

Ambulance Service

Standard Operating Procedures

Version 4.4 – November 2024





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Hamad Medical Corporation

HEALTH • EDUCATION • RESEARCH

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SOP 1: General Administration



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	1.2 UNIFORMS AND PERSONAL HYGIENE
Implementation Date	16 February 2021
Purpose	To provide guidance on HMCAS uniform requirements and presenting an easily identifiable and professional appearance.
Responsibility	ALL HMCAS GROUP STAFF

1. Purpose:

- 1.1 To provide guidance on HMCAS uniform requirements and presenting an easily identifiable and professional appearance.

2. Scope:

- 2.1 This SOP applies to all HMCAS Group Staff.

3. Definitions:

- 3.1 None

4. Procedure:

- 4.1 The HMCAS Executive Director or designee may modify various provisions of the dress code for special functions or specific work assignments.
- 4.2 A high standard of personal appearance and hygiene is imperative to the maintenance of a positive personal, professional, and public image and ensuring appropriate safety standards in the work environment.
- 4.3 Based on the relevant policy, supervisors have the right and duty to judge the appropriateness of the dress and personal hygiene of an employee.
- 4.4 This SOP does not alter the requirement for staff to wear appropriate PPE in hazardous environments as stated in HMCAS [SOP 2.1](#).
- 4.5 All HMCAS staff entitled for uniform shall be issued with the HMCAS approved uniform items according to their duty requirements. Uniform entitlement is non-accumulative and is given per eligibility period or when required (Refer to [Appendix 1.2-1: Uniform Entitlement](#)).
- 4.6 **Uniform Dress Code:**
- 4.6.1 Always wear corporate identification name tag and DHP identification.
- 4.6.2 Wear only the designated colour shirt with shoulder badges and rank epaulettes ([Appendix 1.2-2: Approved Rank Epaulettes](#)).
- 4.6.3 Press uniforms to deliver a neat appearance.
- 4.6.4 Polish and keep boots in a good state of repair.
- 4.6.5 Staff shall remain responsible to care for their uniform and personal issued equipment.
- 4.6.6 Staff should take care of their uniform as guided by the basic uniform care and washing rules ([Appendix 1.2-3: Washing Instructions](#)).
- 4.6.7 Undergarments are provided by the staff and should not be transparent through shirts or trouser. Staff should be guided by HMC HR Policy 3008 (3.7).
- 4.6.8 Socks (black) and sweaters (black or navy blue under sky blue shirt, white under white shirts) is supplied by staff and should be worn according to HMCAS Policy AS 9035 (3.11.8 & 3.11.9).

- 4.6.9 Staff wanting to cover their arms may use navy blue long sleeve T- Shirts under their sky blue HMCAS shirts or a white long sleeve T-Shirt under a white HMCAS shirt purchased at their own cost.
- 4.6.10 Wear uniforms ONLY to and from duty, or whilst operating in an official capacity.
- 4.6.11 Do not wear non-HMCAS approved pins, insignia, or hats.
- 4.6.12 Female staff may wear a dark blue head scarf, neatly tucked in to avoid interference with work performance.
- 4.6.13 Non-clinical staff and senior personnel may wear clothing that is appropriate to the nature of their work. In HMCAS offices, this will usually be business attire (suit, shirt and tie, Qatari national dress, or female equivalent).
- 4.6.14 Administration and Communication staff that perform clinical duties for licensure purposes will be issued two sets of uniform according to their designated duties.
- 4.6.15 Retrieval Service personnel shall wear the following orders of dress:
 - 4.6.15.1 Ground Based Retrieval: AS Uniform.
 - 4.6.15.2 Aero SHAARP: Flight suit with Retrieval Service badge.
 - 4.6.15.3 Commercial on call/Clinical Coordinator: AS Uniform.
 - 4.6.15.4 Commercial Flights: Khaki pants and navy-blue polo shirt without badges or logos.
- 4.6.16 Staff who will cover events should wear their standard/primary uniform.

4.7 **Personal Appearance and Hygiene:**

- 4.7.1 Neatly groom and maintain a conservative hair appearance.
- 4.7.2 Facial hair must not interfere with the appropriate fit of a N95 mask.
- 4.7.3 Draw back hair that is shoulder length or longer.
- 4.7.4 Wash hands with soap and water or use hand gel between patient contacts, upon returning to the work area from the restroom, and after eating.
- 4.7.5 Keep fingernails clean, neatly trimmed, and must always be kept short to prevent injury to clients.
- 4.7.6 Conservative use of cosmetics, fragrances or makeup is permitted.
- 4.7.7 One plain wedding ring is allowed, however, rings with stones, or are not smooth, can compromise the integrity of gloves and should not be worn
- 4.7.8 A wrist watches with a second hand is required.
- 4.7.9 Wearing of loose or dangling jewelry is not permitted.
- 4.7.10 Deodorant or antiperspirant should be used following a daily bath or shower to avoid body odor.
- 4.7.11 Daily oral hygiene (brushing of teeth) is required.

4.8 **Uniform Replacement:**

- 4.8.1 Uniform is supplied to the Staff as listed on [Appendix 1.2-1](#), however full responsibility for maintenance and cleanliness will remain with the wearer, including replacement of damaged/lost uniform.
- 4.8.2 Damaged uniform while performing work duties:

- 4.8.2.1 When uniform needs replacement within the entitlement period; Staff reports the incident to his Immediate Supervisor, submits OVA, waits feedback and approval from the Committee through the Ambulance Service Quality Section to process replacement.
- 4.8.3 Lost, Faded, Ripped or Torn, Physical changes resulting from gaining or losing of weight or Pregnancy:
 - 4.8.3.1 When uniform needs replacement within the entitlement period, Staff submits OVA, waits feedback and approval from the Committee through the Ambulance Service Quality Section to process replacement.
 - 4.8.3.2 This extra uniform can be acquired from the vendor at employee's expense with written authorization from the Management.
- 4.8.4 Lost ID tags/badges must be reported to HMC Security without delay and shall be replaced according to HMC procedure (HR Policy 3010).
- 4.9 **Uniform Requirements for External Services/Institutions:**
 - 4.9.1 All external services/institutions will be accountable to supply uniform to their staff/students scheduled to perform observatory duties in HMCAS that is inline and conforms to the HMCAS operational and safety requirements.
 - 4.9.2 All external services/institutions are liable to supply uniform to their staff/students scheduled to perform clinical observatory duties in the HMCAS vehicles at their own costs which must be conform to safety standards uniformity as required by the HMCAS.
 - 4.9.3 Observers will not be allowed to work on HMCAS vehicles if safety standards are not met and provided to the individual.
- 4.10 **Uniform Request:**
 - 4.10.1 All HMCAS staff shall request uniforms by emailing a Uniform Request Form to the AS Uniform Section at ss-uniform@hamad.qa.
 - 4.10.2 Staff should know their correct uniform sizes. Ensure size is correct before completing the Uniform Request Form. If unsure of size, Staff should try on a sample uniform before submitting a uniform request.
 - 4.10.3 Approval of uniform request is based on eligibility and stock availability.
- 4.11 End of Service/Transfer to other HMC Department
 - 4.11.1 Staff who resigns or transfers to other department within HMC should process End of Service clearance.
 - 4.11.2 Uniform Section issues and verifies the End of Service Uniform Checklist. Staff should return Personal Issued Equipment, Reflective Vest, Life Patrol Helmet, and other items issued within 6 months prior to the date of resignation or transfer. See [Appendix 1.2-5: End of Service Uniform Checklist](#).

5. Attachments:

- 5.1 [Appendix 1.2-1: Uniform Entitlement](#)
- 5.2 [Appendix 1.2-2: HMCAS Rank Insignia](#)
- 5.3 [Appendix 1.2-3: Washing Instructions](#)
- 5.4 [Appendix 1.2-4: Uniform Request Form](#)
- 5.5 [Appendix 1.2-5: End of Service Uniform Checklist](#)

6. References:

6.1 None

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Production
Procedure	1.3 LEAVE AND SHIFT SWOPS
Implementation Date	1 April 2017
Purpose	To outline staff procedures for leave and shift swap requests
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Leave request Procedure:**1.1 Requests for Casual Leave:**

- 1.1.1 Casual leave may be requested after a probation period of 3 months following initial employment date.
- 1.1.2 A maximum of 2 consecutive days leave (3 for Qatari staff) may be requested per leave application.
- 1.1.3 Complete and sign a leave application form (*see attachment*).
- 1.1.4 Request a signature and approval from immediate supervisor.
- 1.1.5 Hand the application form into the scheduling section for final approval.
- 1.1.6 Upon approval, hand in the original document to the administration section.
- 1.1.7 Hand in a copy of the original document to the scheduling section.

1.2 Requests for Annual Leave:

- 1.2.1 Staff are eligible for annual leave after at least 10 months of service.
- 1.2.2 Requests for leave prior to the 10-month period may be granted as unpaid leave at the discretion of the immediate supervisor.
- 1.2.3 Complete online Oracle leave application form.
- 1.2.4 Await automated Oracle approval notification from immediate supervisor.

2. Return from leave:

- 2.1 Complete online Oracle "Return from Leave" application as soon as possible, not exceeding 11 days after returning to duty.
- 2.2 Await automated Oracle approval from immediate supervisor within 3 days.

3. Emergency leave within HMCAS:

- 3.1 In the case of a family or other unforeseen emergency, a request for emergency leave may be made and shall be approved at the discretion of the Senior Operations Manager.

4. Sick Leave:**4.1 Sick whilst on duty:**

- 4.1.1 Inform PTL and immediate supervisor.
- 4.1.2 The PTL shall book the affected unit out of service.
- 4.1.3 Proceed to the staff clinic during office hours or to HGH Accident & Emergency unit outside office hours.
- 4.1.4 Obtain a sick leave form from Romeo or immediate supervisor in order to present to examining doctor.

- 4.1.5 Ensure a completed a sick leave form is returned from the examining doctor at the staff clinic or hospital.
- 4.1.6 At earliest opportunity, present the completed sick leave form to the Staff Clinic for verification stamp.
- 4.1.7 Inform the PTL and the immediate supervisor if sick leave has been granted by the doctor.
- 4.1.8 Upload a copy of the completed sick leave form including Staff Clinic stamp, with the application for sick leave request on the Oracle online system for Immediate Supervisor approval.
- 4.1.9 PTL shall inform Scheduling to get a replacement in place of the affected staff member.
- 4.2 Sick leave before the start of shift: (Minutes before start of shift)
 - 4.2.1 Contact immediate supervisor and inform accordingly.
 - 4.2.2 The immediate supervisor shall inform the scheduling section.
 - 4.2.3 Scheduling shall contact staff for overtime to replace the affected staff member.
 - 4.2.4 Report to the designated hub in uniform.
 - 4.2.5 Follow steps 4.1.3. to 4.1.7
 - 4.2.6 Return home
- 4.3 Sick leave a day before start of shift:
 - 4.3.1 Inform the scheduling section and immediate supervisor.
 - 4.3.2 Proceed to staff clinic and obtain a completed sick leave form signed by the examining doctor.
 - 4.3.3 Upload a copy of the completed sick leave form including Staff Clinic stamp, with the application for sick leave request on the Oracle online system for Immediate Supervisor approval.
 - 4.3.4 Scheduling shall contact staff for overtime to replace the affected staff member.

5. Shift swop procedure:

- 5.1 Inform immediate supervisor on duty of intention to swop a shift.
- 5.2 All requests will be completed electronically using the staff HMCAS Corporate email address.
- 5.3 The form below (Figure 1) is to be completed by both parties prior to the proposed date of the shift swop.
- 5.4 The First Party [Requesting Staff] is to fill in the electronic form with his/her details and rostered shift in the "From" block.
- 5.5 The First Party the emails the electronic form to the Second Party [Accepting Staff] for completion.
- 5.6 The Second Party is to fill in the electronic form with his/her details and rostered shift in the "From" block.
- 5.7 The Second Party is to fill in the electronic form with his/her accepted shift swop in the "To" block.
- 5.8 The completed electronic form is then emailed by the Second Party to the Line Manager for approval and electronic signature.

5.9 Only 3 shift swop requests are permitted per month.



 خدمة الإسعاف
 Ambulance Service

SHIFT CHANGE REQUEST FORM

FIRST PARTY			From		
Name	Corp. No.	Team	Unit	Shift	Date
			To		
			Unit	Shift	Date

SECOND PARTY			From		
Name	Corp. No.	Team	Unit	Shift	Date
			To		
			Unit	Shift	Date

Note:

- * **First Party** (staff requesting the shift change)
- * **Second Party** (staff accepting the shift change)

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	1.5 RED RULES IN AMBULANCE SERVICE
Implementation Date	28 March 2021
Purpose	To identify Red Rules in HMC Ambulance Service which must be followed at all times, without exception.
Responsibility	ALL HMC AMBULANCE SERVICE STAFF

1. Purpose:

- 1.1 To identify Red Rules in HMC Ambulance Service which must be followed at all times, without exception.

2. Scope:

- 2.1 This SOP applies to all HMCAS Group staff who operate a vehicle within the HMCAS fleet.

3. Definitions:

- 3.1 **Red Rules** are used to reduce the probability of harm to patients and employees.
- 3.2 **Red Rules in Ambulance Service** are associated with acts that have the highest level of risk to patient or employee safety.

4. Procedure:

- 4.1 As a result of learnings drawn from the ongoing review of incidents and associated risks related to the work of HMCAS, the following set of explicit, no exception, safety 'Red Rules' have been implemented.
- 4.2 Due to the significant safety implications behind these rules, any contraventions will result in a minimum of three days' pay being withheld, in addition to any related disciplinary action.
- 4.3 **Red Rules:**
- 4.3.1 Never speed above 140 km/h in any location in any HMCAS vehicle. Priority 1 driving excluded.
- 4.3.2 Never drive without a seatbelt.
- 4.3.3 Never talk on, or hold a mobile phone, while driving in any 2-manned unit.
- 4.3.4 Never cross a red light without being on a Priority 1 call.
- 4.3.5 Never work on a public road without wearing a reflective safety jacket/vest.
- 4.4 Red Rule Violation Management Process:
- 4.4.1 Support Service Control Center (SSCC) will send an email with violations recorded on the ACETECH vehicle system to Senior Operations Managers (SOM) each Thursday by 1200 hours.
- 4.4.2 SOMs will review the list to ensure that the concerned vehicle and registered staff was operational when the violation occurred. SOMs must return their findings to HMCAS Quality Department no later than the following Thursday by 1200 hours. If a reply is not received within the timeframe it will be considered that SOMs concur with the recorded violation.
- 4.4.3 Violations confirmed by SOMs, as per para 2, will be sent via the HMCAS Quality Department to the Office of the Executive Director (ED) HMCAS. The list must be sent to the designated person by 1400 hours on Thursday

- 4.4.4 Violations that are contested by SOMs will be sent back to the SSCC for a review of technology or to check if there has been a mistake in the allocation of an individual to a vehicle. Contested violations will be received by SSCC before 1400 hours each Thursday and the SSCC will have 7 calendar days to investigate the query.
- 4.4.5 By Tuesday the following week the office of the Executive Director HMCAS will:
 - 4.4.5.1 Prepare a letter, to be signed by the ED HMCAS, issuing a written warning and notification of 3 days salary deduction in accordance with the Red Rules.
 - 4.4.5.2 Send the warning letter to the relevant AED of the service where the person works. The AED is responsible for speaking to the staff member and issuing the warning letter.
- 4.4.6 For multiple offences the staff member will meet with the Executive Director HMCAS and respective AED. This will be conducted in a timely fashion and a Performance Improvement Plan will be initiated accordingly.
- 4.4.7 In all instances, copies of the following documents will be sent to Corporate HR for inclusion on the staff member's file:
 - 4.4.7.1 Warning Letter
 - 4.4.7.2 Record of meeting between staff member and the AED or AED and ED.
 - 4.4.7.3 Performance Improvement Plan with dates for review.
- 4.4.8 At the end of each calendar month a list of all staff to receive salary deductions is to be prepared, together with a memo from the Ambulance Service Group Chief Executive and sent to the Chief of Human Resources. A copy of this memo is sent to the HR Manager for inclusion on staff files.

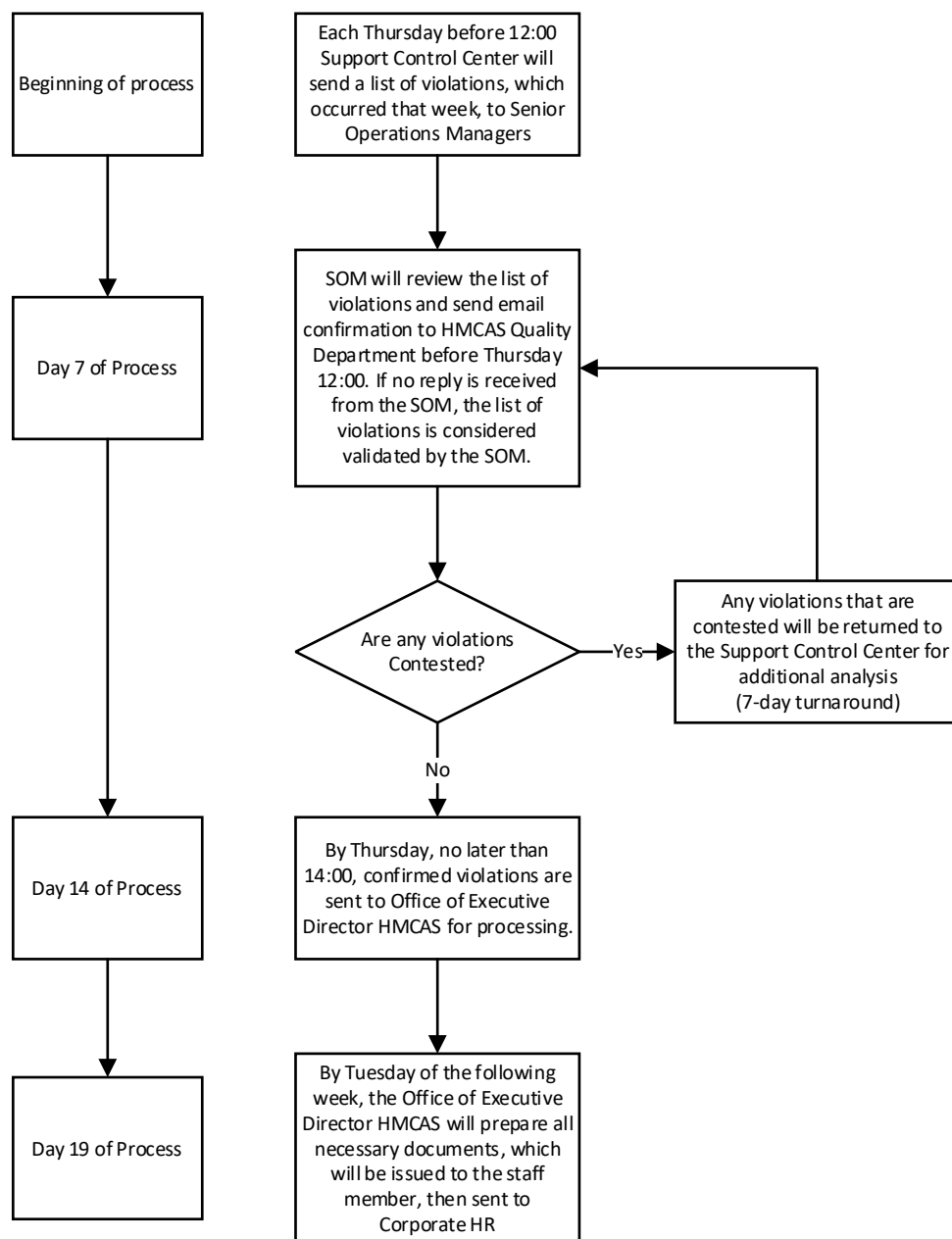
5. Attachments:

- 5.1 [Figure 1: HMCAS Red Rule Violation Management Process](#)

6. References:

- 6.1 None

Figure 1: HMCAS Red Rule Violation Management Process



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Production
Procedure	1.6 SANCTIONS FOR VIOLATING RULES AND REGULATIONS AND ACTS OF NEGLIGENT BEHAVIOR
Implementation Date	1 October 2016
Purpose	To ensure that all operational personnel managed by HMCAS will be sanctioned fairly, appropriately, and equally, when violating regulations or behaving negligently.
Responsibility	ALL HMCAS AMBULANCE SERVICE STAFF

1. General Processes pertaining to Sanctions:

- 1.1 The HMCAS Quality Department, with full support from the relevant operational departments, shall investigate all violations of rules or regulations and acts of negligent behaviour by HMCAS staff.
- 1.2 All investigations shall be initiated by submitting an OVA report as per [SOP 4.9.B](#). After an initial investigation, a more detailed investigation may be recommended.
- 1.3 Staff members or managers directly involved with the incident shall not participate in the investigation.
- 1.4 Upon completion, the investigating team shall present a report with recommendations of sanctions to be imposed.
- 1.5 All reports and recommendations for sanctions shall be reviewed by the HMCAS Executive Team who shall decide the outcome and enforce and implement sanctions as necessary.
- 1.6 Depending on the level of sanction, the Executive Team may elect to engage HMC Corporate HR to impose the requested staff sanction.

2. Levels of sanctions for HMCAS staff:

- 2.1 Additional mandatory training
- 2.2 Verbal warning
- 2.3 Written warning
- 2.4 Written warning combined with suspension from function, role, or duty (including restriction of paid overtime and possible promotion).
- 2.5 Deduction of salary
- 2.6 Suspension from duty
- 2.7 Recommendation of dismissal

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Support Services
Procedure	1.7 AMBULANCES DRIVEN BY NON-MEDICAL PERSONNEL
Implementation Date	01 February 2017
Purpose	To ensure that non-medical personnel is properly guided when driving ambulances to any location across Qatar.
Responsibility	ALL NON-MEDICAL STAFF DRIVING ANY TYPE OF EMERGENCY VEHICLE

1. General Rules:

- 1.1 Vehicle can only be driven by staff holding a Qatari Driving License.
- 1.2 An Ambulance is driven by Non-Medical Staff e.g. Support Services Staff when transporting ambulance to any workshop location across Qatar.
- 1.3 A sign **"NOT IN SERVICE"** must be placed in front and rear side of ambulance and must be visible to the public.

2. Approaching Road Traffic Accident (RTA):

- 2.1 In the event of any Ambulance or Support Vehicle approaching or witnessing incident such as RTA, the Support Services Staff should stop and park the vehicle in a safe location.
- 2.2 The Support Services Staff must quickly assess the scene.
- 2.3 After assessing the scene, immediately call 999.
- 2.4 The Support Services Staff must be familiar with **"Know the Five to Save a Life"**.
- 2.5 The vehicle must remain on scene until the responding Emergency Ambulance arrives.



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Support Services
Procedure	1.8 VEHICLES RUNNING DURING SUMMER SEASON
Implementation Date	01 October 2016
Purpose	To ensure vehicle safety maintenance are considered when temperatures begin to rise in Qatar.
Responsibility	ALL HMC AMBULANCE SERVICE STAFF

1. Basic Safety Precautions:**1.1 Daily vehicle check using Vehicle Safety Checklist.**

- 1.1.1 Ensure fluids, emergency system, lights, mirrors, air conditioning, tires, brake system and all safety items are visually checked before driving the vehicle.
- 1.1.2 Staff must report to Production Supervisor or Fleet Section, if any issues are found.

1.2 Tire Check:

- 1.2.1 Regularly check tires and ensure proper tire pressure.
- 1.2.2 As temperatures rise, under or over inflated tires may put you at risk of tire bursts and blowouts.
- 1.2.3 Staff must report to Production Supervisor or Fleet Section, if any damage or excessive wear to tires is noticed.
- 1.2.4 All tires must be at the same brand on each axle assembly.

1.3 Air Conditioning (AC) System:

- 1.3.1 Check AC system to ensure that it is giving correct amount of air flow.
- 1.3.2 When switching AC on; give ample time for AC to cool down.
- 1.3.3 Pay attention to any strange noises or odours.
- 1.3.4 Staff must report to Production Supervisor or Fleet Section if AC is not working correctly.

2. Shoreline Facility:**2.1 Emergency and Non-Emergency Ambulances:**

- 2.1.1 All ambulances must not be left running at any location unless no shoreline available, the driver must position the gear in the Park position and engage the hand break to activate the Eco Idle mode.
- 2.1.2 All ambulances must be plugged into the shoreline when on standby at a location when shoreline is available. Vehicle engine should be switched off.
- 2.1.3 Engine must remained off when connected to the shoreline so the safety ejection system can be activated when switching on the engine.
- 2.1.4 Staff must report immediately to Production Supervisor if shoreline is defective, damaged or any related issue.
- 2.1.5 All units ending their shift at AS parking areas where shoreline cables are available must ensure to connect their vehicles for charging.
- 2.1.6 Before moving make sure the shoreline is secured in place to avoid driving over it.

2.2 **Marked Prado:**

- 2.2.1 All marked Prado carrying medical equipment can be kept with the engine running at the standby location.
- 2.2.2 All marked Prado must allow ambulances to have access to the shoreline when standby at any location where shoreline is available.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Support Services
Procedure	1.9 UNMARKED RAPID RESPONSE VEHICLE (RRV) ALLOCATION
Implementation Date	01 November 2016
Purpose	To ensure that Rapid Response Vehicle is allocated as required
Responsibility	ALL STAFF UTILIZING UNMARKED RRV

1. General Rules:

- 1.1 Vehicle can only be driven by staff holding a Qatari Driving License or other nationality license approved by the Traffic Department.
- 1.2 A vehicle is allocated to a staff member as part of an emergency response on-call rota.
- 1.3 No vehicle will be permanently allocated to any member of staff without the written approval of the Group CEO.
- 1.4 In the event of other operational requirements, vehicle can be taken from staff.
- 1.5 If Staff goes on leave for five consecutive days and above, it is his/her responsibility to return the vehicle to the Fleet Section.
- 1.6 Fleet Section reallocates vehicle as required.

2. On-Call Vehicle Allocation:

- 2.1 On-Call Staff is allocated with an appropriate vehicle for his/her duty.
- 2.2 Depending upon the on-call profile, vehicle may be shared between staff.
- 2.3 If Staff changes his/her on-call status, it is his/her responsibility to inform the Production Team Leader and FMS Coordinator.
- 2.4 On-Call rota is updated and circulated by Sunday 10:00 of every week.
- 2.5 FMS updated with vehicle and driver details by 11:00 each Sunday.
- 2.6 FMS Coordinator informs PTL by email on updated on-call vehicle list by 11:15 each Sunday.

3. Documentation Responsibilities:

- 3.1 All vehicle check sheets to be completed as required.
- 3.2 Staff is responsible for ensuring that refueling takes place at an approved petrol station.
- 3.3 Petty cash claims for fuel will not be accepted without written approval of the Executive Director.
- 3.4 For accident reporting, it is the Staff responsibility to report to Fleet Section as soon as practical after the accident takes place.

4. Reference:

- 4.1 Vehicle Usage Policy

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	1.11 PHOTO/VIDEO RECORDING
Implementation Date	11 September 2018
Purpose	Actions to be taken by HMCAS should a member of the public, patient, or a patient's family member video/photograph and/or record voice of the staff without their permission
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Introduction:

- 1.1 Social media is an established and accepted part of modern society. However, it can be intrusive to patient care, privacy, and dignity. HMCAS have a duty to protect patient's dignity at all costs. However, staff are reminded to always act in a professional and courteous manner when challenging members of the public and/or family who are filming patient care as those who see the subsequently shared footage will lack the situational awareness of the events and will only observe the behaviour of HMCAS staff.

2. Recording of a patient with their consent:

- 2.1 Where patients are happy to be recorded then this is acceptable behavior.
 - 2.1.1 This should be documented on your ePCR.
- 2.2 Staff members have the right to refuse being filmed. See point 4.

3. Recording of a patient without the patient permission:

- 3.1 Respectfully request those recording clinical care to stop.
- 3.2 Should they refuse:
 - 3.2.1 Shield patient from being recorded.
 - 3.2.2 Update National Command Centre.
 - 3.2.2.1 Ask the patient if they wish to have police requested.
 - 3.2.3 Document incident on the ePCR.
 - 3.2.4 Avoid conflict as this will escalate events.
 - 3.2.5 Remain professional, polite, and non-confrontational.

4. Staff not wishing to be recorded:

- 4.1 Should staff note that they are being recorded and they wish it to stop, but it involves the patient filming their own care then:
 - 4.1.1 Politely and professionally request that recording be stopped.
 - 4.1.2 If able, without affecting patient care, move away from the immediate environment e.g. sit out of shot if the patient is taking self-portraits.
 - 4.1.3 If the patient and/or the person recording the incident refuse to stop filming:
 - 4.1.3.1 Remain professional at all times.
 - 4.1.3.2 Report incident to National Command Centre and direct supervisor.
 - 4.1.3.3 Continue managing the patient in an appropriate manner.
 - 4.1.3.4 Avoid escalating the incident.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Group: Support Services
Procedure	1.12 WOQOD E-FUELING SYSTEM
Implementation Date	19 November 2019
Purpose	To ensure that all staff driving any type of vehicle is properly guided on the WOQODe Fueling system.
Responsibility	ALL HMCAS STAFF

1. General Information:

- 1.1 The WOQOD RFID chip is installed and activated in all Ambulance Service vehicles.
- 1.2 The reader automatically recognizes the required type and quantity of the fuel.
- 1.3 Staff are responsible for ensuring that refueling takes place only at WOQOD station.

2. Use of Fuel Voucher:

- 2.1 The fuel voucher is no longer accepted in any WOQOD station except on the following circumstances:
 - 2.1.1 Unregistered vehicle used as replacement vehicle.
 - 2.1.2 WOQOD e-System failure; and
 - 2.1.3 Refueling of Command and Control Vehicle generators.
- 2.2 The fuel voucher that can be used in those circumstances should ONLY be from the book voucher issued to every Department.
- 2.3 The voucher number that is being issued must be logged and submitted to Support Services Procurement Section through ASContracts@hamad.qa on monthly basis.

3. WOQOD e-System Failure:

- 3.1 If the WOQOD chip is damaged and not working, staff must immediately inform his immediate supervisor.
- 3.2 The supervisor ensures that a single voucher is delivered immediately to the vehicle (at the fuel station).
- 3.3 The supervisor reports to Fleet Section through SS-Fleet@hamad.qa with the vehicle registration number and full details of the problem.
- 3.4 The Fleet Section will liaise with WOQOD for rectification.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	1.13 MANAGEMENT NOTIFICATION PROCESS
Implementation Date	25 February 2020
Purpose	To provide guidelines and a set of rules for the Management Notification process in the Ambulance Service.
Responsibility	HMCAS MANAGEMENT STAFF

1. MAJOR INCIDENT ON CALL WEEKLY ROSTER will be sent via email to all HMCAS Management staff every Sunday.
2. HMCAS Managers are responsible for posting the roster at NCC, HCC, and all HUBS.
3. NCC On-Duty Officers must use the Incident of Interest Group on Microsoft Teams to send out Notifications.
 - 3.1 All information shared on the Incident of Interest Group is confidential.
 - 3.2 This is not a primary activation channel for any such incidents. Fact 24 will continue to be used as the primary activation channel.
 - 3.3 All Incidents of Interest should be posted in the Incidents of Interest Group by the NCC On-Duty Officer, including a brief description of the incident and NAJM pin location.
 - 3.4 Appropriate updates should also be posted.
 - 3.5 Information will be shared in real time at all hours of the day and night. If you need to not be disturbed, it is your responsibility to temporarily mute the notifications.
 - 3.6 In the interest of minimizing necessary disturbances, please do not 'acknowledge' messages (copy, noted, etc. should not be used).
 - 3.7 If you are instructed to take an action or provided information, report only when the action is completed or with the information requested.
4. Operation Command Desk and Clinical Desk must be informed by NCC On-Duty Officer for any Incident listed in Point 7.5.
5. Operation Command Desk On-Duty is the person to communicate any related information from scene to Clinical Desk after being briefed by On-Duty NCC Officer or Distribution Team Leader.
6. The Incidents of Interest listed in points 7.1 to 7.14 require the following actions by the NCC On-Duty Officer:
 - 6.1 Send notification to Incidents of Interest Group in Microsoft Teams.
 - 6.2 Inform the Operation Officers On-Duty.
 - 6.3 Activate On-Call Silver Command
 - 6.3.1 Silver Command at his discretion, choose to notify Gold Command.
 - 6.4 When required, NCC On-Duty Officer will ensure the Team Leaders have notified their In-Line Managers respectively.
7. Incidents of Interest include, but are not limited to:
 - 7.1 Incidents where on initial call are greater than 5 patients in total (**10 patients****).
 - 7.2 Incidents where there are more than 3 patients classified as either Yellow or Red (**5 patients****).
 - 7.3 Incidents with prolonged scene times, i.e. >60 minutes.

- 7.4 Significant Mechanism of Injury (i.e. shootings, explosive related, sign of child abuse, sign of criminal activity and chemical/radiological spills).
 - 7.5 Incidents which **attract media interest**** (incidents in very crowded areas, incidents of public unrest, significant injury, or illness of a patient involved in a major sporting or entertainment event).
 - 7.6 Incidents which involve HMCAS staff (i.e. any on duty ambulance resource collisions with other road users and/or where there are injuries).
 - 7.7 Incidents which may **reflect negatively on HMC****
 - 7.8 Incidents occurring at sensitive locations (Ministry buildings, military installations, embassies, AUAB, DIA, HIA).
 - 7.9 Any airport incident involving whole aircraft, life threatening injury or illness, as well as any potential cases of infection with potential public health consequences.
 - 7.10 Any VIP, VVIP case
 - 7.11 Activation of Major Incident Response (MIR) Plan
 - 7.12 Any significant service failure (in the field or in the control room).
 - 7.13 Other incidents may be included at the discretion of the Operation Officer On-Duty at the NCC.
 - 7.14 ****Incidents that should be escalated by Silver Communication to Silver Command & Gold Command as listed on the Major Incident On-Call Roster.**
- 8. If demand exceeds resource capability, the On-Duty NCC Officer will escalate to Gold Command as listed on the Major Incident On-Call Roster.
 - 9. **Activation of Level 0** will be via FACT 24 as a standard
 - 10. **Activation of Level 1** will be via FACT 24 as a standard
 - 11. **Activation of Level 2** will be via FACT 24 as a standard

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Staff
Procedure	1.14 ABSENTEEISM MANAGEMENT
Implementation Date	01 September 2020
Responsibility	ALL HMCAS STAFF

1. Purpose:

- 1.1 To manage attendance within HMCAS in order to maintain operational readiness, staff health and wellness, ensure staff are fit to care for patients, and to minimize the impact of sick leave absence on work colleagues.

2. Scope:

- 2.1 All HMCAS Staff.

3. Definitions:

- 3.1 **The Bradford Factor:** a trigger tool used to identify patterns of absence related to sickness.
- 3.2 **Bradford Factor Score:** a formula used to calculate a numeric score based on the number of unrelated absence periods (S) squared, multiplied by the total number of days absent (D), equals the Bradford Score (B). The formula is $S^2 \times D = B$.
- 3.3 **Bradford Factor Table:** a matrix with the calculated scores based on the Bradford Factor formula.

4. Procedure:

- 4.1 The Ambulance Service will be utilizing the Bradford Factor Score as a trigger tool to assist Supervisors in identifying staff that require more comprehensive support to reduce their sick leave.
- 4.2 Following the issuance of the previous quarter's sickness report, the supervisor will note the following:
- 4.2.1 Number of separate occasions the staff member has been absent due to sickness.
- 4.2.2 Total number of days sick during the period.
- 4.3 Using the Bradford Factor Table, the supervisor will then determine the Bradford Factor Score and corresponding colour attributable of the staff member.
- 4.4 The score and colour dictate which stage of the management program should be followed in addressing the staff member's absenteeism due to sickness.
- 4.5 **Special considerations related to staff on light duty:**
- 4.5.1 All decisions to place a staff member on light duties will be supported by a report from the Staff Clinic. The report will clearly indicate the start and review date of the light duty period. The report will be shared with the Senior Operations Manager and the Assistant Executive Director for Group Business Services.
- 4.5.2 During a period of light duty, all staff members will/will not be deployed in accordance with the guidance from the Staff Clinic. Furthermore, during the period of light duties, the staff member is not eligible for overtime, nor promotion into any acting position.

The staff member will remain an employee of the cost centre that they are substantively employed under.

- 4.5.3 Upon reaching the light duty review date, the staff member will return to the Staff Clinic for follow up evaluation. The resulting report and decision on whether they are fit to practice in their substantive job will be decided. If it is decided that the staff member is unable to practice as their substantive job, then they will be transferred to an administrative role depending on vacancy and the recruitment selection process by the recruiting department: their pay and benefits may be affected.

5. Attachments:

- 5.1 [Table 1: Bradford Factor Grid](#)
- 5.2 [Table 2: Summary of Actions](#)
- 5.3 [Appendix 1: Green](#)
- 5.4 [Appendix 2: Yellow](#)
- 5.5 [Appendix 3: Orange](#)
- 5.6 [Appendix 4: Red](#)
- 5.7 [Appendix 5: Purple](#)

6. References:

- 6.1 None

Table 1: Bradford Factor Grid

		Number of Separate Occasions of Sickness Absence									
		1	2	3	4	5	6	7	8	9	10
Total Days of Sickness Absence during the Quarter	1	1	4	9	16	25	36	49	64	81	100
	2	2	8	18	32	50	72	98	128	162	200
	3	3	12	27	48	75	108	147	192	243	300
	4	4	16	36	64	100	144	196	256	324	400
	5	5	20	45	80	125	180	245	320	405	500
	6	6	24	54	96	150	216	294	384	486	600
	7	7	28	63	112	175	252	343	448	567	700
	8	8	32	72	128	200	288	392	512	648	800
	9	9	36	81	144	225	324	441	576	729	900
	10	10	40	90	160	250	360	490	640	810	1000
	11	11	44	99	176	275	396	539	704	891	1100
	12	12	48	108	192	300	432	588	768	972	1200
	13	13	52	117	208	325	468	637	832	1053	1300
	14	14	56	126	224	350	504	686	896	1134	1400
	15	15	60	135	240	375	540	735	960	1215	1500
	16	16	64	144	256	400	576	784	1024	1296	1600
	17	17	68	153	272	425	612	833	1088	1377	1700
	18	18	72	162	288	450	648	882	1152	1458	1800
	19	19	76	171	304	475	684	931	1216	1539	1900
	20	20	80	180	320	500	720	980	1280	1620	2000
	21	21	84	189	336	525	756	1029	1344	1701	2100
	22	22	88	198	352	550	792	1078	1408	1782	2200
	23	23	92	207	368	575	828	1127	1472	1863	2300
	24	24	96	216	384	600	864	1176	1536	1944	2400
	25	25	100	225	400	625	900	1225	1600	2025	2500
	26	26	104	234	416	650	936	1274	1664	2106	2600
	27	27	108	243	432	675	972	1323	1728	2187	2700
	28	28	112	252	448	700	1008	1372	1792	2268	2800

Table 2: Summary of Actions

The summary of actions that the Supervisor should take are shown below, a full set of instructions and guidance are for each colour are available in the appendices of this document.

Action	Green 0-49	Yellow 50-124	Orange 125-399	Red 400-649	Purple 650+
Informal Conversation with Supervisor	✓				
Formal meeting with Supervisor		✓	✓	✓	
Formal meeting with Senior Operations Manager			✓	✓	✓
Formal meeting with Assistant Executive Director				✓	✓
Formal meeting with Executive Director					✓
Referral to Staff Clinic		If required	✓	✓	
Meeting recorded and copy placed on staff member's HR file		✓	✓	✓	✓
Formal referral to HR requesting review by HR				✓	
Verbal Warning (where indicated)		✓			
1 st Written Warning (where indicated)			✓	✓	
2 nd Written Warning (where indicated)				✓	
Referral to HR					✓

Appendix 1: Green

This is the first stage of the process and is intended to be informal and supportive. Most staff who enter this stage are unlikely to progress to any further.

Response Structure	Actions
Meeting Structure	Informal meeting between the Supervisor and staff member to discuss their sickness absence over the last three months.
Meeting Agenda	1. Reason/s for the sickness absence? 2. Is sickness likely to occur again? 3. What is the staff member doing to prevent further sickness absence? 4. What can HMCAS do to help the staff member to not take sick leave?
Is Staff Member able to continue to undertake overtime?	Yes
Actions During the Meeting?	Listen, discuss and support
Further Actions?	1. If the staff member has reported that they have regular family commitments (i.e. family member has regular hospital appointments) that have caused them to take sick leave, please consider looking at ways to revise their schedule in accordance with those appointments. 2. If the staff member has reported a chronic or acute condition and is waiting for a hospital appointment, please consider a referral to Staff Clinic (is discussion with your Operations Officer) to request their assistance in expediting the appointment. 3. Inform the staff member what will happen if their sickness absenteeism continues
Warning Letter	No

Appendix 2: Yellow

Response Structure	Actions
Meeting Structure	Formal meeting with the staff member to discuss their sickness absence over the last three months (or further back as required), with the following staff in attendance: Operations Officer
Meeting Agenda	- Reason/s for the sickness absence? - Is sickness likely to occur again? - What is the staff member doing to stop further sickness absence? - What can HMCAS do to help the staff member to not take sick leave? - Actions that will be taken as per the SOP If the staff member's sickness absence has been previously triggered response from this SOP, that should also be discussed at this meeting
Is Staff Member able to continue to undertake overtime?	No, not for the next three months
Actions during the meeting?	- Write notes of the areas discussed and staff members responses. This file note should be kept by the Supervisor with a copy placed in the staff members HMCAS HR file. - Inform the staff member during the meeting that they will receive a verbal warning letter in accordance with the SOP. - If the staff member has reported that they have regular family commitments (i.e. family member has regular hospital appointments) that have caused them to take sick leave, please consider looking at ways to revise their schedule in accordance with those appointments. - If the staff member has reported a chronic or acute condition and is waiting for a hospital appointment, please consider a referral to Staff Clinic (is discussion with your Operations Officer) to request their assistance in expediting the appointment. - Inform the staff member what is likely to happen if their sickness absenteeism continues (orange stage in the guidance)
Referral to HR?	No
Warning letter (where indicated)	If it is proven that there has been abuse of the sick leave process, staff member is issued a verbal warning letter in accordance with the SOP. A copy of this letter should be sent to Corporate HR to place in the staff members HR file.
Further Actions?	A formal copy of meeting notes to be placed on the staff member's HMCAS HR file together with a copy of the verbal warning letter (if indicated).

Appendix 3: Orange

Response Structure	Actions
Meeting Structure	Formal meeting with the staff member to discuss their sickness absence over the last three months (or further back as required), with the following staff in attendance: • Operations Officer • Senior Operations Manager
Meeting Agenda	1. Reason/s for the sickness absence? 2. Is sickness likely to occur again? 3. What is the staff member doing to stop further sickness absence? 4. What can HMCAS do to help the staff member to not take sick leave? 5. Actions that will be taken as per the SOP If the staff member's sickness absence has been previously triggered response from this SOP, that should also be discussed at this meeting
Is Staff Member able to continue to undertake overtime?	No, not for the next three months.
Actions during the meeting?	1. Write notes of the areas discussed and staff members responses. This file note should be kept by the Supervisor with a copy placed in the staff members HMCAS HR file. 2. Inform the staff member during the meeting that they will receive a written warning letter in accordance with the SOP. 3. Inform the staff member that they will be referred to the Staff Clinic for a fitness to practice assessment and if found that they are not fit to practice in their substantive clinical role, they may be transferred to an administration grade under HR Law employment contract. 4. Inform the staff member what is likely to happen if their sickness absenteeism continues (red stage in the guidance).
Referral to Staff Clinic	Yes, request a fitness to practice assessment for the staff member.
Referral to HR	No
Warning letter (where indicated)	If it is proven that there has been abuse of the sick leave process, staff member is issued a 1 st written warning letter in accordance with the SOP. A copy of this letter should be sent to Corporate HR to place in the staff members HR file.
Further Actions?	1. A formal copy of meeting notes to be placed on the staff member's HMCAS HR file together with a copy of the written warning letter. 2. On receipt of the fitness to practice assessment, diarize a meeting with the Senior Operations Manager to discuss next steps and then meet with the staff member to explain the outcome. If the fitness to practice assessment confirms that the staff member is not fit to continue in their substantive role, refer the case to the AED Group Business Services for transfer to HR law.

Appendix 4: Red

Response Structure	Actions
Meeting Structure	Formal meeting with the staff member to discuss their sickness absence over the last three months (or further back as required)., with the following staff in attendance: • Operations Officer • Senior Operations Manager • Assistant Executive Director • AED Group Business Services
Meeting Agenda	1. Reason/s for the continued sickness absence? 2. Review previous actions, meetings and reviews triggered by sickness absence. 3. Is sickness likely to occur again? 4. What is the staff member doing to stop further sickness absence? 5. What can HMCAS do to help the staff member to not take sick leave?
Is Staff Member able to continue to undertake overtime?	No, not for the next six months
Actions During the Meeting?	1. Write notes of the areas discussed and staff members responses. This file note should be kept by the Supervisor with a copy placed in the staff members HMCAS HR file. 2. Inform the staff member during the meeting that they will receive a 2nd written warning letter in accordance with the SOP. 3. If not already undertaken, inform the staff member that they will be referred to the Staff Clinic for a fitness to practice assessment and if found that they are not fit to practice in their substantive clinical role, they will be transferred to an administration grade under HR Law employment contract. 4. Inform the staff member what is likely to happen if their sickness absenteeism continues (purple stage in the guidance).
Warning letter (where indicated)	If it is proven that there has been continued abuse of the sick leave process, a 2 nd written warning letter will be issued, a copy of this letter should be sent to Corporate HR to place in the staff members HR file and a copy kept on their HMCAS HR file.
Referral to Staff Clinic	Yes, request a fitness to practice assessment for the staff member.
Referral to HR?	Yes, formal request to the CHRO by the CEO for formal investigation.
Further Actions?	1. A formal copy of meeting notes to be placed on the staff member's HMCAS HR file together with a copy of the written warning letter. 2. Case discussed with the AED Group Business Services, to agree request for HR Formal Review. 3. A formal copy of meeting notes to be placed on the staff member's HMCAS HR file. 4. On receipt of the fitness to practice assessment, diarize a meeting with the Senior Operations Manager and AED to discuss next steps and then meet with the staff member to explain the outcome.

Appendix 5: Purple

Response Structure	Actions
Meeting Structure	Formal meeting with the staff member to discuss their sickness absence over the last three months (or further back as required)., with the following staff in attendance: • Operations Officer • Senior Operations Manager • Assistant Executive Director • Executive Director • AED Group Business Services • HR Manager
Meeting Agenda	1. Reason/s for the continued sickness absence? 2. Review previous actions, meetings and reviews triggered by sickness absence.
Is Staff Member able to undertake overtime?	No
Actions During the Meeting?	1. Write detailed notes of the areas discussed and staff members responses. Include in the notes all previous action taken regarding sickness absence and include evidence of this from Oracle. This file note should be kept by the Supervisor with a copy placed in the staff members HMCAS HR file. 2. Inform the staff member during the meeting that they are being recommended to Ambulance Service Group Chief Executive Officer for review.
Referral to Staff Clinic	No
Referral to HR?	Yes, formal request to the CHRO by the CEO for review.
Further Actions?	1. A formal copy of meeting notes to be placed on the staff member's HMCAS HR file together with a copy of the written warning letter, if required. 2. If indicated, ED to discuss case with CEO to agree request for employment contract termination. If agreed, the AED Group Business Services will prepare the documentation to support the CEO's letter to the Chief of Human Resources.

On referral to the CHRO recommending contract termination, this process will then be managed by the Chief Executive's office to full conclusion.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	1.15 PERFORMANCE IMPROVEMENT PROCESS
Implementation Date	28 March 2021
Responsibility	ALL HMCAS GROUP STAFF

1. Purpose:

- 1.1 To describe how the Performance Improvement Process will be implemented in the Ambulance Service.

2. Scope:

- 2.1 Applicable to all staff, clinical and non-clinical, except physicians, within the Ambulance Service.

3. Definitions:

- 3.1 **Poor Performance:** Where the level of job performance does not meet the requirements and standards described in the Job Description, and/or where the employee has received an annual performance evaluation score below Meets Expectations/Good.
- 3.2 **SMART Objectives:** Specific, Measurable, Achievable, Realistic, and Timebound.

4. Procedure:

- 4.1 Managers and supervisors will utilize this process for addressing poor performance of HMCAS staff.
- 4.2 Where persistent poor performance is noted, this SOP should be applied to assist managers in order to support staff and address concerns.
- 4.3 [Table 1](#) describes a 3-step process, starting with a face-to-face meeting between the immediate supervisor and employee, where an action plan is agreed and documented. SMART objectives must be utilized.
- 4.4 If the desired improvement is achieved, then no further action may be required. If improvement is not achieved, managers should move to Step 2, which will require further supervision and support.
- 4.5 If the desired outcome is still not achieved, Step 3 will require the engagement of HR to review the documentation and consider the individuals future employment.
- 4.6 Managers should ensure that sufficient supervision and support is available for staff to achieve the desired outcome, and that objectives are clear. It is recommended that employees meet with their immediate supervisor weekly to monitor performance, and provide feedback as needed.
- 4.7 This SOP is linked to the appraisal process, and episodes of poor performance must be reflected in the appraisal grading and comments.

5. Attachments:

- 5.1 [Table 1: Guidelines for Managing Poor Performance](#)
- 5.2 [Figure 1: Performance Improvement Plan Template](#)

6. References:

- 6.1 HR 3020: Employee Performance Improvement (Non-Physician Staff)
- 6.2 HR 3049: Performance Management, Appraisal and Evaluation

Table 1: Guidelines for Managing Poor Performance

STEP	PROCESS	
Step 1: Initial Notification	Initiate Process:	Face-to-face meeting between employee and immediate supervisor to discuss the performance shortfall and the background to why it may have occurred.
	Action	Corrective Action documented with recommended timeframe of 1-3 months.
	If there is successful improvement in performance:	Acknowledgement memo to employee. Meet the employee in person to acknowledge the improvement and encourage ongoing high levels of work. Advise if there is a repeat occurrence of unsatisfactory performance for the same or different reason, the process may resume at the next step as a continuous process.
	If no successful improvement occurs:	Go to Step Two
Step 2: Second Notification	Initiate Process:	Face-to-face meeting between employee and immediate supervisor to discuss the performance shortfall and the background to why improvement has not occurred. Inform employee that failure to improve performance may result in disciplinary action through HR.
	Action	Corrective Action documented. Recommended timeframe of 2 months.
	If there is successful improvement in performance:	Acknowledgement memo to employee. Advise if there is a repeat occurrence of unsatisfactory performance for the same or different reason, counseling may resume at the next step as a continuous process.
	If no successful improvement occurs:	Go to Step Three
Step 3: Third Notification	Department shall forward the issue with all supporting documents to HR. HR in coordination with Legal Department (if applicable) shall verify and finalize the case in accordance with HR Management Law (15) 2016 and HMC By Laws.	

Figure 1: Performance Improvement Plan Template

PERFORMANCE IMPROVEMENT PLAN

Start of Plan Date:		Supervisor's Name:		Employees Name:	
		Designation:		Designation:	
		Corporation Number:		Corporation Number:	
		Signature:		Signature:	
End of Plan Date:		Supervisor's Signature:		Employees Signature:	
Start Date	Performance to be Improved	Expected Results (SMART Objective/s)	Activities / Tasks	Monitoring Frequency	Review / End Date
Support required/in place to help the employee achieve the expected results					
Actual Results (Performance in relation to Expected Results) and any further action					
Any additional information					

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	1.16 HMCAS PROCUREMENT AND CONTRACT PROCESS
Implementation Date	09 April 2020
Purpose	To familiarize department employees with department purchasing practices.
Responsibility	ALL HMCAS MANAGERS AND EXECUTIVES

1. Purpose:

- 1.1 The purpose of this document, hereafter referenced as a Procurement SOP, is to familiarize department employees (whether an Ambulance Service Procurement Coordinator, SCM buyer or an AS staff) with department purchasing practices in a clear, common understanding of goals, benefits and policies as well as what the department expects with regard to performance and conduct. The document provides an overall description of the department's purchasing program.

2. Responsibility:

- 2.1 Ambulance Service Procurement Section is responsible for developing their procurement plans in cooperation with the HMC Supply Chain Management and the Ambulance Service.
- 2.1.1 Project/Procurement Coordinator is the personnel with access rights to the HMC's ERP System (Oracle). They have the relevant authority to assign a requisition for a specific Cost Centre or a range of Cost Centers, as appropriate. They are responsible for coordinating all procurement related activities, including contacting the supplier (through SCM) and communicating the receipt/update to the specific end-user (authorized requestor or a person appointed on his/her behalf).
- 2.1.2 Senior Supervisor is the personnel who closely monitor/assist project coordinator's assignment. He is authorized to assign project coordinator for PR Creation. He manages HMC Ambulance Service's contracts. He is responsible for arranging evaluation meetings with executives for high value tenders.
- 2.1.3 Project Manager – Procurement and Tender is the personnel responsible for managing the core business operations of the procurement team and all its purchases. He is responsible for delegating tasks to the entire team and for any escalation as when required.

3. Guideline/Procedure:**3.1 HMCAS Standard Procurement Process:**

- 3.1.1 The Authorized Requestor (Manager/Executive) must submit a general request by email to the Procurement Section. The requestor must make sure AS Stores/Facilities Section does not have the same item in their stock. Authorized Requestor is responsible for providing accurate specifications to the AS Procurement Section.
- 3.1.2 The request can be clubbed with other requests and clearly stated in the Oracle System.
- 3.1.3 Procurement Coordinator creates the Oracle PR (follows the iProcurement Work Instructions).
- 3.1.4 IT Contracts Management, projects, applications, software/hardware equipment requests may be subjected to further approval cycle, both internal to HMC (HICT portfolio) and external to Ministry of Transport and Communications (MOTC) whether your request is OPEX or CAPEX.

- 3.1.4.1 The following documentation is required for all IT requests:
 - 3.1.4.1.1 IT change request
 - 3.1.4.1.2 Business Case
 - 3.1.4.1.3 To be submitted to HMCAS IT delivery team, who will review before submitting to HICT portfolio.
- 3.1.4.2 The following documentation is required for all IT requests above 500,000 QAR.
 - 3.1.4.2.1 IT change request
 - 3.1.4.2.2 Business Case
 - 3.1.4.2.3 Request for a proposal (RFP)
 - 3.1.4.2.4 a set of MOTC documentation
 - 3.1.4.2.5 To be submitted to HMCAS IT delivery team, who will review before submitting to HICT portfolio.

3.2 **HMCAS Group Oracle PR Approval Process:**

- 3.2.1 Once approved by the Finance Business Partner through email, then the Incomplete PR is submitted in the Oracle system by the Procurement Coordinator. While submitting the PR, a Manager/AED is added as “First Approver”.
- 3.2.2 The chain of approvals for PR value is as follows Requestor > Project Manager - Procurement > HMC Finance > AED (First Approver) > ED > CEO (Final Approver)
- 3.2.3 The Approver can “Request Information” from anyone in HMC through the Oracle system by clicking the search tab and typing in the relevant name.
- 3.2.4 The approver must log into the Oracle system, as they would for the HR Self Service. On the home page they will see all pending notifications with regards to the approvals of PR/requested information.
- 3.2.5 Click on the notification and the approver can choose to approve/reject or request information to anyone in HMC.
- 3.2.6 The Final Approver (EDs) should confirm the following before approving the PRs:
 - 3.2.6.1 Is the quantity requested in line with the current/agreed consumption rates?
 - 3.2.6.2 Is an alternative available or has been considered if the price per unit seems excessive?
 - 3.2.6.3 Is the spend aligned with the Service Delivery Plan (SDP)?
 - 3.2.6.4 SDP includes the ten-year CAPEX budget and the yearly OPEX budget sent to the respective EDs for their respective departments.
 - 3.2.6.5 If yes – approve PR
 - 3.2.6.6 The approver can “Request Information” from Finance Business Partner (Damien Britton) to ensure the coding.
 - 3.2.6.7 If the spend is outside the agreed SDP/budget, he/she must “Request Information” from Department Chief to agree on additional spend.
- 3.2.7 Once approved, the PR is sent to SCM for further process. If it requires specifications from Engineering/Bio-Engineering, then it must be sent to the relevant department before forwarding it to SCM.

- 3.2.8 If PR amount is QAR 200,000 or above, please send it to Minor Tender Committee and if it is above QAR 1,000,000 it is sent to the Tender Committee Secretariat along with the appropriate specifications and RFP.
- 3.2.9 For any PR above QAR 200,000, a Tender Request (TR) has to be created by the AS Procurement Coordinator.
- 3.2.10 SCM/TCS Manager will need to assign a Buyer for processing the request. This must be followed-up by the AS Procurement Coordinator to avoid delay.
- 3.2.11 Buyer will send out RFQs to suppliers/service providers and closing date is set. Procurement Coordinator communicates all updates to the end-user.
- 3.2.12 Quotes are received by the Buyer on the closing date and are provided to AS by way of evaluation memo.
- 3.2.13 Procurement Coordinator documents and files the evaluation memo before forwarding it to the end-user for their evaluation.
- 3.2.14 Acceptance memo/Rejection memo is created as a reply to the evaluation memo. Price, quality, and delivery time should be taken into consideration while evaluating. If approval is required from other departments, it must be addressed to the relevant department and then forwarded to SCM/TCS.
- 3.2.15 The Buyer will create a PO based on the acceptance memo or go for re-quotation depending on AS Procurement Section's reply.
- 3.2.16 For some TCS requests, the Buyer might request for considerations from HMC Internal Audit and HMC Legal Affairs departments before releasing the PO.
- 3.2.17 The PO will be released, and the vendor/service provider will collect it from SCM.
- 3.2.18 Any changes in PO, a memo is sent to Head of Expediting Section detailing the change and its justification.
- 3.3 **HMCAS Procurement Process (for inks, cartridges, toners, water coolers, water coupons, business cards, catering):**
 - 3.3.1 The Authorized Requestor (Stores/Facilities/Executive Secretaries' Office) submits a request to the Procurement Section.
 - 3.3.2 For business cards, the business card request form must be signed and approved by the Executive before the PR is created. A memo must be sent to the Chief of Business Services for approval.
 - 3.3.3 All PRs (except for catering) is catalogued in the Oracle system and is automatically generated as a PO as they are Blanket Purchase Agreement.
 - 3.3.4 As these are BPA POs (except for catering), they are assigned a PO number along with a release number (E.g.: 118130-731).
- 3.4 **Documentation:**
 - 3.4.1 This must be filed as individual files and numbered using the PR number. Subsequent specifications, additional information, memos, and email threads must be attached to the file.
 - 3.4.2 The receiving copies of all PRs and its attaching documents must be scanned and stored in the HMC AS Production server (\\hmcfs03\ASProduction). Must have PR number in the file name so it can be identified or searched easily.
- 3.5 **Contracts Management:**

- 3.5.1 All contracts are to be mentioned in the Outlook Calendar with the reminders kept 8 months/6 months/3 months before expiry. All parties under ASContracts@hamad.qa are notified of the same.
- 3.5.2 HMCAS Procurement must ensure meetings are conducted on a monthly, quarterly, biannually, or annually (based on the total value of the contracts) with each vendor;
 - 3.5.2.1 QAR 10,000,000 and more (in total contracts value) = quarterly meetings.
 - 3.5.2.2 QAR 1,000,000 and more (in total contracts value) = biannual meetings.
 - 3.5.2.3 QAR 500,000 and more (in total contracts value) = annual meetings.
- 3.5.3 During these scheduled meetings, performance is evaluated based on KPIs or other indicators set by the respective contracts.
- 3.5.4 HMCAS Procurement must ensure all previous payments have been processed before processing the latest invoice.
- 3.5.5 HMCAS Procurement must ensure a contract excel sheet is updated every month with the latest incoming and outgoing contracts. Contracts amendments must also be reflected here.

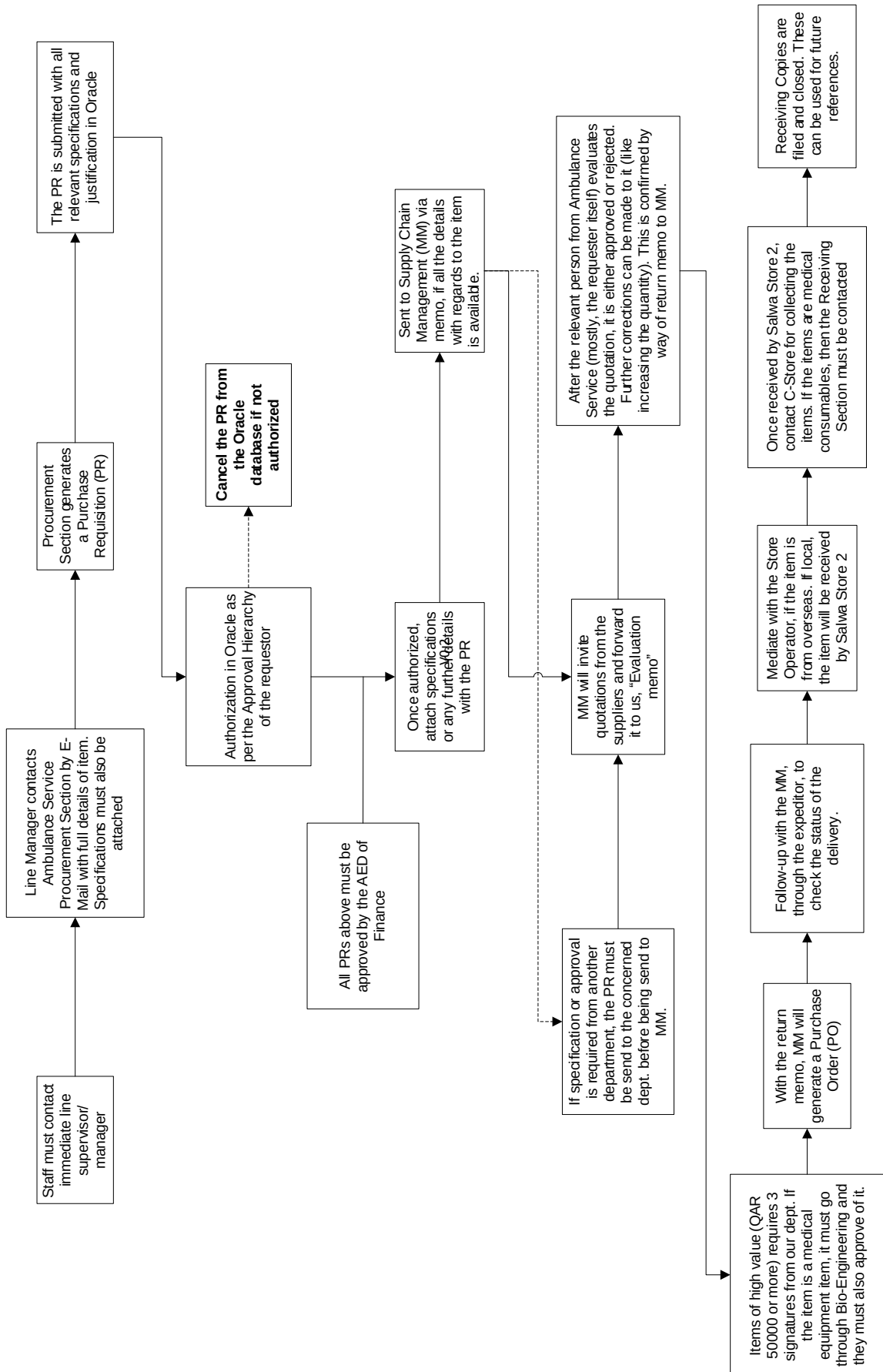
4. Attachments:

- 4.1 PR Procedure
- 4.2 Contract/Procurement Evaluation Process

Attachment 1: Procurement Procedure

V0.2; Feb 2020

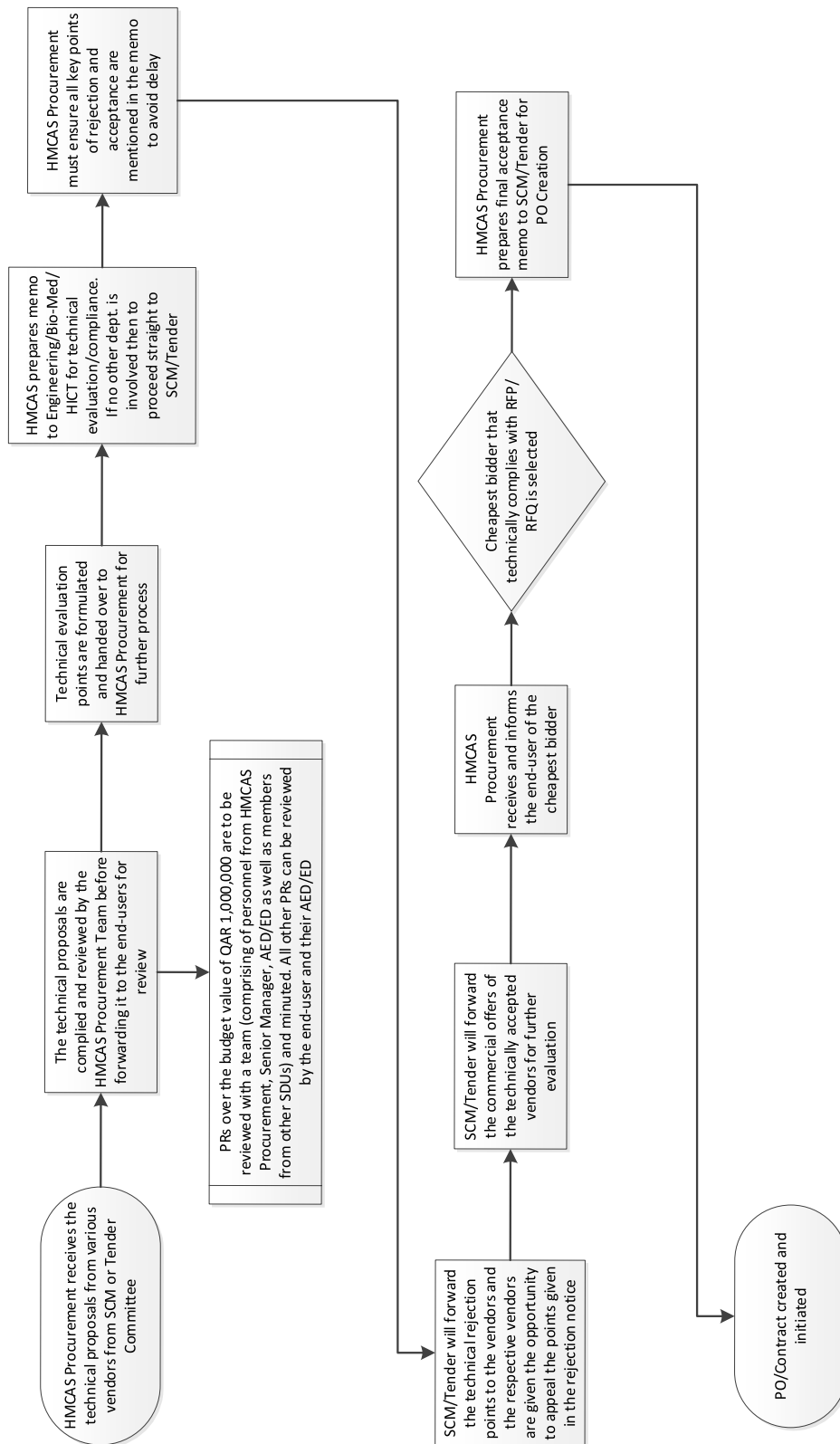
Procurement Procedure (HMC Ambulance Service)



Attachment 2: Contract/Procurement Evaluation Process

VO.2, Feb2020

HMC Ambulance Service Contract/Procurement Evaluation Process



Legend:

HMCAS – Hamad Medical Corporation Ambulance Service
 SCM – Supply Chain Management
 HICT – Health Information and Communication Technology
 PR – Purchase Request
 PO – Purchase Order
 RFP – Request for Proposal
 RFQ – Request for Quotation

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	1.17 DISCLOSURE OF HMCAS STAFF IDENTIFICATION TO EXTERNAL SECURITY AGENCIES
Implementation Date	18 July 2023
Purpose	To establish guidelines and procedures for disclosing staff information to the external security agencies while ensuring the protection of sensitive employee data and compliance with legal requirements.
Responsibility	ALL HMCAS STAFF

1. Purpose:

- 1.1 To establish guidelines and procedures for disclosing staff information to the external security agencies while ensuring the protection of sensitive employee data and compliance with legal requirements.

2. Scope:

- 2.1 The SOP applies to all HMCAS staff.

3. Definitions:

- 3.1 HMCAS Staff information: Personal and confidential information (for example, but not limited to, HMC ID, QID, Qatar Driving License).
- 3.2 External Security Agencies: Authorized law enforcement agencies, National Security agencies responsible for enforcing laws, maintaining public safety, traffic safety or conducting investigations i.e., Qatar Traffic department, Lekhwiya, Al Fazaa, Criminal Investigation Department, ISF etc.

4. Procedure:

- 4.1 HMCAS staff must communicate calmly and politely when asked for their identification.
- 4.2 Politely ask the reason for requesting ID card.
- 4.3 When an external security agency requests HMCAS staff information, the request should be verified for authenticity and legitimacy. Ensure the requester belongs to the authorized external security agency. Politely state that HMCAS Policy requires the requestor:
 - 4.3.1 To be in uniform
 - 4.3.2 Verify reason for requesting the identification
 - 4.3.3 If not in uniform, request verification by their official identification. Without this, no HMCAS staff can provide identification details.
- 4.4 Before disclosing any information please make sure the requester is eligible to receive that HMCAS staff identification.
- 4.5 If HMCAS staff are in doubt about the authenticity of the requester, please make sure to request for your immediate supervisor via NCC notification.
- 4.6 Provide the requested staff information promptly, ensuring accuracy and completeness.
- 4.7 If the requester is police/law enforcement team HMCAS staff can show their HMC ID and/or QID.
- 4.8 If the requester is CID staff can display their HMC ID and/or QID.

- 4.9 HMCAS staff whilst driving Ambulance unit can share their driving license to the traffic patrol team on request.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Group Quality, Patient Safety and Risk Management
Procedure	1.19 CONDUCTING A QUALITY IMPROVEMENT ACTIVITY IN HMCASG
Implementation Date	18 September 2024
Purpose	To ensure that all quality improvement projects are conducted in compliance with the organisation's policies and that all data usage and publication rights are clearly understood.
Responsibility	ALL HMCAS STAFF

1. Purpose:

- 1.1 Ensure that all quality improvement projects are conducted in compliance with HMCASG policies.
- 1.2 To adhere with the Corporate Quality and Patient Safety (QPS) program AS 9040.
- 1.3 Assist the HMCASG staff in ensuring that their Quality Improvement (QI) projects are conducted properly.
- 1.4 All data usage and publication rights are clearly understood.

2. Scope:

- 2.1 All HMCASG staff.

3. Definitions:

- 3.1 Quality Improvement Advisor (QIA): a professional who provides guidance and advice to individuals within an organisation aiming to improve their operational effectiveness and quality of service. The key roles of a QIA are:
 - 3.1.1 Advise on the development and implementation of quality improvement strategies and plans.
 - 3.1.2 Collaborate with management to set quality expectations and goals.
 - 3.1.3 Encourage the dissemination of a continuous improvement culture.
- 3.2 Quality Improvement Mentor (QIM): a professional who reports to the QIA to promote and facilitate the application of quality improvement principles within an organisation by supporting improvement teams and providing them with the necessary coaching and training to foster a culture of continuous quality improvement. The key roles of QIM are:
 - 3.2.1 Assists in identifying areas of potential improvement and implementing improvement strategies and plans within an organisation.
 - 3.2.2 Provides mentoring and coaching for the improvers when conducting their QI projects.
 - 3.2.3 Facilitates training and development to equip improvers with the necessary skills and knowledge to maintain quality standards.
 - 3.2.4 Promotes a positive quality culture within the organisation, fostering engagement and enthusiasm for continuous improvement efforts.

4. QIA and QIM are responsible for ensuring that the HMCAS Executive Director is informed about all the ongoing QI projects.

- 4.1 Service Delivery Unit (SDU) reviewer: A reviewer assesses the QI proposals to comply with SDU priorities, requirements and HMCASG policy.

- 4.2 ASGQPSRM reviewer: The Ambulance Service Group Quality and Patient Safety reviewer who coordinates the QI review process and assesses the QI methodology.

5. Procedure:

- 5.1 Submission to the Ambulance Service Group Quality, Patient Safety and Risk Management:
- 5.1.1 All quality improvement projects must be submitted to the Ambulance Service Group Quality, Patient Safety and Risk Management at the e-mail address HMCAS-QI@hamad.qa for approval prior to commencement.
 - 5.1.2 The submission should include a proposal detailing the purpose, methods, and expected outcomes using the Proposal Form (Appendix 1).
 - 5.1.3 Where MRC approved a study as a quality improvement study, the Improver must forward the proposal and MRC decision letter to HMCAS-QI@hamad.qa to ensure receiving the required HMCASG approval to initiate the QI project.
 - 5.1.4 The QIA and QIM are responsible for ensuring that all QI projects approved by the HMCASGQPSRM are logged in the Life QI platform.
 - 5.1.5 The improvers are responsible for executing the appropriate steps and carrying out required analyses in line with QI processes under the guidance of QIA and QIM.
 - 5.1.6 The QIA and QIM are not directly tasked with conducting the analysis or synthesising project outcomes. Their roles primarily involve providing guidance and support, while the actual execution of the improvement tasks is the responsibility of the designated team or individual.
 - 5.1.7 HMCASG QPS commit to supporting the Improver in preparing the outcome of their projects to be presented in national and international events and publications.
 - 5.1.8 The project outcome is owned by the HMC. The Improver has to obtain approval for publication/presentation in international and national forums by sending a request to HMCAS-QI@hamad.qa.
- 5.2 Compliance with Terms:
- 5.2.1 All quality improvement projects must be conducted in accordance with the terms outlined below.
 - 5.2.2 These terms include, but are not limited to, ethical considerations, patient safety, data security, and confidentiality:
 - 5.2.2.1 Institution Policies and Patient Safety: The project must be conducted in adherence to the institution's policies and procedures, including but not limited to those related to patient safety, confidentiality, data handling, and quality control. It is crucial that respect for patient rights, privacy, and well-being be maintained at all times during the project.
 - 5.2.2.2 Publication and Data Usage: As outlined in the Project Data Usage and Publication Agreement (Appendix 2), all data generated from this project is owned by HMCASG. It is critical the Improver does not publish, disclose, or disseminate any project findings without the express written permission from HMCASG. The Improver is required to sign this agreement and return it within three (3) working days from the receipt of the approval memo.
- 5.3 Non-Compliance:
- 5.3.1 Failure to comply with any of the points mentioned in this SOP could lead to disciplinary measures.

6. Benefits of conducting a Quality Improvement initiative:

- 6.1 Staff who submit QI initiatives will be given priority for nomination to attend QI courses offered by Hamad Healthcare Quality Institute and other relevant institutions.
- 6.2 Participants will receive a formal thank you letter acknowledging their contribution to improving healthcare quality.
- 6.3 The thank you letter can be used to support claims for CPD points with the Department of Healthcare Professions (DHP).
- 6.4 Successful QI initiatives may lead to opportunities for writing and publishing findings in professional journals.

7. Attachments:

7.1 **Appendix 1: Quality Improvement Activity Proposal.**

<i>Surname/Family Name:</i>	<i>Corporation #:</i>	<i>E-mail:</i>
<i>Department:</i>	<i>Supervisor:</i>	<i>Other department(s) involved:</i>

- 7.1.1 What is the problem you are trying to improve?
- 7.1.2 What data will you need?
- 7.1.3 How will you collect this data?
- 7.1.4 Why is this important for patient care, for you and/or HMCASG?
- 7.1.5 How will you measure the effect of any changes you make to solve the problem?
- 7.1.6 When do you hope to start and complete this?
- 7.1.7 Any other information? (if applicable)

7.2 **Appendix 2: Project Data Usage and Publication Agreement:** (See Sample letter below)

- 7.2.1 This agreement entered into on [Date] between Hamad Medical Corporation Ambulance Service Group (HMCASG) and [Improver's Name]. WHEREAS, HMCASG is committed to maintaining the highest levels of quality and professionalism in its operations; WHEREAS, the Applicant has submitted a proposal for a quality improvement project to the Institution, which has been approved on [Date] under reference number xxxxx; NOW, THEREFORE, in consideration of the mutual promises and covenants contained herein, the parties agree as follows:
 - 7.2.1.1 **Confidentiality and Data Usage:** The Improver shall not publish, disseminate, or otherwise disclose any data or findings from the project without the express written consent of HMCASG.
 - 7.2.1.2 **Reporting:** The Improver shall keep HMCASG informed about all progress related to the project and submit a final project report to HMCAS-QI@hamad.qa.
 - 7.2.1.3 **Publication Rights:** If the project's final report is not published within six (6) months from the completion of the project, HMCASG have the right to lead the publication process. The Improver will provide all necessary assistance to the Institution in such a case.

- 7.2.1.4 **Responsibilities:** The Improver is expected to adhere to all ethical guidelines, data protection laws, and other regulations in the execution of the project.
- 7.2.1.5 The Improver acknowledges and agrees to this Agreement by signing below:
- 7.2.1.6 I, _____, confirm that I have read, understood, and agree to all the terms and conditions outlined above on this date [Date].
- 7.2.1.7 Improver's Signature

Appendix 2: Project Data Usage and Publication Agreement

Project Data Usage and Publication Agreement

This agreement entered into on [Date] between Hamad Medical Corporation Ambulance Service Group (HMCASG) and [Improver's Name].

WHEREAS, HMCASG is committed to maintaining the highest levels of quality and professionalism in its operations;

WHEREAS, the Applicant has submitted a proposal for a quality improvement project to the Institution, which has been approved on [Date] under reference number xxxxx;

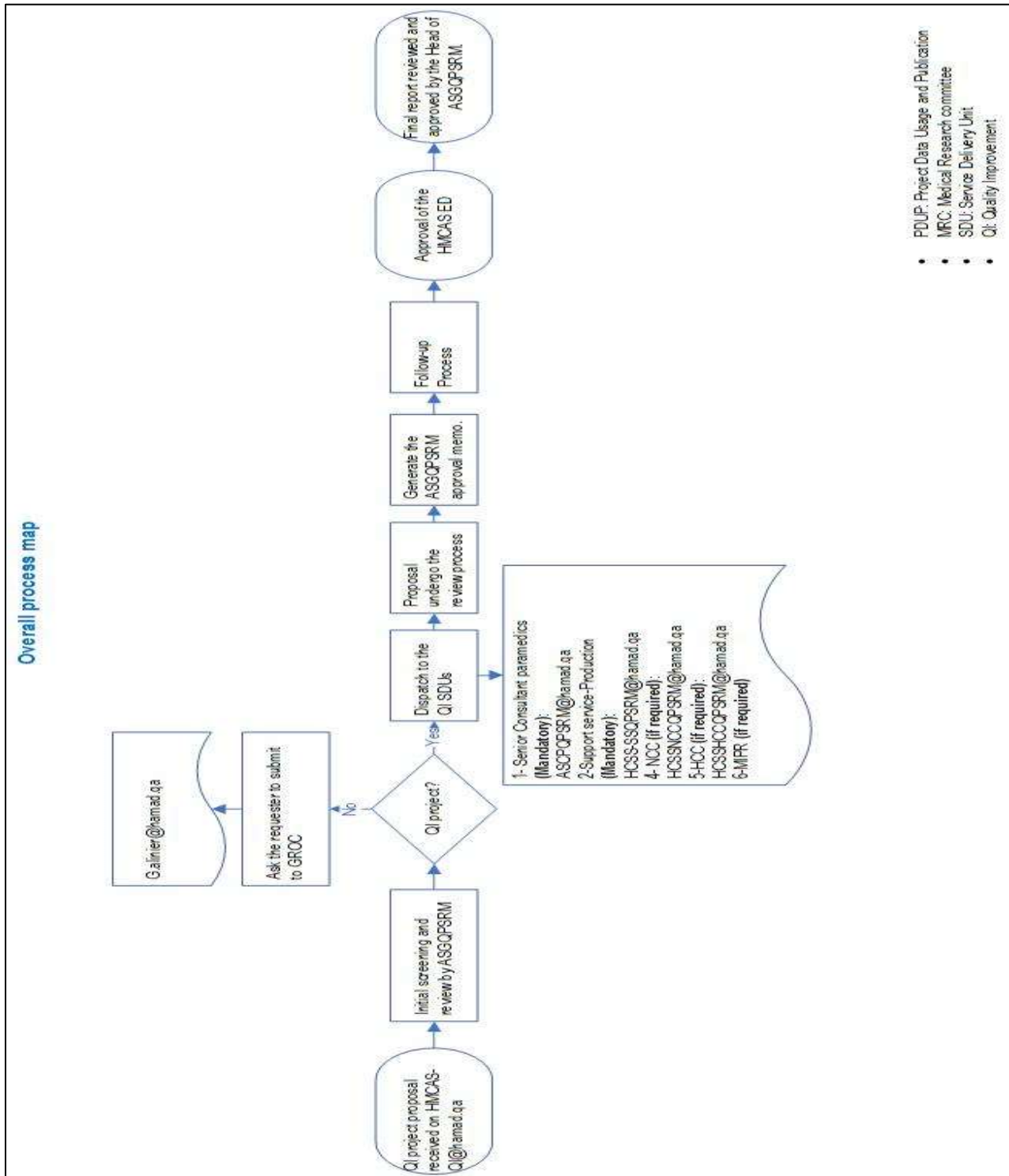
NOW, THEREFORE, in consideration of the mutual promises and covenants contained herein, the parties agree as follows:

- a) **Confidentiality and Data Usage:** The Improver shall not publish, disseminate, or otherwise disclose any data or findings from the project without the express written consent of HMCASG.
- b) **Reporting:** The Improver shall keep HMCASG informed about all progress related to the project and submit a final project report to HMCAS-QI@hamad.qa.
- c) **Publication Rights:** If the project's final report is not published within six (6) months from the completion of the project, HMCASG have the right to lead the publication process. The Improver will provide all necessary assistance to the Institution in such a case.
- d) **Responsibilities:** The Improver is expected to adhere to all ethical guidelines, data protection laws, and other regulations in the execution of the project.

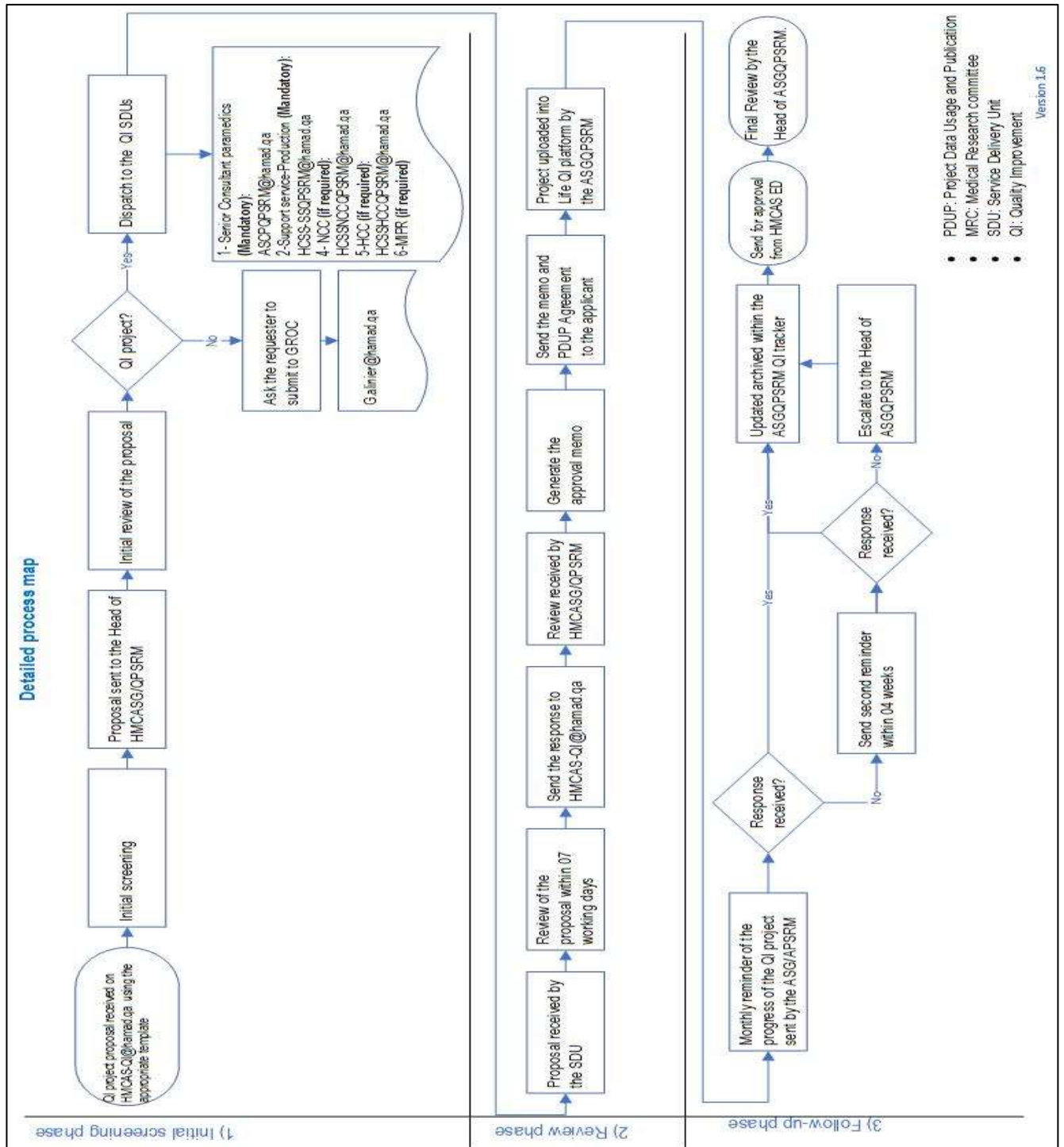
The Improver acknowledges and agrees to this Agreement by signing below:

I, _____, confirm that I have read, understood, and agree to all the terms and conditions outlined above on this date [Date].

Improver's Signature



Detailed process map



SOP 2: Health, Safety, & Security



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.1 PERSONAL PROTECTION & SAFETY
Implementation Date	01 October 2016
Purpose	- To direct and ensure the use of appropriate Personal Protective Equipment (PPE) - To ensure both patient and practitioner safety in hazardous environments
Responsibility	ALL HMC AMBULANCE SERVICE STAFF

1. General considerations for PPE:

- 1.1 Wear only HMCAS Approved PPE
- 1.2 Before operational duties, remove hand jewelry that could damage surgical gloves.
- 1.3 Use hygienic techniques for donning and removing medical gloves.
- 1.4 Perform hand hygiene before and after donning gloves.
- 1.5 Don disposable medical gloves prior to arriving at the patient's side.
- 1.6 Do not wear medical gloves whilst driving.
- 1.7 Replace torn or punctured disposable gloves immediately.
- 1.8 Dispose of disposable PPE at the point of care after endorsing the patient.
- 1.9 Dispose of disposable PPE in designated waste containers.
- 1.10 Thoroughly clean non-disposable PPE prior to next use.
- 1.11 Seek advice from senior staff if unsure about the use of PPE.
- 1.12 Notify a senior HMCAS staff member if a staff member becomes injured.

2. Highly infectious disease situations:

- 2.1 Notify the communications center
- 2.2 Avoid any unnecessary exposure of self and others to the patient.
- 2.3 Wear a second pair of gloves plus eye protection (goggles/face-shield) for additional protection.
- 2.4 Wear a "N95 Mask" or equivalent respiratory mask in the presence of air-borne diseases.
- 2.5 Wear specialized full body PPE for uncommon highly infectious diseases, as directed by a senior clinical HMCAS staff member. See Fig. 2.1-1 & Figure 2.1-2 for donning/doffing PPE.
- 2.6 Follow national recommendations on wearing PPE during outbreaks and in the presence of new emerging and re-emerging diseases.

3. PPE for RTA scenes, Major Incidents, and Industrial Incidents:

- 3.1 ALWAYS don a high-visibility vest BEFORE arrival on the scene.
- 3.2 Wear a firmly secured helmet if there is a perceived head injury risk.
- 3.3 Use face and eye protection in the form of an appropriate face-shield and/or safety goggles.
- 3.4 Wear rescue gloves during special access operations.
- 3.5 Use appropriate hearing protection (e.g. earmuffs/inserts) in high-noise environments (e.g. helicopter, industrial machines, etc.). Enhanced situational and spatial awareness is required since the ability to communicate and concentrate may become diminished.

4. Lighting and Safety:

- 4.1 Position HMCAS vehicles so that the lights could provide light in a dark environment.
- 4.2 Carry HMCAS-issued torch at all times, including during the day.
- 4.3 Do not shine the torch light into other people's faces.
- 4.4 Do not attempt to access a patient in confined/hazardous situations (e.g. crushed vehicle, building collapse, etc.) until:
 - 4.4.1 You have donned appropriate PPE
 - 4.4.2 It safe to do so; and
 - 4.4.3 You receive support from other appropriate stakeholders (Civil Defence/Police on the scene).
- 4.5 Shield the patient from potential debris during extrication from hazardous environments which may include broken glass, exposed sharp metal, non-deployed airbags, construction material and waste, falling debris, hazardous gas, hazardous chemicals, holes, heights, electrocution, and fire.

SEQUENCE FOR **PUTTING ON** PERSONAL PROTECTIVE EQUIPMENT(PPE)

1. GOWN



2. REGULAR FACE MASK OR N95 MASK



3. GOGGLES OR FACE SHIELD



4. GLOVES



SEQUENCE FOR **REMOVING** PERSONAL PROTECTIVE EQUIPMENT(PPE) **EXAMPLE 1**

1. GLOVES

- Outside of gloves are contaminated!
- If your hands get contaminated during glove removal, immediately wash your hands or use an alcohol-based hand sanitizer.
- Using a gloved hand, grasp the palm area of the other gloved hand and peel off first glove.
- Hold removed glove in gloved hand.
- Slide fingers of ungloved hand under remaining glove at wrist and peel off second glove over first glove.
- Discard gloves in a waste container



2. GOGGLES OR FACE SHIELD

- Outside of goggles or face shield are contaminated!
- If your hands get contaminated during goggles or face shield removal, immediately wash your hands or use an alcohol-based hand sanitizer.
- Remove goggles or face shield from the back by lifting head band or ear pieces.
- If the item is reusable, place in designated receptacle for reprocessing. Otherwise, discard in a waste container.



3. GOWN

- Gown front and sleeves are contaminated!
- If your hands get contaminated during gown removal, immediately wash your hands or use an alcohol-based hand sanitizer.
- Unfasten gown ties, taking care that sleeves don't contact your body when reaching for ties.
- Pull gown away from the neck and shoulders, touching inside of gown only.
- Turn gown inside out.
- Fold or roll into a bundle and discard in a waste container.



4. MASK OR RESPIRATOR

- Front of mask is contaminated!--DO NOT TOUCH
- If your hands get contaminated during mask removal, immediately wash your hands or use an alcohol-based hand sanitizer.
- Grasp bottom ties or elastics of the mask, then the ones at the top, and remove without touching the front.
- Discard in a waste container.



5. WASH HANDS OR USE AN ALCOHOL- BASED HAND SANITIZER IMMEDIATELY AFTER REMOVING ALL PPEs.



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.1.A MANAGEMENT OF AMBULANCE IN THE EVENT OF FIRE
Implementation Date	23 October 2024
Purpose	To ensure the safety of ambulance personnel, patients, and bystanders in the event of a fire occurring in an ambulance during operations.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Purpose:

- 1.1 The purpose of this SOP is to establish clear, detailed procedures to ensure the safety of ambulance personnel, patients, and bystanders in the event of a fire occurring in an ambulance during operations. This includes outlining emergency response protocols, evacuation procedures, preventive measures, and post-incident actions.

2. Scope:

- 2.1 This SOP applies to all ambulance operations where a fire-related incident may occur, whether during patient transport, emergency response, or stationary operations. It covers actions to be taken by all ambulance personnel in the event of a fire.

3. Definitions:

- 3.1 **Ambulance Fire:** A fire occurring in the ambulance, including fires originating from the engine, electrical components, or medical equipment, which can pose significant risks to both personnel and patients.
- 3.2 **Oxygen Tank:** Mounted and portable Oxygen accelerates combustion, making these tanks extremely dangerous in the event of a fire.
- 3.3 **RACE Procedure:** (Raise the alarm, Assist with the evacuation of patient/s, Close any doors or windows and Extinguish the fire ONLY if you are confident or safe to do so).
- 3.4 **PASS Method:** (Pull the pin, Aim the nozzle, Squeeze the handle, and Sweep the extinguisher from side to side).

4. Procedure:**4.1 Ensure Personal and Patient Safety:**

- 4.1.1 **Stop the Vehicle Immediately:** If the ambulance is in motion when the fire starts, your priority is to pull over to a safe location away from traffic, structures, or flammable materials (if possible). **Do not** wait to assess the fire; any sign of smoke or fire should be taken seriously. Turn off the engine vehicle.

4.2 Call for Emergency Services:

- 4.2.1 Notify NCC/HCC via current channel by pressing the emergency button on the radio and including the “*break, break, break* – control [vehicle call sign]”. Refer to [SOP 4.5.C](#) to give a message that the vehicle is on fire and help is required and evacuation of the staff and patient should take priority.
- 4.2.2 Provide your exact location and describe the situation, including whether there are hazardous materials on board, like oxygen tanks or flammable medical supplies. This will allow responders to arrive fully prepared.

4.3 Evacuate All Occupants:

- 4.3.1 Get yourself, your partner, and any patients or passengers out of the ambulance as quickly and safely as possible. Time is of the essence because fires in enclosed

spaces can spread rapidly. If you have a patient, follow your training to ensure they are removed carefully but swiftly.

- 4.3.2 (Note: Your safety and the patient's safety are the highest priority. If the fire is progressing quickly, you need to prioritize getting people out over saving equipment).

4.4 Move to a Safe Distance:

- 4.4.1 Maintain a **Safe Distance**: Once everyone is out of the ambulance, move at least **100 meters** away. This is crucial, as vehicle fires can escalate, leading to fuel and oxygen tank explosions or the release of toxic fumes from burning materials such as plastic, chemicals, and medical equipment.
- 4.4.2 Keep Bystanders Away: Ensure bystanders are kept at a safe distance as well. Ambulances carry flammable materials like oxygen tanks, which can explode if the fire reaches them.

4.5 Use a Fire Extinguisher (Only If Safe to Do So):

- 4.5.1 Assess the Fire: If the fire is small and localized and the engine fire hasn't spread, you may consider using a fire extinguisher. However, your first duty is to evaluate whether using the extinguisher is safe.
- 4.5.2 If it's safe to do so, aim the extinguisher at the base of the fire, not the flames, and follow the PASS method.
- 4.5.3 (Note: If the fire has spread to the rear of the ambulance or is growing rapidly, do not attempt to extinguish it. Furthermore, avoid returning to the ambulance if the fire has started near oxygen tanks or medical supplies, as these can pose significant explosive or hazardous risks).

4.6 Medical Assessment:

- 4.6.1 Once the situation is under control, everyone involved should be checked for injuries or smoke inhalation. Even brief exposure to a vehicle fire can result in respiratory issues due to toxic fumes.

4.7 Documentation:

- 4.7.1 Before the end of the shift staff shall:
 - 4.7.1.1 Complete OVA report
 - 4.7.1.2 Obtain the staff incident investigation report
 - 4.7.1.3 (Note: Police report and Staff Investigation report should be completed within 48 hours).

4.8 Post Incident procedure:

- 4.8.1 Follow [SOP 4.6 D](#) Section 5

5. Attachments:

- 5.1 FIRE AND EXPLOSION HAZARDS CAUSED BY OXYGEN CYLINDERS.

6. References:

- 6.1 BOŽENA et al., 2018. FIRE AND EXPLOSION HAZARDS CAUSED. Wit press, 174(7), pp. 149-151.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.2 MEDICAL WASTE HANDLING
Implementation Date	23 June 2024
Purpose	To direct and ensure the appropriate handling and disposal of medical waste
Responsibility	ALL HMCAS STAFF

1. Purpose:

- 1.1 To give clear direction about the segregation, storage, handling, transportation, and disposal of waste in order to ensure the safety of healthcare workers and to minimize the potential risk it presents to the general public and the environment.

2. Scope:

- 2.1 All HMCAS Staff

3. Definitions:

- 3.1 **Healthcare Waste:** any material produced by the health care establishment that is no longer wanted and requires disposal.
- 3.2 **Infectious Waste/Clinical Waste:** has the potential for transmitting infectious diseases, it includes:
- 3.3 **Sharp Waste:** These are the wastes that contain sharp items such as sharps, intravenous needles, blood vials, glass vials and needles, scalpels, lancets, sutures, razors, broken glass or any other sharp objects that cut or sting the body.
- 3.4 **Waste from Human blood, blood products and body fluids:** includes human blood or blood components (serum or plasma). Containers with free-flowing blood or blood components or discarded saturated material containing free flowing blood, blood component and body fluids. Soiled surgical dressings, swabs etc.

4. Procedure:

- 4.1 All HMCAS staff shall comply with this SOP.
- 4.2 Operational staff shall ensure that vehicles are equipped with colour-coded bags for proper segregation.
- 4.3 It is necessary to use the appropriate colour coded bags and labels for collection of wastes and shall be colour coded or have a biohazard label.
- 4.4 Identification, segregation, and packaging of waste shall be done at the point and time of origin by the generator of that waste.
- 4.5 Any spillage of clinical waste (e.g. content of yellow bag) must be dealt with immediately.
- 4.6 **Temporary Storage of Infectious Waste:**
- 4.6.1 The TEMPORARY STORAGE area for Infectious Waste shall have the following provisions:
- 4.6.1.1 The specified location for the collection of infectious waste should be known to all healthcare staff.
- 4.6.1.2 This waste shall be placed in closed and secured containers or plastic bags prior to storage.

- 4.6.1.3 Waste will be temporarily stored in closed containers within the designated area until the scheduled pick-up time. The storage location shall be appropriate and cause no pollution or harm against human health or the environment.
- 4.6.1.4 Storage times for infectious waste (e.g. the time gap between generation and treatment) should normally not exceed 24 hours but can extend up to 48 hours during the cool season.
- 4.6.1.5 Legible stickers should be placed on the storage location stating the nature of the contents in the location.

4.7 **Temporary Storage of Other Healthcare (Non-Infectious) Waste:**

- 4.7.1 The holding site for domestic waste black bag collection shall be covered and fly proof.
- 4.7.2 Domestic waste storage area must be separated from infectious waste areas.

4.8 **Handling of Waste:**

- 4.8.1 In order to prevent inappropriate disposal, infectious waste shall be segregated from non-risk (domestic waste). Clear information, instruction, and training about identifying specific categories of waste should be provided to Staff.
- 4.8.2 Correct colour coded bags shall be used to segregate waste:

TYPE OF WASTE	COLOR CODED BAG
Domestic Waste (Paper tissues and face mask used by the patient, gloves, gowns, apron, face shield, head cover, shoe cover, and face masks including N95 mask used).	Black bag
Infectious Waste (Visibly soiled waste includes human blood or bloody fluids) Blood stained PPE such as: Gloves, face shield, gowns.	Yellow bag labeled with Universal Biohazard sign
Sharps (needles, syringes etc.)	Yellow Sharp Box with the Universal Biohazard sign
Amputated body part (e.g. hand, leg etc.)	Transparent plastic bag covered with bed sheet
Pathological Waste – including Placenta.	Yellow bag
Presence of emerging and re-emerging disease.	Follow Corporate and MOPH recommendations for waste segregation and disposal.

- 4.8.3 Waste bags shall be packed three fourth full (3/4), or as required, and securely closed by tying the top of each bag.

4.9 **Segregation of Waste:**

- 4.9.1 Segregation and packaging of waste shall be done using waste bags and boxes according to the colour code and labelling scheme.
- 4.9.2 Yellow bags with contaminated clinical waste shall not be mixed with black bags
- 4.9.3 Double bagging is recommended when the weight/mass of the waste warrants added packing strength. Infectious waste shall be packed in single bags only.
- 4.9.4 Bags shall be protected from puncture and tearing by preventing contact with external sharp objects.
- 4.9.5 Sharps containers shall be securely closed when three fourth full (3/4), put into yellow plastic bags with the required identification tag and sent for autoclaving.
- 4.9.6 Sharps Containers in response bags shall be always securely closed between uses.
- 4.9.7 Body fluid drains shall be placed in a yellow plastic bag.
- 4.9.8 For used Intravenous set, discard by tying the tube around the empty plastic bag that contained the intravenous fluid and place in the domestic black bin – no cutting. If the IV tubing is connected to glass, cut the IV tubing by retaining the spike inside the bottle and discard in the rigid container for empty bottles. However, all types of blood contaminated tubing should be placed in Yellow Bags to be sent for autoclaving.

4.10 **Linen [Disposable]:**

- 4.10.1 Place dirty linen not soiled with blood or body fluids in domestic bags.
- 4.10.2 Place linen soiled with blood or body fluids in yellow bags.

5. References:

- 5.1 Management of Infectious Waste CL 7249.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Production
Procedure	2.3 SOCIAL CLEANING AND SIMPLE DECONTAMINATION OF VEHICLES AND NON-CRITICAL EQUIPMENT
Implementation Date	23 June 2024
Purpose	To ensure that operational vehicles and non-critical equipment are socially clean and disinfected.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Social cleaning and simple decontamination may be performed at Hubs, Sheltered Spokes and Health Facilities.
2. At the shift start, ensure that vehicles and equipment are socially clean:
 - 2.1 Assess the vehicle and equipment for soiling.
 - 2.2 Where there is soiling from body-substances, the vehicle or equipment should be exchanged, and the soiled equipment sent for Deep Cleaning.
 - 2.3 Cleaning and disinfection should be performed for minor visible soiling.
 - 2.4 Visible non-body-substance soiling should be cleaned.
 - 2.5 External vehicle cleaning should be done as required
3. Cleaning products and tools:
 - 3.1 Social cleaning should be carried out with soap, cloths, vacuum cleaners (inside), mops, soft brooms, and/or high-pressure cleaners (outside).
 - 3.1.1 Brooms cloths and mops should be rinsed and allowed to dry naturally.
 - 3.2 Cleaning and disinfection should be performed with AS approved disinfectant disposable wipes.
 - 3.2.1 Wipes should be discarded in a black bin.
4. Personal protective equipment (PPE):
 - 4.1 The following personal protective equipment should be worn:
 - 4.1.1 Latex or non-latex examination gloves, doubled-up.
 - 4.1.2 Safety glasses, if required
 - 4.2 Following cleaning, disposable PPE must be discarded in a black bin and non-disposable PPE should be wiped with alcohol-based disposable wipes.
5. Cleaning and disinfection must be performed after every patient encounter.
 - 5.1 Assess equipment and work surfaces for significant visible body-substances soiling; where found the vehicle and/or equipment should be exchanged for a pool unit or pool equipment.
 - 5.2 Where there is no significant visible soiling, perform cleaning and disinfection on all non-critical equipment used and working surfaces. This includes, but is not limited to, the following items:
 - 5.2.1 Monitor-defibrillator and attachments
 - 5.2.2 Stethoscopes and other diagnostic equipment
 - 5.2.3 Immobilization equipment
 - 5.2.4 Stretcher and cabinet handles

6. Exchanging of equipment or vehicle should be done when there is significant visible soiling with body-substances.
 - 6.1 If at HGH, exchange the ambulance with pool ambulance unit.
 - 6.2 If near Hub 1, exchange with a pool ambulance unit at the main stores.
 - 6.3 At other hubs exchange with a pool unit at that specific hub.
7. References:
 - 7.1 Cleaning and Disinfection and Sterilization CL 7261
 - 7.2 Infection Control in Housekeeping Department CL 7242
 - 7.3 Infection Control in Ambulance Services CL 9041

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.3.A MAINTENANCE AND CLEANING OF HMCAS RESPONSE AND MEDICATION BAGS
Implementation Date	01 June 2023
Purpose	To provide guidelines for maintaining and cleaning HMCAS bags, as well as preventing and controlling the spread of microorganisms.
Responsibility	ALL HMCAS OPERATIONAL STAFF, HMCAS SUPPORT SERVICES STAFF, HMCASG PHARMACY STAFF AND SUB-CONTRACTORS.

1. Definitions:

- 1.1 **Dirty/Contaminated Bag:** Bag that has been used for patient care and is visibly unclean but is not soiled with blood and/or body fluids.
- 1.2 **Soiled/Contaminated Bag:** Bag that is soiled, stained, or soaked with blood and/or other body fluids, as well as other potentially infectious materials.

2. Storage of Bags in HMCAS Stores:

- 2.1 Clean bags shall be kept in a dust bag and stored on a clean shelf, away from direct sunlight and possible sources of contamination.
- 2.2 Shelves used to store clean bags shall be free of dust and vermin.

3. Procedure/Process:**3.1 Contents of the Bag:**

- 3.1.1 All HMCAS staff are responsible for preparing the HMCAS bags according to the attached checklist, with any additional items added as per patient requirements.
- 3.1.2 HMCAS staff shall replace all medical supplies that have been used on a daily basis and as needed.

4. Cleaning of Bags:**4.1 Routine decontamination of Bags****4.1.1 Social cleaning of HMCAS Bags:**

- 4.1.1.1 The outer surface of the bag shall be cleaned after each use with HMC-approved disinfectant wipes.
- 4.1.1.2 Hand hygiene should be performed prior to cleaning the inside of bag.
- 4.1.1.3 The bag's inner pouches shall be kept clean at all times; end-users shall clean the bag's interior pouch using HMC-approved disinfectant wipes during routine checks and as needed.
- 4.1.1.4 Following manufacturer's recommendations, all equipment shall be cleaned after each use and prior to being placed in the bag.
- 4.1.1.5 Prior to restocking medications, pharmacy staff shall clean the outer surface and inner pouches of the bag using HMC-approved disinfectant wipes.

5. Cleaning of Soiled/Contaminated Bags:**5.1 Washable Medication Bags:**

- 5.1.1 Unused medications shall be removed by the end user and placed in an orange bag before being returned to the pharmacy.

- 5.1.2 The end user shall return dirty and/or soiled medication bags in an orange bag for manual washing.
- 5.2 **Controlled Medication Cases:**
 - 5.2.1 The end user shall place the whole dirty and/or soiled bags without removing their controlled medications in an orange bag and return them to the pharmacy.
- 5.3 **Other washable Bags:**
 - 5.3.1 Packs and equipment shall be placed in an orange bag and returned to the Stores.
 - 5.3.2 Dirty and/or soiled bags shall be placed in an orange bag and surrendered for manual washing.
- 5.4 **Non-washable Bags:**
 - 5.4.1 Packs and equipment shall be placed in an orange bag and returned to the Store.
 - 5.4.2 Non-washable bags that are dirty and/or soiled shall be placed in a yellow bag and discarded.
- 5.5 **Deep Cleaning and Disinfection of HMCAS Bags:**
 - 5.5.1 Cleaning and disinfection shall be performed by a sub-contractor in accordance with the manufacturer's guidelines for each type.
 - 5.5.2 Sodium hypochlorite or HMC-approved disinfectant solution shall be used for manual decontamination.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.3.B INFECTION CONTROL IN HMCAS SUPPLY CHAIN
Implementation Date	19 March 2024
Purpose	To provide guidelines for maintaining and cleaning HMCAS bags, as well as preventing and controlling the spread of microorganisms.
Responsibility	ALL HMCAS OPERATIONAL STAFF, HMCAS SUPPORT SERVICES STAFF, HMCASG PHARMACY STAFF AND SUB-CONTRACTORS.

1. Definitions:

- 1.1 **HMC AS Supply Chain (HMCASSC):** The section responsible for packing and re-packing medical supplies. This process involves disassembling, sorting, cleaning, packing, transporting, and distributing the processed items.
- 1.2 **HMC AS Warehouse (HMC ASW) –** Area where materials are received, stored, and processed before distribution to customers. This includes, but is not limited to:
 - 1.2.1 HMCAS Stores in the hubs and facilities.
 - 1.2.2 Other on-site facilities (for example, pop-up supply outlets for events, conferences, and drive-thru initiatives).
- 1.3 **Outsourced Staff –** A contract employee who has training and expertise on a certain work-related task or role.
- 1.4 **Cleaning –** The removal of visible dust, soil, or any other foreign material.
- 1.5 **Personal Protective Equipment (PPE) –** A specialized equipment worn by staff for protection.
- 1.6 **First-in, First-out (FIFO) –** An asset-management and valuation method in which the assets produced or acquired first are sold, used or disposed of first.
- 1.7 **First-Expiry-First-Out (FEFO) –** The product whose expiration date is due first shall move out of the storage first and the rest shall follow in the same order.
- 1.8 **Shelf life –** The amount of time that a product can be stored and still be considered safe and effective for use, the length of time a material may be stored without deterioration, and the length of time it remains usable.
- 1.9 **Pre-pack Kits –** Pre-assembled medical consumable items.
- 1.10 **Reprocessing –** The cleaning, disinfecting, and repacking of an item that was not used on a patient but has not had its packing breached or damaged.

2. Responsibilities:

- 2.1 **HMC AS Infection Prevention and Control Committee -** responsible for reviewing, revising, and approving the policy.
- 2.2 **HMC AS Operational Staff-** responsible for implementing the policy.
- 2.3 **HMC AS Supply Chain Staff-** responsible for disseminating, educating and implementing the policy.
- 2.4 **Outsourced HMCAS Supply Chain Staff -** responsible for implementing the policy.
- 2.5 **Production Supervisors –** responsible for overseeing the proper delivery, collection, and transportation of returned items in the hubs and other on-site facilities.

3. Physical Design of the Supply Chain:

- 3.1 The HMCASSC Warehouse shall be divided into the following areas:
 - 3.1.1 Receiving Area for returned items

- 3.1.1.1 Initial sorting
- 3.1.2 Sorting Area
- 3.1.3 Cleaning/Decontamination Area
- 3.1.4 Packaging Area
- 3.1.5 Storage Area
- 3.1.6 Distribution Area
- 3.2 Each work area shall be identified with a signage specifying the type of work conducted and its corresponding step within the process.
- 3.3 Work tables shall be made of nonporous materials (e.g., stainless steel).
- 3.4 Separate work areas shall be provided for processing clean and dirty items.
- 3.5 Safety measures (e.g., eye wash station) shall be available for staff use.
- 3.6 Hand washing facilities shall be easily accessible in the sorting and decontamination areas.
- 3.7 Alcohol hand rubs shall be readily available.

4. Procedure/Process:

4.1 Standard Precautions:

- 4.1.1 All supplies used for patient care shall be considered high-risk for potentially infectious material and shall be handled following standard precautions.
- 4.1.2 The common occupational injuries that occur in the supply chain include, but are not limited to:
 - 4.1.2.1 Exposure to Blood and Body Fluids
 - 4.1.2.2 Exposure to hazardous materials and chemicals.
- 4.1.3 Staff exposure to blood and body fluids shall be managed according to CL 7251 Occupational Exposure to Blood and Body Fluids.
- 4.1.4 Hands shall be washed thoroughly, or alcohol hand rub shall be used if hands are not visibly soiled (Refer to Policy CL7241 Hand Hygiene).

4.2 Collection and Handling of Opened and Unused Pre-pack Kits and Other Medical Consumable Items:

4.2.1 HMCAS Operational Staff/End-User

- 4.2.1.1 Partially used or opened pre-pack kits shall not be discarded unless they are soiled.
- 4.2.1.2 End-users shall conduct an initial risk assessment of used and opened pre-packs prior to returning them to the Stores:
 - 4.2.1.2.1 Contaminated items shall not be stored in the same bag as clean and unused medical items.
 - 4.2.1.2.2 Promptly discard contaminated items in accordance with waste management procedures (Refer to CL 7249 Management of Infectious Waste).
 - 4.2.1.2.3 Clean, unused, or damaged pre-pack items shall be placed in designated collection boxes in the Stores for re-packing.
 - 4.2.1.2.4 Collection boxes shall be labelled and stored in a clean area, away from exposure to dust and direct sunlight.
 - 4.2.1.2.5 Opened packages must be inspected by the concerned user. Promptly discard items if there are doubts regarding their integrity including, but not limited to:

- 4.2.1.2.5.1 Tears
- 4.2.1.2.5.2 Holes in packages
- 4.2.1.2.5.3 Water/liquid marks
- 4.2.1.2.5.4 Item is visibly soiled

4.2.1.2.6 If the safety seal is breached or damaged, the end-user shall return the pre-pack items to the Stores for replacement.

4.3 **Transportation of Opened and Unused Prepack Kits and other Medical Consumable Items:**

- 4.3.1 The assigned staff shall be responsible for collecting returned pre-pack items from the Stores and other on-site facilities.
- 4.3.2 Containers used for collecting returned items shall be labelled clearly as 'for used pre-packs only'.
- 4.3.3 Storekeepers shall ensure that collection boxes are lined with transparent or white plastic bags to facilitate the transfer of returned items and prevent the contamination of said items.
- 4.3.4 Returned items shall be sent to the HMCASSC Warehouse in a transparent or white plastic bag and placed in closed container tightly knotted.
- 4.3.5 Unused pre-pack items shall be transported in a clean vehicle.
- 4.3.6 All items transported to the HMCASSC Warehouse shall have a trolley dedicated for transporting returned items. This trolley shall be cleaned and washed along with the transporting vehicle in the ambulance wash bay at the end of each shift and when visibly dirty in between usage.

4.4 **Sorting of Opened and Unused Prepack Kits and other Medical Consumable Items:**

- 4.4.1 Sharps containers, infectious waste bins, and general waste bins shall be provided in the initial sorting area.
- 4.4.2 Staff assigned in the sorting areas shall:
 - 4.4.2.1 Observe hand hygiene before and after wearing proper PPE.
 - 4.4.2.2 Wear appropriate PPE and follow standard precautions while handling returned medical items.
 - 4.4.2.3 Remove and properly discard PPE before leaving the work area.
 - 4.4.2.4 Returned items shall be inspected for the following but not limited to:
 - 4.4.2.4.1 Cleanliness
 - 4.4.2.4.2 Integrity
 - 4.4.2.4.3 Presence of tears in packaging
 - 4.4.2.4.4 Discoloration and watermarks
 - 4.4.2.4.5 Expiration date not discernible
- 4.4.3 Items found with compromised integrity during the inspection process shall be promptly discarded if contaminated, adhering to the established waste management policy (Refer to CL 7249 Management of Infectious Waste).
- 4.4.4 Containers used for collecting and sorting returned items shall be cleaned and disinfected in between usage.
- 4.4.5 Conduct an initial risk assessment of opened and unused pre-pack items:
 - 4.4.5.1 Promptly discard contaminated items in accordance with waste management procedures (Refer to CL 7249 Management of Infectious Waste).

- 4.4.5.2 Damaged, non-contaminated items (for example, opened but unused syringes, O2 kits, cannula, etc) can be cleaned and sent to the Training department for teaching purposes.

4.5 **Cleaning of Work Areas:**

- 4.5.1 The cleaning/decontamination area shall be separated by a physical barrier from other work areas.
- 4.5.2 Observe hand hygiene before and after wearing proper PPE.
- 4.5.3 Wear appropriate PPE and follow standard precautions while handling returned medical items.
- 4.5.4 Cleaning shall be conducted using hospital approved disinfectant.
- 4.5.5 All warehouse staff, in cooperation with the outsourced housekeeping staff, shall observe proper housekeeping to ensure that:
 - 4.5.5.1 Risk of fire is mitigated by proper disposal of waste.
 - 4.5.5.2 Damaged or unused pallets and cartons are immediately removed if considered to be not part of the active operations.
 - 4.5.5.3 Transportation carts shall be disinfected after each use.
 - 4.5.5.4 Cleaning of potential dust collecting areas (for example, air conditioning vents, filters, and shelves) shall be scheduled.

4.6 **Repacking of Medical Items:**

- 4.6.1 Ensure that returned and unused medical items are clean and dry prior to repacking.
- 4.6.2 Pre-pack kits shall be clearly labelled with the following:
 - 4.6.2.1 Name of item
 - 4.6.2.2 Item Code and Lot No.
 - 4.6.2.3 Production Date
 - 4.6.2.4 Expiration Date
- 4.6.3 Where a pre-pack kit consists of items with differing expiration dates, the package label shall reflect the earliest expiration date among the included items.

4.7 **Storage of Prepack Kits and other Medical Consumable Items in the HMCASSC Warehouse and Sub-stores (Hubs and Spokes):**

- 4.7.1 Storage area specification and maintenance should be as follows:
 - 4.7.1.1 Maintained at a temperature no higher than 25°C (75°F) and relative humidity of 30-70%. Temperature and humidity readings should be recorded at least once daily.
 - 4.7.1.2 Exterior shipping cartons shall not be brought into the storage area.
 - 4.7.1.3 Floors and shelving racks shall be cleaned as necessary by housekeeping staff.
 - 4.7.1.4 Storage shelves should be labeled to maintain adequate inventories of clean and sterile products.
 - 4.7.1.5 Medical items shall not be kept near sinks or stored under plumbing valves and traps.
 - 4.7.1.6 All medical items shall be stored 8-10 inches from the floor and 18-20 inches from the ceiling.
 - 4.7.1.7 Packed items shall be placed in properly labeled containers.
 - 4.7.1.8 The storekeeper shall do the following:

- 4.7.1.8.1 Ensure that the integrity of the package is not compromised.
- 4.7.1.8.2 Ensure the proper storage of clean and sterile items:
 - 4.7.1.8.2.1 Clean and sterile items shall not be crushed or grouped with rubber bands.
 - 4.7.1.8.2.2 All packed items shall be preferably stored on shelves, in a dedicated container with labels.
 - 4.7.1.8.2.3 Clean medical items shall preferably be stored in transparent plastic bags for additional protection.
 - 4.7.1.8.2.4 Expiration dates of the stored sterile items shall be checked and recorded on a monthly basis.
 - 4.7.1.8.2.5 Medical supplies shall be arranged to facilitate stock rotation and arranged according to the principles, "first in, first out" (FIFO) and First-Expiry-First-Out (FEFO).
- 4.7.1.9 The Production Supervisor shall maintain stock levels in accordance with the maximum and minimum levels designated by the Operations Officer.

4.8 **Single-Use Disposable Items:**

- 4.8.1 Single-use medical items shall not be re-sterilized as the re-sterilization processes may compromise the safe function (Refer to CL 7278 Management of Single Use Device and Expired Supplies).
- 4.8.2 Items that have expired or have been recalled from circulation due to manufacturer issues and are intended for teaching purposes shall be:
 - 4.8.2.1 Decontaminated
 - 4.8.2.2 Stored in a designated area
 - 4.8.2.3 Labeled clearly as "for training use only"

4.9 **Distribution of Clean Medical Items:**

- 4.9.1 A dedicated trolley shall be used for clean and sterile items. This trolley shall be cleaned at the beginning and end of each shift and when visibly dirty between usage.
- 4.9.2 All clean and sterile supplies shall be placed in covered or enclosed carts with solid bottom shelves and arranged in a manner that prevents crushing, bending, or compressing enclosed items.
- 4.9.3 A vehicle shall be dedicated solely for the transportation of medical items:
 - 4.9.3.1 Vehicles shall be cleaned at the beginning and end of each shift, and when visibly dirty between usage.
 - 4.9.3.2 Containers used for collecting and transporting returned and unused pre-pack kits and other medical consumable items shall not be stored directly on the vehicle's flooring. Staff shall utilize plastic pallets or shelving racks to elevate said items.
 - 4.9.3.3 Vehicles shall be kept at an appropriate temperature whenever possible by ensuring that the air conditioning system is switched on during loading and transportation.
- 4.9.4 Container Systems shall be designed to:
 - 4.9.4.1 Prevent compromise of sterility such as protection from moisture/condensation, insects, vermin, dust and dirt.

- 4.9.4.2 Facilitate easy loading, unloading, and removal of items without contamination.
 - 4.9.4.3 The container shall be of an appropriate size relative to the items and shall be secured in the vehicle for transportation.
 - 4.9.4.4 Environments with high humidity or extreme temperature changes shall be considered and mitigated when transporting medical items to reduce any risk of mishandling.
- 4.10 **Transportation and Handling of Dirty/Used Items for Washing and Disinfection**
 - 4.10.1 Contaminated items shall include, but not be limited to the following:
 - 4.10.1.1 Combi-Carrier
 - 4.10.1.2 Backboards
 - 4.10.1.3 Scoops
 - 4.10.1.4 Stretcher and safety belts
 - 4.10.1.5 Bags
 - 4.10.1.6 Lifepak 15
 - 4.10.1.7 Other immobilization devices such spider straps and head blocks.
 - 4.10.2 If the orange bag can accommodate the item, contaminated items shall be placed in the orange bag.
 - 4.10.3 The assigned staff shall wear proper PPE such as gloves and gown when handling contaminated items.
 - 4.10.4 A dedicated vehicle shall be used for transporting contaminated items:
 - 4.10.4.1 Clean items shall not be transported in the same vehicle as contaminated items.
 - 4.10.4.2 Deep cleaning shall be done on vehicles at the end of each shift, or when there is a visible splash, spillage, or contamination of the vehicle.
- 4.11 **Waste Disposal:**
 - 4.11.1 All waste shall be managed according to Waste Management Policy CL 7249 Management of Infectious Waste.
- 4.12 **Elements of a Recall System:**
 - 4.12.1 HMCASSC shall adhere to system processes for recalling items in the presence of evidence of risks, such as contamination, manufacturer failure, etc.
- 4.13 **References:**
 - 4.13.1 HMC Occupational Exposure to Blood and Body Fluids Policy CL 7251
 - 4.13.2 HMC Hand Hygiene Policy CL 7241
 - 4.13.3 HMC Management of Single Use Device and Expired Supplies Policy CL 7278
 - 4.13.4 HMC Management of Infectious Waste Policy CL 7249

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Production
Procedure	2.4 DEEP CLEANING OF PATIENT TRANSPORTING VEHICLES
Implementation Date	01 October 2016
Purpose	To ensure that patient transporting vehicles ¹ are deep cleaned appropriately and safely on a routine basis.
Responsibility	SUPPORT SERVICES STAFF

1. Deep cleaning may only be performed in the dedicated wash bay at AS-hubs by trained housekeeping staff or trained ambulance attendants.
2. **Personal Protective Equipment (PPE):**
 - 2.1 The following personal protective equipment should be worn:
 - 2.1.1 Tyvek coveralls
 - 2.1.2 Heavy duty gloves
 - 2.1.3 Safety goggles
 - 2.1.4 Ordinary mask
 - 2.1.5 Waterproof boots
 - 2.2 Following deep cleaning, disposable PPE must be discarded in a black bin and non-disposable PPE should be washed by the end-user with sodium hypochlorite or HMC-approved cleaning solutions.
 - 2.3 PPE may be worn for multiple vehicle deep cleans if a break is not being taken between vehicles.
 - 2.3.1 Bottled water may be consumed while wearing PPE, but gloves must be removed and hands thoroughly washed prior to drinking.
 - 2.3.2 Food should not be consumed while in PPE.
3. **Deep Cleaning Tools and Products:**
 - 3.1 HMC-approved disinfectant solutions such as Surfanios and sodium hypochlorite are to be used for manual decontamination.
 - 3.1.1 Sodium hypochlorite solution concentration should be used as per HMC policy, as follows:
 - 3.1.1.1 For routine surface cleaning and small blood spills use 0.05% (500 ppm).
 - 3.1.1.2 For large blood and body fluid spills use 0.5% (5000 ppm).
 - 3.1.2 Any remaining solution should be discarded after 24 hours.
 - 3.2 **Mops:**
 - 3.2.1 Should only be washed with HMC-approved disinfectant solution, never just with water, and kept dry when not in use.
 - 3.2.2 Mop heads should be replaced once per week, on a Sunday morning.
 - 3.2.3 Old mop heads must be discarded in a yellow bin.

¹Patient transporting vehicles are Alpha, PTS, Bariatric and 4x4 ambulances, and the "green busses"

3.2.4 Mop handles should be washed with HMC-approved disinfectant at the end of shift.

3.3 Hydrogen Peroxide Disinfection Machine:

3.3.1 Should only be used according to manufacturer's instructions by staff trained in its use.

3.3.2 A notice must be placed on the doors of any vehicle undergoing Hydrogen Peroxide disinfection.

3.3.3 Hydrogen Peroxide Disinfection Machine should be cleaned after each use.

4. Frequency of Deep Cleaning of Patient-carrying Vehicles:

4.1 Routine deep cleaning of patient-carrying vehicles shall be conducted on a monthly basis, specifically every 30 days, to ensure optimal hygiene and safety standards.

4.2 Immediate deep cleaning is required:

4.2.1 Following the transportation of any cases involving emerging or re-emerging infectious diseases or outbreaks.

4.2.2 Presence of significant blood and body fluid spills.

4.2.3 As per Infection Control risk assessment and recommendations.

4.3 Any vehicle that fails the Adenosine Triphosphate (ATP) test after deep cleaning must be re-cleaned and disinfected until it meets passing criteria for the ATP test.

4.4 Deep cleaning staff must document all vehicles that have been cleaned, ensuring effective tracking and accountability.

5. Cleaning and Disinfection Process:

5.1 Wipe down the interior surfaces of the patient compartment and driver's cab with HMC-approved disinfectant solution. HMC-approved disinfectant solutions, such as Surfanios and sodium hypochlorite, should be used as per HMC policy, as follows:

5.1.1 For routine surface cleaning and small blood spills use 0.05% (500 ppm).

5.1.2 For large blood and body fluid spills use 0.5% (5000 ppm).

5.1.3 This includes:

5.1.3.1 Removing and cleaning stretchers and equipment.

5.1.3.2 Moving supplies so cabinet interiors can be cleaned.

5.1.3.3 Ceilings, walls, and surfaces.

5.1.3.4 Webbing straps, seatbelts, and response bags.

5.2 Clean any visible body substance-soiling with soap and water, then use the disinfectant solution.

5.3 Floors are mopped with HMC-approved disinfectant solution.

5.4 The electrical machines should be cleaned in a dedicated place according to manufacturer's instruction.

5.5 Replace all equipment and supplies in the vehicle, according to checklist.

6. Sodium Hypochlorite Disinfection:

6.1 When disinfecting surfaces and equipment with sodium hypochlorite, ensure a minimum contact time of 10 minutes before wiping.

6.2 After disinfection, use towels soaked in water to wipe off any remaining solution. This prevents surface damage and discoloration.

- 6.3 If HMC-approved disinfectant wipes are not available, use disposable towels soaked with Sodium Hypochlorite 0.05% solution to wipe electronic devices.
- 6.4 Avoid direct spraying on electronic devices, including the driver's compartment:
- 6.5 Apply solution to a cloth or towel, then gently wipe surfaces.
- 6.6 Ensure the towels are not excessively wet to prevent damage to the devices.

7. Disinfection with Surfanios Solution:

- 7.1 Follow manufacturer's instructions for preparing solutions.
- 7.2 When disinfecting surfaces and equipment with Surfanios Solution, ensure a minimum contact time of 5 minutes before wiping.
- 7.3 Avoid direct spraying on electronic devices, including the driver's compartment:
 - 7.3.1 Apply solution to a cloth or towel, then gently wipe surfaces.
 - 7.3.2 Ensure the towels are not excessively wet to prevent damage to the devices.

8. Hydrogen Peroxide Disinfection:

- 8.1 Hydrogen Peroxide disinfection is preferred after the deep cleaning process; however, this step can be omitted if the machine is unavailable due to maintenance or stock of consumables.
 - 8.1.1 Open all cabinets and lockers and the interior window.
 - 8.1.2 Ensure all surfaces are dried.
 - 8.1.3 Return the cleaned medical equipment and bags inside the vehicle, except the linen.
 - 8.1.4 Close all windows and doors.
 - 8.1.5 Place the machine in patient compartment of ambulance and start program.
 - 8.1.6 Keep vehicle closed for 30 minutes after starting the disinfection process.

- 9. Clean the exterior of the vehicle with powdered soap, soft brooms or cloths, and a hose pipe or high-pressure cleaner.

- 10. The exterior of the vehicle may be cleaned while the Hydrogen Peroxide disinfection machine is underway.

- 11. Do ATP testing to check the effectiveness of cleaning.

- 12. Complete and submit vehicle deep clean report.

13. References:

- 13.1 Cleaning and Disinfection and Sterilization CL 7261
- 13.2 Infection Control in Housekeeping Department CL 7242
- 13.3 Infection Control in Ambulance Service CL 9041

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Group
Procedure	2.5 MANAGEMENT OF OCCUPATIONAL EXPOSURE TO BLOOD AND BODY FLUIDS
Implementation Date	23 June 2024
Purpose	To provide guidance on the actions to be taken by staff who have been exposed to blood or body fluids.
Responsibility	ALL HMCAS GROUP STAFF

1. Blood and body fluid exposure includes contact with blood and/or body fluids, whether an infectious disease is suspected or not, and includes any of:
 - 1.1 Parenteral/percutaneous exposure (i.e. needle sticks, contaminated sharp items, human bites).
 - 1.2 Mucous membrane exposure (i.e. splashes into the eyes or mouth).
 - 1.3 Cutaneous exposure (i.e. exposure to non-intact skin).
2. This SOP does not replace but rather compliments HMC Policy CL7251 – Management of Occupational Exposures to Blood and Body Fluids.
3. **Immediate Actions on Exposure by Employee:**
 - 3.1 Immediately wash/irrigate wounds and skin sites that have been in contact with blood or body fluids with water and/or soap and water irrespective if there are cuts or abrasions present (avoid alcohol-based products).
 - 3.2 Apply a sterile dressing as necessary and apply pressure through the dressing if bleeding is still occurring.
 - 3.3 Do not squeeze or rub the injury site.
 - 3.4 Exposed mucous membranes and eyes should be flushed thoroughly with water (remove contact lenses if worn).
 - 3.5 If blood or body fluids are splashed in the mouth, spit them out and then rinse the mouth thoroughly with water.
 - 3.6 If clothing is contaminated, remove clothing and shower if necessary.
4. **Employee Reporting of Exposure Events:**
 - 4.1 The exposed employee shall inform their direct supervisor immediately after exposure.
 - 4.2 The exposed employee shall report to the Emergency Department of the hospital where the patient is being transported.
 - 4.3 If staff reported to HGH/ED, the Romeo on duty will then liaise with the ED Head Nurse (55847803)/ED Nurse Lead (55814439) to facilitate the process of management.
 - 4.4 The Romeo and all immediate supervisors shall inform the ASG Infection Control Practitioner and relay the source patient's health card number.
 - 4.5 The employee may alternatively report immediately to the Staff Medical Center during working hours (0700 hours to 1500 hours), such as in the case where the patient was not transported to an ED.
 - 4.6 It is the responsibility of the exposed employee to follow any post-exposure management and treatment recommendations made by the treating physician and ASG Infection Control Practitioner and to attend all follow-up appointments.

- 4.7 The employee shall complete an OVA and Blood and Body Fluids Exposure Form as soon as reasonably possible after the exposure.
- 4.8 The Quality Department shall forward the OVA and Blood and Body Fluids Exposure Forms to the ASG Infection Control Practitioner.
- 4.9 The exposed employee shall report to the Staff Medical Center the next day for follow-up.

5. References:

- 5.1 HMC Policy 7251 - Management of Occupational Exposures to Blood and Body Fluids.
- 5.2 CDC Guidance for Evaluating Health-Care Personnel for Hepatitis B Virus Protection and for Administering Post Exposure Management.
- 5.3 World Health Organization (WHO), Recommendations on Exposure to Blood and Body Fluids, Chap 8.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.6 OXYGEN MANAGEMENT BY AMBULANCE SERVICES
Implementation Date	15 May 2018
Purpose	Management, control, and storage of oxygen gases contained in pressurized cylinders.
Responsibility	HMCAS OPERATIONAL AND SUPPORT PERSONNEL

1. Regulations Storing Oxygen and Pressurized Cylinders:

- 1.1 Oxygen is a colourless, odourless medical gas which is supplied in pressurized cylinders and is considered hazardous by the OSHA Hazard Communication Standard (29 CFR 1910.1200).
- 1.2 Care is needed when handling and using compressed medical oxygen cylinders.
- 1.3 Compressed oxygen cylinder potential hazards and precautions:
 - 1.3.1 May cause or intensify fire, Oxidizer
 - 1.3.2 Contains gas under pressure, may explode if heated.
 - 1.3.3 Store away from combustible materials
 - 1.3.4 Keep valves and fittings free from grease and oil.
 - 1.3.5 In case of fire: Stop leak, if safe to do so.
 - 1.3.6 Store in well ventilated area
- 1.4 Ruptured cylinders may rocket
- 1.5 All pressurized cylinders are to be routinely inspected.
- 1.6 The end user must carry out a physical cylinder inspection of integrity before and after use.
- 1.7 Pin index valve(s) integrity check must be carried out regularly.
- 1.8 All damaged and suspected damaged cylinders in storage must be reported immediately to line management.
- 1.9 Standard oxygen cylinder colour orientation is standardised white Ratio 1:5 black cylinders, or green Ratio 1:5 silver cylinders.

2. Types of Oxygen Cylinders, Refilling, Collection and Distribution:

- 2.1 All new cylinders are GCE Integrated Valve with the following sizes:
 - 2.1.1 CD Luxfer – 2 Liter
 - 2.1.2 HX Luxfer – 10 Liter
- 2.2 All old cylinder's sizes are the following:
 - 2.2.1 Size D (ALCAN) – 2 Liter
 - 2.2.2 Size M60 (ALCAN) – 10 Liter
 - 2.2.3 Size M6 (ALCAN) – 1 Liter (Bike)
 - 2.2.4 L45X (Luxfer) – 4.7 Liter (LifeFlight)
 - 2.2.5 MOST 30-A1 – 17,000L (MIR)
- 2.3 The refilling, collection, and distribution of oxygen to all required Ambulance Service hub locations are carried out by the third-party contractor.

3. HMCAS Oxygen Storage Area/Cabinet:

- 3.1 The oxygen storage area/cabinets are supervised daily by the Support Service Team to maintain availability of oxygen for immediate deployment within the Ambulance Service operations.
- 3.2 All HMCAS oxygen storage area/cabinet must maintain a standardised maximum and minimum quantity of full and empty oxygen cylinders.
 - 3.2.1 The temporary storage area at Bu Garn has a maximum quantity of:
 - 3.2.1.1 40 x HX Luxfer 10 Liter and
 - 3.2.1.2 48 x CD Luxfer 2 Liter cylinders
 - 3.2.2 The oxygen storage cabinets at hub locations always maintain measurable cylinder quantities against the actual requirements of the current operations as required.
- 3.3 The oxygen storage area/cabinets must contain appropriate securing devices to ensure the stability of free-standing cylinders.
- 3.4 The oxygen storage area/cabinets have the appropriate racking to secure non-free-standing cylinders.
- 3.5 All full and empty cylinders are clearly separated and defined.
 - 3.5.1 Empty cylinders must be placed in RED-labelled area.
 - 3.5.2 Full cylinders must be positioned in GREEN-labelled area.
- 3.6 Available oxygen storage cabinets are to be utilised where appropriate securing devices in specific storage facilities are not available.
- 3.7 The oxygen storage cabinets should be filled only to their maximum capacity. Overstocking storage cabinets is strictly prohibited.
- 3.8 When taking cylinder from the oxygen storage area/cabinet, ensure that entire tag is present.
 - 3.8.1 All new cylinders:
 - 3.8.1.1 Full cylinder is sealed with shrink plastic wrap and the ENTIRE tag is attached.
 - 3.8.1.2 Empty cylinder is unsealed, has NO tag and/or EMPTY tag is attached.
 - 3.8.1.3 If cylinder is found UNSEALED, consider cylinder is empty thus NOT to be taken.
 - 3.8.2 All old cylinders:
 - 3.8.2.1 Full cylinder has the ENTIRE tag attached.
 - 3.8.2.2 Empty cylinder has NO tag and/or EMPTY tag is attached.
 - 3.8.2.3 If cylinder is found WITHOUT TAG, consider cylinder is empty thus NOT to be taken.
- 3.9 The oxygen storage area/cabinet must be easy to access/egress with emergency access.
- 3.10 The oxygen storage area/cabinet is to be well ventilated and have adequate lighting.
- 3.11 The oxygen storage area/cabinet is to be kept clean and tidy all times.
- 3.12 No smoking or naked flame within the vicinity of the oxygen storage area/cabinet.
- 3.13 No oils or fuel or lubricants store in and proximity of the oxygen storage area/cabinet.
- 3.14 HMCAS oxygen storage area/cabinet are to be marked with appropriate and approved signage.
- 3.15 The oxygen storage area/cabinet must comply with HMC corporate fire regulations.

4. HMCAS EMERGENCY GUIDE – CYLINDER MAINTENANCE:

- 4.1 In case of fire, follow the normal evacuation procedures and ensure that emergency services are informed that medical oxygen cylinders are in the premises.
- 4.2 Use fire extinguisher suitable for type of surrounding fire, if only trained to do so.
- 4.3 Oxygen equipment should be isolated from potential sources of combustion.
- 4.4 Move cylinders/containers from fire area if you can without risk.
- 4.5 Evacuate the immediate surrounding area. Isolate up to 800 metres radius.
- 4.6 Shelter behind building to avoid rocketing pressurized cylinders in a fire risk.
- 4.7 Always stay away from tanks engulfed in fire.
- 4.8 Allow compressed oxygen gas to vent under control if cylinder valve is damaged.

5. HMCAS EMERGENCY GUIDE – FIRST AID:

- 5.1 Move injured personnel away and into fresh air.
- 5.2 Call 999
- 5.3 Give CPR if required
- 5.4 Keep warm
- 5.5 Ensure all personnel are aware of material(s) involved and take precautions to protect.

SOP 2.7: Security of Facilities, Vehicles and Equipment



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations and Production
Procedure	2.7.A USE OF SPOKE AND HUB FACILITIES
Implementation Date	1 October 2016
Purpose	To ensure cleanliness, safety and security of spokes and hubs.
Responsibility	OPERATIONAL CLINICAL PERSONNEL (E.G. ALPHA, BRAVO, CHARLIE, DELTA, OSCAR, MIKE, AND EVENT STAFF)

1. Access to Spoke and Hub facilities:

- 1.1 Use **PIN 4349#** to open the door.
- 1.2 Once you enter the spoke, randomly scan for any damage and cleanliness.
- 1.3 Switch off and lock your vehicle.
- 1.4 If you notice any garbage, property damage or any variance, immediately notify immediate production/distribution supervisor.

2. Standby responsibilities at Spoke or Hub building:

- 2.1 Always keep your medication bag, portable radio, and MDT with you when on standby inside the spoke or hub building.
- 2.2 Plug your MDT and portable radio into wall charger while on standby inside the spoke or hub building.
- 2.3 Tag your refrigerated food with your name and date. Refrigerated food items more than 24 hours old shall be discarded without any notification.
- 2.4 Maintain cleanliness inside refrigerator and microwave oven.

3. Responsibilities when leaving a hub or spoke building:

- 3.1 Remember to take along all your personnel belongings, MDT, radio, and medication bag along with you when you are leaving the spoke or hub building.
- 3.2 Throw all domestic waste into domestic bin located at the hub or outside the spoke.
- 3.3 If you are the last person leaving the building, close and lock all windows and doors.

4. Safety & Security of the Spoke and Hub:

- 4.1 Smoking is strictly Prohibited any HMC Compound and property. ([SOP 2.8](#)).
- 4.2 No unauthorized persons are allowed to enter or remain inside the spoke or hub.
- 4.3 No unauthorized vehicles are allowed to park in the parking area of spoke and hub facilities.
- 4.4 Immediately report any violations, variance or concerns to your immediate supervisor (Production, Distribution) and the NCC via Production-1 radio channel.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Critical Care Services (CCS) and Scheduled Ambulance Service (SAS)
Procedure	2.7.B HELIPAD BASE SECURITY
Implementation Date	01 October 2016
Purpose	To ensure Helipad Base Security and Safety.
Responsibility	ALL CRITICAL CARE SERVICES AND SCHEDULED AMBULANCE SERVICE STAFF

1. Background:

- 1.1 Critical Care Services and Scheduled Ambulance Service use the offices and storage space located at the Hamad General Hospital rooftop helipad. Security of the helipad is to be afforded the highest priority for the following reasons:
 - 1.1.1 Use of helipad by LifeFlight, QEAF, and other helicopters.
 - 1.1.2 Physical security of Retrieval Services capital equipment items and consumables.
 - 1.1.3 Integrity of information security and data protection.

2. Open Up Procedure:

- 2.1 At the start of each working day, the following actions are to be taken by the first person reporting for duty.
 - 2.1.1 A visual check is to be conducted of the area around the offices to rule out the presence of any suspect personnel, objects, or packages.
 - 2.1.2 Once unlocked, keys must be secured in the lockable key press on the wall outside the office.

3. Working Day:

- 3.1 During the course of the working day, the following actions are to be taken as necessary:
 - 3.1.1 Visitors to offices are required to show their ID. Contractors and non-HMCAS personnel are to be escorted at all times.
 - 3.1.2 Suspect personnel must be challenged. If a person has no valid reason to be at the Helipad, security must be called to escort them from the premises.
 - 3.1.3 Unnecessary movement outside while aircraft are landing and taking off is to be avoided.
 - 3.1.4 Access to the Store will be by a combination lock. The store is to be locked when unoccupied.

4. Close Down:

- 4.1 The following action is to be taken at the end of each working day and when the compound is to be unoccupied for any period of time:
 - 4.1.1 All lights are to be switched off.
 - 4.1.2 All computer systems are to be locked or shut down.
 - 4.1.3 Portable electric appliances (e.g. kettles, etc.) are to be unplugged.
 - 4.1.4 Air-conditioning units are to be switched off in offices. In areas where temperature sensitive items are stored, air-conditioning must be set to LOW fan speed with a minimum air temperature of 22 degrees Celsius.
 - 4.1.5 All internal doors are to be closed and external doors are to be locked.

4.1.6 Keys are to be placed in the key press and secured using the combination lock.

5. Combination Codes:

5.1 Combination codes for the key press and the store are to be changed every three months.

6. Security Breach:

6.1 Suspected security breaches of any nature are to be reported immediately to the following personnel:

6.1.1 Operations Manager

6.1.2 Site Security Manager

6.1.3 Senior Operations Manager

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Group
Procedure	2.7.C TRAKA KEY MANAGEMENT SYSTEM
Implementation Date	10 October 2023
Purpose	To give clear guidance on the Usage of Traka Key Management System for HMCAS.
Responsibility	ALL HMCAS GROUP STAFF

1. Purpose:

- 1.1 This Standard Operating Procedure (SOP) outlines the steps for the proper usage of the Traka Key Management System to ensure the secure and efficient management of keys and assets within the HMCAS Group.

2. Scope:

- 2.1 This SOP applies to all authorized personnel who are responsible for using the Traka Key Management System within the HMCAS Group.

3. Responsibility:**3.1 Support Control Center:**

- 3.1.1 Responsible for the overall management and administration of the Traka system.
- 3.1.2 Ensure that all authorized personnel receive proper training on how to use the Traka Key Management System with biometric access effectively and securely.

3.2 Location Supervisor:

- 3.2.1 Responsible for overall usage of the Traka System.
- 3.2.2 Should send details of the new staff to Control Support Center for giving access to the Hub.
- 3.2.3 For coordinating with Support Control Center for reporting malfunction or issues regarding the Traka System key cabinet.
- 3.2.4 For safe keeping and operation of Emergency Keys.

3.3 Authorized Users: Individuals who are granted access to keys or assets through the Traka system.

- 3.4 All the above Users: Responsible for informing Support Control Center (SCC) by calling +974 4032 8300 or by emailing (SS-SupportControlCenter@hamad.qa) for Emergency Key Access and for informing malfunction or issue with Traka Key Cabinet.

4. Procedure:**4.1 Accessing the Traka Key Management System with Biometric Authentication:**

Log in to the Traka system using biometric authentication procedure, such as fingerprint scan, to open the door and pick the keys that are in green light indicator. The red light indicator is not accessible until the access provided by the SSC.



4.2 **New Key Request Using Biometric Authentication:**

- 4.2.1 To request access to a key, Please fill Key Access Form and send it to (SS-SupportControlCenter@hamad.qa) with details of mentioning the reason for request and Hub details.
- 4.2.2 The Team will decide and give the access to the required Key in the online Traka System.
- 4.2.3 After the system is Synced, you can access the Key Cabinet and get the required key.
- 4.2.4 For Emergency please call SCC **+974 4032 8300**.

4.3 **Key Retrieval Using Biometric Authentication:**

- 4.3.1 Once the request is approved by the system or authorized personnel, go to the designated Traka cabinet or location.
- 4.3.2 Authenticate yourself using biometric access to open the cabinet and access the requested key indicated by Green light.
- 4.3.3 Close the Cabinet door and it will get locked automatically. If there is alarm sound it means the cabinet is not closed properly, close the door till the alarm sound is off.

4.4 **Using the Key:**

- 4.4.1 Ensure that the key is used responsibly and only for the approved purposes.
- 4.4.2 Do not do unauthorized handover of the key. Only Ambulance Vehicle Keys can be handed over to the next crew during the shift change.
- 4.4.3 Do not share access or leave keys unattended.

4.5 **Returning the Key:**

- 4.5.1 Log in to the Traka system using biometric authentication procedure, such as fingerprint scan, to open the door.
- 4.5.2 Return the key to its designated slot in the Traka cabinet promptly where the red light is blinking.
- 4.5.3 Confirm the return within the Traka touch system.
- 4.5.4 Close the Cabinet door and it will get locked automatically. If there is alarm sound it means the cabinet is not closed properly, close the door and hold it till the alarm sound is off.

4.6 **Reporting Issues:**

- 4.6.1 Report any issues, malfunctions, or damage to keys or the biometric access system immediately to the immediate Supervisor/Support Control Center (SCC) by calling +974 4032 8300 or by emailing (SS-SupportControlCenter@hamad.qa).
- 4.6.2 Do not attempt to repair or modify the Traka key Cabinet System.
- 4.6.3 Location Supervisor need to communicate with Support Control Center for any issues, malfunctions, or damage to keys or the biometric access system with the Key Cabinet.

4.7 **User Access Management:**

- 4.7.1 New User Creation and Access is provided by Support Control Center. The Location Supervisor must send the details of the new employee via email to Support Control Center (SS-SupportControlCenter@hamad.qa).

- 4.7.2 Location Supervisor to assist in registration of biometric access to the new staff at the Key Cabinet.
- 4.7.3 The Support Control Center is responsible for managing user access rights, including adding, modifying, or removing users as necessary in coordination with communication from Location Supervisor.
- 4.7.4 Regularly review, update and remove resigned/terminated staff user access permissions as needed.
- 4.8 **Auditing and Monitoring:**
 - 4.8.1 The Traka system logs all key transactions and biometric access events.
 - 4.8.2 System Logs and transactions should be reviewed monthly to ensure compliance and security.
 - 4.8.3 User Access to be reviewed quarterly.
 - 4.8.4 Conduct internal audits to verify that the system is being used correctly.
- 4.9 **Emergency Procedures:**

In case of emergencies or system failures, contact Location Supervisor and in case not available, contact Support Control Center (SCC) by calling **+974 4032 8300** and follow the instructions as below:

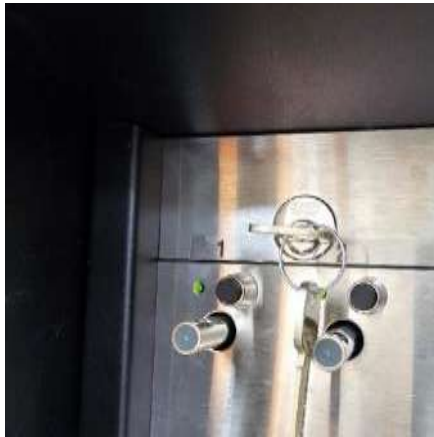
 - 4.9.1 Call Location Supervisor/SCC and report the emergency for accessing the Emergency Key for the Cabinet and Keys.
 - 4.9.2 Answer all the questions and follow instructions from SCC.
 - 4.9.3 Get the pin access from SCC to open the MASTER LOCK and get the **Key A** (Cabinet Emergency Key) and **Key B** (iFob Release Emergency Key).



- 4.9.4 **With Key A – Open the Key Cabinet**



4.9.5 With **Key B** - Turn on the Emergency Key Release



4.9.6 Remove the designated iFob Key for use

4.9.7 Turn off the Emergency Key Release with Key B, close and lock the Key Cabinet Door with Key A.

4.9.8 Return the key to MASTER LOCK

4.9.9 Call back location Supervisor or SCC and mention key used.

4.9.10 Support Control Center should change the pin of the MASTER LOCK.

4.10 **Documentation:**

4.10.1 SCC to Maintain records of all key transactions, user access changes, biometric access events and system maintenance activities.

4.11 **Compliance:**

4.11.1 All Staff to adhere to SOP and any variance will lead to disciplinary actions.

4.12 **Review and Update:**

4.12.1 Periodically review and update this SOP to ensure it remains accurate and aligned with organizational needs and changes in the Traka system.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.8 USE OF TOBACCO PRODUCTS
Implementation Date	1 October 2016
Purpose	To maintain a healthy, clean, and safe environment for all personnel, patients, and visitors.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Smoking of tobacco products (inclusive of but not limited to cigarettes and electronic/smokeless cigarettes):

- 1.1 Do not smoke anywhere inside or outside all HMC premises.
- 1.2 Do not smoke in any HMCAS vehicle, in both driver and patient/passenger compartments.
- 1.3 Do not smoke within 15 meters of any HMCAS vehicle.
- 1.4 Do not smoke/chew/use tobacco products during any patient care activities.
- 1.5 Always present yourself in a professional manner, free of any tobacco odour during all patient care activities.

2. PLEASE NOTE:

- 2.1 'No Smoking' Policy SA 1052 - "this policy is formulated for all Hamad Medical Corporation (HMC) staff in conformance with the Qatar Law No. (20) of the year 2002 on the Control of Tobacco and its derivatives, and to provide a safe environment to protect patients, visitors and staff from Active and Passive Smoking Exposure."
- 2.2 Anyone found to be smoking on-site will be subject to a full and thorough investigation, potentially leading to formal disciplinary action.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.9 FITNESS TO OPERATE
Implementation Date	25 February 2020
Purpose	To provide guidance on fitness to operate within the HMC Ambulance Service.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Commitment:

- 1.1 HMCAS committed to providing a safe and healthy working environment for staff and patients by actively working to optimize staff health and fitness.
- 1.2 HMCAS personnel have a professional responsibility to actively engage with health and fitness guidance.
- 1.3 HMCAS is committed to ensuring staff recover fully following a period of sick leave by ensuring that staff return to work within their required working hours (for a defined period) without completing additional over-time.

2. Medical Fitness:

- 2.1 All staff must undergo a medical assessment at the commencement of their employment with HMCAS, including:
 - 2.1.1 HIV test.
 - 2.1.2 Hepatitis B test.
 - 2.1.3 Tuberculosis test.
 - 2.1.4 Chest X Ray.
 - 2.1.5 Vital signs (including blood pressure).
- 2.2 In the absence of the provision of a list of notifiable diseases by the Qatar Ministry of Public Health, staff are required to report the following occurrences to Staff Clinic:
 - 2.2.1 Exposure/Potential Exposure to: [Diseases notifiable under UK Law](#).
 - 2.2.2 Exposure/Potential Exposure to: [Infectious diseases listed as notifiable by the US Centre for Disease Control](#).
 - 2.2.3 Any illness or injury that may compromise their fitness to operate on a temporary or permanent basis.
- 2.3 Pregnant staff may continue their frontline operational duties until unable to safely continue their patient facing role at which time they will continue their duties in a non-frontline role unless booked on light duty or booked off work by a doctor earlier.

3. Diet and Physical Fitness:

- 3.1 Whilst there is no mandatory fitness requirement for service HMCAS, all personnel are strongly encouraged to maintain a level of personal fitness that will allow them to fulfil their work obligations without unnecessary fatigue or injury and permit optimum longevity. This should incorporate a combination of cardiovascular training (e.g. jogging, walking, cycling), resistance training (e.g. weight training and bodyweight exercises) and flexibility (e.g. yoga/Pilates). Swimming offers a good combination of resistance and Cardio Vascular training with the benefit of reduced impact on bones and joints.
- 3.2 Personnel are strongly encouraged to maintain a healthy weight through making appropriate dietary choices in combination with exercise.

3.3 As healthcare professionals, staff should avoid smoking tobacco. (See Policy SA 1052).

4. Aeromedical Considerations:

4.1 Aeromedical personnel should not donate blood within 72 hours of commencing flying duties.

4.2 Aeromedical personnel must not participate in scuba diving within 24 hours of commencing flying duties.

4.3 No member of staff may undertake aeromedical duties if, for any reason, their physical or mental state is such that it could endanger the safety of the aircraft, crew, or patient(s).

4.4 Aeromedical personnel are duty bound to report concerns about the mental or physical fitness of themselves or others to the appropriate line manager.

4.5 Any aeromedical personnel deemed unfit to perform flying duties should report the same to the appropriate line manager.

4.6 Individuals prone to airsickness should discuss the matter with the appropriate line manager before undertaking any flight duties.

5. Sickness:

5.1 Staff who are suffering from any illness or injury that is contagious or potentially debilitating are not fit for patient-facing duties and should report sick to the Staff Clinic or an alternative medical facility. A sick note/medical report is to be obtained and submitted to the appropriate line manager in accordance with [HMC Sick Leave Procedure](#).

6. Returning to duty post a period of sick leave:

6.1 Following a period of sick leave HMCAS staff are not permitted to work overtime or provide event cover (unless the event is part of their scheduled hours) for a minimum of 10 days unless approved by the appropriate senior operations manager.

6.2 Senior Operational Managers (and above) may extend this period of 'no overtime and/or no events' based on staff sickness record.

6.3 Following a period of sick leave of 3 months or more [SOP 7.7](#) 'returning to service following a break in service' is to be followed.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.10 AMBULANCE SERVICE FATIGUE MANAGEMENT
Implementation Date	1 October 2016
Purpose	To provide guidance on management of fatigue for Ambulance Service personnel.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Commitment and Responsibility:

- 1.1 HMCAS is committed to maintaining a safe and healthy working environment for staff and patients through risk management activities, including active management of fatigue.
- 1.2 To comply with the HMC Health and Safety Mission: *'Protect Life and Asset from Occupational Health and Safety hazards – professionally and with minimum business interruption.'*
- 1.3 This SOP applies to all elements of Ambulance Service.
- 1.4 All personnel, including staff from partner organizations have a professional responsibility to actively engage with Fatigue Management activities.
- 1.5 Staff is duty bound to report when they are fatigued for any reason.
- 1.6 Self-declaration of being 'Not Fit for Duty' due to fatigue demonstrates professional maturity, integrity, strength of character and indicates active participation in risk management and promotion of patient safety.
- 1.7 Operational line managers are responsible for monitoring staff compliance and the provision of mitigating options.
- 1.8 On long distance Transfers and Aeromedical Mission Team leaders are responsible for monitoring team members for signs of fatigue and for the implementation of fatigue management activities, including allocation of rest time for napping, eating, and drinking.

2. Aim:

- 2.1 To establish and maintain a proactive system that minimizes fatigue-related risks to the health, safety, and welfare of HMCAS staff and patients.

3. Fatigue and Work:

- 3.1 Line managers will take active measures to ensure that personnel arrive fit for duty at all times. This includes roster planning that provides adequate opportunity for recovery sleep between shifts and during missions (in the case of aeromedical operations). The following mandatory post-flight rest periods will apply:
 - 3.1.1 24 hours and 2 local nights following flight between UK/Europe and Qatar.
 - 3.1.2 24 hours and 2 local nights following flight between Far East/Thailand and Qatar.
 - 3.1.3 48 hours and 2 local nights following flight between Qatar and United States of America.
 - 3.1.4 Where flights consist of more than one sector, a period of 24 hours rest will apply for each sector to a maximum of 72 hours.
 - 3.1.5 An additional 24 hours in the country might be approved for certain cases.
 - 3.1.6 Deviation from the matrix will be on a case-by-case basis and requires approval from the Operations Manager - Critical Care Services.

3.2 The following behaviour is indicative of fatigue and should prompt the initiation of countermeasures:

- 3.2.1 Nodding off/dozing (microsleep)
- 3.2.2 Hunger and/or thirst
- 3.2.3 Moodiness, irritability, or anger
- 3.2.4 Lack of concentration
- 3.2.5 Eye rubbing
- 3.2.6 Boredom
- 3.2.7 Yawning, 'heavy' eyelids
- 3.2.8 Fidgeting
- 3.2.9 Lack of coordination
- 3.2.10 Unusually quiet and withdrawn

4. Countermeasures:

4.1 The only cure for fatigue is sleep. The following countermeasures can be implemented:

- 4.1.1 'Power Nap'/'Combat Nap'. A short period of sleep less than 30 minutes duration. This provides sufficient time for restorative sleep without entering REM (deep) sleep. This sleep phase has the highest awakening threshold. **Power naps are usually followed by 5-20 minutes of sleep inertia, characterized by confusion and sluggishness. This factor must be considered by team leaders/managers.**
- 4.1.2 Quick fix countermeasures including:
 - 4.1.2.1 Keeping active and mobile, avoiding sitting for long periods.
 - 4.1.2.2 Completion of manual tasks
 - 4.1.2.3 Switching between tasks to avoid monotony
 - 4.1.2.4 Consumption of 150-200mg caffeine (2-3 cups coffee)
 - 4.1.2.5 Static exercise (e.g. jogging on the spot)
 - 4.1.2.6 Avoidance of large/carbohydrate-rich meals
- 4.1.3 Quick fix countermeasures are not a substitute for sleep.

5. Active Management:

5.1 The following principles apply to ground operations:

- 5.1.1 Shifts may not exceed 12 hours (plus travel period to rural dispatch locations) without authorization from line manager.
- 5.1.2 Rest periods between shifts shall not be less than 10 hours.
- 5.1.3 No other duties (meetings, training etc.) should be undertaken during the 10 hours preceding duty.
- 5.1.4 Any fatigue break taken during rostered hours will be considered duty hours.
- 5.1.5 Where operational activity precludes the scope for a full 10-hour rest period, line managers must take action to mitigate the risk, including use of replacement staff, negotiating later start times etc. Where this is not possible, the risk must be escalated to the appropriate Senior Operations Manager.

- 5.1.6 Individual staff members are responsible for ensuring that they obtain optimum rest during the minimum 10 hours rest period prior to duty.
- 5.2 The following principles apply to aeromedical operations:
 - 5.2.1 Team composition will be configured to permit in-mission rest periods as per Crewing Matrix. Deviation from Crewing Matrix may only be authorized by SOM or his designee.
 - 5.2.2 Shifts may not exceed 14 hours without authorization from an Operations Manager - Critical Care Service.
 - 5.2.3 The rest period prior to commencement of aeromedical duties shall not be less than 10 hours.
 - 5.2.4 No other duties shall be undertaken (meetings, training etc.) in the 10 hours preceding duty.
 - 5.2.5 Any fatigue break taken during rostered hours will be considered duty hours.
 - 5.2.6 Where operational activity precludes the scope for a full 10-hour rest period, Team Leaders, in conjunction with Operations Manager - Critical Care Service must take action to mitigate the risk, including the use of replacement staff, negotiating later start times etc. Where this is not possible, the risk must be escalated to Senior Operations Manager - Critical Care Service or his designee.
 - 5.2.7 Individual staff members are responsible for ensuring that they obtain optimum rest during the 10 hours rest period prior to duty.
 - 5.2.8 Aeromedical personnel may not work for more than 6 consecutive shifts without at least one full day off, or work for more than 60 hours per week.
 - 5.2.9 Aeromedical personnel are duty bound to report perceived fatigue affecting other team members.
 - 5.2.10 Aeromedical personnel reserve the right to refuse a mission if they believe that crew, patient, or aircraft safety is compromised as a result of fatigue, following discussion with the Operations Manager - Critical Care Service.
 - 5.2.11 Circumstances likely to break the rules highlighted at 5.2.1 and 5.2.7 must be actively managed by Operations Manager - Critical Care Service, options include crew swaps and pre-positioning of replacement crews.
 - 5.2.12 Roster scheduling must take into account on-call, actual and potential hours worked to anticipate fatigue management issues.
 - 5.2.13 On call personnel activated for a mission will commence a period of 10 hours rest from the point at which they return home after mission completion.

6. Emergent Situations:

- 6.1 Fatigue management guidelines may only be broken in exceptional circumstances. Authority to breach fatigue management may only be granted by an Assistant Executive Director.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.11 WORKPLACE VIOLENCE
Implementation Date	31 January 2017
Purpose	To maintain a healthy and safe violence-free environment for all personnel, patients, and visitors.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Definitions:

- 1.1 **Workplace Violence:** Behaviour in which an employee, former employee, patients or visitor to a workplace inflicts or threatens to inflict damage to property, serious harm, injury, or death to others at the workplace. Behaviours may include any act or threat of physical violence, harassment, intimidation, or other threatening disruptive behaviour, as well as threats and verbal abuse.
- 1.2 **Threat:** The implication or expression of intent to inflict physical harm or actions that a reasonable person would interpret as a threat to physical safety or property.
- 1.3 **Intimidation:** Making others afraid or fearful through threatening behaviour.

2. Procedure following a violent incident at the workplace:

- 2.1 HMCAS does not tolerate ANY behaviour, implied or actual, that constitutes workplace violence.
- 2.2 In the event of a violent act/altercation, immediately inform your direct supervisor.
- 2.3 If the offender is the supervisor him/herself, inform the appropriate section manager.
- 2.4 Before beginning any inquiry, Senior Management and the Human Resources Department shall be consulted.
- 2.5 The Senior Operations Manager (SOM) or the Department Assistant Executive Director (AED) shall be responsible for coordinating a response to the incident.
- 2.6 In instances that involve emergency situations or criminal activity, update your supervisor, and proceed to the hospital for medical attention, as appropriate. These incidents shall be referred to the Police Department for assessment and, if necessary, an inquiry.
- 2.7 In instances when it is not appropriate to refer an incident to the Police Department, the SOM/AED/ED and the Human Resources Director shall attend to the case and make a recommendation regarding the need for an inquiry.
- 2.8 The HMCAS Quality section shall coordinate all inquiry processes.
- 2.9 All parties involved in the reported incident shall be interviewed by the relevant supervisor/manager/AED or ED as appropriate, in order to develop a true and accurate account of the incident.
- 2.10 All inquiries shall be conducted in a manner that demonstrates objectivity, fairness, and a concern for confidentiality.
- 2.11 Inquiries may lead to further actions, depending on liabilities and legal implications.
- 2.12 **Disciplinary actions** may involve a warning (verbal or written), workplace transfer, or termination of employment, depending on the seriousness of the case.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	2.12 HAND HYGIENE
Implementation Date	23 June 2024
Purpose	To ensure that Ambulance staff practicing hand hygiene to prevent and control the risk of transmission of infections.
Responsibility	ALL OPERATIONAL STAFF

1. DEFINITION:**1.1 Hand Hygiene:**

1.1.1 A general term that applies to any action of hand cleansing which includes routine handwashing, antiseptic hand washing, antiseptic hand rubbing and surgical hand antisepsis.

1.1.2 Hand Hygiene is considered as the single most important procedure to prevent the spread of pathogens in the healthcare settings.

1.2 Hand Hygiene Practices:**1.2.1 Hand washing**

1.2.1.1 Routine/Social hand wash: Washing hands with plain soap and water.

1.2.1.2 Antiseptic/Hygienic hand wash: Washing hands with antimicrobial soap and water.

1.2.1.3 Antiseptic Hand Rubbing (or Hand rubbing) – Cleaning hands with the use of a waterless antiseptic hand rub.

1.3 Hand Hygiene Products:

1.3.1 Liquid Soap – Soap that do not contain antimicrobial agents, or that contain very low concentrations of antimicrobial agents effective solely as preservatives.

1.3.2 Alcohol-based (hand) rub (Waterless antiseptic agent) – An antiseptic agent that does not require the use of water. It comes in different formulations e.g. liquids, gels, or foam.

1.3.3 Antimicrobial soap (Antiseptic agent) – An antimicrobial substance which reduces and/or inhibits the growth of microorganisms on living tissues.

2. PROCEDURE:**2.1 Hand Hygiene Practices in the Scene and Ambulances:**

2.1.1 Readily accessible alcohol-based hand rubs should be available in all ambulance vehicles access to all facilities to perform hand washing if needed.

2.1.2 The staff shall perform hand rubbing If hands are not visibly soiled but are contaminated.

2.1.3 When hands are visibly dirty or contaminated with visible material or visibly soiled with blood or other body fluids use the disinfect wipes to remove the visible fluids and use hand rubbing and hand washing is required after hand over the patient in the hospital.

2.2 Hand Hygiene Technique with Alcohol-Based Formulations: (Duration of the entire procedure: 20-30 seconds).

- 2.2.1 Apply a palm full of the product in a cupped hand, covering all surfaces (or follow the manufacturer's specification).
- 2.2.2 Rub hands palm to palm, right palm over left dorsum with interlaced fingers and vice versa.
- 2.2.3 Palm to palm with fingers interlaced
- 2.2.4 Backs of fingers to opposing palms with fingers interlocked
- 2.2.5 Rotational rubbing of left thumb clasped in right palm and vice versa
- 2.2.6 Rotational rubbing, backwards and forwards with clasped fingers of right hand in left palm and vice versa
- 2.2.7 Once dry, your hands are safe
- 2.3 **Hand Hygiene Practices in Healthcare Facilities, Hubs, and Spokes:**
 - 2.3.1 The staff shall perform hand rubbing if hands are not visibly soiled but are contaminated.
 - 2.3.2 The staff shall perform hand washing:
 - 2.3.2.1 When hands are visibly dirty or contaminated with visible material, or visibly soiled with blood or other body fluids, or if exposure to potential spore-forming organisms (e.g. *C. difficile*) is strongly suspected or proven.
 - 2.3.2.2 After using the toilet, after changing diapers, or cleaning up a child who has used the toilet.
 - 2.3.2.3 Before eating
 - 2.3.2.4 After blowing your nose, coughing, or sneezing
 - 2.3.2.5 When caring for patient with diarrhea or suspected exposure to *Clostridium difficile*; *Bacillus anthracis* (Anthrax).
 - 2.3.2.6 After touching garbage
- 2.4 **Handwashing Procedure** (Duration of the entire procedure: 40-60 seconds).
 - 2.4.1 Select a comfortable water temperature
 - 2.4.2 Wet hands with running water
 - 2.4.3 Apply plain soap or antimicrobial soap to cover all surfaces of the hands
 - 2.4.4 Rub hands palm to palm
 - 2.4.5 Right palm over left dorsum with interlaced fingers and vice versa
 - 2.4.6 Palm to palm with fingers interlaced
 - 2.4.7 Backs of fingers to opposing palms with fingers interlaced
 - 2.4.8 Rotational rubbing of thumbs clasped in right palm and vice versa
 - 2.4.9 Rotational rubbing backwards and forwards with clasped fingers of right hand in left palm and vice versa.
 - 2.4.10 Rinse the hands under running water to remove all soap residues keeping the hands lower than the forearms.
 - 2.4.11 Dry (pat dry) hands with paper towel
 - 2.4.12 Use paper towel to turn off the faucet or use your elbow/foot to turn off the faucet to prevent recontamination whichever is applicable.

2.4.13 Dispose paper towels into a foot operated bin

3. General Indications for Handwashing and Hand Antisepsis:

- 3.1 All staff members are required to adhere to the WHO's 'Your 5 Moments of Hand Hygiene' guidelines when providing patient care, demonstrating proficiency in hand hygiene techniques. Before having direct contact with patients.
 - 3.1.1 Before donning gloves for an invasive procedure (i.e. starting an IV).
 - 3.1.2 After contact with body fluids or excretions, mucous.
 - 3.1.3 After contact with a patient's (i.e. taking a blood pressure, lifting patient)
 - 3.1.4 After teaching patient sounding; (including medical equipment)
 - 3.1.5 After removing gloves

4. Monitoring of Hand Hygiene Practices:

- 4.1 The Romeo-on-duty shall be responsible for monitoring hand hygiene practices among staff and shall report staff compliance to the Infection Control Head on a monthly basis.
- 4.2 The monitoring process will focus on the 4th Moment of Hand Hygiene, which occurs after patient contact and the removal of gloves.
- 4.3 Staff monitoring shall take place in specific facilities, namely the Emergency Departments and Dialysis units.
- 4.4 Staff compliance to hand hygiene protocols shall be communicated to the IC Committee and displayed on television screens for staff awareness.

5. References:

- 5.1 WHO Save Lives Clean Your Hands: Guide to Implementation of the WHO Multimodal Hand Hygiene Improvement Strategy Revised August 2009
- 5.2 CDC, Hand Hygiene Guidelines <http://www.cdc.gov/handhygiene/>
- 5.3 HMC Hand Hygiene Policy 2024

How to Handwash?

WASH HANDS WHEN VISIBLY SOILED! OTHERWISE, USE HANDRUB



Duration of the entire procedure: 40-60 seconds



Wet hands with water;



Apply enough soap to cover all hand surfaces;



Rub hands palm to palm;



Right palm over left dorsum with interlaced fingers and vice versa;



Palm to palm with fingers interlaced;



Backs of fingers to opposing palms with fingers interlocked;



Rotational rubbing of left thumb clasped in right palm and vice versa;



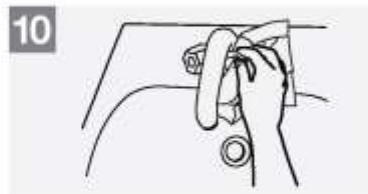
Rotational rubbing, backwards and forwards with clasped fingers of right hand in left palm and vice versa;



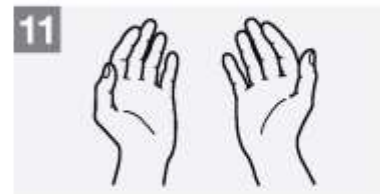
Rinse hands with water;



Dry hands thoroughly with a single use towel;



Use towel to turn off faucet;



Your hands are now safe.



World Health Organization

Patient Safety

A World Alliance for Safer Health Care

SAVE LIVES
Clean Your Hands

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May 2009

How to Handrub?

RUB HANDS FOR HAND HYGIENE! WASH HANDS WHEN VISIBLY SOILED

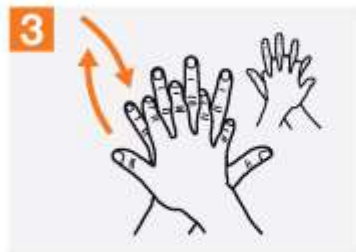
 **Duration of the entire procedure: 20-30 seconds**



Apply a palmful of the product in a cupped hand, covering all surfaces;



Rub hands palm to palm;



Right palm over left dorsum with interlaced fingers and vice versa;



Palm to palm with fingers interlaced;



Backs of fingers to opposing palms with fingers interlocked;



Rotational rubbing of left thumb clasped in right palm and vice versa;



Rotational rubbing, backwards and forwards with clasped fingers of right hand in left palm and vice versa;



Once dry, your hands are safe.



World Health Organization

Patient Safety

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SAVE LIVES
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July 2009

SOP 3: Communications



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.1 NCC CALL ROUTING
Implementation Date	01 October 2016
Purpose	- To direct the routing of emergency and non-emergency calls. - To direct the routing of calls for Patient Transport Services units requesting for Emergency Assistance.
Responsibility	ALL COMMUNICATIONS STAFF

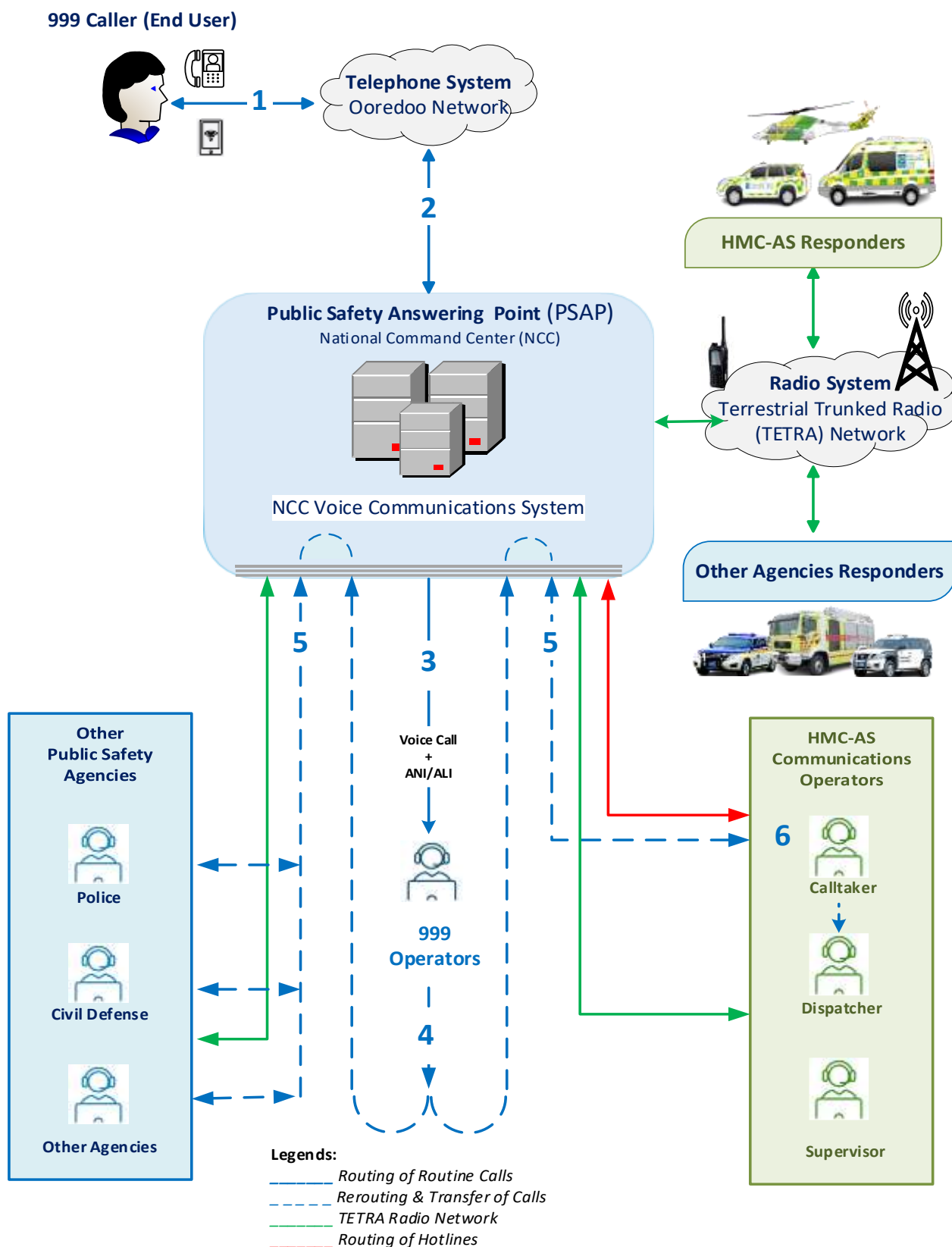
1. HMC Ambulance Service (HMCAS) utilizes the NCC Emergency Services “999” network and acts as a secondary Public Safety Answering Point (PSAP) for Medical Emergency calls.
2. **Medical Emergency Calls are answered first by 999 Operators then routed to HMCAS Call Takers:**
 - 2.1 For direct ‘999’ ambulance requests, the **HMCAS Call Taker should:**
 - 2.1.1 Immediately identify the address of the emergency.
 - 2.1.2 Route the Call for Service (CFS) to the HMCAS Dispatcher without delay.
 - 2.1.3 Process the call through ProQA and update the dispatcher.
 - 2.2 For calls where the ‘999’ caller does not directly request an ambulance:
 - 2.2.1 The 999 operator will route the CFS to the AS dispatcher (and log the HMCAS as a participant to the primary agency being dispatched)
 - 2.2.2 **The HMCAS dispatcher should:**
 - 2.2.2.1 Dispatch an Alpha Unit (priority one/ two based on participant classification).
 - 2.2.2.2 Notify the supervisor
 - 2.2.2.3 Task a call-taker to call-back the caller
 - 2.2.2.4 Override the dispatch priority based on the new ProQA update.
 - 2.2.3 **The HMCAS Call Taker should:**
 - 2.2.3.1 Process the case in the original CFS or link with new CFS, if the voice call has been received and is still online.
 - 2.2.3.2 Make all attempts to call-back the caller if the caller voice not received.
 - 2.2.3.3 Process the case through ProQA and update the dispatcher.
 - 2.2.3.4 Log a note in the free text area of the original CFS about the call-back attempts if the caller does not answer the call-back.
3. **Dedicated Ambulance Service Hotline (44797955):**
 - 3.1 HMC Call Takers shall directly answer the dedicated hotline, usually accessed by HMC facilities and other parties requiring the ambulance service. In these situations, the 999 operators are bypassed.
 - 3.2 The call taker should open a CFS manually only if it is not automatically created in the NJM CAD system.
 - 3.3 The number of hotlines is limited to the number of call takers available to answer the calls. Other incoming calls will be kept on hold until a call taker becomes available.

4. Misrouted Emergency calls:

- 4.1 Identify the location of the caller and verify the information.
- 4.2 Quickly identify non-ambulance calls inadvertently received.
- 4.3 Ask the caller not to hang up and inform him/her about the call transfer.
- 4.4 Transfer the identified calls to the appropriate agency, using the most expedient method.
- 4.5 Verify that the call was transferred successfully.
- 4.6 Notify the shift supervisor when misrouted calls are received.

Call Routing	
1	A 999 call is placed by an end user within network.
2	Ooredoo delivers the emergency call to the Voice Communications System (VCS) switch at the Primary Public Safety Answering Point (PSAP) at the National Command Centre (NCC).
3	The NCC VCS delivers the call with its Automatic Number Identification (ANI) and Automatic Location Identification (ALI) to the 999 operator. The caller's number and address are displayed on the NCC operator's Life-X monitor and in NJM CAD.
4	999 operator triage the call and transfer it to the appropriate agency.
5	The Call is then rerouted back to VCS to be routed to HMC Ambulance Service Call Taker.
6	• The HMC AS call taker receives the call and obtains & confirms caller's number and the incident's location as well the call details: • The HMC AS call taker may stay online with the caller or can terminate the call as determined in the Medical Priority Dispatch System. • After termination of the call, the Ambulance Service Medical Dispatcher or case controller may call back the caller at any time to gather more information about the address/patient/situation. • If the line disconnects or the caller hangs up before the termination of the interrogation, the Ambulance Service operator must call back the caller • When multi agency response is required HMC AS call taker or case controller will notify the required agency via NJM and Life-X console. • When appropriate and possible, the HMC AS Call taker shall transfer the call to the operator of the concerned agency.

How 999 Calls are Routed to the Ambulance Service



SB & LGP 2024

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.2 NCC CALL TAKING
Implementation Date	09 March 2020
Purpose	To ensure effective and efficient receipt and handling of all emergency calls within the HMCAS Communications Centre within the NCC.
Responsibility	ALL COMMUNICATIONS STAFF (SPECIFICALLY CALL TAKERS)

1. GENERAL CALL TAKING STANDARDS:

- 1.1 Prompt Answering: Answer all emergency calls arriving at the Ambulance Service consoles within 5 to 10 seconds.
- 1.2 Order of Answering Priority: Call takers must prioritize answering of calls; first '999' line, then Hotlines, then administrative lines.
- 1.3 Answer all '999' & Hotlines calls with the phrase, **"Ambulance Service, this is (ASMD's name) What is the address of the emergency?"**
"خدمة الإسعاف, معاك (فلان/فلانة) ما هو عنوان حالة الطوارئ؟"
- 1.4 Answer non-emergency line calls with the phrase **"Ambulance Service, this is (ASMD's name) how may I help you?"**
"خدمة الإسعاف, معاك فلان/فلانة, كيف أقدر أساعدك؟"
- 1.5 For non-emergency calls received on emergency lines, Refer to [SOP 3.14](#) and do not transfer the call to an administrative line.
- 1.6 Consider calls reported by HMCAS clinical or administrative staff via the phone to be routine emergency calls and be processed in same way as other 999 calls.
- 1.7 Interrogation and information gathering: Obtain the following minimum basic information from the caller:
 - 1.7.1 The address or exact location of the incident/patient.
 - 1.7.2 The call back number.
 - 1.7.3 The type of emergency.
 - 1.7.4 Medical Priority Scripted Questions and Dispatch Life Support (DLS) Instructions when appropriate using ProQA ®.
 - 1.7.5 All required information for the Computer Aided Dispatch (CAD).

2. STEP-BY-STEP SUMMARY OF TAKING A 999 CALL:

Once the call prompts in Life-X, immediately answer the call within 5 seconds
The Call taker will open a CFS manually if NJM does not open it automatically.
Identify and verify the exact location (zone, street, building number), telephone number and document it accurately in NJM.
Route the CFS for "initial dispatch" as "Priority1" Lights and siren.
Open ProQA to process the questioning and identify the Chief Complaint and the Dispatch Code.
Make sure NJM is updated with ProQA code.
Coordinate with the dispatcher to upgrade or downgrade the responding unit as per ProQA code.

Continue with the ProQA process to complete the questioning and provide Post-Dispatch/Pre-arrival Instructions when appropriate.

Terminate the call as per the MPDS protocol.

Fill in the administrative fields in NJM.

Follow up the case with the dispatcher until the arrival of the crew and call back the caller if needed.

3. ADDRESS IDENTIFICATION & VERIFICATION: To provide all Emergency Medical Dispatchers with approved procedures and practices for obtaining and verifying accurate and complete address and phone number.

3.1 Address Identification & Verification:

- 3.1.1 Address identification and verification is a crucial step to ensure that the emergency responders are dispatched to the correct location.
- 3.1.2 All emergency phone lines will always be answered in the following manner:
“Ambulance Service, this is (ASMD's name) What is the address of the emergency?”
This may be repeated in Arabic.
- 3.1.3 The call-taker shall consider the address method volunteered initially by the caller as the preferred method for the address identification, especially when it is clear and accurate. Call-takers will not prompt the caller to leave the patient's side to look for the Qatar Area Referencing System (QARS) number unless the address provided initially doesn't include enough information to enable the initial dispatch.
- 3.1.4 The call-taker will enter the address or location provided by the caller into the NJM CAD system using the most accurate information available from the caller (This could be QARS number, a numeric address, intersection, business, landmark, etc.)
- 3.1.5 Once the call-taker has entered the address/location into the NJM CAD system, he/she will geo-verify the entered address/location by ensuring that the system returns a valid address or location and it matches the initial information entered and obtained from the caller.
- 3.1.6 After documenting the address in the CAD, the call taker will verify the address (or location) by stating the following: “Please repeat the address/location for verification.” The call-taker will then verify the address details entered in the CAD against the second statement of the caller and shall correct mistakes if any.
- 3.1.7 The location of the event may be available from the Automatic Identification Location (ALI) feature in the NJM; the call taker will check the obtained address from the caller against the displayed location in the NJM.
- 3.1.8 If the address appearing automatically in the NJM CAD matches the initial address provided by the caller and after asking the address identification question “What's the address of the emergency?” the call taker will not ask the verification question “Please repeat the address/location for verification” instead he/she will state to the caller “Your address appears correctly in the map.” However, other details of the address shall be identified and documented properly in the CAD as stated above.

3.2 Unknown/Unclear Address: If the caller is unable to provide a numeric address.

- 3.2.1 The call-taker will say “Please give me the best location/ nearest landmark you have for the incident.”
- 3.2.2 Call-taker will notify the Team leader to follow up with him closely.

- 3.2.3 For all residential (or suspected residential) locations the call-taker will ask “Is this a villa, compound or an apartment?” and correctly enter this information into the CAD system. The call-taker will also ask if the QARS (Qatar Address Referencing System) number is known.
- 3.2.4 If the QARS number is not available and/or for non-residential locations, the call-taker will obtain all necessary access information, which may include: building name, business name, floor number, apartment, office or suite number, specific entrance instructions and intersection or street segment (for roadway incidents).
- 3.2.5 If the caller cannot obtain any of the information above: the call-taker shall advise the caller to give an alternative telephone number of someone who can provide clear address information.
 - 3.2.5.1 When possible and applicable, advise the caller to find and provide the Blue Plate - Qatar Address Reference System (QARS) in the entrance of the building and provide at least 3 consecutive plate numbers (zone, street, building, & Unit number “if available”).
 - 3.2.5.2 When the QARS number is not available, advise the caller to describe the directions to get the ambulance to his location.
 - 3.2.5.3 If all attempts fail, advise the caller to send someone to rendezvous with the ambulance at a mutually known location.
 - 3.2.5.4 If necessary, request an alternative contact number of someone able to guide the ambulance to the incident location. Keep the first caller on the line while the second caller is called. The call taker may choose one of the two option:
 - 3.2.5.4.1 The initial caller shall be kept online for the call interrogation and the second caller shall be contacted by another call-taker or the team leader.
 - 3.2.5.4.2 The initial call taker can put the caller on hold and contact the second telephone number given. If no answer, he should go back to the caller and look for other alternative.
- 3.2.6 The call-taker will seek the assistance of the team leader or the OSCAR any time he has difficulties to identify the exact address.
- 3.2.7 The official HMCAS WhatsApp number might be used as an alternative to find location in challenging situation.
- 3.2.8 When address cannot be identified or patients does not know his location, DTL/Oscar NCC should be notified, they should add police investigation and it is required to notify the corresponding agency by phone to endorse the case.

4. PHONE NUMBER VERIFICATION:

- 4.1 For each call, a call back number should be verified twice by asking the caller “**What is the phone number you are calling from?**”
- 4.2 If the stated number given by the caller matches the Automatic Number Identification (ANI) information in the NJM, this serves as double verification.
- 4.3 If the ANI information does not match the number given by the caller, the call taker will not repeat the number back, but instead, verify the number by asking the caller again to repeat the phone number.

- 4.4 The call taker shall make a cross check verification that the telephone number is typed correctly in the NJM.

5. TRANSFERRING EMERGENCY CALLS TO ANOTHER AGENCY (NON-AMBULANCE REQUESTS):

- 5.1 The call taker shall follow the below steps:
 - 5.1.1 Advise the caller to call 999 again and request the appropriate agency.
 - 5.1.2 Notify the DTL/OSCAR.
 - 5.1.3 Add comments in the CFS that the caller is not requesting Ambulance services and he/she was advised to call back 999 and close the CFS.

6. TAKING NON-STANDARD CALLS:

- 6.1 Abandoned Calls: When the caller disconnects before the can be answered by call taker, call taker should attempt a call back twice. Make additional attempts to contact the caller if there are no other emergency calls in the queue.
- 6.2 Disconnected Calls: When the caller disconnects after being answered by a call taker and before it can be determined if assistance is needed, call taker should attempt a call back twice. Make additional attempts to contact the caller if there are no other emergency calls in the queue.
- 6.3 Silent calls: When the caller is not responding to the call taker and remains silent, call taker will escalate the case to the DTL/OSCAR who will notify NCC officer of all silent calls for any possible transfer to "992" console, to determine if the caller is attempting to report an emergency using a special communications device for hearing impaired individuals.

If the call taker hears background noises indicating an emergency is occurring, such as domestic violence or other medical emergencies, the call taker should initiate the appropriate response and add the relevant agencies to the case.
- 6.4 Misdialed/ Unintentional Calls: when the caller stays on the 999 line and admits a 'misdial' or unintentional call and there is no indication of an emergency situation, no further action required.
- 6.5 Prank calls: when someone calls 999 for a joke, or calls and deliberately hangs up for fun, call taker will Treat as a real emergency until proven otherwise. Call back the called party and query to determine if further action or an emergency response is required.

Call taker will escalate confirmed Prank calls to the DTL/OSCAR

7. NEW CALLS REPORTED VIA RADIO BY ON-DUTY HMC AMBULANCE PERSONNEL:

- 7.1 The dispatcher will instruct a call taker to open a new CFS.
- 7.2 The dispatcher should notify DTL immediately.
- 7.3 The DTL shall ensure that one call taker is in charge of opening the CFS.
- 7.4 The call taker will omit ProQA and document in NJM the appropriate classification (Code):
 - 7.4.1 AS10X for Priority 2
 - 7.4.2 AS10Y for Priority 1
- 7.5 The dispatcher will send extra resources as per the request of the clinical crew on-scene.

8. FOREIGN LANGUAGE CALLS:

- 8.1 When available, the foreign language translation support service shall be used.
- 8.2 **General Guidelines:**

- 8.2.1 MPDS card sets/ProQA™ Paramount, containing English/Arabic language protocols for Emergency Medical Dispatching, shall be made accessible to all call-taking positions.
- 8.2.2 The English card sets/Pro QA™ paramount shall be used by ASMDs skilled in the English language for the interrogation of English-speaking callers to the 999 system, and for the provision of PDI's and PAI's.
- 8.2.3 It is recognized that the standard Arabic language version of the MPDS protocols may not always contain language understood by all persons that speak Arabic, given the cultural and geographic diversity of the Arabic speaking population. Subsequently, ASMD's with the necessary language skills may rearticulate those words and phrases necessary to facilitate the caller's understanding of the question or instructions in their own words after the scripted protocol question or instruction has been attempted.
- 8.2.4 Regardless of the language used by the caller, call receiving and dispatch for medical assistance shall be provided in a standardized manner following approved MPDS protocol scripts for 999 caller interrogation and provision of post-dispatch and pre-arrival instructions.
- 8.2.5 ASMDs with the appropriate language skills must therefore ensure that any translation of the protocol scripts, whether using a foreign-language ProQA or spontaneous translation, reproduces the same meaning, syntax and phrasing as contained in the Arabic/English languages MPDS.
- 8.2.6 No modification or omission of questions is allowed when providing spontaneous translation.
- 8.3 **Managing calls with language barrier:**
 - 8.3.1 Notify the DTL immediately that a language barrier call is in progress.
 - 8.3.2 Attempt to identify the language of the caller by asking what country they are from or interpretation of their response to various language options.
 - 8.3.3 Attempt to obtain the address by using landmarks, shops, or schools in the area.
 - 8.3.4 The Case Controller or DTL will determine if any call taker on duty is able to speak the caller's language. If someone is available:
 - 8.3.4.1 The initial call taker shall inform the caller about the transfer and shall use courtesy even with language barrier cases.
 - 8.3.4.2 The call taker shall transfer the call to the appropriate line extension and shall endorse the case in a reasonable time.
 - 8.3.4.3 The second call taker shall process the case using ProQA and all questions should be translated with an equivalent language. No changes to the questions meaning or order are tolerated.
 - 8.3.5 If no information can be obtained, no call taker with language skills is available. process ProQA and allocate 32-B-4 code (language barrier, no interpreter).
 - 8.3.6 The DTL should contact MOI to determine if their call takers are able to talk in the caller's language and then transfer the call to the MOI call taker to obtain the correct address and incident details.
 - 8.3.7 The DTL or Case Controller shall obtain an update from the MOI call taker and notify the dispatcher.
 - 8.3.8 **Managing Abusive callers:** (Refer to [SOP 3.20](#))

9. DUPLICATE CALLS: (When several calls have been received on the same incident).

- 9.1 When more than one call is received for the same incident, the call-taker must complete the Case Entry interrogation, and then determine if a complete ProQA interrogation is required, based on the caller's answer to Case Entry questions.
- 9.2 The call-taker shall answer every emergency call with the expectation that the caller will be reporting new information not yet received from previous callers, including those cases where an incident record may already exist in the CAD for the address/location provided by the caller and will complete Case Entry according to standard MPDS procedures.
- 9.3 Before determining a reported event to be a duplicate of an active incident, the ASMD must confirm that three information are strictly similar in the below order:
 - 9.3.1 Address/location is the same as an existing incident already entered in the CAD.
 - 9.3.2 Incident description (Chief Complaint, Mechanism of Injury).
 - 9.3.3 The caller statement that he called before (within the same time span) for the same incident or another person called earlier from a different number.
- 9.4 When the caller is a third-party caller (not in the immediate vicinity of the event/victim), and the call-taker determines the event to be a duplicate incident, and no new information is offered by the current caller during the Case Entry interrogation, the ASMD call-taker may terminate the phone call once Case Entry interrogation is completed only after ensuring that another call-taker is processing the call with a second party caller.
- 9.5 If new information is provided by the caller during Case Entry, the call-taker must complete the Key Question interrogation and assign a final coding according to standard procedures.
- 9.6 If the new coding is a lesser priority than the previous code assigned, the response priority will not be changed.
- 9.7 If the new coding is a greater priority than the previous code assigned, consideration shall be given to upgrading the response priority.
- 9.8 When the caller is a first or second-party caller (either a victim/patient or someone with the victim/patient), the ASMD must complete the Key Question interrogation, assign a final coding, and provide DLS instructions according to standard procedures.
- 9.9 If the call taker ascertained that the call is duplicate, he shouldn't dispatch a new CFS. But he should find the original CFS and update the details (location and ProQA) if provided.
- 9.10 If the call taker discovers while processing ProQA that the call is duplicate, he should inform the dispatcher and provide him with any update. The dispatcher should reject the duplicated CFS and link it together.
- 9.11 Confirm that location and incident is the same as an existing incident by different means in NJM.
- 9.12 Open ProQA and complete Case entry according to standard EMD procedures.
- 9.13 Update the NJM with any changes and additional information and inform the dispatcher of any required upgrade or downgrading.
- 9.14 Accurately document contact details of 2nd or 3rd caller in the "COMMENT" section of NJM.
- 9.15 Should the caller be 2nd party, complete the ProQA process including scripted PDI's and PAI's.
- 9.16 Should the caller be a 3rd party and no new information is given, then the call taker must reassure him/her that an ambulance has already been dispatched to the incident.

- 9.17 Once the call taker is sure that the caller is reporting an incident that has already been received and any additional pertinent information is obtained, the caller will be advised that the call information has already been reported. These calls should be disconnected as soon as possible in order not to tie up any emergency lines unnecessarily.

10. POST-DISPATCH INSTRUCTIONS (PDIS):

- 10.1 The call-taker giving PDIs will follow the protocol, giving instructions appropriate to each individual call, and avoiding free-lance information.
- 10.2 PDI's shall be provided to the caller whenever possible and appropriate to do so.

11. PRE-ARRIVAL INSTRUCTIONS (PAIS):

- 11.1 Pre-arrival instructions should be given verbatim when appropriate and possible as per the MPDS (i.e. Cardiac Arrest, choking, unconscious, Childbirth...)
- 11.2 In the event that the caller hangs up before PAIs can be given, the call-taker must make all effort to re-contact the caller.
- 11.3 Call Management when providing Pre-Arrival Instructions:
- 11.3.1 The ASMD will never ask permission to give instructions. The best way to handle reluctant callers is to simply start giving the instructions. Most callers expect to take direction from the trained ASMD on the other end of the phone line.
- 11.3.2 If the caller refuses to follow PAIs, the ASMD will make reasonable efforts to keep the caller online.
- 11.3.3 If the caller refuses to administer aid, the ASMD will ask if there is someone else there to speak to; if so, the ASMD will attempt instructions again with the new caller.

12. EMERGENCY INTER-FACILITY PATIENT TRANSFERS:

- 12.1 Treat as a routine 999 call
- 12.2 If unable to understand medical terminology, then the call must be transferred to another Call Taker capable of handling the call (NOTIFY the CC Emergency/Oscar-NCC).
- 12.3 Follow Protocol 33 (PRIORITY-1) or 37 (PRIORITY-2) via ProQA.
- 12.4 Update the dispatcher accordingly with regards to Zulu/Yankee/X-ray status.
- 12.5 Transfer the call to the CC Emergency via Life-X only if at least one of the below conditions apply:
- 12.5.1 Special equipment required
- 12.5.2 CCP requested by the healthcare facility
- 12.5.3 LifeFlight requested by the healthcare facility
- 12.5.4 Medication or blood transfusion to be monitored or given during ambulance transport.
- 12.6 Where applicable, verify successful transfer of the call to CC Emergency, otherwise notify CC Emergency/Oscar to call back the healthcare facility.
- 12.7 The CC Emergency should update the dispatcher accordingly.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.3 NCC CALL DISPATCHING
Implementation Date	01 October 2016
Purpose	To standardize the HMCAS Communication Centre Dispatch Procedure for emergency cases.
Responsibility	ALL COMMUNICATIONS STAFF, SPECIFICALLY CALL DISPATCHERS

1. General rules for Dispatch:

- 1.1 The dispatcher receiving the request for assistance shall ascertain the nature of the call, the address and other pertinent information as quickly as possible to permit rapid dispatch.
- 1.2 Obtain additional information and relay to responding units after initial dispatch.
- 1.3 Upon receipt, immediately air calls for service in the pending queue.
- 1.4 During extreme circumstances, always dispatch the highest priority first when multiple calls are in the pending queue. If calls are of the same priority, dispatch with the principle of first-come first-served.
- 1.5 One dispatcher is assigned to monitor each operational channel.

2. Dispatch Priority: Units to be dispatched in accordance with the Dispatch Matrix.

DISPATCH PRIORITY	DISPATCH CODE	VEHICLES TO DISPATCH
ONE	ZULU	1 Alpha Unit, 1 Charlie unit, and 1 Delta unit if available, moving with lights and siren . Life Flight to be dispatched if the call is outside Doha.
ONE	YANKEE	1 Alpha Unit & 1 Delta unit moving with lights and siren .
TWO	X-RAY	1 Alpha Unit & Delta unit if available moving without lights and siren .
THREE	Routine Interfacility Transfers	1 Alpha Unit or patient Transport Service Unit (Foxtrot) moving without lights and siren for non-emergency transport reasons

3. Role of the Dispatcher:

- 3.1 At the beginning of the shift to coordinate with the supervisor to make sure that calls belonging to the assigned Hub are highlighted in LIGHT GREY. Verify own assigned geographical area in the CAD (NJM).
- 3.2 Continuously monitor and observe the screen for pending list (screen containing emergency calls in queue for dispatch).
- 3.3 ACTIVATE emergency calls within five seconds (5) of being routed from the call taker.
- 3.4 Once a CFS is activated, immediately visualize the CFS in the map.
- 3.5 Open the unit status screen and scroll to visualize ALL available and relevant HMCAS units close to the incident.
- 3.6 Initiate the dispatch via the MDT by assigning the CLOSEST and most relevant unit to the incident within ten (10) to fifteen (15) seconds and try not to exceed thirty (30) seconds during peaks periods. If the closest unit is on a different radio channel, coordinate with the relevant

dispatcher to instruct the unit to switch radio channels. The field crew shall respond to the call as per the MDT and will not wait for the confirmation message through the Radio.

- 3.7 Confirm the dispatch by calling the unit call sign twice e.g. *"Calling A1.1 - A1.1, this is Control, Over"*. On reply, provide the initial response priority and the location e.g. *"A1.1 Priority one in Gharafa: from Shamal RA towards Sheikh Faisal RA, Over."*
- 3.8 If the ProQA response matrix (X-ray/Yankee/Zulu code) is available, replace the 'response priority' with the 'response matrix code' on initial dispatch.
- 3.9 Inform the crew that "Refer to the MDT for *more information*".
- 3.10 Simultaneously monitor the Responding unit's AVL and unit status updates in NJM. If the MDT is not available, the dispatcher shall coordinate with the crew via the radio and will enter all unit status from his/her NJM console.

4. Dispatcher Information Sharing:

- 4.1 The communication between to the dispatcher and the field crew shall be kept to the minimum and the unit shall read all CFS updates in the MDT. Necessary information to be given to the responding units may include but not be limited to the following:
 - 4.1.1 Address details not documented in the CAD or if guidance to access the patient is necessary.
 - 4.1.2 Additional address information specific to special situation and explicitly said by the caller should be documented in the Call record and relayed to the responding crew e.g. 'patient is in the annexure behind the villa...'
 - 4.1.3 Infection control concerns and risk of Communicable Diseases.
 - 4.1.4 Upgrade or downgrade of the CFS priority
 - 4.1.5 Cancellation of the CFS
 - 4.1.6 All available safety information, including details about a violent situation, presence of weapons, hazardous materials, crime scene or dangerous animals, etc.....
 - 4.1.7 For RTAs information are available in the MDT but if possible the dispatcher can repeat them via the radio for general broadcast to alert supervisors units: type of RTA and Mechanism of Injury, number of vehicles involved, number of patients, the presence of any hazard, fire, or explosion.
 - 4.1.8 Update the responding unit if there are drastic changes in the situation, including the patient's condition or changes in the scene or in the route.

5. Resource allocation:

- 5.1 When ProQA® code is available the Dispatcher shall update the response priority and coordinate resources as required i.e. either, upgrade, downgrade, or override ProQA code.
- 5.2 Assign and deploy extra resources as necessary (in consultation with the control room supervisor) in cases of:
 - 5.2.1 Cases or any situation with one patient who requires more than two paramedics such as Cardiac arrest or poly-trauma patient.
 - 5.2.2 More than one critical victim
 - 5.2.3 Multiple casualties
 - 5.2.4 Safety concerns/high risk situations
- 5.3 If the first assigned unit is unavailable or not answering the radio call within 30 sec.

- 5.3.1 Immediately dispatch the next closest available unit. Do not cancel the initial unit from the CFS.
- 5.3.2 Notify the Distribution Team Leader
- 5.3.3 Contact the initial unit using other alternative methods; call by phone or mobile. If they reply and they accept the CFS then the disaster shall decide, in coordination the DTL, which unit is the closest to continue to the call.
- 5.3.4 The communications supervisor shall notify the Hub supervisor of the unit with delayed deployment and make necessary escalations and follow-up.

6. Follow up with responding units:

- 6.1 Follow up the location and status of the responding unit until arrival on scene.
- 6.2 Document in the “comments” free text area of the CFS record to document all important information and events during response to the scene.
- 6.3 Monitor the accuracy of MDT user unit status capture by the responding crew or accurately capture the reported “On Scene” time if the MDT is not available.
- 6.4 Inquire scene/patient information report from the responding crew.
- 6.5 Confirm the Qatar Early Warning Score of the patient (QEWS) when applicable and accurately document it in the call record.
- 6.6 Follow up the unit’s on-scene duration and escalate if needed.
- 6.7 When units are **leaving the scene**, and moving to the receiving facility:
 - 6.7.1 Monitor the time once the unit reports (10/20) they are leaving the scene.
 - 6.7.2 Verify and confirm the destination, the priority of transport and the number of patients to be transported.
- 6.8 Upon **arrival at the receiving hospital/facility**:
 - 6.8.1 Monitor the reported time in the Call record
 - 6.8.2 Follow up the status of the unit at the facility
 - 6.8.3 Notify the DTL and the HMCAS supervisor located at the receiving facility for units staying more than 15 minutes.
 - 6.8.4 Document any applicable information in the call record e.g. the reason for delay.
 - 6.8.5 Review the documentation in the call record, update it, and complete it accurately.
- 6.9 When the **unit reports availability**:
 - 6.9.1 Log the reported time in the call record
 - 6.9.2 Log all administrative data, including patient details
 - 6.9.3 Close the call record of the incident
 - 6.9.4 Assess the current standby coverage and immediately dispatch the available unit to the standby point.

7. Requests for HELP/ASSISTANCE from Primary Crew while on scene:

- 7.1 See table below for allocation of units when additional crew assistance is requested.

INITIAL DISPATCH PRIORITY	INITIAL DISPATCH CODE	ADDITIONAL VEHICLES TO DISPATCH FOR ASSISTANCE OF INITIAL UNITS ON SCENE
ONE	ZULU	Additional ambulance units or resources as required/requested for the incident according to the primary crew on scene. Inform DTL and Clinical Desk.
ONE	YANKEE	Closest Charlie unit (LF if case outside Doha) and Delta unit. Other Alpha unit if required. Inform DTL and Clinical Desk if required.
TWO	X-RAY	Closest Charlie unit (LF if case outside Doha) and Delta unit. Other Alpha unit if required. Inform DTL and Clinical Desk if required.
THREE	Routine Interfacility Transfers	Closest Alpha unit, Delta unit; Charlie unit (LF if case outside Doha). Inform DTL and Clinical Desk if required

7.2 Requests for Scene Safety assistance:

- 7.2.1 Activate police and/or Civil Defense and inform the Case Controller & DTL about the request as applicable.
- 7.2.2 Notify the on-scene unit that the requested assistance is responding when appropriate.

7.3 When **Special Scene or patient management assistance** is requested.

- 7.3.1 Inform the DTL/PTL the need to activate the requested units e.g. 2nd unit for cardiac arrest cases, Bariatric unit, Green Bus, 4X4 Ambulance.
- 7.3.2 Dispatch the required resources to the incident.

7.4 Unit moving priority 2 or 1 to hospital needing help:

- 7.4.1 Dispatch the closest available Charlie/Delta/Alpha unit
- 7.4.2 Update the dispatched unit with the latest patient's status and conditions.
- 7.4.3 Arrange for a rendezvous point and guide the dispatched unit.
- 7.4.4 Re-establish transport priority towards hospital as needed.
- 7.4.5 Inform DTL and Clinical Desk
- 7.4.6 Record all information in the comments area of the CFS. Information to be documented includes but not restricted to:
 - 7.4.6.1 The time when the first unit requested help
 - 7.4.6.2 Reason for requesting help
 - 7.4.6.3 Safety concerns
 - 7.4.6.4 Rendezvous time of both units

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.4 OBVIOUS DEATH, EXPECTED DEATH & PRE-ARRIVAL CPR INSTRUCTIONS
Implementation Date	25 February 2020
Purpose	To handle obvious death and expected death cases by following the Medical Priority Dispatch System protocol and local procedures approved by the HMCAS Medical Director.
Responsibility	ALL COMMUNICATIONS STAFF (SPECIFICALLY CALL TAKERS AND DISPATCHERS)

1. Protocol 9 (Cardiac or Respiratory Arrest/Death) Obvious Death Criteria:

1.1 **Obvious Death:** A patient's condition that can be identified as incompatible with life, after all information has been obtained on the Case Entry Protocol, and Protocol 9 (Cardiac or Respiratory Arrest/Death).

1.1.1 The call taker shall:

1.1.1.1 Determine the patient to be not conscious and not breathing, through proper application of the MPDS, ensuring specific answers to Protocol 9.

1.1.1.2 Ensure that the presence of at least one of the above 'obvious death' conditions is unquestionable:

1.1.1.2.1 Cold and stiff in a warm environment

1.1.1.2.2 Decapitation

1.1.1.2.3 Decomposition

1.1.1.2.4 Incineration

1.1.1.2.5 Non-recent death (confirmed as being greater than six hours).

1.1.1.2.6 Severe injuries obviously incompatible with life.

1.1.1.2.7 Brain matter expelled

1.1.1.2.8 Evisceration of chest or abdominal organs

1.1.1.2.9 Submersion (confirmed as being greater than 6 hours)

1.1.1.3 Not provide resuscitative measures i.e. PAIs for breaths and chest compression in any 'Obvious Death' situation.

1.1.1.4 Code all "obvious death" cases as 9-B-1 and inform responders of specific obvious death condition.

1.1.1.5 Keep the caller on the line and provide emotional support, where applicable and possible.

1.1.2 All other criteria not mentioned under section 1.2.2 that appears in the Obvious Death, will not be approved for use under the HMCAS Medical Director's authority.

1.2 Expected Death:

1.2.1 Do Not Resuscitate (DNR) case reported by HMC doctors within HMC facilities where caller reporting terminal illness with DNR order.

1.2.2 Resuscitative measures including PAIs for breaths and chest compressions will not be provided in any 'Expected Death' situation.

- 1.2.3 The call taker must get specific answers to all applicable questions on protocol card 9 to arrive at an 'expected death' determination.
- 1.2.4 Code all "expected death" cases as 9-Omega-1
- 1.2.5 If applicable and possible, keep the caller on the line and provide emotional support.
- 1.3 **Cardiac Arrest Pathway Box:**
 - 1.3.1 **(C Only)** Continuous compressions is NOT APPROVED.
- 1.4 **Pre-Arrival Instructions conveyed by Call Takers:**
 - 1.4.1 Attempt pre-arrival instructions on all cardiac/respiratory arrest cases where the obvious death and expected death determinants (9-B-1 and 9-Omega-1) are not applied.
 - 1.4.2 Do not say "Would you like me to tell you how to do CPR?"; simply proceed with PAIs.
 - 1.4.3 If the caller refuses to follow PAIs, make reasonable efforts to convince the caller e.g. say, "The ambulance is on its way, but this is important to give the patient the best possible chance until it arrives." Repeat as necessary.
 - 1.4.4 If the caller is unwilling to administer aid, ask if there is someone else you can speak to.
 - 1.4.5 If no one else is available, keep the caller on the line and provide emotional support.
 - 1.4.6 Make it clear that if they change their mind about providing patient care you will tell them exactly what to do.
 - 1.4.7 If the caller mention that a healthcare provider is on scene.
 - 1.4.7.1 If CPR is in progress, the call taker shall reminder the caller to use the AED and provide AED instructions when possible.
 - 1.4.7.2 If not sure that CPR is in progress the call taker shall start the PAIs until he/she confirms that a healthcare provider is doing CPR to the patient. Otherwise continue the PAIs.
 - 1.4.7.3 Add a note to NJM and escalate to the supervisor and remain online for further assistance until ambulance is on scene.
 - 1.4.8 Remain polite and courteous at all times.

2. Protocol 14 (DROWNING/DIVING/SCUBA ACCIDENT) Obvious Death Criteria:

- 2.1 **Obvious Death:** The following situation is considered Obvious Death:
 - 2.1.1 Submersion for 6 or greater hours

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.5 FIRST PARTY CALLER (PATIENT) “NOT FOUND ON ARRIVAL”
Implementation Date	01 October 2016
Purpose	To ensure appropriate intervention in the case of a first-party caller (patient) appearing to be “not found” or “gone” upon arrival at the scene.
Responsibility	ALL HMCAS COMMUNICATION STAFF

1. Never assume that a first-party caller (patient) “has left” until that has been reasonably established based on the procedure below.
2. If the arriving crews report back to dispatch that nobody appears to be at the address initially given by the caller, the doors and windows are locked, and a call back from dispatch goes unanswered, the following advice should be given to the crew on the scene:
 - 2.1 Make sure the crew are at the correct address
 - 2.2 Knock at the door several times, when applicable
 - 2.3 Search around the scene/house/property to make sure there is no other entrance.
 - 2.4 Wait on scene until the police patrol arrives
3. **The dispatcher** should immediately notify the Shift supervisor of the situation and inform the relevant call taker to call back the caller (if not available any other call taker shall call back).
4. **The call taker shall call back several times** on the verified call-back number. For a minimum of 6 times of call-backs (during low call volume period the call taker can call back up to 15 times).
5. **The call taker shall verify that any call-back number that is not answered is indeed a correct number.**
6. Simultaneously the call taker or the shift supervisor (whoever is available) should notify police by calling the **police NCC console** and brief them with incident details with clear request of dispatching a police patrol to respond to the scene and search for the patient.
7. DO NOT advise responders to break-in to the scene/house.
8. **FIRST PARTY callers classified in the below listed critical determinant codes, are considered to be vulnerable patients** and an extra level of regard is required because they appear to be initially stable, but could later deteriorate, therefore they might warrant a legal break-in by police whenever patient is not found on arrival:

CLINICAL CONDITION	CODE
Allergies (reactions)/Envenomation (Stings, Bites)	2-D-2 Difficulty speaking between breaths
Breathing problems	6-E-1 INEFFECTIVE BREATHING (for obvious extreme respiratory distress 1 st party) 6-D-2 Difficulty speaking between breaths
Burns (Scald)/Explosion (Blast)	7-D-5 Difficulty speaking between breaths
Carbon monoxide/Inhalation/Hazmat/CBRN	8-D-4 Difficulty speaking between breaths
Chest pain/Chest Discomfort (Non-traumatic)	10-D-2 Difficulty speaking between breaths
Heart problems/A.I.C.D.	19-D-2 Difficulty speaking between breaths
Hemorrhage/Laceration	21-D-4 Dangerous haemorrhage
Overdose/Poisoning	23-C-3 Antidepressant (tricyclic) 23-C-5 Narcotics (heroin)
Psychiatric/Mental Health Conditions/Suicide Attempt/Abnormal behavior	25-D-4 Dangerous haemorrhage 25-D-5 Near hanging, Strangulation, or Suffocation (with difficulty breathing) 25-B-3 INTENDING suicide 25-B-4 Jumper (Threatening) 25-B-5 Near hanging, Strangulation, or Suffocation (alert without difficulty breathing)
Stab/Gunshot/Penetrating Trauma	27-D-4 Central wound 27-D-5 Multiple wounds 27-B-3 Serious haemorrhage

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.6 HIGH RISK PREGNANCY COMPLICATIONS
Implementation Date	25 February 2020
Purpose	To handle all pregnancy, childbirth, and miscarriage cases by following the MPDS protocol and local procedures approved by HMCAS Medical Director.
Responsibility	ALL HMCAS COMMUNICATION STAFF, SPECIFICALLY CALL TAKERS

1. **High-risk complications (pregnancies)** are defined as conditions that place the mother, fetus, or both at higher-than-normal risk to complications during or after the pregnancy and birth.
 - 1.1 Provide pre-arrival instructions as indicated by the patient's condition and the DLS links in the MPDS protocols.
 - 1.2 For premature or multiple births, provide delivery instructions whenever possible and safe to give.
 - 1.3 Keep the caller on the line for emotional support, and give exit instructions, if pre-arrival delivery instructions are not necessary.
2. All factors listed below are considered high risk complications during pregnancy and any time the caller reports one or more of the below conditions, the call taker shall select the determinant descriptor "HIGH RISK complications" and code the case as a 24-D-5 (only if higher codes are not applicable).
 - 2.1 **High Risk Complications:**
 - 2.1.1 Premature birth (24-36 weeks)
 - 2.1.2 Multiple birth (equal or greater to 24 weeks)
 - 2.1.3 Bleeding Disorder
 - 2.1.4 Blood Thinners
 - 2.1.5 Cervical cerclage (stitch)
 - 2.1.6 Placenta abruption
 - 2.1.7 Placenta Previa
 - 2.1.8 FEMALE GENITAL MUTILATION
 - 2.2 **Maternal factors: (HMCAS factors):**
 - 2.2.1 Age (younger than age 15, older than age 35)
 - 2.2.2 Weight (Pre-pregnancy weight under 45.35 Kg or obesity)
 - 2.2.3 Height (under 152.4cm/1.5m)
 - 2.2.4 Previous pregnancy complications (stillbirth, fetal loss, preterm labor and/or delivery, small- for- gestational age baby, large baby, seizure, pre-eclampsia, or eclampsia).
 - 2.2.5 More than five previous pregnancies
 - 2.2.6 Bleeding during the third trimester
 - 2.2.7 Abnormalities of the reproductive tract
 - 2.2.8 Uterine fibroids
 - 2.2.9 Hypertension
 - 2.2.10 Rh incompatibility

- 2.2.11 Gestational diabetics
- 2.2.12 Any infection
- 2.2.13 Fever
- 2.2.14 Acute surgical emergency (appendicitis, gallbladder disease, bowel obstruction).
- 2.2.15 Post-term pregnancy
- 2.2.16 Pre-existing chronic illness (such as asthma, autoimmune disease, cancer, sickle cell anemia, tuberculosis, herpes, AIDS, heart disease, kidney disease, Crohn's disease, ulcerative colitis, diabetes).

2.3 **Fetal factors include:**

- 2.3.1 Exposure to infection (especially herpes simplex, viral hepatitis, mumps, rubella, varicella, syphilis, toxoplasmosis, and infection caused by Coxsackievirus).
- 2.3.2 Exposure to damaging medications (especially phenytoin, folic acid antagonists, lithium, streptomycin, tetracycline, thalidomide, and warfarin); exposure to addictive substances (cigarette smoking, alcohol intake and illicit or abused drugs).
- 2.3.3 Prenatal tests indicating a serious health problem (for example, a heart defect).

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.7.A THE EMERGENCY RULE
Implementation Date	01 October 2016
Purpose	To ensure that EMDs complete the minimum MPDS requirements for a safe and effective response during situations of extreme system overload.
Responsibility	ALL COMMUNICATIONS STAFF

1. In situations of extreme over-capacity in the emergency call center, the EMD may suspend Pre-Arrival Instructions (PAIs) and suspend or discontinue Post-Dispatch Instructions (PDIs) once Case Entry, Key Questions and final coding are completed.
2. If there are unanswered emergency calls waiting in the 999 queue, the EMD shall make every effort to answer those calls ensuring that any current case(s) being processed are given a Final Determinant Code and specific responder crews are notified (or will be imminently notified by a co-worker).
3. **Emergency Rule Situations:**
 - 3.1 Make every attempt to complete the interrogation sequence.
 - 3.2 At a minimum, correctly obtain the Case Entry and Key Question information, and assign a final Determinant Coding.
 - 3.3 The following are the acceptable reasons for call takers to suspend or discontinue the MPDS protocol:
 - 3.3.1 All call takers are already busy taking emergency calls, with one or more calls in queue (*waiting to be answered*).
 - 3.3.2 Mass Casualty Incidents (where both phone and radio traffic are extreme).
 - 3.3.3 When instructed by shift supervisor/Oscar NCC.
 - 3.4 Release the caller with the following statement (or similar) ... "I need to hang up now to take another call). Help is on the way. If he/she gets worse in any way, call me back immediately for further instructions." Also, whenever appropriate, say "If he/she becomes less awake and vomits, quickly turn him/her on his/her side."
 - 3.5 Notify the shift Supervisor
 - 3.6 Document in ProQA "free text" comments field that Emergency rule applied.
 - 3.7 If the call taker used manual MPDS flipchart sets, the comment "Emergency rule applied" will be logged in the CAD.
 - 3.8 Once the peak period is over and when possible and appropriate the call taker will make call back attempts to continue the interrogation and provide instructions.
4. **Tracking of Emergency Rule Cases:**
 - 4.1 The EMD call taker will record all incidents where he/she applies the Emergency Rule.
 - 4.2 The supervisor will document in the shift daily report the time when the emergency rule was applied and the trip number of the cases when applicable.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.8 DISPATCHING WITH MOBILE DATA TERMINALS
Implementation Date	01 October 2016
Purpose	To ensure efficient workflow during dispatch via Mobile Data Terminals.
Responsibility	ALL COMMUNICATIONS STAFF

1. The Dispatcher will verify the MDT signal in NJM: *(This will be repeated each time the MDT goes offline)*.
 - 1.1 If the indicator is **RED**, the Dispatcher shall report “**Negative.**” The Responder should then log off, log on again, and repeat the confirmation again with the dispatcher.
 - 1.2 If the indicator is **GREEN**, the Dispatcher shall report “**Affirmative.**”
2. The dispatcher shall call the unit via the radio channel and advise about the incoming P1 or P2 call. If the crew does not get the information on the MDT, they should revert to the emergency procedure.
3. The unit once received the call on MDT should accept the CFS and confirm with the dispatcher immediately.
4. If the unit has an active MDT, the responder should receive the call via the radio and simultaneously accept it through the MDT. All other updates of the unit status will be done **ONLY** via MDT, **EXCEPT** when:
 - 4.1 Calling for additional help
 - 4.2 With Patient
 - 4.3 Reporting a safety issue
 - 4.4 MDT is offline or when there is MDT failure
 - 4.5 In emergency *(Requesting for specific urgent assistance, reporting the number of victims, MCI, and any status not available in the MDT)*.
5. At the end of the call and when becoming available, the responder will update through MDT with “**CLEARED**” status:
 - 5.1 If the unit is already available for service, then they have reported “**Available**” via the radio and the dispatcher has changed the unit from NJM as “**Assigned Available.**”
 - 5.2 If the unit is **NOT** available for service for any reason and the Dispatcher advised them to change to Standby channel and **the PTL will update their unit status to Unavailable status.**
6. Each dispatcher should monitor meticulously the update of UNIT STATUS done by responders and follow up the accuracy with AVL updates.
7. Each dispatcher should monitor the connectivity of MDTs to NJM network and report failure to the responding unit and to the PTL.
8. The PTL should keep an updated record of all units with Active MDT, updates any MDT change as required, and notify the DTL and Oscar on duty with the list of units with Active MDT.
9. MDT ticketing and change from vehicle to vehicle will be the responsibility of Production/Vehicle-Control Supervisors.
10. Production/Vehicle-Control Supervisors should immediately notify the PTL any time MDT is changed from vehicle to another.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.9 DAILY OPERATIONS IN THE COMMUNICATIONS CENTRES
Implementation Date	09 March 2020
Purpose	To ensure staff follow appropriate daily shift procedures.
Responsibility	ALL COMMUNICATIONS STAFF

1. All staff shall familiarize him or herself with, and follow all rules, regulations, policies, and directives of the HMCAS Communications Centres (which include HMC and MOI).
2. **Professionalism:**
 - 2.1 Staff shall conduct themselves in a professional manner as described in OP 4036 (HMC Code of Conduct) and AS 9035 (Dress Code and Personal Hygiene).
 - 2.1.1 Staff will be courteous, kind, patient, and respectful in dealing with the public, field staff and peers.
 - 2.1.2 Therefore, (for example) staff will refrain from the following behaviours, not limited to:
 - 2.1.2.1 Loud talking, boisterous laughter and rough play.
 - 2.1.2.2 Threatening, abusive or insulting/profane language.
 - 2.1.2.3 Improper, insubordinate, or argumentative actions or comments.
 - 2.1.2.4 Publicly criticize, disrespect, or ridicule the service or the corporation.
 - 2.1.2.5 Knowingly make any false statements or misrepresentation of facts.
 - 2.1.2.6 Use service or corporation equipment, supplies and vehicles for personal use.
 - 2.1.3 Staff shall not report for duty under the influence of any controlled substance, narcotic, or hallucinogens to the extent that it impairs their performance of duty in compliance with HR 3001(Alcohol and Drugs Abuse).
 - 2.1.4 There shall be no use of any tobacco products (smoking or chewing) in compliance with SA 1052 (Anti-Smoking).
 - 2.2 Communications Centre information must be kept confidential and not discussed with any other party unless expressed permission is received from the Operations Manager.
3. **Reporting for duty:**
 - 3.1 Staff should be well rested and prepared to perform his/her duties.
 - 3.1.1 Staff are required to attend work fit to safely perform their duties without risk to themselves or others.
 - 3.1.1.1 Staff should manage this personally by ensuring that an individual's time away from work includes sufficient sleep and recovery.
 - 3.1.1.1.1 This will include amongst other things, staff managing any health, lifestyle, personal or family responsibilities that may impact on their fatigue (loss of alertness).
 - 3.1.1.1.1.1 Staff experiencing fatigue should identify the factors that are contributing to fatigue, discuss any issues with their immediate supervisor, make changes as required, including sleeping

patterns, workloads, rosters, and lifestyle behaviors and seek professional help if necessary.

3.1.1.1.2 Supervisors should be alert for signs of drowsiness on the job and insist that an in-shift break be taken when these are observed.

3.1.1.1.3 Furthermore, sleeping on duty shall be cause of disciplinary action.

3.2 Staff are expected to report for duty as scheduled and to notify the immediate Supervisor in a timely manner whenever they are unable to do so as described in HR 3033 9 (Punctuality).

3.2.1 If the staff expects to be late, she/he should contact the duty Supervisor as soon as possible on the day when she/he will be late.

3.2.1.1 This notification is required no later than one hour before the scheduled start of his/her shift.

3.2.2 Staff shall not fail to arrive at work at, or depart from work prior to, the designated time without prior approval, which includes arriving late at the beginning of the duty, returning late from breaks, or an early departure from work.

3.2.3 Failure to report to work on time (being 'tardy') shall be a cause of disciplinary action.

3.3 Staff shall report timeously for duty with sufficient time to receive their assignment, receive a proper endorsement and be prepared to assume responsibility of their position at the scheduled start time.

3.3.1 The Operations Officer has the discretion to assign Team Leaders to certain roles (for example, Production, Distribution, Call-Taker, Dispatcher or Case Controller, etc.) based on the structure of the team, the expertise of the staff.

3.3.2 The Team Leader has the discretion to assign staff to certain roles (for example, Call-Taker, Dispatcher or Case Controller, etc.) based on the structure of the team, the expertise of the staff.

3.3.3 Reporting for duty activities include but are not limited to the following:

3.3.3.1 Be in uniform in accordance with AS 9035

3.3.3.2 Sign in using time/attendance system

3.3.3.3 Staff must sign in with their HMC account followed on the CAD system.

3.3.3.4 Check HMC email for communications memo/policy/procedures notifications and updates.

3.3.3.5 Assure equipment is in working order (telephone, radio, and computer, etc.) and do Microsoft Teams consoles checklist at the beginning of each shift.

3.3.3.6 Assure the console is clean and tidy

3.3.4 Staff shall report any needs/problems to their immediate supervisor.

4. Going off duty:

4.1 Staff shall be responsible for assuring that the on-coming shift receives a proper endorsement and is knowledgeable of issues that will carry over. Going off duty activities include but are not limited to:

4.1.1 Review and endorse on-going calls and pending cases.

- 4.1.2 Review and endorse unit availability and HUB/spoke coverage.
 - 4.1.3 Review and endorse event or specialized unit coverage.
 - 4.1.4 Clean and tidy the console
 - 4.1.5 Sign out using time/attendance system
- 4.2 The Operations Officer and/or Team Leader has the discretion to keep staff on duty beyond their scheduled end of shift based on operational need (as per the accountabilities in the job description).

5. Staff breaks:

- 5.1 Staff breaks are encouraged and should be taken/scheduled as approved by the Team Leader.
- 5.2 Staff working in the control rooms will take one break every 2-hours.
 - 5.2.1 The duration of the breaks is 30 min.
 - 5.2.2 10 min for Prayer (does not count as a regular 'break').
- 5.3 Breaks should not be taken within the first hour or the last hour of the staff's shift.
- 5.4 Breaks should not be combined.
- 5.5 Breaks are subject to recall/change/shortening depending on the workload at any time by the Team Leader.
- 5.6 Food and beverages should only be consumed on breaks, outside of the control rooms and in designated areas.
 - 5.6.1 Water is permitted in the control rooms, but they should be in a spill proof container and placed so as to avoid spills on the console/equipment.

6. Personal communications devices:

- 6.1 The use of any personal communication device (smart-phones, tablets, computers, etc.) during daily operations prohibited in the control rooms.
- 6.2 Personal phone calls can be made during breaks and the duration of the phone call should not exceed the duration of the break.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.10 TELEPHONE SYSTEM FAILURE
Implementation Date	21 September 2021
Purpose	To ensure staff follow appropriate procedures in the case of a telephone system failure.
Responsibility	ALL COMMUNICATIONS (HCS) STAFF

1. If the 999 system fails:

1.1 Immediately notify:

1.1.1 The MOI via established trouble reporting number: **44526-520** or **2345-520**.

1.1.2 If the above number was busy call 44526751 or 2345751. Write down the shutdown timing and take the technician's name.

1.1.3 Ooredoo hotline via established trouble reporting number: 44526-520 or 2345-520.

1.1.4 The on-call Communications Manager (Silver Communications).

2. In case of Telephone System Failure:

2.1 Calls will be routed to the basic PBX backup phone sets located on each console.

2.2 Calls may be transferred from the backup phones by using the 'flash' key.

2.3 There will be no speed dial capability and no ANI/ALI display.

2.4 A determination will be made by the on-call Communications Manager (Silver Communications) about whether there is still enough functional equipment at the NCC or if there is need to evacuate.

2.5 If fifteen (15) minutes has passed after the termination of 999 services and service has not been restored, the following notifications will be made along with any other notifications requested by the on-call Communications Manager (Silver Communications)

2.5.1 Notify Operations Managers, Senior Operations Managers, Directors, and the Executive on call for AS ES and HCS.

2.6 If the outage cannot be resolved quickly and the decision to move to the Backup Centre has been made, follow the Evacuation Plan to request re-routing of calls (to the PTS call center).

2.7 All reported equipment failures must be documented by the Supervisor or Senior Officer on duty via the "OVA" reporting system as per HMC policy.

3. In case of General GSM/Ooredoo shutdown:

3.1 Check if it is general for land line phone and Vodafone GSM.

3.2 Check if the "LIFENET" function or not

3.3 Check the MDT functionality

3.4 Check the AVL functionality

3.5 Check the HIA call center

3.6 Check the internet, Microsoft team, Trust case, WhatsApp...etc.

3.7 Do radio check and keep the unit's crew standby inside the Ambulance.

3.8 All reported equipment failures must be documented by the Supervisor or Senior Officer on duty via the "OVA" reporting system as per HMC policy.

4. If the HCC telephone system fails:

- 4.1 Immediately notify:
 - 4.1.1 HMC Communications Department at **44392210, 66262083** on call technicians.
 - 4.1.1.1 Failure to reach any on those numbers contact **40260334** (bleep station) and ask them to contact the technician.
 - 4.1.2 On duty Communications Centre Supervisor or Operation Officer.
 - 4.1.3 The on-call Communications Manager (Silver Communications).
 - 4.1.4 Request HCC staff at NCC log into HCC Cisco workstations and take calls.
- 4.2 If fifteen (15) minutes has passed after the termination of HCC telephone system and service has not been restored, the following notifications will be made along with any other notifications requested by the on-call Communications Manager (Silver Communications).
 - 4.2.1 Notify the SAS and MHS on-call managers (or any other users like HHCS and Diabetic).
 - 4.2.2 If the issue is Cisco related, the on-call Communications Manager (Silver Communications) will request re-routing of calls to the DID numbers (direct HCC extensions) hunt Group 28201, 28202, 28203, 28204, 28205, 28207.
 - 4.2.3 If the issue is not Cisco related, the on-call Communications Manager (Silver Communications) will request Communications to re-route calls to the emergency message:
 - 4.2.3.1 Thank you for calling the Hamad Medical Corporation Ambulance Service. At this time, we are experiencing technical difficulties and we are unable to attend to your call. We are working to rectify the problem. If this is a life-threatening emergency, please hang up and dial 999. For all other queries, please hang up and dial 40002055, 40442555.
- 4.3 If the outage cannot be resolved quickly and the decision to move to the Backup Centre has been made, follow the Evacuation Plan
- 4.4 All reported equipment failures must be documented by the on-duty Communications Centre Supervisor or Operation Officer via the "OVA" reporting system as per HMC policy and complete a summary report, check list for telephone shut down.

5. If the Clinical Desk telephone system fails:

- 5.1 Immediately contact HMC Communications Department on-call technician at **44392210, 66262083**.
 - 5.1.1 If unable to successfully contact any of these numbers, contact 40260334 (bleep station) and ask them to contact the technician.
 - 5.1.2 Request that all incoming telephone calls from the 98833 IVR are forwarded to the direct dial extension hunt group 28251, 28252, 28253, 28254, 28255.
- 5.2 In addition to 5.1 above immediately notify:
 - 5.2.1 Operations Supervisor NCC (Oscar NCC)
 - 5.2.2 Operations Supervisor for HCC (Oscar HCC)
 - 5.2.3 The on-call Communications Manager (Silver Communications)
 - 5.2.4 Critical Care Services Manager on-call (CCS Manager)

- 5.3 If unable to make outbound calls, ICCS or the NCC Cisco phone should be used to make outbound calls.
- 5.4 If fifteen (15) minutes has passed after the termination of Clinical Desk telephone system and service has not been restored, the following notifications will be made along with any other notifications requested by the CCS Manager on-call or Silver Communications:
 - 5.4.1 Senior Consultant CCP: Clinical Desk
 - 5.4.2 Consultant CCP: Effectiveness
 - 5.4.3 Oscar Retrieval
 - 5.4.4 Oscar LifeFlight
 - 5.4.5 Senior Operations Manager: Scheduled Ambulance Services
 - 5.4.6 Senior Operations Manager: Emergency Ambulance Services
- 5.5 The system failure should be documented in the shift reports and an OVA created by the CC: Retrieval or CC: Emergency as per HMC policy.

6. Testing:

- 6.1 If there have been no failures of the system, the failover system will be tested and documented bi-annually.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.11 RADIO SYSTEM FAILURE
Implementation Date	09 March 2020
Purpose	To ensure staff follow appropriate procedures in the case of a TETRA radio system failure in NCC and/or HCC.
Responsibility	ALL COMMUNICATIONS STAFF

1. Management of a Radio System Failure at National Command Center (NCC):

- 1.1 If a problem with any facet of the radio system arises,
 - 1.1.1 The shift supervisor or the most senior person (staff) will contact NCC Maintenance Team via internal NCC extension line **44526520** or **44526751** and report the problem.
 - 1.1.2 If unsuccessful, the shift supervisor will notify the NCC-MOI Operations Officer (via 45444 or 45333) and escalate the issue to the Communications Operations Manager.
 - 1.1.3 In the event of radio consoles failure, dispatchers will use the backup TETRA Mounted Radios (TMR) located beneath each radio console when available; otherwise the TETRA Handheld Radios (THR) will be used with the existing control channels.
- 1.2 **Complete Radio System Failure at NCC:**
 - 1.2.1 In case of a radio system failure which makes the TETRA radios (mounted and mobile) out of service at NCC the following steps shall be followed:
 - 1.2.1.1 Switch radio operations to a backup radio system if provided by Ministry of Interior (MOI).
 - 1.2.1.2 Contact the radio workshop hotline 2341225 immediately.
 - 1.2.1.3 Notify Operations Managers, Senior Operations Managers, Directors, and the Executive on call of AS ES, SAS and HCS.
 - 1.2.1.4 If radio communications still cannot be made, each dispatcher will contact each unit through individual crew's mobile phones, advise them to update their unit status, and await further instructions.
 - 1.2.1.5 Notify the Oscar on duty via mobile phone.
 - 1.2.1.6 The on-call Communications Manager will report to the Communications Center.
 - 1.2.1.7 The telephone system will become the primary dispatch system for HMCAS and where necessary, some units shall be supplied with a cellular phone (provided by the production supervisor of each hub).
 - 1.2.1.8 Record all relevant cellular phone numbers accurately in the resource management application and make available to all communications staff, supervisors, and managers.
 - 1.2.1.9 Upon restoration of the Radio System's functionality, the on-duty Oscar or the Duty Team Leader (DTL) is responsible for verifying the operational status of the system. This is achieved by directing each dispatcher across all channels to conduct a radio check with all respective units, ensuring comprehensive system functionality.

2. Management of Radio failure at Healthcare Coordination Center (HCC):

- 2.1 In case of radio system failure which renders the TETRA system out of service the following steps will take place:
 - 2.1.1 PTL should verify that it is not a user or individual radio issue.
 - 2.1.1.1 Contact Oscar-NCC to determine if NCC is experiencing any issues.
 - 2.1.2 Switch radio operations to an assigned backup radio system if provided by MOI.
 - 2.1.3 Contact the radio workshop immediately. **(Hotline: 2341225).**
 - 2.1.4 Notify Operations Managers, Senior Operations Managers, Directors and the Executive on call AS ES, SAS and HCS.
 - 2.1.5 If radio communications still cannot be made, each dispatcher will contact each unit through individual crew's mobile phones, advise them to update their unit status, and await further instructions.
 - 2.1.6 The on-call Communications Manager will report to the Communications Center.
 - 2.1.7 The telephone system will become the primary dispatch system for HMCAS and where necessary, some units shall be supplied with a cellular telephone (provided by the HMCAS if available).
 - 2.1.8 Record all relevant cellular telephone numbers accurately in the resource management application of the CAD and make it available to all communications staff, supervisors, and managers.

3. Documentation:

- 3.1 All reported equipment failures must be documented by the PTL or Operations Officer on duty via the "OVA" reporting system as per HMC policy and complete a summary report, check list for radio system shut down.

4. Testing:

- 4.1 If there have been no failures of the system, the failover (backup) system will be tested and documented bi-annually.

Appendix 1: Radio System (Life-X & Tetra) Failure Checklist

Date: DD/MM/YYYY

Time Started: 00:00

Time Ended: 00:00

CATEGORY: **Failure of Radio Dispatch Console** ☐

Gold Major and Control Major Signal ☐

Complete Radio System Failure ☐

NOTIFICATION		Done	Not Done
1	Contact NCC Maintenance Team (Call Ext. '520' up to 2 times)- Preplanned, they notify us.	☐	☐
2	Notify the NCC-MOI Operation Officer and escalate the issue to the Communications Operations Manager (If no answer from line 520 at second attempt)- Preplanned	☐	☐
3	Notify the Operations Manager, Senior Operation Managers, Directors, and CDO (When there is complete radio system failure only)	☐	☐
Backing up <i>In case of Radio Dispatch Console Failure</i>			
4	Dispatcher uses the backup Tetra Mounted Radios (TMR) – Not working	☐	☐
5	Dispatcher uses the Tetra Handheld Radios (THR)- Not working/No service	☐	☐
6	TMR available to operate in the Talk Group	☐	☐
7	Adequate THR available at PTL's desk (at least 8)	☐	☐
8	Multiport Charger with 6 ports and 8 batteries readily available at PTL's desk	☐	☐
In case of Complete Radio System Failure			
4	Switch to a back-up radio system (if provided by MOI) – No back provided	☐	☐
5	Contact Radio Workshop immediately for serious failure case- Preplanned	☐	☐
6	Dispatchers contact unit via crew's cellphones (when back-up radio system is not available). Two CFSs 0988 and 0989	☐	☐
7	Dispatchers notify Oscar Access via cellphone (when back-up radio system is not available). By Oscar command	☐	☐
8	On Call Communications Manage reports to NCC (when back-up radio system is not available).	☐	☐
9	PTL coordinates with Hub Production Supervisors the provision of cellular phones for some units. Informed to check staff names in SMS and notify them to monitor MDT.	☐	☐
Special Considerations			
*	Evacuate to the Backup Communications Center (when there is prolonged system failure and NCC is completely disabled)	☐	☐
Notes: 			

AFTER ACTION**Operations Officer/ Operations Manager**

		Yes	No
10	Where all steps completed on the checklist? If no, why?	☐	☐
11	Where there any obstacles /challenges with completing the checklist? If yes, why?	☐	☐
12	Could anything be done better the next time to mitigate the shutdown?	☐	☐
	Actions done:		

Names and Signatures:

Operations Officer on Duty

Operations Manager

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.12 NJM SHUTDOWN
Implementation Date	13 March 2018
Purpose	To detail the backup systems available in managing calls for service and resources in the event where NJM is not available.
Responsibility	ALL COMMUNICATIONS STAFF

1. GENERAL GUIDELINES:

- 1.1 Staff are responsible for maintaining familiarization with these resources in the event of an unplanned NJM crash or scheduled system down time.
- 1.2 Ambulance Service Medical Dispatchers must routinely review NJM backup procedures and resources.
- 1.3 Each employee is responsible for soliciting assistance from the case controller or team leader if additional directions are needed.
- 1.4 Each employee must ensure they are adequately prepared to follow backup procedures should they become necessary.

2. SHUTDOWN PROCEDURE:**2.1 In the event of a NJM failure or scheduled maintenance:**

- 2.1.1 The Operation Officer for Communications or the Distribution Team Leader should notify immediately the NJM helpdesk line **44526520** or **44526751**.
- 2.1.2 The Operations Officer for Communications or the Distribution Team Leader must immediately notify the On-Call Communications Operations Manager (Silver Communications) and the On-Call Managers/Executive as per the notification procedures.
- 2.1.3 The manual communications record ([Appendix 1](#)) must be utilized to process calls for service and make dispatch assignments. Manual communications record sheets are located at each call-taking/dispatching console and on the Production/Distribution Team Leader desks.
- 2.1.4 A case controller will be assigned to coordinate and endorse the manual sheets between the call-takers and Dispatchers.
- 2.1.5 Manual entry will require the same information as NJM screen to be handwritten in the appropriate space on the manual communications record.
- 2.1.6 Before forwarding the sheet to be dispatcher, Call-taker will enter the necessary information (address, received time, priority of the call, call-taker name, and console number). The dispatch code will be forwarded when available.
- 2.1.7 Dispatcher is required to enter accurately all remaining information including time stamps.
- 2.1.8 The CFS opened during the shutdown will be numbered as follows:
 - 2.1.8.1 1st CFS will take the number YY-MM-DD-9001 (Example: The 1st case opened during the shutdown will be: 170521**9001**).

- 2.1.8.2 2nd CFS will take the number YY-MM-DD-9002 etc... Then the numbers will keep adding for each new CFS (opened during the shutdown) until NJM is back to operation.
- 2.1.8.3 These numbers during the shutdown will be coordinated by the on-duty DTL and will be written on the CFS-manual-sheet before providing it to the dispatcher.
- 2.1.8.4 The dispatcher will then verbalize the Shutdown's CFS number to the Field Crew(s) via radio.
- 2.1.9 The running CFSs during the shutdown will maintain their original NJM CFS number and NO Shutdown's CFS Number will be created for them.
- 2.1.10 The last 4 digits of the Shutdown's CFS Number shall not be used twice during the same day.
- 2.1.11 If a shutdown happens twice during the same day the DTL will continue numbering the CFS starting from the last Shutdown's CFS Number of the previous shutdown.
- 2.1.12 (Example: Two shutdowns happened in May 19th, 2017):
- 2.1.13 1st shutdown happened at 08:00 with 5 cases opened, the numbers will be as follows: 1705199001, 1705199002, 1705199003, 1705199004 and 1705199005.
- 2.1.14 2nd shutdown happened at 14:30 with 2 cases opened the numbers will be as follows: 1705199006, 1705199007.
- 2.1.15 If a shutdown happens the NEXT day the DTL will start numbering the CFS starting from 9001.
- 2.1.16 (Example: 1 shutdown happened May 10th and another happened 11th):
- 2.1.17 1st shutdown happened May 12th at 22:00 with 3 cases opened, the numbers will be as follows: 1705109001, 1705109002 and 1705109003.
- 2.1.18 2nd shutdown happened May 11th at 10:30 with 4 cases opened the numbers will be as follows: 1705119001, 1705119002, 1705119003, 1705119004.
- 2.2 **Once NJM has been restored:**
 - 2.2.1 Calls received during the shutdown will be logged into NJM immediately when NJM application is back in service.
 - 2.2.2 A call-taker shall be assigned or a DTL to ensure that all manually opened CFSs are created inside NJM and a scanned copy of the CFS-Manual-sheet shall be attached to the case in NJM respectively.
 - 2.2.3 Each CFS-Manual-sheet will contain the Shutdown's CFS Number and the "REAL" CFS number created when NJM is back to operation.
 - 2.2.4 A Summary Checklist ([Appendix 2](#)) with a clear crosscheck and link between "REAL" CFS number & "SHUTDOWN'S" CFS number will be sent by the DTL to the below emails:
 - 2.2.4.1 nccquality@hamad.qa
 - 2.2.4.2 AS-NCCTeamLeadersMailGroup@hamad.qa
 - 2.2.4.3 AmbulanceData@hamad.qa
 - 2.2.4.4 ambulanceCDO@hamad.qa

- 2.3 The original copy of the summary checklist and all Manual CFSs shall be sent to the Operations Manager for Communications then to the Quality Improvement Unit for filling.
- 2.4 If NJM is back after Midnight, the same above steps will apply.
- 2.5 PTL will record the time of the shutdown and the return back to service in the PTL Daily Report.

3. Documentation:

- 3.1 Following HMC policy, it is obligatory for the Production Team Leader (PTL) or the Operations Officer present during the shift to document all incidents of equipment failure. This documentation process should be carried out through the “OVA” reporting system. Additionally, they are required to finalize a summary report and a complete checklist, specifically focusing on the procedures for the NJM shutdown. This approach ensures that all equipment malfunctions are systematically recorded and managed, promoting both operational efficiency and accountability within the team.

Appendix 1: Manual Communications Record



Hamad Medical Corporation
AS Department
 Computer Aided Dispatch
CFS SHUTDOWN COMMUNICATION RECORD



DATE: _____ Time: _____

CONSOLE#: _____

Call taker Name: _____

Dispatcher Name: _____

LOCATION OF CALL				DESTINATION					
SITE:		QARS		SITE:					
		BLD #:							
		St. #:	ZONE #:						
ADDRESS:									
CALLER NAME:				PHONE NO:					
RESPONSE CODE		DISPATCHED CODE:							
APPARENT INJURY				TRAUMA CAUSE		PATIENT CONDITION			
1. ABD PAIN	7. SHORT OF BREATH	13. INT BLEEDING	19. BACK PAIN	1. ANIMAL	7. FIRE/FLAME	CONSCIOUS	1.Y	2.N	3.UNK
2. ALLERGY	8. DROWNING	14. NAUSEA/VOMIT	20. BURNS	2. ASSAULT	8. GUN SHOT	BREATHING	1.Y	2.N	3.UNK
3. BEHAVIOURAL	9. FANTING	15. OB/GYN	21. FRACTURE/DIS	3. CHEMICAL	9. RTA	BLEEDING:	1.Y	2.N	3.UNK
4. CHEST PAIN	10. FEVER	16. POISON/OD	22. SMOKE INH.	4. COLLAPSE	10. SPORTS	AGE:			
5. CHOKING	11. HEADACHE	17. PERS DOWN	23. TRAUMA	5. ELECTRICAL	11. STABBING	SEX:			
6. CONVULSION	12. HEAT EXP	18. WEAKNESS	24. WOUNDS	6. FALL	12. WATER ACC	NAME:			
OTHER:			25. N.E. TRANS	OTHER:		NATIONALITY:			
ROAD TRAFFIC ACCIDENT		ROLL OVER:	FIRE/EXPLOSION:	NO OF VEHICLES:		NO OF PATIENTS:		OTHER:	
PRIORITY:		RESPONSE TIMES							
TIME:	1 ST	2 ND	3 RD	4 TH	5 TH	6 TH	7 TH	8 TH	RETURN
UNIT #									
1. RECEIVED									
2. DISPATCHED									
3. TO SCENE									
4. ON SCENE									
5. TRANSPORT									
6. AT HOSPITAL									
7. AVAILABLE									
NO OF PATIENTS									
CASE OUTCOME									
TRANSPORT	TRANS/PT	REFUSED	NO INJURY		FALSE CALL		TREAT/NO TRANS		
TRANS/POLICE		NO PT FOUND:		CANCELLED		DOA		OTHER:	
COMMENTS:									

Appendix 2: Summary Checklist

HAMAD MEDICAL CORPORATION EMERGENCY MEDICAL SERVICES DEPARTMENT NJM SHUTDOWN DTL CHECKLIST

DATE:

Team:

SHUTDOWN DETAILS				CALLS DETAILS		
SHUTDOWN START TIME:				TOTAL NUMBER OF CALLS RUNNING WHEN THE SHUTDOWN HAPPENED:		
SHUTDOWN END TIME:				TOTAL NUMBER OF CALLS OPENED DURING THE SHUTDOWN:		
RUNNING CASES WHEN THE SHUTDOWN HAPPENED				CASES OPENED DURING THE SHUTDOWN		
NJM-CFS-NO Before				SHUTDOWN-CFS-NO	NJM-CFS-NO After Shutdown	
1				1		
2				2		
3				3		
4				4		
5				5		
6				6		
7				7		
8				8		
9				9		
10				10		
11				11		
12				12		
13				13		
14				14		
15				15		
16				16		
17				17		
18				18		

CHECKLIST ROUTING

OSCAR NAME:

SIGNATURE

DATE :

TIME:

NJM TIMING ENTER TO THE SYSTEM BY (SUPERVISOR) NAME:

CORP#:

SIGNATURE

DATE:

TIME:

A copy should be emailed by the DTL to: E-mail: ambulanceCDO@hamad.ga, nccquality@hamad.ga,
ambulancedata@hamad.ga, AS-NCCTeamLeadersMailGroup@hamad.ga

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.13 ACCESS TO THE HEALTHCARE COORDINATION CENTER (HCC)
Implementation Date	01 October 2016
Purpose	To maintain an access-controlled and secure communications centre environment to ensure uninterrupted service delivery.
Responsibility	ALL COMMUNICATIONS CENTER STAFF

1. The HCC control rooms (Call taking and dispatch rooms) is considered as designated **restricted areas**.
2. Do not allow visitors to use the phones, radios, or other HCC equipment.
3. Requests for information, quality reviews, reports and miscellaneous business-related discussions with HCC employees should be conducted either by email, memorandum, inter-office telephone, or incident report.
4. **Access Control:**
 - 4.1 Access shall be limited to staff working inside the HCC and parties with appropriate clearance (i.e. approved Executives, managers, operational officers and IT Project Managers), and shall enter the control rooms through authorized entry points only.
 - 4.2 The primary areas shall be accessed by digital card only.
 - 4.3 Never loan your physical keys, badges, key cards, pin codes, or other physical access control devices to other employees or visitors.
 - 4.4 Keep all doors (including doors of the communications/data equipment rooms) closed and secured to prevent any unauthorized access.
 - 4.5 Do not use emergency exit doors for access; never block or prop open emergency exits.
 - 4.6 Never enter the control room using authorized access to conduct personal business.
 - 4.7 Field/Operations personnel shall not be permitted entry into the control room.
 - 4.8 Patients and family members are prohibited from entering the control rooms at any time.
 - 4.9 When Patients and family members want to endorse the medical certificate should not enter the call center.
 - 4.10 HCC personnel shall not hang around in the control room before or after their shift and shall not enter the control room during off-duty periods.
5. **Visits to the HCC:**
 - 5.1 Visitation is a privilege allowed at the discretion of the management staff and may be revoked at any time.
 - 5.2 Any visitor requiring admission to HCC shall contact either the Operations Officer (Oscar HCC), Operations Manager/AED of HCC for permission.
 - 5.3 In-room tours must be pre-scheduled, and a Senior Supervisor shall be assigned to conduct any approved tours.
 - 5.4 All visitors, vendors and non-direct employees shall be met and escorted (by HCC personnel) throughout the duration of the visit and shall sign in and out in the log book before entering and upon leaving the premises.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.14 CALL-FOR-SERVICE COORDINATION BETWEEN NATIONAL COMMAND CENTER AND HEALTHCARE COORDINATION CENTER
Implementation Date	01 June 2016
Purpose	To maintain an access-controlled and secure communications center environment to ensure uninterrupted service delivery.
Responsibility	ALL HMCAS HEALTHCARE COORDINATION SERVICES PERSONNEL

1. Only “legitimate” emergency shall be handled on the NCC 999 and hotlines systems.
2. Do not tie up the medical emergency telephone network with non-emergency calls since it has a limited number of dedicated trunks/lines/consoles. This could result in a “real” emergency call being delayed, blocked, or routed back to 999 operators.
3. **Non-emergency and Routine Inter-facility calls received at NCC:**
 - 3.1 **Patient’s Transport (non-emergency) calls received on emergency lines (999/hotline) at NCC.**
 - 3.1.1 All calls received through the hotline/999 will be treated as emergency calls and processed accordingly, unless:
 - 3.1.1.1 The caller indicates that the patient requires non-emergency transportation to a residence or a facility other than a hospital Emergency Department and/or,
 - 3.1.1.2 The patient complaint is non-emergency and fits under the Patient Transport Services (PTS) criteria.
 - 3.1.2 If a PTS call is received on the hotline/999 the call taker shall advise the caller that they have called on an emergency line and shall direct the caller to dial the non-emergency line “16099” or “4439 8555”. **Do not transfer the call to the HCC lines**, since this may tie up the 999 and hotlines trunks. The following cases are exceptions and shall be treated as follows:
 - 3.1.2.1 **Doha International Airport (DIA) cases received at NCC:**
 - 3.1.2.1.1 Patients to be transported from DIA arrivals shall be transported in most of the cases to the emergency department and shall be considered as emergency cases, thus If patient transport request from DIA is received at NCC, and the following steps shall be taken:
 - 3.1.2.1.1.1 The call taker shall immediately process it in NJM using the most appropriate ProQA chief complaint.
 - 3.1.2.1.1.2 The dispatcher shall ensure the closest alpha unit is dispatched first along with the closest Delta unit (Delta8 or Delta7). A Charlie unit may be added if required according to caller information and dispatch matrix.
 - 3.1.2.1.1.3 Ideally dispatch units to DIA from Hub1 (standby at the Museum spoke, unless another alpha unit is closer).

- 3.1.2.1.1.4 The call taker and the dispatcher shall immediately notify the DTL about the airport case.
- 3.1.2.1.1.5 If call taker discovers the call to be case of a non-emergency transportation, he/she shall continue processing the call as an emergency and immediately notify the DTL.
- 3.1.2.1.1.6 The DTL shall then call back the caller to verify the nature of the call and coordinate with the HCC to send appropriate resources prior to cancellation of the call at NCC.
- 3.1.2.1.1.7 In case NCC receives a case from Hamad International Airport (HIA) and the aircraft has landed/going to land in a short time.
- 3.1.2.1.1.8 The call shall be processed as above and NCC DTL should coordinate further with the HCC UTL to verify if there is an existing booking or an ambulance dispatched.
- 3.1.2.1.1.9 If there no booking or pre-notification given to NCC, NCC shall continue with the call or the closet or most appropriate ambulance deployed from NCC or HCC.

3.2 **HCC Interfacility calls received on emergency lines at NCC** consist of all patients who have been seen by a nurse or doctor and need further evaluation (higher level of care) or transfer (to a more appropriate facility) from Hamad General Hospital, Heart Hospital, Women's Hospital, Women's Wellness and Research Center (WWRC), Al Khor Hospital, Al Wakra Hospital, Cuban Hospital, Pediatric Emergency Centers (PEC) Al Saad and Al Rayyan, Sidra Medicine, Hazm Mebareek General Hospital and Primary Health Care Corporation (PHCC) facilities.

3.2.1 **Interfacility Protocol 45/46/47 Cases (Appendix A)**: All calls coming from hospitals listed in Appendix A, Sidra or Primary Health Care Corporation (PHCC) facilities should be processed at HCC and ProQA shall be completed under most appropriate protocol. On the occasion that these cases come to NCC:

- 3.2.1.1 Verify the exact location of the patient to ensure the transfer request is being initiated from WITHIN the facility (in a bed).
- 3.2.1.2 If the patient is OUTSIDE of the facility, (means patient not under care of the facility or not registered/admitted) NCC will proceed with the call using Protocols 1-32 (in case HCC received such cases the call taker should direct the caller to call 999 (except the PHCC cases).
- 3.2.1.3 **If the patient is INSIDE of the facility, NCC will proceed with the call using Protocol 37 "Transfer/Interfacility/Palliative Care" and Protocol 33 exclusively for PHCC calls.**
 - 3.2.1.3.1 The call taker should immediately notify the NCC DTL who will contact the HCC UTL to confirm if there is any technical issue with the Cisco Phone system or busyness/availability of the call takers as well as to confirm the availability of the SAS resources, However NCC shall continue with the call (using NJM) if no HCC unit available/more closer or appropriate and if

there is available unit the HCC shall advise the unit to switch the appropriate NCC channel.

3.2.1.3.1.1 If there is no technical issue, an OVA can be raised against the caller.

3.2.1.4 The Oscar NCC will be consulted and decide if the responding NCC units should continue, or if the case should be reassigned to HCC.

3.3 [Interfacility Protocol 33 Cases \(Appendix A\)](#): All calls coming from medical facilities otherwise identified as Protocol 33 facilities shall be processed at NCC and ProQA shall be completed under chief complaint 33 "Transfer/Inter-facility/Palliative Care."

4. **NCC requiring special unit(s) from HCC (e.g. Bariatric, wheelchair, ICU):**

4.1 To manage emergency calls, the NCC team may need to utilize units from the Scheduled Ambulance Service (SAS). The use of those units shall be shortened to the duration of the relevant emergency call and shall return immediately to the pool of the SAS units once cleared available. Take the following steps into consideration any time a specific SAS unit is need at NCC:

4.1.1 The NCC DTL shall inform the HCC UTL that a special unit is required.

4.1.1.1 The NCC DTL may utilize the HCC CAD Tactical Board to view the location and status of available units to assist in choosing the closest and most appropriate unit based on the unit(s) type required, not the unit(s) name.

4.1.2 The HCC UTL shall check the availability of the requested unit(s) and decide which unit to give NCC.

4.1.3 If the unit(s) is not immediately available, the HCC UTL will update the NCC DTL once the special unit is available. The NCC DTL shall follow up with the HCC and/or seek an alternative.

4.1.4 If the unit(s) is available, the HCC UTL shall instruct the HCC Dispatcher to inform the unit to change to the required NCC operational channel and update the NCC DTL accordingly.

4.1.4.1 HCC Dispatcher will place the Call Sign of the selected SAS resource in VLT CAD 'UNAVAILABLE'.

4.1.5 The NCC DTL shall inform the PTL to publish the special unit in NJM so it can be assigned by the dispatcher in the relevant Call for Service (CFS).

4.1.6 NCC Dispatcher monitors the status of the Call Sign throughout the incident response through to CLEAR,

4.1.6.1 The unit shall be instructed to go back the HCC operational channel immediately after finishing the NCC case.

4.1.6.2 NCC Dispatcher will advise the NCC DTL when the Call Sign is CLEAR,

4.1.6.3 NCC DTL notifies HCC UTL that the Call Sign is CLEAR.

5. **HCC requiring special unit(s) from NCC (e.g. Alpha, Charlie, Delta, LifeFlight):**

5.1 The use of Emergency Service (ES) units shall be shortened to the duration of the relevant IFT call and shall return immediately to the pool of the ES units once cleared available. The below steps shall be taken into consideration any time a specific AS unit is needed by HCC:

5.1.1 HCC UTL shall inform the NCC DTL that EAS resource is required to respond to SAS case.

- 5.1.2 Charlie requests are exempted which should be coordinated directly with NCC DTL through Retrieval Desk-EMD as needed, However the RD-EMD must update the UTL and timely escalate if there is challenges to obtain CCP from the NCC DTL.
 - 5.1.2.1 Details include the priority, incident location of the case and designated radio channel.
- 5.1.3 NCC DTL coordinates resource assignment with NCC Dispatcher and NCC DTL advises HCC UTL the Call Sign they will receive.
 - 5.1.3.1 NCC Dispatcher will inform the unit to change to the required HCC radio channel and advise the case response priority (P1 or P2) and shall place the Call Sign of the selected EAS resource in NJM CAD as 'UNAVAILABLE w/HCC.'
- 5.1.4 HCC UTL notifies the HCC Dispatcher of the Call Sign they will receive.
- 5.1.5 HCC Dispatcher dispatches the Call Sign in VTL CAD and calls the Call Sign on the radio.
- 5.1.6 Call Sign comes up on the designated radio channel acknowledging the priority, incident location, and responding from location.
- 5.1.7 HCC Dispatcher advances the status of the Call Sign in VTL CAD and provides any additional details, confirming any details that may have been missed,
 - 5.1.7.1 If HCC Dispatch unable to reach the Call Sign within 2 minutes, HCC UTL will recontact the NCC DTL.
- 5.1.8 HCC Dispatcher monitors the status of the Call Sign throughout the incident response through to CLEAR,
 - 5.1.8.1 The unit shall be instructed to go back the NCC operational channel immediately after finishing the HCC case.
 - 5.1.8.2 HCC Dispatcher will advise the HCC UTL when the Call Sign is CLEAR,
 - 5.1.8.3 HCC UTL notifies NCC DTL that the Call Sign is CLEAR.

6. Medical/Trauma Emergencies calls received at HCC:

6.1 Medical/Trauma emergency calls:

- 6.1.1 If a call of an emergency nature is received at HCC non-emergency lines, the HCC call taker shall advise the caller to call back on 999. **Do not transfer the call to the NCC hotline.**

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.15 CAPTURING UNIT STATUS IN MDT
Implementation Date	01 June 2016
Purpose	- To establish procedures for the use of the Mobile Data Terminals (MDT's) and to define dispatcher and field unit responsibilities. - To define the differentiation between the unit status definition with examples.
Responsibility	ALL OPERATIONS, COMMUNICATION STAFF & CLINICAL FIELD RESPONDERS

1. In line with the technical enhancement in HMCAS most of the response units are equipped with Mobile Data Terminals (MDT) and Automated Vehicle Locator (AVL).
2. **GENERAL GUIDELINES:**
 - 2.1 Provides direct access to the Ministry of Interior Computer-Aided Dispatch system called "Najem" (NJM) which is used by HMC Ambulance Service. Starting from March 9th, 2015, all equipped units shall use MDTs and AVL as the primary tool for receiving and acknowledging routine dispatch assignments and updating unit status, with due regard to crew safety.
 - 2.2 MDTs shall:
 - 2.2.1 not be installed, removed, moved, or modified in any manner by anyone other than authorized Department or technical service personnel.
 - 2.2.2 not be activated or used by anyone other than those personnel properly trained in their use. At no time shall an employee use another log-on I.D. number or password other than their own.
 - 2.2.3 used to transmit official messages only
 - 2.2.4 not be used to transmit any message of a defamatory, derogatory, inflammatory, racist, or sexual nature.
 - 2.3 All MDT transmissions shall be recorded (in some cases, may become public information) and are subject to audit and periodic review for appropriateness and conformance, by the Auditing and Review section. There will be no expectations of confidentiality or privacy.
 - 2.4 All transmissions shall be subject to constant monitoring by Supervisory and Operations personnel. Personnel found to be making personal or inappropriate transmissions using MDTs or radios are subject to disciplinary action. Examples of prohibited transmissions, but not limited to, include:
 - 2.4.1 Unnecessary radio conversations or MDT transmissions;
 - 2.4.2 Argumentative or sarcastic transmissions on the radio or MDT;
 - 2.4.3 Use of profane and obscene language;
 - 2.4.4 Use of personal names during transmission, except in an emergency;
 - 2.5 No member of HMCAS shall access, disclose, nor knowingly permit the access to or disclosure of information within the possession of HMCAS to any person or entity except as necessary in the performance of his/her official duties and in compliance with departmental policy.
 - 2.6 MDT's assigned to employees may be used in training projects/courses and or drills, when the training or drill is related to course work approved by HMCAS. Relevant staff shall receive Communication manager approval prior to using MDT's for training/drills.

- 2.7 Employees shall not use MDT's, whether on or off duty, for the tracking of any outside employment, personal business, or profit-making ventures. All records and information maintained in any MDT assigned to any employee are confidential, the property of HMCAS, and must be made available for inspection to any department supervisor or manager.
- 2.8 Patient history information, personnel information or history, intelligence information, or other confidential information may not be maintained in MDT's unless authorized by an operations manager or higher authority, as part of an ongoing investigation.
- 2.9 Any field unit in the "AVAILABLE on the move" or "Available on STANDBY" status is presumed to be inside the unit and monitoring the MDT for assignments and messages, when away from the unit and not monitoring the MDT, field personnel shall inform the Dispatcher via radio. On the other hand, any status change or acknowledgment that is broadcast by radio voice by a field unit shall be entered in NJM by the dispatcher to avoid errors and duplication of effort.
- 2.10 Field personnel signed onto MDT's shall be responsible for monitoring the terminal and maintaining correct unit status on the CAD system. Any field unit in the "AVAILABLE" or "STANDBY" status is presumed to be inside the unit and monitoring the MDT for assignments and messages.
- 2.11 MDT's shall be used as the default dispatch tool and whenever practical to reduce radio traffic. Obvious crew safety or urgent information will continue to be broadcast via voice. The field unit has the ultimate decision over when they will transition to voice as situations in the field change.

3. PROCEDURE:

- 3.1 Field crew assigned to response units equipped with Mobile Data Terminals shall consider the following:
- 3.2 **MDT connectivity continuously**, dispatchers shall monitor MDTs connectivity via the CAD system (NJM) by verifying continuously the MDT signals (RED/ORANGE/GREEN) in the NJM dispatch interface. In case of disconnection, the dispatcher shall follow up with the unit via the radio to re-establish connection of MDTs with the control room. The units may try the following:
 - 3.2.1 Change location for better signal
 - 3.2.2 "log off" the MDT then "log on" again
- 3.3 **Log-on:** Field Crew shall log on the MDT at the beginning of the shift and shall test the connectivity with the control room.
- 3.4 **Initial Dispatch:**
 - 3.4.1 Receive calls for service via MDT;
 - 3.4.2 Dispatch shall utilize a "minimum voice" technique. Voice dispatch only the heading and location of the incident, (i.e. "Alpha One One, Zulu case in Madinat Khalifa south").
 - 3.4.3 Acknowledge receipt of calls for service by transmitting appropriate MDT "unit status" and by voice through the TETRA Radio.
 - 3.4.4 MDT's may not receive updated information automatically; field crew shall press the "REFRESH" key to receive updates.
 - 3.4.5 Dispatch will voice broadcast any critical supplements and changes to the call while the units are responding.
- 3.5 **Unit Status:** (refer to Figure 3.15-1 "NJM - Unit Status Definition Document"):
 - 3.5.1 "Unit status" shall be captured in NJM to determine the current operational status/availability of response Units as well as recording when they are responding or

proceeding to incidents. The MDT allows Field crews to directly change their own status within NJM. The filed crew shall use the relevant “unit status” as per their movement and position.

- 3.5.2 Field crew shall change their status and provide updates via MDT. The response unit may change to radio voice as situations in the field change, to such an extent that it is not physically possible for the crew to update the MDT. In these situations, any unit status broadcasted through the radio should be entered in NJM by the dispatcher to avoid errors and duplication of efforts.
 - 3.5.3 At the end of the case, responders shall assign their unit “CLEARED” from the case via the MDT. The dispatcher shall then assign them “ASSIGNED AVAILABLE”; the unit shall acknowledge “ASSIGNED AVAILABLE” via the radio.
 - 3.5.4 If the unit is cleared from the call but not available for service for any reason (mechanical/other), the crew shall inform the Dispatcher who shall advise them to change to the production radio channel and coordinate with the Production Team leader (PTL). If required, the PTL may assign them “Unavailable status”. The PTL shall monitor the time and validity of all units assigned “Unavailable status”.
 - 3.5.5 To avoid confusion regarding who/when/how to update the responding unit’s status, the following guidelines have been created. Kindly take note of the important rules, definitions, and scenarios below:
- 3.6 **The Main Rules:**
- 3.6.1 It is the responsibility of the responding crew to update their unit’s status timely via MDT (or via TETRA radio when MDT is not working).
 - 3.6.2 If the MDT freezes/hangs up while enroute to a call, the crew shall immediately notify the dispatcher via TETRA radio. The dispatcher shall thereafter take over the MDT status responsibility from the crew until the unit becomes active again.
 - 3.6.3 The dispatcher shall document the reason in the comments box as to why he/she performed the update (examples: “MDT is hanging. Responding crew cannot update.” “The unit failed to update the status even after being advised to do so. Unit is already on scene.”)
 - 3.6.4 The responding crew shall send their unit’s status update via TETRA radio when:
 - 3.6.4.1 a call is received. The crew shall update their status as they begin to move toward the scene.
 - 3.6.4.2 with the patient (the dispatcher will update this status).
 - 3.6.4.3 “clear”/“available”. The crew will update their status as “clear” and the dispatcher will update the crew status as “available.”
 - 3.6.5 If two unit’s status are transmitted simultaneously, they shall be both entered in the order described in Figure13.5B:
 - 3.6.5.1 Example1: If the crew arrives at the scene and the patient at the same time; they shall enter “on scene” then “with patient” consequently with no time delay.
 - 3.6.5.2 Example2: if the crew are already on the move to a standby location and they receive a call via MDT/RADIO they shall enter “Received” and “to scene” consequently with no time delay.

3.6.6 **“Available” status:**

- 3.6.6.1 **Available - In position:** Unit is available to be dispatched from its assigned standby location/position. It is staffed correctly, has passed the Key Vehicle Safety Check and has complete Equipment and Communication Technology Minimum Equipment.
- 3.6.6.2 **Available - En route to position:** Unit is available to be dispatched but is repositioning/enroute to its assigned location/position. It is staffed correctly, has passed the Key Vehicle Safety Check and has complete Equipment and Communication Technology Minimum Equipment.
- 3.6.6.3 **Available Admin:** Unit is available to be dispatched but the crew is engaged in management approved admin activities. It is staffed correctly, has passed the Key Vehicle Safety Check and has complete Equipment and Communication Technology Minimum Equipment.

3.6.7 **“Unavailable status:**

- 3.6.7.1 **Unavailable Mechanical issue:** Unit is unavailable due to a mechanical/vehicle issue/RTA but it is staffed correctly and has complete Equipment and Communication Technology Minimum Equipment.
- 3.6.7.2 **Unavailable Equipment issue:** Unit is unavailable due to an Equipment or Communication Technology issue but is staffed correctly and has passed the Key Vehicle Safety Check.
- 3.6.7.3 **Unavailable Crew issue:** Unit is unavailable due to a crew issue/incomplete crew but it has passed the Key Vehicle Safety Check and has complete Equipment and Communication Technology Minimum Equipment.
- 3.6.7.4 **Unavailable Refueling:** Unit is unavailable as it requires to or is refueling. It is staffed correctly, has passed the Key Vehicle Safety Check and has complete Equipment and Communication Technology Minimum Equipment.
- 3.6.7.5 **Unavailable with NCC/HCC:** Unit is not available to be dispatched as it is currently reassigned to the NCC/ HCC.

3.6.8 **LF status:**

- 3.6.8.1 **Any LF related status (Standby- Activate- Startup- On station- Landed Hospital) must be updated by IF coordinated accordingly and timely.**

3.6.9 **“With the Patient” versus “On Scene”:**

- 3.6.9.1 **“With the Patient” Status:** This refers to a situation where the crew is on the location of the incident and is standing next to the patient. The crew shall immediately update their status to the NCC via TETRA radio, by transmitting “with the patient.”

3.6.9.1.1 **On Scene” Status Scenarios.**

Cases	Yes	No	Comments
1. You have the exact/actual address of the incident, for instance "QARS"(Qatar Area Referencing System) or street number with gate number. Is the unit 10/10?	p		----
2. You have the meeting/rendezvous point (a roundabout or a landmark).	p		----
3. Is the unit 10/10?			

4. The exact location of the incident is unknown due to language barrier or the caller is new in Qatar and only general area is given. For example, the caller location is in Al Rayan Qadeem (Old Rayan). The unit enters this area. Is the unit 10/10?		p	<i>The 10/10 is the CFS location sign. Otherwise, the 10/10 is the general or main area.</i>
--	--	----------	--

3.6.9.1.2 Confusing Scenarios.

Cases	Yes	No	Comments
1. In a case of RTA (Road Traffic Accident) on a highway and the unit happens to be on the other side of the road where the accident is and needs to take the next bridge or R/A to reach the scene, is the unit 10/10?		p	<i>The unit shall inform NCC that they can see the RTA, but they will take U-turn on the bridge or roundabout before they reach the scene. Dispatcher shall document this in the comment box.</i>
2. The initial location given is reached by the unit and then the caller suddenly changes the location and gave a new one. Is the unit 10/10?	p		<i>The unit is 10/10 in the first location. The dispatcher shall document the changes in location and the secondary 10/10 in the comments box.</i>
3. The unit is following the caller but the CFS location is still very far. Is the unit 10/10?	p		----
4. A unit is on the same street as in CFS location and the unit can see the caller but cannot reach the location due to road obstructions (examples: flood, heavy traffic, RTA with no victim, road closure, etc.). Is the unit 10/10?		p	<i>The unit shall inform the dispatcher that they can actually see the caller, but they cannot proceed to the location due to some road barriers. Dispatcher will document this in the comment(s) box.</i>
5. The unit reported “following the Civil Defence in a case of fire” or “following the police in a case of assault or RTA”. Is the unit 10/10?	p		<i>The answer is yes if the police or Civil Defence guides the crew to the location which is known to them.</i>
6. A call is inside a building (residential/commercial, high-security facility like HIA, Emiri Guard, Doha Port, QP Industrial) but the responding unit is already outside its gate. Is the unit 10/10?	p		----

3.7 **Cancellations and Exchanges:**

3.7.1 Cancellation of units by dispatch: Any cancellation of unit response to a call by dispatch shall be broadcast by voice. Field units shall acknowledge by voice. Dispatch shall be responsible for immediate CAD updates.

3.7.2 Re-directing a unit from a currently assigned call: Dispatch shall voice broadcast the cancellation of the first call before assigning them to the new call. Exchanges will be done by voice and dispatch shall be responsible for the immediate NJM and CAD updates.

3.8 **Changing Unit Location/Transport/Status:**

3.8.1 Field units shall use the MDT to change locations whenever practical. If the unit chooses to voice the update, the dispatcher shall change the unit status in CAD. Terminal to terminal messages shall not be used to change location or status.

3.9 **Dispatch Script/ProQA data (information about the scene and the patient):**

- 3.9.1 Dispatch Script information is automatically sent to the MDT. It will be the field unit's responsibility to read the premise history (ProQA scripted questions/answer).
- 3.9.2 The dispatcher shall give the dispatch script by voice to units not equipped with MDT or any time an MDT is not operational. This is applicable for initial dispatch and for backup units.
- 3.9.3 The dispatch script shall contain information about the incident and the patient as per ProQA scripted questions and as per the answers given by the caller. The dispatch script may contain information such as patient's chief complaint, Mechanism of injury, number of patients, patient's age, and other information (exact location of the patient flat/floor).
- 3.9.4 The dispatch script shall be displayed in the MDT in English. If the MDT displays in Arabic language, the field crew shall immediately notify the control room.
- 3.10 **Emergency Messages:**
 - 3.10.1 If the HMCAS Control room receives an MDT request for emergency assistance, the procedure will be as follows:
 - 3.10.1.1 If the unit is on a call, dispatch shall immediately send a backup unit and then attempt to raise the unit on the relevant radio channel to confirm status. Dispatcher will broadcast by voice that "help is on their way."
 - 3.10.1.2 If the unit is not currently on a call, dispatch will immediately attempt to raise the unit on the relevant channel and confirm location and status.
 - 3.10.1.3 As soon as practical, the unit declaring the emergency shall contact dispatch via voice to confirm the nature of the situation.
 - 3.10.1.4 The MDT does not have to be logged on to send an emergency message.
- 3.11 **Message Acknowledgment:**
 - 3.11.1 Do not assume that messages or information sent via the MDT to dispatch or other MDT's have been read or that action has been taken without receiving acknowledgment.
 - 3.11.1.1 **Log off:** It is the field unit's responsibility to properly log off the MDT at the end of the shift by using the LOGOFF command. The MDT shall not be logged off during the shift unless there is operation failure. The MDT/MDC should not be powered off while it is still logged on. This causes the system to continuously attempt to reach the terminal and can dramatically slow responses to other system users.
 - 3.11.1.2 **Security:** The MDT may be disabled for security reasons by using the "LOCK" function as needed whenever the user is away from the unit. To reactivate the MDT, use the "UNLOCK" function. Do not use the camera of the MDT for any reason.
 - 3.11.1.3 **MDT Operation Failure:** If an MDT fails to operate properly the crew assigned to the unit shall perform trouble shooting procedures before declaring failure. If the MDT is still not working, the crew shall notify dispatch immediately via voice that the unit no longer has MDT capability. It will be the MDT user responsibility to fill out an "MDT failure" form and follow the procedure of changing the MDT.

4. CARE, CLEANING, AND SERVICE OF MOBILE DATA TERMINALS:

- 4.1 All requests for installation, removal, or modification shall be forwarded in writing to the Communications technical support (or communication Production supervisor).
- 4.2 No equipment or items may be attached to MDT other than those specifically designed to be a part of the MDT by the manufacturer.
- 4.3 MDT display screens shall be cleaned with commercial glass cleaner and a soft cloth only.
- 4.4 MDT keyboards shall be cleaned using alcohol and a soft cloth. DO NOT use abrasives, solvents, or acetone to clean MDTs.
- 4.5 Spillage of any type must be removed from the MDT surface as quickly as possible using a soft clean cloth. Any incident of spillage shall be reported on the "MDT Service" form and forwarded to the Communications technical support (Communications production Supervisor).
- 4.6 Required repair or service of MDT's shall be properly documented on the designated MDT repair form. Completed forms shall be turned in to service personnel as quickly as possible after the failure is noted.

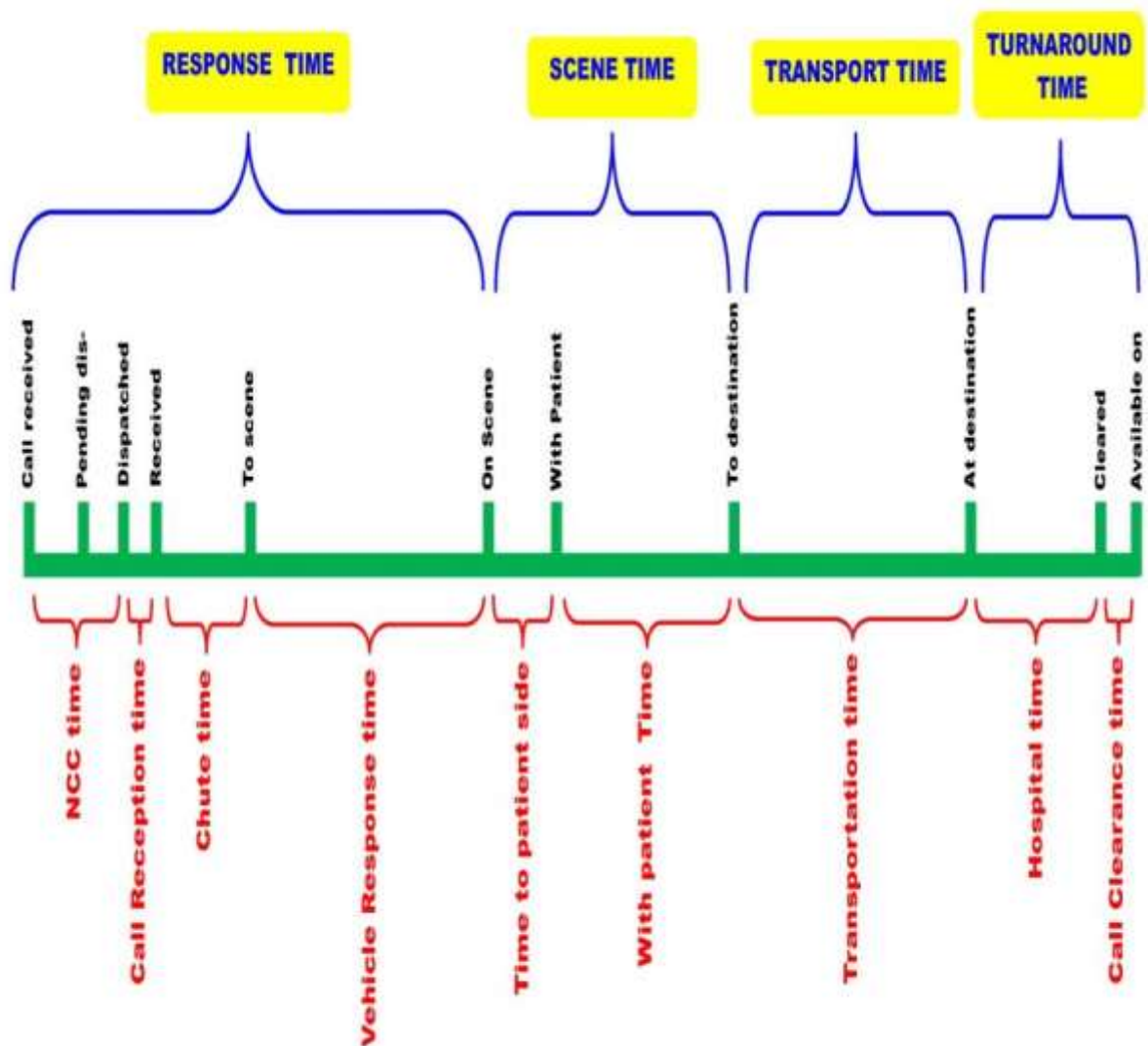
5. Automated Vehicle Locator (AVL):

- 5.1 As part of the technical solution to ensure effective and safe utilization of Ambulance Service resources all units are installed with an AVL system.
- 5.2 Dispatchers shall use the AVL tracking system to assist in assigning the appropriate unit to a call.
- 5.3 Dispatchers shall use the AVL tracking system to guide units more accurately towards the scene of an incident, should the MDT fail.
- 5.4 The AVL does not replace MDT or radio procedures.
- 5.5 The AVL system allows the control room to pinpoint the exact location of all units at all times, ensuring the safety of responding crews by facilitating a quick response for any type of assistance needed.

6. Attachments:



Breakdown of the Ambulance operation time intervals



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.16 MAJOR INCIDENT RESPONSE ON CALL PHONES
Implementation Date	01 June 2016
Purpose	To ensure that on call phones associated with the Major Incident Response On Call Roster work effectively.
Responsibility	NATIONAL COMMAND CENTRE

1. The National Command Centre (NCC) shall host a bank of phones that will be used for on call purposes.
2. The primary use of these phones is so that HMCAS staff can reach a particular call sign at the same number always, regardless of the actual individual/person responsible for the duties and responsibilities associated with that call sign.
3. The Major Incident Response (MIR) On Call Roster shall be sent out from the HMCAS Emergency Services office each Sunday.
4. The Oscar NCC shall:
 - 4.1 Review the worksheet, locating those MIR call signs that have a dedicated On-Call phone.
 - 4.2 Forward the dedicated phone line to the associated HMCAS staff members specified contact number.
 - 4.3 Do a test call to the MIR dedicated phone number to ensure that the HMCAS staff member who is supposed to receive the call, actually receives the call.
5. During the routine afternoon MIR radio communications check, the NCC shall do a test call to the MIR call signs with a dedicated-On Call phone.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations and Production
Procedure	3.17 MANAGEMENT OF CALL SIGN COMMUNICATIONS EQUIPMENT SETS (CCES)
Implementation Date	01 June 2016
Purpose	To identify the activities and processes to be used in distribution and management of call sign communications equipment sets for all response units.
Responsibility	PRODUCTION MANAGERS AND SUPERVISORS, TEAM LEADERS, AND HMCAS OPERATIONAL STAFF

1. Definitions:

- 1.1 Call sign communications equipment sets (CCES) – a Protection Case with a set of communications equipment required for a vehicle to be activated as a call sign.
- 1.2 Vehicle – an ambulance, a response car, or a bus, etc. with a registration number.
- 1.3 Unit – a vehicle with a call sign

2. General Guidelines:

- 2.1 HMCAS operational staff shall:
 - 2.1.1 Ensure familiarization with the use of CCES
 - 2.1.2 Be responsible for CCES while registered to them in line with this SOP.
 - 2.1.3 Request assistance if additional training or direction is needed for the use of CCES.

3. Use of Protection Case and its Contents:

- 3.1 The Protection Case is principally intended as a means of protection for communication equipment/devices (ePCR, NJM/MDT pad, TETRA radios, mobile phone, battery chargers, etc.)
- 3.2 Staff shall use the Protection Case only for storing the above-mentioned devices.

4. Actions by Emergency/Non-Emergency Operational Staff at Commencement of Shift:

- 4.1 Collect the CCES from either the unit's previous crew or from the Hub Production Supervisor if the unit is coming onto shift.
- 4.2 Note the serial number of the CCES and write it on the vehicle checklist.
- 4.3 Check the battery level of the devices and recharge if required.
- 4.4 As soon as practically possible, complete a vehicle checklist which will include the CCES.
- 4.5 Ensure that the serial numbers, physical presence, and serviceability of all CCES items are recorded and reported to Communications immediately after checks. If the unit is required to attend a call before completing the check, comply at the earliest practicable time, normally on completion of the call.
- 4.6 If a unit's vehicle needs to be changed, the attendant shall transfer the CCES to the new vehicle. The Hub Production supervisor shall advise the PTL accordingly.
- 4.7 CCES items shall be kept in their charging dock, where provided, when the attendant is in the vehicle, especially when the vehicle is on the move.
- 4.8 Unless handing over directly to the incoming crew at the end of the shift, carefully replace the items to the Protection Case so that it is as you would expect to find it. Where applicable, both

crew members shall be responsible for the CCES pack and need to agree that the pack is complete.

4.9 If the Protection Case has some missing items:

4.9.1 Record this on the vehicle checklist

4.9.2 Follow procedure as per **Section 5**

5. Actions by Emergency and Non-Emergency Staff in the Event of Equipment Loss, Failure/Damage, and Theft:

5.1 Advise the PTL and Hub Production supervisor as appropriate and act upon their instructions.

5.2 If stolen or lost, the Hub Production supervisor shall notify PTL to inform the police within 48 hours.

5.3 Complete an incident report

5.4 Line management shall provide a replacement CCES and shall interview staff to establish the circumstances surrounding the loss, theft, or damage.

5.5 Hand over a faulty item to the Hub Production Supervisor for replacement and label as "Faulty Equipment" on a RED tag.

5.6 Complete and sign an OVA and defect form after receipt of the new device/equipment.

5.7 Register the new device with the Hub production supervisor who shall notify the PTL.

5.8 The Hub Production Supervisor shall send the faulty unit back by to the Main Store for repair.

5.9 In the case of damage, loss, or a protection case locking fault, CCESs shall be swapped out.

6. Battery Charging of Equipment/Devices by staff:

6.1 Ensure that equipment/devices are charged at all times.

6.2 Outgoing crew shall endorse the CCES to the incoming crew with equipment/devices in good/full battery level.

6.3 Never leave the devices out of charge

6.4 Charging stations are available in the spokes or hubs. Non-Emergency ePCR and MDT units can be charged in the vehicle docks.

6.5 If the battery is not charging/faulty, refer to Section 5

7. Distribution of CCESs

7.1 Issuing CCES:

7.1.1 The initial issue of CCESs shall be coordinated centrally by the equipment Store. Production Managers in the Store shall ensure that CCESs have been delivered to the hubs and that they are signed for by the hub production supervisor. Hub Production Supervisor shall ensure that the Serial Number is recorded on the form at the time of signing.

7.1.2 Issuance of CCESs to the Training shall be managed by the Training Supervisor.

7.2 Record Keeping/Documentation:

7.2.1 HMCAS shall maintain a database of equipment contained in CCESs via an asset tracking system.

7.2.2 Equipment shall be bar-coded and scanned

- 7.2.3 Logistics shall have access to the database for monitoring and checking purposes. This procedure introduces one equipment tag, one Equipment Exchange Record (see Figure 3.17-1– Equipment Exchange Record) and station whiteboards.
- 7.3 **Managing Buffer Stock:**
- 7.3.1 The purpose of buffer stock is to ensure crews can have an immediate replacement in the event of loss or damage of the CCES.
- 7.3.2 Hub Production Supervisors shall maintain a stock of replacement equipment and ensure that arrangements are in place for accessing this stock for emergency replacements on a 24/7 basis.
- 7.4 **Scheduled Maintenance/Checking:**
- 7.4.1 Equipment/devices shall be maintained and repaired by approved contractors in accordance with manufacturer's recommendations and guidelines. This includes function tests and scheduled servicing where appropriate. This process shall be facilitated by the Support Services and shall be undertaken either at the Main Store or at the ambulance stations/hubs.
- 7.4.2 Production Supervisors shall ensure that equipment is made available for maintenance (if informed of pending service visits).
- 7.4.3 A database of all individual items of equipment which are maintained according to the schedule of the contractor maintained by the equipment supplier.
- 7.4.4 Maintenance contractors provide details of all equipment serviced on their visits and this information shall be placed on the database and may be used by Logistics to identify any items missed. These items will then be recalled to the Logistics for servicing within the approved timetables.
- 7.5 **Audit:**
- 7.5.1 The PTL shall ensure that a complete level register of CCESs issued to hubs is maintained, updated, and sent to the Director of Health Care Coordination Services and the Director of Support Services on the last day of each year.
- 7.5.2 The Head of Training Section and Production Supervisor shall ensure the audit of all CCESs distributed to the Training section for education purposes and shall send a copy of the report to the Director of Health Care Coordination Services and the Director of Support Services on the last day of each year.

Ambulance Service
PROTECTION CASE RETURN FORM
EQUIPMENT EXCHANGE FORM

This form must be completed when returning protection cases or any of its content (device/equipment).
You must ensure you return the case and all its content.
Please provide the below details:

Hub: _____ **Date:** _____

Device Name/Serial Number:

Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____
Reason Code: _____

Reasons:	Codes
<i>Damaged</i>	A
<i>Locking Fault</i>	B
<i>Loss</i>	C
<i>Received equipment with error</i>	D
<i>Other (please specify)</i>	E

IMMEDIATE**EQUIPMENT EXCHANGE RECORD**

Date:		Time:		Hub:		Unit Call Sign:		Vehicle No.:	
-------	--	-------	--	------	--	-----------------	--	--------------	--

Item/Device

	TETRA Radio
	Tough Pad
	Mobile Phone

	Charger
	Batteries
	Protection Case

Fault

--

MANUFACTURER'S SERIAL NO:

--

HMC ASSET NO:

--

REPAIR TAG NO:

--

EQUIPMENT EXCHANGE PERSONNEL**Name** (please print)

--

Signature*Incident Report
attached?*

Yes / No

--

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications - Healthcare Coordination Centre
Procedure	3.18 CAREMONX CAD SHUTDOWN
Implementation Date	09 March 2020
Purpose	To ensure appropriate use of backup systems available to manage calls for service and resources in the event CareMonX CAD is not available.
Responsibility	ALL COMMUNICATIONS STAFF

1. Staff are responsible for maintaining familiarization with these resources in the event of an unplanned CareMonX CAD crash or scheduled system down time.
 - 1.1 Scheduled downtime requires coordination with affected stakeholders and pre-notification no less than 48 hours in advance by the ASIS/VTL.
2. Each employee must routinely review CareMonX backup procedures and resources.
3. Each employee is responsible for soliciting assistance from the case controller or team leader if additional directions are needed.
4. Each employee must ensure they are adequately prepared to follow backup procedures should they become necessary.
5. **In the event of a CareMonX failure or scheduled maintenance:**
 - 5.1 User must log off and restart the system to check if failure is user based or individual system shutdown.
 - 5.2 The Operation Officer for Communications or the Utilization Team Leader should notify immediately the OUNAK at 44391444 or via email to get a ticket opened immediately.
 - 5.3 The Operations Officer for Communications or the Utilization Team Leader must immediately notify the On-Call Communications Operations Manager (Silver Communications).
 - 5.3.1 The On-Call Communications Operations Manager (Silver Communications) will be responsible for activating the full Disaster Recovery and Business Continuity Plan when:
 - 5.3.1.1 The primary facility providing the service is inaccessible; as a result, technology processing can no longer continue.
 - 5.3.1.2 A major technology component failure has occurred and is preventing technology capabilities.
 - 5.3.1.3 A power failure has occurred, and technology processing cannot continue.
 - 5.3.1.4 Corruption of data has occurred, and data needs to be recompiled, restored, and synchronized.
 - 5.3.1.5 There is a need to migrate or change IT systems or applications for an extended period.
 - 5.3.2 This Disaster Recovery and Business Continuity Plan when, plus all the other Disaster Recovery documents listed in Table 1 of the plan, should be stored on-site at Ambulance Service Healthcare Coordination Centre (HCC), Corporate HICT – DR/BC section, and at the primary backup site, the National Command Centre, a secure remote location.
 - 5.4 HCC shift supervisor or most senior officer on duty must notify ASIS team on (5509 3574/5518 4726, 55135126) in accordance with [SOP 3.26](#) once they have notified OUNAK and received a ticket number.

- 5.4.1 Any major outage or downtime, affecting the Ambulance Service operations will be considered as disaster event. An outage event will be broadly classified depending upon the downtime foreseen.
- 5.4.2 The downtime (or non-availability of System or physical site) which will be either known (planned) or unknown (unplanned) can be broadly classified as following:
 - 5.4.2.1 Level 1: downtime less than 1 hour;
 - 5.4.2.2 Level 2: downtime greater than 1 hour, but less than 4 hours;
 - 5.4.2.3 Level 3: downtime greater than 4 hours, this is a major incident and will result into disaster declaration.
- 5.5 Communications officer shall utilize the Manual Communications Record (available from the UTL desk) to process calls for service and make dispatch assignments.
 - 5.5.1 Any new incident received during the shutdown will be numbered as follows:
 - 5.5.1.1 1st incident will take the number YYYYMMDD-9000 thru to 9399 for PTS cases (Example: The 1st case opened during the shutdown will be: 20180125-9000).
 - 5.5.1.1.1 2nd incident will take the number YYYYMMDD-9001 etc... for PTS cases. Then the numbers will keep adding for each new CFS (opened during the shutdown) until CareMonX is back to operation.
 - 5.5.1.2 1st incident will take the number YYYYMMDD-9400 thru to 9699 for IFT cases (Example: The 1st case opened during the shutdown will be: 20180125-9400).
 - 5.5.1.3 1st incident will take the number YYYYMMDD-9700 thru to 9899 for RET cases (Example: The 1st case opened during the shutdown will be: 20180125-9700).
 - 5.5.1.4 These numbers during the shutdown will be coordinated by the on-duty UTL and will be written on the incident number manual-sheet before providing it to the dispatcher.
 - 5.5.1.5 The dispatcher will then verbalize the Shutdown's incident number to the Field Crew(s) via radio.
 - 5.5.1.6 The running (active) incidents during the shutdown will maintain their original CareMonX incident number and NO Shutdown incident Number will be created for them.
 - 5.5.1.7 The last 4 digits of the Shutdown's incident Number shall not be used twice during the same day.
- 5.6 Manual entry will require the same information as the CareMonX CAD screen to be handwritten in the appropriate space on the Manual Communications Record.
- 5.7 Call taker shall enter the necessary information in accordance with [SOP 3.22](#) HCC Call Taking before forwarding the sheet to the dispatcher.
- 5.8 The Dispatcher shall accurately capture all remaining information including time stamps.
- 5.9 Once the case is completed/cleared, the Dispatcher shall endorse the manual call record to the UTL for reviewing, completion and signature.

6. Once CareMonX CAD has been restored:

- 6.1 Calls received during the shutdown will be logged in CareMonX CAD.
- 6.2 All running cases during the return of CAD should be processed and closed in CareMonX.
- 6.3 Notify CDO via email the duration & number of calls logged manually if still not entered to CAD.
- 6.4 The Case controller will collect all manual call records endorse it to the UTL for documentation and filing.
- 6.5 All reported equipment failures must be documented by the UTL or Operations Officer on duty via the "OVA" reporting system as per HMC policy and complete a summary report, check list for CAD shut down.

7. Testing:

- 7.1 If there have been no failures of the system, the failover system will be tested and documented bi-annually.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.19.A NCC EVACUATION PLAN
Implementation Date	01 October 2022
Purpose	This procedure identifies the disaster recovery site communications center to be used when the National Command Center (NCC) is to be evacuated and describes the levels of evacuation.
Responsibility	ALL COMMUNICATIONS (HCS) STAFF

1. PROCEDURE:

- 1.1 An evacuation will be considered for any situation that puts HMC-AS staff at the NCC and TCC out of service or in danger that it cannot safely and effectively operate at its co-located TCC/MOI facility in Duhail.
- 1.2 When it is deemed necessary to evacuate the TCC some staff will relocate to a disaster recovery site identified by the management team.
 - 1.2.1 If the problem is limited to only one section of the control room, the operations in that area may relocate to another control room. In such situation HMCAS consoles in the TCC will relocate to the old building of NCC at the Joint Operation Center (JOC) which includes dedicated consoles for HMCAS.
 - 1.2.2 If the problem impacts the entire buildings-complex, partial operations will move to the disaster recovery site located within the designated TCC/MOI primary disaster recovery site at Bin Omran Area while the same portion of the operations will move to the HMC-AS headquarter and some will remain standby for back to service.
 - 1.2.3 TCC/MOI primary disaster recovery site is located at Civil Defense Headquarter and featured with 2 consoles for Team leaders (1 DTL & 1 PTL). All HMCAS communications staff will be allowed to enter the building by presenting both HMC-ID and NCC access card.
 - 1.2.4 HMC-AS headquarter backup center will be at HCC call center and will be featured as follows:
 - 1.2.4.1 8 Consoles at HCC call center room number AS-1-0120, 4 dispatching, 3 call taking and 1 Team Leader. The hotline 403-28201 for call takers will be provided to access CISCO DIDs 403 28201, 2 8202, 28203, the dispatchers will be provided to access to 403-28204, 28205, 28206, 28207. 55181032 mobile number for NCC supervisor.
 - 1.2.4.2 3 consoles for the Clinical Desk at HCS Quality office room number AS-1-0113; 1 Clinical Coordinator, 1 LifeFlight Coordinator and 1 EMD: Retrieval Coordinator. The contact number 4439-8833 for the Clinical Desk will be unchanged through IVR access when staff login to CISCO Finesse for DIDs 403-28222-28223. The hotline number 403-28499 is retained if the IVR experiences any issues while the Clinical Desk is at HMCAS HQ.
- 1.3 Once all disaster recovery site positions are staffed and operational, dispatchers shall notify all on duty supervisors and Managers that the relocation is complete and broadcast over all primary channels that service is now operational from the disaster recovery site.
- 1.4 Staff may transport themselves to the designated disaster recovery via personal vehicles with no less than two people per vehicle. If staff vehicles are unable to be used, Oscar-NCC will

- arrange to transport them to the designated disaster recovery site using HMC AS resources or HMC Transport.
- 1.5 If the evacuation is extended beyond shift change, the coming shift will report to the designated disaster recovery site directly via personal vehicle (One team leader from group1 shall notify the incoming shift staff to which location they should respond).
 - 1.6 Anytime an alarm or emergency condition affects the TCC building, Oscar-NCC or their designee must establish close contact with Communications Operations Manager to ensure all available facts regarding the incident are known to aid in the decision regarding evacuation.

2. EVACUATION PLAN:

- 2.1 To allow for uninterrupted emergency services; during an evacuation below steps will take place simultaneously by OSCAR NCC or his delegate at TCC.
 - 2.1.1 Immediately notify the on-call management team and SME: Clinical Desk. The HMCAS emergency services manager will be notified first to start their intradepartmental notifications in a cascade manner.
 - 2.1.2 Notify by mobile or radio the HCC on duty supervisor to prepare all the 11 backup consoles at HCC and HCS Quality office.
 - 2.1.3 Coordinate with the 999-officer to activate a radio channel between TCC 999 supervisor and one HMCAS ASMD call taker for endorsing emergency requests. (Radio folder and channel names will be arranged as per availability).
 - 2.1.4 The call taker monitoring the 999-radio channel will log the received cases in NJM-Shutdown paper forms or in a list including the exact address, the nature of incident and the caller telephone number. A coordinator or a team leader will assist to endorse these cases promptly to the group of dispatchers (refer to NJM shutdown SOP).
 - 2.1.5 ProQA will be omitted and all cases will be dispatched as priority 1.
 - 2.1.6 Instruct all dispatchers to keep monitoring their radio channels through TETRA while evacuating the building and document the current CFSs and unit status in a list in A4 paper for each dispatcher.
 - 2.1.7 The operational channel will be merged as follows:
 - 2.1.7.1 Dispatcher1: Doha 1 and South channel
 - 2.1.7.2 Dispatcher2: Doha 2 and North channel
 - 2.1.7.3 Dispatcher3: South 1 channels
 - 2.1.7.4 Dispatcher4: South 2 and South2 channels
 - 2.1.8 If the address is not clear and not complete the dispatcher will ask the case controller to call back the caller for clarifications.
 - 2.1.9 Inform 999 officers not to forward 999 calls yet to HMCAS headquarter-HCC dedicated CISCO branch until a group of call taking consoles are ready.
 - 2.1.10 All staff will move to the TCC/MOI assembly point while monitoring the channels and processing calls.
 - 2.1.11 To ensure accountability for staff Oscar-NCC or the most senior person in charge will do a head count prior to releasing staff to go to the designated disaster recovery locations or to remain in standby at TCC.
- 2.2 OSCAR NCC to start the move to the backup centers and will divide the staff to three groups.

- 2.2.1 Group1 that consists of the 2 Team Leader (PTL&DTL) will move with their private vehicles to the designated TCC/MOI disaster recovery site at Bin Omran area to ensure the following tasks:
 - 2.2.1.1 Activate the consoles
 - 2.2.1.2 Monitor the activities
 - 2.2.1.3 Liaison with MOI and other agencies
- 2.2.2 Group2 that consists of 50% of the EMDs, 50% of the Clinical Desk staff (1 Clinical Coordinator, 1 LifeFlight Coordinator, 1 EMD: Retrieval Coordinator), OSCAR communication and 1 Team leader. will be transported by an HMC van if available, otherwise with their private vehicles to the HMC-AS HQ. once the team arrive, they shall ensure the followings:
 - 2.2.2.1 Activate 3 Call taking Consoles
 - 2.2.2.2 4 Dispatch consoles
 - 2.2.2.3 1 Team leader Console
 - 2.2.2.4 3 Clinical Desk consoles
 - 2.2.2.5 Once all consoles are ready for service OSCAR NCC will inform MOI officer to activate the transfer all 999 calls to the hotline 403-28201.
- 2.2.3 Group3 that consists of the remaining staff will stay at the assembly point in a state of readiness to get the TCC operational once back to service.

3. NOTIFICATION:

- 3.1 Notify Silver Communications and NCC Operations Manager. The manager or designee will notify additional administrative personnel as needed but shall include the EAS, Clinical Desk and LifeFlight on-call managers.
- 3.2 NCC Operations Manager will announce and declare the evacuation of the TCC/MOI building in collaboration with Oscar-NCC and MOI officers.
- 3.3 Notify HCC 402 8210/8209/8219
- 3.4 HMC-AS on call manager will be kept updated with all details and steps of the activation.
- 3.5 Notify the NHICC 402 6555

4. DOCUMENTATION:

- 4.1 All evacuations process must be documented by the Operations Officer on duty or designee:
 - 4.1.1 Document via "OVA" reporting system as per HMC policy.
 - 4.1.2 Complete a summary after action report including learning points and challenges
 - 4.1.3 Complete the checklist for all evacuation steps

5. DEACTIVATING THE PLAN:

- 5.1 The Communications Operations Officer will follow the same process as for activating the plan and relay the need for reversal of the plan back to TCC/MOI. Group3 will resume TCC/MOI operation, followed by Group2 and then Group1
- 5.2 Notify NCC Operations Manager
- 5.3 Notify the NHICC 402 6555
- 5.4 Notify HCC 402 8210/8209/8219

6. TESTING:

- 6.1 At the recovery sites all telephone lines, CAD and radio channels of the disaster recovery locations should be tested prior to being used.
- 6.2 Testing of all the backup consoles will be done weekly by TCC staff using a standardized checklist.
- 6.3 A complete exercise of evacuation and relocation will be done twice a year, one in each semester.
- 6.4 The checklist will be saved under the evacuation documents on the communication shared drive.
- 6.5 All reported equipment failures must be documented by the Supervisor or Operation Officer on duty via the "OVA" reporting system as per HMC policy. The initiator of the OVA shall make all required follow up to solve the failure within 24 hours. Any unsolved failure shall be escalated to the executive via official communications.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.19.B HCC EVACUATION PLAN
Implementation Date	01 October 2016
Purpose	This procedure identifies the backup communications center to be used when the Healthcare Coordination Center (HCC) is to be evacuated and describes the levels of evacuation.
Responsibility	ALL COMMUNICATIONS (HCS) STAFF

1. An evacuation will be considered for any situation that puts HMC-AS Healthcare Coordination Center (HCC) out of service or in danger that it cannot safely and effectively operate at Ambulance Service HQ.
2. When it is deemed necessary to evacuate the Healthcare Coordination Center all personnel will relocate to a backup center identified by HCS management.
 - 2.1 If the problem is limited to only one section of the control room, the operations in that control room may relocate to another control room.
 - 2.2 If the problem impacts the entire building, all operations will move to the backup center located within the National Command Center (NCC), which is the primary disaster recovery site.
3. All telephone lines, CAD, and radio channels of the backup communications center should be tested prior to being used.
 - 3.1 Testing will be done weekly by HCC staff inside the NCC staff using a standardized checklist.
 - 3.2 The checklists will be saved under the evacuation documents at HCC.
 - 3.3 \\hmcfs05\EMS\Communication Staff files\PTS COMMUNICATIONS\HCC REPORTS\HCC WORKSTATION CHECKLIST.
 - 3.4 All reported equipment failures must be documented by the Supervisor or Senior Officer on duty via the "OVA" reporting system as per HMC policy.
4. Anytime an alarm or emergency condition affects the building, the Communications Operation Officer (Oscar-HCC) or their designee must stay in close contact with Communications Operations Manager to ensure all available facts regarding the incident are known to aid in the decision regarding evacuation.
5. **STAGED EVACUATION:**
 - 5.1 To allow for uninterrupted emergency services; during a staged evacuation staff will divide into two groups.
 - 5.1.1 The first group (Group "A") will relocate to the designated backup center and activate the equipment and confirm phone lines have been redirected.
 - 5.1.1.1 "Group A" should consist of the UTL, Case controller ,1 dispatcher and 2 call-takers (those staff who can get access to NCC).
 - 5.1.2 Once this has occurred, the remaining staff (Group "B") will then relocate to the backup center.
 - 5.2 During a staged evacuation, the HCC Operation Officer will remain with Group "B" at HCC.
 - 5.3 The determination of which supervisor remains with which group may be made by the Communication Operation Manager or senior person in charge on a case by case basis.
 - 5.4 During a staged evacuation, staff may arrange transport or transport themselves to the backup center via personal vehicle.

- 5.4.1 If the evacuation is extended beyond shift change, the oncoming shift will report to the backup center directly via personal vehicle (UTL shall notify the incoming shift where to report).
- 5.5 Once all backup center positions are staffed and operational, dispatchers shall notify all on duty supervisors and Managers that the relocation is complete and broadcast over all primary channels that service is now operational from the backup center.
- 5.5.1 This will also occur during reversal of the plan from NCC to normal operations at HCC.

6. IMMEDIATE EVACUATION:

- 6.1 Immediate evacuations of all staff will occur without delay due to an imminent threat.
 - 6.1.1 An example would be the building filled with smoke.
 - 6.1.2 If evacuation is due to a bomb threat, DO NOT transmit on mobile/portable radio equipment during the evacuation or within 300 feet of the building.
- 6.2 All staff will report to the HCC assembly point then to designated transportation center.
- 6.3 To ensure accountability for staff the OSCAR-HCC or the most senior person in charge will do a head count prior to releasing staff to go on to the backup center.
- 6.4 Staff may transport themselves to the backup center via personal vehicles with no less than two people per vehicle.
- 6.5 If staff vehicles are unable to be used OSCAR-HCC will arrange to transport them to the backup center by HMC AS green bus or coordinate with fleet section to provide transport car.

7. ACTIVATING THE PLAN:

- 7.1 HCC Operations Manager will announce and declare the evacuation of the HCC building in collaboration with OSCAR-HCC.
 - 7.1.1 Together they will define the method and level of the evacuation (Staged or IMMEDIATE).
- 7.2 HCC Operations Officer/UTL will notify Oscar NCC/DTL.
- 7.3 NCC Operations Officer/DTL, PTL will take in charge the activation of the HCC workstations and coordinate staff access to NCC.
- 7.4 Retrieval desk will undertake all HCC incoming calls and ongoing cases at CAD.
- 7.5 HCC Operations Manager will be updated with all details and steps of the activation.

8. NOTIFICATION OF EVACUATION SHALL BE MADE TO:

- 8.1 Notify Silver Communications and HCC Operations Manager. The manager or designee will notify additional administrative personnel as needed but shall include the SAS on-call managers (or any other users like Diabetic).
- 8.2 Notify the hospital/Bleep station 40260334.
- 8.3 Notify HMC communications center 443 92210/6626 2083.

9. DEACTIVATING THE PLAN:

- 9.1 HCC Operation Manager Officer/OSCAR-HCC will follow the same process as for activating the plan and relay the need for reversal of the plan back to HCC. Test calls will follow to ensure a successful reroute back to HCC.
- 9.2 Notify HCC Silver Communication
- 9.3 Notify the hospital/Bleep Station 44260334

9.4 Notify HMC communications center 443 92210 Mobile: 6626 2083

10. Documentation:

- 10.1 Immediate evacuations of all staff will occur without delay.
- 10.2 All staff will report to the HQ assembly point.
- 10.3 HCC Operations Officer/UTL will notify Oscar NCC/DTL.
- 10.4 NCC Oscar will notify Retrieval desk to undertake all HCC incoming calls and ongoing cases at CAD.
- 10.5 HCC Operation Officer/UTL will notify Communication Manager and SAS and silver communication.

11. TESTING:

- 11.1 The Evacuation Plan should be tested annually with documentation of adherence to the plan and lessons learned for improvement purposes.
- 11.2 To ensure update implementation into the computers the Oscar/UTL will ensure the restart of the computers twice a week, Tuesday and Friday at low call volume periods.
- 11.3 To ensure the functionality of the HCC Phone number 16099 a regular checking on hourly basis is done by the operation officer or supervisor by dialing the number and getting connected to the agent.

HCC Evacuation Plan
CHECKLIST



خدمة الإسعاف
Ambulance Service

HCC Supervisor:
Corporation #:

Date:
Shift:

Time of Evacuation:

ACTIONS			
Unit/Facility: HCC/Retrieval Control room		Yes	No
1	STAGED EVACUATION Plan needed	☐	☐
2	IMMEDIATE EVACUATION Plan needed	☐	☐
3	Contact HMC Control room: 4439-2333/3888/7386	☐	☐
4	Notify ON CALL Silver Communication	☐	☐
5	Notify Communication Manager	☐	☐
6	Make evacuation action plan and coordinate amongst Team Leaders, Operation Officers, Call Receivers and Dispatchers	☐	☐
7	Ensure that UTL documents time of Evacuation	☐	☐
8	Emergency exits and evacuation route identified at end of the hall way	☐	☐
9	Assembly point identified outside the building near the parking area	☐	☐
10	If necessary, evacuate HCC all personnel will relocate to a backup center identified by HCS management	☐	☐
11	HCC Operations Manager notify NCC as the backup center for the evacuation plan	☐	☐
12	Retrieval Desk undertake all Radio communication and ongoing CAD cases.	☐	☐
13	Transport arranged for staff going to backup center?	☐	☐
14	Staff at backup center notified to set up for arrival of HCC	☐	☐
15	Evacuation Plan Deactivation	☐	☐
16	Notify hospital Bleep station and HMC communications Centre if needed	☐	☐

Signature:

**Incident Timings:**

AFTER ACTION		Yes	No
Operations Officer/Manager			
1	Where all steps completed on the checklist? If no, why?	☐	☐
2	Where there any obstacles /challenges with completing the checklist? If yes, why? <u>Unfamiliarity with steps and action plan</u>	☐	☐
3	Could anything be done better the next time to mitigate the shutdown?	☐	☐
4	Action items (if any):	☐	☐

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.20 CUSTOMER SERVICE COMMUNICATIONS FOR HEALTHCARE COORDINATION SERVICE CALL CENTERS
Implementation Date	13 March 2018
Purpose	- To standardize the approach of NCC and HCC staff in interacting with external and internal users of HCS Communication Centers. - To provide all ASMD with a well-defined procedure for managing callers and providing those callers with sufficient reassurance, emotional comfort, and customer service. - To ensure the delivery of highest customer service at HCS.
Responsibility	ALL COMMUNICATIONS (NCC & HCC) STAFF

1. SOP STATEMENT:

- 1.1 Ambulance Service Medical Dispatchers (ASMDs) shall handle all calls for ambulance assistance, whether emergency or non-emergency, using established caller management and customer service practices. It is understood that not all callers will be calm and/or cooperative. ASMDs will anticipate those situations and respond as trained professionals following the MPDS.
- 1.2 HMCAS expects all HCC & NCC staff to exhibit the highest level of customer service at HCS. This applies to all internal and external communications via telephone.

2. DEFINITIONS:

- 2.1 ASMD:
 - 2.1.1 Ambulance Service Medical Dispatcher: Title given to a Calltaker/dispatcher that has successfully completed the internationally accredited dispatch training program.
- 2.2 MPDS:
 - 2.2.1 Medical Priority Dispatch System: (Flip chart and Software) is a medically approved system used to dispatch the most appropriate medical resource to the person calling for service.
- 2.3 PAIs:
 - 2.3.1 Pre-Arrival Instructions: Life-sustaining Information and instructions given to the caller to follow while awaiting the arrival of the ambulance.
- 2.4 PDIs:
 - 2.4.1 Post-Dispatch Instructions: General instructions given to all callers to follow while waiting for the ambulance to arrive.

3. CASE OPERATION PROCEDURE:**3.1 Opening the call:**

- 3.1.1 For HCC, the greeting script will include:
 - 3.1.1.1 "Ambulance Service - This is _____ (staff name) speaking. How may I help you?"
 - 3.1.1.2 **خدمة الإسعاف, فلانة/فلان معاك, كيف أقدر أساعدك؟**
- 3.1.2 For NCC emergency and hotline calls, the ASMD will use:

3.1.2.1 "Ambulance Service, This is _____ (staff name) speaking, What is the address of the emergency?"

3.1.2.2 *خدمة الإسعاف, فلانة/فلان معاك, ما هو عنوان حالة الطوارئ؟*

3.1.3 For NCC routine administrative calls, the below script will be applicable:

3.1.3.1 "Ambulance Service, This is _____ (staff name) speaking. How may I help you?"

3.2 **Obtaining and confirming information:**

3.2.1 The below information will only be required for HCC cases (Refer to the Calltaking [SOP 3.22](#)).

3.2.2 Ask politely for the caller's information and properly document in the CAD.

3.2.2.1 *"May I have your name, corporation number, and telephone number, please?"*

3.2.3 Ask about the patient's information:

3.2.3.1 *"May I have the patient's HC number, name, exact location, destination, and the reason for transport?"*

3.3 **Using the Hold Guidelines:**

3.3.1 Put the caller on hold only when absolutely necessary for a few seconds or no more than 1 minute and use the Hold/Mute button properly, making sure the caller will not hear you when you put him/her on hold.

3.3.2 Never interrupt the caller just to put them on hold.

3.3.3 When necessary, do wait for a pause in the conversation before you put them on hold.

3.3.4 Tell them you are going to put them on hold and explain the reason before placing the caller on hold. "Mr. Ben, can you please hold on the line while I retrieve the information? {Pause for a response}.

3.3.5 Be sure to listen to the response.

3.3.6 Inform the caller that you will return to their call in as soon as possible. "Thank you. I will be back with you shortly. {Caller on hold}.

3.3.7 Thank the caller for waiting immediately when you return to the call and start the conversation "Thank you for holding Mr. Ben. I can now help you?..."

3.3.8 After placing your caller on hold, check back periodically (between 30-45 seconds). Give them the option to continue to hold if it takes longer than 60 seconds to find information or offer to call them back.

3.3.8.1 *"Ms. Maryam, I will check to see if your appointment is confirmed. Can you please hold for a minute?" {Pause for a response}.*

3.3.8.2 *"Thank you. I will be right back." {Caller on hold}.*

3.3.8.3 *"I'm sorry for the inconvenience. Your request is missing some information, May I have your complete name please?"*

3.3.9 When placing multiple calls on hold, remember to return to the first caller you placed on hold first.

3.3.10 Always apologize for any inconvenience or long hold and thank them for their patience and understanding.

- 3.3.11 Don't put the caller on hold multiple times in one call.
- 3.4 **Transferring Calls:**
- 3.4.1 Ask valid questions and ensure that the call will be transferred to the appropriate party.
- 3.4.2 Inform the caller to whom he/she will be transferred and state the reason for the transfer.
- 3.4.2.1 *"I need to transfer your call to _____ so that he/she can answer your question".*
- 3.4.3 Before transferring a call, confirm that the person to whom the call is being transferred is available.
- 3.4.4 As applicable, call taker may have to endorse pertinent information and provide a brief description/summary of what was discussed to the person to whom the caller is being transferred.
- 3.4.5 When receiving a transferred call, the call taker should always mention his/her facility name. This is to ensure correct transfer to a department/section.
- 3.5 **Politeness, courtesy and listening behavior:**
- 3.5.1 Always sound courteous, calm, and respectful throughout the call, no matter how mad a customer may be or behave. Avoid sounding unprofessional. (*Avoid saying: "Hang on." "Hold on." "Who's calling?" "I can't help you. You'll have to speak to someone else."*)
- 3.5.1.1 *Instead of saying "I can't hear you, speak up!" use "I am having a little difficulty hearing you. Can you please speak up?"*
- 3.5.2 Utilize the words *"Would you please?"*, *"Thank you"*, *"Sorry for the inconvenience"*, or other relevant statements as appropriate. Use the words *"your question,"* or *"your concern,"* or *"this situation,"* instead of *"your problem,"* or *"your complaint."*
- 3.5.3 Show empathy by saying *"I understand your concern and would be glad to assist you to resolve it"* or similar statements when needed.
- 3.5.4 After helping them with their request(s), ask if there is anything else you can help them with. *"Is there anything else I can help you with?"*
- 3.5.5 Do use the caller's first name or title to establish a personal connection with the caller. (*Jassim, Bill, Mr. Jones, Mrs. Stevens...*)
- 3.5.6 When a caller speaks, listen to what he or she has to say without interruptions.
- 3.5.7 Speak with the normal speed to avoid the impression of "rushing" the call.
- 3.6 **Show interest while handling the call:**
- 3.6.1 Maintain a caring voice tone and pitch using a clear voice that is neither too loud nor too low. Words should be pronounced and said slow enough that a caller is able to understand what is being said to him/her.
- 3.6.2 Positively answer all queries with confidence.
- 3.6.3 Paraphrase or summarize long details to confirm understanding.
- 3.7 **Conversation skills:**
- 3.7.1 Refrain from using jargon, abbreviations, technical or medical terms such as HCC, NCC, AP, and CCPs, etc.

- 3.7.2 Take control of the conversation. Keep the focus. Politely but firmly direct the caller on answering all questions as you ask them "Sir/Madam, it's important I get this information so I can help you."
- 3.7.3 Use correct language (English or Arabic) in words, phrases, and greetings.
- 3.7.4 Refrain from personal conversations. Focus on the concern(s) of the caller.
- 3.7.5 Process the call in an appropriate manner and close it within a suitable time frame.

3.8 **Escalation Procedure:**

- 3.8.1 Try to manage the caller's issue once or twice depending on the caller's tolerance. "Manage up" as appropriate when customers insist to talk with a supervisor.
- 3.8.2 For abusive, threatening, or vulgar language, let the caller know that you may need to escalate the call to your supervisor if he/she will continue with this unacceptable behavior.

3.9 **Closing the call:**

- 3.9.1 Call taker is not allowed to close the line abruptly during the conversation.
- 3.9.2 End the phone call professionally after completing all required questions/Instructions as per the protocols.
- 3.9.3 The call taker will make it clear to the caller that the conversation is finished, and the call will be terminated.
- 3.9.4 In routine disconnects the call-taker will wait for the caller acknowledgement and will let him/her hangup first.
- 3.9.5 In urgent disconnects call-taker will wait for the caller acknowledgement and will close the line immediately to take another call.
- 3.9.6 Always state the following 'send off' script:
 - 3.9.6.1 For HCC:
 - 3.9.6.1.1 "Thank you for calling Ambulance Service."
 - 3.9.6.1.2 "شكرا على اتصالك بخدمة الإسعاف"
 - 3.9.6.1.3 "Please call us back for updates or if anything changes. Goodbye!"
 - 3.9.6.2 For NCC: Please refer to the MPDS protocols (routine/urgent disconnect instructions).

4. **CALLER MANAGEMENT:** The ASMD will use the below procedures in managing callers with ProQA as described in [SOP 3.2](#) for NCC or [SOP 3.22](#) for HCC.

4.1 **Caller reassurance and explanations:**

- 4.1.1 Once Case Entry is complete, or whenever necessary to maintain caller focus, the ASMD will use reassuring statements and brief explanations. For example;
 - 4.1.1.1 "The ambulance is on the way to help you. Please stay on the line. I need to gather some important information so that I can tell you what to do to help the patient." OR "My partner is dispatching the ambulance right now; I'm going to get more information and give you some instructions before help arrives."

- 4.1.1.2 *For HCC cases with initial dispatch: Call taker may reassure the caller by stating “the ambulance has been arranged and I need to ask some additional questions, stay reassured this will not delay the response.”*
- 4.1.2 Politely but firmly focus the caller on answering all questions as you ask them. If callers lose their focus, get agitated or uncooperative, say:
 - 4.1.2.1 *“Sir/Madam it’s important I get this information so I can tell you exactly what to do.” Repeat this as often as necessary using the same phrasing.*
- 4.2 **Coping with Distressed, Hysterical, Rude, Angry and/or uncooperative Callers:**
 - 4.2.1 It is recognized that some callers will be highly distressed, uncooperative and, at times, abusive. Callers behave this way because they are frightened and feel helpless. When faced with these callers, the ASMD will maintain a professional demeanor and caring approach.
 - 4.2.2 The ASMD will always remain calm and courteous.
 - 4.2.3 The ASMD will maintain normal speaking volume and a professional, caring voice tone, avoiding yelling and any display of anger or contempt.
 - 4.2.4 Whenever possible, the ASMD will give clear, brief explanations as to what he/she is doing and why. For example, explain to the caller that they will need to ask some questions (so that you can get the proper priority).
 - 4.2.5 The ASMD will, when necessary, obtain and use a caller’s first name or title (Jassim, Bill, Mr. Jones, Mrs. Stevens, etc.)
 - 4.2.6 The ASMD will use ‘REPETITIVE PERSISTENCE’:
 - 4.2.6.1 Give the caller an action, followed by a reason for complying with the action.
 - 4.2.6.2 Repeat this, using the same phrasing and in a calm, level voice, as often as necessary until the caller listens and cooperates.
 - 4.2.7 The ASMD will continually reassure callers that he/she is there to help. It may be necessary to repeat this. The ASMD will explain that trained help is on the way and repeat it when necessary.
 - 4.2.8 The ASMD will, when necessary, use ‘POSITIVE AMBIGUITY.’ Do not ‘lie’ to the caller, even if motivated by kindness. Do not make promises or create unrealistic expectations for the caller. Examples follow:
 - 4.2.8.1 The caller asks, “How long will the ambulance take to arrive?” The ASMD will say, “The ambulance is on the way. It will be with you as soon as possible.” DO NOT GIVE AN EXACT TIME FOR ARRIVAL OF RESPONDING CREW(S). The ambulance may break down or get stuck in traffic.
 - 4.2.8.2 The caller asks, “The patient is going to be all right, isn’t s/he?” The ASMD will say, “Everyone will do the best they can to help.”
 - 4.2.8.3 The caller asks, “Will this save him/her?” The ASMD will say “This will help give him/her the best possible chance.”
 - 4.2.9 During pre-arrival instructions, the ASMD will give the caller firm but gentle encouragement. For example, if the caller says, “Nothing’s working!” The ASMD will say, “Don’t give up. You’ve got to keep doing it. This will keep him/her going until the ambulance crew arrives.”

- 4.2.10 When the caller is unable to answer questions or perform rescue techniques after the ASMD makes multiple attempts to employ caller management techniques, the ASMD will calmly ask the caller to let him speak to someone else.
- 4.2.11 The ASMD will never make any statements that foster or create feelings of helplessness, guilt, or panic in a caller.
- 4.2.12 The ASMD will never threaten a caller in any way, or engage in any discriminatory, derogatory, or demeaning behavior toward the callers, patients, family members or bystanders, explicitly or implicitly, through language, attitude, or voice intonation.
- 4.3 **Managing Abusive callers:** The ASMD should distinguish between manageable behaviors such as hysteria, distress, anger, etc. and unmanageable abuse.
 - 4.3.1 Abuse is when an ASMD is subjected to a personal attack, especially when it is based on, race, age, gender, or religion, it is considered hateful and it is not accepted nor tolerated by HMCAS. ASMD should:
 - 4.3.2 Attempt to de-escalate the situation. It is important not to respond to the behavior in the same way. ASMDs must not abuse callers, despite the caller abusing the ASMD.
 - 4.3.2.1 Allow the caller to speak. Try not to talk over the caller.
 - 4.3.2.2 Display empathy and compassion. Use words and phrases consistent with professional, compassionate communication to acknowledge the caller's feelings. e.g. "I'm here to help you", "we're doing this together", "I understand that" (explain what they have said, so they know you are listening).
 - 4.3.2.3 Use appropriate speech. Speak clearly, concisely, pace your words allowing the caller to understand, use a pitch that is not too high or low and use a firm and even tone – ensuring microphone placement is not distorting the volume.
 - 4.3.2.4 Use the caller's name or title.
 - 4.3.3 If the caller fails to stop and the ASMD finds the abuse intolerable, ask to speak to someone else at the address. If there is no one else available, the ASMD should seek assistance from their supervisor for immediate intervention.
 - 4.3.4 (For NCC) If no triage has been completed, code the incident 32B01 UNKNOWN.
 - 4.3.5 Document "ABUSE" into the incident comments, so the call will be flagged for review by a supervisor or Manager.
 - 4.3.6 In situations where a caller becomes abusive after the triage process is complete, ASMD may utilize the Urgent Disconnect script and must also notify their supervisor. Use of Urgent Disconnect will be in line with the "Urgent Disconnect SOP" and must include the reading of the protocol-specific PDI's. ASMD may escalate to his/her supervisor if required at any time.
 - 4.3.7 When an ASMD refers an abusive caller to their supervisor, they should take over the call, allowing the ASMD to disconnect. They will explain that they are the ASMD's Supervisor and that they will arrange help for them. The remainder of the call process should then be followed. (If the call cannot be transferred, ASMD will inform the caller that he/she will be called back, the supervisor should not delay calling back the caller and should be prepared to deal with the situation).
 - 4.3.8 Call Termination Process (Supervisor):
 - 4.3.8.1 In situations where the caller remains abusive after speaking with a supervisor (and the Supervisor finds this intolerable) the caller is to be

advised that his/her behavior is not acceptable and provide 3 warnings that the call may be disconnected if the abuse will continue.

4.3.8.2 If the abuse continues and it is intolerable the Supervisor is to advise "Help is being arranged, I am hanging up now, if anything changes call us back.

4.3.8.3 The Supervisor who terminates the call should enter a comment in CAD saying that the caller was abusive towards the ASMD.

4.3.9 Follow up and Reporting Process:

4.3.9.1 Supervisors/Oscar should immediately debrief abusive caller incidents with the concerned ASMD as soon as practicable, check that no further support is required.

4.3.9.2 Supervisors are required to escalate to the Quality unit any call with Abuse to review the incident and provide any further support/coaching that may be identified.

4.3.9.3 In cases where the supervisor believes further action is required, they should escalate this to the management and document the incident through the OVA process.

4.4 **Providing Pre-Arrival Instructions:**

4.4.1 The ASMD will never ask permission to give instructions (i.e. Do not say "*Would you like me to tell you how to do CPR?*" or "*I can give you CPR instructions if you want to try.*"). The best way to handle reluctant callers is to simply start giving the instructions. Most callers expect to take direction from the trained ASMD on the other end of the phone line.

4.4.2 If the caller refuses to follow PAIs, and only if there is no safety concerns will say, the ASMD:

4.4.2.1 "*The ambulance is on its way, but this is important in order to give the patient the best possible chance/care until professional help arrives.*" Repeat as necessary.

4.4.2.2 *If the caller still refuses to administer aid, the ASMD will ask if there is someone else there to speak to; if so, the ASMD will attempt instructions again with the new caller.*

4.5 **Third Party Calls:**

4.5.1 The ASMD will not assume that third party callers know nothing, even if they say they know nothing. 3rd party caller can provide some valuable information about the call.

4.5.2 The ASMD will always attempt to ask all Case Entry and Key Questions of third-party callers.

4.5.3 Whenever possible, the ASMD will try to convert a 3rd party caller to a 2nd party caller by asking them if he/she can go to the patient to render aid. If they agree, the ASMD will give PDIs and PAIs.

4.5.4 Once the ASMD has made several attempts to gather information without success, and only after finishing ProQA interrogation and identifying the final coding he/she may choose to terminate the call, once it has been determined that the caller has no further information and cannot or will not get close to the patient or the incident.

5. QUALITY ASSURANCE AND CALL MONITORING:

5.1 To ensure compliance with the established Customer Service procedures within NCC & HCC, a sample size from the total calls (emergency and non-emergency) will be subjected to random quality reviews.

5.2 The below ten (10) key HCC specific customer service metrics will be measured for evaluation:

Key Metrics	Competencies
1. Opening the call	Call taker used the standard greeting script Call taker identified her/himself to the caller
2. Obtaining and confirming information	Asked for the caller's name Asked for Caller's corporation number Asked for Caller's telephone number Asked for patient Health Card number Asked for patient's name Asked for patient's exact location Asked for patient's destination Asked about the reason for transport/transfer Asked about the requested priority/acute level Captured the caller's information
3. Using the Hold guidelines	Calltaker used the Hold/Mute button when necessary Calltaker informed the caller before placing him/her on hold and explained the reason. The hold duration was less than or equal to 1 minute (when possible and applicable) Calltaker kept in touch during long hold Calltaker thanked the customer for holding
4. Transferring Calls	Calltaker asked valid questions to ensure the call is transferred to the correct party when appropriate Calltaker informed the caller where & why they were transferred to Calltaker transferred the call to the right department/section when appropriate Calltaker mentioned his/her facility name when receiving a transferred call
5. Politeness, courtesy and listening	Calltaker sounded courteous & respectful throughout the call Calltaker used "please" when appropriate Calltaker used "thank you" when appropriate Calltaker apologized using "Sorry for the inconvenience" or other statements when appropriate Calltaker showed empathy using "I understand your concern and would be glad to assist you to resolve it" or similar statements when appropriate Calltaker asked if caller wanted help with anything else, when applicable Calltaker used the caller's name throughout the call & used the right prefix Dr./Mr./Ms./Mrs. or Sir/Madam (when applicable) Did not interrupt the caller Did not rush the call
6. Interest shown while handling the call	The Calltaker maintained proper tone, pitch, volume, and pace throughout the call. Positively answering all queries with confidence when appropriate

- Used paraphrasing/summarizing to confirm understanding
7. Conversation skills Avoided usage of “jargon” (technical/medical terms), abbreviations, or acronyms.
Calltaker was in control of the conversation.
Calltaker was professional with correct use of language either in English or Arabic.
(in wordings, in phrases, or greetings)
Refrained from turning the conversations personal with the caller
The way of handling the call—processing and closing—is within a suitable time frame.
8. Escalation Tried once or twice (based on the caller's tolerance) to address the caller's issue.
Procedure Escalated the call on caller's insistence without upsetting him.
9. Closing the call Calltaker used the “send-off” script
10. Prohibited Calltaker employed a statement that fostered/created feelings of anger, guilt, or panic
Behaviors in the caller.
Calltaker yelled at the caller.
Calltaker used derogatory, offensive, demeaning, or discriminatory language or tone
of voice.
Calltaker showed disrespect for the caller.
Calltaker used threatening statements
Calltaker made any sound that may be interpreted as offensive or derogatory such as
laughing, chuckling, sighs of frustration, etc.
Calltaker blamed the caller for any of his/her actions
Calltaker disconnected call without sufficient reason

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.21 HCC CALL ROUTING
Implementation Date	09 March 2020
Purpose	To direct the routing of calls for the Transfer and Retrieval Services
Responsibility	ALL COMMUNICATIONS STAFF

1. HMC Ambulance Service (HMCAS) national Healthcare Coordination Centre (HCC), which provides call center and control room operations for the Scheduled Ambulance Service (SAS) and Ambulance Service (AS) Critical Care Services (CCS).
 - 1.1 SAS is accessed by patients, family members, care givers and providers through 16099 or 974 443- 98555.
 - 1.2 CCS is access by practitioners at 974 443-98833
 - 1.3 All phone lines as well as the chat/broadcast functions are recorded for quality assurance and improvement purposes.
 - 1.4 Extensions numbers should not be given to anyone EXCEPT;
 - 1.4.1 MIR 'on call staff' SILVER or GOLD category,
 - 1.4.2 HCC staff,
 - 1.4.3 NCC DTLs/PTLs to reach HCC UTLs,
 - 1.5 If an NCC EMD staff need to transfer a caller to HCC, they must use 16099 or 4439 8555 and navigate through the phone system.
 - 1.5.1 NCC is NOT allowed to transfer these types of calls to direct extensions.
2. **Phone System Essentials:**
 - 2.1 The designated staff should be at the right desk/room at all times according to the desk/room assignments and the pre-assigned role of each individual staff.
 - 2.2 Staff are expected to log in to the CISCO agent (or supervisor if applicable) at the beginning of the shift and log out at the end of the shift.
 - 2.2.1 If going on break or a temporary absence, staff will use the NOT READY statuses and select the most relevant reason with notification to the shift UTL.
 - 2.2.2 Staff should not have to log out to switch users (change desks for dispatcher rotation).
 - 2.2.3 Staff are expected to regularly monitor the dashboard to monitor their readiness and see potential calls waiting on the queue they are assigned to.
 - 2.2.3.1 Supervisors (Oscar, UTL, CC) will have the ability to monitor ALL queues to see potential calls waiting on the queues.
 - 2.2.3.1.1 Staff working in Retrievals Desk, will need to monitor their queues as well.
 - 2.2.3.2 Supervisors (Oscar, UTL, CC) will have the ability to monitor their readiness.
 - 2.2.3.2.1 This is important to ensure that someone who is NOT READY has not left themselves in the status by mistake so they can answer calls presented in the ACD.
 - 2.2.4 Supervisors are authorized to make queries about readiness as well as update the NOT READY status on behalf of the staff so they can receive calls.

2.2.4.1 The status NOT READY “Call No answer” means that the agents phone logged out after the phone was not answered within 15 seconds of a call ringing to it, UTL have to follow up and take necessary action as needed.

2.2.4.2 The status NOT READY SUPERVISOR means the ‘supervisor’ initiated the status for the agent from the supervisor desktop.

2.3 Staff are expected to answer all calls within 15 seconds (1-2 rings), 90% of the time.

3. Agent queues:

3.1 Calls will be routed via the Automatic Call Distributor (ACD) to the agent which is not busy (on rotation) in accordance to what queues it is assigned to (attached).

3.2 Staff will be presented in the dashboard with the language and option the caller selected in the IVR (for example PTS _ARABIC or RET English).

3.2.1 It is important to pay attention to the caller’s selection when answering the phone.

3.2.2 The standard call opening should be in the same language selected by the caller which is displayed on Cisco Agent Desktop. (Refer to [SOP 3.22](#) for further details).

4. Misrouted calls:

4.1 If any staff receives a call that is not for them, for example the caller selected PTS when they meant to select national diabetic center hotline:

4.1.1 The staff will advise the caller they have reached the incorrect section and that they will assist them in reaching the correct section and to please hold the line.

4.1.2 The staff will then transfer the call back into the IVR and navigate the options to get the caller where they need to go, and then release the line.

4.1.3 Incorrect pathway calls should not be transferred to the agent extensions directly.

4.2 Notify the shift supervisor when misrouted calls are received.

4.2.1 Callers, who are misusing the system by not accurately navigating it, for example pressing 1 thereby entering the Arabic queue when they do not speak Arabic, should be reported.

4.2.2 The high priority calls (IFT/PHCC) might be routed to any available agent (CT) regardless the selected language whenever the selected language /agents are busy and here the CT should process the call without any delay.

4.3 On occasion, the Retrievals Desk may receive IFT and CCS calls.

4.3.1 If someone on the Retrievals Desk received an IFT call, it should be processed according to [SOP 3.22](#) (HCC Call Taking).

4.3.2 If someone on the Retrievals Desk received an CCS call, unless otherwise described in a call handling process, the staff will consult the CC Retrieval to see who can best handle the call and then transfer it to their extension.



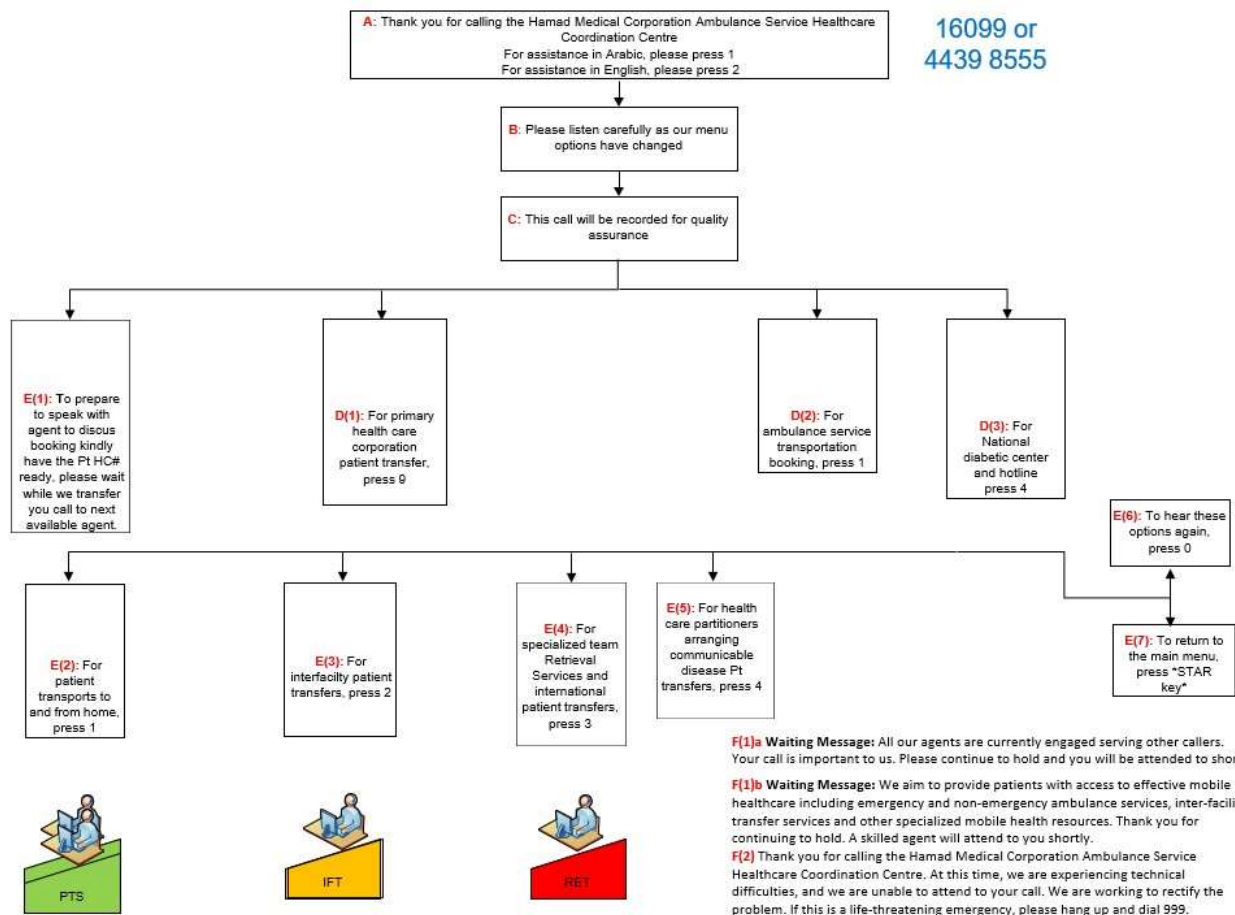
English and Arabic	28218
All extensions able to receive Arabic and English calls (the IVR relay on the staff users name)	28217
	28216
	28215
	28214
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	28211
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	28204
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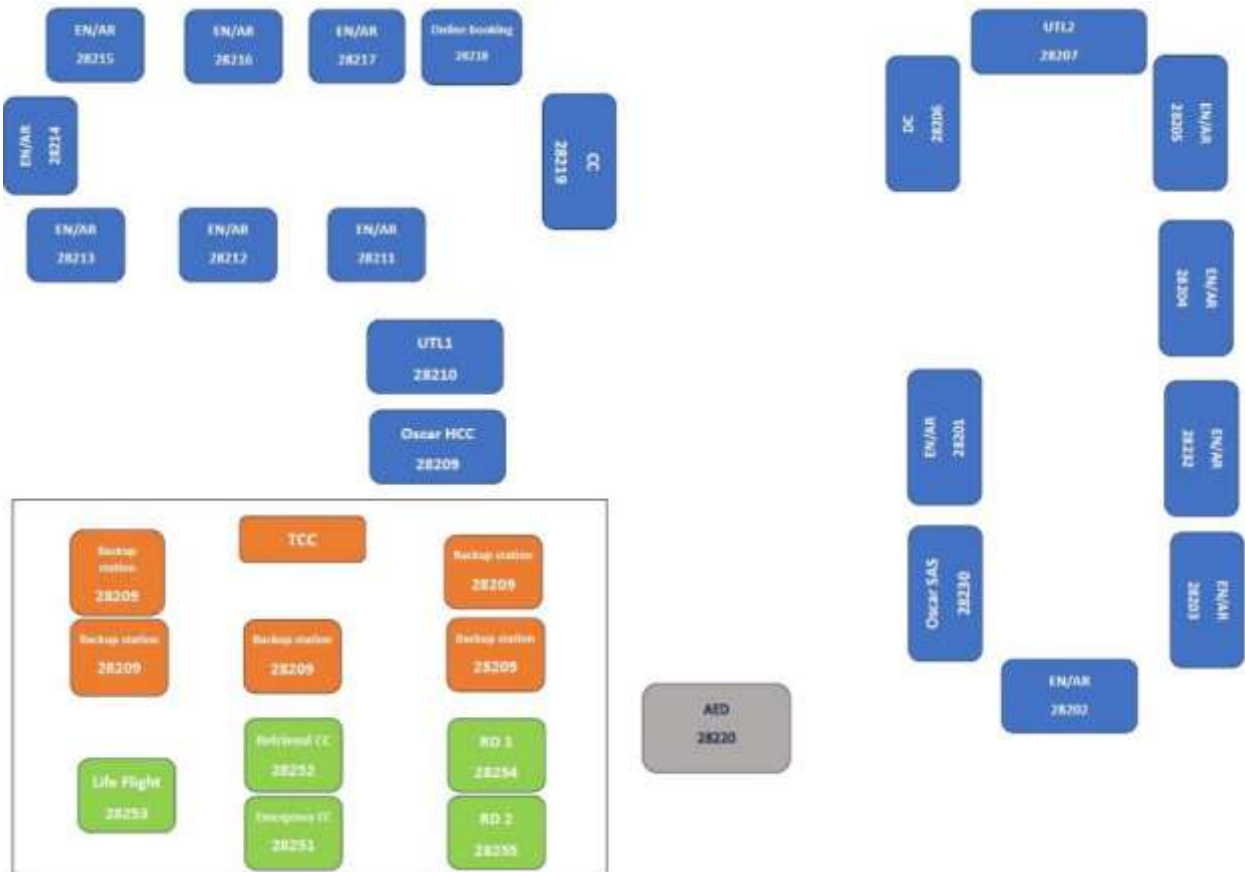


English and Arabic	28218
All extensions able to receive Arabic and English calls (the IVR relay on the staff users name)	28217
	28216
	28215
	28214
	28213
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	28211
	28201
	28202
	28203
	28204
	28232
	28205
	28206
	28207



English and Arabic	28251
All extensions able to receive Arabic and English calls (the IVR relay on the staff users name)	28252
	28253
	28254
	28255





Call Routing for Ambulance Service Healthcare Coordination Centre

1	A 16099/974 4439 8555 call is placed by an end user.
2	The caller reaches an Interactive Voice Response (IVR) which greets the caller in Arabic and English advising them to select number 1 for Arabic language and number 2 for English language.
3	The caller will choose one of the services by pressing respected number as follow: • For Primary Healthcare Cooperation patient transfer press 9. • For Ambulance service transportation booking press 1. • For National diabetic and hotline press 4. • To repeat this information press 0.
4	When the caller presses 1 for Ambulance service transportation bookings he will have the following options: • For patient transport to/from home press 1. • For interfacility patient transfer press 2. • For specialized team retrieval services and international patient transfer press 3. • For healthcare practitioners arranging communicable disease patient transfer press 4. • To repeat this information press 0. • To go to main menu press*.
5	The HMC AS call taker receives the call and obtains & confirms the patients demographics including HC number as well as caller name and number and the location and destination of the patient to process a booking query or request: a. Calls will be processed in the Computer Aided Dispatch system (CAD) by section (PTS, IFT RET) and use ProQA whenever applicable. b. After termination of the call, the Ambulance Service Medical Dispatcher or case controller may call back the caller at any time to gather more information about the patient if seemed to be necessary. c. Anytime the line disconnects or the caller hung-up before the termination of the call, the Ambulance Service operator must call back the caller.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.22 HCC CALL TAKING
Implementation Date	13 April 2021
Purpose	To ensure effective and efficient receipt and handling of all the calls within the HMCAS Communications Center - Healthcare Coordination Center (HCC).
Responsibility	ALL COMMUNICATIONS STAFF (SPECIFICALLY CALL TAKERS)

1. General Call Taking Standards:

- 1.1 Answer all the calls arriving at the Ambulance Service consoles within 1 to 2 rings.
- 1.2 Answer all the calls coming to HCC with the phrase **“Ambulance Service, this is _____ (staff name) speaking. How may I help you?”**
- 1.3 For emergency calls received by HCC lines, advise the caller to call back on 999. **Do not transfer the call to the NCC hotline.**
- 1.4 Obtain the following minimum basic information from the caller:
 - 1.4.1 The address or exact location of the incident
 - 1.4.2 The caller name and call back number
 - 1.4.3 The type of call (Emergency, PTS, IFT, RET)
 - 1.4.4 Chief complaint/reason for transfer
 - 1.4.4.1 As needed for IFT or RET, ask if the case has been coordinated with the receiving facility.
 - 1.4.5 Complete all necessary information required for the Computer Aided Dispatch (CAD) for regular/ routine calls which may include:
 - 1.4.5.1 Caller name and contact number
 - 1.4.5.2 Name, age, gender, nationality, and HC number of the patient.
 - 1.4.5.2.1 By entering the HC number in the CAD, the rest of the patient details appear automatically. Therefore, the call taker shall verify the name and the age additional to the HC number.
 - 1.4.5.3 Weight Actual or estimated (inclusive of body mass).
 - 1.4.5.4 The exact location/facility where the patient will be picked up includes floor/unit, room, and bed.
 - 1.4.5.4.1 If the transport is from a residence, determine/document if there are any stairs or other obstacles present that prevent access to the residence that may require special equipment or additional manpower.
 - 1.4.5.5 Destination where the patient is going exactly, for example floor/unit/ward, room, and bed.
 - 1.4.5.5.1 If the transport is to a residence, determine/document if there are any stairs or other obstacles present to gain access to the residence that may require special equipment or additional manpower.

- 1.4.5.6 As needed for IFT or RET, obtain receiving facility contact number and contact person.
 - 1.4.5.6.1 This can be used to verify transfer details and confirm acceptance of the patient.
- 1.4.5.7 What is the patient diagnosis and pertinent medical history?
 - 1.4.5.7.1 Although the HCC CAD utilizes the 'chief complaint' field to capture the reason for transfer, the patient diagnosis and clinical condition should be documented in 'special considerations' as to the reason the patient has to go by ambulance.
- 1.4.5.8 Medical needs and transport requirements (may include O2/suction/IV therapy, or ambulatory requirements based on patient request consistent with 1.4.5.4.1 and/or 1.4.5.3, etc.)
- 1.4.5.9 Call taker shall ask mobility questionnaire whenever applicable.
- 1.4.5.10 Will additional personnel be accompanying the patient?
- 1.4.5.11 Does the patient have any communicable disease?
- 1.4.6 Call in take is not complete unless all fields in CAD are filled with responses to questions from (points 1.4.5.1 thru 1.4.5.10).
 - 1.4.6.1 If the answer to the question is not applicable, the call taker will select NONE in the applicable CAD field.
- 1.4.7 For emergency calls, as to not delay Ambulance dispatch, limited key information will be gathered (points 1.4.1 thru 1.4.5.2 refer to point 9 detailed steps).
- 1.4.8 For booked cases;
 - 1.4.8.1 COLLECTION TIME - means the time the ambulance crew needs to "Collect" the patient from Home/Facility.
 - 1.4.8.2 APPOINTMENT TIME - means the time the patient is expected to see the Doctor, or rather begin their procedure/treatment/assessment.
- 1.5 Consider calls reported by HMCAS clinical or administrative staff via the HCC line to be routine calls and be process the same way as other calls.
- 1.6 Report any unusual call/demand to the shift supervisor immediately.

2. HCC Call taking Process (Step by Step):

Once the call comes into the screen, immediately answer the call within 1-2 rings.

The call taker will be opening CAD call taker board which related to the section assigned (PTS, IFT, RET).

Identify and verify the exact location and destination of call (zone, street, building number), telephone number and document it accurately in CAD.

Check the availability of booking by reviewing the hourly cases for PTS/IFT scheduled calls.

Use ProQA to process the questioning and identify the ProQA Chief Complaint and the Dispatch priority for IFT cases. Also obtain patient diagnosis and clinical condition.

For Scheduled Ambulance Requests advice the caller to fill and send patient transportation request form Via Email or Through online booking request form. If the nurse requesting immediate transfer for a Scheduled ambulance request (patient going to same or low level of care facility) use

ProQA to triage the case and escalate the request to shift supervisor/Oscar or clinical desk for further decision.

Terminate the call once all the information obtained and the booking time confirmed.

Fill in all fields in CAD.

3. Address identification & verification:

- 3.1 Verify all addresses reported by asking the caller to repeat the address once again.
- 3.2 If exact location is unknown/unclear, notify the UTL/shift supervisor immediately and do the following:
 - 3.2.1 Advise the caller to find the Blue Plate - Qatar Address Reference System (QARS) in the entrance of the building and provide the 3 consecutive plate numbers (zone, street and building number).
 - 3.2.2 When the QARS number is not available, advise the caller to describe the directions to get the ambulance to his location.
 - 3.2.3 If necessary, request an alternative contact number of someone able to guide the ambulance to the location.
 - 3.2.4 If all attempts fail, advise the caller to send someone to rendezvous with the ambulance at a mutually known location.

4. Taking non-standard calls:

- 4.1 If a call disconnects/is dropped before it can be determined if assistance is needed, attempt a call back twice. Make additional attempts to contact the caller if there are no other calls in the queue, escalate the Issue to UTL/Oscar on duty.
- 4.2 Prank calls: Treat as a real emergency until proven otherwise. Call back the called party and query to determine if further action or an emergency response is required.

5. New calls reported via radio by on-duty HMC Ambulance Personnel:

- 5.1 The dispatcher of the channel will need to verify if this is an emergency call request or a transport request.
- 5.2 For Non-Emergency Transport request, dispatcher will need to verify if the call is not booked in the CAD system.
- 5.3 Ask the call taker to open a new request at CAD.
- 5.4 If it's an emergency case, take basic information and notify the UTL/shift supervisor.
- 5.5 Instruct the unit to change to respective emergency radio channels (Channels) and report the incident.
- 5.6 Shift supervisor/Operation officer will need to call NCC operation officer desk and report the incident. (Refer to [SOP 3.14](#) for more details).
- 5.7 Extra resources will be sent as per the request of the clinical crew on-scene.

6. Foreign Language calls:

- 6.1 Process the call as per normal
- 6.2 Alert the Case Controller and UTL on duty

- 6.3 Attempt to identify the language of the caller by asking what country they are from or interpretation of their response to various language options.
 - 6.4 Attempt to obtain the address by use of landmarks, shops, or schools in the area.
 - 6.5 The Case Controller will determine if any HMCAS staff on duty is able to speak the caller's language and transfer the call to the appropriate line extension accordingly.
 - 6.6 UTL or Case Controller can contact NCC (National Command Center) to obtain more information and get assistance.
7. **Redundant Calls:** (When several calls have been received on the same call);
- 7.1 Confirm that patient HC number, location and incident is the same as an existing CAD case record.
 - 7.2 Update the CAD with any changes and additional information and inform the dispatcher of any changes.
 - 7.3 The call taker must reassure the caller that an ambulance has already been dispatched to the call or the case already booked and updated and disconnect the call.
8. **Inter-Facility Patient Transfers:**
- 8.1 Diagnostic, procedures, assessments, and consultations as well as Airport cases and other prescheduled appointments shall be treated as routine HCC calls.
 - 8.2 For IFT calls that require ProQA in accordance with [Appendix 3.22](#), call taker should use ProQA after taking basic information.
 - 8.3 **Basic Information** includes the caller details, patient details, patient pick up and patient destination.
 - 8.4 If unable to understand medical terminology, then the call must be transferred to another Call Taker capable of handling the call notify the UTL or Case Controller.
 - 8.5 Once information obtained from the caller and the case confirmed is not a routine IFT call, and requires emergency intervention, escalate as necessary to the Clinical Desk for clinical review and triage.
 - 8.5.1 If the case is referred to by another hospital (not where the patient is located), the call taker will contact the sending facility to verify the transport details.
 - 8.5.2 For example, Sidra Medicine calls HCC and needs a patient moved from PEC Al Sadd to Sidra. After completing the interrogation, the call taker will contact PEC Al Sadd to confirm.
 - 8.6 Notify the UTL/shift supervisor
 - 8.7 If the nurse requesting immediate transfer for a Routine or scheduled ambulance call (patient going to same or low level of care facility) use ProQA to triage the case and escalate the request to shift supervisor/Oscar or clinical desk for further decision.
 - 8.8 Call taker must book the case in the CAD and not refer the caller to do online booking.
9. **HMC Immediate transfer/PHCC calls:**
- 9.1 For PHCC calls once call taker obtains exact location immediately shall notify case controller/UTL to start initial unit dispatch. This is a mandatory step for both P1 and P2 calls.
 - 9.1.1 **The call taker shall ensure the followings:**
 - 9.1.1.1 The call taker shall continue gathering patient information (HC Number) confirming patient name and age then do **Partial registration**.

- 9.1.1.2 If a call appears to be an extreme emergency, or the caller stated that the patient is not yet admitted or particularly PHCC cannot handle this type of emergency the call taker shall immediately notify the case controller or UTL.
- 9.1.1.3 Process the interrogation without delay. If the patient's HC number is unknown by the caller or not available for any valid reason call taker can type "00000000" and add patient info manually and if the caller is not able to answer any question, then document as unknown and do Partial Registration.
- 9.1.1.4 After obtaining the minimum information the call taker should reassure the caller that resources are coordinated for dispatch immediately.
- 9.1.1.5 Continue the interrogation through CAD and ProQA and update the Dispatch controller and UTL.

9.1.2 Once notified the Dispatcher shall ensure the followings:

- 9.1.2.1 If SAS unit can be deployed, the dispatcher will immediately dispatch the crew via the radio towards the PHCC and documentation in CAD can be added when the CAD is available to the dispatcher.
- 9.1.2.2 If a SAS unit is not available, the dispatcher shall immediately notify the UTL who shall start coordinating a resource from EAS by calling NCC DTL. UTL shall request for the closest unit to PHCC to change to the tango channel for a P1 or P2 call. Refer to [SOP 3.14](#) for more details.
- 9.1.2.3 Dispatch controller and UTL shall closely follow up all PHCC calls/any immediate transfer from the receipt time to on scene time and shall mitigate any response time delay.

9.2 HMC Immediate Transfer

9.3 The call taker shall do partial registration at any time of interrogation if the case is going to ICU or has a critical complaint. (P1 obvious cases, code blue) otherwise the case should be processed as normal IFT and process the call in ProQA until final code.

10. Online Booking Requests:

- 10.1 The dedicated call taker for online booking must frequently check the online booking dashboard for any pending requests either by monitoring the pop-up messages or refreshing the search option continuously.
- 10.2 Upon receiving a new request, the call taker must process it immediately by either acceptance or rejection.
- 10.3 The call taker shall accept the booking request whenever the information is complete, and a slot is available and there is no need to call the facility (unless required for the exceptional situation only) also the call taker should not expect a confirmation call from the facility.
- 10.4 The Call taker shall reject any request with incomplete/unclear information or whenever the requested slot is not available. It's mandatory to add a comment about the rejection reason to justify the rejection. Example: mention what exact info is missing or propose a new slot based on the availability.
- 10.5 Case Controller should double check the dashboard frequently for requests that need to be actioned and to inquire/coordinate with the call taker accordingly.
- 10.6 The Call taker/case controller may have to consult the Clinical desk for some special cases for further screening if needed.

- 10.7 If the nurse requesting immediate transfer for a Routine or scheduled ambulance call (patient going to same or low level of care facility) escalate the request to shift supervisor/Oscar or clinical desk for further decision or if advised call the facility and triage the case by using ProQA.
- 10.8 Call taker must book the case in the CAD and not refer the caller to do online booking.

11. Email Booking Requests:

- 11.1 Call taker must check the shared mailbox (AmbulanceBooking@hamad.qa) frequently throughout the shift for any new email request for ambulance booking.
- 11.2 Upon receiving a new email-booking request call taker shall validate the request and process booking the request in CAD with No delay based on slot availability.
- 11.3 If the request is accepted, the call taker shall book the case accordingly and attach the booking request to the relevant CAD number and doesn't need to call back unless required.
- 11.4 Once the call taker processes the email request, he/she should reply back to the sender within 15 mins as acknowledgement or acceptance of the request or otherwise to reply back as rejection or inquiring further about any missing/unclear info and if requested time slot is full, reject the booking and propose a new time.
- 11.5 When the booking is completed, 'mark /flag the email as complete so that staff will know that the booking is complete and avoid any duplication.
- 11.6 In case another Call taker received a follow up call and processed the request he/she must immediately inform the dedicated email-booking person as well as to notify the case controller or UTL to avoid any duplication.
- 11.7 The call taker may refer/consult the case to clinical desk whenever needed. Case Controller should double check the shared email inbox frequently during the shift for requests that need to be actioned and to inquire/coordinate with the call taker accordingly.
- 11.8 If a request seemed to be an emergency transfer, the call taker must immediately notify the UTL or the Oscar or clinical desk or if advised call the facility and triage the case by using ProQA.
- 11.9 Call taker must book the case in the CAD

12. MTPS Protocols 45-46-47:

- 12.1 All calls for ambulance requests must be processed in ProQA based on the below definition for each protocol:
 - 12.1.1 **Protocol 45:** Specialized Unscheduled Up-Care Transports; Used when a patient is transported to a higher, more acute level of care.
 - 12.1.2 **Protocol 46:** Scheduled Interfacility Transfers (Routine): Used when a patient is routinely transported to another facility with a similar level of care.
 - 12.1.3 **Protocol 47:** Mental Health Transfer: Used when a patient with diagnosis or symptoms of mental illness is transported to a medical or support facility.
- 12.2 **Case Exempted from ProQA:** Ambulance request received via booking, emails and return trips (unless there is an upgrade in care or request by nurse for immediate transport which call taker must process the call in ProQA and notify immediate supervisor or clinical desk).

Guide for Protocol Use

Protocol Number & Title	Definitions/Explanations
Protocol (45): Specialized Unscheduled Up-Care Transports <i>Initial dispatch required</i>	• Transports to a higher level of care for evaluation or stabilization of an acute or chronic problem. • Use to offer a response level based on a joint medical professional and EMD evaluation of the patient's medical condition and transport need. • Example: A patient transfer from HGH-ED to HH-Cath lab, AKH to HGH or HH ... or immediate urgent procedure cases involving PICUs, CICUs, NICUs, PECs and SIDRA.
Protocol (46): Scheduled Inter-facility Transfers (Routine)	• The patient transport is to or from a medically supervised environment for routine treatment, procedure, checkup, palliative care, or relocation. Continuity of current care may be required. • Use to facilitate the relocation of a patient based on the level of transport care needs and the time constraints required for scheduling. • The time of scheduling are divided into: ✓ Immediate: a request that requires transfer within 1 hours from the time of booking. ✓ Scheduled: a request that requires transfer within 24 hours from the time of booking. ✓ Pre-Scheduled: a request that requires transfer after 24 hours from the time of booking. • Example: A patient transfer for routine appointment; routine diagnostic procedures; Ward to Ward; ICU to a ward.
Protocol (47): Mental Health Transfers	• Transport of patient with diagnosis or symptoms of mental illness for medical clearance, mental health evaluation, admission, routine appointment, or relocation. • Use to facilitate the transfer of a mental health patient when requested by medical care/psychiatric facility or public safety officer. • The time of scheduling are divided into: ✓ Immediate: a request that requires transfer within 1 hours from the time of booking. ✓ Scheduled: a request that requires transfer within 24 hours from the time of booking. ✓ Pre-Scheduled: a request that requires transfer after 24 hours from the time of booking. • Transfers to the Mental Health Hospital.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.23 HCC CALL DISPATCHING
Implementation Date	02 June 2019
Purpose	To standardize the HMCAS Communication Center Dispatch Procedure for cases received by HCC.
Responsibility	ALL COMMUNICATIONS STAFF, SPECIFICALLY CALL DISPATCHERS

1. General rules for Dispatch:

- 1.1 The dispatcher receiving shall use the CAD dispatcher board for the assigned section (PTS, IFT, RET).
- 1.2 PTS dispatcher needs to review all the cases and bookings to plan the unit calls based on the locations and special requirements.
- 1.3 Obtain additional information and relay to responding units whenever needed.
- 1.4 Acknowledge the total number of units and the type of ambulances at the beginning of dispatching.
- 1.5 One dispatcher is assigned to monitor each operational channel.
- 1.6 All units responding to cases shall be tracked in CAD, assigned to an actual incident:

2. Dispatch Priority:

- 2.1 Units to be dispatched in accordance with the Dispatch Matrix.
- 2.2 In particular for all PHCC or whenever applicable for IFT immediate transfer obvious P1 cases the dispatcher shall initially dispatch the response unit as P1 based on the initial information (Partial resignation).
- 2.3 Dispatcher to follow up and should advise the response unit to downgrade the response or continue as P1 based on the final ProQA response code.

3. Role of the Dispatcher:

- 3.1 Monitor and continuously observe the screen for pending list.
- 3.2 Coordinate with the Case Controller or shift supervisor to make sure that calls are dispatched based on the criteria.
- 3.3 Review CAD maps to visualize the case's location.
- 3.4 Open the unit status screen to visualize ALL available HMCAS units close to the cases location.
- 3.5 Assign the CLOSEST unit to the case and monitor the timing and the collection time. If a special unit is needed and is on a different radio channel, coordinate with the relevant Dispatcher to instruct the unit to switch radio channels.
- 3.6 Initiate the dispatch by calling the unit Call sign twice e.g. "Calling Tango Alpha One, One – Tango Alpha One, One". On reply, advise the unit that they have a call and to check the MDT for more details. If MDT not available, the unit to be guided by Radio with the case details.
- 3.7 Simultaneously update the unit status in CAD accurately and monitor the accuracy of MDT user updates (when the MDT is available).
- 3.8 Reply on all the requests popping up on dispatching board and update the status accordingly (stood down, breaks or unit unavailability).

- 3.9 Must be fully aware with the unit assigned in fixed location as starting point and should assigned to the nearest patient in their area.
- 3.10 Should closely monitor the location and movements of the units.
- 3.11 Must coordinate with duty UTL to account the different type of ambulance available to be dispatch accordingly to needs of the patient.
- 3.12 Document important information, incident and event that transcribe during the call and logged in the CAD Call Notes.

4. Dispatcher Information Sharing:

- 4.1 Necessary information to be given to the responding units should include but not be limited to the following:
 - 4.1.1 All details of the address (Zone name/number, Street name/number, building name/number including the floor and flat number if applicable).
 - 4.1.2 Landmarks near the address e.g. names of Roundabouts and buildings.
 - 4.1.3 Additional address information specific to special situation and explicitly said by the caller should be documented in the Call record and relayed to the responding crew e.g. 'patient is in the annexure behind the villa...'
- 4.2 Update the unit with further information about the location and the patient condition as soon as it is received and updated in CAD case record. This should include:
 - 4.2.1 Additional details of the location if required
 - 4.2.2 Patient status change
 - 4.2.3 Patient information or details
 - 4.2.4 All available safety information, including the presence/risk of infection (e.g. communicable diseases).
 - 4.2.5 Guide the responding unit to the identified location through the shortest and quickest route or by setting a rendezvous point with the caller or with other responding agencies.

5. Resource allocation:

- 5.1 Assign and deploy extra resources as necessary (in consultation with shift supervisor) in cases of:
 - 5.1.1 More than one patient found on scene
 - 5.1.2 Special requirement or consideration
 - 5.1.3 Safety concerns/high risk situations
- 5.2 If the first assigned unit is unavailable or didn't answer the radio call within 30sec:
 - 5.2.1 Call the unit again by radio
 - 5.2.2 Notify the UTL/shift supervisor
 - 5.2.3 Contact the unit using other alternative methods; call by phone or mobile.
 - 5.2.4 The communications supervisor shall notify the operations supervisor of the delayed unit and make necessary notification and follow up.

6. Follow up with responding units:

- 6.1 Follow up the location and status of the responding unit until arrival on scene.

- 6.2 Using the “comments” free text area of the call record, document all information and events during response to the scene.
- 6.3 Accurately capture the reported “On Scene” time (or monitor accuracy of MDT user unit status capture when applicable).
- 6.4 Follow up the status of the unit on scene
- 6.5 Document all reported information or events in the CAD.
- 6.6 **Upon arrival at the receiving hospital/facility or the residence:**
 - 6.6.1 Log the reported time in the CAD Call record
 - 6.6.2 Follow up the status of the unit at the facility or residence.
 - 6.6.3 Notify the UTL and the operations supervisor for units staying more than 15 minutes and/or not answering radio.
 - 6.6.4 Document any applicable information in the call record e.g. the reason for delay.
 - 6.6.5 Review the documentation in the call record, update and complete it accurately.
 - 6.6.6 For units returning same patient back, dispatcher informs call taker to make the return case to be assigned for the same responding unit.
- 6.7 **When the unit reports availability:**
 - 6.7.1 Log the reported time in the call record
 - 6.7.2 Log all administrative data
 - 6.7.3 Close the call record of the incident
 - 6.7.4 Assess the current standby coverage and immediately dispatch the available unit to standby point.

7. **Requests for HELP/ASSISTANCE from Primary Crew while on scene:**

- 7.1 When requests for Scene Safety assistance is made:
 - 7.1.1 Notify the Case Controller & UTL about the request as applicable, dispatch operation supervisor if needed.
 - 7.1.2 Notify the on-scene unit that the requested assistance is responding when appropriate, for example like MIR1 or other TRS unit.
 - 7.1.2.1 If the unit requesting assistance is at a facility and needs a CCP (Charlie), consider utilization of the Retrievals Charlie by coordinating with the TRS Clinical Coordinator.
 - 7.1.3 Have any secondary units switch to the channel of the primary unit needing assistance.
 - 7.1.4 Monitor in system any other unit near the drop location of patient for help.
- 7.2 In cases where help is not available, should coordinate with supervisor and unit inform unit requesting for help.
- 7.3 When **Special Scene or patient management assistance** is requested:
 - 7.3.1 Inform the UTL the need to activate the requested units e.g. ICU unit, MIR unit, Green Bus, 4X4 Ambulance or wheelchair unit.
 - 7.3.2 Dispatch the required resources to the incident.
 - 7.3.3 Have any secondary units switch to the channel of the primary unit needing assistance.

8. **Unit moving to hospital and/or residence needing help (patient condition change):**

- 8.1 Instruct the unit to change to Clinical 2 channel and consult if Charlie unit needed.
- 8.2 The closest available Charlie unit will be dispatched by NCC.
- 8.3 The unit will update with the latest patient's status and conditions.
- 8.4 Arrange for a rendezvous point and guide the dispatched unit.
- 8.5 Re-establish transport priority towards hospital after Charlie Unit rendezvous if HCC dispatcher handled the case not NCC dispatcher.
- 8.6 Inform UTL/shift supervisor.
- 8.7 Record all information in comments area of the call records. Information to be documented includes but not restricted to:
 - 8.7.1 The time when the first unit requested help
 - 8.7.2 Reason for requesting help
 - 8.7.3 Safety concerns
 - 8.7.4 Rendezvous time of both units

9. **Delayed Cases:**

- 9.1 All cases with a potential delayed response must be escalated to UTL/Dispatch controller.
- 9.2 UTL/Dispatch controller or delegatee must call the caller (patient/family/nurse) to inform them of the delay.
 - 9.2.1 A 'DELAYED RESPONSE' is defined as an expected arrival greater than the collection time.
- 9.3 This call should be made between 15-30 minutes at minimum prior to the collection time and documented in the case notes.
- 9.4 Advanced calling allows those with scheduled appointments the opportunity to cancel the ambulance BEFORE it arrives at the pick-up location.
- 9.5 When necessary and possible, HCC staff may provide the best 'Estimated Time of Arrival' to the caller (patient/family/nurse) in consultation/coordination with the Dispatch Controller or assigned crew.
 - 9.5.1 Understanding that this is an estimate, it is appropriate that HMC staff just simply advise that the ambulance will be there as soon as possible.
 - 9.5.2 Understanding that situations change, if the response is extended beyond the time initially described, additional calls will be made as needed to the caller (patient/family/nurse) by the UTL to keep them informed.
- 9.6 HCC staff will not make 'excuses' as to the delay. For example, citing staffing issues, mechanical issues, 'too busy', etc. UNLESS approved by the UTL.
- 9.7 As an example, at 09:30 as a part of the calling process the HCC staff can simply state,
 - 9.7.1 "Regarding the ambulance booking for 10:00... I apologize, but it looks like we are running behind schedule and the ambulance will arrive between 10:15-10:20, is that okay?"
- 9.8 If the booking has an appointment time, HCC staff can simply state,
 - 9.8.1 "Regarding the ambulance booking for 10:00... I apologize, but it looks like we are running behind schedule and the ambulance will arrive between 10:15-10:20. I

understand you have an appointment at 11:00. Do you know if the doctor/facility will see you if you arrive late? Based on travel time, we won't get there until 11:30."

- 9.9 For other non-urgent cases like discharges and returns, it may also be appropriate to state,
 - 9.9.1 "Regarding the ambulance booking for 10:00... I apologize, but it looks like we are running behind schedule. The ambulance is coming from "X" area and based on traffic conditions could arrive within the next 30-45 minutes, is that okay?"
- 9.10 HCC staff will always leave it in the hands of the caller (patient/family/nurse) to decide whether the ambulance will continue AS WELL AS coordinate the delay and acceptance with the doctor/facility.
 - 9.10.1 If IFT, the sending facility needs to communicate with the receiving facility to avoid any unnecessary travel.
- 9.11 In addition to documenting the delay, HCC staff will document the result:
 - 9.11.1 CALLER INFORMED OF DELAY, EXPECTED PICK UP NOW 10:20 AND THEY ARE OKAY.
 - 9.11.2 CALLER INFORMED OF DELAY, EXPECTED PICK UP NOW 10:20 AND THEY STATED THE DOCTOR WILL STILL SEE THEM IF THEY ARRIVE LATE, THEY ARE OKAY.
 - 9.11.3 CALLER INFORMED OF DELAY, EXPECTED PICK UP NOW 10:20 AND THEY STATED THEY WILL MISS THEIR APPOINTMENT, CANCEL.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.24 MANAGING OFFSHORE CALLS AT NCC
Implementation Date	13 March 2018
Purpose	To standardize response procedure for Offshore cases requiring a helicopter evacuation of a medical/trauma patient to the Emergency Department.
Responsibility	ALL COMMUNICATIONS (NCC) STAFF

1. Background and Purpose:

- 1.1 HMCAS regularly receive requests to transport medical patients from offshore areas including oil rigs, ships/boats and islands to hospital. Due to the limited access, these cases are serviced by LifeFlight; however, offshore operations impose additional safety risks on HMCAS staff and patients requiring unique dispatch procedures.
- 1.2 This SOP should be read together with [SOP 4.11.A.14](#) which sets out the procedures relating to the specific LifeFlight aircraft dispatch, coordination, and offshore operations.

2. Definitions:

- 2.1 **A7S** stands for “Alpha 7 Sierra” which is the disaster management control center operated by Qatar Energy and, among other responsibilities, is responsible for coordinating offshore medical evacuations.
- 2.2 **DTL** means Distribution Team Leader
- 2.3 **LFC** means LifeFlight Coordinator

3. Call taking and Dispatching – Calls originating from A7S:

- 3.1 A7S will call the HMCAS NCC hotline (40442555/40002055) to request helicopter medivac support.
- 3.2 The HMCAS call taker will receive the call and immediately notify the DTL of the request.
 - 3.2.1 DTL must immediately notify the LifeFlight Coordinator of the incoming A7S offshore request and add a note in the CFS of the notification.
 - 3.2.2 DTL should assist the call-taker as necessary and monitor the case progress.
 - 3.2.3 DTL must ensure that the appropriate “A7S Offshore Retrieval Checklist” is used as a guide on actions to be taken and it is completed appropriately for all A7S offshore cases. ([Appendix 3.24 NCC-LF Offshore Checklist](#)).
- 3.3 The HMCAS call taker will take the caller’s name and the address of the patient (*designated helideck name and helideck code*) from A7S and document this properly and accurately in NJM.
- 3.4 The HMCAS call taker will reconfirm the designated helideck name and code by repeating this back to the A7S caller.
- 3.5 The HMCAS call taker will identify and verify the telephone number of the caller.
- 3.6 The HMCAS call taker will advise the caller to stay online for more questions and route the NJM CFS for dispatch.
 - 3.6.1 While the call-taker continues with ProQA interrogation, the HMCAS dispatcher (responsible of that geographical area) will receive the case and assign LF to the CFS and update the LFC.

- 3.7 The HMCAS call taker will continue ProQA interrogation using Protocol 33 and confirm if there is any communicable disease or contamination with a hazardous substance by asking *“Is the patient suspected with a communicable disease or contamination by a hazardous substance?”*.
- 3.7.1 If the answer is **YES**, the HMCAS call taker must immediately inform the DTL and the LFC, then complete ProQA and provide PDI/PAI as applicable.
- 3.7.2 If the answer is **NO**, the HMCAS call taker will remain online with the caller and complete the ProQA and provide PDI and PAI as applicable.
- 3.8 Once the HMCAS call taker has completed call taking, and before closing the call, he/she should advise the A7S caller that the LFC will contact them shortly.
- 3.9 Once the HMCAS call-taker has closed the call with A7S they must immediately update the LFC and DTL, advising that they have finished with A7S and the case is handed over to the LFC. The call-taker must add a note in the CFS of the completion of call-taking and handover to LifeFlight Coordinator.

4. Call taking and Dispatching – Calls NOT originating from A7S:

- 4.1 Patients or bystanders might call 999 requesting ambulance assistance for calls located offshore.
- 4.2 The HMCAS call taker will receive the call and handle the case following [SOP 3.2](#) and [SOP 3.3](#).
- 4.2.1 If the patient is in a moving vessel, and an appropriate meeting point at a mainland port where an ambulance can be dispatched can be arranged, the call-taker shall assign the address pinpoint at this meeting point and should continue with ProQA interrogation.
- 4.2.1.1 If necessary and appropriate the call-taker may stay online with the caller for logistic coordination until an ambulance is with the caller.
- 4.2.1.2 The dispatcher must assign the nearest appropriate Alpha and, if applicable, Delta unit according to the ETA of the patient's arrival at the meeting point. If the patient's ETA exceeds 15-minutes, the Ambulance Service response should be priority 2.
- 4.2.2 If the patient is in a moving vessel, and an appropriate meeting point at a mainland port where an ambulance can be dispatched cannot be arranged, the call-taker must inform the DTL as in 4.3 below, add the coastguard to the CFS, and continue with ProQA interrogation providing all necessary PAIs.
- 4.2.2.1 Closure of the case can only be done after full endorsement of the case to the coastguard or the search and rescue team.
- 4.3 The HMCAS call taker must immediately update the DTL as soon as they become aware of the case being an offshore case.
- 4.3.1 The DTL must update the LFC of the case as soon as they are notified and advise them the case is not from A7S.
- 4.3.2 The LFC should immediately call A7S advising them of the incoming call and check whether they are aware of the case or not.
- 4.3.2.1 If the case is being coordinated by A7S, the LFC should advise A7S that we will be waiting on their confirmation and the Offshore Retrieval Request Form before activating LifeFlight. This must be clearly documented in NJM at the time of the call by the LFC/DTL.

4.3.2.2 If the case is unknown to A7S, the case should be handled following 4.2 above.

5. Notification Procedure:

- 5.1 In accordance with [SOP 4.11.A.14](#):
 - 5.1.1 The LFC will contact A7S when call taking is complete to verify further details and update them of LifeFlight dispatch and ETA as soon as possible.
 - 5.1.2 The LFC will notify the Consultant Critical Care Paramedic: Effectiveness and SME: LifeFlight as soon as possible.
- 5.2 LFC must update the DTL of LifeFlight status, ETA and keep them informed of mission progress.
- 5.3 DTL will keep Oscar NCC updated on case progress as necessary and appropriate.
- 5.4 Oscar NCC will ensure regular notification of the Management On-call if needed.
- 5.5 Oscar NCC will oversee the management of the case and, where necessary and in collaboration of LifeFlight and Clinical Desk management, provide feedback to the staff.
 - 5.5.1 If necessary the Oscar NCC, SME: LifeFlight or SME: Clinical Desk may conduct or request further review from QIU team.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service
Procedure	3.26 HMCAS INFORMATION SYSTEMS ON CALL
Implementation Date	13 March 2018
Purpose	To deliver 24/7 support for operations on the key operational Ambulance Service IT Systems.
Responsibility	ALL AMBULANCE SERVICE STAFF

1. Definitions:

- 1.1 **ePCR:** Electronic Patient Care Record
- 1.2 **CAD:** CareMonX computer Aided Dispatch System
- 1.3 **MDT:** CareMonX Mobile Data Terminal
- 1.4 **SMS:** Scheduling Management Systems
- 1.5 **FMS:** Fleet Management Systems
- 1.6 **Biometrics:** Time and Attendance
- 1.7 **OOH:** Out-of core hours 07:00 - 15:00 Sunday – Thursday, excluding HMC public holidays)

2. Procedure:

- 2.1 Any failure of an IT Device (ePCR/MDT/iPhone) or Ambulance Service software application (incident) identified by staff of Ambulance Operations, Fleet, Scheduling or HCC users must be reported to the on-duty hub/production/supervisor.
- 2.2 For both Hardware and Software, the hub/production/supervisor will raise an incident with HICT helpdesk either by phone or by email to HICT@hamad.qa, also copied to ASIS_Support@hamad.qa
 - 2.2.1 Email will auto generate a HICT helpdesk incident number.
- 2.3 Details to be provided are:
 - 2.3.1 Category – this describes the application with the fault
 - 2.3.1.1 ePCR Category is 'Ambulance Service – ePCR Application'
 - 2.3.1.2 MDT Category is 'Ambulance Service – MDT Application'
 - 2.3.1.3 Fleet Management Category is 'Ambulance Service – FMS Application'
 - 2.3.1.4 Scheduling System Category is 'Ambulance Service – SMS Application'
 - 2.3.1.5 Control Room CAD Category is 'Ambulance Service – CAD Application'
 - 2.3.2 Priority:
 - 2.3.2.1 Priority 1: Total failure for all users for the application.
 - 2.3.2.2 Priority 2: Failure of application key functionality affects more than 50 % of users.
 - 2.3.2.3 Priority 3: Single User affected, unable to work.
 - 2.3.3 Incident Description:
 - 2.3.3.1 Provide in as much detail as possible the details of the incident.
- 2.4 The staff member must complete an OVA form if there is a damaged device detailing the nature of the damage and the method by which the damage occurred.

- 2.4.1 The OVA number will be given to the Hub/Production supervisor.
- 2.5 For hardware issues:
 - 2.5.1 Staff will exchange the faulty device with the Hub/Production supervisor.
 - 2.5.2 Hub/Production supervisor will arrange to bring the defective unit to the IT delivery team (Ambulance HQ) with Helpdesk reference and OVA form during normal office hours (Sunday – Thursday 07:00- 15:00).
 - 2.5.3 The IT Delivery team log the incoming device and replace the defective unit with another device.
 - 2.5.4 This will be returned to the hub/production supervisor.

3. Notification:

- 3.1 The Staff member will then endorse the incident to the Hub/Production supervisor and the staff member will continue their duty.
- 3.2 HICT Helpdesk will verify the incident with assistance from HICT User Support and IT delivery team.
- 3.3 HICT User Support, HICT Helpdesk or the IT Delivery Team will call /email the Hub/Production supervisor to provide support for the resolution of the incident.
- 3.4 Hub/Production supervisor will inform Production Manager and Oscar on duty. Production manager will inform the IT delivery support number with the HICT ticket number for further assistance. Oscar will inform Silver Communications.
- 3.5 For escalation of the incident, the on-call Silver Communications will then escalate to the IT Delivery Team and HCS Executives.
- 3.6 For software issues that cannot be resolved with the IT Delivery team and/or the Local Service Engineer, HICT User Support will then raise the incident to Valentia who will be in contact with the Hub/Production supervisor until resolved.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	3.27 EMD CERTIFICATION
Implementation Date	25 February 2020
Purpose	To inform all Ambulance Service Medical Dispatchers (ASMD's) of the requirements for EMD certification and re-certification.
Responsibility	ALL COMMUNICATIONS STAFF (SPECIFICALLY CALL TAKERS AND DISPATCHERS)

1. Ambulance Service Medical Dispatchers Training PROCEDURE:**1.1 EMERGENCY MEDICAL DISPATCHER CERTIFICATION:**

- 1.1.1 All current and future personnel employed in the position of Ambulance Service Medical Dispatchers (ASMD) are required to obtain Emergency Medical Dispatcher (EMD) and the Emergency Telecommunication (ETC) Certifications with the International Academy of Emergency Dispatch (IAED).
- 1.1.2 Hamad Medical Corporation Emergency Services will provide the necessary training and re-training opportunities to facilitate obtaining and maintaining this certification.
- 1.1.3 In the event that an employee does not pass the certification examination on the first attempt, he/she will be provided with supportive training based on feedback received from the IAED. Student who does not pass the certification exams will then be invited to take the re-test.
- 1.1.4 Should the ASMD still be unsuccessful in passing the re-test, they will be invited to participate in a complete EMD course in the future. They may then take the certification examination and, if necessary, the re-test.

1.2 RECERTIFICATION:

- 1.2.1 ASMDs are required to maintain current EMD and ETC certifications. This requires completion of a minimum of 24 hours Continuing Dispatch Education (CDE) per two-year period, achieving a passing score in an open book EMD/ETC examinations at two-year intervals, and maintaining current CPR certification.
- 1.2.2 Hamad Medical Corporation Health Coordination Services Quality Improvement & Training section will provide all necessary opportunities for completion of the Continuing Dispatch Education requirement and CPR recertification. It will also maintain CDE records and a record of EMD/ETC certification status.
- 1.2.3 CDE Program Management:
 - 1.2.3.1 The Medical Dispatch Quality Review team shall be responsible for defining the topics that the CDE program will address based on the QI reports of previous month.
 - 1.2.3.2 Appropriate CDE topics may be identified in a number of ways:
 - 1.2.3.2.1 As a result of the Quality reviewers' recommendations
 - 1.2.3.2.2 Via requests from EMDs
 - 1.2.3.3 The Quality Improvement and training section for Communications shall be responsible for the following:
 - 1.2.3.3.1 Scheduling educational opportunities as necessary to address the needs identified above.

- 1.2.3.3.2 Ensuring that necessary educational opportunities are:
 - 1.2.3.3.2.1 Delivered by qualified personnel
 - 1.2.3.3.2.2 Adequate in their content/format to address the identified learning need/objective.
 - 1.2.3.3.2.3 Relevant to EMDs and their associated work
 - 1.2.3.3.2.4 Attended by all EMDs unless it is designed for specific role in the call centre like the case controller or the Supervisor.
- 1.2.3.3.3 Ensuring that appropriate records are maintained regarding the CDE program in the QIU filing system and for each EMD individually.
- 1.2.3.3.4 Ensuring that a CDE lesson plans are completed to an adequate standard for all classroom-based education.

1.2.4 Types Of CDE:

- 1.2.4.1 The minimum CDE requirement in any given year shall be 12 hours of completed CDE per EMD, at least eight hours of which shall be didactic in nature.
- 1.2.4.2 In addition to the CDE hours, types, and topics discussed above, each EMD must maintain current CPR certification.
- 1.2.4.3 The bulk of the subject matter accepted as fulfilling IAED requirements will be directly related to the science of Emergency Medical Dispatch and the use of the MPDS and ProQA software. However, other Ambulance Service-related material will be considered by the NAED for its educational relevance.
- 1.2.4.4 The following are acceptable formats and their associated maximum hours for CDE:
 - 1.2.4.4.1 Official Academy-Created Educational Products and Programs (no Maximum) (Journals, Advancement series, navigator...)
 - 1.2.4.4.2 Workshops and seminars (16 hours maximum)
 - 1.2.4.4.3 Attendance at planning and management meetings (8 hours maximum)
 - 1.2.4.4.4 Quality assurance and case review (8 hours maximum)
 - 1.2.4.4.5 Review of EMS related audio, video, and written materials (4 hours maximum)
 - 1.2.4.4.6 Teaching (4 hours maximum)
 - 1.2.4.4.7 MPDS protocol review (4 hours maximum)
 - 1.2.4.4.8 Miscellaneous, such as ride-along (4 hours maximum)

1.2.5 CDE Program Objectives:

- 1.2.5.1 Development of a better understanding of telecommunications and of the EMD's specific roles and responsibilities.
- 1.2.5.2 Improving skill in the use or application of all component parts of the MPDS, including interrogation and prioritization.

- 1.2.5.3 Providing opportunities for discussion practice of skills, and for constructive feedback of performance.

1.3 CERTIFICATION EXPIRATION, REVOCATION OR SUSPENSION:

- 1.3.1 Should an ASMD's EMD certification expires or become void due to suspension or revocation, the employee will be removed from EMD call-taking until such time as the EMD certification is reinstated or renewed.

1.4 ADDITIONAL TRAINING REQUIREMENTS:

- 1.4.1 All personnel employed in the position of ASMD (Call takers/Dispatcher) are required to obtain the technical training that allow them to operate efficiently in their area of assignment.
- 1.4.2 The following are the mandatory orientations that both call takers and dispatcher should complete prior to be signed off to process emergency calls:
 - 1.4.2.1 ProQA training
 - 1.4.2.2 Communication's Standards Operating Procedures and Policies
 - 1.4.2.3 Mapping System including
 - 1.4.2.4 NJM application: The Computer Aided Dispatch (CAD) application for resources management and deployment of units.
 - 1.4.2.5 Life-X system (Telephone & Radio systems operation Consoles).
 - 1.4.2.6 Telephone techniques
 - 1.4.2.7 Radio system operation and voice procedures
 - 1.4.2.8 Mobile Data terminals (MDT)
 - 1.4.2.9 Mass Casualty Incident orientation

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Group
Procedure	3.28 IT EQUIPMENT ALLOCATION AND USE
Implementation Date	06 October 2020
Purpose	To give clear guidance on the allocation and use of Ambulance Service IT Equipment Request.
Responsibility	ALL HMCAS GROUP STAFF

1. Purpose:

- 1.1 The purpose of this Policy is to give clear guidance on the allocation and use of Ambulance Service IT Equipment Request. The policy encompasses all IT Equipment Request used within the Ambulance Service Group.

2. Scope:

- 2.1 This SOP applies to all staff working within the HMCAS Group.

3. Definitions:

- 3.1 Shared PC or Workstation and related Monitor(s).
 3.2 Personally: Issued PC or Workstation and related Monitor(s).
 3.3 Shared Laptop(s)
 3.4 Personally: Issued Laptop
 3.5 Shared desktop Telephone
 3.6 Personally: Issued desktop Telephone
 3.7 Printers & Scanner
 3.8 Accessories

4. Sub – Definition PC's – Laptops:

- 4.1 Standard Computer – PC
 4.1.1 Single or Dual Monitors
 4.1.2 Application for Administrative users, primary use MS office Application.
 4.2 High Demand - Workstation
 4.2.1 General 24/7 Workstation
 4.2.2 Dual or Quad Monitors
 4.3 Standard Laptop
 4.3.1 Suitable for general use
 4.4 Mid-Range - Laptop
 4.4.1 Suitable for use in a truly mobile environment/support
 4.5 High performance Laptop
 4.5.1 Data Analytics
 4.5.2 Media
 4.6 Laptop docking stations
 4.6.1 Standard & Mid-Range – Laptops only

5. Sub – Definition – Printer:

- 5.1 Large network Floor standing Printer
- 5.2 Small network printers
- 5.3 Personal Printer
- 5.4 Desk Scanner

6. Sub – Definition – Telephone:

- 6.1 Desktop Phone Analogue
- 6.2 Desktop Cisco IP Phone (without call centre licences)
- 6.3 Desktop Cisco IP Phone (with call centre licences)
- 6.4 Wireless Cisco Wireless IP phone

7. Sub – Definition – Accessories:

- 7.1 Screens stand and related arms
- 7.2 Webcam & related connecting cables (USB & Cisco Phone)
- 7.3 Headsets (USB & Cisco phones)
- 7.4 HDMI (5, 10, 20 metres)
- 7.5 5 metre Power cables
- 7.6 5 metre monitor extender cables and adaptors
- 7.7 5 metre USB Keyboard or Monitor extender cable
- 7.8 Keyboard
- 7.9 Mice
- 7.10 Laptop Car Charger
- 7.11 Mobile Tablet
- 7.12 Toughpad
- 7.13 Fax Machine

8. Responsibility:

- 8.1 Executive Director of Healthcare Coordination & Support Services:
 - 8.1.1 Responsible for approval and disseminating this policy.
- 8.2 Assistant Executive Director for IT Delivery and Transformation Programme:
 - 8.2.1 Responsible for coordinating the development, approval, and dissemination of this policy.
- 8.3 Head of IT Delivery and Transformation Programme:
 - 8.3.1 Responsible for developing, revising, updating, and implementing this policy.
- 8.4 All HMCAS Staff are responsible to comply with the policy.

9. Priority Guidelines:

- 9.1 IT Delivery and Transformation Programme Priority Criteria for the allocation new IT Equipment (PC's – Laptops), Priority shall be given to requests related to:
 - 9.1.1 Operational key roles where a new shared PC is required.

- 9.1.2 Newly joining staff to Ambulance Service, who is not a direct replacement for an existing member of staff, in other words additional member of staff, who will not be performing a shared role, for example Warehouse Supervisor, VCS Controller, Production Supervisor or some performing (24/7) role.
- 9.1.3 Any staff changing department /section who is not a replacement for existing member of staff, in other words additional member of staff, who will not be performing a shared role.
- 9.1.4 Any staff who are using a temporary laptop or personal laptop because there were no PC's available.
- 9.1.5 Staff using a Laptop or PC's that is not suitable for upgrade to the latest approved operating system.
- 9.2 PC's & Laptop's Data Back Up's:
 - 9.2.1 The end user will be responsible for backing up their personal data.
- 9.3 Multiple requests for PC's – Laptops replacements:
 - 9.3.1 Not all the PCs or Laptops within a department/section – can be replaced all at the same time despite the request. The priority will be given to PC's or Laptops that are not suitable for upgrade to the latest approved operating system; followed by PC's or Laptops aged more than 5 years old;
 - 9.3.1.1 Equipment will not be replaced without the commitment from the requester to return all existing equipment, including all accessories as they originally received).
 - 9.3.2 Multiple requests may result in the creation of a mini project, which will be scheduled in line with available resources.
- 9.4 Shared device – The shared devices (any IT equipment using by more than one person). Will be requested and approved by the section AED and then ownership will be transferred - assigned to that AED, as they are eligible to have more than PC or Laptop assigned to them within corporate asset register.
- 9.5 Loan Equipment - The lending equipment is purely based on availability and IT manager decision. It is this person who has been loaned the equipment will be responsible in case of loss or damage.
- 9.6 Moving of IT equipment - Under no circumstances should any IT equipment be moved without informing IT Delivery and Transformation Programmes through email to HMCASITdelivery@hamad.qa, at least 3 days prior to the planned move.
- 9.7 Re-assigning of IT equipment -Under no circumstances should any IT equipment be reassigned within department/section all IT equipment must be returned to the IT Delivery and Transformation Programme.
- 9.8 Printer Requests:
 - 9.8.1 Large Network Floor standing printers shall be distributed and replaced by IT Delivery and Transformation Programme.
 - 9.8.2 Personal MFP printers will no longer be issued to individual staff or small teams of the staff.
 - 9.8.2.1 Exception to the above rule, MFP Printers may be installed at Temporary or Remote sites, when installing a Large Network Floor standing printer's is not cost effective (based on usage or space), this decision rest solely with IT Delivery and Transformation Programme.

9.9 Telephone:

9.9.1 Personal issued Telephones:

9.9.1.1 Shall be personally issued and will be assigned to individual who is responsible for this phone until such time:

9.9.1.1.1 The telephone is replaced

9.9.1.1.2 The person leaves the section or department, at which time the telephone will re-assigned to the next person using this workspace or returned to the IT Delivery and Transformation Programme.

9.9.2 Share Phones:

9.9.2.1 Shared Phones shall be assigned to the Assistant Executive Director for the department or section where the phone is being deployed.

9.9.2.2 Or when the telephone is returned to the IT Delivery and Transformation Programme department.

9.9.2.3 Or shall be assigned to the elected AED by the ED should the AED leave the organization.

9.10 Accessories:

9.10.1 Request for additional Accessories, will be based upon whether there is a new requirement, for example staff have been authorized, to have a new TV Monitor and requires a HDMI cable.

9.10.2 Replacement of an existing Accessories (damaged IT equipment):

9.10.2.1 End user will be required to report all damaged IT equipment to the HMC IT Helpdesk (OUNAK).

9.10.2.2 End user shall be required to provide the Helpdesk Ticket number at the time of the request, before a replacement will be provided.

10. Process:

10.1 IT Delivery and Transformation Programme Process for the allocation and IT Equipment usage:

10.1.1 The Authorized Requestor (Assistant Executive Director (AED) or senior manager with the approval from AED) must submit a general request by email to HMCASITdelivery@hamad.qa addressing the Head of IT Delivery and Transformation Programmes.

10.1.2 The requestor may combine several requests into a single request, to IT Delivery and Transformation Programme, by email to HMCASITdelivery@hamad.qa addressing the Head of IT Delivery and Transformation Programmes.

10.1.3 All other IT usage will be as per HMC HICT IT Usage Policy.

11. Purchasing Request:

11.1 All request for purchasing of IT equipment should be made in the first instance to IT Delivery and Transformation Programmes by email to HMCASITdelivery@hamad.qa addressing the Head of IT Delivery and Transformation Programmes for assessment.

11.2 The following documentation may be required for any IT requests, that is valued in excess 20,000 QAR.

11.2.1 IT change request

11.2.2 Approved Budget

11.2.3 Business Case

11.2.4 To be submitted to IT Delivery and Transformation Programme
HMCASITdelivery@hamad.qa address to AED IT Delivery and Transformation
Programme, who will review before submitting to HICT portfolio.

12. Attachments:

12.1 None

13. References:

13.1 None

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Health Coordination and Support Services Training and Quality Improvement Department
Procedure	3.29 DHP AND TRAINING REQUIREMENTS FOR AMBULANCE SERVICE MEDICAL DISPATCHERS
Implementation Date	08 December 2020
Purpose	To ensure that all Health Coordination Services (HCS) ASMD Staff are on-boarded appropriately and receive consistent orientation and training required by HMC and JCI.
Responsibility	ALL HCS AMBULANCE MEDICAL DISPATCHERS

1. ONBOARDING OF AMBULANCE SERVICES MEDICAL DISPATCHERS (ASMD):

- 1.1 **Minimum Required Orientation:** This Standard Operating Procedure (SOP) describes the minimum required orientation and training for all Ambulance Services Medical Dispatchers (ASMD) to be completed during the initial induction to meet HMC, International Academy of Emergency Dispatch (IAED) and Joint Commissions International (JCI) requirements.
- 1.2 **HMC Human Resources (HR) Orientation:** Upon arrival, all new staff will initially report to HR to complete orientation procedures and documentation. Once this process is completed, he/she will then report to the Health Coordination Services (HCS) Training Department for the on-boarding training.
- 1.3 **HMC-HCS On-Boarding:**
 - 1.3.1 HCS Training Unit will assist the new ASMD staff with completion of the training based on the below checklists:
 - 1.3.1.1 HMC Corporate Orientation Checklist
 - 1.3.1.2 HMC-AS Orientation Checklist
 - 1.3.1.3 Departmental/Job Specific (ASMD) Orientation Checklist
 - 1.3.2 The new staff member will attend the following lectures required by the corporate the HMCAS group:
 - 1.3.2.1 Dress Code
 - 1.3.2.2 Training Unit House Rules
 - 1.3.2.3 HMCAS Infection Control (or HMC e-learning module equivalent).
 - 1.3.2.4 HMCAS Fire Safety (or HMC e-learning module equivalent).
 - 1.3.2.5 HMCAS Environmental Safety (or HMC e-learning module equivalent).
 - 1.3.2.6 HMCAS Quality Improvement and Patient Safety (HMC e-learning module equivalent).
 - 1.3.2.7 HMC Major Incidents (e-learning module)
 - 1.3.2.8 Introduction to the OVA reporting platform
 - 1.3.3 The new staff member will be issued the following documents and attend the relevant training as preparation to the ASMD job:
 - 1.3.3.1 HMCAS group Orientation Book (Hard Copy)
 - 1.3.3.2 HMCAS group Standard Operating Procedures Books (Soft Copy)
 - 1.3.3.3 HMCAS group policies and operation plans (9032, 9012 etc.) (Soft Copy)

- 1.3.3.4 HCS Training Log book
- 1.3.3.5 Emergency Medical Dispatch (EMD) Course Training Material (soft or hard copy).
- 1.3.3.6 Emergency Telecommunication Course (ETC) Training Material (soft or hard copy).
- 1.3.3.7 Software training and relevant materials (ProQA, NJM, Life-X, mapping, MDT...)
- 1.3.3.8 Corporate Orientation Guideline Document (Hard copy)
- 1.3.4 The new staff member will complete their N95 Fit Test. it is required to be performed prior to the staff member attending any patient facing shifts and ride along with Alpha units.
 - 1.3.4.1 The N95 fit test is conducted by the AS Training Unit prior to the staff member's initial ride along with field crews.
 - 1.3.4.2 The N95 mask is used to help protect staff against airborne pathogens. Staff members need to ensure that they have an effective facial fit in order to ensure proper protection.
 - 1.3.4.3 Should the staff member "fail" the fit test he/she will be re-tested with a different size/mask until the correct fit is found.
 - 1.3.4.4 Should a "failed" test be the result of the staff member refusing to remove facial hair, the staff member will be required to sign a waiver. A copy of the waiver will be saved in the staff member's file and a copy will be sent to Operations.
- 1.3.5 HCS Training Unit will assist the staff with the application for his/her HMC username creation (this can only be done once the staff member has received his/her Corporate ID card which is the responsibility of the HMC HR department to issue during the HR Orientation).
- 1.3.6 HCS Training Unit will start a hard file for the new staff member to house all records and documentation pertaining to the above orientation/training. Once the staff member training data is loaded onto the Scheduling Management System (SMS), the relevant documentation will then be uploaded onto their electronic profile.
- 1.3.7 All the above training represents the minimum requirements before staff members attend patient-facing call taking/dispatching or any ride along.
- 1.4 **Ambulance Service Medical Dispatch Course and Initial Training:**
 - 1.4.1 HCS Training Unit will provide an orientation for all newly hired Ambulance Service Medical Dispatchers (ASMD) for both call taking and dispatching skills. This course is required for staff before they can apply licensure with the Department of Healthcare Professions. The Course will include but not limited to:
 - 1.4.1.1 The didactic course generic for both National Command Center (NCC) and Health Coordination Center (HCC) and will include all materials relevant for both centers (Customer Service, EMD/ETC courses) some specific training will be given to the staff according to their center of assignment (the Computer Aided Dispatch, Telephone system);
 - 1.4.1.1.1 Basic Life Support Course (BLS)
 - 1.4.1.1.2 Emergency Medical Dispatch Course (EMD)

- 1.4.1.1.3 Emergency Tele-communication Course (ETC)
 - 1.4.1.1.4 ProQA, CAD, Life-X, and Cisco software Courses
 - 1.4.1.2 Mentorship orientation: the new employee will attend the mentorship period in the call center of his/her full-time assignment. Either NCC or HCC. The student will be assigned to work with a full time mentor for a period that vary from 1 month to 3 months.
 - 1.4.1.2.1 Call taking
 - 1.4.1.2.2 Dispatching
- 1.4.2 During the ASMD course the training unit will assist the staff to register in the following HMC Hospitality training courses: (staff are encouraged to attend these courses within the first two years of their joining date).
 - 1.4.2.1 Hospitality Courses:
 - 1.4.2.1.1 World of Communication
 - 1.4.2.1.2 World of Service
 - 1.4.2.1.3 World of Welcome
 - 1.4.2.1.4 World of Opportunities
 - 1.4.3 Optional Courses: Hamad International Training Center (not mandatory but highly preferred for supervisors and only after approval of the Operation manager).
 - 1.4.3.1 Pediatric Life Support Course (PLS)
 - 1.4.3.2 International Trauma Life Support (ITLS)
 - 1.4.3.3 Major Incident Medical Management and Support Course (MIMMS)
 - 1.4.3.4 HMC AS MIR Overview (CPD)
 - 1.4.4 Upon completion of training and prior to release into Control Centers, each newly qualified staff member will receive their respective onboarding letter. Copies of the privileging letter will be stored in the staffs training and HR files.

Title	Recertification Period	Delivery	JCI Required	DHP Required	IAED Required	Department required
Corporate Orientation	N/A	Online	☒	☐	☐	☒
Facility Orientation	N/A	Training Unit	☒	☐	☐	☒
Departmental Orientation	N/A	Training Unit for ASMDs.	☒	☐	☐	☒
Ambulance Service Clinical Medical Dispatch Course	N/A	Training Unit	☒	☒	☐	☒
EMD Course	2 Years	Training Unit	☐	☒	☒	☒

ETC Course	2 Years	Training Unit	☐	☒	☐	☒
BLS Course	2 Years	AS Training Unit/HITC	☒	☒	☒	☒
All Safety programs	Course Specific	Online	☒	☐	☐	☒

2. ANNUAL TRAINING REQUIREMENTS AND MINIMUM MEDICAL DISPATCH HOURS FOR ASMDs:

2.1 Introduction:

- 2.1.1 Licensure with DHP is dictated for all HMC Health Coordination Service ASMD staff members.
- 2.1.2 In order to be able to recertify and maintain medical dispatch currency the EMD/ETC courses and re-license with DHP, ASMD staff will be required to meet minimum annual training requirements.
- 2.1.3 The minimum standards are a combination of those requirements set forth by the Department of Healthcare Professions (DHP), Joint Commission International (JCI), and the HCS executive management members.
- 2.1.4 All minimum requirements as described in this section must be met by all ASMD licensed staff.

2.2 Definitions:

- 2.2.1 Continuous Professional Development (CPD) is any training, course, or learning activities, which meet the standards of DHP and is accredited or accepted by the DHP for CPD hours.
- 2.2.2 The three primary HCS Core CPD activities run by the HCS Training and QI Department that run continuously and cover the following topics:
 - 2.2.2.1 Dispatch Life Support (DLS) Protocols Course
 - 2.2.2.2 Trauma Protocols Course
 - 2.2.2.3 Medical Protocols Course

2.3 Minimum Requirements for Ambulance Service Medical Dispatchers:

- 2.3.1 Continuous EMD related Professional Development hours – minimum of 24 hours per 2 years as outlined by IAED.
- 2.3.2 In-date HMC Basic Life Support (BLS) Course must be completed every 2 years.
- 2.3.3 DLS protocols Course must be completed once every year.
- 2.3.4 Trauma protocols Course must be completed once every year.
- 2.3.5 Medical protocols Course must be completed once every year.
- 2.3.6 To maintain the privilege of ASMD, staff must maintain shift hours as ASMD or ASMD related supervisory jobs – minimum of 2 to 4 shifts per month. HCS deem the following areas of ASMD practice as being patient care for maintenance of clinical hours:
 - 2.3.6.1 Any hours allocated shift at HCC and/or NCC for call taking, dispatching, Quality review, Dispatch training or Supervisory roles.
 - 2.3.6.2 Any hours allocated shift within MCU/MCC unit and/or deployed special operation role where the ASMD is required to respond to Major Incidents.

- 2.3.6.3 Any hours allocated shift at Hamad International Airport watch-room.
- 2.3.6.4 Covering the on-call rota where the nature of the period of on-call is to respond following a Major Incident activation and/or to provide Medical Dispatch command-based decisions or Medical Dispatch advice outside of a major incident.
- 2.3.6.5 HCC/NCC Event coverage

3. **REPORTING OF COMPLIANCE TO MANDATORY CLINICAL TRAINING:**

- 3.1 **Reporting Frequency:** A compliance report will be provided to the HCS Head of Professions each end of semester.
- 3.2 **Report Content:**
 - 3.2.1 Percentage of staff who that are compliant with each of the HCSS Core Continuing Professional Development training modules:
 - 3.2.1.1 Dispatch Life Support (DLS) Protocol Course
 - 3.2.1.2 Trauma Protocol Course
 - 3.2.1.3 Medical Protocol Course
 - 3.2.1.4 CDE Courses
 - 3.2.2 Percentage of staff that are compliant with the HMCAS group offered safety courses will be tracked and provided by HMCAS training unit:
 - 3.2.2.1 Fire Safety
 - 3.2.2.2 Infection Control
 - 3.2.2.3 QPS Training
 - 3.2.2.4 Environmental Safety
- 3.3 **DHP Validation of Clinical Training Compliance Reports:**
 - 3.3.1 Continuing Professional Development courses is highly recommended to be accredited by the DHP. Signed attendance records and course content shall be maintained, as required by DHP, for review and verification.

4. **SCHEDULING OF CORE HCS CPD COURSES FOR ASMDs:**

- 4.1 **Scheduling of CPD sessions:**
 - 4.1.1 It is the responsibility of the ASMDs to coordination with HCS training unit to book their core CPD courses.
 - 4.1.2 Each department within HCS should maintain a member within the department to act as a liaison between his/her department, HCSS Training Unit and the Scheduling supervisor regarding the scheduling of their staff to CPD activities.
 - 4.1.3 Staff should monitor their schedule and ensure that they access the online content when applicable and complete all pre-course activities at least 2 weeks prior to attending the course.
 - 4.1.4 The Training Unit will release a CPD schedule to all departments each 3 months.
 - 4.1.5 All scheduling of CPD events should be done by the Scheduling supervisor and an updated schedule of events should be sent to the Training Unit.
 - 4.1.6 Any queries regarding the scheduling of Training sessions should be taken up with the staff member's department liaison or the Scheduling supervisor directly.

- 4.1.7 All staff should be scheduled for the CPD activity a minimum of 2 weeks prior to the scheduled activity.
- 4.1.8 Staff are NOT to be scheduled to attend CPD training the day AFTER a night shift.
- 4.2 **Cancellation of Attendance on a Core CPD activity:**
 - 4.2.1 The HCS Core CPD activities are considered as any other normally assigned duty, failure to attend is the same as failing to attend a regular duty shift. All relevant procedures for attendance or cancellation are to be followed.
 - 4.2.2 If a staff member becomes sick resulting in him/her being unable to attend the CPD activity (If schedules in mandatory shift), normal HMC sick leave procedure should be followed.
 - 4.2.3 Staff are not permitted to cancel their attendance on a HCS Core CPD activity unless the cancelation has been approved by their Operations Manager – staff may not cancel training to:
 - 4.2.3.1 Undertake paid over-time
 - 4.2.3.2 Attend a paid event
 - 4.2.3.3 For casual leave (unless agreed by Operations Manager)
 - 4.2.4 Failure to attend an allocated CPD training activity is classed as being 'absent' and is a disciplinary event.
 - 4.2.5 Staff who frequently cancel training sessions will be subject to review by the Head of Training and their line manager.
 - 4.2.6 The Operations Manager or their designate is responsible for contacting the Scheduling supervisor to notify them of the approval of a staff member's cancelation of attendance.

5. EMERGENCY DRIVER TRAINING:

- 5.1 Only staff approved by the Operation Manager can attend the Emergency driving course.
- 5.2 Selected HCS staff will attend the driving training facilitated by HMC AS training unit.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Communications
Procedure	3.30 HANDLING PARTICIPANT CASES
Implementation Date	16 August 2022
Purpose	To ensure that EMDs effectively process ambulance request from other emergency agencies at National Command Center (NCC/TCC).
Responsibility	ALL COMMUNICATIONS STAFF

1. **Criteria of responding to assistance requests from other emergency agencies at TCC:** These guidelines should be adhered to when government emergency agencies request that Ambulance service respond to call for assistance under the following conditions, HMCAS Medical emergency Dispatch center will deploy the closest resources as requested, whenever possible.
 - 1.1 Any kind of emergency where a patient(s)/victim(s) is reported to be on scene.
 - 1.2 Any kind of emergency where a patient(s)/victim(s) is entrapped, and medical care maybe required at extrication.
 - 1.3 High index of suspicion for requiring medical assistance for public individuals or emergency services personnel such as search and rescue, active fire with no patients, (**assault and Domestic issue**).
 - 1.4 Major incident confirmed or potential even if no victims reported. (**Such as strike cases**).
2. **Required information for assistance requests from other emergency agencies at TCC:** The following information are required to be added in NJM prior to any call for service requests.
 - 2.1 Exact location with all egress details. Documentation in NJM must be verified and completed preferably with QARS details or additional comments to describe the best access to the patient/scene.
 - 2.2 All safety details (Aggressive patients, use of weapon, dangerous animals, and other hazards).
 - 2.3 Number of patients/victims
 - 2.4 Brief description of the Nature of illness or mechanism of injury.
 - 2.5 METHANE report if major incident
3. **Process of requesting assistance by other emergency agencies at TCC:** All governmental agencies can request HMC AS assistance as following:
 - 3.1 Add HMCAS (EMS) to the Call for Service (CFS) as participant agency and route the CFS to HMCAS dispatcher with all details in point2 accurately added to CFS.
 - 3.2 Highly preferred that 999 operators to transfer the caller to HMCAS call taking consoles with the same CFS that was routed to the HMCAS dispatcher.
 - 3.3 999 operators or any other agency requesting the assistance should avoid transferring or routing duplicated calls with the same CFS not to cause confusion and overload at HMCAS consoles.
 - 3.4 In case of high acuity incident, major incident or any other outstanding situation the officer/agent of the agency requiring HMCAS assistance should call HMCAS on-duty officer (OSCAR) to endorse the cases.
4. **Process of handling requests of assistance from other emergency agencies at TCC:** When receiving a CFS as participant the ASMD dispatcher should activate the CFS and read the details carefully:

- 4.1 If the case does not require an ambulance dispatch such as car stuck, event, GIS balloon footage etc., the ASMD shall reject the CFS and close it. (After consult with DTL).
- 4.2 If the case seems duplicated with another existing CFS, the ASMD must follow all the steps of duplicated calls [SOP 3.2](#) point 8.
- 4.3 If the call is not rejected and not a duplicate, ensure initial dispatch as follows:
 - 4.3.1 If the case is a Road Traffic Accidents and/or a death cases the ASMD dispatcher will deploy the **closest unit Priority 1** (a solo unit with Lights and Siren). Deployment can include the closest Alpha or Delta unit unless more information is available requiring additional resources.
 - 4.3.2 For all other participant calls the ASMD dispatcher will deploy the **closest unit Priority 2** (a solo unit no Lights and Siren) unless more information is available requiring additional resources.
- 4.4 For all processed participant calls ASMD dispatcher will endorse the CFS to a call-taker to start the call back attempts immediately.
- 4.5 If call back attempts are successful (within 2 minutes), the ASMD complete the ProQA interrogation, update the code, and notify the dispatcher. The dispatcher will send resources according to the ProQA code and must notify the DTL. (Call back attempt should not more than 2 times).
- 4.6 If the second call back attempts is not successful (within 2 minutes), continue the initial dispatch and DTL will coordinate with the owner agency when required, upgrade or downgrade the response priority/resources if needed.
- 4.7 OSCAR NCC and OSCAR Command may add more resources (Delta, Charlie, Additional Alpha units, GB, MIR...) to the call based on additional information provided by other agencies through NJM or verbal conversation.

SOP 4: Emergency Operations



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.1.A PATIENT & FAMILY RIGHTS – CONSENT, PRIVACY & CONFIDENTIALITY
Implementation Date	01 January 2017
Purpose	To ensure that due diligence is given to patient and family rights with regard to consent, privacy, and confidentiality.
Responsibility	ALL HMCAS STAFF

1. HMCAS has a legal and ethical obligation to ensure privacy and confidentiality of all patients.

2. Patient & Family Rights:

2.1 The patient and/or patient's family has the right to expect and receive the following from HMC Ambulance Service personnel:

- 2.1.1 To access HMC Ambulance Service regardless of race, religion, national origin, beliefs, values, language, age, or disability.
- 2.1.2 To receive care and services without unnecessary delay.
- 2.1.3 To receive respectful and considerate care at all times in a safe, secure, and non-threatening environment and to maintain the patient's personal dignity.
- 2.1.4 To have physical privacy during assessment and care and to avoid unreasonable, unwanted interference into the private affairs.
- 2.1.5 To have all records and medical information treated as confidential.
- 2.1.6 To accept or refuse medications, treatments, or procedures and to be informed of medical consequences of such refusal.
- 2.1.7 To make the decision to withhold or withdraw treatment and ceasing of resuscitative effort.
- 2.1.8 To be protected from physical assault.
- 2.1.9 To protect the valuables and possessions from loss or theft.
- 2.1.10 To express a concern or a complaint regarding the quality of care and/or service received.

3. Consent:

3.1 Refers to the provision of approval or agreement from the patient or legal guardian for treatment and interventions following thoughtful consideration.

3.2 HMCAS personnel shall:

- 3.2.1 Obtain general consent from patients for assessment, treatment, and transportation at the point of first contact.
- 3.2.2 Consider verbal or Implied consent is an acceptable means of receiving the patient's or his/her representative's [e.g. parent or legal guardian] general consent before performing procedures and this should be documented on the electronic Patient Care Report (ePCR). The Ambulance Service deems implied consent exists through the 999 call for service and/or through booked request for transportation via Retrieval Service UNLESS the patient verbally with-holds consent.
- 3.2.3 Perform necessary high-risk procedures and interventions without obtaining any consent only in a life-threatening emergency situation or where the patient is

incapacitated with a reduced level of consciousness. Patient's inability to consent or the non-availability of a representative should be documented on the ePCR.

- 3.2.4 Exercise implied consent regarding the rights of minors and incompetent adults, through their representatives if available.
- 3.3 A competent patient has the right to refuse any treatment, procedures and/or transportation. Medical consequences of refusal should be explained to the patient. The patient's refusal to consent should be documented in the ePCR before leaving the scene ([SOP 4.8.D](#)).
- 3.4 The ambulance service does not operate a system of 'treat and release' but in cases where patients are treated and discharged on scene (following refusal of transportation to hospital), or in cases where patients are not treated because of refusal of treatment or where patients refuse care the ePCR must be signed by the patient, family member or guardian ([SOP 4.8.D](#)).
- 3.5 Where a patient refuses treatment and/or treatment and subsequently refuses to sign the ePCR the ambulance crew should.
 - 3.5.1 Inform the National Command Centre
 - 3.5.2 Request the attendance at the patient's location of a duty supervisor.
 - 3.5.3 Document the refusal to sign ePCR within the comment section of the ePCR.
- 3.6 The ambulance service does not obtain consent nor offer advice on organ donation. Where a patient or a family member wishes to discuss organ donation then this will be handed over to the receiving hospital.

4. Patient Privacy:

- 4.1 Privacy in healthcare is an individual's desire to limit the disclosure of personal health information and avoidance of notice or display. Patients trust us with private information, so out of respect for our patients we must ensure that it stays that way.
- 4.2 HMCAS employees **shall NOT**:
 - 4.2.1 Release any Protected Health Information
 - 4.2.2 Take pictures or videos of patients or a patient's injuries (regardless of the inability to identify the patients face), unless given written permission by the patient.
 - 4.2.3 Take pictures or videos of any accident scene or vehicle damage even if in public view, unless an HMCAS vehicle is involved in the accident.

5. Patient Confidentiality:

- 5.1 Confidentiality carries the responsibility for limiting disclosure of private matter, including the responsibility to use, disclose, or release such information only with the knowledge and consent of the individual concerned.
- 5.2 HMCAS employees **shall NOT**:
 - 5.2.1 Discuss Patient information with any person that is not directly involved in the treatment of the patient.
 - 5.2.2 Discuss patient information when it can be overheard by people not directly involved in the care of the patient.
 - 5.2.3 Discuss patient information in a public place.
- 5.3 Discussion of patient information should be restricted to HMCAS employees and staff from receiving facilities that are involved in the patient's treatment.
- 5.4 Store all recorded information in a manner which only authorized personnel have access to it.

- 5.5 DO NOT copy ePCR's or NJM detailed reports in any way unless permitted by an HMCAS Officer.
- 5.6 Any request for information from an individual that is not an HMC employee should be referred to the HMCAS Quality Improvement Manager.
- 6. **Under no circumstance should an HMCAS employee divulge patient information to the media.**

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.1.B ABUSE AND NEGLECT OF CHILDREN AND ADULTS
Implementation Date	25 February 2020
Purpose	To support the identification of patients who are suffering abuse.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Purpose:

- 1.1 This SOP is designed to support staff in understanding what actions to take in a case of suspected neglect or abuse. This SOP is based on HMC policy for 'abuse and neglect of children and adults' (CL7217).

2. Definition:

- 2.1 **Abuse:** a harmful or injurious treatment of another human being that may include physical, sexual, verbal, psychological, emotional, intellectual, or spiritual maltreatment.
- 2.2 **Neglect:** failure to meet a dependent person's basic, physical, medical, educational, and social needs and/or emotional deprivation, and/or desertion.
- 2.3 For the purpose of this policy HMC defines children as being under the age of 14.

3. Actions on Transportation to Hospital:

- 3.1 The primary purpose of the Ambulance Service is to report episodes of suspected abuse and/or neglect to the receiving hospital so that the hospital can comply with CL7217.
- 3.2 Confirming a case of suspected neglect or abuse is a complex process which is outside of the scope of an ambulance service and may result in a criminal investigation. Therefore:
- 3.2.1 Avoid making judgements or being overly emotive.
- 3.2.2 Remain professional at all times.
- 3.2.3 Contact a duty supervisor to attend the scene.
- 3.2.4 Clearly record your observations and findings on your ePCR.
- 3.2.5 DO NOT record your opinions.
- 3.3 On arrival at hospital handover your concerns/findings to the receiving doctor.

4. Actions on Patient/Family Refusal for Transfer:

- 4.1 Request supervisor to attend the scene and up-date the NCC.
- 4.2 Encourage patient/family to attend hospital.
- 4.3 Where patient continues to refuse AND no family member is present offer to contact police.
- 4.4 Document the patient's response to this offer on your ePCR.
- 4.5 Where family are present and the family refuse to allow the patient to go to hospital. Inform NCC (regardless of call sign) and request NCC OSCAR updates NCC senior police officer.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.1.C CONFIRMATION OF DHP QUALIFICATIONS OF CLINICAL ESCORTS AND/OR FIRST RESPONDERS ON SCENE
Implementation Date	25 February 2020
Purpose	To confirm that private nursing staff and/or physicians are qualified with Department of Healthcare Professions (DHP) should they form part of a patient's clinical escort.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Escorting clinicians provided by Hamad Medical Corporation (HMC):

- 1.1 HMCAS has a service level agreement with HMC hospitals covering roles and responsibilities during inter facility transfers and therefore confirmation of qualifications is not required from HMC supplied clinical escorts.
- 1.2 The referring hospital/doctor is responsible for ensuring that any escorting clinician provided by HMC is qualified and privileged to provide ongoing care.

2. Escorting clinicians provided by SIDRA hospital:

- 2.1 HMCAS has a service level agreement with SIDRA covering roles and responsibilities during inter facility transfers and therefore confirmation of qualifications is not required from SIDRA supplied clinical escorts.
- 2.2 The referring hospital/doctor is responsible for ensuring that any escorting clinician provided by SIDRA is qualified and privileged to provide ongoing care.

3. Private home care nurses and/or private physicians:

- 3.1 HMCAS paramedics will request to see the private home care nurse (and/or physician) DHP license.
- 3.2 Private home care nurses/physicians may continue to provide.
 - 3.2.1 Ongoing nursing care and care of activities of daily living.
 - 3.2.2 Care of patient's personal medical equipment if required during transportation.
- 3.3 In the event of a clinical emergency private healthcare nurses and/or physicians may assist (but not direct) emergency care.

4. Healthcare provider providing immediate medical care as a member of the public:

- 4.1 On arrival immediate deal with any threat to life.
- 4.2 Obtain a history from the bystanders and associated healthcare provider whilst assessing the patient in accordance with CPG1.
- 4.3 Request to see a valid Department of Healthcare Professions license from any bystander reporting to be a healthcare provider.
- 4.4 Members of the public (to include Healthcare Providers) may assist the ambulance service with basic life support but they may not:
 - 4.4.1 Direct patient care
 - 4.4.2 Operate HMCAS equipment

SOP 4.2: Shift Handover, Vehicle Check & Safety



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.2.A ROUTINE OPERATIONAL SHIFT HANOVER VEHICLE CHECKING
Implementation Date	01 September 2020
Purpose	To ensure that routine operational vehicles are checked appropriately prior to use.
Responsibility	CREW OF ALPHA, CHARLIE, DELTA, OSCAR, TANGO, BIKE, EVENT, AND SPECIAL OPERATIONS UNITS

1. All staff working on the vehicle is responsible for keeping it clean and tidy.
2. The oncoming and outgoing crew must carry out a detailed handover and check of the vehicle and equipment **TOGETHER**.
3. Check and test all vehicle equipment at the commencement of the shift.
4. **Commencement of Shift:**
 - 4.1 All operational staff are required to report for duty fifteen (15) minutes prior to the commencement of their shift.
 - 4.2 Collect allocated vehicle keys and hand-held radio from the PTL or from staff handing over and ensure that the radio identification number is documented by the PTL.
 - 4.3 A 10-minute time limit is allocated to check a vehicle and have it operational, except in unusual circumstances.
 - 4.4 Ensure batteries of all equipment have at least 50% battery capacity prior to becoming available for calls. At least one back-up battery should always be fully charged.
 - 4.5 Ensure large oxygen cylinders are replaced or have a level of at least 50% capacity. At least one large cylinder should always be full.
 - 4.5.1 KEEP the main oxygen tap in the "ON" position - DO NOT CLOSE this tap.
 - 4.5.2 Ensure that the flow dial is kept at "0" (ZERO) to prevent leakage of oxygen.
 - 4.5.3 Remove plastic wrap from ALL portable cylinders - MUST be ready for immediate use. Ensure all portable cylinders are full, with no more than one cylinder at min. 50% full.
 - 4.6 Report any vehicle/equipment problems to the PTL.
5. **Completion of Shift:**
 - 5.1 Complete all relevant HMCAS documentation accurately and comprehensively prior to leaving the base.
 - 5.2 Replenish/replace any used stock/consumables/equipment in the vehicle.
 - 5.3 Ensure all equipment is present and accounted for and in working order.
 - 5.4 Ensure that the vehicle fuel level is above 75%.
 - 5.5 The unit must remain ready to respond at all times.
 - 5.5.1 Staff may not leave until their relief has arrived and a complete handover is done.
 - 5.5.2 Where there is no relief that will take over the unit, the unit must remain fully prepared to respond until completion of the shift time.
 - 5.6 Document and report any unusual incidents during the shift.
 - 5.7 Record any equipment that was left at a hospital or handed over to a Life Flight crew and notify the PTL if it was not possible to retrieve this equipment during the remainder of the shift.

- 5.8 If the vehicle is not going to be used upon completion of the shift, ensure that the vehicle is adequately stocked, cleaned, and locked and the keys handed in to the HMCAS stores or a HMCAS Supervisor for safe keeping.

6. The appropriate mechanical checklist for the vehicle type must be completed fully:

- 6.1 Start the vehicle and allow it to idle during the checking process.
- 6.2 Briefly test all lights, indicators, warning devices and sirens, taking into consideration the surrounding environment and population.
- 6.3 If there is a failure or deficiency of an essential element (as per check list), notify the production supervisor or PTL for unit change or repair.
- 6.4 Note any other failures or deficiencies on the checklist.
- 6.5 Provide details for failures or deficiencies on a Mechanical Defect Report and attach to the checklist.
- 6.6 The vehicle should be clean without visible soiling or dust. Where cleaning is required, report to the production supervisor to determine availability of cleaning staff.
- 6.7 Where fuel levels are below half, report to the production supervisor to determine availability for refuelling before refuelling.

7. The appropriate clinical equipment and supplies checklist must be completed fully:

- 7.1 Check electronic clinical equipment for currency of service date and specific tests as follows:
- 7.1.1 Monitor/Defibrillator: user test, attach printout to checklist.
- 7.1.2 Suction unit: suction test
- 7.1.3 Glucometer: as per Ambulance Paramedic duty checklist
- 7.1.4 Ventilator: as per Critical Care Paramedic duty checklist
- 7.2 Note the HBE number of all electronic clinical equipment on the checklists.
- 7.3 If there are failures or deficiencies of essential equipment or supplies (as per check list), which are unavailable at the Hub, report to a supervisor or PTL for further instructions.
- 7.4 Note any other failures or deficiencies on the checklist.
- 7.5 Equipment failures should be managed according to [SOP 4.3.A](#).

8. Radio test and booking on:

- 8.1 Staff should book available with the PTL, within 10 minutes of shift start, after completing the essential mechanical and clinical checks. Do not book available until oxygen system is ready.
- 8.2 Staff must report:
- 8.2.1 Unit, vehicle, radio, and monitor/defibrillator numbers;
- 8.2.2 Vehicle fuel level and
- 8.2.3 Names of the staff and any observers or students present.
- 8.3 Once staff have booked on with the PTL they should notify the appropriate Dispatcher of their location.
- 8.4 Where a unit has two radios (two portable, or one portable and one vehicle), use one radio for the initial communication with the PTL and the other for communication with the Dispatcher to ensure that both radios are working correctly.
- 8.5 If a radio fails, the test it should be reported to a supervisor or PTL for replacement.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.2.B NON-ROUTINE OPERATIONAL AND NON-OPERATIONAL SERVICE VEHICLE CHECKING
Implementation Date	01 October 2016
Purpose	• To ensure that HMCAS vehicles are checked regularly with respect to: • Non-routine operational vehicles viz. MCI vehicles, "Green Busses", the Bariatric ambulance. • Non-operational vehicles viz. travel cars, vans and decommissioned ambulances used for other purposes.
Responsibility	HUB PRODUCTION SUPERVISORS, MANAGERS WITH ASSIGNED VEHICLES AND STORES STAFF & ANY OTHER APPLICABLE STAFF

1. Following the use of any vehicle:
 - 1.1 Ensure that fuel level is above 75%;
 - 1.2 Replace used stock and equipment and
 - 1.3 Clean the vehicle and perform surface decontamination.
2. The above responsibilities of staff that used the vehicle do not take away from the responsibilities of the staff checking the vehicle, as described below.
3. Utilize and complete the appropriate mechanical and clinical equipment checklists for the vehicle type.
 - 3.1 Start the vehicle and allow it to idle during the checking process.
 - 3.2 Test all lights, indicators, warning devices and sirens.
 - 3.3 If there is a failure or deficiency of an essential element, report the vehicle to the PTL as out of service; deficiencies should be corrected with urgency.
 - 3.4 Note other failures or deficiencies on the checklist and rectify as soon as practically possible.
 - 3.5 Provide additional details for any failure or deficiency on an OVA and document the OVA number on the checklist.
 - 3.6 The vehicle should be clean without visible soiling or dust.
4. Utilize and complete the appropriate clinical equipment and supplies checklist:
 - 4.1 Check electronic clinical equipment for full battery levels, currency of service date and specific tests as follows:
 - 4.1.1 Monitor/Defibrillator: user test, attach printout to checklist.
 - 4.1.2 Suction unit: suction test
 - 4.1.3 Glucometer: as per Ambulance Paramedic duty checklist
 - 4.1.4 Ventilator: as per Critical Care Paramedic duty checklist
 - 4.2 Note the HBE number of all electronic clinical equipment on the checklists.
 - 4.3 If there are failures or deficiencies of essential equipment or supplies, which are unavailable at the Hub, notify the supervisor or PTL.
 - 4.4 Inspect generators and battery charging systems.
5. Perform a Radio test with the PTL, indicating that the vehicle has been checked.
6. Considerations for MCI vehicles, "Green busses" not in active duty.

- 6.1 During the checking process, drive the vehicle for 5 to 10 minutes instead of just idling the vehicle.
- 7. All operational vehicles must be checked at the start of the shift.

SOP 4.3: Medical Equipment & Medication Management



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.A ISSUING & MAINTENANCE OF MEDICAL EQUIPMENT & SUPPLIES
Implementation Date	01 October 2016
Purpose	To ensure that all operational personnel manage HMCAS medical equipment appropriately.
Responsibility	OPERATIONAL CLINICAL PERSONNEL (E.G. ALPHA, CHARLIE, DELTA, BIKE AND EVENT STAFF)

1. On every shift, complete the approved equipment checklists and note the following on the checklist in the allocated space:
 - 1.1 Biomedical Engineering (HBE) Number (found on the piece of equipment concerned)
 - 1.2 Date of next service
 - 1.3 Presence of any defects/malfunction of the equipment.
2. If a piece of equipment (e.g. LifePak monitor, Suction Unit, LUCAS device or Glucometer) has reached its due date for service, return that piece of equipment to the HMCAS stores and request a replacement.
3. If a piece of equipment is found to be defective or malfunctioning:
 - 3.1 Return the identified equipment to the HMCAS stores or Hub supervisor.
 - 3.2 Complete an OVA report
 - 3.3 Request a replacement for the defective equipment from the HMCAS stores.
4. Carefully manage all medical equipment to avoid damage or malfunction:
 - 4.1 Do not leave equipment in direct line of traffic when on the scene of an RTA.
 - 4.2 Ensure that equipment is sent in for service at least 2 weeks before the service date.
 - 4.3 Handle all equipment with care when setting up and packing away.
5. **Issue/Re-stocking of Supplies/Consumables:**
 - 5.1 When supplies (e.g. IV fluids, oxygen masks, etc.) have been used on a call, re-stock supplies either at the main HMCAS storeroom OR in any HMC ED's or Hubs.
 - 5.2 The Production Supervisor or Production Team Leader (PTL) may authorize the HMCAS unit to restock at HMCAS Base or arrange for stock to be transported to the unit's location.
 - 5.3 Ensure that a "STOCK REQUISITION" is completed appropriately and legibly.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.B MANAGEMENT OF NON-CONTROLLED MEDICATIONS BY AMBULANCE PARAMEDICS & CRITICAL CARE PARAMEDICS
Implementation Date	06 July 2021
Purpose	To ensure that all clinical operational personnel manage HMCAS approved non-controlled medications appropriately.
Responsibility	OPERATIONAL CLINICAL PERSONNEL

1. All medications shall be dispensed from any of Medication Dispensing Locations (MDLs) which include; relevant HMC hospital ED (Emergency Department)/AS (Ambulance Service) interface, and the respective AS hub.
2. All medications shall be stored and handled with respect to AS 9002 policy and in accordance with the manufacturer's instructions to maintain the quality, efficacy, and security of the medications. Unless otherwise directed, all stored or transported medications shall be kept out of direct sunlight and within an air-conditioned environment.
3. A HMCAS authorized personnel will be available 24/7 to assist with dispensing of non-controlled medications.
4. Do not leave medications unattended (e.g. in an unlocked vehicle or in an unlocked office).
5. If the HMCAS vehicle is not being handed over to oncoming HMCAS healthcare professional, then return and sign in the medication bags to the medication dispensing location.
6. Medication dispensing locations' staff shall check expiry dates of medications inside medication bags at least once monthly.
7. Any inappropriate medication labeling shall be communicated to supervisor and AS pharmacy department through an OVA.
8. Medications are considered acceptable for use up to:
 - 8.1 The end of the expiry month where a MONTH-YEAR expiry date is stated OR
 - 8.2 The end of the stated day date where a DATE-MONTH-YEAR is stated.
9. **Re-stocking of Non-Controlled Medications:**
 - 9.1 When medications need to be restocked it can be obtained from the closest medication dispensing location
 - 9.2 Complete the "MEDICATION REQUISITION" form legibly.
 - 9.3 If there is no relevant incident/trip number (as in cases of expired medications or general restocking of medication bag), then write the word "Restock" in the "Trip No." block.
 - 9.4 Place all received medications immediately into your assigned medication bag.
10. **Broken ampoules:**
 - 10.1 In cases of broken medication ampoule/vial/bottle or spilled medications:
 - 10.1.1 Safely discard the broken pieces of the medication ampoule/vial/bottle.
 - 10.1.2 Replace the medication at the appropriate medication dispensing location following the usual re-stocking procedure.
 - 10.1.3 On duty Emergency Operations Officer
 - 10.1.4 In case Pentrox® vial is broken, raise an OVA.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.C CONTROLLED MEDICINES MANAGEMENT BY CRITICAL CARE PARAMEDICS
Implementation Date	06 July 2021
Purpose	To ensure Critical Care Paramedics manage HMCAS approved controlled medications appropriately.
Responsibility	CRITICAL CARE PARAMEDICS

1. All controlled medications shall be dispensed as pre-packed Controlled Medication Cases (CMCs) with predefined standard content, from approved Medication Dispensing Locations (MDLs) (such as HGH - ED pharmacy, Helipad pharmacy and AS hubs). Within these locations, the pre-packed narcotic cases are stored inside prepositioned secured control medication cases cabinets.
2. Only a licensed Critical Care Paramedic (CCP) shall be eligible to be issued controlled medications.
3. A designated pharmacy staff is 24/7 available to assist with dispensing & receiving of CMCs.
4. **At the beginning of the shift:**
 - 4.1 Pharmacy staff are responsible for preparing the ready-for-dispense, pre-packed CMCs in accordance with the agreed predefined standard quantities of controlled medications.
 - 4.2 Request a pre-packed CMCs from the pharmacy staff at the medication dispensing location.
 - 4.3 Complete the relevant controlled medications register confirming the receipt of CMCs.
 - 4.4 Open the CMCs inside the medication dispensing location and ensure that it contains the correct and intact standard quantities of the controlled medications.
5. **At the end of the shift (returning the CMCs):**
 - 5.1 Ensure the CMCs are clean before returning to the medication dispensing location.
 - 5.2 Both pharmacy staff and the CCP will sign in the relative register to confirm the return of the CMCs.
 - 5.2.1 Pharmacy staff is responsible for confirming stock count inside CMCs.
6. **Re-stocking of controlled medications:**
 - 6.1 If controlled medication is used during patient contact and requires re-stocking:
 - 6.1.1 Attend the appropriate MDL and request a new ready-for-dispense CMC from the pharmacy staff, return the used CMC together with the residual of used controlled medications (as per point 7). These 3 processes (returning used CMCs, returning residuals, receiving new CMCs) must be documented in the relative registers and signed by both pharmacy staff and the CCP or doctor.
 - 6.1.2 Due to large volume of medication High Acuity Transfer Services may exchange medication on an ampoule for ampoule bases.
 - 6.1.3 Pharmacy staff is responsible for reviewing and confirming both the contents of the returned, partially used CMCs and the residuals of used narcotics (in accordance with the filled-in controlled drug administration form) as per point 7.
 - 6.1.4 Pharmacy staff is responsible for recording the count of remaining ready-for-dispense and returned CMCs in the CMC cabinets.

7. Returning of residual controlled medications:

- 7.1 If a CCP has used any controlled medications, they will need to return residual medication and the empty ampoule to the appropriate MDL.
- 7.2 Used empty syringes (with the needles removed) and the glass head/neck of broken ampoules (once all residual medication has been drawn out) should be discarded.
- 7.3 The empty ampoules/vials together with the syringe containing residuals of withdrawn controlled medication, closed with a stopper (without sharps) are to be placed in a Plastic Zipper Storage Bag (PZSB) and returned to the appropriate MDL, together with a completed "Controlled Drug Administration Form".
- 7.4 High Acuity Transfer Services to adhere to agreed process for discarding controlled medication whilst on overseas mission.
- 7.5 Any controlled medication prepared, but not administered to a patient, must be annotated in the free text section of the ePCR (e.g. "Midazolam prepared and drawn up, but not administered due to seizure self-termination").

8. Issues Requiring Escalation and Reporting:

- 8.1 In the context of HMCAS controlled medications, generally escalation and reporting will be between the Pharmacy staff and Oscar Effectiveness Officer and/or Clinical Manager on call by phone and then by email. All correspondence needs to be documented accordingly.
- 8.2 In the event of an accidentally broken ampoule or spilled controlled medication:
 - 8.2.1 Whenever possible, attempt to retrieve as much of the spilled medication as possible (by drawing the fluid into a syringe).
 - 8.2.2 Attempt to salvage all the pieces of the broken ampoule.
 - 8.2.3 Place the syringe containing the salvaged medication together with salvaged pieces of ampoule in a PZSB.
 - 8.2.4 Complete a "Controlled Drug Administration Form" (with brief details in the "Remarks" section) and place the completed form in the PZSB.
 - 8.2.5 Replace the CMC as per points 6 and 7.
 - 8.2.6 Notify the pharmacy staff and Effectiveness Officer. If Effectiveness Officer is unavailable, then contact the on-call Consultant Paramedic.
 - 8.2.7 Submit an OVA describing the breakage of the ampoule.
- 8.3 Any discrepancy in narcotic drug count will be escalated by the Pharmacy Supervisor as per AS 9004.

9. Staff and situations where controlled medications may be held for protracted periods:

- 9.1 CCPs who fulfill an on-call capacity either as duty officers and/or on the Major Incident Response roster and/or provide surge capability may hold a CMC for the duration of their on-call period.
 - 9.1.1 A record must be held within the appropriate MDL that the CMC has been signed out as per point 4.
 - 9.1.2 The CMC must be returned and signed in as per point 5.
 - 9.1.3 The maximum duration that a CMC drug box can be held by CCPs who fulfil an on-call capacity either as duty officers and/or on the Major Incident Response roster is one calendar month.

9.1.3.1 All CMCs must be returned by the last day of the month to ensure contents remain in-date.

9.1.4 CCPs who fulfil an on-call capacity either as duty officers and/or on the Major Incident Response roster must comply with points 7 and 8.

9.2 CCPs on overseas missions and/or when providing protracted care at a location within Qatar and/or overseas may retain a CMC:

9.2.1 A record must be held within the appropriate MDL that the CMC has been signed out as per point 4.

9.2.2 The CMC must be returned and signed in as per point 5.

9.2.3 There is **NO** maximum time period that CCPs on overseas missions and/or when providing protracted care at a location within Qatar and/or overseas may retain a CMC for. This is to reflect the nature of the duration of these events. However, an estimated date of return must be provided to the pharmacy during receiving the CMC.

9.2.4 CCPs collecting a CMC that will be used for a protracted time period should request a CMC with a long expiry date.

10. Storage of CMC:

10.1 CMCs must not be left unattended in a response unit or car and should remain with the responsible clinician.

10.2 For CCPs holding CMCs for protracted periods:

10.2.1 When on duty, CMC must not be left unattended in a response unit or car and should remain with the responsible clinician.

10.2.2 When off duty BUT STILL ON CALL, the CMC must either be with the responsible clinician or locked within a static location.

10.3 CCPs on overseas missions and/or when providing protracted care at a location within Qatar are to comply with point 10.2.2.

11. Major incident medication:

11.1 Major Incident supplies of controlled medication are available in HMCAS main pharmacy. These may be transported to an incident site as part of the major incident response. This is primarily the responsibility of either a CCP or a Pharmacist and/or a Pharmacy Technician. In extra ordinary circumstances, Silver Commander may request that a LICENSED Ambulance Paramedic transport-controlled medication.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.D NEAR PATIENT BLOOD TESTING
Implementation Date	25 February 2020
Purpose	To ensure supply, functioning and training of near patient blood testing equipment for operational units.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Blood Glucose Meter:**1.1 General considerations for point of care glucose testing:**

- 1.1.1 Initial equipment issued via Support Services will be checked and calibrated by Support Services Equipment section.
- 1.1.2 Glucose testing strips will be issued, via the Ambulance Service Group operational stores within HUB locations via Support Services which are in date, ready for use with test strips which are recommended by the manufacturer.
- 1.1.3 Test strips will be discarded 6 months after the first opening date of the test strip container as recommended by the manufacturer, when expired, or whichever comes first. All containers must be labelled with the first opening date. Labels will be supplied to all Hub Stores by Support Services.
- 1.1.4 The glucometer machine will be issued by the Ambulance Service (AS) Sub-Stores located in HUBs as part of the Ambulance Paramedic or Critical Care Paramedic response bag and will be returned to HUB Stores as part of standard rotation of equipment as required.
- 1.1.5 Calibration of the glucometer machine will be done according to the AS process guided by the manufacturer's recommendations by the Store-man, Production Supervisor, or trained Paramedic. Recommendations as follows:
 - 1.1.5.1 When a new vial of strips is ordered and opened for that specific glucometer.
 - 1.1.5.2 When the glucometer or test strips is suspected not to be working correctly.
 - 1.1.5.3 When unexpected blood glucose results were repeatedly recorded with discrepancies.
 - 1.1.5.4 When a blood glucose machine was dropped or accidentally damaged.
 - 1.1.5.5 When a new battery change is required.
- 1.1.6 In the event of equipment failure, discrepancies with blood glucose test results or accidental breakage, an OVA and equipment defect report should be completed and submitted followed by replacing the defective unit with a working unit.
- 1.1.7 All calibration tests must be documented by the person calibrating the glucometer on a glucometer test log sheet/log book located at respective Hub and Store locations.
- 1.1.8 All staff required to perform glucose testing and glucometer calibration will receive training during their induction period and/or during their Ambulance Paramedic course by the AS Training section in accordance with the manufacturer's recommendations.
- 1.1.9 Subsequent training will occur as and when a new blood glucose machine is introduced into operational use.

2. iStat:

- 2.1 The iStat is exclusively used by Transfer and Retrieval Ambulance and Critical Care Paramedics.
- 2.2 Transfer and Retrieval Ambulance and Critical Care Paramedics are trained on the correct use of the iStat during their induction period.
- 2.3 Refresher skills drills are conducted once yearly in Service or as part of a Refresher training.
- 2.4 iStat machines and test cartridges are stored, maintained, and issued at request by Production Transfer and Retrieval.

3. Critical Values:

- 3.1 The Ambulance Service does not define critical values because near patient blood testing is carried out by the person requesting the test.
- 3.2 Values above or below a treatment threshold are defined where appropriate in CPG.
- 3.3 The decision to treat is based on clinical judgement taking clinical data and patient condition into account.
- 3.4 Results of near patient blood tests are recorded within the paper or electronic Patient Care Record.
- 3.5 High and low levels of blood glucose that may require treatment are outlined in CPG 7.
- 3.6 The lower level of haemoglobin that may require treatment is outlined in [SOP 5.4.F.4](#) Blood Transfusion for Transfer and Retrieval Service.
- 3.7 The decision to act on results in respect pO₂, pCO₂ or pH are based on clinical judgement by the transferring physician or Critical Care Paramedic.

Glucometer Calibration

Do a Control Solution test:

- When a new vial of strips is ordered and opened for that specific glucometer.
- When the glucometer or test strips is suspected not to be working correctly.
- When unexpected blood glucose results were repeatedly recorded with discrepancies.
- When a blood glucose machine was dropped or accidentally damaged.
- When a new battery change is required.

Note:

- Use only the approved Control Solution provided to your Glucometer strip device.
- Control solution test must be done at room temperature (20-25°C). Make sure your meter, test strips and control solution are at room temperature before testing.
- Write the discard date on the control solution bottle **3 months after the first opening date.**

Glucometer Test Strip Request Process

1. Glucometer test strip must be requested from AS Sub-stores, AS Hub Production Supervisor locations and Romeo locations.
2. Stores staff, Production Supervisors and clinical staff must perform a calibration on the glucometer using the approved control solution.
3. Calibration test should be:
 - Done **2 times** according to the manufacturer's process using the new test strip with the test solution (high and low).

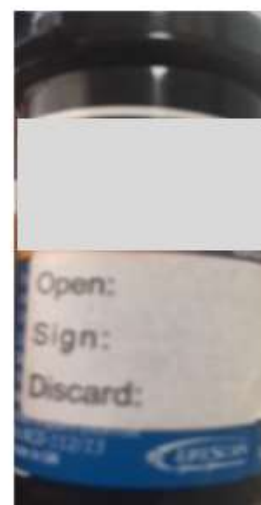
Note:

NORMAL solution ranges should be within the expected parameters printed on the test solution vial.

- Record the two (2) readings (High and Low) on log sheet provided at the respective locations.
 - Label the expiry date of the test strip vial from the date opened (**expiry date is 6 months from the date the vial is opened or expiry date indicated on the vial whichever comes first**).
4. If the two (2) results are outside the parameters specified on the test solution vial then:
 - Complete an equipment defect report form and OVA.
 - Replace the defective device with a new glucometer and new strips followed by the above process.
 - The defective device should be returned to the Equipment section for maintenance.

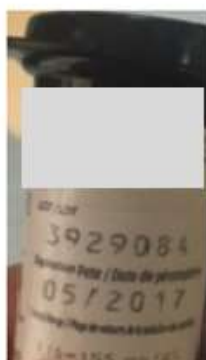
Test Strip Labelling Process:

Open:
Sign:
Discard:



Note:

Write discard date 6 months after opening date.



Do not place label over the expiry date of the container.



Write discard date 3 months after opening date.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.E HIGH ALERT AND LOOK ALIKE SOUND ALIKE MEDICATIONS
Implementation Date	04 March 2020
Purpose	To maximize patient safety when using High Alert Medications
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Definition:

- 1.1 High Alert Medications (HAM): Medications that are associated with a high percentage of errors or have greater risk for adverse outcomes or sentinel events if error occurs, as well as Look-Alike-Sound-Alike (LASA) medications (refer to AS 9010 Policy).
- 1.2 These drugs include, but are not limited to;
 - 1.2.1 Intravenous Inotropic and/or chronotropic drugs and/or vasoconstrictors.
 - 1.2.2 Intravenous vasodilators
 - 1.2.3 Intravenous concentrated electrolytes
 - 1.2.4 Intravenous irritant agents
 - 1.2.5 Intravenous antiarrhythmic agents
 - 1.2.6 Intravenous sedation and/or anti-epileptic
 - 1.2.7 Intravenous opiates

2. Look-alike Sound-alike medication are deemed to exist where:

- 2.1 SOUND-alike is where the drug name is similar to a different drug.
- 2.2 LOOK-alike where the packaging is similar to a different drug.
- 2.3 A complete list of medications deemed to be High Alert medications and/or LOOK-alike/SOUND-alike medications is appended with this SOP. The list of High Alert medications and/or LOOK-alike/SOUND-alike medication also indicates in which division of the Ambulance Service (999/Emergency or Transfer and Retrieval Services) that these medications are available for use.

3. Governance:

- 3.1 The Ambulance Service Clinical Guidelines and Standards Committee will make recommendations to the Ambulance Service Executive Governance Committee for all medications carried within the Ambulance Service including high alert medications. The Ambulance Service Executive Governance Committee, in conjunction with the HMCAS Group Medical Director, will agree on all newly recommended medications. Whenever any new medication is added, it is examined whether it is a HAM or LASA or not.

4. Generic procedures for High Alert Medication and LOOK-alike/SOUND-alike:

- 4.1 All Ambulance Service Drug bags are held in the Ambulance Service Pharmacy unless issued to a member of the ambulance service including;
 - 4.1.1 Critical Care Paramedic 999/Emergency, LIFEFLIGHT or Retrieval.
 - 4.1.2 Ambulance Paramedic 999/Emergency or Scheduled Ambulance Service (SAS) (this includes production supervisor).
 - 4.1.3 Ambulance Service doctor
 - 4.1.4 Hamad Medical Corporation Doctor

4.2 Ambulance service drug bags are only allocated to specific grades of clinician either a Critical Care Paramedic (CCP), Ambulance Paramedic (AP), medical doctor (member of the ambulance service or member of transfer team from Hamad Medical Corporation) or nursing staff employed by the ambulance service. The type of drugs contained within these drug bags reflect the TRAINING of the individual as well as the scope of practice outlined by Department of Healthcare Professions.

4.3 An independent check of all medication of all High Alert or LOOK-alike/SOUND-alike will be instigated prior to drug administration.

5. Procedure/Processes for High Alert Medication:

5.1 A list of High Alert medication will be contained within this SOP.

5.2 All High Alert medications will be identified by RED label stating HIGH ALERT.

6. Procedure/processes for LOOK-alike/SOUND-alike:

6.1 A list of Sound-alike are to be contained within this SOP.

6.2 All SOUND-alike medications names will be typed by using the Tallman lettering (emphasize parts of dissimilarities by using upper case letters).

6.3 Photos of updated Look-Alike pairs are sent by pharmacy through an alerting email to CCP and AP supervisors to be disseminated to all CCPs and APs.

6.4 All LOOK-alike/SOUND-alike medications will be kept separate from their LASA pair(s).

6.5 The potential error regarding LOOK-alike/SOUND-alike medications will be considered when introducing a new medication.

7. Attachment:

7.1 List of High Alert medications ([Figure 4.3.E](#))

7.2 List of LOOK-alike and SOUND-alike medications. ([Figure 4.3.E](#))

Figure 4.3.E

Drug	Class	HAM	Sound-Alike Pair(s)	EAS/SAS	CCS/RS
Adenosine 6mg/2ml	Antiarrhythmic			Yes	Yes
Atropine 1mg/10ml (1:10,000) PFS	Antiarrhythmic	Yes		Yes	Yes
Atropine 0.5mg/1ml (1:1,000)	Antiarrhythmic	Yes		Yes	Yes
CISTRACurium	Paralytic	Yes	VECUronium SUXAmethonium ROCUronium	No	Yes
DIGOxin	Antiarrhythmic	Yes	doxaPRAM	No	Yes
Dihydralazine	Anti-Hypertensive	Yes		No	Yes
DOBUTamine	Inotropic	Yes	DOPamine	No	Yes
DOPamine	Inotropic	Yes	DOBUTamine	No	Yes
Doxapram doxaPRAM	Respiratory Stimulant	Yes	DIGOxin	No	Yes
EPINEPHrine/Adrenaline 1mg/10ml 1:10,000 PFS	Inotropic	Yes	ePHEDrine	Yes	Yes
EPINEPHrine/Adrenaline 1mg/1ml 1:1,000	Inotropic	Yes	ePHEDrine	Yes	Yes
EPINEPHrine/Adrenaline 0.3mg In <i>EPIPEN</i> Auto-Injector	Inotropic	Yes	ePHEDrine	No	No
ePHEDrine	Inotropic/ Vasoconstrictor	Yes	EPINEPHrine	No	Yes
Esmolol 10mg/ml	Antiarrhythmic	Yes		No	Yes
Esmolol 250mg/ml	Antiarrhythmic	Yes		No	Yes
Etomidate	Sedative	Yes		No	Yes
Glyceryl Trinitrate (Infusion)	Vasodilator	Yes		Yes	Yes
Haloperidol	Sedative	Yes		No	Yes
Labetalol	Anti-Hypertensive	Yes		No	Yes
NOREPINeprine	Inotropic/ Vasoconstrictor	Yes		No	Yes
PhenylEPHRINE	Inotropic/ Vasoconstrictor	Yes	PhenyTOIN	Yes	Yes
PhenyTOIN	Anti-Epileptic/ Antiarrhythmic	Yes	PhenylEPHRINE	No	Yes

Drug	Class	HAM	Sound-Alike Pair(s)	EAS/SAS	CCS/RS
Potassium Chloride 15%	High Concentrated Electrolyte	Yes		No	Yes
Propofol	Sedative	Yes		No	Yes
VECUronium	Paralytic	Yes	CISTRACurium SUXAmethonium ROCUronium	No	Yes
Insulin	Insulin	Yes		No	Yes
SUXAmethonium	Paralytic	Yes	CISTRACurium VECUronium ROCUronium	Yes	Yes
ROCUronium-	Paralytic	Yes	CISTRACurium VECUronium SUXAmethonium	Yes	Yes
Fentanyl 100mcg/2ml	Opiate	Yes		Yes	Yes
Fentanyl 500mcg/10ml	Opiate	Yes		No	Yes
Ketamine 500mg/10ml	Sedative	Yes		Yes	Yes
Midazolam 15mg/3ml	Sedative	Yes		Yes	Yes
HAM: High Alert Medication LASA: Look-Alike, Sound-Alike Medication		EAS: Emergency Ambulance Services SAS: Scheduled Ambulance Services CCS: Critical Care Services RS: Retrieval Services			

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

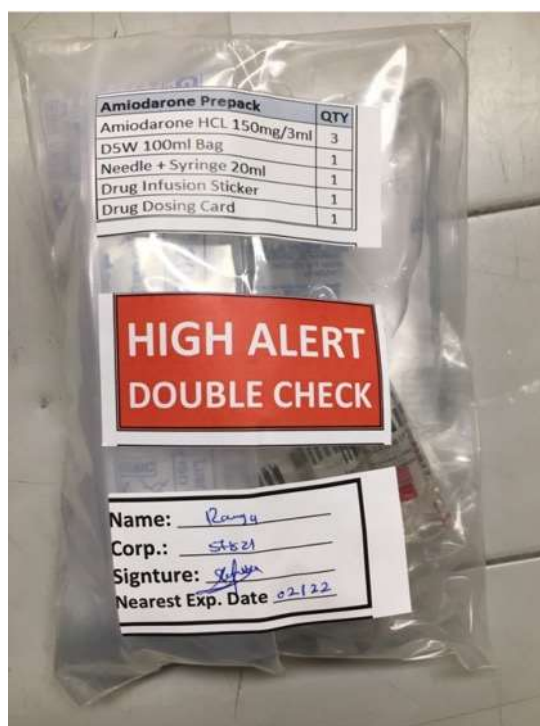
Sector	Ambulance Service Operations
Procedure	4.3.F ADMINISTRATION OF HIGH ALERT AND LOOK-ALIKE AND SOUND ALIKE MEDICATION
Implementation Date	06 July 2021
Purpose	To maximize patient safety when administering Look-Alike/Sound-Alike and High Alert Medications.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. The aim of this Standard Operating Procedure is to outline the procedures to be undertaken when administering either a High Alert Medication or a Look-Alike and/or a Sound-Alike medication.
2. **Identification of high risk, look alike and sound alike medication:**
 - 2.1 Look-Alike or Sound-Alike (LASA) medications shall be identified by:
 - 2.1.1 Utilizing TALL MAN lettering for labeling
 - 2.1.2 Physically separated from their LASA pair(s)
 - 2.1.3 Look-Alike/Sound-Alike label (Figure 1)
 - 2.2 Each High Alert Medication will be labelled with a red label stating, "HIGH ALERT" (Figure 2).
 - 2.3 Prior to administration of medication, the following procedures are required:
 - 2.3.1 Administer medication as per Clinical Practice Guidelines.
 - 2.3.2 Confirm any allergies (whenever possible) and check what medication has recently been administered and record on ePCR.
 - 2.3.3 In an emergency act in the patient's best interests according to all applicable professional, ethical and medicolegal frameworks and Hamad Medical Corporation Patient and Family Rights.
3. **Packing and Storage of medication:**
 - 3.1 Pharmacy is responsible for packing **all** medication into predefined medication bag/container compartments.
 - 3.2 CCPs and Ambulance Paramedics are not permitted to move medication between these predefined compartments.

Figure 1: Example of High Alert and Look-Alike, Sound-Alike Warning Labels:



Figure 2: Example of High Alert Medication Labelling:



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.G LP15 TRANSMISSION GUIDELINES
Implementation Date	08 January 2019
Purpose	To provide guidance and troubleshooting steps when LP15 transmission errors/failures occur.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Transmission of ECG data to the Toughpad ePCR:
 - 1.1 Select and transmit "ALL" when transmitting ECG and vital signs data to the Tough pad ePCR on completion of a case.
 - 1.2 Once ECG and vital signs data has been transmitted to the Toughpad ePCR, open and view the ECG data file in the ePCR to ensure that all data has been successfully transmitted and is complete prior to closing the case.
 - 1.3 Should an ECG data transmission to the ePCR fail, restart the ePCR Toughpad (using the Microsoft Windows Start Menu Restart option) and the LP15, enter the archives and attempt to re-transmit the ECG data again. Ensure you select the correct case from the archives.
 - 1.4 Should this fail, request assistance from the Clinical Documentation Officer (CDO). Should the CDO be unable to successfully transmit the ECG data to the ePCR, print out two complete 'Code Summary' and hand one copy to the CDO on duty, together with the relevant case identifying details (CFS and LP15 HBE number). Hand the second 'Code summary' to the receiving hospital/facility.
 - 1.5 The CDO shall complete an OVA for any transmission failures and include the CFS and LP15 HBE number in the OVA.
 - 1.6 Document in the ePCR that the ECG data failed to transmit.
 - 1.7 Notify the CDO if it is a Critical Case/Death in Care and hand the LP15 to the CDO. The CDO is to notify the Quality Team by sending an email to ASGQPSRM@hamad.qa with the CFS number and relevant case details. The CDO will retrieve the ECG data from the LP15.
 - 1.8 DO NOT open a new case until you have completed and closed the previous case. This is to prevent the wrong ECG and vital signs data being transmitted to the wrong case.
 - 1.9 It is mandatory to transmit LP15 ECG data to the Toughpad for all cases, regardless of severity.
2. Conducting the daily "User Test":
 - 2.1 Turn on the LP15.
 - 2.2 Press the "Options" button and select "User Test".
 - 2.3 The User Test will then be conducted and once complete will automatically print out the results of the test – either "User Test Passed" or "User Test Failed".
 - 2.4 Should the User Test fail, return the LP15 immediately to the production supervisor and replace it with a functioning LP15 which has passed the user test.
 - 2.5 The Production Supervisor shall complete an OVA reporting the equipment failure. The OVA must include the LP15 HBE number and where the LP15 was sent for inspection.
3. If 12-Lead ECG fails to transmit to NCC:

- 3.1 Check the modem cable is securely plugged in. Do not attempt to remove and plug the modem cable back in.
- 3.2 If indoors, place the LP15 near a window. Press "Transmit" and ensure the selected "SITE" is "NCC". Press "SEND" to re-attempt the transmission of the 12-Lead ECG to the NCC.
- 3.3 If you are in a remote/rural area with poor network signal, attempt to re-transmit the 12-Lead ECG once you are mobile to hospital and in an area with a stronger network signal.
- 3.4 Should these steps fail, notify the Clinical Desk that you are unable to transmit a 12-Lead ECG to them. Notify your Production Supervisor and arrange to have the LP15 replaced.
- 3.5 The Production Supervisor shall complete an OVA which will include the CFS number, LP15 HBE number and where the LP15 was sent for inspection.
4. Keep the 4-lead ECG cable plugged in the LP15 at all times, regardless of whether you are using the 4-lead ECG or not.
5. Pair the ePCR Toughpad with your assigned LP15 at the start of every shift, before leaving the Hub in order to change the Toughpad if any pairing issues are identified.
6. The ePCR Toughpad is required to be restarted at the start of your shift, or earliest convenience following the correct Microsoft Windows Restart procedure. Do not do a hard shut down by holding down the Toughpad power button.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.G.1 ZOLL X TRANSMISSION GUIDELINES
Implementation Date	18 September 2024
Purpose	To provide guidance and troubleshooting steps when ZOLL transmission errors/failures occur.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Transmission of ECG data to the Toughpad ePCR (Retrieving data from ZOLL):
 - 1.1 Receiving vital signs and ECG from ZOLL via Bluetooth through manual pairing from EPCR.
 - 1.2 EPCR Should be very close to ZOLL so EPCR can discover ZOLL and pair successfully.
 - 1.3 Note that a configuration is needed from ZOLL only from ePCR.
 - 1.4 Press on ZOLL ECG on the Menu Bar.
 - 1.5 **Press on (Get Current Case) to transmit vitals for the current patient, or you can transmit vitals from Archive by pressing on (Get Archive cases).**
 - 1.6 EPCR discovering ZOLL to pair with, for successful pairing keep ZOLL and ECPR very close.
 - 1.7 From the window select the name of ZOLL, then Press (OK) if the same name displays:
 - 1.7.1 To very ZOLL name:
 - 1.7.1.1 Select the Bluetooth icon on ZOLL monitor screen, by using the arrow on the ZOLL.
 - 1.7.1.2 Then press the circle bottom (beside the arrow).
 - 1.7.1.3 Check the "Hostname" that appears on the screen and double-check the number with the one that appears on the Tough Pad.
 - 1.8 Should an ECG data transmission to the ePCR fail, restart the ePCR Toughpad (using the Microsoft Windows Start Menu Restart option) and the LP15, enter the archives and attempt to re-transmit the ECG data again. Ensure you select the correct case from the archives.
 - 1.9 Should this fail, request assistance from the Clinical Documentation Officer (CDO). Should the CDO be unable to successfully transmit the ECG data to the ePCR, print out two complete 'Code Summary' and hand one copy to the CDO on duty, together with the relevant case identifying details (CFS and LP15 HBE number). Hand the second 'Code summary' to the receiving hospital/facility.
 - 1.10 The CDO shall complete an OVA for any transmission failures and include the CFS and LP15 HBE number in the OVA.
 - 1.11 Document in the ePCR that the ECG data failed to transmit.
 - 1.12 Notify the CDO if it is a Critical Case/Death in Care and hand the LP15 to the CDO. The CDO is to notify the Quality Team by sending an email to ASGQPSRM@hamad.qa with the CFS number and relevant case details. The CDO will retrieve the ECG data from the LP15.
 - 1.13 DO NOT open a new case until you have completed and closed the previous case. This is to prevent the wrong ECG and vital signs data being transmitted to the wrong case.
 - 1.14 It is mandatory to transmit LP15 ECG data to the Toughpad for all cases, regardless of severity.

2. Conducting the daily “User Test”:
 - 2.1 Turn on the LP15.
 - 2.2 Press the “Options” button and select “User Test”.
 - 2.3 The User Test will then be conducted and once complete will automatically print out the results of the test – either “User Test Passed” or “User Test Failed”.
 - 2.4 Should the User Test fail, return the LP15 immediately to the production supervisor and replace it with a functioning LP15 which has passed the user test.
 - 2.5 The Production Supervisor shall complete an OVA reporting the equipment failure. The OVA must include the LP15 HBE number and where the LP15 was sent for inspection.
3. If 12-Lead ECG fails to transmit to NCC:
 - 3.1 Check the modem cable is securely plugged in. Do not attempt to remove and plug the modem cable back in.
 - 3.2 If indoors, place the LP15 near a window. Press “Transmit” and ensure the selected “SITE” is “NCC”. Press “SEND” to re-attempt the transmission of the 12-Lead ECG to the NCC.
 - 3.3 If you are in a remote/rural area with poor network signal, attempt to re-transmit the 12-Lead ECG once you are mobile to hospital and in an area with a stronger network signal.
 - 3.4 Should these steps fail, notify the Clinical Desk that you are unable to transmit a 12-Lead ECG to them. Notify your Production Supervisor and arrange to have the LP15 replaced.
 - 3.5 The Production Supervisor shall complete an OVA which will include the CFS number, LP15 HBE number and where the LP15 was sent for inspection.
4. Keep the 4-lead ECG cable plugged in the LP15 at all times, regardless of whether you are using the 4-lead ECG or not.
5. Pair the ePCR Toughpad with your assigned LP15 at the start of every shift, before leaving the Hub in order to change the Toughpad if any pairing issues are identified.
6. The ePCR Toughpad is required to be restarted at the start of your shift, or earliest convenience following the correct Microsoft Windows Restart procedure. Do not do a hard shut down by holding down the Toughpad power button.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.H DISCARDING PARACETAMOL AND IBUPROFEN SYRUP
Implementation Date	28 January 2020
Purpose	To outline the process of disposing of oral paracetamol and ibuprofen syrup once opened.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Definition:

- 1.1 HMCAS supplies paracetamol and ibuprofen syrup for the treatment of pain and mild pyrexia (as per HMCAS Clinical Practice Guidelines). Paracetamol and ibuprofen syrup may be administered to adults as well as children.
- 1.2 Once opened and the initial dose is administered, the 'opened' bottle should be discarded.

2. Process:

- 2.1 Administer the correct dose as indicated by the Clinical Practice Guidelines.
- 2.2 When measuring paediatric treatment dose, use the provided measuring 'cap' (you may use the age defined treatment doses indicated on the bottle).

3. Disposal:

- 3.1 Once the bottle has been opened it must be discarded at the end of the patient treatment episode.
- 3.2 Where multiple paediatric patients are anticipated (such as during event cover) a single bottle of paracetamol and ibuprofen syrup may be used for multiple patients.
 - 3.2.1 The paracetamol and ibuprofen syrup bottle should then be discarded at the end of the event.
- 3.3 Discard by placing the bottle in to the yellow bag within the ambulance.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.I SELECTING AMBULANCE SERVICE MEDICATIONS AND EQUIPMENT
Implementation Date	25 February 2020
Purpose	To outline internal HMCAS processes for selecting medication and equipment.
Responsibility	ALL LICENSED HMCAS PARAMEDICS AND CRITICAL CARE PARAMEDICS

1. Background and Purpose:

- 1.1 This SOP outlines the process of selecting medications and equipment for the Ambulance Service.
- 1.2 HMCAS Ambulance Paramedics and Critical Care Paramedics are authorized to administer classes of medication (by DHP). However, the type, dose, and indication for the use of any medication is agreed upon by the Medical Director in association with the Head of Professions.

2. Choice of Medication:

- 2.1 Proposed medications will be identified by the Head of Professions through the following processes:
 - 2.1.1 Following clinical audit
 - 2.1.2 Following international changes in clinical practice
 - 2.1.3 In response to a request from outside agencies where this is deemed to be clinically appropriate (e.g. Heart Hospital for STEMI patients).
 - 2.1.4 Following case review
 - 2.1.5 Upon HMCAS staff request where this is deemed to be clinically appropriate.
- 2.2 New medication will be agreed with the Medical Director and where deemed appropriate the new medications will be discussed at the HMCAS Clinical Forum which is chaired by the Medical Director.
 - 2.2.1 Additional external expert advice will be sorted as deemed appropriate by the Medical Director.
- 2.3 Newly recommended medications will be signed off into clinical practice by the Ambulance Service Executive Governance Committee.

3. Clinical Practice Guidelines:

- 3.1 Clinical Practice Guidelines will be reviewed at least annually.
- 3.2 Clinical Practice Guidelines will be published on the HMCAS Learning Management System and on the ePCR tablet.

4. Ambulance Service equipment:

- 4.1 The purchase and evaluation of ambulance equipment will be overseen by the HMCAS equipment and uniform committee.
- 4.2 All new equipment will be evaluated, and appropriate feedback provided to the equipment committee and/or HMC Department of Material Management.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.3.J AIR LIFTING AND MOVING DEVICES
Implementation Date	07 September 2021
Purpose	To provide guidance on lifting and moving bariatric patients considering safety measures for both staff and patient.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. HoverJack:

- 1.1 The HoverJack is considered as an air moving and lifting device used to transport patient whose weight or girth poses a potential health risk for the paramedics responsible for the lifting or evacuating patients. The key function of the HoverJack are:
 - 1.1.1 The HoverJack device is used to lift a patient in supine position from the floor to bed or stretcher height.
 - 1.1.2 The HoverJack device can also be used to evacuate patient's downstairs in the event of an emergency.
 - 1.1.3 For patient whose weight or girth poses a potential health risk for the paramedics responsible for the lifting or evacuating said patients.

**2. HoverMatt:**

- 2.1 The HoverMatt is used to assist paramedics with patient transfers, positioning, turning and proning of patients whose weight represents a patient safety risk. During use air simultaneously escapes from the holes on the underside, reducing the force needed to move the patient by 80-90%.
- 2.2 For patient whose weight or girth poses a potential health risk for the paramedics responsible for repositioning or laterally transferring said patients.



3. **Air Supply:**

- 3.1 The Air Supply is an electric device used to inflate, with different levels, the HoverJack and the HoverMatt.



4. **Safety Precautions:**

4.1 **HoverJack:**

- 4.1.1 Make sure patient safety straps are secured before moving.
- 4.1.2 Move the HoverJack using the transport straps and/ or the transport handles located along the top perimeter.
- 4.1.3 Never use patient safety straps to pull the HoverJack, as they may tear.
- 4.1.4 When moving a patient on the inflated HoverJack, use caution and move slowly.
- 4.1.5 Additional paramedics are recommended when moving or evacuating a patient over **113kg**.
- 4.1.6 Never attempt to move a patient on an uninflated HoverJack.
- 4.1.7 Never leave a patient unattended on an inflated HoverJack.
- 4.1.8 Use only attachments and/or accessories that are authorized by HMCAS.

4.2 **HoverMatt:**

- 4.2.1 For safety always use minimum two people opposite sides during patient transfer.
- 4.2.2 Additional paramedics are recommended when moving/ transferring a patient over 340kg.
- 4.2.3 Never leave a patient unattended on an inflated device.
- 4.2.4 If possible, transfer from higher surface to a lower surface.
- 4.2.5 Ensure patient is centered on receiving equipment prior to deflation.
- 4.2.6 Ensure that transfer surfaces are as close as possible and lock all wheels.
- 4.2.7 Never attempt to move a patient on an uninflated HoverMatt.
- 4.2.8 Use only attachments and/or accessories that are authorized by HMCAS.

4.3 **Air Supply:**

- 4.3.1 Ensure that the power cord is deployed so that it is free from hazard.
- 4.3.2 Avoid blocking the air intakes of the Air Supply.
- 4.3.3 Avoid electric shock. Do not open Air Supply.
- 4.3.4 The Air Supply must only be connected to a supply main with **protective earth**.

- 4.3.5 The Air Supply is not compatible with DC power supplies.
- 4.3.6 Use only attachments and/or accessories that are authorized by HMCAS.

5. Disinfection and Maintenance:

5.1 THE HoverJack:

- 5.1.1 The HoverJack should be periodically inspected to ensure the following:
 - 5.1.1.1 All deflation valves are equipped with a red cap
 - 5.1.1.2 The red caps are intact
 - 5.1.1.3 All patient safety straps are attached
 - 5.1.1.4 Transport handles and straps are attached
 - 5.1.1.5 Inflation valves are all self-sealing with no evident leakage
 - 5.1.1.6 There are no punctures and tears
 - 5.1.1.7 Do not launder the HoverJack
 - 5.1.1.8 In between patient use, the HoverJack should be wiped down with a cleaning solution as directed by HMCAS infection control recommendations for surface cleaning.
 - 5.1.1.9 A gentle scrub brush can also be used on the affected area if necessary, then clean the area according to HMCAS cleaning protocols.

5.2 The HoverMatt:

- 5.2.1 Is a single use device - Do not launder or reuse.
- 5.2.2 Prior to use, a visual inspection should be performed on the HoverMatt to ensure that there is no visible damage.
- 5.2.3 The HoverMatt should have all of its patient safety straps and handles.
- 5.2.4 If any damage is found that would impact safe use, then DO NOT USE, and request a replacement HoverMatt.

5.3 Air Supply:

- 5.3.1 After use, the surface of the Air Supply should be wiped down with a cleaning solution as recommended by HMCAS.
 - 5.3.1.1 Do not spray cleaners/liquids directly on the Air Supply
 - 5.3.1.2 Spray the cleaning solution onto on cleaning clothe and then apply to Air Supply unit.
- 5.3.2 Prior to use visually inspect the Air Supply:
 - 5.3.2.1 To ensure the power cord is not damaged.
 - 5.3.2.2 If damage remove from service and report to production.
- 5.3.3 The air filter must be assessed quarterly, and filter cleaned.
- 5.3.4 Between uses with an airborne isolation patient, the air supply filters can be removed and disinfected or replaced if HMCAS requires.

6. Usage:

- 6.1 The airlifting and moving set (HoverJack/HoverMatt/Air Supply) are available on the MIR first response units and can be dispatched upon request.



7. Manpower:

- 7.1 A minimum of 8-10 staff should be deployed to operate the HoverJack Device.
- 7.2 If any occasion the above number of the staff not available due to manpower or space limitation safe to do, it's not safe to do the stairwell evacuation in term of patient safety first, and paramedic safety.
- 7.3 Activating and deployment of the device it will the responsibility of Senior supervisor on scene and coordination with bronze clinical.
- 7.4 Crew on scene (Delta & paramedics) must assess the scene (stairwell wide and sharp point) for the proper movement of the device.
- 7.5 The MIR unit are overall IN CHARGE of the extrication.

8. Key safety points:

- 8.1 **Maximum weight** for stairwell evacuation - **300kg. (700lbs).**
- 8.2 **Maximum weight** for vertical lift - **545 kg. (1200lbs).**
- 8.3 Activation and deployment should be for the patient weight **200kg** above.
- 8.4 When moving a patient on inflated evacuation HoverJack, use caution and move slowly.
- 8.5 Never use patient safety strap to pull evacuation HoverJack.
- 8.6 Make certain that patient safety strap is secured before moving.
- 8.7 Ensure patient is attached and stabilized while on evacuation HoverJack.
- 8.8 Do not release multiple chambers at once.
- 8.9 Ensure that patient remains centered on evacuation HoverJack and that his/ her head does not slump forward and restrict breathing during evacuation.
- 8.10 For stairwell evacuation, deflate chambers **3 & 4.**
- 8.11 **Do not launder**

7 STEPS TO SAFETY WITH THE HOVERJACK®

1. Place the deflated HoverJack® Air Patient Lift on the floor with Valve #1 against the floor.

*Check that the (4) HoverJack red deflate valve caps are screwed on properly and tightened securely.



2. To position patient on HoverJack, first log-roll HoverMatt® Air Transfer Mattress under patient. Ensure patient is centered on HoverMatt and attended.



3. Insert air supply hose into one of the two entries at the foot-end of the HoverMatt. Press the appropriate button to inflate the transfer mattress.



4. After full inflation, push HoverMatt on an angle, head or feet-first, on top of HoverJack. Ensure patient is centered on the HoverJack, then press appropriate button to stop air flow and deflate HoverMatt. Loosely secure HoverJack patient safety straps; straps will tighten during inflation.



5. To inflate HoverJack, start with Valve #1 closest to the floor. Place air supply hose nozzle over the valve, then press the appropriate button to initiate air flow. "LOOK, LISTEN, FEEL" to ensure each chamber is fully inflated before inflating next chamber. Inflate chambers 2-4, as needed, in succession using the same technique.

LOOK: The patient's body will stop rising when the chamber is fully inflated.

LISTEN: You will hear a pitch change.

FEEL: HoverJack will be hard to the touch.



6. Use transport handles to move inflated HoverJack to desired location. Before transferring patient using the HoverMatt, unbuckle HoverJack patient safety straps. Transfer patient as described in step 4. Ensure patient is centered on receiving equipment prior to deflation.



7. Follow facility's cleaning protocol, deflate HoverJack and recap the 4 deflate valves with red caps. Return system to assigned storage location.



7 STEPS TO SAFETY WITH THE HOVERMATT®

BEFORE INFLATING HOVERMATT

1. Patient is informed regarding what to expect during transfer.



2. Patient is centered and secured on HoverMatt; At least two caregivers are present to move patient.



3. Transfer surfaces are as close as possible with **brakes locked on sending and receiving surface.**



FOR TRANSFER

4. Turn on Air Supply. Insert hose into hose entry (located at bottom of HoverMatt). Inflate mattress.



Single-Patient Use HoverMatt: Insert Air Supply hose at a 45-degree angle.

Reusable HoverMatt: Insert Air Supply hose straight-on.

5. Receiving staff member is alongside of receiving surface. Staff member on sending side pushes HoverMatt at an angle, either headfirst or feet first. Once half-way across, receiving caregiver should grasp closest handles and pull to desired location.



AFTER TRANSFER

6. Ensure patient is centered on receiving surface.



7. Turn off Air Supply to deflate HoverMatt and raise side rails.



www.HoverMatt.com
800-471-2776

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Pharmacy Section
Procedure	4.3.K TRANSPORTING HMCAS MEDICATIONS AND STAFF TRAVELLING DURING DUTY.
Implementation Date	17 January 2023
Purpose	To govern the process of transporting HMCAS medications by pharmacy staff and the travelling of staff from one location to another during duty.
Responsibility	HMCAS PHARMACY STAFF

1. Definitions:

- 1.1 **Authorized vehicles for medication transportation:** Vehicles under the direct management of HMCAS or HMC Transport Department.

2. Procedure/Process:

- 2.1 HMCAS pharmacy staff may occasionally need to accompany HMCAS medications and transport them from one location to another.
- 2.2 HMCAS pharmacy staff are not permitted to use their private vehicles for neither the purpose of transporting HMCAS medications nor for the purpose of individual traveling from one location to another during duty.
- 2.3 There are cases when a staff may travel one journey without medications but may be asked to carry medications with him in his return. Therefore, private cars are not allowed for transportation during duty (unless an approval is granted from the staff superior in advance).
- 2.4 HMCAS pharmacy staff must use an authorized vehicle for medication transportation and their travelling across locations during duty.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.4 EMERGENCY UNIT DISTRIBUTION - HUB AND SPOKE MODEL
Implementation Date	01 October 2016
Purpose	To direct and coordinate HMCAS resources for public access, including Hub and Spoke distribution, standby procedure, and allocation of escalated resources.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. The HUB and SPOKE model are designed to ensure that the public have rapid access to delivery of emergency care. See [Appendix 4.4.A](#):
2. The HUB represents the locations where ambulance personnel start and end their shifts:
3. The SPOKES represent the various satellite/standby locations where HMCAS operational vehicles will be deployed:
4. **Procedure:**
 - 4.1 Spoke standby allocation is restricted to available hub resources.
 - 4.2 Vehicles must not exceed a maximum of three [3] hours of standby at any spoke point where there is no physical structure provided to accommodate staff during standby.
 - 4.3 When the call volume for a particular hub increases, the escalation to cover the spoke will be initiated by the Distribution Team Leader/Oscar NCC.
5. **Resource Escalation:**
 - 5.1 When the call volume for any particular hub increases, the escalation to cover Spoke 1 of that particular hub is as follows:
 - 5.1.1 Vehicles from Spoke 2 – 4 of that particular hub will be redeployed to cover Spoke 1.
 - 5.1.2 If these units are called away - Charlie Unit assigned to the hub will be deployed to spoke 1.
 - 5.1.3 If the Charlie unit is called away - Delta Unit assigned to this hub will be deployed to spoke 1 and On-Call Mike Units/Oscar Units will be notified.
 - 5.1.4 If Delta unit is called away On-Call Mike Units/Oscar Units will be deployed to Spoke 1, and all On-Call Executive Team will be notified.
 - 5.1.5 On-Call Executive Team will be deployed to Spoke 1, and Mike 1 notified.
 - 5.2 When less than 30% of units are available over the entire system and are not expected to be available within the next 30 minutes the following procedure should be initiated by the DTL/Oscar NCC.
 - 5.2.1 Contact the On-Call Silver Command
 - 5.2.2 Call in extra staff if approved by Silver Command On duty.
 - 5.3 When less than 10% of units are available over the entire system the following procedure will be initiated by the DTL:
 - 5.3.1 Announce low coverage situation as general broadcast on the radio.
 - 5.3.2 Hold all Priority 2 and inter-facility transfers
 - 5.4 **References:**
 - 5.4.1 [Appendix 4.4.A Emergency Unit Distribution - Current Hub and Spoke locations](#)

- 5.4.2 [Appendix 4.4.B Map of current Hub and Spoke \(Doha\)](#)
- 5.4.3 [Appendix 4.4.C Map of current Hub and Spoke \(Qatar\)](#)
- 5.4.4 [Appendix 4.4.D Unit availability escalation procedures](#)

SOP 4.5 Emergency Unit Activation



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.5.A ACTIVATION VIA CONTROL CENTRE
Implementation Date	01 October 2016
Purpose	To outline communications procedures during routine activation via the National Command Centre (NCC).
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. HMCAS units primarily operate on Operational (Distribution) channels for the dispatch of ground units and LifeFlight. The most used channels are detailed in the table below:

ASSIGNMENT	Folder	CHANNEL
HUB-1	MAIN-OPS	AS-Doha 1
HUB-2	MAIN-OPS	AS-Doha 2
HUB-4	MAIN-OPS	AS-South 1
HUB-5	MAIN-OPS	AS-South 2/AS-South 3
HUB-6	MAIN-OPS	AS-North
HUB-7	MAIN-OPS	AS-South
HUB-8	MAIN-OPS	AS-Airport
CHARLIE UNITS (CCP)	MAIN-OPS	AS-North 3
LIFEFLIGHT	MAIN-OPS	Air-Med 1/Air-Med 2
CLINICAL DESK	MAIN-OPS	Clinical 1
Coordination between Charlie and Alpha Crew	MAIN-OPS	Clinical 2
PRODUCTION TEAM LEADER (PTL) - NCC	MAIN-OPS	Production 1
PRODUCTION TEAM LEADER (UTL) - HCC	MAIN-OPS	Production 2
SAS - PTS	MAIN-OPS	AS-Tango 1
SAS - PHCC	MAIN-OPS	AS-Tango 2
SAS - IFT	MAIN-OPS	AS-Tango 3
SAS - RETRIEVAL	MAIN-OPS	AS-Tango 4
SEALINE OPERATIONS (WEEKENDS)	MAIN-OPS	AS-North 2
In case of MCI	MAJOR-1/MAJOR-2	Follow the Dispatcher's instructions
Events/MIR/Drills/Exercises/Training/Other	Other folders will be activated as needed for events, special events, MCI, and other.	Other channels will be activated as needed for events, special events, MCI, and other.

2. Response Priority Definitions:

2.1 Priority Codes and examples:

PRIORITY CODE	EXAMPLES OF UNITS DISPATCHED
Priority 1/Zulu code	Alpha Unit & CCP unit (Charlie) Priority One, moving with lights and siren.
Priority 1/Yankee code	Alpha Unit Priority One, moving with lights and siren.
Priority 2/X-ray code	Alpha Unit Priority Two, moving without lights and siren.

2.2 Responding Unit/s need to follow the instructions of the Dispatcher.

3. Responsibilities of the dispatcher:

3.1 Assign the CLOSEST unit to the incident via NJM/MDT.

- 3.2 Initiate the dispatch by calling the unit Call sign via the radio twice e.g. "Calling A1.1 - A1.1" to Confirm the CFS reception.
- 3.3 Request the unit to change their radio channel to the appropriate one, If the closest unit does not belong to the same radio channel.
- 3.4 Notify the responding unit of any additional safety and/or infection control concerns.

4. Responsibilities of the responding crew:

- 4.1 Accept the CFS via the MDT immediately
- 4.2 Answer radio **within 10 seconds** of the radio dispatcher's transmission.
- 4.3 Carefully read details in the MDT and listen to the details of the radio call and response priority as conveyed by dispatcher.
- 4.4 Confirm location details with dispatcher and if necessary, request step by step guidance.
- 4.5 Update the unit status in the MDT accordingly and use radio when necessary.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.5.B NON-CONTROL CENTRE ACTIVATION
Implementation Date	01 October 2016
Purpose	- To ensure appropriate activation of emergency services when it is requested directly by the public or external agencies. - To ensure efficient resource management via the National Command Centre (NCC).
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. HMCAS emergency vehicles may be notified of an incident/patient directly by the public or other agencies including but not limited to the police, civil defence, military, etc.
2. **ALWAYS** notify the NCC when a HMCAS vehicle is responding to an incident.
3. When staff receives an emergency call directly from any source other than the NCC, the following procedure applies:
 - 3.1 Immediately notify the dispatcher of the approximate location of the call while proceeding to the incident.
 - 3.2 Notify the NCC if there is a need to respond out of the allocated spoke area.
 - 3.3 When applicable, allow the member of public to accompany the crew in the HMCAS vehicle.
 - 3.4 If it is necessary to follow a guide (member of public or police vehicle) to the incident, give clear and concise directions to the dispatcher to ensure rapid assistance from other units responding to the incident.
 - 3.5 On arrival on scene, follow routine procedures as if you were activated by the NCC.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.5.C RADIO COMMUNICATION & REPORTING
Implementation Date	01 October 2016
Purpose	To ensure a uniform radio procedure for the communication and reporting of basic information between field crew and the Control.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. INTRODUCTION:

- 1.1 HMCAS Staff should be able to operate the radio equipment in accordance with the HMC Ambulance Service SOP
- 1.2 HMCAS staff are required to use approved radio voice procedures and strive to become proficient communicators.
- 1.3 Comprehensive and continuing training is critical to the successful use of communications equipment. All HMCAS staff (new personnel and existing staff) must be trained to use the radio equipment properly in conjunction with all the accessories.
- 1.4 It is the responsibility of the staff to report to his immediate supervisor to solicit assistance if additional training or direction is needed for the use of radio equipment.

2. BASIC OPERATING RULES:

- 2.1 **Official Language:** The Ambulance Service radio communication's official language is English. In an emergency if a language barrier exists, the operator may use his/her native language.
- 2.2 **Control of Communications:** The Communication Center has control of communications, and all units shall comply with all instructions in matters relating to the order and time of transmission. In some occasions (events/Clinical Coordination...) an operator from outside the communications can be designated as a channel controller.
- 2.3 **Recordings:** All radio communications are automatically recorded. Recordings are confidential and will be monitored/released for authorized purposes only.
- 2.4 **Care of Equipment:** HMCAS personnel assigned with a TETRA radio is solely responsible for its security and will be held accountable for any loss or damage, all operators should ensure that their radio device is:
 - 2.4.1 Kept with them and constantly monitored.
 - 2.4.2 Secured either with a belt holster or place in the trousers radio pocket or vest.
 - 2.4.3 Operational and battery charged.
 - 2.4.4 Set to the correct talk group and to a suitable volume so it can be heard at all times.
 - 2.4.5 All TETRA radios shall be accounted for at shift change by the HMCAS stores representative and be recorded in the TETRA Radio Duty Logbook in the HMCAS stores or hub.
- 2.5 **TETRA Radio Channel Allocation (see [SOP 4.5.A](#)) Radio Reporting:**
 - 2.5.1 **Arrival report:** Upon assessment of the patient, transmit the following information to the NCC Dispatcher being as concise as possible:
 - 2.5.1.1 Qatar Early Warning Score [Q.E.W.S] – Within 2 minutes if possible.
 - 2.5.1.2 Delta or Charlie assistance required or not.

- 2.5.1.3 Requirement for more Ambulances and number, if applicable.
- 2.5.1.4 Requirement for Civil Defense, Traffic Police, Life Flight etc., if applicable.
- 2.5.1.5 Before transporting the patient to hospital, update the NCC Dispatcher as follows:
 - 2.5.1.6 Unit number, destination, and ETA
 - 2.5.1.7 Patient age, sex, and nationality
 - 2.5.1.8 Level of consciousness and orientation
 - 2.5.1.9 Chief complaint and paramedic impression, including:
 - 2.5.1.9.1 Severity
 - 2.5.1.9.2 Symptoms
 - 2.5.1.9.3 Pain score on a scale of 0-10
 - 2.5.1.9.4 Mechanism of trauma/pertinent scene information
- 2.5.2 Other reports:
 - 2.5.2.1 IMIST-AMBO report ([SOP 4.8.B](#) & [4.8.L](#))
 - 2.5.2.2 METHANE report ([SOP 4.10.A](#))
 - 2.5.2.3 Reporting unit collision ([SOP 4.6](#))

2.6 **Urgency & Distress Communications Procedure:** A unit having a message of higher precedence than the routine transmission in progress may break in and suspend the transmission in the following manner:

- 2.6.1 Urgency Communications: The proword 'BREAK' spoken three times means cease transmission and listen for an incoming URGENT message. Silence will be maintained until the unit breaking-in has passed its message. In most cases this 'break' will be requested for Urgency Communications with important information of concern such as Ambulance RTA, patient condition has deteriorated with CPR in progress, Major incident,...

Example:

Unit
BREAK-BREAK-BREAK, This is Alpha one one I have important message

- 2.6.2 Distress Communications: Scenarios involving life-threatening situations to personnel or people and requiring immediate assistance will have the highest priority; the channel is to be cleared of any other transmissions not pertaining to the emergency in progress. The proword to be used is "Emergency, Emergency, Emergency" The Distress call format will be as follows:
 - 2.6.2.1 The distress signal "EMERGENCY" spoken three times;
 - 2.6.2.2 The prowords "THIS IS";
 - 2.6.2.3 The call sign of the unit in distress spoken three times.
 - 2.6.2.4 The nature of the distress and the kind of assistance required.

- 2.6.2.5 Any other important information (Exact Location, Number).
- 2.6.2.6 Radio Silence can be imposed by the Control using the proword **"SILENCE"** three time.
- 2.6.2.7 If Radio Silence was imposed, the control will lift it using the proword **"SILENCE LIFTED"**.

Example A: Distress Call:

Unit
"EMERGENCY, EMERGENCY, EMERGENCY, Control, this Alpha one three, Alpha one three, Alpha one three, We are facing an attacker armed with knife, we need police immediately. Our current location is Al Watan Petrol Station."

Example B: Radio Silence

Control
"All units except Alpha one three this is control, SILENCE, SILENCE, SILENCE out."

Example C: Lifting Radio Silence

Control
"All units this is control, SILENCE LIFTED out."

- 2.7 **Closing Down/Terminating the use of an established talk-group or channel:** In some situations, such as special events, Major Incidents, etc. When an established talk-group is no longer required and is to be closed down, notice must be given to all units on that specific talk-group/channel for closing down or terminating the use of the established talk-group. No unit should close down without prior permission from the Control.

The Proword **CLOSE DOWN** will be used

Example A: Motor GP event

Control	Unit
"All Units, All Units, All Units. THIS IS Control. CLOSE DOWN OUT. "	<i>(No reply required.)</i>

Example B: End of Major Incident

Control	Unit
"All Units, All Units, All Units. THIS IS Control. We are terminating use of CONTROL MAJOR CHANNEL	<i>(No reply required.)</i>

All units shall return to their normal operational channels OUT.”	
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2.8 **Changing a Radio/Talk Group/Channel:** In some circumstances, it may be necessary for the unit to change channels or talk groups for a certain task or assignment.

2.9 The proword **SWITCH TO** is used to direct unit(s) to change to another talk-group/channel.

2.9.1 When your unit has been instructed to change to another channel for dispatch, remain on the new channel until the call is completed.

2.9.2 On completion of the call, update the dispatchers that you will switch back to original channel and update the other dispatcher once you are in the channel.

Example:

Step A. Control directing a unit to change to a new channel

Control	Unit
“Charlie One, SWITCH TO major incident channel OVER.”	“Control, Charlie One, switching to Major Incident Channel, OVER.”

3. SHIFT CHANGE PROCEDURE FOR OPERATIONAL CREW.

3.1 At the start of the shift, the Production Supervisor/Staff announce availability on the Standby Channel with the PTL and relay the following information:

3.1.1 Unit Call Sign

3.1.2 Vehicle registration number

3.1.3 LP15 HBE number, MDT number

3.2 Change to the duty channel and announce availability and standby location to the NCC Dispatcher.

3.3 Notify the NCC Dispatcher and the Production Supervisor/PTL that you are “Out of service” or “Handing over” the vehicle to the next shift, or any emergency for vehicle or equipment failure.

4. APPENDIX A-Authorized Procedure Words (Prowords) in HMC Ambulance Service:

APPENDIX A

Authorized Procedure Words (**Prowords**) in HMC Ambulance Service

Proword or Phrase	Meaning/Definition/Instructions
ACKNOWLEDGE	Confirm that you have received and understood the message.
AFFIRMATIVE	This means YES or Correct. Normally used when a question is asked, and the reply is YES.
ALL AFTER	Used with “SAY AGAIN” Meaning to repeat parts of message after a specific word. (Example: SAY AGAIN ALL AFTER “ Street ”.)
ALL BEFORE	Used with “SAY AGAIN” To repeat parts of a message before a specific word (Example: SAY AGAIN ALL BEFORE “ Street ”.)
ALL BETWEEN	Used with “Say again” to define the part of the message to be repeated do not use unless it is related to a written message.

Proword or Phrase	Meaning/Definition/Instructions
ALL UNITS	Call signs for all units in the channel
BE ADVISED	For imparting/conveying information
BREAK, BREAK, BREAK	I have an URGENT Message, All units will immediately cease Transmission (Used only for URGENT Communications).
CLOSE DOWN	An order over the radio to close a specific channel.
CONFIRM	Have I received the following ... or Did you receive the message?
COPY	Message is heard and understood.
CORRECTION	An error has been made in last transmission. Correction transmission follows.
DISREGARD	Consider this transmission as not sent/ignore what has just been said.
EMERGENCY, EMERGENCY, EMERGENCY	I have a message of Life-threatening Importance – All other transmissions must cease.
ETA	Estimated Time of Arrival
EXERCISE, EXERCISE, EXERCISE	Used to indicate that the following message is practice/exercise and not a real situation.
FIGURES	Group of one or more numerals. The numbers are said by each numeral.
GO AHEAD	Proceed with your message
GRID	A Grid reference to follow
HOW DO YOU READ?	What is the readability of my transmission?
I SAY AGAIN	I repeat all or specified part of a message to clarify or emphasize. (Remember we don't use the word "repeat".)
I SPELL	I will spell the next word or group of words phonetically. (See Phonetic Alphabet).
I READ BACK	The following is my reply to your request to read back.
READY FOR SERVICE	A unit is available to respond.
LONG MESSAGE	What follows is a long message. This is to make the receiver ready to receive and/or write the message.
MINIMISE	Reduce transmission – to essential messages only.
RESPONDING	En route/ Responding to the call.
LOUD & CLEAR	(Used with RADIO CHECK Signal Strength). Can receive and understand transmissions
MONITOR	Listen on (Channel/talkgroup).
MORE TO FOLLOW	More information to follow.
NEGATIVE	No, or that is not correct.
NO FURTHER ASSISTANCE NEEDED	No assistance required.
NOTHING HEARD	(Used with RADIO CHECK Signal Strength). No transmission received.
ON SCENE	Arrived at the scene.
OUT	I have finished my transmission and no response is expected.

Proword or Phrase	Meaning/Definition/Instructions
OUT OF SERVICE	A unit is unavailable to respond.
OVER	My transmission ended, and I expect a response from you.
PRIORITY 1	Responding to a call with lights and siren.
PRIORITY 2	Responding to a call without lights and siren.
PROCEED	To go or to continue.
RADIO CHECK	What is my signal strength and readability?
READ BACK	Repeat all or the specified part of this message back to me exactly as received.
READABLE	(used with RADIO CHECK Signal Strength). Can receive and understand transmissions (good readability 90–100%).
RECEIVED	Message/Transmission is completed/received.
RENDEZVOUS	Meeting place of two or more responding units. Also a message to say “Go to the place or meet.”
SAY AGAIN	Meaning repeat your last transmission. Alternative way to say “COME AGAIN”. (Do not use the word “REPEAT”)
SPEAK SLOWER	Reduce the speed of transmission.
SPEAK FASTER	Increase the speed of transmission.
SILENCE, SILENCE, SILENCE	An international expression to advise that a distress situation is in progress.
SILENCE LIFTED	An international expression to indicate that the distress situation has ended.
STANDBY	Stay alert, further information to follow /Do not transmit again until called upon.
SWITCH TO	Change to a new radio channel.
THIS IS	The originator call sign of the transmitting UNITS follows.
TO	The addressee(s) immediately following is (are) addressed for action.
UNABLE TO LOCATE	Unable to find the location
UNREADABLE	Can receive but cannot understand transmissions (readability of less than 10%).
Approved 10 codes	
10 – 10	Crew arrived on scene
10 – 20	To hospital/health facility
10 – 70	Arrived at hospital/health facility
10 – 40	Available

5. Used Radio Models:

5.1 AIRBUS TH9, THR9I & TH1n



6. APPENDIX E Ambulance Service Fleet Map:

APPENDIX E Ambulance Service Fleet Map

Appendix E (HMC-AS Terra Fleet Map)																		Special Groups
999, SAS & Event Talk Groups																		
Folder 1	Folder 2	Folder 3	Folder 4	Folder 5	Folder 6	Folder 7	Folder 8	Folder 9	Folder 10	Folder 11	Folder 12	Folder 13	Folder 14	Folder 15	Folder 16	Folder 17		
EVENT-01	EVENT-02	EVENT-03	EVENT-04	EVENT-05	EVENT-06	EVENT-07	EVENT-08	EVENT-09	EVENT-10	EVENT-11	EVENT-12	EVENT-13	MAIN-OPS	MAJOR-1	MAJOR-2	SHARED	AS SPECIAL NHCC	
CONTROL-1	CONTROL-2	CONTROL-3	CONTROL-4	CONTROL-5	CONTROL-6	CONTROL-7	CONTROL-8	CONTROL-9	CONTROL-10	CONTROL-11	CONTROL-12	CONTROL-13	AS-Data 1	CONTR-MAJ-1	CONTR-MAJ-2	COMMON-1	AS SPECIAL 1 NHCC 1	
SILVER-1	SILVER-2	SILVER-3	SILVER-4	SILVER-5	SILVER-6	SILVER-7	SILVER-8	SILVER-9	SILVER-10	SILVER-11	SILVER-12	SILVER-13	AS-Data 1	GOLD-MAJ-1	GOLD-MAJ-2	COMMON-2	AS SPECIAL 2 NHCC 2	
PRODUCTION-1	PRODUCTION-2	PRODUCTION-3	PRODUCTION-4	PRODUCTION-5	PRODUCTION-6	PRODUCTION-7	PRODUCTION-8	PRODUCTION-9	PRODUCTION-10	PRODUCTION-11	PRODUCTION-12	PRODUCTION-13	AS-North	SILVER-MAJ-1	SILVER-MAJ-2	COMMON-3	AS SPECIAL 3 NHCC 3	
CLINIC-1	CLINIC-2	CLINIC-3	CLINIC-4	CLINIC-5	CLINIC-6	CLINIC-7	CLINIC-8	CLINIC-9	CLINIC-10	CLINIC-11	CLINIC-12	CLINIC-13	AS-North 2	SILVER-MED-1	SILVER-MED-2	MOD-1	AS SPECIAL 4 NHCC 4	
G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	G-Event	AS-North 3	SILVER-OPS-1	SILVER-OPS-2	MOD-2	NHCC 5	
													AS-South	SILVER-SUPP-1	SILVER-SUPP-2	MOLEMS	NHCC 6	
													AS-Data 2	SILVER-SPEC-1	SILVER-SPEC-2	MOD-SSB	NHCC 7	
													AS-South 1	BRONZ-MED-1	BRONZ-MED-2	MOD-CAF	NHCC 8	
													AS-South 2	BRONZ-OPS-1	BRONZ-OPS-2	MOD-ASF	NHCC 9	
													AS-South 3	BRONZ-SUPP-1	BRONZ-SUPP-2	MOD-ENVI	NHCC 10	
													Clinical 1	BRONZ-SPEC-1	BRONZ-SPEC-2	MOD-OSTM	NHCC 11	
													Clinical 2	PROD-MAJ-1	PROD-MAJ-2	MOD-HMVA	NHCC 12	
													AS-Targo 1	Auxiliary	Auxiliary	Sh-Drct		
													AS-Targo 2			Sh-Tfr1		
													AS-Targo 3			Sh-Tfr2		
													AS-Targo 4			Shal-Fire		
													Air-Med 1			Shal-Fire		
													Air-Med 2			Sha-medc		
													Production 1					
													Production 2					
													AS-Support					
													Mobil Doctor 1					
													Mobil Doctor 2					
													Auxiliary					

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.5.D USE OF MOBILE DATA TERMINALS
Implementation Date	01 October 2016
Purpose	To ensure efficient workflow during dispatch via Mobile Data Terminals.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Make sure the MDT is fully operational by testing the MDT is "ON" and the Username is active. Remember that MDT should be endorsed **LOGGED ON**, not **LOGGED OFF**.
2. Do not turn off or shut down the MDT especially when changing shift; simply just switch user (by log off).
3. The MDT could be restarted once a day (preferably on the evening shift) after dispatcher notification.
4. If the MDT is malfunctioning, report to the Production Supervisor to replace the MDT.
5. Login with username (use of a training username or other colleagues' username is strictly prohibited).
6. Once the MDT is fully operational, report to the PTL via the standby channel and confirm whether the MDT is linked to the assigned vehicle.
7. Once the PTL gives the "go ahead" reply, report to the control room via operational radio channel to do the radio check and report "MDT active".
8. Responders shall be advised via radio about the incoming P1 or P2 call on the MDT. If information is not received on the MDT, revert to the standard procedure.
9. If the unit has an active MDT, receive the call via the radio and simultaneously accept it through the MDT. All other updates of the unit status will be done via MDT ONLY, EXCEPT when:
 - 9.1 Calling for additional help
 - 9.2 Reporting a safety issue
 - 9.3 MDT is offline or when there is MDT failure
 - 9.4 In emergency (Requesting for specific urgent assistance, reporting the number of victims, MCI, and any status not available in the MDT).
10. At the end of the call and when becoming available, update the unit status through the MDT as "CLEARED".
11. Responsibilities of the PTL:
 - 11.1 Keep an updated record of all units with Active MDT.
 - 11.2 Update any MDT change as required.
 - 11.3 Notify the DTL and Oscar on duty with the list of units with Active MDT.
12. Responsibilities of Production/Vehicle-Control Supervisor:
 - 12.1 Ticket MDTs appropriately
 - 12.2 When necessary, change MDT from one vehicle to another vehicle.
 - 12.3 Immediately notify the PTL if any MDT is changed from one vehicle to another.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.5.E DELTA DISPATCH PROCESS
Implementation Date	25 January 2020
Purpose	To provide guidance on the dispatch criteria for the Delta units.
Responsibility	ALL HMCAS DELTA STAFF

1. The Ambulance Service Supervisor commonly named DELTA oversees delivery of Hamad Medical Corporation Ambulance Service (HMCAS) activity within his Service Delivery Area. This oversight should be reviewed against the appropriate Service Delivery Plans that match resource to demand and are consistent with the HMC corporate service delivery framework. The DELTA is required to work with colleagues from all Directorates to ensure sufficient service delivery is achieved. The DELTA is required to monitor and ensure quality of the service in his area as measured through existing and future performance indicators. The DELTA is required to take on appropriate supervisory roles during Special Events, Multiple Casualty Incidents (MCI) and other major incidents as required.
2. **The DELTA is dispatched by NCC to the following case types based on the Dispatch Matrix:**
 - 2.1 **All cases where other emergency and safety departments are deployed**, such as Civil Defense, ISF, Traffic, Police, Customs, Coast Guard.
 - 2.2 **All cases within governmental and official instances**, like Ministries, Embassies, Police Stations.
 - 2.3 **All cases with safety concern**, such as behavioral and psychiatric disorder, cases within jails, strikes, suicide attempts, and maritime accidents.
 - 2.4 **All cases with confirmed High Mechanism of Injury**, long fall, building collapse, blast injuries, trapped casualties, Human Run Over, Crushing Injuries.
 - 2.5 **All cases of undetermined status of life**, Cardiac Arrest, Agonal Breathing, Peri-Arrest status, undeniable death, entrapment.
 - 2.6 **All cases where the DELTA has been notified by TETRA radio that the DELTA will be the initial responding unit.**
3. **The DELTA should monitor the following:**
 - 3.1 All Yankee cases with predefined medical complaint and/or traumatic injury.
 - 3.2 All Yankee cases within Health Care Facilities: Hospitals, Health Centers, Private Clinics.
 - 3.3 All Yankee cases where his response time would exceed the scene time allocated to the dispatched unit.
4. **The DELTA should self-dispatch to the following incidents:**
 - 4.1 Whenever DELTA backup is requested.
 - 4.2 Whenever the DELTA witnesses any emergency status.
5. **The DELTA should DIVERT from their initial dispatch, as follows:**
 - 5.1 Whenever their presence on scene would be beneficial for HMCAS service delivery to the patient, to the patient's family, or to any incident involving interference from other emergency agencies.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.5.F CANCELLATION OF CHARLIE OR LIFEFLIGHT
Implementation Date	07 September 2021
Purpose	Procedure to cancel a Critical Care Paramedic Unit or LifeFlight.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Background and Purpose:

- 1.1 This SOP provides guidance for the cancellation of the responding Charlie unit or LifeFlight. The following procedure must be followed when an on-scene unit needs to cancel a Charlie unit or LifeFlight.

2. Acronyms:

- 2.1 CCP: Critical Care Paramedic
 2.2 AP: Ambulance Paramedic
 2.3 ECG: Electrocardiogram
 2.4 CC Emergency: Clinical Coordinator: Emergency
 2.5 AirMed 1: Primary TETRA Radio frequency for LifeFlight
 2.6 Clinical 1: Primary TETRA Radio frequency for CC: Emergency
 2.7 Clinical 2: TETRA Radio frequency used for coordinating a rendezvous with CCP

3. Cancellation of a responding Charlie unit:

- 3.1 When the transporting unit requires cancellation of the responding CCP unit, the unit must notify the dispatcher of their intention change to Clinical 2. The dispatcher must acknowledge the request and ask the responding CCP unit to switch to Clinical 2 in accordance with [SOP 4.8.L](#).
- 3.2 The transporting unit must ensure that a full set of vital signs has been completed and provide an IMIST-AMBO report to the responding CCP unit on Clinical 2. The responding CCP will decide to continue to the scene, set a rendezvous point, or stand down from the case if the CCP determines that CCP assistance is not required.
- 3.2.1 Where a CCP is required, the transporting unit and Charlie unit must remain on Clinical 2 to coordinate a rendezvous point.
- 3.2.2 Where the on-scene unit have completed a clinical assessment and determines that a CCP is not required, the on-scene unit must contact their dispatcher and advise the dispatcher that Charlie is not required.
- 3.3 The dispatcher must acknowledge the request and cancel and assign the responding CCP unit as available.
- 3.4 The on-scene unit should return to their original dispatch channel and transport the patient to receiving facility avoiding unnecessary delays in transport.

4. Cancellation of LifeFlight:

- 4.1 Any on-scene CCP or Delta may directly cancel LifeFlight via the regular dispatch channel. However, all other unit must consult with CC: Emergency for the cancellation of LifeFlight.
- 4.2 LifeFlight must be cancelled as soon as possible when it is identified that LifeFlight is not required either for LifeFlight capabilities or for the Critical Care resources.

- 4.2.1 The on-scene unit must notify the dispatcher of their intent to change to Clinical 1, and the dispatcher must acknowledge the request to switch channels.
 - 4.2.2 The on-scene unit must endorse the patient to CC: Emergency, using the IMIST-AMBO handover and include information on the patient's location.
- 4.3 The CC: Emergency must account for the following criteria before confirming the request to cancel LifeFlight:
 - 4.3.1 Geographic location of the patient and need for retaining resources in the area.
 - 4.3.2 Whether transport by air would provide clinical benefit to the patient.
 - 4.3.3 The possible deterioration of the patient and the patient requiring a CCP transport.
- 4.4 If it is agreed that LifeFlight should be cancelled:
 - 4.4.1 The on-scene unit must return to their regular dispatch channel after the consultation and advise the dispatcher that LifeFlight is cancelled.
 - 4.4.2 The CC: Emergency must immediately update the LifeFlight Coordinator who will cancel LifeFlight over AirMed 1 and update the CFS notes.
- 4.5 Where LifeFlight will continue to the case:
 - 4.5.1 The on-scene unit must switch to AirMed 1 and maintain radio communication with the Flight Coordinator and LifeFlight crew.
 - 4.5.2 The CC: Emergency must immediately advise the LifeFlight Coordinator of the updated incident and clinical details.
 - 4.5.3 The LifeFlight Coordinator must update the CFS notes and update the LifeFlight crew on the rendezvous location and clinical update.

SOP 4.6: Emergency Unit Response



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.6.A DRIVING OF EMERGENCY VEHICLES
Implementation Date	01 October 2016
Purpose	- To ensure safe and lawful driving behaviour when responding to emergencies, as well during routine driving and parking. - To ensure safe and lawful driving behaviour during transportation of patients.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Before moving the emergency vehicle, ensure that **ALL** occupants of the emergency vehicle are buckled-in safely, including strapping/buckling-in of the patient on the stretcher.
2. Fasten vehicle doors properly to ensure they do not open whilst the vehicle is on the move. **NEVER** move an ambulance vehicle whilst any doors are open.
3. Secure equipment properly to avoid injuries to passengers.
4. Ensure that the rear step of the ambulance is in the 'up' position before moving off.
5. Proceed with due caution to other road users and adhere to safe driving practices at all times especially whilst responding to emergencies.
6. Never drive PRIORITY-1 unless trained in PRIORITY-1 driving techniques.
7. Use emergency warning devices [lights and sirens] only for calls that fit criteria classified as a medical emergency [Priority-1], as determined by the dispatch matrix.
8. In extreme low-resource situations, the on-duty Executive Team member may authorize vehicles to function with lights and sirens in non-Priority-1 situations.
9. As applicable you may use two types of Priority 1 driving; Priority 1 Response and Priority 1 Transport.
10. Priority-2 driving implies normal driving behaviour, following all road rules and excludes the use of emergency lights and sirens.
11. When possible, the driver should not operate the handheld radio whilst driving.
12. **Priority-1 Response Driving Guideline:**
 - 12.1 These guidelines should be followed for all response driving to a priority one case.
 - 12.2 An emergency vehicle **DOES NOT** have the automatic right to proceed through a red traffic light across traffic.
 - 12.3 By operating emergency lights and sirens, the HMCAS vehicle requests the permission of other motorists for the right of way.
 - 12.4 Report to the NCC any significant delays in response due to congested traffic.
 - 12.5 Ambulances **MUST** come to a **complete stop** precisely in the following two positions when approaching a **red traffic signal**:
 - 12.5.1 **At the pedestrian lane** at any intersection or crossing.
 - 12.5.2 **At the yellow box.**
 - 12.5.2.1 When directed by a law enforcement officer.
 - 12.5.2.2 Red traffic lights and STOP signs. If in a quiet suburb with high visibility, the driver may yield at STOP signs **ONLY IF SAFE TO DO SO**.

12.5.2.3 When the driver cannot account for all lanes of traffic when crossing an intersection.

12.5.2.4 When other intersection hazards are present.

12.5.2.5 When encountering a stopped bus, due to the risk of alighting passengers.

12.5.2.6 When there are any other potential hazards or adverse conditions.

12.6 Ambulances **are not permitted** to use the **contraflow lane** unless police forces are actively controlling traffic during a **Major Incident**.

12.7 Whilst responding Priority 1, the emergency response vehicle may exceed the legal posted speed limit by a maximum of 20 km/h e.g. driving a maximum of 100km/h in an 80km/h zone.

12.8 Be alert for other responding emergency vehicles. The right of way for two [2] responding emergency vehicles shall be as follows:

12.8.1 When approaching **red traffic signals or intersections**, **priority** shall be given to **Civil Defence vehicles**. Ambulance drivers should allow Civil Defence vehicles to proceed first as they are the first who secure the scene.

12.8.2 Vehicle having the right of way by traffic control devices.

12.8.3 Vehicles that will be travelling through an intersection without negotiating any turns will have preference over vehicles having to turn.

12.8.4 The first vehicle in the intersection shall have the right of way.

12.9 When following another responding vehicle, allow a following distance of 1.5 vehicle lengths per 10 km/h of speed in order to stop safely should the lead vehicle stop abruptly.

12.10 Be aware of the possibility that motorists may pull into your path after yielding to the lead vehicle[s].

13. **Vehicle safety on-scene:**

13.1 Secure a safe ambulance parking within a designated or appropriate parking space.

13.2 Ensure parking selected is able to provide safety for yourself, your patient and other road users.

13.3 Vehicles should have their engines turned off or the Run Lock system should be activated, secure by locking with key/remote and keys to remain with AP crew and the vehicle key should only be shared with HMC AS Staff.

13.4 If vehicles on-scene need to be left idling for AC and emergency lights, ensure that all vehicles doors are locked.

13.5 Prior to leaving the scene, ensure all equipment is onboard and secured and ensure all doors are appropriately locked.

13.6 Ensure all patient, all patient's escorts (HMC Staff/Family/Friends/Police, etc.) and AS staff are well secured prior to leaving the scene.

14. **Priority-1 Transportation to Hospital using Warning Devices:**

14.1 These guidelines should be followed for all driving while transporting a patient to hospital where based on the patient's condition rapid transport to hospital is the recommended action in the interest of best patient care.

14.2 **NEVER** use HMCAS vehicles to provide an escort for an ambulance conveying a critical patient to hospital.

14.3 **Adhere to all speed limits** during Priority-1 driving to hospital with a patient on board.

- 14.4 Avoid sudden harsh braking and jerking of the vehicle.
- 14.5 Drive in a way that allows for effective patient management without undue patient care disruption.
- 14.6 If there is a need to stop the ambulance to undertake any clinical procedure/intervention, do so in a safe and responsible manner (as directed by [SOP 4.8.I](#)).
- 14.7 An emergency vehicle **DOES NOT** have the automatic right to proceed through a red traffic light across traffic.
- 14.8 By operating emergency lights and sirens, the HMCAS vehicle requests the permission of other motorists for the right of way.
- 14.9 Bring the emergency vehicle to a complete stop under the following circumstances.
 - 14.9.1 When directed by a law enforcement officer.
 - 14.9.2 Red traffic lights and STOP signs. If in a quiet suburb with high visibility, the driver may yield at STOP signs **ONLY IF SAFE TO DO SO**.
 - 14.9.3 When the driver cannot account for all lanes of traffic when crossing an intersection.
 - 14.9.4 When other intersection hazards are present.
 - 14.9.5 When encountering a stopped bus, due to the risk of alighting passengers.
 - 14.9.6 When there are any other potential hazards or adverse conditions.
- 14.10 Contra-flow driving is extremely dangerous and should **NEVER** be used.
- 14.11 Be alert for other responding emergency vehicles. The right of way for two [2] responding emergency vehicles shall be as follows:
 - 14.11.1 Vehicle having the right of way by traffic control devices.
 - 14.11.2 Vehicles that will be travelling through an intersection without negotiating any turns will have preference over vehicles having to turn.
 - 14.11.3 The first vehicle in the intersection shall have the right of way.
 - 14.11.4 Be aware of the possibility that motorists may pull into your path after yielding to the lead vehicle[s].

15. Reversing/backing-up of vehicles:

- 15.1 Prior to reversing, the driver and attendant/passenger must agree a safe path for the vehicle to take.
- 15.2 The attendant/passenger shall act as guide where ever possible.
- 15.3 Do not reverse the vehicle until the guide has checked that the area into which the vehicle is to travel is clear.
- 15.4 The guide shall stand in the position from which they can best be seen and heard by the driver in their nearside mirror.
- 15.5 Verbal instructions alone are inadequate as they may be drowned by the vehicle engine or other noise. Use agreed hand signals.
- 15.6 When the guide's view is restricted from the front or side of the vehicle, he/she shall take up a position towards the rear of the vehicle and eye contact must be maintained between the driver and the guide.
- 15.7 The guide must not position themselves between the reversing vehicle and a static object (wall, post, etc.).

- 15.8 Reversing cameras MUST NOT be relied upon in isolation but are to be used as an aid to the driver.
16. In case of an RTA with an HMCAS vehicle refer to [SOP 4.6.D](#).
17. **In the case of a family/accompanying party request to upgrade the Alpha unit priority from P2 to P1 while transporting patient to hospital:**
- 17.1 The patient, family/relatives on board, family escorting unit, or any other non-HMCAS person is **NOT** allowed to demand an upgrade to P1 driving.
- 17.2 Explain to the requesting person/s that it is unnecessary to upgrade the unit to P1 since:
- 17.2.1 The patient is clinically stable;
- 17.2.2 There is no clinical benefit to rush to hospital;
- 17.2.3 There is an increased risk to safety of the patient, other passengers, the crew on board and public road users.
- 17.3 If the situation is hostile and you have no choice, immediately notify NCC dispatcher and upgrade to P1 driving with caution (as per bullet-point 13 above).
- 17.4 Immediately request for a Qatari liaison officer to contact the patient attendant to speak with the crew.
- 17.5 Request MOI assistance to arrange a police officer to meet the ambulance at the receiving facility.
- 17.6 As soon as possible, escalate the incident to immediate supervisor, AS supervisor at receiving facility and Operations Officer on shift.
- 17.7 After handover of the patient to receiving facility document the incident on the OVA system.
- 17.8 A Qatari liaison officer shall contact the family within 24 hours to reaffirm the inappropriateness/risk/dangers of the family request to upgrade to P1 driving.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.6.B PRAYER DURING HMCAS WORKING HOURS
Implementation Date	01 October 2016
Purpose	- To ensure that emergency calls are not delayed during Islamic Prayer times, whilst HMCAS staff are on active duty. - To ensure that there is ample opportunity for HMCAS Islamic staff to pray during working hours.
Responsibility	ALL HMCAS STAFF

1. HMCAS shall endeavour to provide a place of Islamic worship with adequate ablution facilities and religious paraphernalia in each of its locations.
2. **General Rules:**
3. Emergency calls have precedence over the right to pray.
4. Standby at a spoke point always takes precedence over the right to pray.
5. Priority-1 and Priority-2 calls **cannot wait** until an employee finishes his or her prayer.
6. A Priority 3 Call is not an emergency and **can wait** until an employee finishes his or her prayer.
7. A Spoke Point without Facilities is a geographically optimized response point where no physical structure is provided for HMCAS staff, thus requiring employees to remain in their vehicles whilst waiting for calls; e.g. roundabouts and petrol stations.
8. When employees are on standby at spoke points without facilities, the HMCAS Communications Centre shall attempt to allow the unit to return to a location with prayer facilities at prayer time; however operational priorities and the needs of all employees to have equal opportunity for refreshment and rest (as well as prayer) will be taken into account.
9. If an employee is interrupted while praying, every attempt will be made to allow him or her to return to a location with prayer facilities as soon as possible, taking into account operational priorities.
10. **Priority-1 and Priority-2 Calls:**
 - 10.1 If praying and your unit is dispatched, respond **immediately** to the call.
 - 10.2 Ensure that while praying, a member of HMCAS staff is monitoring the radio.
 - 10.3 If all employees at a location are praying at the same time, notify the NCC Dispatcher and monitor both the radio and mobile phones.
 - 10.4 If a call comes in during prayers the employee monitoring the radio shall immediately notify the praying crews and respond to the call without delay.
11. **Attendance of Prayers at a Mosque:**
 - 11.1 First request permission from the HMCAS Communications Centre before attending prayers at a mosque.
 - 11.2 Permission shall only be granted for employees to attend a mosque at an operationally suitable location and operational priorities will need to be considered.
 - 11.3 If monitoring of the radio is impossible, notify the HMCAS Communications Centre to contact the unit by mobile phone.
 - 11.4 Be in a position for a quick exit if praying in a mosque.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.6.C EMERGENCY RESPONSE AND NON-EMERGENCY DRIVING DURING ADVERSE WEATHER CONDITIONS
Implementation Date	01 October 2016
Purpose	To ensure safe and defensive driving principles are used when driving in adverse weather conditions.
Responsibility	ALL HMCAS OPERATIONAL CLINICAL PERSONNEL (E.G. ALPHA, BRAVO, CHARLIE, DELTA, OSCAR, MIKE, AND EVENT STAFF)

1. **During general difficult driving conditions**, where traction, vision and handling implications are likely to occur or has occurred:
 - 1.1 Inform NCC of adverse weather condition/s.
 - 1.2 Reduce speed and move over to a slow lane that will allow for safer driving conditions, keeping both hands firmly on the steering wheel.
 - 1.3 Be courteous to other road users and be mindful of road conditions, signage and posted speed limits.
 - 1.4 Increase the distance between your vehicle and the vehicle ahead, to allow for safe stopping.
 - 1.5 Obtain maximum visibility by turning on low-beam headlights. Request permission from NCC to turn on emergency lights.
 - 1.6 Keep the windows and windshield clear.
 - 1.7 Stow loose items securely and keep the dashboard free of clutter.
 - 1.8 Frequently check mirrors for approaching vehicles.
 - 1.9 Accelerate and brake as smoothly and gently as possible, avoiding sharp turns and sudden stops (improves ride quality and promotes patient safety and comfort).
2. **Driving in rain or fog conditions:**
 - 2.1 Follow all procedures for general difficult driving conditions and;
 - 2.2 Be aware of slow-moving vehicles.
 - 2.3 Be aware that vehicles using hazard lights are likely to be stationary.
 - 2.4 Avoid flooded roads and large puddles; seek an alternative route.
 - 2.5 If visibility is poor, pull over safely and completely off the road; turn on the hazard lights; wait until visibility improves and it is safe to resume driving.
 - 2.6 Avoid harsh acceleration and braking. If roads are slippery, consider using cadence braking.
3. **Driving in windy/sand storm conditions:**
 - 3.1 Follow all procedures for general difficult driving conditions and;
 - 3.2 Prepare to make adjustments in speed and steering to compensate for wind conditions.
 - 3.3 Safely pull over if necessary, to allow strong gusty winds to subside.
 - 3.4 Anticipate gusts of wind in rural areas and when overtaking, crossing culverts or bridges, or being passed by large vehicles.
 - 3.5 Be aware of slow-moving vehicles.
 - 3.6 Be aware that vehicles using hazard lights are likely to be stationary.

- 3.7 If visibility is poor, pull over safely and completely off the road; turn on the hazard lights; wait until visibility improves and it is safe to resume driving.

4. Driving in sun-glare conditions:

- 4.1 Follow all procedures for general difficult driving conditions and;
- 4.2 Use the sun visor appropriately and wear sunglasses if appropriate.
- 4.3 Use lane markers to help maintain lane discipline.
- 4.4 If visibility is poor pull over safely and completely off the road; turn on the hazard lights; wait until visibility improves and it is safe to resume driving.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.6.D RTA WHILE DRIVING EMERGENCY VEHICLES
Implementation Date	01 October 2016
Purpose	To ensure the safety of HMCAS Staff, Patients and Bystanders.
Responsibility	ALL HMCAS OPERATIONAL CLINICAL PERSONNEL (E.G. ALPHA, BRAVO, CHARLIE, DELTA, OSCAR, MIKE, AND EVENT STAFF)

1. If a HMCAS vehicle is involved in an RTA/collision the crew shall:

- 1.1 Stop the vehicle, ensure scene safety, and park the vehicle in a safe place (if possible).
- 1.2 Ensure access to the victims, treat, and facilitate transport to hospital if possible.
- 1.3 Notify NCC/HCC via current channel
- 1.4 Inform PTL, then immediate supervisor

2. Injury to staff /patient and safe transportation:

- 2.1 Injury to the staff or patient being transported.
 - 2.1.1 Assess the scene for safety issues and call NCC for help and other unit(s).
 - 2.1.2 Communicate with the opposite driver/passengers to reach **agreement** to allow transfer of patients to the hospital before proceeding to the police station.
 - 2.1.3 If the opposite driver is **not in agreement**, call for another unit(s) and stay ON SCENE until arrival of the police.
- 2.2 Provide medical treatment to the victim if possible, until another unit arrives on the scene.
- 2.3 Park the vehicle in a safe place and maintain communication with public and police officers.

3. Collision with another vehicle:

- 3.1 If there are any patients, follow **Section 2** above.
- 3.2 If there are no patients, communicate with the opposite driver to reach agreement to proceed to the police station for police report; if you do not reach agreement, wait on scene for the police after parking the vehicle in a safe place (if possible).
- 3.3 If possible, take photographs of the damage to the vehicle/s for documentation reasons.
- 3.4 Before the end of the shift, the involved staff shall;
 - 3.4.1 Complete an OVA report
 - 3.4.2 Obtain the police report
 - 3.4.3 Obtain the staff RTA investigation report
 - 3.4.4 **(Note: Police report and Staff Investigation Report should be completed within 48 hours).**

4. Collision with a fixed object:

- 4.1 Follow **Section 3** above
- 4.2 Provide a copy of your Driving License and QID to Fleet, and report to immediate supervisor with the mentioned documents.
- 4.3 **(Note: Police Report should be completed within 10 days)**

5. Post RTA procedure:

- 5.1 Staff shall be invited to the local RTA panel meeting in the event of any RTA related damage (vehicle, TETRA radio, MDT, touch screen struck, water or other unexplained damages).
- 5.2 The local RTA panel shall include representation from Operations, Production, Quality, Driver Training and Fleet Supervision.
- 5.3 The Executive RTA panel shall include representation from Operations, Production, Quality, Driver Training and Fleet Management and shall:
 - 5.3.1 Be chaired by an Assistant Executive Director.
 - 5.3.2 Confirm or adjust the Local RTA Panel Decision to meet the criteria for suspending staff from driving immediately.
 - 5.3.3 Decide internal sanctions and if necessary, recommend overall sanctions to the Executive team.

6. Situations that may result in disciplinary action:

- 6.1 Blatant disregard for [SOP 4.6.A](#)
- 6.2 Repeated “at fault” RTAs or collisions.
- 6.3 Staff may be suspended from driving in the case of serious incidents as detailed in the table below.

<i>SERIOUS INCIDENT (Stop from driving)</i>	<i>NOT SERIOUS INCIDENT</i>
Accident occurs while crossing the Red-light	Minor Damage
Failure to follow SOP	Not a mistake of staff
Accident results in injured victims	
Accidents puts vehicle out of service	
Negligent behaviour and actions	
Repeated RTA > 2 in 1 month	
Executive Panel Decision	

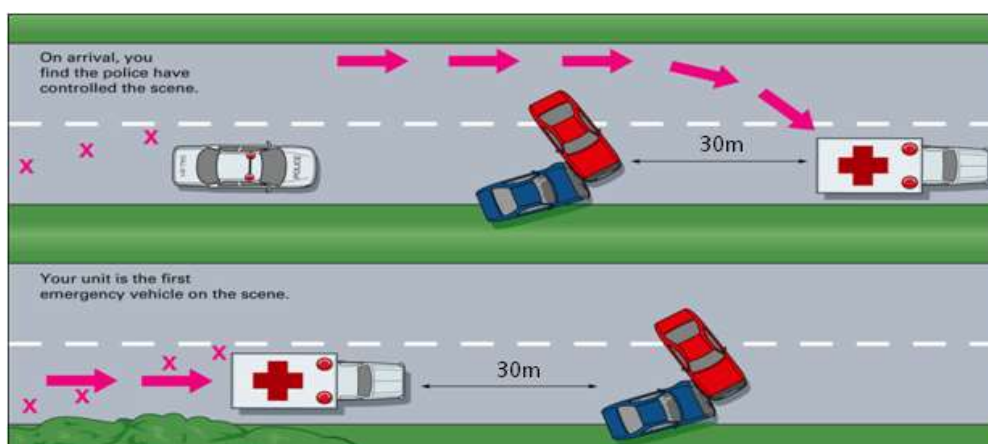
SOP 4.7: Emergency Scene Management



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.7.A EMERGENCY SCENE ARRIVAL AND SIZE-UP
Implementation Date	01 October 2016
Purpose	To ensure a safe general response of HMCAS staff to emergency calls and emergency scenes.
Responsibility	ALL HMCAS OPERATIONAL CLINICAL PERSONNEL (E.G. ALPHA, BRAVO, CHARLIE, DELTA, OSCAR, BIKE AND EVENT STAFF)

1. HMCAS staff shall base their actions and decisions on the HMCAS Code of Conduct.
2. **Arrival on an Emergency Scene:**
 - 2.1 Notify the NCC Dispatcher of arrival on scene and conduct a hazards assessment.
 - 2.2 Park the responding vehicle in a safe place, as close as possible to the point of patient contact.
 - 2.3 Identify obvious and potential hazards, and where possible, identify access and egress routes, notifying the NCC where required.
 - 2.4 If hazards exist or if there is uncertainty regarding the safety of personnel or patients, request assistance from the NCC. Use reasonable judgement based on available when electing to delay patient care.
 - 2.5 If no hazards exist that will prevent safe patient access, approach the patient ensuring appropriate Personal Protective Equipment [PPE] is worn.
 - 2.6 Carry the minimum primary response equipment which includes the Primary Response Bag, Oxygen cylinder, Suction Unit and ECG Monitor/Defibrillator and Medication Bag.
3. **Arrival on the Scene of a Road Traffic Accident (RTA):**
 - 3.1 Ensure that the vehicle's location and position provides safety while working on scene.
 - 3.2 If other agencies have secured the scene already, position the emergency vehicle past the RTA scene (or uphill/upwind in the presence of hazardous materials). This provides a safe area to load the patient/s at an already secured scene.
 - 3.3 If no other agencies have secured the scene yet, position the emergency vehicle well behind the RTA [minimum 30 meters] in the fend-off position as illustrated below.
 - 3.4 The fend-off position ensures that the emergency vehicle becomes a protective barrier to personnel and patients and deflects other vehicles that may collide into the active RTA scene.



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.7.B PREPARING A LANDING ZONE (LZ)
Implementation Date	01 October 2016
Purpose	To outline critical safety aspects for selecting and preparing a safe landing zone for helicopter emergency operations.
Responsibility	ALL HMCAS OPERATIONAL STAFF, SPECIFICALLY DISTRIBUTION SUPERVISORS

1. LifeFlight helicopter crews need assistance selecting suitable landing zones, warning pilots of potential hazards and securing landing zones during flight operations.
2. **Selecting a Daytime Landing Zone:**
 - 2.1 Determine if the area is large enough to land a helicopter safely, no smaller than 70m x 70m [Fig. 1].
 - 2.2 The surface should be flat and firm (ideally, a road). Avoid loose, powdery sand or dirt.
 - 2.3 The area should be free of any loose materials that might be blown into the rotors or engines [e.g. laundry, cardboard boxes, plywood etc.].
 - 2.4 The surface should be as flat and level as possible having a less than 10-degree slope.
 - 2.5 Landing on major highways may only take place if the road has been closed and access is controlled by the police.
3. **The Touchdown Area:**
 - 3.1 The touchdown area should be at least a 70m x 70m during daylight hours. [Fig. 1].
 - 3.2 A non-emergency personnel/bystander clearance of 100m from the edge of the LZ. [Fig. 2].
 - 3.3 The landing site must be clear of obstructions (vehicles, trees, poles, and wires). [Fig. 3].
 - 3.4 The landing site must be free of building rubble, posts, and large rocks.
 - 3.5 Electric and telephone wires are a particular hazard to helicopters, as they are much easier to see from the ground looking up than from a helicopter looking down into the ground clutter.

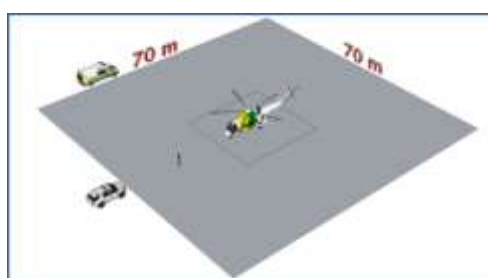


Figure 1

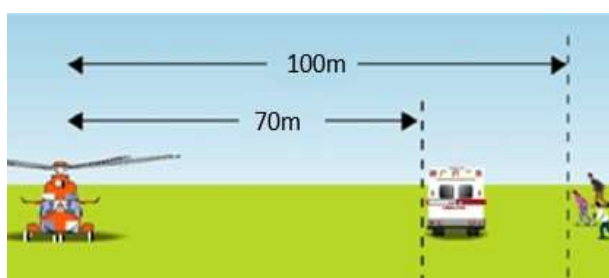


Figure 2

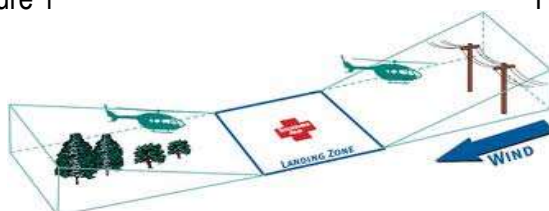


Figure 3

4. Helicopter Landing Zone Preparation:

- 4.1 Preparation of a landing site is an essential function of ground personnel. Proper preparation of a safe LifeFlight landing site should include the following:
 - 4.1.1 Clear the site of all loose objects (such as hats, paper, linen, etc.) that could be swept up in the rotor wash and sucked into the engine intakes or the rotors.
 - 4.1.2 Secure the landing area with enough ground personnel to keep all unauthorized bystanders and vehicles away.

5. Landing Zones at Night:

- 5.1 LifeFlight helicopters will only land at designated Helicopter Operating Sites [HOS] in the desert or 'no road' environments and in compliance with point 2.5 above for major highways at night.
- 5.2 Turn off unnecessary lights during the landing operation to help preserve the pilot's night vision.
- 5.3 Pilots will utilize Night Vision Goggles (NVG) for night flying operations.
- 5.4 During night flight operations, all spotlights, floodlights, and handheld lights should be pointed at the ground - NEVER at the helicopter.
- 5.5 All non-essential lights should be turned off, and any vehicle lights used to illuminate the landing zone should be on low beam.
- 5.6 LifeFlight helicopters are equipped with landing lights for night operations and will use them for landing. [Fig.4].

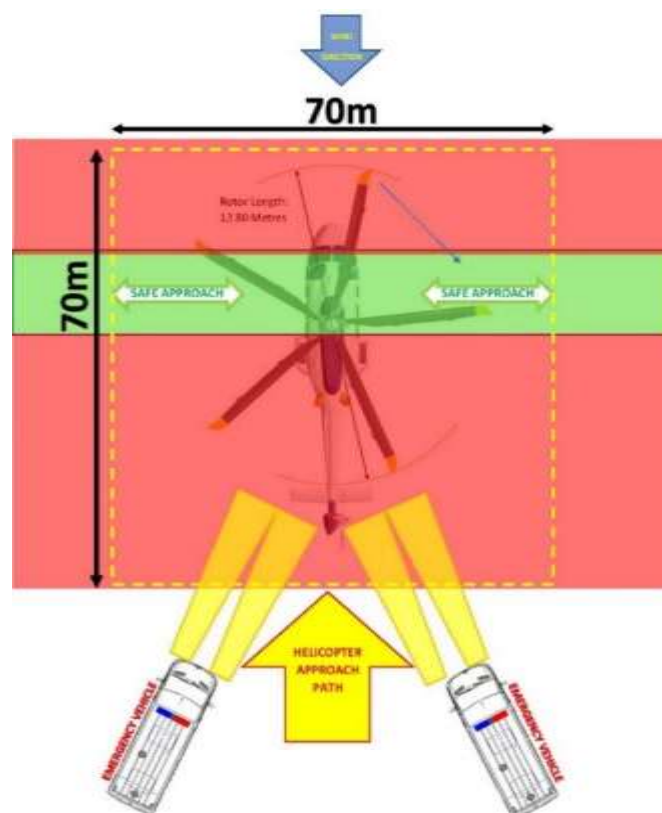


Figure 4

6. Landing Zone Coordinator Responsibilities:

- 6.1 Command and secure the LZ.
- 6.2 Establish radio contact with LifeFlight on AirMed 1 Channel.
- 6.3 Assist flight crew in locating the LZ.
- 6.4 Information required by LifeFlight Crew:
 - 6.4.1 Landmarks (Malls, Water/cell phone towers, Schools, Sport or football fields, Major Road intersections/round about, etc.).
 - 6.4.2 Type of LZ [i.e. field or road]
 - 6.4.3 LZ surface [i.e. sand, grass, concrete, gravel, dirt]
 - 6.4.4 Boundaries of the LZ [i.e. trees, houses, wires, emergency vehicles]
 - 6.4.5 Approach and departure pathways.
 - 6.4.6 Landing Zone Hazards (power lines, camel fences, vehicles, oblivious bystanders, or any obstacles that might interfere with landing).
 - 6.4.7 Notify the Flight Crew when you hear the helicopter and when you see the aircraft.
 - 6.4.8 Direct the aircraft to your location using the clock method, remembering the pilot is facing the 12 o'clock position [Fig. 5].
 - 6.4.9 Contact the flight crew after landing to determine any safety issues.



Figure 5

7. Helicopter Safe on the Ground:

- 7.1 Once the helicopter has landed, wait for the rotor blades to stop and for the pilot to signal before anyone approaches the helicopter. [Fig. 6]
- 7.2 Always approach the helicopter in full view of the pilot-between 10 and 2 o'clock of the cockpit and ensure the pilot sees you and waves you forward before approaching the helicopter. [Fig. 7].
- 7.3 Keep a crouching, low body profile to show others that you are aware of the rotor hazard.
- 7.4 If you have a helmet, secure the chin strap. Do not wear hats, caps or headscarves; avoid anything loose, such as a stethoscope around your neck.
- 7.5 Never carry anything overhead, including IV fluids.
- 7.6 Carry all long objects such as LBB's parallel to the ground.
- 7.7 Collapse stretcher IV poles before approaching the helicopter.

- 7.8 Always walk; never run, to and from a helicopter.
- 7.9 **Never** approach or depart from the uphill side if the landing zone is uneven. Always use the downhill side where you are in the pilot's view. [Fig. 6].
- 7.10 **NEVER** approach the helicopter from the rear, the LZ Coordinator is to ensure that **NO-ONE**, including flight crew, will approach the rear of the helicopter.
- 7.11 No Vehicles should approach within 50 metres of the aircraft unless directed by the flight crew.
- 7.12 Under no circumstances will any vehicle drive under the helicopter rotor blades. [Fig.9]
- 7.13 **Follow Flight Crew instructions at all times.**

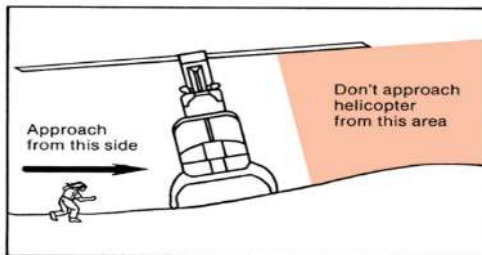


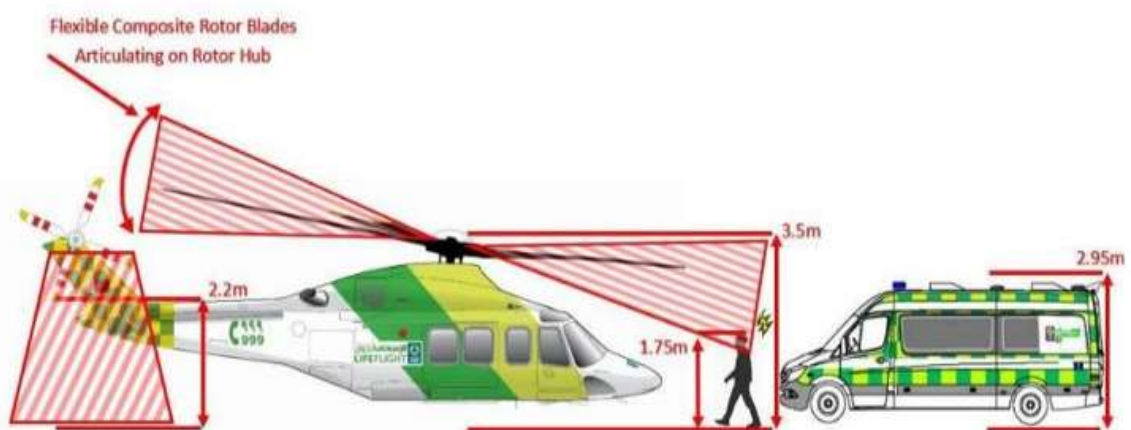
Figure 6



Figure 7



Figure 9



8. Preparing the Patient for Helicopter Transport:

- 8.1 Secure anything that may be disturbed by rotor wash.
- 8.2 Protect the patient and yourself against debris/dirt blown.
- 8.3 Use eye protection and protect exposed skin.
- 8.4 Assist the helicopter crew in securing patient to stretcher.
- 8.5 Patient should be inside the ambulance in preparation for transport to helicopter.

9. Preparing for Departure:

- 9.1 Clear all ground personnel away from the helicopter before the engine starts.
- 9.2 No one may approach the helicopter after engines have started.
- 9.3 Re-establish radio contact with the Flight Crew on AirMed 1 and confirm the LZ is secure.
- 9.4 Notify the Flight Crew immediately if an unsafe situation develops.
- 9.5 Always give the highest priority to the safety of the helicopter and its crew.

10. Designated Helicopter Operating Sites:

- 10.1 Helicopter Operating Sites (HOS) at Truck Weigh Stations will facilitate the landing of LifeFlight helicopters closer to incidents along freeways.

11. Designated HOS GPS coordinates are detailed in [Appendix 4.7.B-1](#).

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.7.C SCENE SAFETY
Implementation Date	25 February 2020
Purpose	To provide guidance on maintaining the highest level of safety while conducting on-scene operations.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. The goal of this SOP is to ensure safety of Ambulance Service staff, patients, and members of the public while on scene of each call.
2. **Approaching the scene:**
 - 2.1 When approaching the scene, the Ambulance Service crew should scan the area for hazards such as electrical power lines, spilled fluids, etc.
 - 2.2 Any identified hazards should be reported to NCC and assistance requested from Delta and/or police.
3. **Parking on scene:**
 - 3.1 All Ambulance Service vehicles will be parked in a safe area.
 - 3.2 Prior to parking on the scene, precautions for hazardous situations will be observed as follows:
 - 3.2.1 Park clear of any overhead hazards, such as electrical power lines.
 - 3.2.2 Park uphill from any spilled liquids.
 - 3.2.3 In case of fire, the crew must park a safe distance away from the scene.
 - 3.3 Do not park in a manner that will obstruct exit from the scene.
 - 3.4 The driver should attempt to park in an area where reversing is not required when parking or leaving the scene.
 - 3.5 The ambulance should be parked in a manner to allow for immediate and safe exit from the scene.
4. **On Scene Operations:**
 - 4.1 Ambulance Service crews will enter a private residence after receiving appropriate permission.
 - 4.2 While approaching the patient, the ambulance crew should be alert for any potential hazards, always making sure the scene is safe to provide effective care.
 - 4.3 **When on scene with RTA:**
 - 4.3.1 The ambulance must be parked after the site of the accident in a safe place as far from the flow of traffic as possible.
 - 4.3.2 All crew must wear a reflective safety jacket/vest if the RTA is on a public road.
 - 4.3.3 If the ambulance is parked on a public road, the emergency lights should remain on in order to warn other drivers.
 - 4.4 If the crew encounters any violent patient or family member, notify NCC immediately and retreat to a safe location until additional resources arrive.
 - 4.5 If Ambulance Service staff are given orders to perform an unsafe action or action outside of their scope of practice, the crew shall not perform such action.

- 4.5.1 If the individual continues to insist that staff take unsafe actions, notify Oscar Command at NCC, and escalate to Senior Management.
 - 4.5.2 While waiting for a Delta to arrive, every effort should be made to avoid confrontation with members of other agencies.
- 5. All Ambulance Service staff shall maintain a professional manner at all times when dealing with bystanders and family members. If there is any resistance or conflict, immediately ask for assistance and stop all unnecessary conversation.

SOP 4.8: Patient Treatment and Transportation



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.A SELECTING APPROPRIATE RECEIVING FACILITIES FOR SPECIAL CLINICAL CASES
Implementation Date	15 October 2019
Purpose	To ensure that patients are transported to the appropriate destination facility and unit in terms of presenting clinical condition.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Receiving Facility Abbreviations:
 - 1.1 **HGH-ED:** Hamad General Hospital: Emergency Department
 - 1.2 **HGH-TRU:** Hamad General Hospital: Trauma Resuscitation Unit
 - 1.3 **SIDRA:** Sidra Medicine
 - 1.4 **PEC:** Paediatric Emergency Centre
 - 1.5 **NCCCR:** National Centre for Cancer Care and Research
 - 1.6 **AWH:** Al Wakra Hospital
 - 1.7 **CH:** Cuban Hospital
 - 1.8 **HH:** Heart Hospital
 - 1.9 **AKH:** Al Khor Hospital
 - 1.10 **AAH:** Aisha Bint Hamad Al-Attiyah Hospital
 - 1.11 **HMGH:** Hazm Mebaireek General Hospital
 - 1.12 **MGH:** Mesaieed General Hospital
2. The Ambulance Service aims to bring the 'the right patient, to the right hospital at the right time.'
3. Unless there is an agreed patient specific pathway then all patients will be delivered to the nearest appropriate Emergency Department.
4. Agreed pathway patients will be taken to the appropriate receiving facilities as detailed in the table below.
5. For all trauma bypass cases, contact the Clinical Desk to confirm HGH-TRU acceptance/bypass criteria ([Appendix 4.8.A](#)).
6. **All** paediatric **medical cases** up to the age of **18 years** will be transported to SIDRA.
7. **All** paediatric **trauma cases** less than **14 years**, including those who meet **Trauma Bypass Criteria** can be taken to SIDRA. The following exceptions apply:
 - 7.1 If the patient does not meet Trauma Bypass Criteria, they are to be treated in a facility appropriate for that geographical area.
 - 7.2 In cases when an adult family member is also injured in the same incident, both patients are to be transported to HGH-TRU if they meet Trauma Bypass Criteria. In these cases, the objective is not to split the family unit (i.e. all family members should be treated at the same facility).
8. In cases of uncertainty, contact the Clinical Desk.

CLINICAL CONDITION	ADULTS	PAEDS
Near drowning and drowning WITHOUT associated/suspected trauma	HGH-ED	SIDRA
Near drowning and drowning WITH associated/suspected trauma	HGH-TRU	SIDRA
Electrocution WITHOUT associated/suspected trauma	HGH-ED	SIDRA
Electrocution WITH associated/suspected trauma	HGH-TRU	SIDRA
Envenomation	HGH-ED	SIDRA
Hanging	HGH-TRU	SIDRA
Adult patients with burns > 20% body surface area burns	HGH-TRU	SIDRA
Facial and neck burns regardless of %age of body surface area affected	HGH-TRU	SIDRA
Burns associated WITH trauma (e.g. explosion), regardless of %age of body surface area affected	HGH-TRU	SIDRA
Paediatric patients with > 10% body surface area burns	-	SIDRA
Cancer patients already attending NCCR and require urgent care	NCCR-ED	-

- 8.1 If patient or family request NCCR when the patient is stable, then the Clinical desk can contact NCCR for admission. Urgent care patients should be taken to the respective ED.

9. If the patient or patient's family member refuses to go to SIDRA, the following actions are to be completed:

- 9.1 Politely and respectfully encourage them to use SIDRA as this is a specialist pediatric facility.
- 9.2 If they continue to refuse:
- 9.2.1 Inform NCC of the refusal
- 9.2.2 Document within your ePCR
- 9.2.3 Hand this information over to the receiving facility

10. Inclusion Criteria for Direct Transfer to the HMC Heart Hospital:

- 10.1 The criteria below MUST be determined directly by a CCP or by Clinical Desk (via ECG telemetry). All CCPs/Clinical Desk are to note that only 12-lead ECGs meeting Heart Hospital admission criteria are to be transmitted to Heart Hospital. Heart Hospital and its staff are not in a position to provide online ECG advice.
- 10.1.1 Adult patients with ST elevation myocardial infarction (STEMI) identified on 12-lead ECG with supporting history with or without chest pain (refer to CPGs).
- 10.1.2 Adult patients with ST segment depression in leads V1 – V3 identified on 12-lead ECG indicative of a posterior STEMI (refer to CPGs).
- 10.1.3 Left bundle branch block (LBBB) identified on 12-lead **ECG WHERE THE CCP** believes the patients **SIGNS AND SYMPTOMS** are suggestive of a STEMI. Isolated LBBB without symptoms suggestive of a STEMI should be transported to the closest Emergency Department.
- 10.1.4 Cardiac arrest **WITNESSED** by HMCAS staff with return of spontaneous circulation (ROSC) where the pre-arrest 12-lead ECG was indicative of a STEMI.
- 10.1.5 Where patients insist on transportation to Heart Hospital, but their 12-lead ECG does not meet admission criteria, the attending clinician should advise the patient and/or

patient's family that this is not feasible and should attempt to persuade them to accept transportation to the appropriate facility. If they continue to insist, the NCC should be updated.

10.1.6 Where other healthcare service providers (e.g.: health centers, clinics etc.) insist that a patient is transported to Heart Hospital, but the patient's ECG does not meet admission criteria, the Clinical Desk is to contact Heart Hospital and confirm acceptance. If the patient has not been accepted, the discretion lies with the attending CCP regarding the final destination for the patient.

10.1.7 All other presenting arrhythmia are to be transported to the closest Emergency Department.

11. Where a receiving specialist hospital cannot receive the patient (to include pathway patients) then the patient should be taken to the nearest emergency department.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.B PATIENT HANDOVER AND ENDORSEMENT
Implementation Date	01 October 2016
Purpose	To facilitate accurate transfer of information and safe and effective transition of care within HMCAS and to healthcare facilities.
Responsibility	ALL OPERATIONAL STAFF (INCLUDING TRS DOCTORS)

1. Handover occurs every time the care for a patient is transferred from one healthcare provider to another, either within the Ambulance Service or to another healthcare facility (HCF). Every patient must be formally handed over and received through verbal and/or written communication.
2. As the treating clinician, decide which HCF (and specifically which section at the HCF) is most appropriate for the patient and communicate this information to the HMCAS staff member driving the ambulance. If in doubt, seek advice from the Dispatcher/ Clinical Desk.
3. As the treating clinician/clinical team, conduct the handover (in the case of the treating clinician being mentored then the senior HMCAS clinician will monitor the handover and correct any omissions).
4. The receiving health care facilities include, but are not limited to:
 - 4.1 Hamad General Hospital (HGH)
 - 4.2 Heart Hospital
 - 4.3 Paediatric Emergency Centres (PEC) [Al Sadd & Al Rayyan]
 - 4.4 Al Wakra Hospital
 - 4.5 Al Khor Hospital
 - 4.6 Women's' Hospital
 - 4.7 Al Amal Hospital (National Centre for Cancer Care and Research)
 - 4.8 Cuban Hospital
5. Use the following acronym to guide verbal handover ([See Figure 4.8.B-1](#)):

I	Identify the patients age, gender, and time of onset of complaint.
M	Describe the Mechanism of Injury or current Medical Complaint .
I	List the Injuries sustained (trauma patients) or information ab.
S	List the Signs and Symptoms .
T	List the Treatment given and response thereto.
A	List any Allergies the patient has.
M	List any Medications the patient is taking.
B	Summarize the patient's Background , i.e. Past Medical History.
O	Describe any Other issues , e.g. social situation, organ donation.

6. On completion of handover, ensure that the receiving healthcare provider signs the Patient Care Report.
7. **Pre-arrival notification for seriously ill or injured patients:**

- 7.1 While enroute to hospital, as the treating clinician, update the Clinical Desk with an IMIST-AMBO report, via the CLINICAL-1 radio channel, in order to facilitate early pre-notification of the hospital. *This is not necessary for planned high acuity patient transfers (TRS).*
- 7.2 Update the dispatcher to notify Romeo 1 (in the case of patients being transported to HGH) or the Clinical Desk (for all other healthcare facilities) for the following situations:
 - 7.2.1 If a representative from the HCF cannot be reached.
 - 7.2.2 If assistance (e.g. police, security, or extra personnel to help carry the patient or equipment) is required at the receiving HCF.
 - 7.2.3 If the patient carries a highly infectious disease.

8. Handover to an alternate HMCAS clinician

- 8.1 *This step is only applicable when the treating HMCAS clinician is handing over a patient to another HMCAS clinician.*
- 8.2 Responsibilities of the clinician that is receiving the patient:
 - 8.2.1 Identify self, greet, and acknowledge the primary HMCAS treating clinicians as well as the family members/friends/colleagues/bystanders.
 - 8.2.2 Indicate willingness to receive the patient handover and accept responsibility thereof.
 - 8.2.3 Facilitate the physical transfer of the patient and direct colleagues (for example, when handing a patient to LF, the instructions given by the LF personnel should be adhered to).
 - 8.2.4 Continue to be transport the patient to the nearest most appropriate HCF.
- 8.3 Responsibilities of the clinician that is handing over:
 - 8.3.1 Clearly identify self as the treating clinician.
 - 8.3.2 Introduce the patient's family/friends/colleagues to the receiving HMCAS Clinician.
 - 8.3.3 Commence with the verbal handover of the patient whilst preparing for the physical movement of the patient if necessary (for example, to the LifeFlight (LF) helicopter stretcher).
 - 8.3.4 Use the acronym "IMIST-AMBO" to guide the patient handover.

9. Ambulance Paramedic requesting assistance or advice from a Critical Care Paramedic:

- 9.1 Notify the dispatcher of your intention to switch to CLINICAL-2 radio channel, in order to speak to the CCP on the relevant Charlie unit.
- 9.2 Provide a brief **IMIST-AMBO** report to the CCP, clearly indicating your patient's condition.
- 9.3 The CCP shall give clinical advice to the AP as necessary.
- 9.4 The CCP may decide to continue responding to the scene/rendezvous point or may instruct the Alpha unit to proceed directly to hospital, if appropriate.
- 9.5 Both the Alpha unit and the Charlie unit shall return to their regular dispatch channel after the consultation has been concluded.

10. Upon arrival at the HCF, the HMCAS treating clinician shall do the following:

- 10.1 Avoid using abbreviated words during the handover.
- 10.2 Identify and verbally confirm which receiving HCF doctor or nurse will accept responsibility for the continued care of the patient.
- 10.3 Clearly identify self as the treating clinician.

- 10.4 Introduce the patient's family/friends/colleagues and commence with the verbal handover.
- 10.5 Use the acronym **IMIST-AMBO** to guide the patient handover.
- 10.6 If appropriate highlight any key social issues and/or indicate if the patient and/or patients relative has enquired about organ donation.
- 10.7 Assist the HCF doctor/nurse receiving the patient within clinical scope of practice.
- 10.8 Ensure efficient transfer of the patient to alternate monitoring and treatment devices, minimizing interruption of the clinical treatment being administered to the patient.
- 10.9 The HMCAS treating clinician should briefly re-evaluate the patient's clinical presentation during the handover. If the patient's clinical presentation or parameters differ significantly from the initial handover then this could indicate a pre-morbid state and should be highlighted to the receiving doctor/nurse and documented accordingly in the "Treatment" section of the ePCR.
- 10.10 Once the receiving HCF doctor or nurse has indicated that they are satisfied with the handover, ensure that they sign the ePCR to acknowledge receipt and responsibility for the patient. If the HCF doctor or nurse's name and HMC Staff number is illegible then request clarification.
- 10.11 Remove HMCAS equipment that is not needed and notify the NCC of unit availability accordingly.

IMIST-AMBO

Clinical Handover Protocol

A tool that enables a quick, seamless and safe patient handover

I

M

I

S

T

Identify patient name, age, gender and onset time

Mechanism/ Medical Complaint

Injuries/ Illness/ Information related to complaint

Signs and Symptoms

Treatment and Trends

A

M

B

O

Allergies

Medications

Background History

Other Information

ROLE OF TREATING PARAMEDIC:

- Review handover details before arrival at hospital
- Introduce self and announce, "Please listen for IMIST report!"
- Deliver the IMIST report uninterrupted over 20 to 30 seconds while the patient remains on the ambulance stretcher (only necessary care e.g. IPPV or CPR should continue).
- Safely move patient to hospital bed after IMIST report
- Continue with AMBO report after patient has been moved to hospital bed
- Encourage questions on completion of IMIST and again at the end of AMBO

ROLE OF EMERGENCY DEPARTMENT CLINICIANS:

- Identify ambulance team leader
- Observe "Hands off, eyes on" for 20 to 30 seconds while the patient remains on the ambulance stretcher, until the IMIST report is delivered
- Ensure handover remains interruption free
- Ask questions only between the IMIST and AMBO and upon completion of AMBO report



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.C COMMUNICATIONS FOR VERBAL ORDERS GIVEN VIA RADIO/TELEPHONE
Implementation Date	01 October 2016
Purpose	To ensure correct, consistent, and smooth communication during verbal orders via radio or telephone.
Responsibility	OPERATIONAL CLINICAL PERSONNEL (E.G. ALPHA, CHARLIE, DELTA, BIKE AND EVENT STAFF)

1. The requirement for verbal orders, within the ambulance service, is limited by the existence of the Clinical Practice Guidelines. Where a verbal order is required then a written record, within the ePCR, is required so as to provide a formal record.
2. Sender of the verbal order:
 - 2.1 Speak slowly and clearly to send an order to the receiver of the verbal order.
 - 2.2 Request 'read-back of verbal order' to confirm receipt of exact order by the receiver and thereby preventing communication errors.
3. Receiver:
 - 3.1 Write down sender's orders
 - 3.2 Repeat order back to sender to confirm that order was clearly received.
 - 3.3 Document clearly on the ePCR
 - 3.3.1 The verbal order drug name, dose route of administration.
 - 3.3.2 The name of person providing the order
4. Keep all radio communications clear, short, and concise.
5. At handover to receiving hospital staff, provide documentation confirming all treatment (including verbal orders) rendered to the patient.
6. Hand over the patient in accordance with [SOP 4.8.B](#).

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.D NON-TRANSPORT AND RELEASE OF PATIENTS
Implementation Date	01 October 2016
Purpose	To ensure that appropriate steps are taken when treatment and/or transportation of a patient is refused.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. A key principle of the Ambulance Service is to provide treatment and transportation to all persons who request it, therefore:
 - 1.1 Avoid suggesting or implying that transport to a HCF might not be necessary for a patient. Other role players (e.g. police) may also feel that further medical care is not necessary.
 - 1.2 If it is suspected that a person is abusing the Ambulance Service, discuss with a supervisor after the treatment and transportation is completed.
 - 1.3 If a patient is violent or otherwise a danger to the HMCAS personnel, then seek the assistance of the police and escalate the matter to a supervisor.
 - 1.4 Treating and transporting patients in order of priority following triage is NOT considered refusal of treatment or transport.
2. **Refusal of treatment and/or transport by a patient:**
 - 2.1 Soon after arriving at the patient's side and while simultaneously performing the functions stipulated in the HMCAS clinical practice guidelines and directives, communicate to the patient (and other role players) that transport will be offered in an ambulance or helicopter (if appropriate) to the most appropriate Health Care Facility (HCF).
 - 2.2 As the HMCAS treating clinician, when possible:
 - 2.2.1 Encourage the patient to have a clinical assessment.
 - 2.2.2 Attempt to acquire a history and a baseline set of vital signs in line with CPG 1.
 - 2.2.3 Attempt to get all the role players (especially the patient) to agree that further clinical follow-up at the HCF is the best course of action and to attempt to allay any fears or concerns that the patient (and others) may have.
 - 2.2.4 Make at least three attempts to convince the various role players that the patient should ideally be transported to an HCF. Each attempt should use a different approach (not merely repeating oneself).
 - 2.2.5 Gain the trust of the various role players by being gentle and empathetic (avoid threatening approaches or body language).
 - 2.2.6 Empower the patient by asking why they are reluctant to go to a HCF and attempt to dispel any concerns.
 - 2.2.7 Paraphrase questions and words to ensure that the patient understands what is being said.
 - 2.2.8 Remind the role players that the HMCAS staff members are merely concerned about the patient's well-being and want what is best for the patient.
 - 2.2.9 Remind the role players that the HMCAS is free of charge.
 - 2.2.10 Use an interpreter for patient communication if necessary.

- 2.2.11 Ensure that only one person at a time is talking to the patient or primary role player, to avoid anxiety and discomfort.
- 2.2.12 Crouch or kneel next to the patient to be at their eye-level (as opposed to looking down overbearingly at them).
- 2.2.13 Explain to the role players that the HMCAS staff members are also concerned about the role player's safety and liability if the patient deteriorates.
- 2.2.14 Encourage the patient and/or role players to call for an ambulance again at a later stage if the need arises if it is ultimately decided that the patient will not be transported to an HCF.
- 2.2.15 Escalate the matter to a Senior HMCAS staff member if the patient's clinical presentation suggests that there is an imminent clinical risk.
- 2.3 If the patient refuses transportation but accepts treatment, then provide the patient with that treatment, if it is clinically safe to do so, and leave the patient on scene.
- 2.4 If the patient refuses treatment but accepts transportation, then transport the patient to the appropriate facility. Clearly communicate to the receiving health facility the patient's refusal of treatment.
- 2.5 Request the patient to read the "Patient Refusal of Ambulance Service" section on the electronic Patient Care Record (ePCR) if he/she can be reasonably determined to be competent to make an informed decision to refuse further care should be. Alternatively, request a parent or primary caregiver of the patient to complete and sign the "Patient Refusal of Ambulance Service" section.
- 2.6 If a translator assisted with communicating details to the patient then that person's details (name, surname, identification/passport number) should be included in the "Assessment" or "Treatment" section of the ePCR. Ensure that this translator/witness also signs the ePCR next to their details.
- 2.7 A witness (preferably a family member) should then complete and sign at the bottom of this section.
- 2.8 Assist in answering any questions the role players may have and all correspondence with role players (including any signatures that are written on documents) should preferably be witnessed by a fellow HMCAS staff member.
- 2.9 There are some rare occasions that all of the role players will refuse to cooperate or agree to sign the ePCR. In these instances, notify the Clinical Desk.
 - 2.9.1 Escalate the situation to the consultant CCP effectiveness and Oscar access to assist.
 - 2.9.2 If consultant CCP effectiveness or Oscar access are not available to assist, then inform Oscar command accordingly and complete the ePCR; indicating that the role players refused to cooperate with the procedure.
 - 2.9.3 In the notes section of the ePCR, detail the circumstances and also the attempts that were made to try and convince the role players that the patient should go to hospital.
 - 2.9.4 At least two HMCAS staff members involved in the scene should ensure that the details in the ePCR are accurate.
 - 2.9.5 All HMCAS members involved should ensure that they are added as practitioner in the ePCR.

3. Refusal of treatment and/or transportation by a patient not competent to make decisions:

- 3.1 Discuss with the patient's representative the need for forced treatment and/or transportation.

- 3.2 If forced treatment and/or transportation is considered, then the assistance of the police should be sought, and the matter escalated urgently.
- 3.2.1 If the patient is assessed and found to be in a safe, non-urgent, stable clinical condition then leave patient in the care of their representative and request them to sign the ePCR refusal section on the patient's behalf.

4. Refusal of treatment and/or transportation by the police on behalf of the patient:

- 4.1 If the patient is assessed and found to be in a safe, non-urgent, stable clinical condition then leave the patient in the care of the police and request them to sign the ePCR refusal section on the patient's behalf.
- 4.2 If it is believed that a real and immediate threat to the patient's life or health exists, then staff should escalate the case urgently.

5. Refusal of treatment and/or transportation by a parent, spouse, or other person on behalf of the patient:

- 5.1 If the patient is assessed and found to be in a safe, non-urgent, stable clinical condition then leave the patient in the care of the responsible person and request them to sign the ePCR refusal section on the patient's behalf.
- 5.2 If it is believed that a real and immediate threat to the patient's safety, life or health exists then staff should seek the assistance of the police and escalate the case urgently.

6. Non-treatment where there are no patients; e.g. response to a minor road traffic accident:

- 6.1 Search the scene to identify all persons involved in the incident.
- 6.2 Ask each person if they require any clinical assessment, assistance, or transportation to a health care facility.
- 6.3 When no patients are identified, report this to the Communications Centre (for recording in the Call Record) and document it in the ePCR.
- 6.4 There is no need to have any person sign refusal of treatment and/or transport as there are no patients.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.E SAFEGUARDING PATIENT'S VALUABLES
Implementation Date	01 October 2016
Purpose	To ensure responsible management of possessions and valuables of patients in the care of HMCAS.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. To reduce the instances requiring the need to safeguard patient's belongings, encourage patients to look after their own valuables, unless their condition is such that they are unable to take adequate care of their belongings. [e.g. unconscious, spinal motion restricted etc.]
2. **Procedure:**
 - 2.1 Encourage the patient to leave non-essentials items at home or in the care of companion or family members.
 - 2.2 Return all identification documents [QID, Health Card, Passport etc.] received from patients or their representative to the person who provided it after the necessary data has been captured.
 - 2.3 Do not routinely hand over identification documents to the receiving facility staff.
 - 2.4 In situations where the patient's condition is such that they cannot safeguard their property, it is the responsibility of HMCAS staff to ensure the safety of those valuables.
 - 2.5 When possible, ensure valuables are handed to family and the ePCR signed as confirmation.
 - 2.6 Where patient's property [e.g. shoes or jewelry] has been removed for assessment, retain these items for safekeeping and endorse them to the receiving facility staff.
 - 2.7 List all valuables taken over on the electronic Patient Care Report [ePCR].
 - 2.8 Ensure that both ambulance crew members sign the ePCR.
 - 2.9 Place valuables into a plastic bag e.g. jewelry, seal and ensure that the valuables are hand-delivered to the receiving hospital staff.
 - 2.10 In the event that no bag is available, ensure that the valuables are hand-delivered to the receiving hospital staff.
 - 2.11 On arrival at the hospital, all valuables are to be handed over to the receiving hospital staff.
 - 2.12 Request the receiving responsible person to sign on the ePCR an acknowledgement of receipt.
 - 2.13 Document the endorsement in the free text area of the 'Assessment'/'Treatment' blocks or in the 'Notes' section on the ePCR.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.F MANAGEMENT OF PATIENTS IN POLICE CUSTODY
Implementation Date	01 October 2016
Purpose	• To ensure appropriate management of patients who are in police custody • To ensure appropriate communications with police when dealing with patients in police custody.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. If a patient that requires treatment is a suspect who needs to be taken into custody by the police, then a senior HMCAS staff member/DTL **MUST** be notified:
 - 1.1 Approach the most senior police representative near the patient, with the intention of convincing them to allow the patient to receive treatment and transportation to a Healthcare Facility (HCF).
 - 1.2 Remind the police representative that the police processes can continue enroute and at the HCF (police officials at the hospital could facilitate any additional police processes required).
 - 1.3 Encourage a police representative to accompany the patient/suspect in the back of the ambulance or follow the ambulance (at a safe distance) to the hospital.
 - 1.4 Inform the DTL and Romeo 1 that a suspect is enroute to the hospital and to please have security and/or police representatives available to assist should the need arise.
2. On some occasions the police representative might refuse to allow the HMCAS treating clinician access to the patient or do not allow the patient to be transported to hospital using HMCAS resources.
 - 2.1 Avoid treating patients in police vehicles; patients should preferably be transported and managed by HMCAS operational staff in HMCAS designated vehicles.
 - 2.2 Notify a senior HMCAS staff member and/or Clinical Desk/DTL accordingly.
 - 2.3 Explain to the police representative that the police will be assuming responsibility for care of the patient once the HMCAS resources leave the scene.
 - 2.4 Request the police representative assuming responsibility for the care of the patient/suspect to read the "Patient Refusal of Ambulance Service" section on the back of the ePCR.
 - 2.4.1 Cross out the word "Patient" in the section that reads "Name and Signature of Patient" and write "Police official" instead.
 - 2.4.2 Request the police official assuming responsibility for the patient/suspect to write his name and corporation number in the space provided and sign accordingly.
 - 2.4.3 Request a second police official to sign as witness in the "Witness" section (and include his corporation number next to his signature).
3. The police representatives may not be aware of this procedure and may be reluctant to cooperate with these steps.
 - 3.1 Approach police officials respectfully and explain that the approach is a "Standard Operating Procedure" laid out by the Hamad Medical Corporation's Ambulance Service.
 - 3.2 Notify DTL and/or a senior HMCAS staff member.
 - 3.3 Document details of the correspondence in the "Assessment"/"Treatment" section of the ePCR.
 - 3.4 If possible, include in the ePCR the names of the police representatives and the police vehicle numbers (if applicable).

3.5 To avoid conflict details, record all details in a non-threatening manner.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.G PATIENT ESCORTS/ACCOMPANIMENT
Implementation Date	01 October 2016
Purpose	To ensure that safe procedures are adhered to when family/associates of the patient ride-along in the ambulance, enroute to hospital.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. A maximum of 4 (four) persons are allowed in the back of an ambulance i.e. 1 (one) patient on the stretcher and 3 (three) other sitting persons.
2. All persons in the back of the ambulance **MUST** be wearing a seatbelt during transport.
3. Family member/s of the patient being transported may be allowed to accompany the patient unless:
 - 3.1 There is insufficient space in the back of the ambulance to accommodate the extra person/s.
 - 3.2 The care of the patient will be compromised because of limited space.
 - 3.3 The family member/s are hostile and is perceived to be an interference to patient care.
4. Do not allow accompanying persons in the ambulance to sit in the front compartment of the ambulance as this may interfere with the driving of the ambulance, especially in the case of intimidating persons.
5. Allow the mother of paediatric patients accompany the patient.
6. If a family member/patient associate is following behind the ambulance in a private vehicle, warn them to keep a safe distance behind the ambulance.
7. If the vehicle following the ambulance begins to hoot and flash headlights in an attempt to force the ambulance to drive faster, continue to follow normal driving procedures safely and report this immediately to the NCC.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services Clinical Desk
Procedure	4.8.H RENDEZVOUS WITH CRITICAL CARE PARAMEDIC
Implementation Date	01 June 2022
Purpose	To ensure appropriate and efficient resources to provide effective patient care.
Responsibility	ALL STAFF ASSIGNED TO CLINICAL DESK

1. Background:

- 1.1 To provide the best and most appropriate level of care to all patients and prevent unnecessary transport delays, all calls dispatched as ZULU code, according to the dispatch matrix, will receive a Critical Care Paramedic response.
- 1.2 The on-scene unit should provide treatment to all patients according to the Clinical Practice Guidelines and, where appropriate, transport the patient as soon as possible to rendezvous with the responding CCP (Charlie Unit or Life Flight).

2. Principles of Rendezvous Point Selection:

- 2.1 In cases where the CCP Unit will not be able to rendezvous with the transporting unit before they reach the receiving hospital the responding CCP should be stood down.
- 2.2 Choose an appropriate rendezvous point so that the transporting Unit does not stop and wait for the responding CCP. The CCP should arrive at the rendezvous point first and wait for the transporting Unit.
- 2.3 Where the transporting unit will rendezvous with a Charlie or Retrieval CCP unit:
 - 2.3.1 The agreed rendezvous point should always be in the direction of the hospital from the scene.
 - 2.3.2 The transporting unit should NOT travel in a direction away from the receiving facility to meet with the CCP unit, this practice is inefficient and delays definitive care where the transporting unit could reach the receiving facility sooner.
 - 2.3.3 Rendezvous units must not stop in a round-about as it is unsafe and leads to traffic congestion. Rather stop on the side of the round-about exit road in the direction of the hospital (see Figure 4.8.H-1 below).
 - 2.3.4 Due to limited opportunities for vehicles to change directions between rendezvous points, the responding CCP unit should not pass a rendezvous point unless it is certain that they will reach the next rendezvous point before the transporting unit does.
- 2.4 Where the transporting unit will rendezvous with LifeFlight:
 - 2.4.1 The agreed rendezvous point should be the nearest most appropriate HOS or other safe alternative landing zone as determined by the LifeFlight Coordinator in collaboration with the Pilot in Command if necessary.
 - 2.4.2 Wherever reasonably possible the selected HOS or landing zone should be in the direction of the receiving hospital; however, it is acknowledged that this is not always possible, and where necessary, the transporting unit may travel in a direction other than toward the receiving facility.
 - 2.4.2.1 The decision to travel in a direction other than toward a receiving facility to rendezvous with LifeFlight should be discussed with a Clinical Coordinator and/or the Consultant CCP: Effectiveness.

- 2.4.3 The LifeFlight Coordinator should advise the transporting unit on Airmed 1 of the selected rendezvous HOS or landing zone as soon as reasonably possible as well as the ETA of LifeFlight.
- 2.4.4 If rendezvous with LifeFlight is not possible:
 - 2.4.4.1 The LifeFlight Coordinator must inform the DTL and Clinical Coordinator as soon as possible.
 - 2.4.4.2 A Charlie or Retrieval CCP should be dispatched to rendezvous with the transporting unit if likely to reach the unit before they reach the receiving hospital.
 - 2.4.4.3 The transporting unit should proceed to the receiving facility and coordinate a rendezvous with a Charlie or Retrieval CCP.

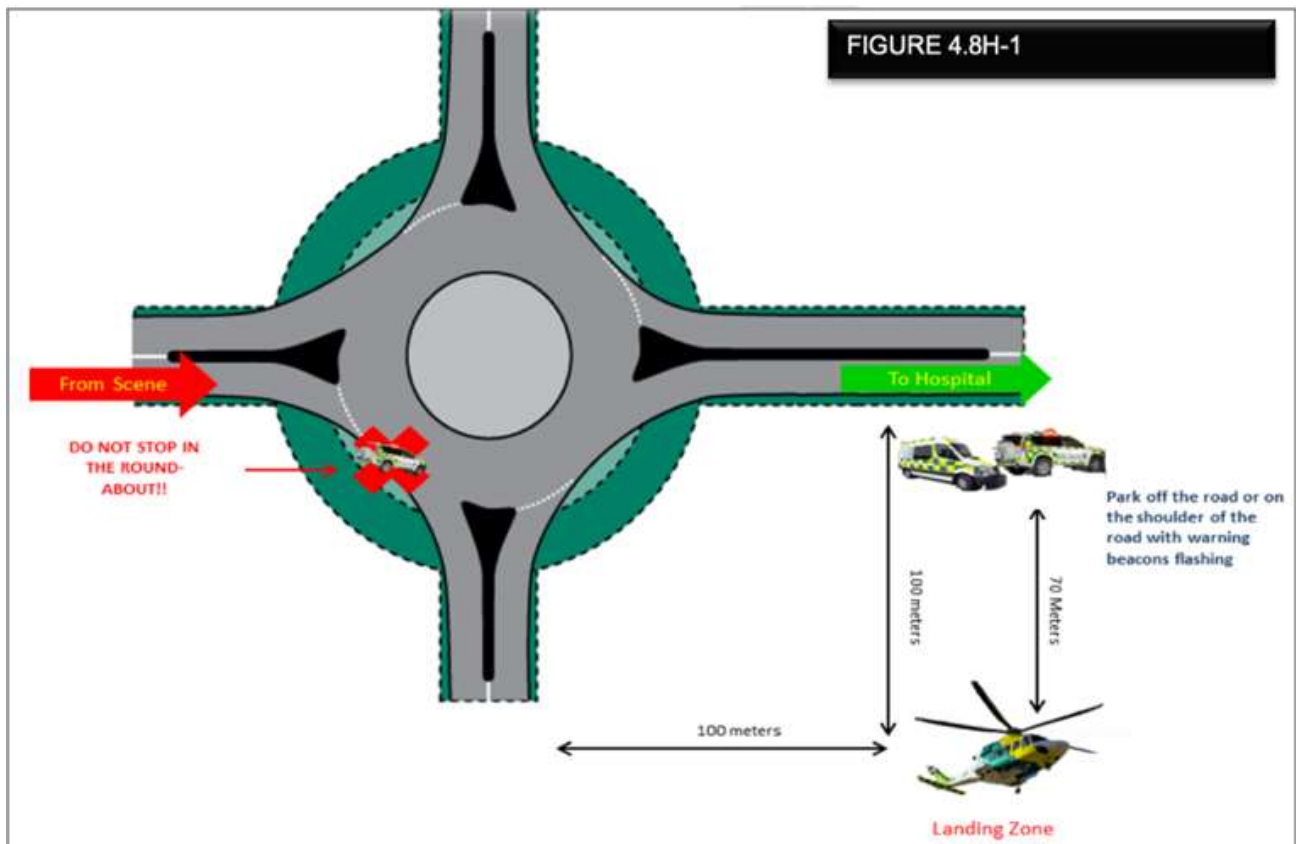
3. Rendezvous Procedure – Charlie or Retrieval CCP:

- 3.1 The on-scene crew should request CCP assistance from their dispatcher as soon as the need for a CCP is identified.
- 3.2 The transporting unit and responding CCP unit should communicate directly over Clinical 2 to identify an appropriate rendezvous point.
- 3.3 The transporting unit should maintain radio contact with the responding CCP unit over Clinical 2 until both unit's rendezvous or the rendezvous is cancelled.
- 3.4 Minimize time delays at the rendezvous point and resume transportation at the earliest possible opportunity to ensure reduced scene to hospital transport times.
- 3.5 Responding units should follow [SOP 4.6.A](#) for priority 1 driving to hospital.
- 3.6 The transporting unit should follow [SOP 4.8.I](#) if stopping for clinical procedures is necessary.
- 3.7 The transporting unit and CCP unit should update their MDT (per [SOP 3.15](#)) and dispatcher when at the rendezvous point.

4. Rendezvous Procedure – LifeFlight:

- 4.1 The on-scene crew should request CCP assistance from their dispatcher as soon as the need for a CCP is identified.
- 4.2 On dispatch of LifeFlight, the LifeFlight Coordinator should request, through the DTL, for the transporting unit to change to Airmed 1.
 - 4.2.1 If required, the LifeFlight Coordinator should request, through the DTL, for a Delta unit to be dispatched to the case.
- 4.3 The transporting unit should change over to Airmed 1 upon request of their dispatcher and remain on Airmed 1 until released by the LifeFlight Coordinator.
- 4.4 The LifeFlight Coordinator must advise the transporting unit of the HOS or landing zone location and the ETA of LifeFlight.
 - 4.4.1 If necessary, the LifeFlight Coordinator should guide the transporting and responding units to the HOS or landing zone selected for rendezvous.
- 4.5 The transporting unit should update their MDT (per [SOP 3.15](#)) and LifeFlight Coordinator when at the rendezvous point.
- 4.6 The responding and transporting units should secure the landing zone and prepare the patient in accordance with [SOP 4.7.B](#).

- 4.7 Minimize time delays at the rendezvous point and resume transportation at the earliest possible opportunity to ensure reduced scene to hospital transport times.
- 4.8 The LifeFlight Coordinator should request the transporting unit to return to their dispatch channel as soon as LifeFlight and taken-off from the HOS or landing zone.



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.I STOPPING THE AMBULANCE FOR CLINICAL PROCEDURES
Implementation Date	01 October 2016
Purpose	To ensure that safe procedures are followed when an ambulance that is enroute to hospital is required to temporarily stop to perform a clinical procedure.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. It may be necessary to temporarily stop the ambulance to perform certain interventions in time-critical patients while enroute to hospital. This is referred to as a **“Procedure Stop”**.
2. **General procedure:**
 - 2.1 Communicate to the driver, the need for a procedure stop.
 - 2.2 Identify a **safe position to stop** the ambulance.
 - 2.3 It may be necessary to obstruct traffic in order to complete a vital clinical procedure.
 - 2.4 Stop the ambulance in a safe position, with emergency lights switched on.
 - 2.5 Update the NCC Dispatcher about the need to stop the ambulance e.g. *“Alpha 5.2, Procedure stop”*.
 - 2.6 Inform the driver when the procedure is completed.
 - 2.7 Resume transport as quickly as possible, with a maximum of 3 minutes stoppage time.
 - 2.8 Update the NCC Dispatcher as soon as the ambulance is mobile again e.g. *“Alpha 5.2, Continuing to hospital”*.
3. **Examples of Procedure Stops:**
 - 3.1 Acquisition of a 12-lead ECG tracing (after setting up equipment whilst mobile).
 - 3.2 Vascular access (after setting up equipment whilst mobile).
 - 3.3 Rapid Sequence Intubation (after setting up drugs and equipment whilst mobile).
 - 3.4 Other critical interventions that may be required based on the reasonable judgement of the attending clinician.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.J EMERGENCY BLOOD TRANSFER PROTOCOL
Implementation Date	16 January 2018
Purpose	- To guide efficient communication between HMCAS Communications Centre and HGH Blood Bank. - To expedite delivery of urgent blood units/products to a receiving hospital.
Responsibility	EMDS, DTLs AND ALPHA AND DELTA UNIT STAFF

1. Hamad General Hospital Blood Bank will activate the Emergency Blood Transfer Protocol in the following manner:
 - 1.1 Call the NCC and ask for the DTL.
 - 1.2 Provide the DTL with the following information:
 - 1.2.1 'Emergency blood transfer protocol has been activated and it is not a test'
 - 1.2.2 Receiving hospital name
 - 1.2.3 Blood Bank contact name and number
2. The DTL shall ensure that the following occurs:
 - 2.1 Open a new Call for Service
 - 2.2 Enter the ProQA code – 33-C-7
 - 2.3 Dispatch an appropriate Delta unit to the Blood Bank
 - 2.3.1 Dispatch an Alpha unit for the first transportation if no Delta unit is available in reasonable proximity.
 - 2.4 Provide the Delta unit with contact person's details at the Blood Bank and receiving hospital.
 - 2.5 Update the Blood Bank and receiving hospital regarding estimated time of arrival of the HMCAS unit.
3. The dispatched unit should perform the following:
 - 3.1 Respond Priority One to the Blood Bank.
 - 3.2 Collect and sign for packaged blood products.
 - 3.3 Transport the package Priority One to the receiving hospital.
 - 3.4 Deliver and sign over the packaged blood products.
 - 3.5 Complete ePCR and attach any documentation.
4. This process may be repeated a number of times for the same patient.
 - 4.1 Subsequent transport of blood may be performed by an Alpha unit.
5. If blood spillage occurs:
 - 5.1 The unit should:
 - 5.1.1 Manage spill in the same manner as blood from a haemorrhaging patient.
 - 5.1.2 Report blood spill immediately to DTL.
 - 5.1.3 Raise an OVA for the incident.
 - 5.2 DTL should immediately ensure the following:

- 5.2.1 Inform the Blood Bank and receiving hospital.
- 5.2.2 Dispatch another Alpha or Delta unit to collect a new blood package.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.K PREVENTION AND MANAGEMENT OF PATIENT FALLS
Implementation Date	16 November 2021
Purpose	To reduce the risk of patient falls during the pre-hospital care phase.
Responsibility	ALL OPERATIONAL CLINICAL PERSONNEL

1. Identifying patients at risk of falls:

- 1.1 All patients treated/attended to by the Ambulance Service are at risk of falling and therefore be treated as such.

2. Patient Handling and Transfer Procedures for Fall Prevention:

- 2.1 Do not allow any patient to walk unnecessarily, patients clinically suitable for transport on stretcher should remain strapped and transferred into receiving facility on stretcher.
- 2.2 At least one clinician should walk with/next to any self-mobilizing patient.
- 2.3 The movement of a patient from scene (field of incident, hospital bed, and home care bed) to the ambulance should be done using an approved carrying device such as a stretcher, carry sheet, scoop stretcher, long spinal board (as an extrication device) or a stair chair.
- 2.4 Secure all patients to the ambulance stretcher using shoulder straps/chest straps, and leg straps.
 - 2.4.1 For any patient refusing any safety restraint during transport (for example, chest straps), explain all risks, document the explanation and note the patient's refusal.
- 2.5 Request assistance with carrying patients via additional Ambulance Service personnel such as Delta, Bariatric unit or a second ambulance dispatched to scene. Civil Defense can be contacted, if required, via the National Command Centre.
- 2.6 Transfer the patient from the transporting vehicle to the healthcare facility or patient residence, using HMCAS approved safe lifting and moving techniques and following all above listed points.
- 2.7 Move patients from ambulance stretcher or stair chair to hospital bed with at least three staff to ensure safe transition.
- 2.8 Clearly communicate the lifting/moving plan to all persons involved prior to moving the patient in any direction or disposition and communicate all steps to the patient, ensuring their understanding and acknowledgment of the explanation.
- 2.9 Never use the side door to move patient from stretcher to wheelchair due to high trip hazards.

3. General Processes for management of a patient that has fallen:

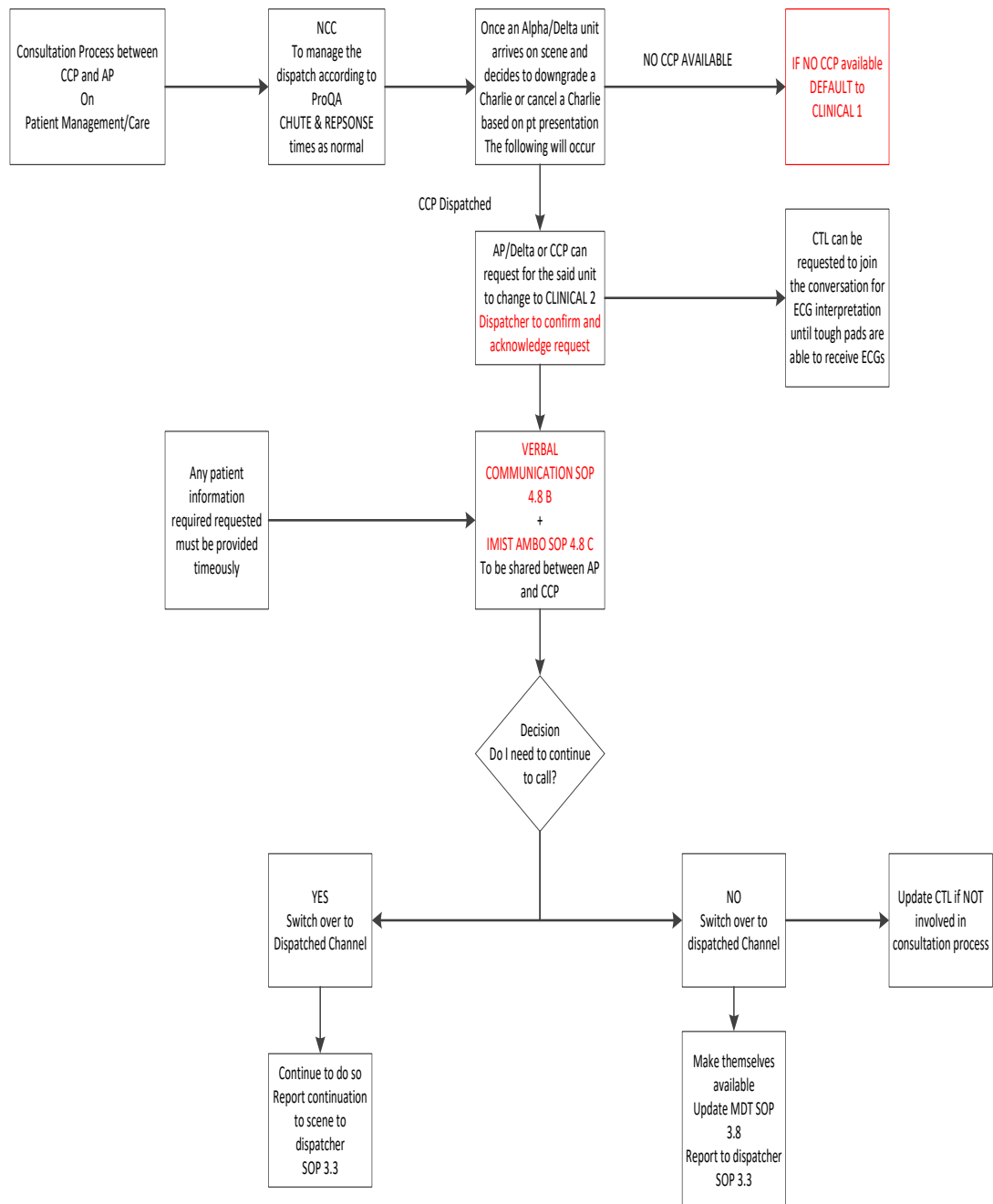
- 3.1 Assist patient immediately.
- 3.2 Request appropriate assistance if needed.
- 3.3 Notify NCC and escalate to immediate supervisor. On-call manager shall be notified by the NCC Officer on duty.
- 3.4 Treat and transport patient to the appropriate hospital and notify hospital staff of fall episode.
- 3.5 Submit an online OVA report as soon as possible.
- 3.6 Report any defective equipment as necessary.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.L UTILIZATION OF CLINICAL-2 RADIO CHANNEL
Implementation Date	05 March 2017
Purpose	To facilitate direct radio communication between the Ambulance Paramedics and Critical Care Paramedics.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. All Ambulance Paramedics (AP) and Critical Care Paramedics (CCP) are requested to use the CLINICAL-2 radio channel for communicating directly with each other when required, whilst adhering to correct radio communication etiquette.
2. The National Command Centre (NCC) shall continue to manage the dispatch process as normal.
3. Once an ALPHA or DELTA unit arrives on scene and the crew decides to either downgrade/cancel or request clinical advice from a CHARLIE unit, the following procedure shall be followed:
 - 3.1 The on-scene treating clinician (AP) shall switch and request the responding CCP to switch to the CLINICAL-2 radio channel in order to give a quick IMIST AMBO clinical report ([SOP 4.8.B](#)).
 - 3.2 The dispatcher will confirm and acknowledge receiving the request to switch channels.
 - 3.3 If the dispatcher has already downgraded or cancelled the CHARLIE unit, the CCP may request the on-scene ALPHA crew to switch to the CLINICAL-2 radio channel in order to receive a quick IMIST AMBO clinical report.
 - 3.4 The NCC Clinical Desk may be requested to join the radio conversation on the CLINICAL-2 channel for transmitted ECG interpretation.
4. After receiving the IMIST AMBO report and clarifying the condition of the patient, the CCP will:
 - 4.1 Decide to continue to the scene/rendezvous point to assist the ALPHA crew or
 - 4.2 Give clinical advice to the ALPHA crew and elect to make the CHARLIE unit available for the next Call For Service (CFS).
5. To ensure correct, consistent, and smooth communications over the CLINICAL-2 radio channel, please adhere to [SOP 4.8.C](#).
6. Utilization of Clinical 2 - Flow Pathway:

Clinical 2 - Flow Pathway:



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.M COHORTING OF PATIENTS AWAITING BEDS AT HAMAD GENERAL HOSPITAL
Implementation Date	10 October 2018
Purpose	Outline the process and procedure for cohorting patients at Hamad General Hospital when there are NO beds available to off load patients.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Introduction:

- 1.1 The function of this Standing Operational Procedure is to outline the process for cohorting patients within the four bedded holding bays at Hamad General Hospital when there are no immediate beds available to off load an ambulance trolley.
- 1.2 The premise is that 1) care will continue to be provided by HMCAS until a bed is available whilst 2) ensuring the maximum number of ambulances are available for deployment to support community calls.

2. The When:

- 2.1 Cohorting will be instigated by the HMCAS ROMEO officer (based at HGH) under the following circumstances
 - 2.1.1 Once one or more ambulances are waiting for a bed space within RIAMS and there is no bed available
 - 2.1.2 On the understanding that the selection criteria (outlined in Section 4) are adhered too and the paramedics selected to cohort have been briefed.

3. The How:

- 3.1 There must be **THREE** licensed ambulance paramedics available to monitor cohorted patients with one paramedic being present at all times. TWO of the paramedics must be active Emergency Service Ambulance Crews or a Critical Care Paramedic.
 - 3.1.1 The first ambulance crew that arrives will initially act as the supervisory cohorting team until they are released by the ROMEO (overview of key functions are listed in [Appendix 4.8.M](#)).
 - 3.1.2 The Clinical Documents Officer will act as the third supervisory clinician.
 - 3.1.2.1 The clinical documents Officer will be responsible for ensuring ePCR and/or paper PCR are up-loaded on to CERNER.
 - 3.1.2.1.1 The preferred method of record keeping will be the transfer of one ePCR record to second ePCR held by the CDO.
 - 3.1.2.1.2 A paper IMIST-AMBO form will be completed by the handing over crew as part of the handover to the Cohorting Ambulance Paramedic team.
 - 3.1.2.1.2.1 The IMIST-AMBO handover form must be completed in full.
 - 3.1.2.1.2.2 The IMIST-AMBO form Must include ALL medications administered prior to handover.
 - 3.1.2.1.3 Paper PCRs will be used at the discretion of the ROMEO officer.

- 3.2 Once handover to the Cohorting ambulance team the accompanying ambulance paramedics will:
 - 3.2.1 Add themselves as 'practitioner' on the ePCR
 - 3.2.1.1 They will document any changes in patient condition and/or additional drug administration in the ePCR
 - 3.2.1.2 Should a patient deteriorate then crew should be immediately escalated to bay 1 (or if appropriate Trauma)
 - 3.2.1.2.1 ePCR should be completed
 - 3.2.1.2.2 A paper code summary, from the LP15, should be printed and handover to the admitting hospital team.
- 3.3 A maximum of FOUR patients are cohorted at any one time UNLESS instructed, via GOLD COMMAND, that more than four patients can be cohorted.
 - 3.3.1 The maximum number of paramedic to patients is one two-person crew to **FOUR** patients plus a single Clinical Documentations Officer. Therefore, if more than four patients are to be cohorted then a second two-person ambulance crew is to be made available.
- 3.4 At all times ONE ambulance paramedic must be supervising the cohort area thereby allowing two ambulance paramedics to transport the waiting patient to an in-hospital bed space as identified by Emergency Department staff.
- 3.5 That adequate supplies of oxygen and cardiac monitoring are available.
- 3.6 Patients requiring monitoring are to be attached to a new LP15, so that the current LP15 can be released back to the ambulance once all data is transferred to the ePCR, a code summary should be printed.

4. Exclusion Criteria:

- 4.1 Patients who meet an agreed pathway are **NOT** to be cohorted
 - 4.1.1 Stroke
 - 4.1.2 STEMI
 - 4.1.3 Trauma
 - 4.1.4 QEWS > 5
- 4.2 Patients who require admission to Bay 1 are NOT to be cohorted.
- 4.3 Pregnant patients are NOT to be cohorted
- 4.4 Patients with mental health issues are NOT to be cohorted.
- 4.5 Children are NOT to be cohorted
- 4.6 Patients who have experienced a SEIZURE are NOT to be cohorted.
- 4.7 Patients requiring more than 4lpm of nasal oxygen are NOT to be cohorted.
 - 4.7.1 SpO2 for those patients receiving oxygen must be greater than 95% for non-COPD patients and greater than 88% for patients with confirmed COPD (this does not include asthma which is not a COPD diagnosis).

5. Key Roles and Functions of Staff Cohorting:

- 5.1 **Romeo**
 - 5.1.1 Ensure cohorting AP's are briefed

- 5.1.2 Ensure observation report is recorded every 30 minutes
- 5.1.3 Maximum of four patients to be cohorted unless instructed otherwise by GOLD command.
- 5.1.4 Ensure ePCR has all patients' details transferred
- 5.1.5 Ensure that IMIST-AMBO form completed
- 5.2 **Clinical Documentation Officer**
 - 5.2.1 If not wearing HMCAS identifiable uniform put on Hi-Vis tabard 'saying paramedic'.
 - 5.2.2 Take handover from ambulance crew.
 - 5.2.2.1 Ensure ePCR is transferred to your tuff-pad/note book.
 - 5.2.2.2 Ensure IMIST AMBO form is completed:
 - 5.2.2.2.1 All medication
 - 5.2.2.2.2 Complete set of vital signs
 - 5.2.2.2.3 Crews call sign, date, and time
 - 5.2.2.3 If monitoring is required attach a new LP15 allowing crew to be released.
 - 5.2.3 Ensure you remain in the area, observing patients, at all times once cohorting is commenced UNLESS you are transporting patients to an Emergency Department bed.
 - 5.2.4 Ensure you remain in the area, observing patients, at all times once cohorting is commenced.
 - 5.2.5 Ensure ePCR has all patients' details transferred.
 - 5.2.6 Ensure that IMIST-AMBO form completed
 - 5.2.7 If patients deteriorate act in accordance with CPG's and ensure events documented on ePCR.
- 5.3 **Cohorting Ambulance Team**
 - 5.3.1 If not wearing HMCAS identifiable uniform put on Hi-Vis tabard 'saying paramedic'.
 - 5.3.2 Take handover from ambulance crew.
 - 5.3.2.1 Ensure ePCR is transferred to your tuff-pad/note book.
 - 5.3.2.2 Ensure IMIST AMBO form is completed:
 - 5.3.2.2.1 All medication
 - 5.3.2.2.2 Complete set of vital signs
 - 5.3.2.2.3 Crews call sign, date, and time
 - 5.3.2.3 If monitoring is required attach a new LP15 allowing crew to be released.
 - 5.3.3 Ensure you remain in the area, observing patients, at all times once cohorting is commenced UNLESS you are transporting patients to an Emergency Department bed.
 - 5.3.4 Ensure ePCR has all patients' details transferred.
 - 5.3.5 Ensure that IMIST-AMBO form completed
 - 5.3.6 If patients deteriorate act in accordance with CPG's and ensure events documented on ePCR.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.N BROUGHT IN DEAD (BID) PROCESS PATHWAY
Implementation Date	01 October 2019
Purpose	To provide guidance on all deaths transported to hospital WHERE active resuscitation in NOT ongoing.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Definition:

- 1.1 All deceased patients where resuscitation efforts have been ceased (following consultation with a CCP) who are being transported to HMC facilities.
- 1.2 All deceased patients where HMCAS is assisting the police to remove a body from a public place to a HMC mortuary.

2. Process:

- 2.1 The deceased patient is to be transported in an Ambulance with the Air Conditioning turn on. Driving should comply normal Qatar road laws.
- 2.2 The deceased patient is to be treated with respect and dignity maintained by ensuring the body is covered at all times.
- 2.3 Where resuscitation was attempted it should be fully documented within the ePCR and subsequently the following should be recorded.
 - 2.3.1 The resuscitation procedure
 - 2.3.2 Time resuscitation ceased
 - 2.3.3 Remove all IV cannulae, LTA, and ETT
 - 2.3.4 Cover all wounds with gauze
- 2.4 Where resuscitation was NOT attempted the following should be recorded:
 - 2.4.1 History of all the events leading to the discovery of the patient.
 - 2.4.2 Physical examination of the patient to include signs of injury.
 - 2.4.3 Any medications found
 - 2.4.4 Measures taken in the management and care of the patient.
 - 2.4.5 Any forced entry by the care giver to gain access to the patient and items moved to access the patient.
 - 2.4.6 Name of Police Officer present
- 2.5 The deceased patient should be transported to the closest HMC mortuary (Hamad General Hospital, Al Wakra or Al Khor) adhering to the process outlined in the 'Brought in Dead (BID) Admission Pathway' ([Appendix 4.8.N](#)).
- 2.6 The ePCR should be completed and then signed by the mortuary clerk and/or technician once signed the ePCR should be printed and a copy left with the mortuary clerk.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.O TRANSPORTING IMMUNOCOMPROMISED/CHEMOTHERAPY PATIENTS
Implementation Date	25 February 2020
Purpose	To provide direction on the handling and transportation of immunocompromised/chemotherapy patients.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Procedure:

- 1.1 Identify patient group:
 - 1.1.1 Patients receiving chemotherapy
 - 1.1.2 Patients who EITHER 'self-identify' or are medically diagnosed as being immunocompromised.
- 1.2 Perform strict hand hygiene before and after handling patients.
- 1.3 Ambulance crew to follow hand hygiene ([SOP 2.12 Hand Hygiene](#)), apply gloves and wear a surgical face mask.
- 1.4 A surgical mask should be worn by the patient during transportation unless the patient refuses. Any refusal should be documented on the ePCR.
- 1.5 Complete patient handover to receiving facility.
- 1.6 If the patient has an associated infectious disease, follow [SOP 4.8.P](#) Transporting Patients with Suspected or known of Infectious Disease.
- 1.7 Clean and disinfect equipment after transporting the patient and perform social cleaning on ambulance surfaces.

2. References:

- 2.1 Guidelines for Care of the Immunocompromised Patient - infection control.ucsfmedicalcenter.org/.../Sec%204.1%20Immunocompromised%20Pt. Jan 2015
- 2.2 HMC Policy CL 7233 Categories of Isolation

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.P TRANSPORTING PATIENTS WITH SUSPECTED OR KNOWN INFECTIOUS DISEASE (EMERGING AND RE-EMERGING DISEASES)
Implementation Date	25 February 2020
Purpose	To provide direction on the handling and transporting patients with suspected or known Infectious Disease to ensure the safety of healthcare workers and to prevent the potential risk of infection to other patients and the general public.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Procedures:

- 1.1 National Command Centre (NCC) and Healthcare Coordination Centre (HCC) will triage calls to ascertain risk of infectious disease and will update the transporting ambulance crew accordingly.
- 1.2 Apply standard precautions including hand hygiene before and after contact of the patient and avoid touching your face.
 - 1.2.1 **Airborne Precaution:** N95 mask should be worn by crew standard surgical mask to be worn by patient.
 - 1.2.2 **Contact Precaution:** Gloves and gown should be worn by crew.
 - 1.2.3 **Droplet Precaution:** Crew to wear surgical mask and eye protection. Patient is to wear a surgical mask.
- 1.3 Family members and other contacts of infectious patients should not ride in the Ambulance. Where a family member is required to be transported with the patient (e.g. a child) then the family member should be evaluated for fever and if present the family member should be instructed to wear a surgical mask.
- 1.4 Wherever possible use vehicles that have a separate driver and patient compartment and close communicating windows between driver and patient compartments.
- 1.5 Patient compartment extraction fans should be turned on.
- 1.6 Aerosol-generating procedures (e.g. nebulizer treatment) should be minimized during transportation, unless clinical necessary.
- 1.7 NCC or HCC (as appropriate) must notify the receiving facility about the arrival of the patient and their suspected or known infectious risk.
- 1.8 For confirmed infectious cases, contact production supervisor for deep cleaning of the transporting ambulance (as per [SOP 2.4 Deep Cleaning of Patient Transporting Vehicles](#)).
- 1.9 For suspected infectious cases, contact production supervisor for deep cleaning of the transporting ambulance (as per [SOP 2.3 Social Cleaning and Simple Decontamination of Vehicles and Non-Critical Equipment](#)).
 - 1.9.1 This SOP maybe subject to change during periods of suspected outbreaks of infectious disease (e.g. MERS or COVID-19) – specific advice will be provided as directed by the Ministry of Public Health.

2. References:

- 2.1 Ambulance Services Infection Control Policy AS 9041
- 2.2 CL 7279 Infection Prevention and Control Program for Emergency Preparedness and Control for Communicable Diseases.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.Q VULNERABLE AND HIGH-RISK PATIENT TRANSPORTATION
Implementation Date	25 February 2020
Purpose	To ensure that all Operational Staff are aware of processes when managing Vulnerable and High-Risk Patients in line with HMCAS CPG, SOP's, Staff Directives/Circulars.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. All patients encountered by HMCAS are vulnerable either due to acute illness/injury and/or chronic age-related condition. Therefore, HMCAS paramedics and critical care paramedics shall care for all patients in-line with corresponding HMCAS Clinical Practice Guideline (CPG).
2. Assessment and reassessment of all patients shall be based on the clinical presentation of the patient in accordance to HMCAS CPG 1.
3. All Staff should plan care in collaboration with the patient, parent, and/or family to meet the specific needs of HMCAS patients.
4. Patient care shall be documented in the electronic Patient Care Report (ePCR) in accordance to [SOP 4.9.A](#).
5. Specific subgroups of patient represent heighten vulnerability due to their underlying medical needs and frequent contact with the ambulance service for routine transportation.
6. These patients include:
 - 6.1 Frail elderly (refer to vulnerable patient section of CPG)
 - 6.2 Paediatric (refer to paediatric section of CPG)
 - 6.3 Patients with tracheostomy (refer to vulnerable patient section of CPG)
 - 6.4 Patients receiving dialysis (refer to vulnerable patient section of CPG)
 - 6.5 Patients receiving Chemotherapy (refer to [SOP 4.8.O](#) Transporting Immunocompromised Chemotherapy Patients)

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.R TRANSPORTING PATIENTS WITH SUSPECTED OR KNOWN INFECTIOUS DISEASE IN THE GREEN BUS
Implementation Date	12 March 2020
Purpose	To give a clear direction in handling and transporting patients with a suspected or known infectious disease and to ensure the safety of healthcare workers and to prevent the potential risk of infection to other patients.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. This SOP is to provide guidance for Ambulance Service staff who are assigned to the GREEN BUS.
2. The Ambulance Service supervisor is responsible for overseeing the transportation process.
3. **Equipment:**
 - 3.1 Minimize the amount of equipment inside the green bus.
 - 3.2 Do not place equipment on the patient seats.
 - 3.3 Equipment must be placed in the rear of the vehicle.
 - 3.4 Green Bus staff must follow green bus checklists and equipment setup.
4. **Personal Protective Equipment (PPE):**
 - 4.1 The following PPE must be available:
 - 4.1.1 Gloves
 - 4.1.2 Gown
 - 4.1.3 Mask (surgical/N95)
 - 4.1.4 Eye protection
 - 4.1.5 Goggles
 - 4.1.6 Portable hand sanitizer
 - 4.1.7 Disinfectant wipes
5. **Procedure:**
 - 5.1 Only stable cases can be transported via the green bus.
 - 5.2 Suspected and confirmed cases must not be transported together on the same bus.
 - 5.3 The maximum number of patients to be transported in the green bus is FIVE (5).
 - 5.4 Patients must be seated 1 meter from each other.
 - 5.5 Full PPE must be worn prior to approaching patients.
 - 5.6 Provide a surgical mask to each patient before entering the bus.
 - 5.7 Sanitize patients' hands before entering the bus.
 - 5.8 Instruct patients to keep their masks intact, and not to touch their eyes.
 - 5.9 Keep air ventilation switched on non-recycle mode.
 - 5.10 Use yellow bags to dispose of wipes and tissues used by patients.
 - 5.11 After transporting patients, doff PPE in the designated area at each facility.

- 5.12 Perform social cleaning on both the equipment and other frequently-touched surfaces after each patient transportation, using HMC-approved disinfectant wipes.
- 5.13 Send the vehicle for deep cleaning at the end of each shift.
- 5.14 Any used equipment must be manually decontaminated with an HMC-approved disinfectant.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services Clinical Desk
Procedure	4.8.S CANCELLATION OF RESPONDING CCP UNITS
Implementation Date	15 June 2022
Purpose	To standardize and guide the procedure for cancelling responding CCP units.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Background:

- 1.1 To provide the best and most appropriate level of care to all patients and prevent unnecessary transport delays, all calls dispatched as ZULU code, according to the dispatch matrix, will receive a Critical Care Paramedic response.
- 1.2 The on-scene unit should provide treatment to all patients according to the Clinical Practice Guidelines and determine the need for a CCP based on their initial impression and clinical assessment.
- 1.3 On-scene Ambulance Paramedics should only cancel a CCP unit after performing an initial assessment.
- 1.4 Where it is determined that no CCP is required, the on-scene unit should cancel the responding CCP without delay.

2. Cancellation of Charlie or Retrieval:

- 2.1 The on-scene unit should provide a situation report to the dispatcher in accordance with Clinical Practice Guidelines and inform the dispatcher that they do not require a CCP.
- 2.2 The dispatcher must acknowledge the situation report and confirm no need for a CCP on-scene.
- 2.3 The dispatcher should then stand down the responding CCP making them available in NJM.
- 2.4 The on-scene unit is not required to change to Clinical 1 to cancel a CCP response.
- 2.5 The on-scene unit may consult with their responding CCP unit over Clinical 2 if they wish to discuss the case before cancelling the CCP.

3. Cancellation of LifeFlight:

- 3.1 If LifeFlight has been dispatched, or put on standby, and is not required, the LifeFlight unit should be cancelled without delay.
- 3.2 If the cancelling on-scene unit is an Alpha unit, cancellation should be confirmed with the Clinical Desk on Clinical 1.
 - 3.2.1 The on-scene Alpha unit must advise their dispatcher that they wish to cancel LifeFlight and will change to Clinical 1.
 - 3.2.2 The dispatcher will acknowledge the message and update the DTL.
 - 3.2.3 The Alpha Unit will change to Clinical 1 and consult the Clinical Coordinator providing a full endorsement of the patient following [SOP 4.8.B](#) and request confirmation to cancel LifeFlight.
 - 3.2.4 The Clinical Coordinator should consider the clinical risk for deterioration likely to require CCP support and if transport by LifeFlight will facilitate improved rural coverage.
 - 3.2.5 If LifeFlight is not required, as confirmed on Clinical 1:

- 3.2.5.1 The Clinical Coordinator will confirm with the unit that they will cancel LifeFlight.
 - 3.2.5.2 The Clinical Coordinator will notify the LifeFlight Coordinator to cancel LifeFlight.
 - 3.2.5.3 The LifeFlight Coordinator will update NJM and NCC DTL as soon as LifeFlight is cancelled.
 - 3.2.5.4 The on-scene unit will change back to their dispatch channel and update the dispatcher that LifeFlight has been cancelled.
- 3.3 Where the cancelling on-scene unit is a CCP, Delta, Oscar Access, or Consultant CCP: Effectiveness, LifeFlight can be cancelled without Clinical 1 consultation.
 - 3.3.1 The on-scene cancelling unit (Delta, Oscar Access, or Consultant CCP: Effectiveness) will contact the dispatcher and request LifeFlight cancellation.
 - 3.3.2 The dispatcher should immediately update the DTL and LifeFlight Coordinator to cancel LifeFlight.
 - 3.3.3 The LifeFlight coordinator will update NJM and assign the LifeFlight unit available.
 - 3.3.4 The LifeFlight Coordinator will update the Clinical Coordinator: Emergency of the cancellation.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.T REQUEST & DISPATCH OF SPECIALIZED UNIT
Implementation Date	01 January 2024
Purpose	To provide guidance on HMCAS Specialized Unit activation procedure, roles, and responsibilities.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Aim:

- 1.1 This document aims to ensure the correct dispatch of an appropriate specialized response unit when a request is made by the Ambulance Service (AS) crew on scene.

2. Definitions:**2.1 Box Type Ambulance (ICU):**

- 2.1.1 AS Bariatric Ambulance: is an ICU ambulance designed to accommodate a bariatric patient on a stretcher regardless of their clinical condition.
- 2.1.2 A Bariatric Patient: can be a patient with body width of equal to or greater than 60cm (2x ruler length).
- 2.1.3 A Bariatric Patient: is a patient whose weight is equal to or greater than 150Kg or a patient with a large body physique with BMI of >40.

$$\text{Body Mass Index (BMI)} = \frac{\text{Weight(Kg)}}{\text{Height(cm)}^2}$$

2.2 Wheelchair Ambulance (Long Vehicle):

- 2.2.1 A Wheelchair Ambulance is a wheelchair-accessible ambulance that provides non-emergency transportation.

3. Equipment:

- 3.1 Standard Ambulance equipment shall be provided by HMCAS.
- 3.2 Specialized Ambulance Type and Equipment.

Equipment	Type of Ambulance		Equipment Measurement				
	Ford Ambulance	Box Type ICU/Long Vehicle Wheelchair Ambulance	Stretcher Weight	Max Stretcher Load limit	Stretcher Width	Stretcher Length	Stretcher Height(cm)
999-Ferno Stretcher	YES	NO	36 kg	270 kg	55 cm	210 cm	High 98.7 Low 35.6
Stryker Stretcher	NO	YES (limited access)	66 Kg	318 kg	58 cm	206 cm	High 108 Low 63
Electronic Stair-Chair	NO	YES	25.67 Kg	227 kg	51.6 cm	-	95.24
Wheelchair	NO	YES	36.74 Kg	317.5kg	76.2 cm	-	-

Sliding Sheet	YES	YES	-	317.5kg	152.4 cm	203.2cm	-
Ramp	NO	YES	-	-	-	-	-

4. Process of Activation:

- 4.1 The first unit on scene assesses the patient condition then confirms and verbalizes through the radio for assistance.
- 4.2 Oscar Command should contact the unit on scene to confirm the type of assistance required ensure the correct dispatch of the specialized unit (Flow Diagram Below).
- 4.3 Directly after gathering all necessary information related to the case the Oscar Command shall contact the Healthcare Coordination Center (HCC) Supervisor on duty and provide all the information.
- 4.4 The HCC will instruct the crew of the appropriate Specialized Unit to switch the relevant radio channel and report to the NCC dispatcher for further instructions.
- 4.5 The Oscar Command should notify the DTL at NCC so that the dispatcher can dispatch and follow the appropriate resource accordingly.
- 4.6 SAS Supervisor should be dispatched to scene if no ES Supervisor or Oscar enroute/on scene already.

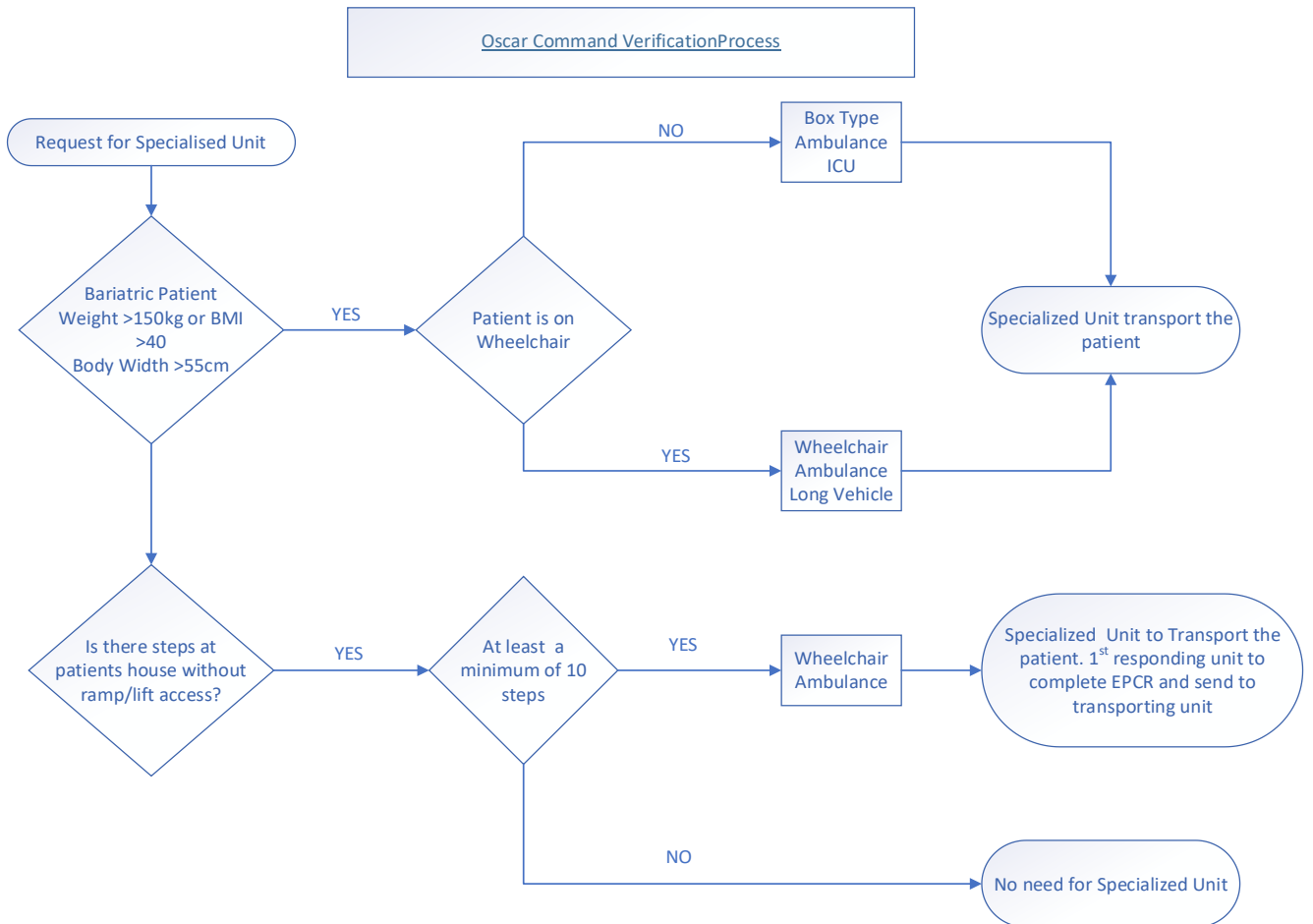
5. Criteria for Activation:

- 5.1 Box Type Ambulance:
 - 5.1.1 The Common name used to request a specialized ambulance with bariatric stretcher is **Box type Ambulance (ICU)**.
 - 5.1.2 After assessing the patient and identifying the nature of help needed, the initial unit must follow the process of activation as provided in section 4.
 - 5.1.3 After endorsing the patient and assisting in the movement of the patient to the Specialized Unit, the initial unit should book and the Specialized Unit will transport the patient to the hospital.
 - 5.1.4 First Alpha unit on scene if not transporting the patient should complete EPCR and send to the unit that will be transporting the patient.
- 5.2 Wheelchair Ambulance:
 - 5.2.1 **If the patient has his own wheelchair and insists for a wheelchair ambulance**, the initial unit can request for the Wheelchair Ambulance following the process of activation in section 4. The Wheelchair Unit assisting the patient shall transport the patient to the hospital and the first unit on scene shall book available. If no Wheelchair Ambulance available, the Alpha Unit will transport the patient on a stretcher and not delay on scene for a wheelchair ambulance.
 - 5.2.2 **If there are steps inside the patient house (10 steps) and no lift available**, the initial unit can request for Wheelchair Ambulance with Electronic Stair Chair to help them to move the patient to the ambulance safely, following the process of activation as outlined in section 4. The requesting unit should continue with the patient and the Specialized Unit book available. If no Wheelchair Ambulance available, the NCC must dispatch alternate units to help move the patient.
 - 5.2.3 **If there are 3-5 steps at the main entrance of the patient's house**, NCC should provide additional units if required to carry the patient safely.

6. References:

6.1 Refer to Addendum for Oscar Command Verification Process

Addendum A: Bariatric Verification Process



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.8.U MENTAL HEALTH TRANSFERS
Implementation Date	01 January 2024
Purpose	To provide guidance on HMCAS Mental Health Transfers between Facilities procedures, roles, and responsibilities.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Purpose:

- 1.1 Scheduled Ambulance Service will provide a triple paramedic crewed ambulance for all scheduled Mental Health inter-facility transfers from ED to the Receiving Mental Health Facility (Rawdat Al Khail or Mesaieed Hospital).

2. Scope:

- 2.1 Standard Ambulance equipment shall be provided by HMCAS. Any additional equipment must be supplied by the transferring facility, whose staff are knowledgeable/privileged about operating the equipment.
- 2.2 All HMCAS crew should adhere to the management of a Mental Health patient as mandated within the Ambulance Service CPG 7.1 Acute Behavioral Disturbance.

3. Definitions:

- 3.1 **What is Mental Health?** It is "a state of well-being in which the individual realizes his or her abilities, can cope with the normal stresses of life, can work productively and fruitfully, and is able to contribute to his or her community." (WHO,2022).

4. Procedure:

- 4.1 Two fully stocked ambulances will be prepared and ready for deployment daily, one positioned in Hub 7 and one positioned at Scheduled Ambulance Service spoke point.
- 4.2 The HCC will utilize CAD to prioritize and dispatch the closest unit to transfer the patient safely and efficiently.
- 4.3 Patients originating from the community will be managed accordingly to AS CPG and transported to the nearest ED.

5. Operating Hours:

- 5.1 The unit located at Hub 7 will commence duty between the hours of 08:00-20:00 (12-hour shifts on a 4-day cycle).
- 5.2 The unit located at Scheduled Ambulance Service Spoke Point will commence duty between the hours of 12:00-00:00 (12-hour shifts on a 4-day cycle).
- 5.3 Outside of these operating hours, the facility is expected to provide a nurse escort to facilitate smooth and safe movement of the patient. If no escort is available, the booking time should be negotiated within operational timing in above points.

6. Completion of Transfer:

- 6.1 At completion of transfer HMCAS will not return the hospital transfer team or equipment.

7. Logistics:

- 7.1 HMCAS will be responsible for post-use cleaning of the ambulance and fault reporting.

8. Reference:

- 8.1 World Health Organization. 2022. Mental Health: Concepts in mental Health, available online: [Mental health \(who.int\)](#), accessed 01 May 2024.

SOP 4.9: Documentation



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.9.A ELECTRONIC PATIENT CARE RECORDING
Implementation Date	01 October 2016
Purpose	- To ensure that all patient care and findings are accurately recorded. - To ensure clinical documentation is appropriately managed.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. All patient contacts requires the completion of an electronic Patient Care Record (ePCR):

- 1.1 Ensure that the treatment recorded accurately reflects the clinical/physical findings.
- 1.2 Clearly document refusal of treatment and attested thereof by the patient.
- 1.3 Carry out a periodic pain score assessment of patients presenting with pain during their care and accurately record this on the ePCR.
- 1.4 Record a provisional diagnosis on the ePCR.
- 1.5 Justify prolonged scene time on the ePCR.
- 1.6 Attach the vital signs record ('Code Summary') to the ePCR.

2. Documentation Responsibilities:

- 2.1 The first treating clinician documents the following:
 - 2.1.1 Patient Complaint;
 - 2.1.2 Initial Patient Medical History obtained [including medications and allergies];
 - 2.1.3 Clinical assessment, signs, and symptoms;
 - 2.1.4 Preliminary Vital Signs and
 - 2.1.5 Treatment initiated **and** response to treatment.
- 2.2 If a CCP continues care of the patient to hospital, he/she should document any further assessment and treatment and note any changes in patient diagnosis or condition.
- 2.3 The completion of the ePCR is the responsibility of the highest qualified transporting clinician. The CCP must document a stand down ePCR if they have assessed a patient but will not accompany the patient to hospital.
- 2.4 Should the electronic PCR platform fail or is not available, a paper PCR should be used.
- 2.5 For paper hardcopy PCR, hand-over the "blue" copy of the PCR with the patient to the receiving facility. Ensure that receiving clinician signs the "white" copy (original) of the PCR.
- 2.6 Do not copy PCRs in any way unless permission is received from the immediate supervisor.
- 2.7 The Paper PCR should be handed over to the CDO once completed or kept secure until the end of shift where it can be handed over to the CDO or supervisor.
- 2.8 Handle all toughpads/clinical documents with clean hands/clean gloves. If a toughpad becomes contaminated clean the toughpad with the approved wipes, if a document becomes contaminated with body fluids, make a duplicate and destroy/shred the original.
- 2.9 All printed ECG strips must include patient identifiers (in accordance with [SOP 4.9.C](#) and International Patient Safety Goals) when handed over to another treating clinician. This can be in the form of a hospital sticker or handwritten by the clinician.

3. Filling in the data fields on the PCR:

- 3.1 Ensure all applicable data fields in the ePCR are completed. If using a paper PCR, write in neat PRINT style and not “joined-up” (cursive) text, using a black pen and complete all applicable data fields. If using a paper PCR, delete written errors by placing a ruled line through the text to be deleted. Sign and date the deleted error.
- 3.2 Where the option in the ePCR exists to select “Yes”, “No” or “Unknown” from a drop-down list:
 - 3.2.1 Where the patient/guardian is able to understand and answer the question, select Yes or No (For example: the patient does have an allergy – select yes and document the type of allergy, or the patient does not have an allergy or is not aware of any known allergies, select no).
 - 3.2.2 Where the patient/guardian is unable to understand or answer the question, select unknown (For example: select unknown if there is a language barrier, patient is unconscious or the patient/guardian is unable to verbalise for some other reason. Document the presence of language barrier or other explanation for why options are unknown eg unconscious, neck trauma that would impair vocalisation).
- 3.3 Accurately document all medications used in the relevant medication section.
- 3.4 If clinical skills are performed, appropriately document the skill performed in the relevant care management section.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.9.B REPORTING OF OCCURRENCES/VARIATIONS/ACCIDENTS (OVA) AND ADVERSE DRUG REACTIONS
Implementation Date	01 October 2016
Purpose	To establish a systematic reporting of events occurring during routine patient care and practice to aid the quality improvement process.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. An OVA Report should be filled in under the following general circumstances:

- 1.1 Any event or circumstance, which results in an **unintended or unnecessary harm** to any person or loss/damage to HMC or personal property.
- 1.2 An event or situation that could have resulted (near miss) in an accident, injury, or illness, but did not, either by chance or through timely intervention.
- 1.3 An occurrence that results in a loss to a third party due to bodily injury, or property damage or destruction.
- 1.4 An event that departs from expectations (variance); an act contrary to a usual rule i.e. a difference between what is expected and what actually occurs.
- 1.5 An unplanned, unexpected, and undesirable event (accident), which occurs suddenly and results in damage, injury, or harm.

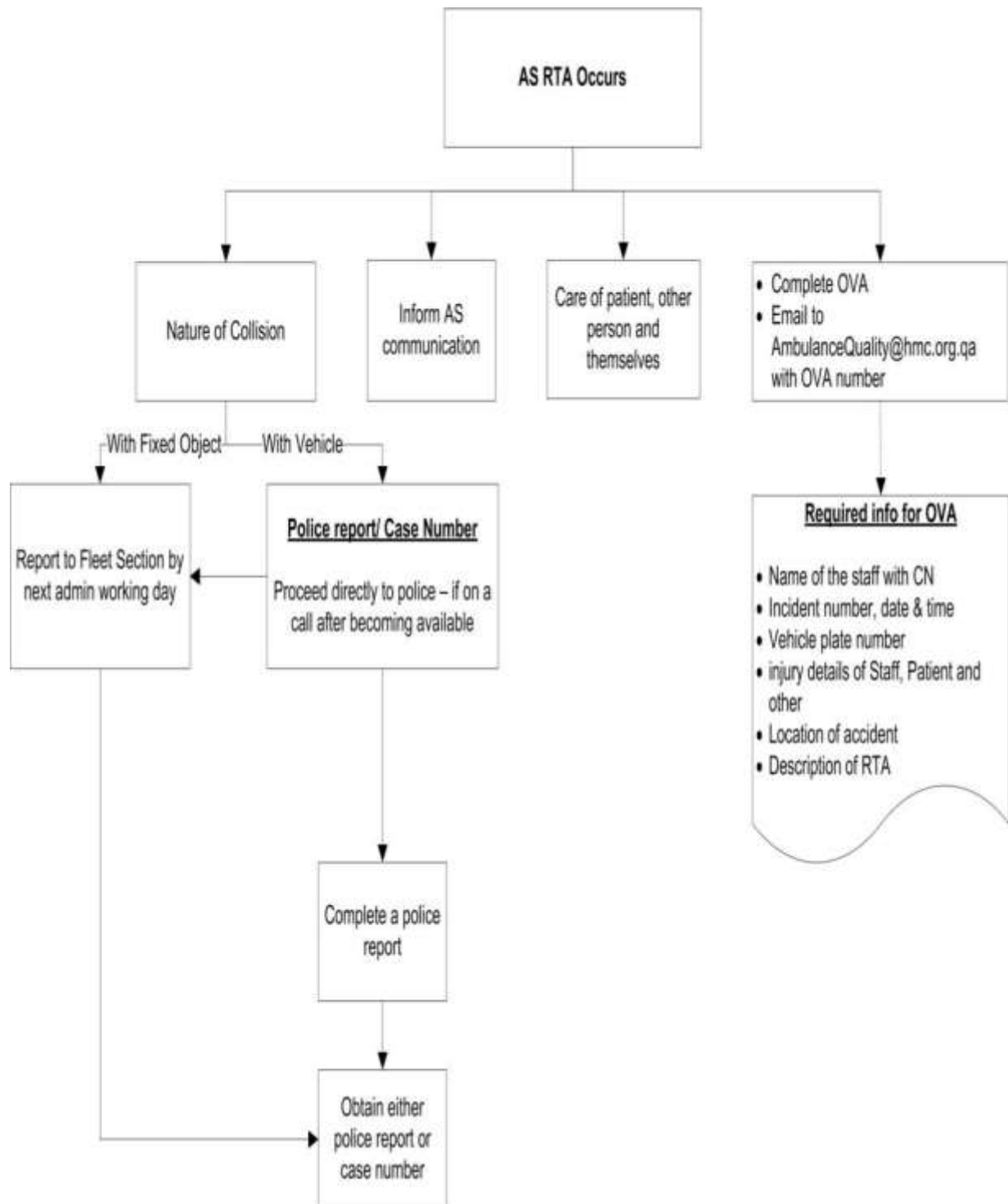
2. Reporting of Adverse Drug Reactions:

- 2.1 An adverse drug reaction is any undesirable effect of a drug (or combination of drugs) beyond its expected therapeutic effects occurring during clinical use.
- 2.2 Report the findings of a witnessed suspected adverse drug reaction through the HMCAS Communications Centre, via the Clinical Desk.
- 2.3 The minimum standard information required by the Clinical Desk are:
 - 2.3.1 Age of patient
 - 2.3.2 Chief complaint and medical history
 - 2.3.3 Trade/generic name of suspected drug
 - 2.3.4 Description of adverse reaction/s
- 2.4 In the PCR, document all the events associated with the suspected drug, including the date, time, and notifications.
- 2.5 Submit an online OVA report through the HMC intranet (Click on [HIS Applications Gate](#) → [Groups](#) → [Other Applications](#) → [OVA Incident Reporting System](#)). Hand-written reports are not permitted.

3. Reporting Road Traffic Accidents involving HMCAS vehicles:

- 3.1 In the case of an RTA involving a HMCAS vehicle, the following procedure (Figure 4.9B-1) should be followed:

Figure 4.9B-1



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.9.C ACCURATE IDENTIFICATION OF PATIENTS
Implementation Date	01 October 2016
Purpose	To ensure that all patients are accurately identified during transition from the pre-hospital to in-hospital phase of care.
Responsibility	ALL HMCAS OPERATIONAL CLINICAL PERSONNEL (E.G. ALPHA, CHARLIE, DELTA, BIKE AND EVENT STAFF)

1. The ambulance service will use two unique patient identifiers prior to transport and prior to discharge to a receiving individual or organization. The primary unique identifiers are patients NAME, Qatar Identification Documentation (QID) and Health Card (HC).
 - 1.1 On arrival at a patient's side, introduce self and clarify patient's name.
 - 1.2 Politely request the patient (or family member/friend) to provide his/her QID to confirm identity.
 - 1.3 If the patient is not in possession of a QID (visitor/new resident of Qatar) then request a copy of patient passport to verify identity.
 - 1.4 Record patient's full name and birth date on the ePCR.
 - 1.5 For mass-casualty incidents, identify patients by patient triage card numbers initially.
 - 1.6 Pediatric patients must be identified by their own identifiers and not those of their parents. If the patient is a newborn and has been registered with a name, they will be identified as a baby of _____.
2. **If none of the above-mentioned patient identifiers are available or possible:**
 - 2.1 Document the patient's name as "Unknown" on the ePCR.
 - 2.2 Identify the patient by the CFS and ePCR number until the correct relevant details can be updated by the MOI or relevant parties. DO NOT DELAY if the patient requires rapid transportation to a definitive care facility.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.9.D CARE AND MANAGEMENT OF THE ELECTRONIC PATIENT CARE RECORD PLATFORM
Implementation Date	01 November 2016
Purpose	To ensure that the HMCAS electronic patient care record (ePCR) platform is appropriately accessed, utilized, and managed.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Management of the Electronic Patient Care Toughpad®:

- 1.1 Complete a patient care record for all patients assessed and/or treated ([SOP 4.9.A](#)) on a Toughpad® (ePCR). The ePCR data is automatically transmitted to an electronic server every 10 minutes.
- 1.2 Use the paper PCR if the ePCR is unavailable or malfunctioning. Notify your production supervisor for advice/replacement as soon as possible.
- 1.3 Receive the Toughpad® in good working order as defined in [SOP 3.17](#).
- 1.4 At the start of shift, record on the vehicle checklist and report the following to the PTL:
 - 1.4.1 ePCR serial number;
 - 1.4.2 MDT serial number;
 - 1.4.3 Toughpad® serial number; and
 - 1.4.4 LifePak 15 device number
- 1.5 Do not use the Toughpad® for internet browsing or accessing any other software application except the ePCR application. Report any violations to the immediate supervisor.
- 1.6 Transmit all monitored vital signs and ECG data via Bluetooth from the multi-parameter monitor (LP15 or other) to the ePCR platform, before officially signing the patient over to the receiving hospital staff.
- 1.7 If a CCP assesses a patient but does not ride along to the hospital, he/she shall note his/her credentials in the appropriate field on the ePCR.
 - 1.7.1 If either the AP or CCP is using a paper PCR, the ePCR Trip_ID must be noted on the paper PCR and given to the facility at handover when a Trip_ID (barcode) is requested.
 - 1.7.2 At handover, the AP/CCP shall ensure that the receiving facility staff endorses the paper PCR.

2. Handover Procedure:

- 2.1 On completion of the verbal handover at the receiving facility, complete the mandatory information on 'Incident information' tab to generate a bar code.
- 2.2 The receiving facility staff shall scan the ePCR generated bar code in order to transfer the record to the hospital.
- 2.3 Continue to finalize the outstanding ePCR details/information.
- 2.4 Where there is a problem with the device (low battery or malfunction) or Bluetooth, print and handover the code summary (LP15 or other) with the CFS number, for the facility staff to attach the patient sticker and file manually.

- 2.5 At handover, the receiving clinician shall sign and note his/her credentials in the appropriate ePCR screen using the Toughpad® stylus.
- 2.6 Save and close the case normally unless there is no server connectivity during handover of a critically ill/injured patient - print the ePCR at the receiving facility and hand over to the receiving facility staff with a code summary.
- 2.7 If there is a problem with the device e.g. low battery or malfunction, use the paper PCR. Notify the Romeo or Clinical Documentation Officer at the facility or Production Supervisor when available.
- 2.8 Manually advise the Trip_ID and the patient's Health Card Number, if the receiving facility is unable to scan the bar code from the device.
- 2.9 For a non-HMC facility or an HMC Facility not using Cerner®, the receiving clinician shall sign and note his/her credentials in the appropriate screen using the Toughpad® stylus as a 'non-HMC Staff'. Advise the receiving person to request a fax/email copy via email address: ambulanceCDO@hamad.qa.

3. **Cleaning Procedure:**

- 3.1 Handle the ePCR with clean hands or clean gloves in accordance with hand hygiene instructions.
- 3.2 If drops of water or other stains get on the surface of the LCD panel, wipe them off immediately (failure to do so may result in staining).
- 3.3 The ePCR device shall be cleaned twice daily or when visibly soiled or between patient encounters or between each staff handling.
- 3.4 To clean the LCD panel:
 - 3.4.1 wipe with a disposable soft damp cloth such as gauze.
 - 3.4.2 clean with HMC approved non-alcoholic wipes (e.g. Medizar), if potentially soiled with blood or other infectious material.
- 3.5 For all areas, other than the LCD panel, clean with HMC approved non-alcoholic solution (Medizar).

4. **CAUTION:**

- 4.1 Do not use benzene, thinner, or rubbing alcohol since it may adversely affect the surface causing discoloration, etc.
- 4.2 Do not use commercially-available household cleaners and cosmetics, as they may contain components harmful to the surface.
- 4.3 Do not apply water or detergent directly to the computer, since liquid may enter inside of the computer and cause malfunction or damage.

SOP 4.10: Major Incident Response (MIR)



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.10.A MIR FIRST UNIT ON SCENE
Implementation Date	01 October 2016
Purpose	To standardize the role and responsibility of the first arriving unit on scene at a major incident.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. FIRST AMBULANCE OR RESPONSE DRIVER arriving on scene (See Figure 4.10.A-1):
 - 1.1 Book 'On Scene' with NCC on designated channel.
 - 1.2 Park as near to the scene as safety permits; do not leave vehicle.
 - 1.3 Don high visibility vest and safety helmet.
 - 1.4 Assume role of Silver Control on the channel instructed by NCC.
 - 1.5 LEAVE FLASHING Emergency lights and hazards warning lights ON.
 - 1.6 Maintain communication link between ambulance paramedic attendant and NCC.
 - 1.7 Provide initial METHANE report to NCC.
 - 1.8 Request additional resources as required.
 - 1.9 Hold all staff at your vehicle until they are briefed by silver command.
 - 1.10 Ensure all staffs wear appropriate PPE and have triage packs.
2. If the first arriving unit is single crewed, then he/she will be responsible for carrying out the ATTENDANT duties described below.
 - 2.1 Don high visibility vest and safety helmet
 - 2.2 Assume role of Silver Commander
 - 2.3 Carry out a scene size up of the incident
 - 2.4 Report back to NCC using the METHANE:
 - 2.4.1 **M** - MAJOR INCIDENT DECLARED at appropriate LEVEL 1/2/3.
 - 2.4.2 **E** – EXACT LOCATION with map reference if possible.
 - 2.4.3 **T** - TYPE OF INCIDENT with details of structure, vehicles.
 - 2.4.4 **H** – HAZARDS present and potential.
 - 2.4.5 **A** – ACCESS ROUTES and suitable Rendezvous Point.
 - 2.4.6 **N** – NUMBER OF PATIENTS and severity.
 - 2.4.7 **E** – EMERGENCY SERVICES present and required.
 - 2.5 Declare a major incident based on criteria as set out by HMCAS policies.
 - 2.6 Think command and control and designate roles.
 - 2.7 Issue ACTION CARDS (available from Delta/Charlie/Oscar units)
 - 2.8 Instruct the second arriving crew to assist with triage.
 - 2.9 In liaison with other emergency services establish:
 - 2.9.1 Access and egress routes to and from scene

2.9.2 Ambulance Casualty Clearing Points

2.9.3 Ambulance Control Point

2.9.4 Ambulance Parking Point

2.9.5 Ambulance Loading Point(s)

2.10 Consider potential HAZARDS when designating the above.

3. The first crew on scene shall not attempt to rescue/treat casualties until relieved of their initial "First on Scene" roles by Ambulance Service supervisors or managers.

APPROACH TO A SIGNIFICANT/MAJOR INCIDENT

APPROACH EVERY SCENE AS A POTENTIAL HAZARDOUS INCIDENT

If you see hazardous materials at the scene:

- **DO NOT APPROACH**
- Stay back a safe distance, up-wind from the incident
- Inform NCC

IF THERE ARE MORE THAN 3 RED/YELLOW PATIENTS



FIRST UNIT ON SCENE

- Put on high visibility vest and helmet
- Carry out scene size-up
- Count the number of patients and their severity status Red/Yellow/Green)
- Report back to NCC using the METHANE Report
 - M** – Declare as potential Major Incident
 - E** – Exact Location of incident
 - T** – Type of Incident
 - H** – Hazards
 - A** – Access and Egress Route
 - N** – Number and Status of Patients
 - E** – Emergency Services Present and Required
- Instruct SECOND ARRIVING UNIT: - DRIVER to set up a **PARKING** area
- ATTENDANT to start **TRIAGE**



SECOND UNIT ON SCENE

- Put on high visibility vest and helmet
- Prepare to take the **BRONZE PARKING** role -You will be responsible for setting up a parking area for Ambulances
- Prepare to take the **BRONZE PRIMARY TRIAGE OFFICER** role - You will be responsible for Triaging Patients

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.10.B MAJOR INCIDENT SCENE SETUP
Implementation Date	01 June 2016
Purpose	To establish an orderly flow of patients from scene through the transport area.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. On arrival of MIR Trucks at the scene:

- 1.1 Park the trucks in a safe area and report to silver command to designate the setup location.
- 1.2 Park the MIR truck on the right side and logistic truck on the left side.
- 1.3 Off-load the container and keep it attached to the truck if there is space to accommodate it.
- 1.4 Consider lighting if the incident is at night.



- 2. Setup:** Divide the staff into two teams, one to set the blue and yellow tarpaulin, one to set the red then green tarpaulin.
- 2.1 Put down the blue tarpaulin first to start secondary triage early (see the diagram).
 - 2.2 Place the Spine boards, scoop stretchers and basket stretchers on the right side of the tarpaulin.
 - 2.3 Put down the red tarpaulin then yellow and green.

- 2.4 Place portable oxygen cylinder (small cylinders), LP15 and Pax treatment bags on tarpaulins, starting with the blue tarpaulin.
3. **Erect the BLUE shelter** (secondary triage tent) first; if possible, erect the red shelter simultaneously.
 - 3.1 Assign a minimum of five staff to erect the tent, four (4) staff and one (1) MIR supervisor to guide and supervise the procedures as well as staff safety.
 - 3.2 The location of the tent should be the safest area facing the incident.
 - 3.3 The required equipment in the blue shelter are as follows:
 - 3.3.1 Four NATO stretchers with ground stabilizer (if the ground surface is not stable). Place the stretchers in the middle of the shelter, with the head facing the shelter exit door.
 - 3.3.2 Place the blue Zarges in the middle of the back to back NATO stretchers.
 - 3.3.3 Four LP15 or (LP12)
 - 3.3.4 Four small oxygen cylinders and four portable suctions.
 - 3.3.5 Minimum of four tent lights maximum of six.
 - 3.3.6 Fire extinguisher near the shelter exit in a safe place, fixed with the tent straps.
 - 3.3.7 Two (2) air conditioner (AC) minimum per tent; keep the AC outside the tent using the AC hose from the tent AC holes.



4. **Erect the RED shelter** second if it is not possible to do at the same time with the blue shelter.
- 4.1 Assign a minimum of five (5) staff to erect the tent, four (4) staff and one (1) MIR supervisor to guide and supervise the procedures and safety of the staff. The tent should be located on the far-left side of the truck containers.
- 4.2 The required equipment in the red shelter are as follows:
- 4.2.1 Six (6) NATO stretchers with ground stabilizer (if the ground surface is not stable).
Locate three on each side, keeping the stretcher head facing the middle of the tents.
 - 4.2.2 Six (6) red Zarges, one next to each stretcher
 - 4.2.3 Six (6) LP15 or (LP12)
 - 4.2.4 Six (6) Oxylog 1000
 - 4.2.5 Six (6) portable suction units
 - 4.2.6 Minimum of four (4) tent lights maximum of six (6) lights
 - 4.2.7 Fire extinguisher near the shelter exit in a safe place, fixed with the tent straps
 - 4.2.8 Two (2) Air conditioner (AC) minimum per tent; keep the AC outside the tent and use the AC hose from the tent AC Holes.
- 4.3 Keep the mass oxygen delivery system in the tent near the exit door without causing an obstruction.
- 4.4 **NOTE:** Connect all six outlets and the alarm system. Check the pressure and Oxylog connectors after attaching. Keep the oxygen delivery tubing on the inner side of the tent.



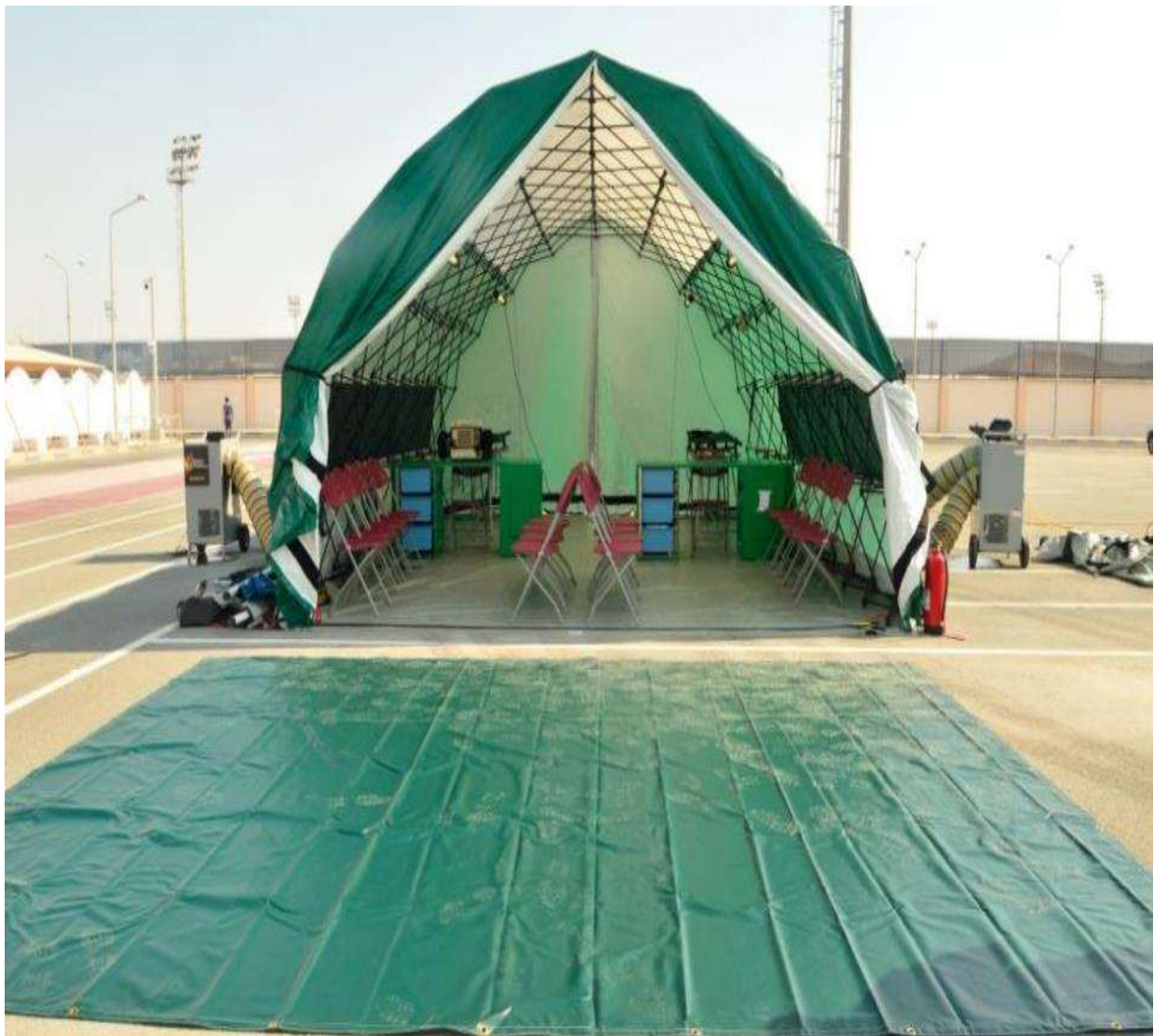
5. Erect the Yellow shelter:

- 5.1 Assign minimum of five (5) staff to erect the tent, four (4) staff and one (1) MIR supervisor to guide and supervise the procedures as well as staff safety.
- 5.2 The location of the tent should be on the left side of the truck and on the right side to the red tent.
- 5.3 The required equipment in the yellow shelter are as follows:
 - 5.3.1 Eight (8) NATO stretchers, with ground stabilizer (if the ground surface is not stable).
 - 5.3.2 Four (4) stretchers on each side, two (2) near the tent entrance, two (2) near the tent exit and two (2) next to each other in the middle of the tent on each side.
 - 5.3.3 Four (4) yellow Zarges container (see the diagram page 92)
 - 5.3.4 Eight (8) LP15 or LP12
 - 5.3.5 Four (4) portable suction
 - 5.3.6 Minimum of four (4) tent lights maximum of six (6)
 - 5.3.7 Fire extinguisher near the shelter exit in a safe place, fixed with the tent straps.
 - 5.3.8 Two (2) Air conditioner (AC) minimum per tent; keep the AC outside the tent and use the AC hose from the tent AC Holes.
 - 5.3.9 Keep the mass oxygen delivery system in the tent near the exit door without causing an obstruction.
 - 5.3.10 NOTE: Connect all eight (8) outlets and the alarm system. Check the pressure and Oxylog Connectors after attaching. Keep the oxygen delivery tubing on the inner side of the tent.



6. Erect the Green tent (shelter) last:

- 6.1 Assign a minimum of five (5) staff to erect the tent, four (4) staff and one (1) MIR supervisor to guide and supervise the procedures and staff safety.
- 6.2 Place the green tent on the right side of the MIR and logistic containers.
- 6.3 The required equipment in the blue shelter are as follows:
 - 6.3.1 Two (2) NATO stretchers, with ground stabilizer (if the ground surface is not stable). Place them on either side of the shelter next to the exit door, with the head facing the tent wall.
 - 6.3.2 Twenty chairs, six (6) on each side of the tent and eight (8) in the middle.
 - 6.3.3 Two (2) green Zarges in the tent
 - 6.3.4 Two LP15 or LP12
 - 6.3.5 Minimum of four (4) tent lights, maximum of six (6)
 - 6.3.6 Fire extinguisher near the shelter exit in a safe place, fixed with the tent straps.
 - 6.3.7 Two (2) Air conditioner (AC) minimum per tent, keep the AC outside the tent and use the AC hose from the tent AC Holes.



7. Erect the Loading shelter (white coloured tent):

- 7.1 Assign minimum of five (5) staff to erect the tent, four (4) staff and one (1) MIR supervisor to guide and supervise procedures and staff safety.
- 7.2 The location of this tent is behind the red and yellow tent towards the middle area.
- 7.3 Equipment required in this shelter are as follows:
 - 7.3.1 Two (2) air conditioner (AC) if available; keep the AC outside the tent and use the AC hose from the tent AC holes.
 - 7.3.2 Draft table

8. Erect the incident Morgue shelter:

- 8.1 MIR production supervisor shall facilitate getting this shelter from logistic village warehouse.
- 8.2 Assign minimum of 6 staff to erect the tent. Four (4) staff and one (1) MIR supervisor to guide and supervise the procedures and staff safety.
- 8.3 Place this tent behind the red tent.
- 8.4 The required equipment in the blue shelter are as follows:
 - 8.4.1 Two (2) air conditioners (AC) minimum per tent, the AC should keep outside the tent and use the AC hose from the tent AC holes.
 - 8.4.2 Body bags
 - 8.4.3 Blue sheets
 - 8.4.4 Patient care reports



9. Erect the survivor Reception shelter:

- 9.1 Assign minimum of 8 staff to erect the tent. Four (4) staff and one (1) MIR supervisor to guide and supervise procedures and staff safety.
- 9.2 Place this tent behind the green tent.
- 9.3 The equipment required are as follows:
 - 9.3.1 Two (2) ACs if available; keep the AC outside the tent and use the AC hose from the tent AC holes.
 - 9.3.2 Chairs if available

10. Re-packing the equipment:

- 10.1 The same team allocated to a tent shall re-pack the equipment and the tent used at the incident.
- 10.2 Check all generators, wires, equipment for any damage.
- 10.3 Report any damage immediately to your Oscar or manager.
- 10.4 Check the fuel level and refuel the truck and generators.
- 10.5 Re-stock all missing or used items
- 10.6 Write defect reports if required

11. General considerations:

- 11.1 Safety is a top priority for staff, patients, colleagues, survivors.
- 11.2 Don't leave the generators unattended; one (1) MIR staff has to attend to the generators all the time.
- 11.3 Teamwork is the key to success.
- 11.4 Ask for help if you cannot do the task allocated to you, and if you are unfamiliar with any equipment and/or procedure.
- 11.5 Don't leave your area of responsibility without informing your team leader.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.10.C DECONTAMINATION (DECON) SETUP (SCENE LAYOUT), RESPONSE AND MITIGATION
Implementation Date	01 October 2016
Purpose	Response and Mitigation to Hazardous Materials.
Responsibility	ALL HMCAS OPERATIONAL (PART 1) AND HAZMAT STAFF (PART 2)

- PART 1:** The '1-2-3' system (to be used when the cause of the incident is unknown).
- When attending emergency calls, adopt the '1-2-3' system to ensure staff safety and to identify a possible chemical incident/attack. Follow up with the use of the **METHANE** report as soon as practical.

[1]	: ONE casualty	Approach using normal procedures.
[2]	: TWO casualties	Approach with caution, consider all options. Report on arrival and update control.
[3]	: THREE or more casualties	DO NOT approach the scene: ✓ Withdraw ✓ Contain ✓ Isolate yourself ✓ REQUEST SPECIALIST HELP (HAZMAT TEAM)

- PART 2:** HAZMAT team considerations.
- Protective Clothing:** HMCAS uses *LEVEL C* or *LEVEL D* Protective Clothing.
 - Before use of protective clothing:
 - Determine that the clothing material is correct for the specified task at hand.
 - Visually inspect for imperfect seams, non-uniform coatings, any tears, and malfunctioning closures.
 - Hold up to light and check for pinholes
 - Flex the product and observe for cracks
 - Observe for other signs or shelf deterioration
 - If the product has been used previously, inspect inside and out for signs of chemical attack (discoloration, swelling and stiffness).
 - Pressurize glove to check for pinholes. Blow into glove, then roll gauntlet towards fingers or inflate glove and hold under water. In either case, no air should escape.
 - During the work task, periodically inspect for:
 - Closure failure
 - Tears
 - Punctures
 - Seam discontinuities

- 4.2.5 Evidence of chemical attack such as discoloration, swelling, stiffening, and softening (keep in mind, however, that chemical permeation can occur without any visible effects).

5. LEVEL C protective clothing:

- 5.1 Not Acceptable for Chemical Emergency Response
- 5.2 Support Function Protective Garment (meets NFPA 1993)
- 5.3 Full-face piece, air-purifying, canister-equipped respirator
- 5.4 Chemical resistant gloves and safety boots
- 5.5 Two-way communications system, hard hat
- 5.6 OPTIONAL: Face shield, escape SCBA
- 5.7 *Protection Provided:* The same level of skin protection as Level B, but a lower level of respiratory protection. Liquid splash protection but no protection to chemical vapours or gases.
- 5.8 *Used When:* Contact with site chemical(s) will not affect the skin. Air contaminants have been identified and concentrations measured. The site and its hazards have been completely characterized.
- 5.9 *Limitations:* Protective clothing items must resist penetration by the chemical or mixtures present. Chemical airborne concentration must be less than IDLH (Immediately Dangerous to Life and Health) levels. The atmosphere must contain at least 19.5% oxygen.

6. LEVEL D Protective Clothing:

- 6.1 Not Acceptable for Chemical Emergency Response
- 6.2 Coveralls, safety boots/shoes, safety glasses or chemical splash goggles
- 6.3 OPTIONAL: Gloves, escape SCBA, face-shield
- 6.4 *Protection Provided:* No respiratory protection, minimal skin protection
- 6.5 *Used When:* The atmosphere contains no known hazard. Work functions preclude splashes, immersion, potential for inhalation, or direct contact with hazard chemicals.
- 6.6 *Limitations:* This level should not be worn in the Hot Zone. The atmosphere must contain at least 19.5% oxygen.
- 6.7 The type of equipment used, and the overall level of protection should be re-evaluated periodically as the amount of information about the HAZMAT incident situation or process increases. HMCAS HAZMAT should upgrade or downgrade their level of protection only with concurrence with the Team Leader/Bronze Decon/Bronze Tactical.

7. Donning Procedure:

- 7.1 Establish and practice periodically for donning the various ensemble configurations that the HMCAS HAZMAT team may use.
- 7.2 The procedures assume the wearer has previous training in respirator use and decontamination procedures.
- 7.3 Once the equipment has been donned, its fit should be evaluated. If the clothing is too small, it will restrict movement, increase the likelihood of tearing the suit material, and accelerate wearer fatigue. If the clothing is too large, the possibility of snagging the material is increased, and the dexterity and coordination of the wearer may be compromised.
- 7.4 Inspect clothing and respiratory equipment before donning.

- 7.5 Standing or sitting, step into the legs of the suit; ensure proper placement of the feet within the suit; then gather the suit around the waist.
- 7.6 Depending on type of suit: put on long-sleeved inner gloves (similar to surgical gloves). Secure gloves to sleeves, for suits with detachable gloves (if not done prior to entering the suit). Additional over gloves, worn over attached suit gloves, may be donned later.
- 7.7 Put on hard hat, if needed
- 7.8 Raise hood overhead carefully so as not to disrupt face seal
- 7.9 Begin to secure the suit by closing all fasteners on opening
- 7.10 Secure all belts and/or adjustable leg, head, and waistbands
- 7.11 Have assistant first ensure that wearer is breathing properly and then make final closure of the suit.
- 7.12 Have assistant check all closures
- 7.13 Have assistant observe the wearer for a period of time to ensure that the wearer is comfortable, psychologically stable, and that the equipment is functioning properly.

8. Doffing procedures:

- 8.1 Remove any extraneous or disposable clothing, boot covers, outer gloves, and tape.
- 8.2 Have assistant loosen and remove the wearer's safety shoes or boots.
- 8.3 Have assistant open the suit completely and lift the hood over the head of the wearer.
- 8.4 Remove arms, one at a time, from suit. Avoid any contact between the outside surface of the suit and the wearer's body--and lay the suit out flat behind the wearer. Leave internal gloves on, if any.
- 8.5 Sitting, if possible, remove both legs from the suit.
- 8.6 After suit is removed, remove internal gloves by rolling them off the hand, inside out.
- 8.7 Remove internal clothing and thoroughly cleanse the body.

9. User Monitoring:

- 9.1 The wearer must understand all aspects of clothing/equipment operation and their limitations; during protective clothing use, end users should be encouraged to report any perceived problems or difficulties to their team leader. These malfunctions include, but are not limited to:
 - 9.1.1 Degradation of the protection ensemble;
 - 9.1.2 Perception of odours;
 - 9.1.3 Skin irritation;
 - 9.1.4 Unusual residues on clothing material;
 - 9.1.5 Discomfort;
 - 9.1.6 Resistance to breathing;
 - 9.1.7 Fatigue due to respirator use;
 - 9.1.8 Interference with vision or communication;
 - 9.1.9 Restriction of movement; and
 - 9.1.10 Physiological responses such as rapid pulse, nausea, or chest pain.

10. Before HMCAS HAZMAT team members undertake any activity in their protective ensembles, the anticipated duration of use should be established. Several factors limit the length of a mission, including:

- 10.1 Air supply consumption as affected by wearer work rate, fitness, body size, and breathing patterns;
- 10.2 Suit ensemble permeation, degradation, and penetration by chemical contaminants, including expected leakage through suit or respirator exhaust valves (ensemble protection factor);
- 10.3 Ambient temperature as it influences material chemical resistance and flexibility, suit and respirator exhaust valve performance, and wearer heat stress and
- 10.4 Coolant supply (if necessary).

11. Response and Mitigation Plan: Hazardous Materials:

- 11.1 A response and mitigation plan shall be developed and set up before any HMCAS HAZMAT personnel or equipment is allowed to enter areas where the potential for exposure to hazardous substances exists. The response and mitigation plan should:
 - 11.1.1 Determine the number and layout of decontamination stations;
 - 11.1.2 Determine the decontamination equipment needed;
 - 11.1.3 Determine appropriate decontamination methods;
 - 11.1.4 Establish procedures to prevent contamination of clean areas;
 - 11.1.5 Establish methods and procedures to minimize wearer contact with contaminants during removal of personal protective clothing and
 - 11.1.6 Establish methods for disposing of clothing and equipment that are not completely decontaminated.
- 11.2 The plan should be revised whenever the type of personal protective clothing or equipment changes, the use conditions change, or the chemical hazards are reassessed based on new information.
- 11.3 The response and mitigation process for decontamination should consist of a series of procedures performed in a specific sequence. Each procedure should be performed at a separate decontamination line in order to prevent cross contamination.
- 11.4 Decontamination lines should be separated physically to prevent cross contamination and should be arranged in order of decreasing contamination, preferably in a straight line. Separate flow patterns and stations must be established to isolate workers from different contamination zones. Entry and exit points to exposed areas should be conspicuously marked. Dressing stations for entry to the decontamination area should be separate from redressing areas for exit from the decontamination area.
- 11.5 All equipment used for decontamination must be decontaminated and/or disposed of properly. Buckets, brushes, clothing, tools, and other contaminated equipment should be collected, placed in containers, and labelled. All spent solutions and wash water should be collected and disposed of properly. Clothing that is not completely decontaminated should be placed in plastic bags, pending further decontamination and/or disposal.
- 11.6 Decontamination of workers who initially come in contact with personnel and equipment leaving exposure or contamination areas will require more protection from contaminants than decontamination workers who are assigned in the decontamination line.

12. Storage:

- 12.1 Clothing must be stored properly to prevent damage or malfunction from exposure to dust, moisture, sunlight, damaging chemicals, extreme temperatures, and impact.
- 12.2 Guidelines for general storage of protective clothing include:
- 12.3 Potentially contaminated clothing should be stored in an area separate from unused protective clothing.
- 12.4 Potentially contaminated clothing should be stored in a well-ventilated area, with good air flow around each item, if possible.
- 12.5 Different types and materials of clothing and gloves should be stored separately to prevent issuing the wrong material by mistake.
- 12.6 Protective clothing should be folded or hung in accordance with manufacturer instructions.

13. Maintenance:

- 13.1 In no case should you attempt to repair equipment.
- 13.2 All repairs made must be recorded for the specific clothing along with appropriate inspection results (if clothing has been sent for manufacturer repair).

14. Actions for HAZMAT vehicles arriving at the Scene:

- 14.1 Park the vehicles in a safe area and the Team Leader report to Silver Command to coordinate the decontamination setup location in conjunction with the relevant external agency.
- 14.2 Park HAZMAT 0 vehicle on the warm-zone/cold-zone border.
- 14.3 Park the CBRNe truck on the warm-zone/cold-zone border.
- 14.4 Off load the container and keep it attached to the truck if there is enough space on scene and if it doesn't negatively impact access or egress routes or hamper the safe movement of patients.
- 14.5 Consider lighting if the incident is at night.
- 14.6 Ensure that wind direction is observed (if the HAZMAT truck is on scene, make use of the windsock).

15. Phase 1 (minimum 5 team members):

- 15.1 Set up the individual decontamination with the entrance 2m back from the border of the hot-zone. Set up the basin in the tent.
- 15.2 Don the appropriate level of PPE (Level C/D) to undertake decontamination operations.
- 15.3 Remove the water heater from the vehicle.
- 15.4 Set up the 100-gallon waste bladder on the side of the tent that has the sprayer connection.
- 15.5 Set up the sump-pump in the decontamination shelter basin and connect the grey hose to the bladder.
- 15.6 Ensure that both valves are in the open position.
- 15.7 Run appropriate voltage (blue connections) extension cable from electrical box to distribution point.
- 15.8 Secure the electrical box to the frame of the tent with zip-ties.
- 15.9 Setup the water heater on side of the waste bladder furthest away from the decontamination tent.

- 15.10 Connect the high-pressure hose (fire hose) to the tent and run out towards the cold-zone border.
- 15.11 Run appropriate voltage (blue connections) extension cable from water heater to distribution point.
- 15.12 Connect both the low-pressure hoses to the bottom fittings on the water heater - red hose to wash, blue hose to rinse.
- 15.13 Connect the low-pressure hoses to the tent with the red hose on the inlet closest to the bladder and the blue hose on the opposite side of the tent.
- 15.14 Setup two conveyors meeting at the exit of the decontamination tent so that they extend out either side of the tent.
- 15.15 Place the conveyors on the side of the sprayer connection.
- 15.16 Place one floor riser in the tent running parallel to the conveyor system and one floor riser at the exit of the tent running parallel to the conveyor system.
- 15.17 Place one foldable bin with bin bags in place at the entrance to the tent on the side opposite to the conveyor.
- 15.18 Place a second foldable bin with bin bag in place with patient belonging bags inside next to it.
- 15.19 Place a plastic bin at the exit to the tent on the opposite side of the conveyor system.
- 15.20 Connect tent lights to the frame at both the entrance and the exit of the decontamination tent. Light 1 at entrance with the long cable on the right and light two at the exit with the long cable on the right.
- 15.21 Lights are to be connected in series.
- 15.22 All excess cabling to be rolled and secured to the tent frame with zip-ties.
- 15.23 Run appropriate voltage (yellow connections) extension cable to lights from distribution point.
- 15.24 Don the appropriate level of PPE (Level C/D) to undertake decontamination operations as a safety person/relief.
- 15.25 Place disposable towels and suits/foil blankets at the exit of the tent on the side opposite to the conveyor system.
- 15.26 Assume roles and responsibilities of Decontamination Team Leader.

16. Phase 2. (minimum 9 team members):

- 16.1 Continue with decontamination operations as required.
- 16.2 Setup 2-line decontamination approximately 5m laterally from the 100-gallon waste bladder.
- 16.3 Setup the basin in the tent.
- 16.4 Setup two 500-gallon waste bladders between the two lines decontamination and the 100-gallon waste bladder.
- 16.5 Place the first bladder inline next to the 100-gallon bladder.
- 16.6 Place the second bladder behind the first bladder.
- 16.7 Setup the sump-pump in the basin and connect the grey hose to the second waste bladder.
- 16.8 Run appropriate voltage (blue connections) extension cable from electrical box to distribution point.
- 16.9 Secure the electrical box to the frame of the tent with zip-ties.

- 16.10 Ensure that both valves are in the open position on the second bladder.
- 16.11 Ensure that the valve is closed on the first bladder.
- 16.12 Hang the interior curtain that separates the wash basin from the de-robing area.
- 16.13 Hang the centre curtain between the conveyor section and the walking section.
- 16.14 Connect the hand sprayers.
- 16.15 Connect low pressure hoses to the water heater (top two connections) – red hose to wash, blue hose to rinse.
- 16.16 Connect low pressure hoses to tent – red hose to inlet closest to hot-zone, blue hose to inlet closest to cold-zone.
- 16.17 Setup a conveyor from the curtain out towards the cold-zone.
- 16.18 Align the conveyor towards the side of the tent closest to the bladders leaving enough room for decontamination personnel to be able to work between the conveyor and the tent.
- 16.19 Place a foldable bin outside the re-robing area.
- 16.20 Place disposable towels inside the re-robing area.
- 16.21 Place suits/foil blankets in the re-robing area.
- 16.22 Place two floor rises inside the basin running longitudinally through the basin.
- 16.23 Place two floor rises in the re-robing area.
- 16.24 Place two floor rises just outside the re-robing area.
- 16.25 Setup tent lights in the 2-line decontamination.
- 16.26 Connect tent lights to the frame at both the entrance and the exit of the decontamination tent.
- 16.27 Light 1 at entrance with the long cable on the right.
- 16.28 Light 2 in the decontamination wash-down area. Wiring run through the tent secured to the ceiling of the tent with zip-ties.
- 16.29 Light 3 at the exit with the long cable on the right.
- 16.30 Lights are to be connected in series.
- 16.31 All excess cabling to be rolled and secured to the tent frame with zip-ties.
- 16.32 Run appropriate voltage (yellow connections) extension cable to lights from distribution point.
- 16.33 Switch water over from individual decontamination to 2-line decontamination.
- 16.34 Ensure water is flowing on both the wash and rinse lines.
- 16.35 Move the two conveyors from the individual decontamination to the 2-line decontamination.
- 16.36 Connect them to the single conveyor in situ running out towards the hot-zone.
- 16.37 Commence decontamination operations in the 2-line decontamination unit.
- 16.38 Ensure individual decontamination is maintained in operational state to be used for emergency personnel decontamination.

17. General consideration for the use of the Face Piece:

- 17.1 The Promask face piece is designed to protect the eyes and face and respiratory system in an environment requiring air purifying protection.
- 17.2 The respirator must not be used where oxygen is deficit less than 19.5 atmospheres.

- 17.3 The face piece is intended to seal against the user's face and allows the user to breathe the air drawn directly through the filtration media (negative pressure) or through breathing tube assembly (positive pressure).
- 17.4 Type: tight fitting face piece with adjustable five-point head harness.
- 17.5 Material: butyl rubber
- 17.6 Size: regular and small
- 17.7 Limitations of use of the face piece:
 - 17.7.1 It may not be used by personnel with facial hair that obstructs the seal of the face piece and users face or have any other condition which would prevent a good face seal.
 - 17.7.2 Do not used with eye glass
 - 17.7.3 Do not use if the user fails to fit the respirator and face piece to his/her face.
 - 17.7.4 Do not use if any components are missing, damaged or worn.
- 17.8 Use of the facemask: before use, verify that the face piece is complete and not damaged.
 - 17.8.1 Inspect the following:
 - 17.8.1.1 Face piece condition
 - 17.8.1.2 Head harness condition, buckles, and roller
 - 17.8.1.3 Visor condition and cleanliness
 - 17.8.1.4 Integral nose cup condition and intactness including inhalation valve
 - 17.8.1.5 Speech diaphragm condition
 - 17.8.2 After use:
 - 17.8.2.1 Decontaminate the mask as per manufacturer's instruction
 - 17.8.2.2 Dry the face mask and wipe it properly
 - 17.8.2.3 Check the face piece for any damage, missing or malfunction components
 - 17.8.2.4 Store the mask properly for future use
 - 17.8.3 System assembly (positive pressure):
 - 17.8.3.1 Remove the seal plugs from both ends of the breathing tube.
 - 17.8.3.2 Inspect the valve seat inside face piece for any damage or missing.
 - 17.8.3.3 Thread the male end of breathing tube into the female thread of the face piece until hand tight.
 - 17.8.3.4 Inspect the gasket inside of the female end of breathing tube, do not use if the gasket is damaged or missing.
 - 17.8.3.5 Attach the female end of breathing tube into the blower assembly until hand tight.
 - 17.8.4 System assembly (negative pressure):
 - 17.8.4.1 Check the cartridge for expiratory date
 - 17.8.4.2 If protective seal is damaged do not use the cartridge
 - 17.8.4.3 The presence of snub cup is important

- 17.8.4.4 Check the filtration media at the facemask and the blanking plug must be correctly connected to the face piece.
- 17.8.4.5 Remove the protective seal of the filter
- 17.8.4.6 Thread the filtration media to the left or right air inlet connector of the face mask until a hand tight seal is achieved.
- 17.8.4.7 Ensure the blanking plug is installed in the opposite air inlet connector.
- 17.8.5 Donning of the facemask:
 - 17.8.5.1 Loosen the head harness.
 - 17.8.5.2 Don the face mask by first placing the jaw end on your face, then pull the head harness over the head.
 - 17.8.5.3 Starting with the lower straps on both sides, adjust the head harness evenly so that the cradle lies centred on the back of the head.
 - 17.8.5.4 Fit check the face mask for proper seal by closing the air inlet opening by hand, create a vacuum to make the mask press onto user face if does not press readjust the head harness.
- 17.8.6 Doffing of the facemask:
 - 17.8.6.1 Loosen the head harness
 - 17.8.6.2 Pull the head harness over the head
 - 17.8.6.3 After use, the face mask must be decontaminated, maintained, and stored properly.
- 17.8.7 Failure to fit the respirator and face piece to the user may result in limited or no respiratory protection and may expose the wearer to substances which can cause serious personal injury or death.

18. Water heater storage:

- 18.1 The water heater injection system should be drained, cleaned, and dried after each use before storage.
- 18.2 If it is to be stored more than two months drain fuel from the burner unit.
- 18.3 Solution unit:
 - 18.3.1 Empty and wash the solution container, fill with clean water
 - 18.3.2 Open the wash output valve and flush the system with clean water 3-5 minutes.
 - 18.3.3 Disconnect the feed tube from the container
 - 18.3.4 Empty and dry the solution container
 - 18.3.5 Reconnect the fuel tube to the container
- 18.4 Blow out procedure:
 - 18.4.1 Disconnect water supply
 - 18.4.2 Open rinse valve (lower right hand with blue tip)
 - 18.4.3 Close wash valve (lower left hand with red tip)
 - 18.4.4 Connect air supply with proper air fitting to air valve on the water supply line.
 - 18.4.5 Allow sufficient time for water to drain out of the rinse valve until no visible flow is present.

- 18.4.6 Open wash valve (lower left-hand side with red tip)
- 18.4.7 Open filter valve
- 18.4.8 Close rinse valve (lower right-hand side with blue tip)
- 18.4.9 Allow sufficient time for water to drain out of the wash valve until no visible flow is present.
- 18.5 Solution Injector:
 - 18.5.1 Unscrew the small black retaining nut at the button of the injection system.
 - 18.5.2 Carefully pull down and remove the fuel tube assembly from the injection system.
 - 18.5.3 Depress the black button on top of the blue injector housing; allow all water to drain.
 - 18.5.4 Remove water from the fuel tube assembly, grasp the solution container and twirl it rapidly in a circular motion (do in care).
 - 18.5.5 Unscrew the blue body cover from the injection, clean, and dry the interior of the housing and exposed piston.
 - 18.5.6 Reattach tube assembly to injection system
- 18.6 Burner unit:
 - 18.6.1 Rinse with clean water sparingly and wipe down with clean water.
 - 18.6.2 Do not put water over or inside the heater chamber.
 - 18.6.3 Allow water heater to dry for 24 hours in an open area.
 - 18.6.4 If necessary, to purge all fuel from the burner, follow these steps:
 - 18.6.5 Disconnect the feed tube inside the fuel can from the container cap assembly.
 - 18.6.6 Run the heater until the burner shuts down.
- 18.7 **SAFETY NOTES:**
 - 18.7.1 Never put hands or face over the exhaust opening on the Heater Stack.
 - 18.7.2 Never put any item or material down into the Heater Stack.
 - 18.7.3 Never touch the Heater Stack Shroud or Heat Chamber during burner operation.
 - 18.7.4 Always lift the Heater by its handles and hand holders over the wheels.
 - 18.7.5 Always pack and store the Heater in an upright position, never on its side.
 - 18.7.6 Never adjust the set points on the Burner Thermostat controller.
 - 18.7.7 Never cover or limit operation of the Pressure Relief Valve during operation.

SOP 4.11 LifeFlight

SOP 4.11.A: LifeFlight Operations



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.1 DAILY LIFEFLIGHT OPERATIONS
Implementation Date	01 April 2017
Purpose	To provide HMC AS staff members with guidance and relevant information regarding daily operations from Al Udeid Air Base, when participating in the LifeFlight Program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or aeroplane as part of their official duties within the State of Qatar.
2. The Consultant Effectiveness will monitor staff members' compliance with this procedure.
3. **Leaving the LifeFlight hangar at Doha Air Base:**
 - 3.1 Do not leave the hangar premises while on active duty unless deploying on a mission/ tasking.
 - 3.2 In extremis, one crew member may be permitted to leave the hangar and go to the maintenance hangar if necessary.
 - 3.2.1 Always inform the second crew member, the pilots and the flight coordinator and always remain contactable by radio and mobile phone.
 - 3.2.2 One crew member shall remain in the hangar at all times to ensure no delay in flight activation.
 - 3.3 In the event of an aircraft being withdrawn from service due to maintenance, equipment & crew shortages, or weather;
 - 3.3.1 Notify the Consultant Effectiveness and SME: LifeFlight immediately.
 - 3.3.2 The Medical Flight Crew shall be deployed on the LifeFlight Charlie response vehicle for the remainder of their shift or until the aircraft is made available for service.
 - 3.3.3 Under such circumstances, staff are permitted to leave Al Udeid base to fulfil their duty subject to approval by the Consultant Effectiveness.
4. **Standing down of aircraft:**
 - 4.1 In the event of the aircraft being stood down due to maintenance issues, staffing issues or weather, the following procedure is to be followed:
 - 4.1.1 Notify the Consultant Effectiveness, SME: LifeFlight immediately and the Senior Operations Manager (SOM) on call, who shall subsequently notify the Executive Team Member on call.
 - 4.1.2 Notify the Flight Coordinator to update the dispatch system in NCC.
 - 4.2 Aircraft out-of-service (OOS) due to inclement weather for more than one (1) hour:
 - 4.2.1 The crew from the 2nd on-call helicopter shall deploy the LifeFlight Charlie unit until such time that the weather improves.
 - 4.2.2 The Flight Coordinator shall obtain regular updates from the pilots as well as from online weather services every 30 minutes and record this information on the Flight Coordinator report.
 - 4.2.3 One helicopter shall always remain in service in case of an Offshore or International Retrieval mission.

- 4.3 Aircraft OOS due to repairs and maintenance:
 - 4.3.1 The crew shall man the LifeFlight Charlie unit until such time that the aircraft is reported by the Maintenance Engineers to be back in service.
 - 4.3.2 The Flight Coordinator shall get updates from the pilots on an hourly basis and update the Consultant Effectiveness, who shall subsequently update the SOM on call.
 - 4.3.3 Should the aircraft be out of service for consecutive shifts, duty staff should alternate CHLF deployments to distribute the duties fairly.
- 4.4 Aircraft OOS due to staffing issues:
 - 4.4.1 Make all attempts to find replacement medical staff or pilots to fulfil the necessary staff shortage.
 - 4.4.2 In the case of medical crew shortage, the on-duty team member shall be reassigned to other duties, as deemed appropriate by the Consultant Effectiveness, until the crew shortage has been resolved.
 - 4.4.3 In the case of pilot shortage, the medical crew will man the LifeFlight Charlie ground unit until a replacement pilot is found.

5. Re-fuelling of Aircraft:

- 5.1 Standard refuelling of the aircraft:
 - 5.1.1 Shall be conducted by the Maintenance Engineers in conjunction with the Qatar Emiri Air Force (QEAF) refuelling staff at Doha Air Base.
 - 5.1.2 During this time, the aircraft shall be shut down and all staff will remain outside of the aircraft.
- 5.2 Refuelling of the aircraft with LifeFlight crew on board, with engines running:
 - 5.2.1 Both medical crews shall exit the aircraft in communication with the pilots and assist in safely securing the aircraft during refuelling.
 - 5.2.2 Once refuelling is complete, medical crews shall enter the aircraft under the direction of the pilots.
- 5.3 Refuelling of the aircraft with a patient on board:
 - 5.3.1 The door closest to the fuel tank shall remain closed and the door opposite it shall be unlatched but remain closed due to noise and fumes.
 - 5.3.2 At least one medical crew (appropriate to the level of medical care required for the patient) shall be unrestrained and remain with the patient at all times.
 - 5.3.3 The second medical crew shall be outside the aircraft to assist in securing the aircraft during refuelling.
 - 5.3.4 Entry and exit of the aircraft shall be under direction of the pilots.
 - 5.3.5 Due to the high risks involved when refuelling with a patient on board the helicopter, inform the Flight Coordinator immediately to notify the SME: LifeFlight as well as the SOM on call.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.2 HELICOPTER OPERATIONS AT HAMAD GENERAL HOSPITAL
Implementation Date	14 August 2018
Purpose	To provide HMCAS staff members and other role players with guidance and relevant information regarding the safe operation of multiple helicopters approaching Hamad General Hospital (HGH) simultaneously.
Responsibility	ALL LIFEFLIGHT STAFF & OTHER HELICOPTER OPERATORS

1. ABBREVIATIONS:

- 1.1 **FATO:** Final Approach and Take-Off area
- 1.2 **HE:** (HOTEL ECHO) Hamad Emergency West elevated helipad
- 1.3 **HG:** (HOTEL GOLF) Hamad General East elevated helipad
- 1.4 **HGH:** Hamad General Hospital
- 1.5 **HMCAS:** Hamad Medical Corporation Ambulance Service
- 1.6 **NCC:** National Command Centre, State of Qatar
- 1.7 **Stand:** A parking area for aircraft at a heliport

- 2. This procedure shall apply to any situation or instance where more than one helicopter is approaching Hamad General Hospital with the intention of delivering patients to or transporting patients from the hospital.
- 3. Any helicopter operator in the State of Qatar wishing to utilise the helipads at HGH shall comply with this procedure.
- 4. The SME: LifeFlight will be responsible for monitoring compliance with the Standard Operating Procedure.
- 5. The LifeFlight Flight Coordinator in the NCC shall coordinate all helicopter operations into and out of HE.
- 6. HE shall be the default helipad for ALL helicopter traffic.
- 7. HE has 1 FATO and 1 Stand. The first aircraft to land at HE, will vacate the FATO to the helicopter Stand.
- 8. The LifeFlight Flight Coordinator shall in the first instance request a ground ambulance to be dispatched timeously to the correct helipad from the HCC Dispatcher to assist the flight crew with the transfer of the patient to Hamad Hospital Facilities.
- 9. Should the HCC report that there is no available IFT resources, the LifeFlight Flight Coordinator shall in the second instance request a ground ambulance from the NCC Dispatcher to assist with the patient transfer.
- 10. As per the LifeFlight HOTEL ECHO Helipad Activation Process, Romeo 1 shall be informed by either the HCC or NCC Dispatcher to support the responding ground ambulance to access the helipad. See [Appendix 4.11.A.2](#).
- 11. The LifeFlight Flight Coordinator shall communicate with the responding ground ambulance personnel to don full personal protective equipment and not to exit the waiting area without instruction from the Flight Crew.
- 12. The Flight Coordinator shall ensure that approved ground fire safety procedures are activated for every helicopter landing and take-off at designated helipads.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.3 HELICOPTER OPERATING SITES
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding helicopter operating sites within the State of Qatar when participating in the LifeFlight Program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program.
2. The SME: LifeFlight will be responsible for monitoring staff members' compliance with this procedure.
3. **Day Operations:**
 - 3.1 All designated Helicopter Operating Sites (HOS) shall be available for use unless prior 'out of service' notification has been given or the sites are marked for restricted access. See [SOP Appendix 4.7.B-1](#).
 - 3.2 Adhere to pre-notification for these sites, as stated in the HOS document.
4. **Night Operations:**
 - 4.1 Only dedicated night HOS shall be utilized as stated in the relevant HOS document.
 - 4.2 Adhere to pre-notification for these sites, as stated in the HOS document.
5. **HOS Document:**
 - 5.1 The HOS document shall be updated regularly and distributed by SME: LifeFlight whenever changes to the status of HOS sites occur or new sites are added.
 - 5.2 Any additional HOS shall be decided upon by senior management and either accepted or rejected in accordance with HOS standards and approval by the relevant stakeholders.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.4 FLIGHT FOLLOWING
Implementation Date	14 August 2018
Purpose	To provide HMCAS staff members with guidance and relevant information regarding communications, reporting and emergency procedures during flight following, when participating in the LifeFlight Program.
Responsibility	ALL AEROMEDICAL STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program.
2. In conjunction with key stakeholders from partner agencies and HMCAS National Command Centre Operations Officer, the SME: LifeFlight is responsible for ensuring that a robust system of flight following is maintained.
3. This procedure interfaces with the Qatar Emiri Air Force (QEAF) Operating Procedure 60 – 2 and Qatar Air Traffic Control procedures.
4. On a day-to-day basis, the SME: LifeFlight shall delegate the responsibility of flight following to the Flight Coordinator.
5. **Definitions:**
 - 5.1 **Landing Zone (LZ):** Helicopter Landing Site set up on an ad hoc basis, close to the scene of an incident.
 - 5.2 **Helicopter Operating Site (HOS):** Permanent, not purpose-built structure pre-designated for use as a temporary Helicopter Landing Site as and when required e.g. sports pitches and vehicle weigh stations.
 - 5.3 **Heliport:** Permanent structure, purpose-built for helicopter landing, take-off, and related activities.
 - 5.4 **NCC:** National Command Centre of the State of Qatar containing the primary communications centre for the HMCAS.
 - 5.5 **Lost Communications:** The loss of the primary means of communication between the NCC and an aircraft for a period of 5 minutes or more.
 - 5.6 **Flight Following:** The process of tracking the movement and activity of an aircraft from activation through to end of the mission.
 - 5.7 **Aircraft Overdue:** The time at which 15 minutes have elapsed since the ETA at a given location or reporting time.
 - 5.8 **Incident/Accident Procedure:** The actions to be taken when an aircraft is classed as overdue or is anticipated to be overdue as a result of other evidence (e.g. failure to establish communications using the 'lost communications' procedure and/or evidence from SKYTRAC satellite tracking system suggesting that aircraft has been subject to an incident or accident).
 - 5.9 **LifeFlight Flight Coordinator (FC):** The Flight Coordinator operates in conjunction with the HMCAS Dispatch team to provide a seamless service on a 24-hour basis.
6. **Actions at commencement of shift:**
 - 6.1 The Pilot In Command (PIC) shall inform all aircraft crew of pertinent Notices to Airmen (NOTAMs) as part of the shift brief.
 - 6.2 A roster of Persons on Board (POB) shall be transmitted to the FC.

7. LifeFlight Tasking Likelihood:

- 7.1 Zulu case (as per dispatch matrix) within agreed operational geographical response area.
- 7.2 HMCAS Ground Unit requesting assistance from LifeFlight aircraft.
- 7.3 Major Incident Response
- 7.4 Non-emergency events such as training exercises and Public Relations events, etc., based on the submission of a formal Helicopter Tasking Request.

8. Emergency Call and Dispatch:

- 8.1 LifeFlight shall be dispatched to emergency calls as per dispatch matrix ([SOP 3.3](#)).
- 8.2 The FC shall dispatch available LifeFlight assets accordingly, as detailed below.
- 8.3 During the call receipt and dispatch phase, all relevant information shall be captured within the existing Computer Aided Dispatch (CAD) system.

9. Start-up and take off of the LifeFlight Aircraft:

- 9.1 Maintain a 'sterile cockpit' throughout the start-up and take-off phase.
- 9.2 Irrespective of the nature of the mission, once the decision has been made to task a helicopter, the following actions shall be taken:
 - 9.2.1 Flight Coordinator:
 - 9.2.1.1 Notify QEAF Engineering personnel of tasking
 - 9.2.1.2 Confirm location and Grid Reference of the patient/incident/event to which the aircraft is being dispatched.
 - 9.2.1.3 Notify aircraft of Grid Reference of incident/event
 - 9.2.1.4 Notify Air Traffic Control of tasking
 - 9.2.1.5 Logs time of aircraft departure from Base and during mission.
 - 9.2.1.6 Notify appropriate personnel at Heliports/HOS that are likely to be used (enables the appropriate action to be taken and agencies to be present).
 - 9.2.2 Aircraft Crew:
 - 9.2.2.1 Transmit 'start-up' radio call
 - 9.2.2.2 Transmit 'taxi' radio call
 - 9.2.2.3 Transmit 'lifting' radio call

10. Aircraft Airborne:

- 10.1 Once the aircraft is airborne the following actions shall be taken:
 - 10.1.1 Aircraft Crew:
 - 10.1.1.1 Transmit 'airborne' radio call, including ETA, fuel endurance and POB.
 - 10.1.1.2 Transmit flight report every 10 minutes – to include Position, ETA and 'Ops Normal' message (or details of any variance from 'Ops normal').
 - 10.1.1.3 Transmit 'scene visual' radio call once on station over the destination LZ/scene.
 - 10.1.1.4 Establish communications with Ground Unit(s) as appropriate using TETRA Network on the 'AirMed-1' talk group.

10.1.2 Flight Coordinator:

10.1.2.1 Log details of all calls, flight reports and log SKYTRAC information every 10 minutes.

10.1.2.2 Transmit details of LZ/HOS including hazards, suggested direction of approach, etc. to the aircraft.

10.1.2.3 Transmit relevant details of patient/incident to the aircraft.

10.1.3 Coordinating Ground Unit (usually a Delta call-sign):

10.1.3.1 Establish communications with Aircraft on 'AirMed 1' talk group.

10.1.3.2 Notify aircraft of hazards, wind direction, possible approach etc. (either by direct transmission or relayed through the FC).

10.1.3.3 Inform of location of rendezvous points.

10.1.3.4 Coordinate ground transport movement at the scene (in conjunction with other agencies).

10.1.3.5 Transmit updates from scene as necessary.

11. Final Approach, Landing and Loading:

11.1 Maintain a 'sterile cockpit' throughout the final approach and landing phase however, radio silence may be broken if critical, life-saving information need to be communicated (e.g. unsafe LZ conditions whilst the aircraft is on short finals).

11.2 The aircraft crew shall transmit the following radio calls as appropriate and shall be logged by the FC:

11.2.1 *'Finals'*

11.2.2 *'Landed'*

11.2.3 *'Hot Load' or 'Shutting Down'*

12. Subsequent mission activities:

12.1 For the remainder of the mission, all subsequent take-off, flight legs and landings shall be managed using the reporting/logging procedures highlighted at sections 6-8 above. In addition to this, the following actions shall be taken:

12.1.1 Flight Coordinator Actions:

12.1.1.1 Transmit details of subsequent LZ/HOS Sites/Heliport to be used.

12.1.1.2 Record take-off time from scene/pick up point.

12.1.1.3 Record landing time at drop off point/receiving facility.

12.1.1.4 Record take-off time from drop off point/receiving facility.

12.1.1.5 Record landing time back at base.

12.1.1.6 Record the time the helicopter is available again (after refuelling & restocking).

12.1.2 Gain consent to transport.

12.1.2.1 The patient, or a relative with authority to act on their behalf, must give their consent to flight.

12.1.2.2 Companions to paediatric patients are permitted following a risk assessment by the Pilot in Command in collaboration with the medical crew.

- 12.1.2.3 Where a patient or relative refuses to be flown against the advice of the senior clinician present, they are to sign the HMCAS ePCR to indicate refusal. Refer to [SOP 4.8.D](#). They may be discharged at scene or be transported to receiving facility by a Ground Unit if flight is refused.
- 12.1.2.4 In the case of violent/combative patients or aggressive companions, a risk assessment shall be made by the Pilot in Command in collaboration with the medical crew. Transport may be refused if the patient is deemed to pose a threat to the aircraft and/or crew.
- 12.1.2.5 Under no circumstances will any patients with known communicable infections/diseases be transported in any LifeFlight aircraft.
- 12.1.3 Notification of Receiving Facility:
 - 12.1.3.1 Refer to SOP [4.8.A](#) for selecting appropriate receiving health facilities.
 - 12.1.3.2 The FC shall notify the receiving facility (either directly or via the NCC Clinical Desk) of the patient's details and other pertinent information, including in-flight updates and shall confirm bed availability.
 - 12.1.3.3 Aircraft crew shall transmit any changes to current ETA, fuel endurance and POB as changes occur.

13. End of Mission:

- 13.1 The mission ends once the aircraft has returned to base and is shut down.
- 13.2 When an aircraft is re-tasked before returning to base, end of mission is considered to be the time at which the aircraft takes off from its last landing point, or the time at which the aircraft switches to the new mission (if no patient is on board).

14. Incident and Accident:

- 14.1 In the event of an aircraft incident (lost communications and aircraft overdue) or an aircraft accident (**See Figure 1 below**):
 - 14.1.1 If 5 minutes after contact window has expired, the FC shall:
 - 14.1.1.1 Attempt to contact the aircraft using alternative communications means (aircraft GSM telephone, aircraft Satellite phone, SkyMail application, aircrew personal mobile phones, HMCAS resources on scene at aircraft's destination).
 - 14.1.1.2 Simultaneously check SKYTRAC and inform Operations Officer: NCC.
 - 14.1.1.3 Notify the SME: LifeFlight with the following brief:
 - 14.1.1.3.1 Aircraft identification
 - 14.1.1.3.2 Last known position
 - 14.1.1.3.3 Direction of Travel
 - 14.1.1.3.4 Names of POB
 - 14.1.1.3.5 Phase of mission and last fuel endurance reported
 - 14.1.1.3.6 Actions taken to locate
 - 14.1.2 If 15 minutes after contact window has expired the FC shall:
 - 14.1.2.1 Inform destination LZ and confirm any visual contact with aircraft.
 - 14.1.2.2 Notify the SME: LifeFlight and Operations Officer: NCC.

- 14.1.2.3 Dispatch second call LifeFlight aircraft for aerial reconnaissance of last known position as per SKYTRAC.
- 14.1.2.4 Under guidance from the SME: LifeFlight, notify the following agencies:
 - 14.1.2.4.1 Qatar Armed Forces General Headquarters (GHQ).
 - 14.1.2.4.2 QEAF Air Operations Command (AOC) – requesting Search and Rescue activation.
 - 14.1.2.4.3 Qatar Police Department
 - 14.1.2.4.4 Qatar Civil Defence
 - 14.1.2.4.5 Air Traffic Control Services at both Doha International Airport and Hamad International Airport.
 - 14.1.2.4.6 Qatar Civil Aviation Authority
- 14.1.2.5 Upon notification and briefing, the SME: LifeFlight shall notify the HMCAS Executive Management team and brief them accordingly.

15. Aircraft Accident/Prolonged Overdue Procedure:

- 15.1 When an accident is confirmed, or an aircraft has been overdue for a period of more than thirty minutes, the following action shall be taken:
 - 15.1.1 Compile hard copy transcripts of all reports, recordings and documentation relating to the incident/accident to include the following:
 - 15.1.1.1 ProQA screenshot printouts
 - 15.1.1.2 CAD screenshot printouts
 - 15.1.1.3 SKYTRAC tracking logs of the last mission for the relevant aircraft.
 - 15.1.1.4 Recordings of all data and voice transmissions as well as relevant telephone calls.
 - 15.1.1.5 Relevant screenshots from the Qatar Geo-Portal Ambulance Response System.
 - 15.1.1.6 POB roster with emergency contact details for relatives.
 - 15.1.2 An immediate blanket embargo shall be imposed on ALL personnel making unofficial communication (written, spoken and electronic) to the public (including friends and family) and the media (including TV companies, radio companies, newspapers, social media, websites, and blogs).
- 15.2 Once wreckage has been discovered, response to the incident shall take place as appropriate. The QEAF shall lead scene management and investigation.
- 15.3 Where Next of Kin are resident outside the State of Qatar, notification of incident will be facilitated via the staff member's Embassy.
- 15.4 Once Next of Kin have been informed, the Executive Director: HMCAS will make a statement to the media followed by further statements to the HMCAS internal audience.

16. Incident/Accident Drills:

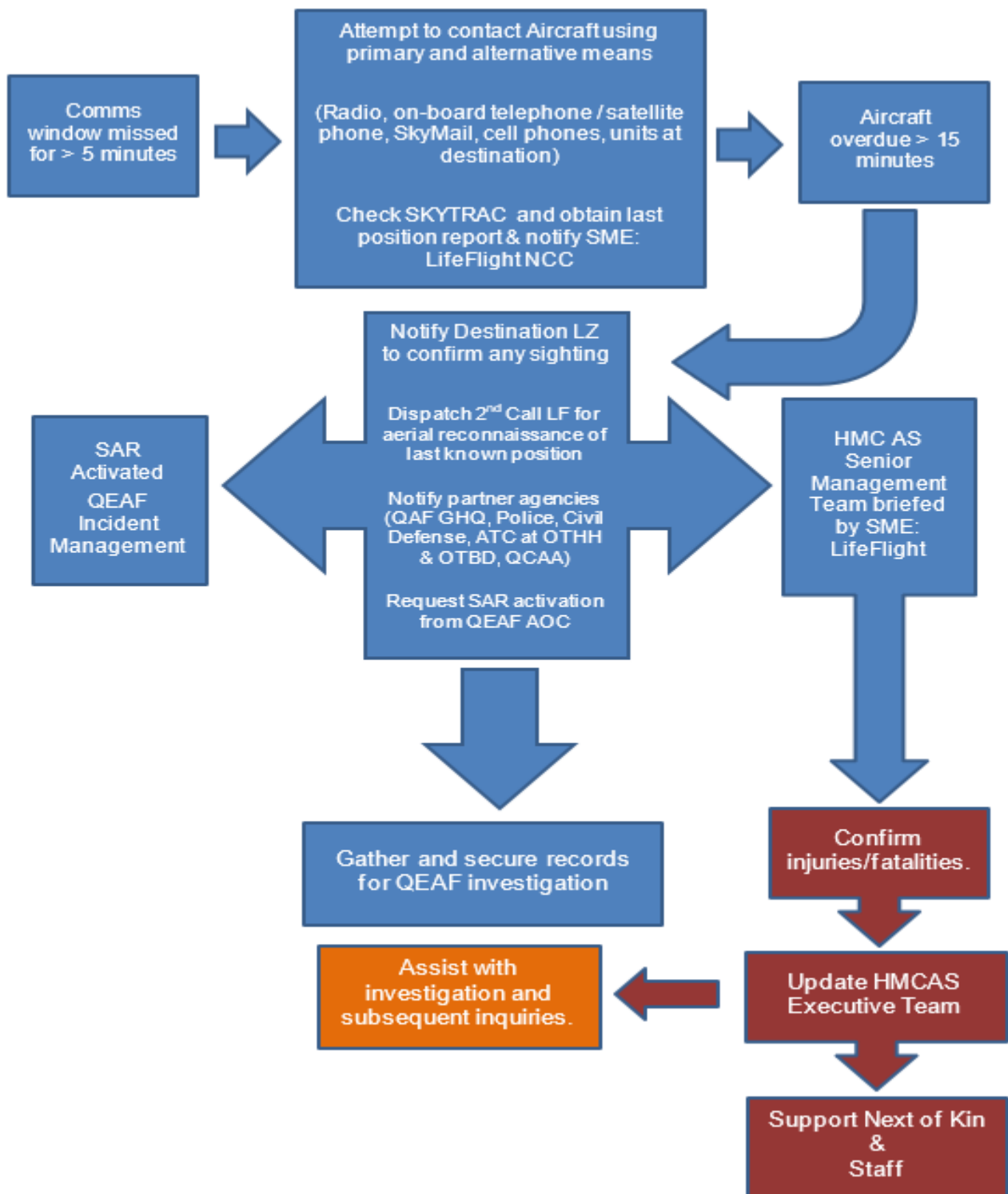
- 16.1 Action drills shall be conducted on a quarterly basis to practice the Incident/Accident Plan and shall involve the following stakeholders:
 - 16.1.1 Pilots
 - 16.1.2 Medical Crew

16.1.3 NCC Personnel

16.1.4 QEAF representatives

16.1.5 HMCAS Administrative Personnel

Figure 1



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.5 LIFEFLIGHT CREW CONFIGURATION & SCHEDULING
Implementation Date	01 April 2017
Purpose	To provide HMC AS staff members with guidance and relevant information regarding the configuration of LifeFlight crew and scheduling when participating in the LifeFlight Program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or aeroplane as part of their official duties within the State of Qatar.
2. The SME: LifeFlight will be responsible for monitoring staff fitness levels as well as their general health and compliance with this procedure.
3. **LifeFlight Crew Configuration:**
 - 3.1 All medical flights shall maintain a minimum of two medical attendants with at least one Critical Care Paramedic (CCP).
 - 3.2 All LifeFlight crew shall adhere to active flight crew requirements and must be deemed fit to fly at all times ([SOP 4.11.B.3](#)).
 - 3.3 In an emergency and on an unanticipated, infrequent basis, non-scheduled or ad hoc LifeFlight staff may be added as an additional medical team member only if they meet prescribed flight crew requirements ([SOP 4.11.B.2](#)) and are deemed fit to fly. ([SOP 4.11.B.3](#)).
 - 3.4 HMCAS Managers may be scheduled to work as ad hoc crew provided that they meet prescribed flight crew requirements ([SOP 4.11.B.2](#)) and are deemed fit to fly. ([SOP 4.11.B.3](#)).
4. **International Retrieval Missions:**
 - 4.1 There shall always be a minimum of two CCPs as part of the medical team.
 - 4.2 All staff shall adhere to Flight and Duty times. ([SOP 4.11.B.3](#)).
 - 4.3 Doctors seconded to LifeFlight for International Retrieval Missions shall:
 - 4.3.1 Adhere to prescribed flight crew requirements ([SOP 4.11.B.2](#));
 - 4.3.2 Complete the prescribed Specialized High Acuity Adult Retrieval training program;
 - 4.3.3 Be deemed fit to fly ([SOP 4.11.B.3](#)).
5. **Duty Schedule:**
 - 5.1 Adhere to the Flight and Duty hours stipulated in ([SOP 4.11.B.3](#)).
 - 5.2 A Day Shift shall be scheduled from 04h45 to 16h45 and a Night Shift from 16h45 to 04h45.
 - 5.3 Do not exceed 16 hours continuous duty in a 24-hour period unless attending a late call-out.
 - 5.4 The Workforce Management Department designee shall notify (in writing) all LifeFlight staff if there are any changes to shift times with a minimum of 48 hours notice.
 - 5.5 The Workforce Management Department designee shall produce the LifeFlight roster and shall be approved by the Consultant Effectiveness.
 - 5.6 The roster shall be completed and distributed to staff at least two weeks in advance.
 - 5.7 A minimum of two Flight CCP's (F-CCP) and two Flight AP's (F-AP) shall be scheduled per shift to man the two aircraft on duty.

- 5.8 One F-AP will be scheduled per shift to perform Flight Coordinator duties within the NCC.
- 5.9 Medical crew shall be scheduled on a four-day-on, four-day-off rotation.
- 5.10 Over-time shifts may be undertaken in line with prescribed rest periods and duty limitations. Written approval must be obtained from the Consultant Effectiveness to ensure duty restrictions are adhered to.
- 5.11 On request, married couples in the LifeFlight program may be scheduled in the same team.
- 5.12 LifeFlight crew shall be scheduled alternating shifts on the LifeFlight aircraft and on Charlie ground units to ensure skill maintenance, as operational coverage needs dictates.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.6 GENERAL & CLINICAL RESPONSIBILITIES DURING A LIFEFLIGHT SHIFT
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding general and clinical responsibilities, when participating in the LifeFlight Program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or aeroplane as part of their official duties within the State of Qatar.
2. The SME: LifeFlight will be responsible for monitoring staff members compliance with this procedure
3. **Checklists:**
 - 3.1 Complete the prescribed checklists (according to the associated technical standard) during every operational duty:
 - 3.1.1 Complete a Shift checklist for the aircraft and/or the LifeFlight Charlie vehicle(s) at the commencement of every **12-hour** shift period.
 - 3.1.2 If scheduled on a day duty on the last Thursday of the month, complete a monthly comprehensive expiry date checklist.
 - 3.2 Submit all checklists daily, via the predetermined checklist drop-off location or electronically to the Production Supervisor: LifeFlight.
4. **Patient Care Continuity and Reporting:**
 - 4.1 Provide ethical, respectful, and confidential patient care to all patients as prescribed by the HMCAS Clinical Practice Guidelines.
 - 4.2 Transfer of patient care from the medical team on the aircraft shall be to a medical team at the receiving hospital.
 - 4.3 For general patient handover and endorsement procedures, refer to [SOP 4.8.B](#).
 - 4.4 For appropriate recording of patient care and handling of documentation, refer to [SOP 4.9.A](#).
 - 4.5 Complete all other documentation included in continuous quality and assessment as required.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.7 DECEASED PATIENT ON LIFEFLIGHT
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding the deceased patient whilst participating in the LifeFlight program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or aeroplane as part of their official duties within the State of Qatar.
2. The SME: LifeFlight will be responsible for monitoring staff compliance with this procedure.
3. **Death in Flight:**
 - 3.1 In the event that a death occurs during the transportation of a patient in the LifeFlight aircraft:
 - 3.1.1 Notify the Flight Coordinator immediately who shall notify the Consultant Effectiveness on duty.
 - 3.1.2 Continue flying to the receiving facility
 - 3.1.3 Handover the deceased to the necessary staff at hospital as per [SOP 4.8.B](#).
4. **Deceased patient on scene with no medical intervention by HMCAS/LifeFlight:**
 - 4.1 In the event that there is a deceased patient on scene who did not receive any medical intervention, follow standard HMCAS procedures.
 - 4.2 The Police shall remove the deceased from the scene.
 - 4.3 In rare instances such as a deceased patient in an offshore location, refer to the SME: LifeFlight or Consultant Effectiveness for advice.
5. **Deceased patient on scene with medical intervention by HMCAS/LifeFlight:**
 - 5.1 In the event that a patient has received medical interventions by HMC AS and/or LifeFlight medical crew and has been declared dead on scene:
 - 5.1.1 The ground ambulance crew shall move the deceased to hospital.
 - 5.1.2 If required, the LifeFlight medical crew shall complete all necessary documentation and handover such to the ground ambulance crew.
 - 5.1.3 Notify the Flight Coordinator if LifeFlight staff are the only medical crew on scene and await further instructions from the Consultant Effectiveness on duty.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.8 NON-EMERGENCY AND AD HOC LIFEFLIGHT MISSIONS
Implementation Date	01 April 2017
Purpose	To outline the procedures for LifeFlight helicopter operations when tasked with non-emergency, ad hoc missions including training and public relations missions.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or aeroplane as part of their official duties within the State of Qatar.
2. The SME; LifeFlight will be responsible for monitoring staff compliance with this procedure.
3. **Definitions:**
 - 3.1 Non-Emergency Ad Hoc Mission: Any LifeFlight mission not dispatched by the NCC as part of an emergency medical response.
 - 3.2 Training Mission: Any mission undertaken for the purpose of training LifeFlight crew members or any other staff associated with LifeFlight operations. This also includes participation of LifeFlight helicopters in joint training exercises with other agencies.
 - 3.3 Event Standby Missions: Any mission undertaken to provide medical coverage for a pre-arranged event.
 - 3.4 Public Relations Mission: Any mission undertaken in support of HMCAS Public Relations Department or independently to promote the LifeFlight program.
 - 3.5 Ad Hoc Landing Zone: Any Landing Zone not used as part of the normal operations of the LifeFlight program. These are typically located close to the event, exercise or PR campaign and are typically unmarked.
4. **Requests:**
 - 4.1 All requests for the involvement/participation of LifeFlight helicopters or crew in Public Relations or marketing activities shall be directed, in writing, to the Executive Director: HMCAS.
 - 4.2 All other requests for involvement/participation of LifeFlight helicopters or crew must be directed, in writing, to the SME: LifeFlight.
 - 4.3 These requests shall be reviewed subject to the following criteria:
 - 4.3.1 Relevance
 - 4.3.2 Aircraft availability
 - 4.3.3 Staff availability
 - 4.3.4 Safety restrictions
5. **Authorizations:**
 - 5.1 The Executive Director: HMCAS shall authorize all non-emergency, ad hoc missions related to public relations or marketing.
 - 5.2 The Assistant Executive Director of Critical Care Services shall authorize all non-emergency, ad hoc missions related to training, special events or orientation flights.

6. LifeFlight Tasking:

- 6.1 Following receipt of a letter from the requesting agency/organization or an internal request via the LifeFlight Tasking Request Form (Form No: HMCAS/A/LF/2), the SME: LifeFlight or a delegated representative shall:
 - 6.1.1 Meet with the requesting agency/organization at the site and identify potential ad hoc landing zones with photos (where applicable), GPS coordinates and potential hazards around the landing zone.
 - 6.1.2 Complete the HMCAS/A/LF/1 form
 - 6.1.3 Authorize tasking as appropriate
 - 6.1.4 Distribute authorized tasking (in PDF format) to predetermined distribution list via e-mail.
 - 6.1.5 Brief (telephonic/in person) the Flight Commander: Training, 23 (EMS) Squadron (QEAF) where applicable.
 - 6.1.6 Brief (telephonic/in person) the HMCAS Flight Coordinator, Pilot in Command and medical flight crew executing the tasking.
 - 6.1.7 Confirm any additional arrangements (e.g. other HMCAS resource deployment, involvement of other agencies, etc.)
 - 6.1.8 Execute tasking mission
 - 6.1.9 Debrief (telephonic/in person) the Pilot in Command and medical flight crew involved.
 - 6.1.10 Feedback to HMCAS Senior Management as and when required.

7. Radio Communications during Non-Emergency/Ad Hoc Missions:

- 7.1 Maintain standard radio communications and flight following between crew and Flight Coordinator.
- 7.2 For some special events & training exercises that require the monitoring of additional TETRA radio channels, utilize a portable TETRA radio, bearing in mind that airborne communications on this radio may not be possible.

ENQUIRIES

Mr R Muller
Mobile +974 6625 5628
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خدمة الإسعاف
Ambulance Service

LIFELIGHT SPECIAL EVENT/ EMS TASKING REQUEST

1	DATE OF REQUEST:			
2	REQUESTED BY:			
	Name		Office	
	Designation		Mobile	
	email		Fax	
3	DATE OF EVENT:		TIME: From:	To:
4	NAME OF EVENT:			
5	DESCRIPTION OF EVENT:			
6	DESCRIPTION OF LANDING AREA:			
	Name of Premises/ Location		GPS Coordinates	
	Physical Address			
	Surface	Size	Other	
	Hazards/ Obstacles			
7	CONTACT PERSON DURING EVENT:			
	Name		Office	
	Designation		Mobile	
	email		Fax	
8	WILL REFRESHMENTS BE SUPPLIED BY EVENT ORGANISERS?			

Form No: HMCAS/A/LF/2

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.9 LEAVE AND DUTY EXCHANGE ON LIFEFLIGHT
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding annual leave, sick leave, casual leave, injury on duty and duty exchanges whilst on the LifeFlight program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program.
2. The SME: LifeFlight will be responsible for monitoring staff compliance with this procedure.
3. **Annual Leave:**
 - 3.1 An Annual Leave Planner shall be projected by the Workforce Management Department designee and approved by Consultant Effectiveness and Operations Manager: Critical Care Services.
 - 3.2 No more than two CCP's and two FAP's shall be permitted leave within the same month to ensure adequate staff coverage for duties, unless approved by the Consultant Effectiveness.
 - 3.3 Submit all written leave applications to the Consultant Effectiveness no less than two months prior to leave dates requested.
 - 3.4 The Consultant Effectiveness shall review the leave request and approve/refuse the application within 7 days.
 - 3.5 Leave applications may be permitted within a shorter notice period if pre-approved by the Consultant Effectiveness.
 - 3.6 Emergency leave may be approved at short notice at the discretion of the Consultant Effectiveness.
4. **Casual Leave:**
 - 4.1 Casual leave requests shall be permitted if another staff member is able to fulfil the requested duty (depending on operational availability).
 - 4.2 A leave request form shall be submitted to the Consultant Effectiveness no less than 7 days prior to date requested.
 - 4.3 The Consultant Effectiveness shall review the leave request and approve/refuse the application within 2 days.
5. **Sick Leave:**
 - 5.1 Please refer to general sick leave procedures in [SOP 1.3](#).
 - 5.2 Staff shall inform the Workforce Management Department designee of the outcome of their visit i.e. fit for work or booked off duty and expected duration.
6. **Injury on Duty:**
 - 6.1 Immediately notify the Flight Coordinator if an injury has been sustained whilst on duty.
 - 6.2 The Flight Coordinator shall immediately inform the Consultant Effectiveness and the Operations Manager: CCS on call.
 - 6.3 Follow sick leave procedure detailed in [SOP 1.3](#).

7. Duty Exchanges:

- 7.1 No more than three duty exchanges per month shall be permitted.
- 7.2 Duty exchanges shall not be permitted for the shift cycle immediately before or after approved annual leave dates.
- 7.3 Duty exchange forms shall be completed and submitted electronically to the Consultant Effectiveness with a minimum of 3 days notice prior to the shift requested
- 7.4 The Consultant Effectiveness shall approve/refuse the request and notify the Workforce Management Department accordingly.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.10 MANAGEMENT OF BARIATRIC PATIENTS BY LIFEFLIGHT
Implementation Date	24 October 2017
Purpose	To provide HMCAS staff members with guidance and relevant information to facilitate appropriate transportation and triage of bariatric patients by LifeFlight.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or aeroplane as part of their official duties within the State of Qatar.
2. Whilst there are varied definitions of Bariatric; in the Aeromedical environment, the classification is applied to any person whose weight or physical dimensions exceed the capability of standard equipment used by the HMC Ambulance Service.
3. The procedure ensures that the Medical Flight Crew evaluates the capabilities of the stretcher system being used in the LifeFlight aircraft to determine its appropriateness for each patient's transportation.
4. Due to the logistical challenges with managing these patients, these missions are commonly longer than the non-bariatric transportation.
5. This procedure should be read in conjunction with the Transfer and Retrieval Services Bariatric Transfers [SOP 4.8.T](#).
6. LifeFlight Bariatric Patient Decision Tool:
 - 6.1 The purpose of the tool is to provide the LifeFlight Medical Crew with adequate information as to the configuration and capabilities within the patient compartment of the AW139 aircraft.
 - 6.2 Maximum patient length – consider the actual length of the aircraft stretcher: 1980mm.
 - 6.3 Maximum patient shoulder width – ensure that the patient can be secured in the aircraft and does not present an egress hazard to the crew (see fig A).

6.3.1 Figure A



- 6.4 Maximum length of safety belts – make conservative judgements when securing the patient onto the stretcher (see fig B).

- 6.5 Weight and balance calculations are absolute to ensure Category - A performance at elevated heliports and patient dimensions should not compromise flight safety.
- 6.6 The Pilot in Command assumes final authority for all safety and operational decisions.

6.6.1 Figure B



7. Bariatric Unit Activation:

- 7.1 Patients exceeding the equipment or aircraft limitations should be referred to the National Command Centre for Bariatric Unit dispatch (as per [SOP 4.8.T](#)).
- 7.2 Whilst awaiting the appropriate assistance, the patient must receive all necessary care and reassurance as outlined in the HMCAS Clinical Practice Guidelines.
- 7.3 The patient's delay in transportation must be professionally communicated, explaining the safety limitation within the aircraft, with the patient or responsible person.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.11 LIFEFLIGHT HOT LOADING PROCEDURE
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding LifeFlight hot loading procedures.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program.
2. The procedure outlines fundamental safety considerations when undertaking Hot Load activities.
3. The Flight CCP shall be responsible for ensuring overall scene safety and compliance with the Hot Load procedure.
4. The SME: will be responsible for monitoring staff compliance with this procedure.
5. Normal operating procedure shall be to load and unload patients with the aircraft engines shut down.
6. **Definition of Hot Load:** In certain circumstances, it might be necessary to load a patient while the rotors are turning, and the Pilot in Command maintains power to the main and tail rotors. This is called a "Hot Load" and may need to be performed due to:
 - 6.1 Low aircraft voltage after a short transit
 - 6.2 Low ambient temperature which may affect restart
 - 6.3 The high ambient temperature requiring a long period (25 to 30 minutes) for the engines to cool sufficiently to be restarted.
7. The left side stretcher will be the default Hot Load stretcher, unless circumstances dictate otherwise and have been agreed by the crew prior to landing.
8. Upon arrival at the designated landing zone, the LifeFlight CCP shall exit the aircraft and go outside the rotor disk to the on-scene personnel while the pilot maintains power to the rotors.
 - 8.1 The LifeFlight AP shall exit the aircraft to the 2 o' clock position outside the rotor disc to provide safety, with direct sight of the tail rotor and provide information to the Pilot in Command.
 - 8.2 Should the Flight AP be required to assist the patient, the Non-Flying Pilot will exit the aircraft to the 10 o' clock position to provide scene safety.
 - 8.3 At all times, there shall be communication between the Pilots and medical crew utilizing the Polycon® or TETRA® radio systems.
 - 8.4 No civilian bystanders will approach or assist in transferring the patient to the aircraft.
 - 8.5 Loading and unloading must be carried out under the supervision of the Non-Flying Pilot. Great care must be taken in these circumstances to ensure that in the "heat of the moment" loading personnel and/or onlookers do not approach the tail rotor without supervision.
 - 8.6 Only appropriately briefed HMC Ambulance Service personnel with appropriate PPE (eye and hearing protection) will assist with the manual handling of the patient to the aircraft.
 - 8.7 The duty medical flight crew shall ensure that all patients, whenever possible, wear hearing protection. (As a general rule, patients will not be on the headset intercom system).
9. Due to risk of injury, Pilots should not participate with the lifting of patients. If Pilot assistance becomes necessary however, then only the Co-Pilot should assist.

- 10.** In certain circumstances, it may be necessary to land and deploy the medical crew and primary response equipment. The aircraft will then take off and remain in a safe orbit or reposition the aircraft to a safe location away from the scene, awaiting further instructions. This procedure may need to be performed due to:
- 10.1 Unpredictable traffic or bystander conditions
 - 10.2 Unstable landing surface
 - 10.3 Remote location of scene from acceptable access

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.12 LIFEFLIGHT CONFIGURATION & PASSENGER SAFETY
Implementation Date	24 October 2017
Purpose	To provide HMCAS staff members with guidance and relevant information on the aircraft configuration and passenger safety information for operational service with LifeFlight.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. Medical Flight Crew members and any HMC staff member seeking to serve on LifeFlight will comply with this procedure.
2. The SME: LifeFlight will be responsible for monitoring staff flight crew status and compliance with this procedure.
3. **Aircraft Configuration:**
 - 3.1 **1 - Patient configuration:** (Staff orientation, Retrieval mission or Offshore deployment):
 - 3.1.1 Consists of 1 aircraft stretcher and 2 attendant seats as a minimum.
 - 3.1.2 As per the supplement to the AB/AW 139 Rotorcraft Flight Manual, up to 4 attendant seats may be accommodated.
 - 3.2 **2 - Patient Configuration:** (Routine LifeFlight Operational Model):
 - 3.2.1 Consists of 2 aircraft stretchers and 2 attendant seats as a maximum.
 - 3.2.2 As per the supplement to the AB/AW 139 Rotorcraft Flight Manual, up to 2 attendant seats may be accommodated.
 - 3.2.3 LifeFlight operations shall not transport more than 1 patient in the 1-patient configuration, or more than 2 patients in the 2-patient configuration.
4. **Passenger/Patient Safety Briefing:**
 - 4.1 LifeFlight Medical Crew shall provide a safety briefing to all passengers and competent patients prior to departure including:
 - 4.1.1 **S**eatbelts (operation and fastened for take-off, taxiing & landing)
 - 4.1.2 **A**ction in case of passenger discomfort
 - 4.1.3 **F**ire Extinguisher (location and operation)
 - 4.1.4 **E**mergencies (exit and evacuation plan)
 - 4.1.5 **T**alking (patients will generally not be on headset, use of ear defenders for excess noise)
 - 4.1.6 **Y**our questions (encourage patient/passenger to speak if they feel ill or nervous)
 - 4.1.7 Use quick reference card for hot loading or language barrier.
5. **Patient Escorts and Accompaniment:**
 - 5.1 Escorts are not routinely transported unless in exceptional circumstances following a risk assessment by the Pilot in Command in collaboration with the medical crew.
 - 5.2 Deviation from standard operating procedure requires escalation to the NCC and SME: LifeFlight.

- 5.3 Companions to paediatric patients are permitted following a risk assessment by the Pilot in Command in collaboration with the medical crew.



APPROACHING/LEAVING HELICOPTER



ATTENTION

!!Communication will be difficult during flight due to noise!!
Crew might need to apply monitoring equipment to ensure you get best possible care, some include:



Stickers on chest to monitor Heart



Oxygen through nose



Oxygen Monitor on finger



Blood Pressure



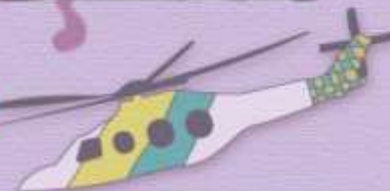
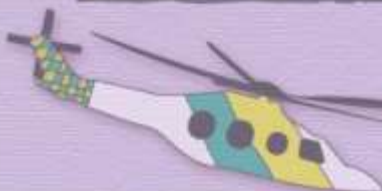
IMPORTANT



Always

Follow the Instructions of the Flight Crew
Remain Seated

Keep arms and legs within the zone of the stretcher
Stay in aircraft unless crew allows exit and escorts away
Keep Ear protection on
Keep loose items secure



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.11.A.13 LATEX ALLERGY PRECAUTIONS
Implementation Date	24 October 2017
Purpose	To provide HMCAS staff members with the guidance and relevant information regarding latex allergies of staff and patients.
Responsibility	ALL OPERATIONAL STAFF

1. Staff and latex allergy:

- 1.1 All staff shall notify their immediate supervisor of any personal allergies, including latex allergy.
- 1.2 A record of staff allergies shall be noted in their personnel files.
- 1.3 Staff with latex allergies shall inform fellow crew members to prevent accidental latex contact during the shift.

2. Patients and latex allergy:

- 2.1 Obtain an appropriate history during routine clinical assessment to identify the risk of latex allergy.
- 2.2 A latex allergy shall be noted by all medical staff managing the patient and annotated on the ePCR and handed over to hospital staff on arrival at the hospital prior to any staff being in contact with the patient.
- 2.3 When possible, every effort must be made to obtain an allergy history for unconscious patients.

3. Equipment:

- 3.1 A list of equipment containing latex shall be made available to staff and in-patient treating vehicles, including aircraft.
- 3.2 Latex free gloves shall be available for use by staff with latex allergy and on patients with latex allergy.
- 3.3 All staff shall be vigilant of which equipment is safe to use on patients with latex allergy.
- 3.4 Be prepared and follow the relevant Clinical Practice Guideline in the event of a latex allergic reaction.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.A.14 LIFEFLIGHT OFFSHORE ACTIVATION AND OPERATIONS
Implementation Date	13 March 2018
Purpose	To provide LifeFlight personnel with safety performance goals necessary to assure safe offshore helicopter operations. The framework is aligned with QEAF (Qatar Emiri Air Force) standards for safe mission pre-flight planning and preparation.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. Background and Purpose:

- 1.1 This regularly receives requests to transport patients from offshore areas including oil rigs, vessels and islands to hospital. Due to the limited access, these cases are serviced by LifeFlight.
- 1.2 This SOP provides the procedures for LifeFlight offshore missions activation and coordination specific the LifeFlight Coordinator and operational LifeFlight staff and outlines fundamental pre-flight planning and safety considerations when undertaking offshore operations.
- 1.3 This SOP should be read together with [SOP 3.24](#) which sets out the procedures relating to the NCC call-taking and dispatch of offshore cases.

2. Definitions:

- 2.1 **A7S** stands for “Alpha 7 Sierra” which is the Joint Operations Centre operated by Qatar Energy and, among other disaster response responsibilities, is responsible for coordinating offshore medical evacuations.
- 2.2 **LFC** means LifeFlight Coordinator
- 2.3 **DTL** means Distribution Team Leader
- 2.4 **PiC** means Pilot in Command

3. LifeFlight Activation and Following:

- 3.1 In accordance with [SOP 3.24](#), the LFC will be notified as soon as possible of an incoming offshore case from A7S.
 - 3.1.1 The LFC must immediately proceed to their console, add a note in NJM that they were notified of the case, open the LifeFlight@hamad.qa mailbox and monitor the inbox for the Offshore Retrieval Request Form from A7S.
 - 3.1.2 The LFC must advise the dispatcher of which aircraft to assign to the case.
 - 3.1.3 The LFC must notify the assigned LifeFlight crew of the incoming case providing the general location (specific locations should only be provided once confirmed with A7S).
 - 3.1.4 The LFC must open DaWinci and locate the vessel using the helideck code and name provided in the Offshore Retrieval Request form. The location of the vessel should be monitored until LF report on-station.
 - 3.1.5 The LFC must assess the flight conditions including weather, visibility, wind and any other conditions that may inhibit safe mission activation or impose unreasonable risk to the aircraft or crew. If necessary, the LFC should discuss any concerns with the PiC.
- 3.2 In accordance with [SOP 3.24](#), once the NCC call-taker has completed ProQA interrogation with the A7S caller, they will endorse the case to the LFC directly or through the DTL.
 - 3.2.1 The LFC must immediately update the LF call-sign timestamp to “Received” in NJM.

- 3.2.2 The LFC must then call A7S, request to speak to the sender of the Offshore Retrieval Request Form (as named on the form) and confirm the below details, by reading them back to the A7S caller:
 - 3.2.2.1 Specific helideck name, code and GPS coordinates, and confirm this is the location of the patient.
 - 3.2.2.2 Nearest refuelling point (name, code, and GPS coordinates).
 - 3.2.2.3 Confirm if there is any infectious disease or contamination. If there is hazardous material contamination, they must confirm the patient is undergoing decontamination procedures and advise A7S we are unable to transport patients contaminated with hazardous materials.
- 3.2.3 After completing the phone call with A7S (3.2.2 above) the LFC will send the Offshore Retrieval Request Form to the LifeFlight CCA on MS Teams and contact the crew on Airmed 1 to dispatch the aircraft, confirming the case and ensuring the flight crew have received the Offshore Retrieval Request Form in MS Teams.
 - 3.2.3.1 The LFC must update the NJM timestamp of the aircraft to "LF-Activated" and update the DTL. The LFC should ensure to update NJM timestamps and notes throughout the mission with all flight tracking information.
- 3.2.4 Once the LFC knows the ETA of the aircraft to the offshore helideck they must note it in NJM, update the DTL and call A7S back advising them of the ETA. If necessary, the LFC may now obtain further patient information which can be provided to the medical team.
- 3.3 The LFC must continue to track the progress of the mission confirming mission status and aircraft location at least every 10-minutes and continue further mission coordination following SOP 4.11.A.4.
 - 3.3.1 Due to limited TETRA® network coverage, radio transmissions should be attempted at least every 10 (ten) minutes or when possible to indicate "Ops Normal". Alternative communication methods, as guided by management, may be used where TETRA® network coverage is ineffective, to confirm mission and crew status.
 - 3.3.2 The LFC shall monitor the aircraft with SKYTRAC® and confirm arrival and departure times with the Helicopter Landing Officer [HLO] of the platform/vessel.

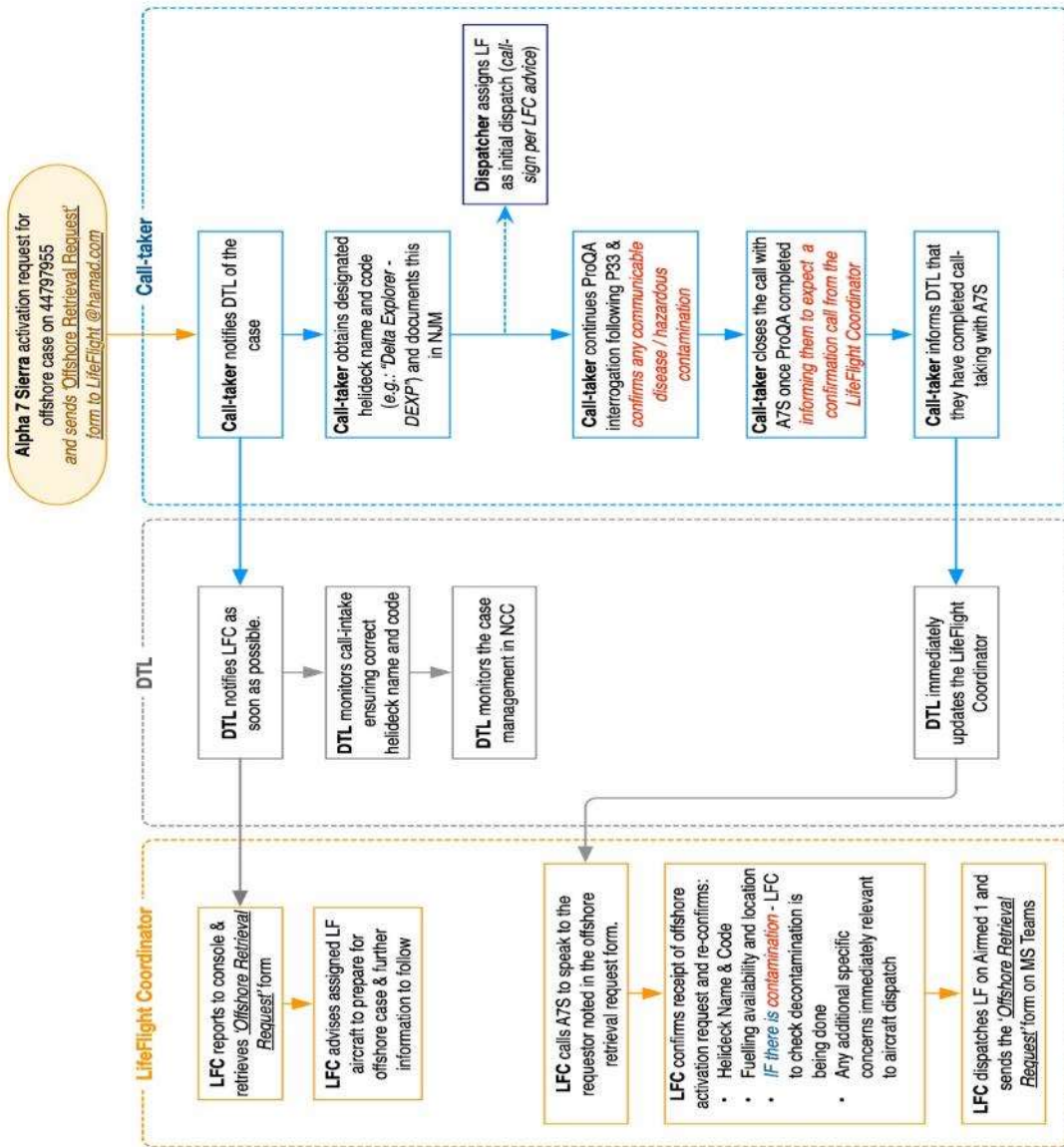
4. Pre-flight Planning and Aircraft Preparation:

- 4.1 Flight planning is the responsibility of the PiC and should include:
 - 4.1.1 Consideration will be given to the weather, visibility and any known hazards that may impose unnecessary risk to the mission or crew.
 - 4.1.2 Estimation of flight time and estimated ground idle time with consideration of refuelling requirements.
 - 4.1.3 Refuelling, if required, should be planned for the outbound flight in which the patient is not onboard the aircraft. If this is not possible, the Medical Crew may disembark at the destination Helideck, allowing refuelling to occur during patient assessment and packaging.
- 4.2 There will be a mission safety briefing given by the PiC, or his delegate, and attended by all flight crew for the mission.
- 4.3 Aircraft preparation should be planned and completed as soon as notified of the offshore mission as follows:
 - 4.3.1 For confirmed 1 (one) patient evacuation:

- 4.3.1.1 Remove the right stretcher to ensure ease of emergency egress during aircraft ditching.
- 4.3.1.2 Secure the primary response bag on the right, aft of the door, with the provided cargo net.
- 4.3.2 For confirmed 2 (two) patient evacuation:
 - 4.3.2.1 Ensure both stretchers are correctly installed, and the right stretcher is free of all equipment to ensure ease of emergency egress during aircraft ditching.
- 4.3.3 Life vests, for the crew, must be donned prior to boarding the aircraft. Patient life vests must be donned prior to patient loading and if possible, deployment and emergency procedures must be explained to the patient prior to boarding the aircraft.

5. Offshore Helideck Operations:

- 5.1 Upon arrival at the designated landing zone, the Medical Crew shall exit the aircraft under instruction of the HLO whilst the pilot maintains power to the rotors.
- 5.2 The Medical Crew shall follow the instructions of the HLO to access the patient(s) and return to the aircraft under his supervision.
- 5.3 At all times, there shall be communication between the Pilots and Medical Crew utilizing the Polycon® or TETRA® radio systems. The HLO may also relay communications between the Medical Crew and Pilots.
- 5.4 The Medical Crew must ensure the patient life vest has been donned prior to entering the aircraft.
- 5.5 The left stretcher will be the default Hot Load stretcher, unless circumstances dictate otherwise or as guided by the PiC.
- 5.6 The Medical Crew shall ensure that all patients, whenever possible, wear hearing protection. (As a general rule, patients will not be on the headset intercom system).
- 5.7 In certain circumstances, it may be necessary to land and deploy the Medical Crew and primary response equipment. The aircraft will then take-off and remain in a safe orbit or reposition to a nearby refuelling station and, when appropriate, return and collect the Medical Crew and patient(s).



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	4.11.A.15 LIFEFLIGHT SELF DISPATCH
Implementation Date	01 September 2022
Purpose	To ensure appropriate activation of emergency resources, when requested directly by Military or external agencies.
Responsibility	ALL LIFEFLIGHT AND NCC PERSONNEL

1. HMCAS may be notified of an incident/patient directly by other agencies including but not limited to the police, civil defense, military, etc.
2. HMCAS and QEAF have agreed to self-dispatch LifeFlight at Military request to ensure rapid deployment of lifesaving assistance to all military personnel within the State of Qatar and its territorial waters.
3. The request is facilitated by the Officer in Command of 23 Squadron of Wing 3 at Doha Air Base. The 23 Squadron OC has the authority to task the aircraft as the point of contact to ensure dispatch without undue delay.
4. This is for various scenarios including parachute accidents, ejection of pilots over water/land, SAR operations for downed aircraft and incidents during live-fire exercises that are otherwise classified to military security clearance only.
5. As with all missions, a pre-flight briefing to determine the safety of all personnel for the mission must be undertaken.
6. The medical crew shall inform the LifeFlight Flight Coordinator during activation to open a CFS with information to follow.
7. **ALWAYS** notify the NCC when LifeFlight is responding to an incident.
8. When the Flight Coordinator receives an emergency call directly from any source other than the NCC, the following procedure applies:
 - 8.1 Immediately notify the dispatcher of the approximate location of the call while proceeding to the incident.
 - 8.2 Notify the SME: LifeFlight of the mission tasking
 - 8.3 Notify the Operations Officer NCC, Oscar Command and the Consultant CCP Effectiveness on duty if there is a need to dispatch other HMCAS or partner agencies to the incident.
 - 8.4 The Flight Coordinator shall undertake routine flight following and the air crew will communicate using the routine communication procedure for the various phases of flight, including: Airborne, POB, endurance and ETA.
 - 8.5 On arrival at the scene, LifeFlight personnel are to follow routine procedures as if activated directly by the NCC.

SOP 4.11.B: LifeFlight Health & Safety



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.B.1 USE OF MEDICAL CREW RESTRAINTS
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding the proper use of medical crew restraints whilst on the LifeFlight program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program.
2. The SME: Lifeflight will be responsible for monitoring staff compliance with this procedure.
3. **Operating as Medical Crew:**
 - 3.1 Medical personnel shall be secured at all times during take-off and landing.
 - 3.2 Medical personnel shall inform the pilots whenever there is a situation that requires temporary removal of their restraints.
 - 3.3 Once the task is completed, the medical personnel shall inform the pilots that they are re-secured in their restraints.
4. **Operating as Crewman:**
 - 4.1 In the event of LifeFlight personnel being required to perform crewman type duties they may be permitted by the pilots to be unrestrained from the seat.
 - 4.2 In this case, wear a safety harness that is attached to the aircraft hard point at all times during these operations.
 - 4.3 Crewman safety harnesses are not a substitute for the approved, installed restraining system on the helicopter seats and may never be used as a substitute restraint system.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.B.2 PHYSICAL FITNESS & ASSESSMENT PROCEDURE
Implementation Date	24 October 2017
Purpose	To provide HMCAS staff members with guidance and relevant information on the physical standards and training to be completed prior to and throughout operational service with LifeFlight.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. Medical Flight Crew members and any HMC staff member seeking to serve on LifeFlight will comply with this procedure.
2. The SME: LifeFlight will be responsible for monitoring staff flight crew status and compliance with this procedure.
3. **Fitness Evaluation:**
 - 3.1 Any HMCAS staff member involved in or wishing to be involved in the LifeFlight Program must pass an initial fitness evaluation and a minimum of six monthly reassessments thereafter.
 - 3.2 Fitness evaluations will be conducted every 3 months with assessment dates to be announced via staff memoranda.
 - 3.3 LifeFlight staff shall not exceed 6 months between LifeFlight fitness currency evaluations.
4. **Fitness Test Components:**
 - 4.1 The LifeFlight Physical Fitness Assessment is a physical assessment used to measure an individual's upper body strength, cardiovascular fitness, and competence in both land and water-based activity.
 - 4.2 The test comprises 3 aquatic events, conducted either in a swimming pool or open water:
 - 4.2.1 100m Freestyle swim
 - 4.2.2 10m Underwater swim
 - 4.2.3 10 Minutes treading water
 - 4.3 And 2 land events:
 - 4.3.1 2.4km Run
 - 4.3.2 100m Stretcher carry with 80kg load
5. **Helicopter Medical Transport Course (HMTTC):**
 - 5.1 Prior to starting operations on the aircraft, all LifeFlight staff shall complete the HMTTC, presented by the HMCAS Education Department.
 - 5.2 Annual refresher courses shall be undertaken to maintain currency.
6. **Crew Resource Management Course (CRMC):**
 - 6.1 Prior to starting operations on the aircraft, all LifeFlight staff shall complete a CRMC at an off-site accredited institution.
 - 6.2 Annual refresher courses shall be attended to maintain currency.
7. **Helicopter Underwater Escape Training (HUET) & Helicopter Emergency Escape Device (HEED) Training:**

- 7.1 Prior to starting operations on the aircraft, all LifeFlight staff shall complete HUET and HEED training at an off-site accredited institution.
- 7.2 HUET and HEED certification is valid for 4 years from completion of the course.
- 7.3 Annual refresher courses shall be attended to maintain currency.

8. Clinical and Technical Skills maintenance:

- 8.1 All LifeFlight staff shall undertake appropriate clinical refresher courses held by the HMCAS as set out in the HMCAS Training Plan, including annual HMCAS infection control training.
- 8.2 All LifeFlight staff shall undertake annual refresher courses in infection control held by HMCAS Infection Control Department.
- 8.3 All LifeFlight staff shall undertake a minimum of two (2) x 12-hour operational shifts on the aircraft per month in order to maintain active Flight Crew status.

9. Pregnancy:

- 9.1 Flight duties while pregnant are allowed unless 20 weeks gestation has been exceeded.

- 10. New pregnancy of female LifeFlight staff shall be reported to the SME: LifeFlight as soon as possible. A letter from her Gynaecologist confirming pregnancy will be required as soon as possible. Pertinent medical history or complications during previous pregnancies which may affect the ability to fly should be contained in the letter.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.B.3 FITNESS TO FLY
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding their medical fitness when participating in the LifeFlight Program.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or aeroplane as part of their official duties within the State of Qatar.
2. The SME: LifeFlight will be responsible for monitoring staff members' fitness levels as well as their general health and compliance with this procedure.
3. **Flight and Duty Hours:**
 - 3.1 LifeFlight shifts may not exceed 14 hours without authorization from the Consultant Effectiveness.
 - 3.2 The rest period prior to commencement of LifeFlight duties shall not be less than 10 hours. No other duties, meetings, training, etc. may be undertaken within the 10 hours preceding the LifeFlight activity (i.e. LF Shifts, LF Training, Events, etc.).
 - 3.3 Medical Flight Crew must have adequate rest and sleep in the preceding 24 hours before a LifeFlight shift.
 - 3.4 Medical Flight Crew may not work more than 6 consecutive shifts without at least one full day off or work more than 60 hours per week.
 - 3.5 Medical Flight Crew members should not donate blood within 72 hours of a flight duty.
 - 3.6 Medical Flight Crew members must not undertake any flying duties within 24 hours after Open Water Scuba Diving.
4. **Unfit To Fly:**
 - 4.1 No individual shall act as a member of the crew of a HMCAS LifeFlight helicopter, if for any reason, their physical or mental condition is such that it could endanger the safety of the aircraft, the crew, or the patients.
 - 4.2 Flight crew members who have concerns about the fitness of any of their fellow crew members on duty are strongly encouraged to contact the Consultant Effectiveness on duty or the SME: LifeFlight and report their concerns and should avoid entering into arguments with fellow crew members about the same.
 - 4.3 Flight crew members are not obliged to fly with any other crew member who is perceptibly unfit to perform flying duties.
 - 4.4 HMCAS staff members who are suffering from any sickness/injury that is contagious or potentially debilitating should not undertake flying duties. Personnel that are sick or injured must report to the Staff Clinic or a suitable alternative Medical Facility. A medical report/sick note shall be submitted to the SME: LifeFlight who will decide whether the staff member is cleared for flying duties or not.
 - 4.5 Individuals prone to motion sickness should not undertake flight duties.

5. Medications:

- 5.1 Medications that impair judgement and/or cause drowsiness should not be taken prior to or during flight duty time. Examples of these types of medication include (but are not limited to) the following:
 - 5.1.1 Antihistamines
 - 5.1.2 Alpha-adrenergic blocking agents
 - 5.1.3 Antidepressants
 - 5.1.4 Anti-emetics
 - 5.1.5 Codeine based analgesia
- 5.2 Enquiries regarding medication and fitness to fly are to be directed to the SME: LifeFlight.

6. Attributes required to function safely and effectively in an aeromedical environment:

- 6.1 Fatigue and flight operations:
 - 6.1.1 The following warning signs should alert flight crew members of a fatigue-related dangerous situation:
 - 6.1.1.1 Eyes going in and out of focus
 - 6.1.1.2 Head bobs involuntarily
 - 6.1.1.3 Persistent yawning
 - 6.1.1.4 Wandering or poorly organized thoughts
 - 6.1.1.5 Spotty near-term memory
 - 6.1.1.6 Missed or erroneous performance of routine procedures
- 6.2 Situational awareness:
 - 6.2.1 For a HEMS pilot or flight crew member, situational awareness involves having a mental picture of the existing inter-relationship of location, flight conditions, configuration, and energy state of the aircraft as well as any other factors that could potentially affect its safety such as:
 - 6.2.1.1 proximate terrain;
 - 6.2.1.2 obstructions;
 - 6.2.1.3 airspace reservations;
 - 6.2.1.4 landing zone infringements; and
 - 6.2.1.5 weather systems
 - 6.2.2 The flight crew shall have an understanding of the current state and dynamics of the aeromedical system and shall be able to anticipate future change and developments.
 - 6.2.3 In terms of situational awareness, medical flight crew shall:
 - 6.2.3.1 Extract information from the environment;
 - 6.2.3.2 Integrate this information with relevant internal knowledge to create a mental picture of the current situation;
 - 6.2.3.3 Use this picture to direct further perceptual exploration in a continual perceptual cycle; and
 - 6.2.3.4 Anticipate future events

- 6.3 Flight crew members may not fly if their actual body size prevents the helicopter's seat belts & restraining system from securing them to the seat.
- 6.4 Flight crew members must demonstrate their ability to lift and carry a stretcher with a minimum load of 80kg.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.B.4 CLEANING OF LIFEFLIGHT (LF) HELICOPTERS
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding the cleaning of all operational LF helicopters.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. SCHEDULED CLEANING & DECONTAMINATION:**1.1 Daily cleaning:**

- 1.1.1 At the start of each shift the Medical Flight Crew on duty will inspect the interior of the helicopter for cleanliness.
- 1.1.2 Basic cleaning and dusting shall be performed by the Medical Flight Crew on duty (e.g. removal of sand from the floor).
- 1.1.3 The Medical Flight Crew on duty shall perform cleaning and disinfection of both the equipment and other frequently-touched surfaces after each patient transportation, using HMC-approved disinfectant wipes.
- 1.1.4 If any bio-hazardous contamination of the aircraft is found (typically after the transportation of patients), the aircraft should be cleaned on site, if at Hamad Emergency Helipad ("Hotel Echo") or contact the designated Ambulance Attendants to facilitate deep cleaning and simple decontamination at the LifeFlight Base.
- 1.1.5 Depending on the extent of contamination, the helicopter may be booked Out of Service for the duration of the cleaning task.

1.2 Weekly cleaning:

- 1.2.1 All operational LF helicopters will undergo social cleaning and simple decontamination twice a week on Sundays and Wednesdays at 0700H.
- 1.2.2 The designated ambulance attendants will perform these cleaning tasks at the LifeFlight Hangar at Doha Air Base.
- 1.2.3 The Flight Coordinator in the NCC will coordinate with all the role players to facilitate the cleaning task.

1.3 Monthly deep cleaning:

- 1.3.1 All operational LF helicopters will have their Medical Interiors completely removed for cleaning and disinfecting once per month.
- 1.3.2 The AgustaWestland engineers at Doha Air Base will perform the removal and re-fitment of the Medical Interiors.
- 1.3.3 The removed Interiors will be cleaned and disinfected by the Ambulance Attendants.

2. CLEANING RESTRICTIONS:**2.1 Detergents and cleaning solutions:**

- 2.1.1 Only HMC approved detergents and cleaning solutions may be utilized for cleaning of LF helicopters and equipment.

2.2 Equipment removal:

- 2.2.1 No equipment may be removed from operational LifeFlight helicopters for cleaning purposes by anyone other than LifeFlight Medical Crew Members.
- 2.2.2 All equipment handed in to the HMCAS Production Department for cleaning has to be recorded in the equipment book and signed for by a representative from the HMCAS Production Department.
- 2.2.3 If equipment must be removed for a prolonged period, loan equipment should be obtained from HMCAS Production Department to ensure continuity of the LifeFlight service.

SOP 4.11.C: LifeFlight Equipment



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.C.1 LIFEFLIGHT PERSONAL PROTECTIVE EQUIPMENT
Implementation Date	24 October 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding the proper use, storage, and care of Personal Protective Equipment (PPE) and related items within the HMCAS.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program.
2. The SME: Lifeflight will be responsible for monitoring staff members' compliance with this procedure.
3. Aeromedical crew members shall be issued with the following PPE:
 - 3.1 NOMEX® Flight Suit and NOMEX® Flying Gloves:
 - 3.1.1 Do not make any alterations to the issued flight suits without approval from the OM.
 - 3.1.2 Wear the Nomex® suit on all helicopter flights.
 - 3.1.3 Always wear the standard shoulder rank insignia on the flight suit at all times.
 - 3.1.4 Fasten all suit closures during use
 - 3.1.5 Clean Nomex® garments either at home, by industrial laundry or dry cleaning.
 - 3.1.6 Use NOMEX® Flying Gloves during helicopter operations only.
 - 3.2 HAIX AIRPOWER® Boots:
 - 3.2.1 Always wear issued boots when performing helicopter duties to ensure adequate ankle support and toe protection.
 - 3.2.2 Staff who have not been issued with HAIX boots may wear other boots if approved by the SME: Lifeflight. Only black boots may be worn with flight suits
 - 3.2.3 Keep boots should clean at all times.
 - 3.2.4 Avoid using flammable cleaning/polishing products on boots.
 - 3.3 MSA® GALLEY LH250 Helicopter Helmet:
 - 3.3.1 Ensure that the helmet fits comfortably & snugly.
 - 3.3.2 Attach only supplied, approved headlamps/accessories to the helmet.
 - 3.3.3 Do not attach/tie/paste any other item in any way to the helmet
 - 3.3.4 Graphics, paint, artwork, stickers, writing or any markings of any kind shall not be allowed on helmets except the official, supplied HMCAS markings.
 - 3.3.5 Do not drill/make any holes into any part of the helmet.
 - 3.3.6 Clean and preserve helmets in strict adherence to the manufacturer's instructions.
 - 3.3.7 Helmets will be inspected every three months by the SME: Lifeflight in strict adherence to the manufacturer's warranty recommendations. Report any damage or defects to any helmet immediately.
 - 3.3.8 If a helmet is dropped from a height greater than one (1) meter, report it immediately, even if no damage is apparent as the integrity of the helmet's shell may be compromised.

- 3.3.9 Keep the helmet visor (smoked or clear depending on time of day) in the lowered position during all helicopter operations especially when helicopter doors are open while the engines are running or during flight.
- 3.3.10 Tighten the chin strap and fasten securely for the duration of wear.
- 3.3.11 Store helmets in the provided helmet bag (to protect it from dust), inside the provided carry bag to protect from damage.
- 3.3.12 Store helmet away from direct sunlight, preferably in an environment with an ambient temperature less than 30° C and relative humidity < 65%.
- 3.4 Custom-made/Disposable Hearing Protection:
 - 3.4.1 ALWAYS wear hearing protection when working on the Apron at Doha Air Base.
 - 3.4.2 ALWAYS wear hearing protection (helmet or plugs) when working around any running helicopter, primarily when operating in a ground-supporting role.
- 3.5 Rescue Knives and Tools:
 - 3.5.1 The supplied personal rescue tools are to be carried at all times during flying operations.
- 3.6 BLACK 5.11® Mission Ready 2.0 Bag:
 - 3.6.1 Store all supplied PPE in this bag during transportation and when not in use.
 - 3.6.2 PPE equipment may not be stored or transported in any other bag or case.
 - 3.6.3 Do not use this bag for any other purpose.
- 3.7 LSC® QUAD LOCK Aviation Safety Belt:
 - 3.7.1 Wear the safety belt in all circumstances where one or both crew members are required to operate on-board an airborne helicopter with open doors and crew members not strapped into seats (e.g. during pattering or HSAR operations).
 - 3.7.2 Wear the safety belt above the hips and adjust for a tight fit, ensuring that both buckles are latched.
 - 3.7.3 Attach the tether to a secure approved load-rated aircraft fitting and verify the safety hook is secured and locked.
 - 3.7.4 Adjust the tether length to remove excess slack to prevent inadvertently exiting out aircraft.
 - 3.7.5 Maintain and inspect safety belts in strict adherence to the manufacturer's instructions.
- 3.8 SWITLIK X-BACK MOLLE® Air Crew Vest:
 - 3.8.1 DONNING INSTRUCTIONS:
 - 3.8.1.1 Put vest on with zipper and buckle in front (Do not wear life vest under clothing).
 - 3.8.1.2 Zip vest up
 - 3.8.1.3 Fasten snap buckle
 - 3.8.1.4 Adjust "X" straps for proper fit
 - 3.8.1.5 Fold excess waist strap, as necessary, and secure in strap stowage pockets.
 - 3.8.2 INFLATION & USE:
 - 3.8.2.1 Do not inflate life vest while inside aircraft.

- 3.8.2.2 After exiting the aircraft, pull sharply down on the inflation beaded handles.
- 3.8.2.3 To access oral inflation tube, lift the lower cover flap on the front cover.
- 3.8.2.4 Emergency strobe will illuminate on contact with water.
- 3.8.3 MAINTENANCE & CARE – Always inspect life vest prior to donning:
 - 3.8.3.1 Check for damage to the outer covering, supporting vest material and straps. If any damage is evident, the vest should be inspected by authorised service centre.
 - 3.8.3.2 Check to ensure that the zip on the outer covering is secure all along the side and that the ends are secure with Velcro fastener.
 - 3.8.3.3 Check to ensure that the inflation lanyard is in place at the bottom of the vest, secured with Velcro fastener in its proper location.
- 3.9 HEED 3® Helicopter Emergency Egress Device:
 - 3.9.1 The device will be attached to the Switlik X-Back Air Crew Vest using the mole attachment.
 - 3.9.2 PRE-USE CHECK:
 - 3.9.2.1 Check Pressure Indicator - Dial Gauge Indicator needle is within green zone (range from 2600 to 3000 psi).
 - 3.9.2.2 Check for obvious physical damage, broken or loose parts.
 - 3.9.3 TO USE THE HEED:
 - 3.9.3.1 Pull D-ring on flap of Holster to open.
 - 3.9.3.2 Grab HEED regulator head and pull out of the Holster. This action will release the Mouthpiece Cover.
 - 3.9.3.3 Place the HEED regulator into user's mouth.
 - 3.9.3.4 If the system is used underwater press purge button or exhale sharply to expel water from the regulator prior to inhalation.
 - 3.9.3.5 Continue to inhale and exhale to a safe and normal rate of ascent.
 - 3.9.4 NOTE:
 - 3.9.4.1 The HEED utilizes a balanced regulator which means it will provide air in any orientation including the regulator being upside down or sideways.
- 4. Staff members shall individually be responsible for the safekeeping and basic care of their own, issued Personal Protective Equipment.
- 5. All issued Personal Protective Equipment MUST be present prior to commencing LifeFlight duties.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.C.2 CARE OF PERSONAL PROTECTIVE EQUIPMENT
Implementation Date	01 April 2017
Purpose	To provide HMCAS staff members with guidance and relevant information regarding the proper care and cleaning of Personal Protective Equipment (PPE) and related items within the HMCAS.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member undertaking duties within the LifeFlight program.
2. The SME: LifeFlight will be responsible for monitoring staff compliance with this procedure.
3. **Care of NOMEX® Flight Suits:**
 - 3.1 Read and carefully follow manufacturer's laundering instructions.
 - 3.2 Launder Nomex® garments separately from other garments to avoid the transfer of flammable fibres or other materials.
 - 3.3 Hand or machine wash in water not exceeding 60°C using a good proprietary washing powder or detergent that does not contain optical whiteners or high levels of oxidizing agents. The best results are given by using a liquid detergent with near-neutral pH and water with minimum alkaline content.
 - 3.4 Do not use pure soap powders as these may affect the flame retardant properties of the garment.
 - 3.5 The machine cycle should not exceed 1 hour and do not exceed 2/3 of the machine capacity.
 - 3.6 Ensure that the rinse cycle is sufficient to remove all residual soap/detergents.
 - 3.7 **DO NOT USE BLEACH, PEROXIDE, OR CAUSTIC WASHING AGENTS.**
 - 3.8 Preferably line dry, not in direct sunlight and hang inside out to avoid fading of the outer surface.
 - 3.9 The garment may be tumble-dried at low heat
 - 3.10 If required, iron with a medium/hot iron
 - 3.11 The garment can be dry-cleaned
 - 3.12 Store Nomex® garments in a dry place away from heat and direct sunlight.
4. **Care of NOMEX® Flying Gloves:**
 - 4.1 Wear Nomex® gloves at all times during missions unless patient examination gloves are indicated for patient treatment.
 - 4.2 Do not store bundled/ folded tightly into pockets but rather stored flat (e.g. inside helmet) to avoid creation of permanent creases in the material.
 - 4.3 Avoid contact with oils and fuel as this will degrade the leather and Nomex® material.
 - 4.4 Wash only as per Nomex® manufacturer's recommendations as described for Nomex® flight suits in 3.1 to 3.12.
5. **Care of Flying Boots:**
 - 5.1 Read and carefully follow the manufacturer's usage, care, and cleaning instructions.

- 5.2 Hand wash with a shoe brush or other soft brush and warm water (at a temperature no higher than 40°C). Scrub lightly with the wet brush to remove dirt and other soils.
- 5.3 Air dry protective footwear slowly, never with forced hot air or on a heat source. A boot dryer is acceptable.
- 5.4 Remove the insole and stand it up inside the boot to allow the absorbent insole to dry without being misplaced.
- 5.5 Keep out of direct sunlight and away from open windows, hot ovens, or radiators.
- 5.6 Apply a professional grade silicone-based shoe polish. Those that contain petroleum-based products are flammable and therefore unsuitable.
- 6. Care of MSA® GALLET LH250 Helicopter Helmet:**
 - 6.1 Wear personal issue perspiration absorbing Nomex® skull cap under the helmet to improve the hygiene inside the helmet and prolong the inner lining's life.
- 7. Care of Custom-made Hearing Protection:**
 - 7.1 Read and carefully follow the manufacturer's usage, care, and cleaning instructions.
- 8. Underwear:**
 - 8.1 All clothing (T-shirts & underwear) worn underneath the Flight Suit shall be wool, Nomex® or 100% cotton (i.e. no synthetics materials e.g. Lycra, spandex, polyester, etc.).
 - 8.2 Wear only plain white and navy blue T-shirts underneath the flight suit with no visible brands or logos.
 - 8.3 Do not wear any metal-containing underwear (e.g. underwire bras, etc.).
 - 8.4 Wear only wool or Nomex® socks.
- 9. Jewellery:**
 - 9.1 Do not wear any loose or exposed jewellery during helicopter operations (hoop earring, bangles, bracelets etc).
- 10. Care of CAMELBAK® Rehydration Pack:**
 - 10.1 The issued CAMELBAK® is designed for use during missions for rehydration purposes of individual crew members.
 - 10.2 Take care to avoid cross contamination.
 - 10.3 Crew members may fill the hydration bladder with any fluids they choose, although HMCAS will not replace hydration bladders or piping damaged by fluids other than water.
- 11. Care of Rescue Knives and Tools:**
 - 11.1 The Leatherman® Wave and Gerber® Folding Tool are provided as utility equipment during helicopter operations and shall be carried at all times during flying operations.
 - 11.2 Store the tools in secure pockets which are available on the flight suit or the Camel Back®.
- 12. Repair and Replacement of Damaged or Lost PPE:**
 - 12.1 Repair/replacement of damaged equipment may take extended periods of time (especially custom-made flight suits) since most of the PPE is imported.
 - 12.2 Any PPE damaged/destroyed/lost by user negligence, misuse or abuse shall be repaired/replaced at the user's own expense.
 - 12.3 Any PPE found abandoned, abused, or stored incorrectly will be confiscated by management.

- 12.4 Report lost or stolen PPE immediately to the SME: LifeFlight and register a case with the Police.
- 12.5 Complete an Occurrence/Variance/Accident report for any component of PPE that is damaged or lost. Each item must be reported in separate OVA's.
- 12.6 All issued PPE must be returned to the Senior Production Supervisor at the end of employment as part of the Termination and Separation process, the cost of any outstanding PPE will be recovered from the employees end of service settlement.

13. Standard Issued PPE to be Returned at the end of Service:

- 13.1 MSA Gallet LH250 Helmet
- 13.2 Remix Pro Headlamp
- 13.3 Nomex
- 13.4 Flight suit
- 13.5 LF Jacket
- 13.6 Nomex Flight Gloves
- 13.7 Rescue Gloves
- 13.8 Leatherman Folding Tool
- 13.9 Gerber Knife
- 13.10 Leatherman Raptor Rescue Shears
- 13.11 Camelbak
- 13.12 HAIX Boots
- 13.13 HDS2 Tetra Earpiece

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations (LifeFlight)
Procedure	4.11.C.3 MEDICAL EQUIPMENT, CONSUMABLES AND ASSET MANAGEMENT
Implementation Date	01 April 2017
Purpose	To provide HMC AS staff members with guidance and relevant information regarding the management of medical equipment to ensure the safe and effective delivery of the LifeFlight service. Policy Ref.: SA 1056 AS Medical Equipment Management Plan.
Responsibility	ALL OPERATIONAL LIFEFLIGHT STAFF

1. This procedure will apply to any HMCAS staff member or Medical Flight Crew member actively involved in the LifeFlight program or seconded to fly on any other helicopter or airplane as part of their official duties within the State of Qatar.
2. The SME: LifeFlight shall delegate to the Production Supervisor the task of tracking equipment's location and maintaining a database of equipment availability and due dates for service and repair.
3. The Production Supervisor shall be responsible for ensuring that sufficient items of medical equipment are serviceable and available for use in order to support the operational effectiveness of the LifeFlight Service.
4. **Definitions:**
 - 4.1 **Medical Equipment:** Non-consumable items of equipment used for patient care, including, but not limited to: electrical medical equipment (electro-med) and equipment used for immobilization, transport, and extrication of patients.
 - 4.2 **User group:** Personnel employed within the Emergency Aeromedical Service who operate point-of-care medical equipment items.
 - 4.3 **Equipment Support:** The process of servicing, maintenance, and repair of front-line medical equipment.
 - 4.4 **Bio Medical Engineering (BioMed):** The internal provider of Equipment Support services within Hamad Medical Corporation.
 - 4.5 **Decontamination:** The process of cleaning an item of equipment to ensure that it is free of blood, body fluids and other bio-contaminants.
 - 4.6 **Asset management** is the process by which the SME: LifeFlight and other members of the User group are aware of the location and status of all medical equipment.
5. **Ground Transportation Platforms:**
 - 5.1 The Production Supervisor shall be responsible for ensuring that vehicles are presented for servicing and repair when required, in line with the existing HMCAS fleet management system. This responsibility may be delegated to a capable member of staff.
 - 5.2 Users of these vehicles shall thoroughly inspect these vehicles using the prescribed vehicle checklists in line with [SOP 4.2.A](#).
6. **Air Ambulances:**
 - 6.1 The Qatar Emiri Air Force (QEAF) shall manage the servicing and repair of air ambulances through AgustaWestland.
 - 6.2 Flight Medical Crew shall be responsible for conducting daily checks of medical equipment and installations and a general check of the serviceability of the side doors, seats, and crew safety harnesses.

- 6.3 Report faults and problems immediately to the Pilot in Command or the Operations Officer.
- 7. Extrication and immobilization equipment:**
- 7.1 Check all extrication and immobilization equipment daily for serviceability and completeness.
- 7.2 Immediately report any damaged, faulty, lost or worn items to the Production Supervisor.
- 7.3 The Production Supervisor shall arrange for the item to be replaced as soon as possible through HMCAS Production Department.
- 7.4 NEVER use any faulty items of equipment on a patient.
- 8. Electric Medical Equipment (ELECTRO-MED):**
- 8.1 All electro-med equipment items shall be approved for use by BioMed and formally brought into service.
- 8.2 All BioMed approved equipment shall display a yellow barcoded label with a unique identifying HBE number. Other labels that may be affixed include warranty labels and 'next test due' labels.
- 8.3 **NEVER use any item of electro-med equipment if it does not display BioMed approval.**
- 9. Trials and Demonstration Equipment:**
- 9.1 Equipment being trialed/tested for fitness purposes shall also be subject to BioMed approval.
- 9.2 A blue 'Demo Unit' label shall be affixed to all trial equipment.
- 10. User Training and Competence:**
- 10.1 The SME: LifeFlight shall ensure that all personnel are trained in the use of all serviceable items of equipment.
- 10.2 Individual personnel shall demonstrate competence in the use of serviceable equipment.
- 10.3 A copy of competency certificates shall be retained in the Users Personnel File.
- 10.4 Training on newly acquired equipment will be facilitated through the HMCAS Training Department.
- 10.5 The SME: LifeFlight shall ensure that any internal competence training provided on in-service equipment complies with the equipment manufacturers' recommendations.
- 11. User Checks and Maintenance:**
- 11.1 The User Group shall ensure that electro-med equipment is maintained in a serviceable condition and that daily pre and post use checks are carried out in accordance with the manufacturers' recommendations.
- 11.2 User check sheets shall be retained on file for a period of 5 years.
- 11.3 Maintain batteries in a charged state and account for ancillary items of equipment (charging units, power cables, ECG electrodes, BP Cuffs etc.) at the beginning and end of each shift. Ensure that such items are recovered when handing over a patient to a receiving facility.
- 11.4 Complete the daily check sheet for each item of electro-med equipment.
- 11.5 Conduct a rapid trouble-shooting assessment if an item of equipment is found to be non-functional (e.g. check power source and battery status, ensure leads and cables fully plugged in). If the fault cannot be rapidly rectified, source a replacement item of equipment from the LifeFlight Store.
- 11.6 Decontaminate faulty electro-med equipment using alcohol or non-caustic detergent before being passed to the HMCAS Production Department for repair arrangements with BioMed.

- 11.7 **NEVER place a faulty item of electro-med equipment on a shelf where it may be inadvertently re-used.**
- 11.8 **NEVER** cannibalize any item of electromedical equipment (ancillaries taken for use on another item of equipment). Actions such as this have the potential to cause Serious Untoward Incidents resulting in death or injury to a patient. DISCIPLINARY ACTION MAY BE TAKEN AGAINST PERSONNEL FOUND TO BE IN VIOLATION OF THIS INSTRUCTION.

12. Pre-packed medical/surgical kits:

- 12.1 Place opened pre-packed kits in the allocated box inside Doha Air Base Medical Store Room or at the relevant areas at HGH or HMCAS main stores for re-packing.
- 12.2 Partially used kits are not to be discarded unless soiled.

13. Medical Stock Maintenance:

- 13.1 Maintain stock levels in accordance with the maximum and minimum levels designated by the SME: LifeFlight.
- 13.2 Communicate replenishment requests to the Flight Coordinator who shall ensure that the stock (including non-medical items e.g. water) is ordered and ready to be transported to Doha Air Base with the oncoming duty personnel.
- 13.3 Transport partially used prepacked kits and other non-medical items (e.g. empty water bottles) to the main HMCAS stores at the end of the shift.

14. Re-stocking of Equipment and Sundries (*excluding controlled medications*):

- 14.1 Replace portable oxygen cylinders from the HGH A&E Ambulance Attendant Store or at the HMCAS main stores.
- 14.2 If a patient is transported to HGH, replenish all prepacks and oxygen cylinders from the stores at the A&E Ambulance Attendant Store.
- 14.3 If a patient is transported to Heart Hospital or PEC Al Sadd, replenish all prepacks from the HMCAS main stores.
- 14.4 If a patient is transported to any other facility, replenish all prepacks from the Doha Air Base LifeFlight store room.
- 14.5 If a patient is treated on scene and not transported, replenish all stock used from the Medical store-room at the Doha Air Base LifeFlight Hangar upon return.

15. Replenishment of Controlled Medications:

- 15.1 All controlled medications are only available from the HMCAS main stores. Please see [SOP 4.3.C](#) for specific controlled medications procedures.
- 15.2 If a patient has been treated and transported to hospital, complete all necessary paperwork, and restock any disposables at the relevant area as detailed above.
- 15.3 If at HGH, use the Ambulance Attendant vehicle to drive to the HMCAS main stores to restock controlled medications.
- 15.4 If the patient is not transported to HGH and restocking is to be conducted at the HMCAS main stores, then restock all equipment and sundries together with controlled medications.

SOP 5.1: Retrieval Services (RS): General



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMCAS Retrieval Services (RS)
Procedure	5.1.A RETRIEVAL SERVICES DRIVING
Implementation Date	01 October 2016
Purpose	To provide guidance on driving for Retrieval Services operations.
Responsibility	ALL RETRIEVAL SERVICES OPERATIONAL STAFF

1. Background:

- 1.1 This SOP complements HMCAS [SOP 4.2.A](#) and [SOP 4.6.A](#). Instead of replacing the provisions made within these publications, it expands upon certain areas with reference to the unique mission sets that are conducted by Retrieval Services.

2. Implications for RS Drivers:

- 2.1 RS drivers must comply with the following instructions:
- 2.1.1 'Priority 1' driving techniques must only be used under extreme circumstances (such as when a patient suddenly and unexpectedly deteriorates in-transit).
 - 2.1.2 Careful driving at moderate speeds will benefit the patient most and must be the normal driving technique applied even for the most high acuity RS patients.
 - 2.1.3 The decision for the driver to drive under Priority 1 conditions must be informed by the patient's condition and rests with the senior clinician present. HMCAS [SOP 4.6.A](#) gives specific instructions for 'Priority 1' driving including the direction that no HMCAS ambulance may exceed the stated speed limit by more than 20 km/h. These instructions **must** be adhered to.
 - 2.1.4 The 'box body' shape of the ICU Ambulances hinders the situational awareness of the driver. The co-driver must act at all times to aid the driver's situational awareness (pointing out hazards etc.) and must always dismount to guide the driver in reversing.

3. Vehicle Familiarization and Licensing:

- 3.1 Line Managers are responsible for ensuring that all driving personnel have a current drivers' license that is appropriate to the vehicle being driven.
- 3.2 Individuals are responsible for reporting any health changes or personal circumstances that may affect their ability to or competence in driving HMCAS RS vehicles.
- 3.3 All drivers must undergo familiarization training and be assessed as competent in the operation of the vehicle being driven.
- 3.4 Personnel employed on driving duties in units that do not deploy daily must conduct two hours of practical driving per shift to maintain currency and familiarity with the vehicle.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.1.B MANAGEMENT OF EMERGENCY PATIENTS BY RETRIEVAL UNITS
Implementation Date	01 October 2016
Purpose	To provide guidance on management of Emergency Patients encountered by Retrieval Services personnel.
Responsibility	ALL RETRIEVAL SERVICES OPERATIONAL STAFF

1. Situation:

- 1.1 Retrieval Services crews may be faced with Emergency Patients. It is anticipated that this will be as a result of:
 - 1.1.1 Unexpected and sudden deterioration in the condition of non-emergency patients being transported.
 - 1.1.2 'Flagging down' of Retrieval Services ambulances by members of the public at the scene of an RTA or medical emergency.
 - 1.1.3 RTA involving (or in close proximity to) a Retrieval Services vehicle.

2. Priority:

- 2.1 When faced with Emergency Patients, Retrieval Services crews must take immediate action and administer treatment as per Ambulance Service CPG's.

3. Communication:

- 3.1 Following/during immediate action, the Retrieval Services crew must notify the HCC that they have an emergency patient. The HCC will notify the crew to switch to a suitable NCC channel.
- 3.2 The Retrieval Services crew must communicate a situation report to NCC on switching channels, this must include:
 - 3.2.1 Unit call sign
 - 3.2.2 Location
 - 3.2.3 Nature of the emergency
 - 3.2.4 Number of patients and priority (including QEWS score)
 - 3.2.5 Treatment given/in progress
 - 3.2.6 Back up requested
- 3.3 NCC, in conjunction with the Clinical Desk, will make a decision on whether the crew should start or continue transport to the appropriate facility. Additional resources will be dispatched as required taking level of care, distance to receiving facility, ability to drive Priority 1 into account.

4. Handover and Responsibility:

- 4.1 If Emergency Ambulance Services assets are not available or not required, Emergency Patients will be transported to an appropriate facility by a Retrieval Services crew with NCC approval. If required, a CCP will accompany the patient.
- 4.2 Emergency Patients may be transported by a Retrieval Services crew, provided the unit can provide a level of care commensurate with that which can be provided by the appropriate Emergency Ambulance Service crew.

5. Commensurate level of care:

- 5.1 Emergency Patients may only be transported to hospital by Retrieval Services if it meets the following criteria:
 - 5.1.1 The unit has Ambulance Paramedic capability.
 - 5.1.2 Capable of providing full spinal immobilization if clinically indicated.
 - 5.1.3 Has a fitted stretcher.
- 5.2 No patient shall experience a lesser standard of care as a result of them being transported by a Retrieval Services Unit in lieu of an Emergency Ambulance Service Unit.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.1.C AIRPORT OPERATIONS
Implementation Date	01 October 2016
Purpose	To provide guidance on the general principals to be followed when operating at airports.
Responsibility	ALL RETRIEVAL SERVICES OPERATIONAL STAFF

1. Background

1.1 Retrieval Services personnel from all service offerings are involved in the transfer of patients into and out of Qatar via Commercial, Military and Official aircraft at the following strategic air transport hubs:

- 1.1.1 Hamad International Airport
- 1.1.2 Doha International Airport
- 1.1.3 Al Udeid Air Base

2. Access:

2.1 Retrieval Services personnel entering strategic air transport hubs in the course of their duties do so entirely at the discretion of the operating authority (Hamad International Airport and Qatar Emiri Air Force).

2.2 Personnel not in possession of access passes to these establishments may be granted temporary access via the following personnel (accessed through the management chain):

- 2.2.1 Al Udeid Air Base/Doha International Airport: Senior Supervisor for Public Relations.
- 2.2.2 Hamad International Airport: Senior Operations Manager for Emergency Service.

2.3 No RS staff member is to use their access cards for any personal gain, e.g purchase discounts, access to passenger terminals without official business, etc.

3. Security Considerations:

3.1 Hamad International Airport and Al Udeid Air Base are key installations within the State of Qatar. Access by Retrieval Services personnel is limited to those areas that are essential to the execution of their duties.

3.2 Photography in all forms, including the capture of still and video footage by smart phones is strictly prohibited in these locations without authority.

3.3 Retrieval Services personnel, vehicles, patients, and relatives may be subjected to search by security personnel, including metal detection and X-Ray. All personnel are expected to comply.

3.4 Each location will be entered via a pre-arranged access point.

4. Immigration:

4.1 Personnel leaving national territory of the State of Qatar on duty must be in possession of the following documentation:

- 4.1.1 Passport
- 4.1.2 Qatar ID
- 4.1.3 Valid exit permit from Ministry of the Interior of the State of Qatar if applicable.
- 4.1.4 Valid entry visa for the flight destination

- 4.2 When required, exit permits will usually be arranged through the Senior Operations Manager for Emergency Service. If not available, the request will be escalated through RS management.
- 4.3 All RS staff traveling abroad, officially or unofficially, should be doing so without their HIA, DIA, Al Udeid Air Base. All access cards should be handed to Sierra Retrieval prior to departure.

5. Airside Operations:

- 5.1 When HMCAS personnel and vehicles are operating on 'airside', the following precautions must be taken without fail:
 - 5.1.1 Do not proceed onto the apron without permission from Airport/Airbase staff.
 - 5.1.2 Vehicles are not to exceed 25 km/h and must be driven on the margins of the apron.
 - 5.1.3 For purposes of loading and unloading patients, vehicles must be parked a minimum of 25 meters from the aircraft unless using a spotter.
 - 5.1.4 When driven 'airside', HMCAS vehicles must display dipped headlights, four-way hazard warning lights.
 - 5.1.5 Personnel are to wear PPE including high visibility/reflective vests. Hearing protection is to be worn in the presence of running aircraft engines. Hats are not to be worn.
 - 5.1.6 Personnel must not walk under aircraft wings, approaching aircraft as directed by the Air Load Master and/or ground staff.
 - 5.1.7 Rotary wing aircraft should only be approached once the aircraft engine has shut down unless briefed otherwise. In such case safety precautions should be used as briefed.
 - 5.1.8 Loose articles must be secured and not permitted to present a foreign object damage (FOD) threat to the aircraft.
 - 5.1.9 Smoking of any description is strictly prohibited.

6. Documentation:

- 6.1 The following documents must be presented to Ground Staff/Air Movements staff:
 - 6.1.1 Passenger manifest
 - 6.1.2 Medical equipment manifest including details of Dangerous Air Cargo.
 - 6.1.3 Baggage manifest
 - 6.1.4 Passports of patient and accompanying medical personnel.
 - 6.1.5 Pharmaceuticals list
 - 6.1.6 Letters of authorization for carriage of controlled medications
- 6.2 A copy of the passenger list (staff and patients) must be lodged with the Clinical Coordinator in either paper or electronic format prior to departure.
- 6.3 Authority to emplane and/or patient medical report must be obtained from the International Medical Affairs Office of the Ministry of Public Health prior to departure.

SOP 5.2: Scheduled Ambulance Service (SAS)



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Scheduled Ambulance Service (SAS)
Procedure	5.2.A SCHEDULED AMBULANCE SERVICE (SAS) SUPERVISOR DUTIES
Implementation Date	01 October 2016
Purpose	To ensure Supervisor Duties are adhered to.
Responsibility	SAS SUPERVISORS

1. Day Shift Duties:

- 1.1 The SAS Supervisor will start duty at 0500hrs daily at which point a handover will be conducted between the night and day shift personnel to confirm the following:
 - 1.1.1 Vehicles available for day shift
 - 1.1.2 Staff available for day shift
 - 1.1.3 Staff sickness
 - 1.1.4 Operational matters that have arisen during the night that warrant escalation (complaints, incidents, accidents).
 - 1.1.5 Any ongoing incidents
- 1.2 Following takeover of duty, the Day Shift Supervisor will move to the Fahad Bin Jassim Renal Unit (FBJ) and conduct the following activities:
 - 1.2.1 Troubleshooting of problems, issues, or complaints
 - 1.2.2 Assisting with heavy patients
 - 1.2.3 Random inspection of crews and ambulances for the following:
 - 1.2.3.1 Cleanliness (interior/exterior)
 - 1.2.3.2 Completion of vehicle and equipment checks.
 - 1.2.3.3 Staff turnout and appearance (including carriage of Corporate ID).
 - 1.2.4 Unless summoned to assist elsewhere, the SAS Supervisor will remain at FBJ until next Dialysis change over.
- 1.3 The SAS Supervisor will relocate to Hamad General Hospital Outpatient Department (OPD), where he will continue to undertake the troubleshooting, assistance and inspection activities highlighted above, returning to FBJ at 1000hrs to continue the same until 1200hrs.
- 1.4 The SAS Supervisor may be called to assist with problems encountered by any SAS crew within Doha city limits.
- 1.5 Incidents occurring outside of Doha (North of Umm Slal Mohammed intersection, Duhail and Qatar University, West of Al Shamal Road/Doha Expressway and South/South-West of F Ring Road) will be referred to the 999 Delta from the nearest hub.
- 1.6 The SAS Supervisor will engage in routine administration tasks, including completion of the daily schedule and responding to emails, while continuing to act as the focal point for all issues involving front-line units and working in collaboration with Operations Production to resolve problems.
- 1.7 The SAS Supervisor will work to activate the units for the night shift and hand over any live issues to the oncoming Night Shift SAS Supervisor.
 - 1.7.1 The day-night handover takes the same form as the handover received at the start of the day shift.

2. Night Shift Duties:

- 2.1 The SAS Supervisor for the night shift will start duty at 1700hrs at which point the handover will be conducted.
- 2.2 Following takeover of duty, the SAS Supervisor will complete the process of activating units for the night shift.
- 2.3 The SAS Supervisor will act as the focal point for all SAS matters throughout the night, monitoring the activities of SAS Units on shift.
- 2.4 The SAS Supervisor will finalize the daily roster and forward it via email to the following personnel:
 - 2.4.1.1 Operations Officer: Scheduled Ambulance Services.
 - 2.4.1.2 Senior Operations Manager: Scheduled Ambulance Services.
- 2.5 The SAS Supervisor will conduct random visits to crews on shift, checking the following:
 - 2.5.1.1 Cleanliness of ambulances (interior and exterior).
 - 2.5.1.2 Completion of vehicle and equipment checks.
 - 2.5.1.3 Staff turnout and appearance (including carriage of Corporate ID and Personal Protective Equipment).
- 3. The SAS Supervisor will complete the daily report and conduct a review of the day's ePCR's, checking each for legibility and fullness of completion.
- 4. The SAS Supervisor will work in collaboration with Operations Production to resolve any issues ahead of the day ahead.
- 5. The SAS Supervisor will begin the activation of units for the coming day shift before handing over duty to the oncoming Day Shift SAS Supervisor at 0500hrs.

Day Shift		Night Shift			
0600hrs	FBJ Renal Unit	1800hrs	Night unit monitoring	Evening and night shift focal point	Night unit activation
0700hrs		1900hrs			Finalize daily roster and send email to SAS, TRS, HCC
0800hrs	OPD	2000hrs			
0900hrs		2100hrs			Random ambulance inspection
1000hrs	FBJ Renal Unit	2200hrs			
1100hrs		2300hrs			Daily report and ePCR Summary
1200hrs	Daily schedule and admin	0000hrs			
1300hrs	Emails	0100hrs			
1400hrs	Focal point/work alongside Production	0200hrs			
1500hrs		0300hrs			Work alongside Production
1600hrs		0400hrs			
1700hrs	Night unit activation	0500hrs			Day unit activation

	Operational troubleshooting and checks
	Routine administration, correspondence, reports and returns
	Access and Effectiveness

SOP 5.3: Inter-Facility Transport



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Scheduled Ambulance Service (SAS) and Retrieval Services (RS) Operations
Procedure	5.3.A EMERGENCY TRANSFERS FROM HGH ED TO HEART HOSPITAL
Implementation Date	25 February 2020
Purpose	To provide guidance on HMCAS emergent transfers from HGH ED to Heart Hospital procedures, roles, and responsibilities.
Responsibility	ALL RETRIEVAL SERVICE, SCHEDULED AMBULANCE SERVICE AND HCC COMMS PERSONNEL

1. Role Description:

- 1.1 HMCAS will provide a double paramedic crewed ambulance for the emergency inter-facility transfer of a STEMI patient from Hamad General Hospital to the Heart Hospital.

2. Equipment:

- 2.1 Standard Ambulance equipment shall be provided by HMCAS. Any additional equipment must be supplied by the transferring facility.

3. Dispatched Ambulance:

- 3.1 The first available ambulance (either Emergency Ambulance Service or Scheduled Ambulance Service) will be dispatched. Benchmark time from dispatch to arrival at the referring facility for an unstable STEMI patient going for PCI is less than 10 minutes.
- 3.2 The first available ambulance (either Emergency Ambulance Service or Scheduled Ambulance Service) will be dispatched. Benchmark Time from dispatch to arrival at the referring facility for a Stable STEMI patient going for admission in specialist facility for further care is less than 30 minutes.
- 3.3 Dispatch will be coordinated by the Healthcare Coordination Center in conjunction (where an interfacility ambulance is not available) with the National Command Center.

4. Hospital to Hospital Communication:

- 4.1 The referring hospital will contact the Healthcare Coordination Centre to request an ambulance for transfer.

5. Ambulance Activation:

- 5.1 The Physician in charge of the case at the referring facility will determine the acuity of the transfer, based upon the following criteria:
 - 5.1.1 Patients ventilated, on continuous medication infusion or otherwise unstable requires augmentation by the referring facility staff. The responding ambulance will report directly to the HGH Emergency Department.
 - 5.1.2 Patients that are stable but have been diagnosed with a STEMI (ST segment elevation myocardial infarction) will be transferred by the AP crew without escort. The ambulance will report directly to the HGH Emergency Department.
 - 5.1.3 Patients not ventilated, on medication infusions (within AP scope of practice) or who are otherwise stable, require augmentation by the receiving facility staff will be transported by the AP crew.

6. Completion of Transfer:

- 6.1 At completion of transfer HMCAS will not return the hospital transfer team or equipment.

7. Logistics:

7.1 HMCAS will be responsible for post-use cleaning of the ambulance and fault reporting.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Schedule Ambulance Service (SAS) and Retrieval Services (RS) Operations
Procedure	5.3.B PAEDIATRIC EMERGENCY CENTRE (PEC) TRANSFERS
Implementation Date	25 February 2020
Purpose	To provide guidance on HMCAS PEC unit activation procedure, roles, and responsibilities.
Responsibility	ALL OPERATIONAL STAFF AND HCC COMMS PERSONNEL

1. Role Description:

- 1.1 PEC Transfers are conducted by Scheduled Ambulance Services in conjunction with specialist personnel from the PEC hospital for the purpose of transporting paediatric patients between fixed facilities within the State of Qatar.

2. Crew Composition:

- 2.1 HMCAS will provide two Ambulance Paramedics who will be augmented by a transfer team of hospital specialists.
- 2.2 The escorting clinicians are employees of Hamed Medical Corporation and are responsible for the continuation of patient care.

3. Equipment:

- 3.1 Standard primary response equipment shall be provided by HMCAS. Any additional equipment must be supplied by the transferring facility.

4. Hospital to Hospital Communication:

- 4.1 Before activating the ambulance service, the patient will have been referred to the receiving facility and the referral accepted.
- 4.2 All communication with the receiving facility will be carried out by the referring facility.

5. PEC Unit Activation:

- 5.1 The benchmark time from dispatch to arrival at the referring facility for unstable patients is less than 10 minutes.
- 5.2 The benchmark time from dispatch to arrival at the referring facility for stable patients is less than 20 - 45 minutes.
- 5.3 The Physician in charge of the case at the referring facility will determine the acuity of the transfer, based upon the following criteria:
 - 5.3.1 Patients ventilated, on continuous medication infusion or otherwise unstable requires augmentation by the referring facility staff.
 - 5.3.2 Patients that are stable but being transferred to OPD for testing require a staff nurse from the referring facility.
 - 5.3.3 Patients not ventilated, on medication infusions within an AP scope of practice or who are otherwise stable. These patients will be transported by the AP crew.
 - 5.3.4 Patients not ventilated, on medication infusions outside the AP scope of practice (will require the Treating Clinician doctor to validate the need of the infusion for transport or alternatively the referring facility to provide a nurse escort to monitor the infusion).
 - 5.3.5 Refer to [SOP 3.22](#) HCC Call Taking.

6. Completion of Transfer:

- 6.1 At completion of transfer, HMCAS will only return hospital transfer staff (PEC Al Sadd only) and equipment if:
 - 6.1.1 There are no other PEC calls pending
 - 6.1.2 HMCAS is not in escalation
 - 6.1.3 HCC approves the return of staff

7. Logistics:

- 7.1 HMCAS will be responsible for post-use cleaning of the ambulance and fault reporting.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Scheduled Ambulance Service (SAS) and Retrieval Services (RS) Operations
Procedure	5.3.C NEONATAL INTENSIVE CARE UNIT (NICU) TRANSFERS
Implementation Date	25 February 2020
Purpose	To provide guidance on HMCAS NICU Unit activation procedure, roles, and responsibilities.
Responsibility	ALL OPERATIONAL STAFF AND HCC COMMS PERSONNEL

1. Role Description:

- 1.1 NICU Transfers are conducted by Scheduled Ambulance Service and/or Retrieval Services in conjunction with specialist personnel from Women's Hospital NICU for the purpose of transporting high acuity neonatal patients between fixed facilities.

2. Crew Composition:

- 2.1 HMCAS shall provide two Ambulance Paramedics who will be augmented by a transfer team of hospital specialists.
- 2.2 The escorting clinicians are employees of Hamed Medical Corporation and are responsible for the continuation of patient care.

3. Equipment:

- 3.1 Women's Hospital NICU shall provide a 'ParAid' stretcher, neonatal incubator, and all medical consumables. Any stretchers must be removed from the ambulance prior to arrival at Report Point.

4. Hospital to Hospital Communication:

- 4.1 Before activating the NICU unit, the patient will have been referred to and accepted by the receiving facility.
- 4.2 All communication with the receiving facility will be carried out by the referring facility.

5. NICU Unit Activation:

- 5.1 Benchmark time from dispatch to arrival at the referring facility (or pickup point at Women's Hospital Emergency Department) is less than 45 minutes.
- 5.2 The HMCAS NICU Unit will report to the referring facility based upon the following criteria:
 - 5.2.1 When a patient is being transferred from the referring facility, using the referring facility staff and equipment, the NICU Unit will report directly to the referring facility and meet the transferring staff.
 - 5.2.2 When a patient is being transferred from the referring facility, using the NICU team and equipment from Women's Hospital, the HMCAS NICU Unit will report to the Women's Hospital Emergency Department entrance and meet with the transferring staff.
- 5.3 Activation decision:
 - 5.3.1 Activation will be initiated on the order of the Neonatologist on-call.
 - 5.3.2 Refer to [SOP 3.22](#) HCC Call Taking

6. Completion of Transfer:

- 6.1 At completion of transfer, HMCAS will return hospital transfer team to Women's Hospital Emergency Department or referring hospital if staff and equipment is not from Women's

Hospital. The NICU 'ParAid' stretcher, incubator and associated medical equipment will be removed.

7. Logistics:

- 7.1 HMCAS will be responsible for post-use cleaning of the ambulance and fault reporting. Women's Hospital NICU will be responsible for reconstituting and post use checks of the specialist equipment and ParAid stretcher.

8. Hours of Service:

- 8.1 Until the service has sufficient capacity to provide a dedicated vehicle, the following platforms will be used as available in the following order:
- 8.1.1 IFT Unit
 - 8.1.2 Airport Unit
 - 8.1.3 PEC Unit
 - 8.1.4 Bariatric Unit
 - 8.1.5 Retrieval Services Unit

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	5.3.E AMBULATORY PATIENT TRANSPORT TO CARRY-OUT INTERFACILITY TRANSFERS
Implementation Date	01 January 2024
Purpose	To provide guidance on HMCAS transfers of patients between HMC Medical Facilities, and Roles and Responsibilities.
Responsibility	ALL HMCAS OPERATIONAL STAFF

1. Purpose:

- 1.1 HMCAS will provide a single/double paramedic crewed vehicle for Interfacility Transfers of **Stable Ambulatory/Stable wheelchair** patients who has been identified by the referring facility staff.
- 1.2 This may include patients going to Out-Patient Departments (OPD) for further investigation (physiotherapy, radiology, ophthalmology, ultra-sound, dentistry, podiatry etc.), for follow-up consultation, for hospital admission and/or continuity of care.

2. Equipment:

- 2.1 The Specialised Patient Carrier (SPC) will carry basic lifesaving equipment i.e. Portable Oxygen, Lifepak 1000, Basic Response Bag with a Bag Valve Mask.
- 2.2 The SPC's will carry a wheelchair to ensure smooth facilitation of a patient from a hospital bed to the vehicle or from the vehicle to a hospital bed.
- 2.3 The SPC's will have a Mobile Data Terminal (MDT), to ensure HMCAS monitors location and timing (time of dispatch till available) ensuring swift turnaround of AS resources and availability to service the next call.
- 2.4 The SPC's will have an Electronic Patient Care Report (EPCR), to ensure continuity of care and clinical documenting for patient transport:
 - 2.4.1 Patients transported by SPC will have a single set of vital signs recorded for every patient encounter.
 - 2.4.2 EPCR clinical documents for patients transported in SPC will be exclude from HMCAS Clinical Documenting Audit.

3. Scope:

- 3.1 To ensure safe transport of stable ambulatory/wheelchair patients who do not require an Ambulance to transport them between HMC Healthcare facilities.

4. Definitions:

- 4.1 **Specialised Patient Carrier:** The procurement of these vehicles is to transport the Stable Ambulatory/Wheelchair patient. These vehicles comes fully fitted to maintain service delivery.
- 4.2 **Stable patient:** A patient with vital signs within normal clinical parameters for that age group.
- 4.3 **Ambulatory patient:** A patient who can perform activities of daily living by themselves i.e. without assistance of a nursing aid/allied healthcare worker (refer to Annexure A or Annexure B).

5. Procedure:

- 5.1 Hospital to Hospital Communication

- 5.1.1 The referring facility will contact the Healthcare Coordination Center (HCC) on 16099 to request a booking slot for Interfacility transfer.
- 5.1.2 HCC Call Taker can with the use of Annexure A or Annexure B, interrogate the call to ascertain the Right Resource to the Right Patient at the Right Time.
- 5.1.3 The premise of a nurse contacting the HCC for a booking slot is considered after both facilities have agree to transport and receive the patient accordingly.

6. Ambulance Activation:

- 6.1 The Healthcare worker taking clinical care of the patient, will call the Healthcare Coordination Center (HCC) on 16099 and answer all of Computer Aided Dispatch (CAD) questions.
- 6.2 If the patient meets both definitions of being a Stable Ambulatory/wheelchair patient, then SPC unit can be used to transport the patient safely to his/her appointment.
- 6.3 Inclusion criteria for Adult and Pediatric patients:
 - 6.3.1 Patients who have an IV cannula with no attachments (Adult and Pediatric).
 - 6.3.2 Patients who have a CVP in-situ with no attachments (Adult).
 - 6.3.3 Pediatric patients being carried by parent/care giver.
 - 6.3.4 Pediatric patients who are not carried but can safely be transported in a child car seat.

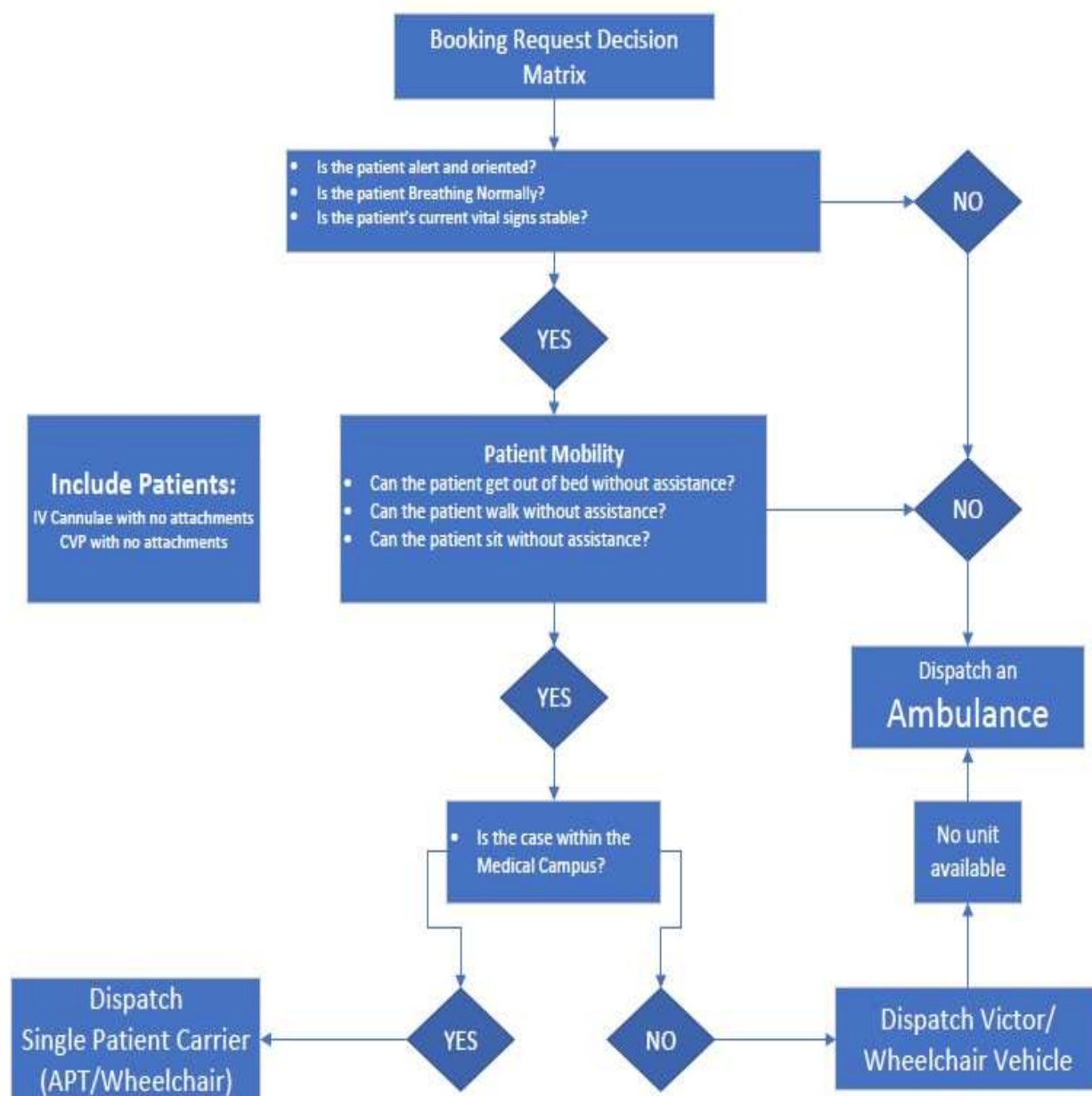
7. Logistics:

- 7.1 HMCAS will be responsible for pre- and post-cleaning of the minivans and fault reporting.

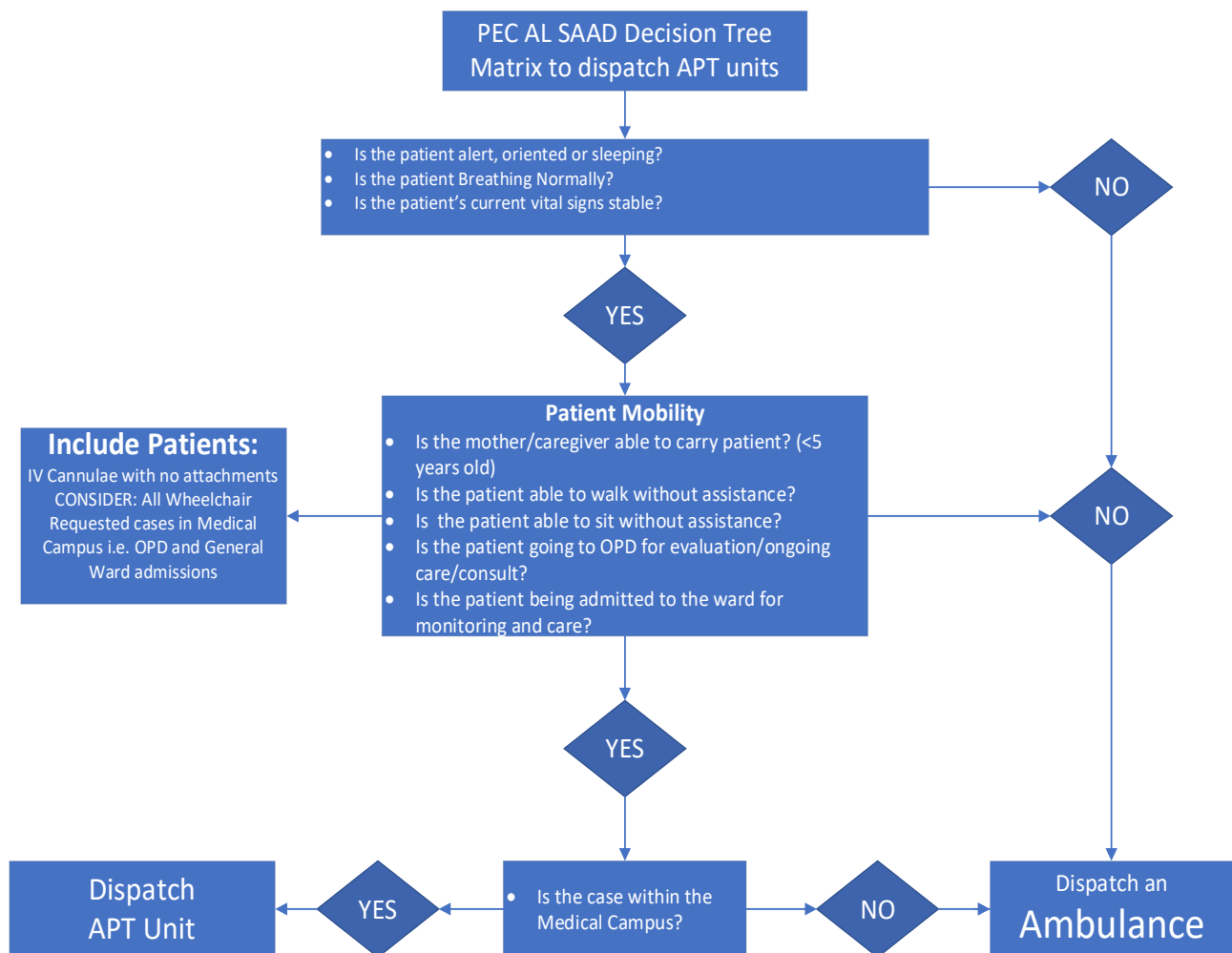
8. Hours of Service:

- 8.1 The SPC units will be in service 12 hours a day, 5 days a week (subject to change based on demand).

Annexure A: Adult Ambulatory Calls



Annexure B: Pediatric Decision Tree



SOP 5.4: Retrieval Services

SOP 5.4.A Retrieval Services Operations



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.A.2 HIGH ACUITY ADULT TRANSFERS
Implementation Date	01 October 2016
Purpose	Effective end to end management of SHAARP Transfers.
Responsibility	ALL RETRIEVAL SERVICE AND HCC PERSONNEL

1. Role Description:

- 1.1 High Acuity Adult Transfers are conducted by Retrieval Service in conjunction with specialist personnel from Hamad General Hospital MICU for the purpose of transporting high acuity adult patients between fixed facilities within Qatar.

2. Crew Composition:

- 2.1 HMCAS will provide 2 x Ambulance Paramedics and X1 Critical Care Paramedic who will be augmented by a Hamad Medical Corporation Physician from the MICU GH.

3. Equipment:

- 3.1 HMCAS will provide all necessary equipment with a specialized ParAid trolley for all High Acuity Transfer cases.

4. Activation time:

- 4.1 Retrieval Service crew and ICU Ambulance will be ready to deploy within 2 hours of activation.

5. Activation decision:

- 5.1 Activation will be initiated on the order of the receiving ICU Physician together with the on-call Retrieval Doctor.

6. Means of Activation:

- 6.1 High Acuity transfers will be activated by telephone call to the Retrievals desk from the receiving Physician.

7. Logistics:

- 7.1 HMCAS will be responsible for post-use cleaning of the ambulance, fault reporting, and for reconstituting and post use checks of the specialist equipment and ParAid stretcher.

8. Day Service:

- 8.1 Until the service has sufficient capacity to provide a 2nd dedicated vehicle and enough manpower for 24/7 coverage, the Retrieval Team will provide a service between the hours of 10:00 and 22:00.

9. High Acuity Transfer Criteria:**9.1 High Acuity Transfer:**

- 9.1.1 Scheduled transfer of patients between intensive care units.

9.2 Hospital Transfer:

- 9.2.1 Unscheduled transfer of a Level 1-3 patients between hospitals requiring a response time appropriate to the patient's condition. Team consist of 2 Ambulance Paramedics and an appropriate hospital escort as required. Equipment provided by the hospital.

- 9.2.2 Unscheduled Hospital transfers shall only be authorized by the Retrieval Consultant.

- 9.3 Retrieval Services shall not service a case from a hospital to an OPD.

10. On-call roster:

- 10.1 Each month the MICU Doctors shall provide the Retrieval Service with the monthly on-call list for High Acuity Transfers. In the event of a Doctor On-Call being unable perform his duties, the Operations Manager – CCS is to be notified in advance.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.A.3.2 COMMERCIAL FLIGHT OPERATIONS: REPATRIATION TO QATAR UPON COMMERCIAL AIRCRAFT
Implementation Date	13 March 2018
Purpose	To standardize the approach to mission planning and execution for the Repatriation of patients from international destinations to Qatar upon commercial aircraft.
Responsibility	ALL HMCAS RETRIEVAL SERVICES PERSONNEL

1. General process pertaining to the transport of patients upon commercial aircraft:

- 1.1 The request for Retrieval Service (RS) escort for the repatriation of patients from International Destination to Qatar upon commercial aircraft may come from a number of sources. Regardless of the source of financial approval, the process outlined in this SOP must be followed to ensure the safe transport of patients.

2. Communication with External Parties/Agencies:

- 2.1 All communication with external parties/agencies should ideally be via the Retrievals Desk.
- 2.2 All internal and external email communication should include the "Retrieval Service Mission Coordination Team" email group members:
 - 2.2.1 Retrievals Desk
 - 2.2.2 Hamad General Hospital MICU Retrieval Consultant
 - 2.2.3 Operations Manager - CCS
 - 2.2.4 Clinical Coordinator
 - 2.2.5 Sierra Retrieval (all)
- 2.3 Direct communication with external agencies may be conducted by Retrieval Service team members in instances where complex details need to be explained.
- 2.4 If communication with an external party is required to be held without involving the Retrievals Desk, a summary email summarizing the outcome of the communication is required to be sent to the Retrieval Service Mission Coordination Team mail group.

3. Recording of Communication:

- 3.1 All communication with internal (HMCAS) or external parties/sponsors is to be recorded.
- 3.2 All paper records must be scanned and filed.
- 3.3 All electronic documents must be emailed to the Retrievals Desk to be saved on to the CAD.
- 3.4 Any information passed via phone must be put into email and sent to the receiving person(s) as soon as possible after the phone conversation.
 - 3.4.1 All RS personnel are to encourage external parties to do this also.
 - 3.4.2 If an external party is unable to provide written confirmation via email/fax or hard copy, it is the RS member's responsibility to put the details into email and send it to the relevant person for confirmation.

4. Mission Sponsorship/Financial Responsibilities:

- 4.1 Financial sponsorship of the patient's treatment, travel and medical escort costs may be from (but not limited to) the following parties:
 - 4.1.1 Hamad Committee for Treatment Abroad (HCTA)

- 4.1.2 International Medical Affairs Office (IMAO)
- 4.1.3 Supreme Committee for Treatment Abroad (SCTA)
- 4.1.4 Ministerial Committee for Treatment Abroad (MCTA)
- 4.1.5 Third Party
- 4.2 The requesting party is responsible for all financial costs repatriating (see point 4.1) the patient including:
 - 4.2.1 Flight tickets of escorting medical staff
 - 4.2.2 The payment of excess baggage
- 4.3 Any agreement for HMCAS to cover costs requires authorization of the AED – Clinical Services.

5. Repatriation Mission Request Notification:

- 5.1 Notification of a requirement for RS medical escort, regardless of source, must be made in writing to the Retrieval Desk. This may be via email, fax or hand delivered letter.
- 5.2 Any documents received by hand or fax must be scanned and sent to the retrievals desk by email for filing.
- 5.3 Any member of the HMC Ambulance Service, who receives a request for RS escort on commercial aircraft, must be passed to the Retrievals Desk.
- 5.4 The Retrieval Desk will automatically assign a “GOAL ID” upon receipt of notification of any patient who is intended to travel with Retrieval Services.

6. Clinical Approval:

- 6.1 Upon receipt of the request for escort, the Clinical Coordinator shall gather all pertinent clinical information regarding the patient. If available details are not sufficient, this may require a conference call to the treating physician.
 - 6.1.1 Conference call should include Retrieval Consultant or designee and the treating physician abroad.
 - 6.1.2 The Retrieval Consultant may assign the Clinical Coordinator to do the conference call and report back to him with clinical details via email.
- 6.2 The Retrieval Consultant will provide provisional clinical approval of RS personnel to escort the patient.
- 6.3 After review of the clinical information presented by the Clinical Coordinator, and the patient is suitable to travel upon a commercial aircraft, the Retrieval Consultant will provide the following information to the requesting party via email:
 - 6.3.1 The recommended skill level of care required to manage the patient during the flight (e.g. personnel skilled in the use of ventilator and capable of intensive care support). And/or:
 - 6.3.2 The recommended team composition to manage the patient during the flight.
 - 6.3.3 The type of seat configuration required to manage the patient (if required).
 - 6.3.4 The Retrieval Consultant may delegate the responsibility of this correspondence to the Clinical Coordinator or other RS team member.
- 6.4 The Retrieval Consultant may deny further RS involvement in the escort of patients if:

- 6.4.1 The patient is defined as “low risk”. i.e. the patients’ clinical condition does not require Critical Care Paramedic level of care.
- 6.4.2 The patient is not “fit to fly”. i.e. the patients’ clinical condition makes the patient unsuitable for travel upon commercial aircraft.
- 6.4.3 The case is assigned to a different department e.g. Heart Hospital or Trauma ICU.
- 6.5 If the patient is “not fit to fly” the Retrieval Consultant will send the rationale to the Mission Sponsor with a recommendation of how to proceed with the patient’s transport (e.g. implement stabilization measures and reassess at a later date/change of destination/arrange for private air ambulance).
- 6.6 Patients may require liaison between the Retrieval Consultant and other specialists in order to prepare the patient for the flight/implement stabilization measures.
 - 6.6.1 The Retrieval Consultant may send the required correspondence with these persons directly or delegate this responsibility to the Clinical Coordinator or other RS team members.
- 6.7 If the patient is “fit to fly” but not suitable for Commercial aircraft (e.g. bariatric) then the Retrieval Consultant together with RS team, shall make a recommendation on what aircraft type to use to the requesting committee.

7. Medical Information Form (MEDIF):

- 7.1 The current treating physician abroad shall be responsible for completing a MEDIF accurately and then send it to Qatar Airways. See [Appendix 5.4.A.3.2-1](#).
- 7.2 The current treating physician who completes the MEDIF shall also send a copy of the MEDIF to Retrievals Desk. Retrievals Desk shall liaise with the treating physician to email a copy.
- 7.3 MEDIFs should be completed within the given timeframe requested by the airline.
- 7.4 No MEDIF is to be provided to a patient or family member without it first being sent to the airline.
- 7.5 Retrieval Consultant should be notified immediately once a MEDIF has been completed by the treating physician and be given the MEDIF Number.

8. Mission Sponsor Approval:

- 8.1 The requesting party will confirm that the requirements for RS escort have been met (e.g. team composition and/or other specific requirements).
- 8.2 If the requesting party does not meet the requirements for RS escort (e.g. approval of a team less than the Retrieval Consultant recommended team composition), the Retrieval Consultant, Clinical Coordinator, Sierra Retrieval, and Operations Manager - CCS will jointly agree how the escort will proceed.

9. Mission Preparation: Initial Patient Visit:

- 9.1 Initial patient visit will be the conference call between Retrieval Consultant and the treating physician abroad.
- 9.2 All clinical details will be emailed to the Clinical Coordinator to keep record.
- 9.3 When a flight date has been confirmed, a conference call should be conducted at least 2 days prior to flight to ensure the patient is still clinically “fit for flight”.
- 9.4 Any change in clinical condition should then be emailed to the requesting party.

10. Mission Preparation: Logistics:

- 10.1 Sierra Retrieval will work with the Mission Sponsor to organize the booking of flights.
 - 10.1.1 Sierra Retrieval will inform the sponsoring party of available flight dates and names.
 - 10.1.2 Return flights shall be booked in accordance with the agreed planned date to return the patient to Qatar.
- 10.2 Where required (and if not already assigned), Sierra Retrieval will request from the sponsoring party the names of additional medical escorts (Doctors, Nurses and Respiratory Therapists etc.) who shall escort the patient.
- 10.3 Sierra Retrieval will coordinate preparation of the required equipment for the mission.

11. Mission Communications:

- 11.1 The Retrievals Desk will start a Mission Microsoft TEAMS group for the mission prior to the Pre-Flight visit being conducted.
 - 11.1.1 The TEAMS Group will be titled with the GOAL ID of the patient, the point of origin and the destination of the flight.
 - 11.1.2 The group will include the following members:
 - 11.1.2.1 All mission medical crew
 - 11.1.2.2 Sierra Retrieval
 - 11.1.2.3 Clinical Coordinator
 - 11.1.2.4 Retrieval Consultant
 - 11.1.2.5 Operations Manager - CCS

12. Mission Preparation: Pre-flight patient visit:

- 12.1 A final conference call will be conducted 2 days prior to team departure from Qatar.
- 12.2 The same criteria outlined in point 9 above will be reassessed.
- 12.3 If the patient has deteriorated and is therefore not “fit to fly” the Retrieval Consultant will notify the requesting committee if the case is to be postponed or cancelled due to the patient’s condition.
- 12.4 Sierra Retrieval will inform the Retrieval Service Mission Coordination Team and the Retrieval Consultant of any operational changes which require the cancellation or postponement of the flight.
- 12.5 The Retrievals Desk will notify the sponsoring party of any requirement to cancel or postpone the mission.
- 12.6 A final call needs to be made by the Retrievals Desk to the referring facility 3 hours prior to the team departing from Doha. If the patient has then deteriorated this will give the team enough time to cancel the flight and put the mission on hold until the patient has been stabilized.

13. Mission Preparation: Receiving Facility Confirmation:

- 13.1 The Retrievals Desk must make contact with the receiving facility to ensure the patient is expected, prior to the team’s departure from the referring destination.
- 13.2 The Retrievals Desk will email HGH Bed Management all patient details and relevant clinical records to ensure a bed has been booked in an appropriate unit.
- 13.3 The Retrievals Desk will notify mission TEAMS group that the receiving facility has confirmed they are expecting the patient.

14. Mission Preparation: Medical Attaché Confirmation:

- 14.1 The Retrievals Desk must make contact directly with the Medical Attaché (or assigned attaché coordinator) and outline any specific transport requirements.
 - 14.1.1 The Medical Attaché Request document template (RS.O.0009) should be used for this purpose. See [Appendix 5.4.A.3.2-2](#).
- 14.2 Telephonic or email confirmation from the attaché must have been received by the Retrievals Desk prior to departure from Doha.
- 14.3 The Retrievals Desk will notify the mission TEAMS group that confirmation has been received.

15. Mission Preparation: Emergency Contact Details:

- 15.1 The Retrievals Desk will ensure that all medical crew members accompanying the patient outside of RS have completed the Emergency Contact Details form (RS.O.0006) prior to departure. See [Appendix 5.4.A.3.2-3](#).

16. Mission Approval: Mission Tasking document:

- 16.1 A Tasking document (RS 2016 01 Ver 1.2) is required to be completed for every commercial expatriation mission. See [Appendix 5.4.A.3.2-4](#).
- 16.2 The Retrievals Desk is responsible for ensuring the Tasking Form is completed and may contact any person within and outside of HMCAS in order to attain all the required information.
 - 16.2.1 Before contacting the Mission Sponsor, the Retrievals Desk should first contact Sierra Retrieval to attain the required details.
- 16.3 The Tasking document shall be submitted a minimum of 24 hours prior, where possible, to the flight departure time.
- 16.4 The Tasking document is to be submitted to the Operations Manager – CCS or designee for approval.
- 16.5 Operations Manager – CCS or designee will email the approved Tasking Form to the Retrievals Desk.
- 16.6 Retrievals Desk shall email the Tasking document to AED-Clinical Services, SOM – CCS, Ops Manager – CCS, and SME Clinical Desk.

17. High Risk Mission Considerations:

- 17.1 Patients who are deemed to be at high risk of deteriorating during flight, but due to outside influence will continue to travel the following is to be conducted.
 - 17.1.1 The Operations Manager - CCS on-call must be notified.
 - 17.1.2 The patient and/or family must be informed of the risks of flight.
 - 17.1.3 The Retrieval Consultant (or designee) must confirm with the patient and family that the flight can't be diverted.
 - 17.1.4 The patient (or authorised family member) must sign the Acknowledgement of Agreement on High Risk Travel (See [Appendix 5.4.A.3.2-7](#)).
 - 17.1.5 The Retrieval Consultant will inform the carrying airline of the patient's condition and any requirements for in flight diversion.
 - 17.1.6 Confirmation must be obtained from the carrying airline stating that they have agreed that the patient may still travel on board the flight.

18. Mission Team Leader Pre-Departure Responsibilities:

- 18.1 A designated Team Leader will be appointed once the mission has been confirmed and the team selected.

- 18.2 The Mission Team Leader is required to provide a timeline for the major mission steps:
 - 18.2.1 Time of team rendezvous (and location)
 - 18.2.2 Time of departure to airport – HIA
 - 18.2.3 Time of arrival at the airport – Destination
 - 18.2.4 Approximate time of customs/immigration clearance at destination (in destination time)
 - 18.2.5 Time at Hotel
 - 18.2.6 Time at patient side
 - 18.2.7 Time departing patient to Airport
 - 18.2.8 Wheels up time
 - 18.2.9 Time at destination Doha
- 18.3 The Mission Team Leader should also outline the following requirements to the Retrievals Desk:
 - 18.3.1 Ambulance requirements (both local and at destination)
 - 18.3.2 Destination transport requirements from the Medical Attaché
 - 18.3.3 Any additional requirements
- 18.4 The Mission Team Leader is required to provide the following information to the Retrievals Desk:
 - 18.4.1 Hotel/Accommodation details
- 18.5 The Mission Team Leader should also assist the Retrievals Desk in ensuring all other mission team members (both RS and non-RS members) have completed their responsibilities outlined in point 19 below.

19. Mission Team Members Pre-Departure Responsibilities:

- 19.1 Mission Team Members must provide the following information to the Retrievals Desk:
 - 19.1.1 Provide the Retrievals Desk their Hotel/Accommodation details.
 - 19.1.2 Provide the Retrievals Desk their return flight booking details.
- 19.2 The Retrievals Desk is responsible for ensuring that these details are attained prior to departure.
- 19.3 These details should be entered into the mission TEAMS group.

20. Mission Pre-departure Tasks:

- 20.1 The Retrieval Desk will arrange local ambulances as required.
- 20.2 Sierra Retrieval will ensure all equipment is prepared.

21. Mission Execution: Mission Communications:

- 21.1 The mission team leader shall ensure the following notifications are put on the mission TEAMS group:
 - 21.1.1 At team rendezvous location
 - 21.1.2 Departing to Airport
 - 21.1.3 At airport
 - 21.1.4 Cleared Immigration/Customs

- 21.1.5 At Gate
- 21.1.6 On Board Aircraft
- 21.1.7 Departing/Wheels up
- 21.1.8 Arrived at Destination/Wheels Down
- 21.1.9 Disembarked aircraft
- 21.1.10 At Hotel
- 21.1.11 Departing Hotel for patient
- 21.1.12 At patient side
- 21.1.13 Loading patient into ambulance
- 21.1.14 Departing for Airport
- 21.1.15 Awaiting Boarding
- 21.1.16 Boarded aircraft, patient loaded
- 21.1.17 Wheels up
- 21.1.18 Wheels Down
- 21.1.19 Cleared Customs/Immigration
- 21.1.20 With HMCAS Ambulance, Departing for HGH
- 21.1.21 At HGH, patient handover being completed
- 21.1.22 Retrieval Service Mission Complete (once all equipment is returned)
- 21.2 The Retrieval Desk will continue to monitor the status of all members until all members have returned to Qatar.
 - 21.2.1 The Retrieval Desk will notify “Mission Complete: All Team Members have returned to Qatar” once this has occurred.

22. Mission Execution: Safety Checks:

- 22.1 Safety checks need to be conducted by each mission member.
- 22.2 The mission team leader is responsible for informing non-Retrieval Service members of safety check requirements.
- 22.3 Each team member should notify on the TEAMS group “Safety Check”.
- 22.4 Safety Checks should be conducted every 24 hours until mission is completed.
- 22.5 Safety Check times can be adjusted by the team member as required provided the time of the next safety check in time is stated on the mission TEAMS group.
- 22.6 The Retrieval Desk is responsible for monitoring that all medical team members have conducted their safety checks at the designated time.
- 22.7 If a team member fails to check in:
 - 22.7.1 The Retrievals desk must request a safety check update from the team member.
 - 22.7.2 If the TEAMS message has not been delivered successfully (two “grey-ticks” on the message) the Retrievals Desk must send an SMS to the team member.
 - 22.7.3 After 30 minutes of no contact, the Retrievals desk must call the team member on their mobile.

- 22.7.4 If no contact is made, the Retrievals Desk must call the team members Hotel/Accommodation phone number. – A message should be left with the hotel reception if they are unable to contact.
- 22.7.5 If still no contact is made, the Retrievals Desk should notify Sierra Retrieval On call.
- 22.7.6 The Sierra Retrieval on call shall then escalate to the Operations Manager on call.

23. Mission Documentation:

- 23.1 Upon completion of the mission, the Mission Team Leader is to ensure that the EPCR is complete and that the case is closed.
 - 23.1.1 All mission documentation should be returned and no documentation are to remain in the possession of the clinical team.
 - 23.1.2 Post Mission Case Reviews and Audits will be conducted in coordination with the Quality and Patient Safety Team.

24. Appendices:

- 24.1 [Appendix 5.4.A.3.2-1: Medical Information Form \(MEDIF\)](#)
- 24.2 [Appendix 5.4.A.3.2-2: RS.O.0009 Medical Attaché Form](#)
- 24.3 [Appendix 5.4.A.3.2-3: RS.O.0006 Emergency Contact Details Form](#)
- 24.4 [Appendix 5.4.A.3.2-4: RS 2016 01 Ver 1.2 Mission Tasking Form](#)
- 24.5 [Appendix 5.4.A.3.2-7: CL711 Acknowledgement of Agreement on High Risk Travel](#)

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.A.8 EXTRACORPOREAL MEMBRANE OXYGENATION (ECMO) PATIENT TRANSFERS
Implementation Date	01 October 2016
Purpose	Effective end to end management of Retrieval Transfers.
Responsibility	ALL RETRIEVAL SERVICE PERSONNEL

1. Role Description:

- 1.1 ECMO Transfers are conducted by Retrieval Service in conjunction with specialist personnel from Hamad Hospital MICU for the purpose of transporting ECMO patients between fixed facilities within the Sovereign Territory of the State of Qatar.

2. Crew Composition:

- 2.1 HMCAS will provide 2 x Ambulance Paramedics, augmented by a transfer team of hospital specialists and x 1 Retrieval Service CCP.
- 2.2 Sierra Retrieval shall be part of the team to ensure adequate transportation of Hospital ECMO Team and ECMO Equipment to the referring facility.
- 2.3 The Hamad Medical Corporation Hospital ECMO Team consist of x1 ECMO Consultant, x1 ECMO Nurse and x1 Perfusionist.

3. Equipment:

- 3.1 Retrieval Services will provide a 'ParAid' stretcher, ICU Ambulance, and monitoring equipment. HGH MICU will provide ECMO Specific equipment.

4. Activation time:

- 4.1 Retrieval Service crew and ambulance will be ready to begin the transfer within 90 minutes of activation. Rapid Response activation of initial team by Sierra Retrieval. Collect hospital team and move to referring facility. ICU Ambulance and rest of the team follow under 90 min with the rest of the equipment.

5. Activation decision:

- 5.1 Activation will be initiated on the order of the ECMO Consultant on-call.

6. Means of Activation:

- 6.1 ECMO transfers will be by telephone call to HCC Communications Retrieval Desk.

7. Report Point:

- 7.1 The report point for pick-up of ECMO transfer team will be outside Hamad Hospital Accident and Emergency Department.

8. Logistics:

- 8.1 HMCAS will be responsible for post-use cleaning of the ambulance and fault reporting. HGH will be responsible for reconstituting and post use checks of the specialist equipment.

9. Daytime Service Provision:

- 9.1 Until the service has sufficient capacity to provide a dedicated vehicle between 07:00 and 19:00 the ECMO service will be provided by the Retrieval ambulance.
- 9.2 In case the Retrieval Ambulance is not available, an ICU type IFT ambulance will be used.
- 9.3 Call Activation:

- 9.3.1 ECMO Consultant calls Retrievals Desk (RD) and activates ECMO case.
- 9.3.2 Retrievals Desk will inform Production and Clinical Coordinator to activate Retrieval Vehicle and team.
- 9.3.3 Sierra Retrieval immediately goes to HGH ED to meet ECMO Team and proceed to referring facility.
- 9.3.4 Retrievals Team to be at HGH ED within 45min of activation to load equipment and go through pre-brief and checklist.
- 9.3.5 Rest of team to be ready to leave HGH ED within 90min of activation to referring facility.

10. Night Service Provision:

- 10.1 Until the service has sufficient capacity, ECMO calls at night will be serviced by Retrieval unit, subject to staff availability and Retrieval unit activity.
- 10.2 Activations outside of Retrieval unit operations should be escalated to the Ops Manager – CCS.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.A.9 AEROMEDICAL TRANSPORT OF BARIATRIC PATIENTS USING QEAF AIRCRAFT
Implementation Date	01 October 2016
Purpose	Notify staff of the steps required to transport Bariatric Patients on QEAF Strategic Transport Aircraft.
Responsibility	ALL RETRIEVAL SERVICE PERSONNEL

1. Eligibility:

- 1.1 Subject to approval from the Ministry of Public Health, Bariatric patients that cannot be transported on commercial flights will be transported by HMCAS Retrieval Service using military aircraft.

2. Service Provision:

- 2.1 HMCAS Retrieval Service will provide a bedside to bedside service from home or hospital to receiving facility using ground and air transport platforms.

3. Vehicles:

- 3.1 By definition, these patients are too large to fit into standard ambulances. In the absence of dedicated 'super-bariatric' ambulances, the following vehicles are required to transport bariatric patients.
- 3.1.1 **ICU Ambulance:** For carriage of medical equipment and gases
 - 3.1.2 **Catering truck:** For transport of patient and bariatric bed
 - 3.1.3 **Low loader truck:** For transportation of Fork Lift
 - 3.1.4 **Fork Lift:** For loading of patient into catering truck
 - 3.1.5 **Sierra Retrieval vehicle:** For transport of Operations Production staff and additional personnel as required.
 - 3.1.6 **Green Bus:** For transport of additional hospital staff to Al Udeid Air Base.
 - 3.1.7 **Panel Van:** For transportation of additional equipment if required.

4. Operational Phases:

- 4.1 Pre-assessment of patient by a competent medical authority (Medical Director/ICU Consultant).
- 4.2 Assessment of access and egress at patient's home
- 4.3 Route reconnaissance from patient's home to hospital
- 4.4 Admission to hospital for pre-flight work up (bed to be booked)
- 4.5 Arrangement of bed at receiving facility
- 4.6 Arrangement of flight, visas and exit permits for patient, escorting relatives, and staff.
- 4.7 Movement to hospital
- 4.8 Pre-flight work up
- 4.9 Movement to Al Udeid Air Base
- 4.10 Aeromedical transport
- 4.11 Handover to receiving hospital team

- 4.12 Transport to receiving facility
- 4.13 Recovery of team and equipment to Doha

5. Communications:

- 5.1 Command and Control will be executed through Channel Tango 4.
- 5.2 Standard call signs will be used. Additional non-standard call-signs will be agreed during mission planning.

6. Command and Control:

- 6.1 Mission Team Leader will be responsible for all aspects of tactical command and control.

7. Recording and Reporting:

- 7.1 Retrievals Desk will be responsible for tracking and recording mission activity and providing situational updates to all call signs.

8. Escalation:

- 8.1 Problems and untoward events that cannot be resolved at the tactical level must be escalated by the team leader to Sierra Retrieval on call or Ops Manager - CCS.

9. Further Guidance:

- 9.1 These missions are sufficiently complex to require detailed planning and briefing and firm command and control throughout all phases of execution. Further guidance is contained in Retrieval Services Standing Contingency Plan for Aeromedical Transport of Bariatric Patients by Qatar Emiri Air Force Strategic Transport Assets.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.A.10 AEROMEDICAL LOGISTIC PLANNING CONSIDERATIONS
Implementation Date	01 October 2016
Purpose	To provide guidance for planning logistic aspects of aeromedical missions.
Responsibility	ALL RETRIEVAL SERVICE FW FLIGHT PERSONNEL

1. Team Leader:

- 1.1 Retrieval Service Critical Care Paramedics are responsible for planning logistic aspects of international aeromedical missions. This SOP gives guidance on considerations that inform logistic planning.

2. General Principles:

- 2.1 Extended duration Inter-Facility Missions are to be planned as bedside to bedside transports with the logistic support plan conforming to the operational plan. The principles of logistic support are:
- 2.1.1 **Simplicity.** The more complex the logistic support plan is, the more likely it is to fail.
 - 2.1.2 **Conformity.** The logistic plan must conform to and synchronize with the operational plan.
 - 2.1.3 **Economy.** Manpower and equipment are limited resources; the logistic support plan should be as lean as possible, avoiding unnecessary use of staff, time, space, and resources.
 - 2.1.4 **Flexibility.** The logistic plan must be flexible enough to deal with unexpected demands and changes to the operational plan.
 - 2.1.5 **Foresight.** The logistic plan must anticipate likely eventualities resulting from the operational plan.
 - 2.1.6 **Robustness.** The logistic plan must have in-built resilience to withstand increased demand.

3. Logistic Considerations:

- 3.1 Logistic considerations to support Inter-Facility Missions must take account of the '4Ds' of Logistic Support:

3.1.1 Destination:

- 3.1.1.1 The following factors can be considered under 'Destination':

- 3.1.1.1.1 What is the destination of the mission?
- 3.1.1.1.2 What is the climate/temperature?
- 3.1.1.1.3 What local healthcare provision can be used?
- 3.1.1.1.4 Where is the receiving facility?
- 3.1.1.1.5 Where is the most appropriate Airport of Disembarkation in relation to the receiving facility?
- 3.1.1.1.6 What are the best routes for ground transport – in Qatar, at destination?
- 3.1.1.1.7 Are there any endemic diseases/health warnings in force at the destination?

- 3.1.1.1.8 What airports might be used enroute for Technical Stops or Emergency Diverts?

3.1.2 **Duration:**

- 3.1.2.1 The following factors can be considered under 'Duration':

- 3.1.2.1.1 What is the expected total mission duration?
- 3.1.2.1.2 What is the flying time/journey time?
- 3.1.2.1.3 What is the expected time of each leg of the journey?
- 3.1.2.1.4 What is the expected duration of planned Technical Stops?
- 3.1.2.1.5 What is the expected duration of any delays (e.g. clearing immigration/customs etc.
- 3.1.2.1.6 What is the potential impact of duration on cold-chain items?

3.1.3 **Distance:**

- 3.1.3.1 The following factors can be considered under 'Distance':

- 3.1.3.1.1 Transit times between facilities in destination country and within the State of Qatar.
- 3.1.3.1.2 Location of Emergency Departments on route in case of emergency stops during road moves.
- 3.1.3.1.3 Fuel requirements for road moves.
- 3.1.3.1.4 Total mission time and safe working hours – need for additional team and pre-positioning for multi-sector journeys.
- 3.1.3.1.5 Traffic conditions and requirement for additional escort.

3.1.4 **Demand:** The following factors can be considered under 'Demand':

- 3.1.4.1 Number of patients and level of care required
- 3.1.4.2 Additional standard equipment requirements (more of the same)
- 3.1.4.3 Specialist equipment requirements (e.g. NICU/Bariatric eqpt)
- 3.1.4.4 Medical gas requirements
- 3.1.4.5 IV Fluids
- 3.1.4.6 Non-standard pharmaceuticals
- 3.1.4.7 Medical consumables
- 3.1.4.8 Dangerous Air Cargo
- 3.1.4.9 Accommodation, feeding and transport requirements for personnel.

4. **Medical Logistics:**

- 4.1 For medical logistic planning, the '4C's can be asked of each of the '4C's:

4.1.1 **Capacity:**

- 4.1.1.1 Is there a bed available at the receiving facility? If not, are there alternative facilities that can accept the patient?
- 4.1.1.2 What transport platform is required to accommodate the patient, the team(s) and accompanying family members?

4.1.1.3 Is there a requirement for additional vehicles for elements of the journey (e.g. panel van, green bus etc.)?

4.1.2 Capability:

4.1.2.1 Do the transporting platforms and the receiving facility have capability required to meet the needs of the patient?

4.1.2.2 What is the skillset and team composition required to provide optimum care for the patient?

4.1.3 Continuity:

4.1.3.1 The mission is to be planned as a bedside to bedside transport.

4.1.3.2 Is there a need for technical stops at facilities enroute?

4.1.3.3 Is there a need for additional teams or pre-positioned teams?

4.1.4 Convenience:

4.1.4.1 Are there closer facilities that offer the same capability for the patient with reduced risk associated with longer distances to be travelled?

4.1.4.2 Does a particular facility offer greater convenience for the patient?

5. Logistic Push/Pull:

5.1 Once initiated, Retrievals Service operations must be self-sufficient. This implies a degree of 'over provision' to be able to cater for unexpected events and delays. The governing principle is that on routine aeromedical missions, sufficient stocks are carried to cater for 24 hours of operation.

5.2 At the end of each mission, replenishment will be provided based on stocks consumed.

6. Cargo Manifesting:

6.1 As the Retrievals Service matures, all standard medical modules are to be weighed and manifested to allow Air Load Masters, ground staff and aircrew to complete weight and balance reports.

7. Dangerous Air Cargo:

7.1 A list of Dangerous Air Cargo is to be provided

8. Prohibited items:

8.1 Prohibited items are not to be carried

9. IT Equipment:

9.1 Regardless of destination, all IT equipment, including cell phones, is to be fully charged at commencement of the mission.

10. Transport of vehicles:

10.1 If vehicles are to be transported on military transport aircraft, fuel tanks are to be ¼ full and the condition of tires inspected to ensure that there is no damage and no punctures. Vehicles will be embarked under the strict control of the Air Load Master.

11. Controlled Medications:

11.1 Personnel carrying controlled medications must be in possession of an official letter from the Ministry of Public Health stating their qualification and requirement to carry them. Utmost care must be taken of controlled medications, including safeguarding them from theft, loss, or

damage. Controlled Medications may only be carried in the box provided by Hamad Medical Corporation Ambulance Service.

12. Passenger Manifest:

- 12.1 A passenger manifest must be completed to include details of medical personnel, patients and relatives/friends accompanying and Passport, Qatar ID number and Residence Permit and Exit/Entry visa details.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.A.13 EMERGENCY RATION PACKS
Implementation Date	01 October 2016
Purpose	To provide guidance on direction on the use of emergency ration packs.
Responsibility	RETRIEVAL SERVICE AEROMEDICAL PERSONNEL

1. Purpose:

- 1.1 Emergency Rations are provided to Retrieval Service personnel undertaking aeromedical retrievals on the following platforms:
 - 1.1.1 Qatar Emiri Air Force fixed wing aircraft
 - 1.1.2 LifeFlight AW139 rotary wing aircraft
- 1.2 These rations are for emergency use only and are not to be consumed as routine. Examples of emergency situations include:
 - 1.2.1 Aircraft out of service in mission with delayed return
 - 1.2.2 Aircraft crash leading to a survival situation
- 1.3 The following do not constitute emergency situations:
 - 1.3.1 Delay in departure from Airport of Embarkation in Qatar

2. Ration Contents:

- 2.1 The emergency ration provides:
 - 2.1.1 2 x 'Meals Ready to Eat' (MRE) per person
 - 2.1.2 3 x litres of water per person
- 2.2 Each MRE contains sundry items and a self-heating pack.

3. Use of the MRE:

- 3.1 All contents of the MRS can be eaten cold, but the main meal items taste better cooked.
- 3.2 Instructions for the use of the heater are included in the pack and are self-explanatory.
- 3.3 In the absence of a rock, the user will need to use their discretion as to what constitutes a suitable "or something" (note, do not use flammable material).
- 3.4 MRE heaters should not be used on-board the aircraft.

4. Safety considerations:

- 4.1 Thermal injury can result from hot food and liquids.
- 4.2 MRE heaters are classed as 'Dangerous Air Cargo' and must not be from their packaging in-flight.

5. Routine meals:

- 5.1 As part of their personal mission preparation, staff should carry 2 x meals and sufficient water for 12 hours duration at their own expense.
- 5.2 Poor personal preparation does not constitute reasonable grounds for consumption of emergency rations.

SOP 5.4.E: Retrieval Services Quality Management & Quality Improvement



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Service (RS) Operations
Procedure	5.4.E.1 RETRIEVAL SERVICE OPERATIONAL CURRENCY
Implementation Date	13 March 2018
Purpose	To ensure that operational staff are operationally competent, assessed and signed off before taking on any Retrieval Service Case/Mission.
Responsibility	ALL RETRIEVAL SERVICE STAFF

1. Purpose:

- 1.1 Retrieval Service is providing patient transport across multiple platforms locally and internationally. Critical Care Paramedics (CCP) and Ambulance Paramedic (AP) transporting patients within Retrieval Services shall be operationally signed off before performing duties.

2. Aeromedical Retrieval Operational Requirements:

- 2.1 All RS CCP's and AP's shall be orientated annually on the following aircrafts:
 - 2.1.1 C130
 - 2.1.2 C17
 - 2.1.3 Qatar Airways Commercial Aircraft
- 2.2 All RS CCP's shall be orientated annually on the following aircraft:
 - 2.2.1 A7MED
 - 2.2.2 AW139
- 2.3 For all Commercial/C130 and C17 missions, both CCP's and AP's assigned to their first mission will be paired with another experienced RS CCP whom will assess the following:
 - 2.3.1 The CCP demonstrates competence in leadership skills throughout the mission.
 - 2.3.2 The CCP/AP clearly demonstrates knowledge of equipment, logistics and mission specific requirements.
 - 2.3.3 The CCP/AP clearly demonstrates adequate knowledge with regards to mission administrative matters i.e. Narcotic letter, SCH letter, MSDS Form, PCR/ePCR and MEDIF'S.
 - 2.3.4 The CCP/AP demonstrates the ability and confidence to perform when experienced staff are not immediately available for consultation.
 - 2.3.5 Ensures proper planning and preparation of the mission prior to take-off.
 - 2.3.6 The CCP/AP understands and demonstrates the importance of continuous liaison with the family and/or patient during flight.
 - 2.3.7 *Unless approved by Assistant Executive Director that the staff member can operate without being paired on his/her first mission.
- 2.4 Upon completion of the mission the experienced RS CCP shall complete the Operational Competency Sheet (Appendix A, next page) and report back to the Operations Manager - CCS on operational readiness or remedial action to be taken.
- 2.5 Visa requirements:
 - 2.5.1 Retrieval Service Staff require entry to the following countries:
 - 2.5.1.1 UK

2.5.1.2 Schengen zone

2.5.1.3 USA

2.5.2 Adhoc Visas might be required

2.5.3 Retrieval staff are responsible for their visas and are to ensure that they maintain the validity thereof.

2.5.4 Staff should keep their personnel file up to date on the shared drive with valid and updated documentation.

3. Ground Operational Requirements:

- 3.1 All CCP's and AP's will undergo orientation on all transport platforms together with experienced staff and Sierra Retrieval.
- 3.2 All newly joined staff shall work with experienced crew until they are operationally and clinically signed off by the Operations Manager.
- 3.3 All newly joined staff shall work a minimum of six months on the ground operations platforms before being assigned to an Aeromedical Retrieval, unless otherwise approved by the Operations Manager.

APPENDIX A

Date:

Candidate name:

Candidate Corp Number:

Evaluator name:

Evaluator Corp Number:

SECTION ONE: Operational Planning and Preparation outcome(s) being assessed:

Subject	Competent		Comments
	YES	NO	
1 Patient Visits			
Patient visit: At home			
Assesses patient at home for any possible risks/complications during flight			
Appropriately identifies required interventions to ensure the patient is safe for flight (e.g. IV line insertion)			
Ensures that the patient and any escorting family have visas in place			
Access and Egress to home assessed and identifies any risks to safe extrication			
Identifies if the patient's weight/size will impact the safety of air travel on commercial aircraft			
Correctly assesses the appropriateness of a commercial aircraft for this patient			
Patient Visit: At Hospital			
Assesses the patient for any possible risks/complications during flight			
Appropriately identifies required interventions to ensure the patient is safe for flight (e.g. IV line insertion)			
Ensures that the patient and any escorting family have visas in place			
Identifies if the patient's weight/size will impact the safety of air travel on commercial aircraft			
Correctly assesses the appropriateness of a commercial aircraft for this patient			

2	Patient preparation					
	Patient preparation at Home:					
	Correctly identifies if the patient will travel on a wheelchair or stretcher					
	Inserts IV line (if required)					
	Inserts / assists insertion of Foleys/Diaper (if required)					
	Assesses the patient for the requirement of anticoagulants (e.g. bedridden patients)					
	Patient preparation at Hospital:					
	Correctly identifies if the patient will travel on a wheelchair or stretcher					
	Notes the number and type of infusions					
	Assesses the ventilator requirements (FiO2 etc.) status and correctly identifies the oxygen consumption requirements for the flight					
	Identifies the type of medication and monitoring access available (e.g. CVC, Art line, Peripheral etc.)					
	Assesses the patients dietary requirements (intake and route) and assesses the potential for it to be stopped or reduced for the flight.					
3	Medication preparation					
	If the patient is on infusions, liaise with Charge RN to prepare enough for 24hours.					
	Ensure the patient from home has enough Medication for the duration of flight and 1 more day.					
	Medications correctly signed out of HMC AS Pharmacy					
	Liaise with Clinical Leader if any extra Medication should be added.					
	Ensures sign out of Istat Cartridges when signing out Medication					
4	Equipment preparation					
	For patient at home: Liaises with Sierra Retrieval for preparation of minimum equipment					

	For patient at Hospital: Liaises with Sierra Retrieval for preparation of minimum equipment as well as additional:			
	- Syringe drivers if patient is on more than 1 infusion - Converter for Economy class to keep equipment charged - Extension lead for equipment in Economy class - Vacuum mattress for stretcher case			
	Liaise with Charge RN for Adult Diapers when necessary			
	Oxygen tube connector to connect to QA Commercial Oxygen for the Hamilton.			
	Extra Batteries in accordance to duration of flight. Cannot charge equipment in Business class.			
5	Flight Pack			
	Ensures the following are prepared for flight:			
	- MEDIF - SCH Letter - Narcotics Letter - MSDS Forms - Any patient medical records - PCR - Tasking Form			
6	Transportation of patient			
	Identifies if the patient should be transported via private transport or ambulance			
	Identifies if the patient will be moved by wheel chair or stretcher			
	Identifies the requirement for a Medilift			
	Ensure the team knows entrance to HIA will be via South Gate using an Ambulance for a stretcher case.			

7	Team brief			
	Conducts team briefing prior to departure on the plan if the patient should deteriorate.			
	Discusses the roles and responsibilities for patient treatment (i.e. who is in charge of what, Dr, RN, CCP and RT. CCP will always be the Team Leader)			
	When there are 2 CCP's on a mission: Clearly defines which CCP shall be the Team Leader and the other shall take the Clinical lead together with the Dr.			

SECTION TWO: In-Flight Procedures outcome(s) being assessed:

	Subject	Competent		Comments
		YES	NO	
1	Liaison with CSD			
	Liaises with the CSD once onboard the aircraft to explain the current condition in an understandable manner.			
	Asks the CSD where the nearest Oxygen bottle is and to have it ready if needed.			
2	Liaison with patient/family			
	Ensures continuous liaison with the patient/family to assure him/her about the patient's current condition.			
	In the event the patient is awake and conscious, continuously assesses the patients comfort and reassess vital signs at appropriate intervals.			
3	Equipment Storage			
	Stores the equipment in an appropriate and safe manner where it can be easily and quickly accessible in Business class.			
	Stores the equipment that is not needed and places those on charge that is currently in use for an Economy Stretcher case.			
	Maintains constant awareness of where all the equipment, consumables and medication are stowed to have quick and easy access when needed.			
	Liaises with the Cabin crew to place cold chain medication in their onboard fridge (which is designated for medication storage only).			
	Ensure that all equipment in use during stretcher cases is secured safely and out of the way, yet easily accessible to the team.			
4	Updates Mission Whatsapp Group			
	Always update the group on all patient and team movements:			
	Team readiness			
	Departing from referring facility with patient			

	At Airport				
	Boarding Aircraft				
	Update (if possible) during flight when connected to Wifi regarding patient condition				
	Landed at destination				
	En-route to Hospital				
	Patient handed over safely to receiving facility				
	Team safely at accommodation				
	Team boarding for flight back to Doha				
	Provides safety update within 12 hours of your stay at the destination country and every 24 hours thereafter				
	Update the group of any adverse events				
5	Performs Team Leader tasks				
	Ensures Team is ready on time				
	Ensures all equipment is ready together with Sierra Retrieval, and returned after mission				
	Ensures safe transportation of the patient and team together with Sierra Retrieval and the Retrievals Desk.				
	Liaises with external stakeholders during the mission where needed. i.e. The CSD, Embassy official, Medical Attaché etc.				
	Ensures safety of all team members' passports.				
	Escalates any operational concerns to Sierra Retrieval.				
	Escalates any clinical concerns to the Associate Medical Director TRS				

CCP Operationally signed off? : YES / NO

CCP Signature:

Operations Officer Signature:

Evaluator Signature:

SOP 5.4.F: Retrieval Services Quality Assurance & Improvement



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.F.4 MANAGEMENT AND ADMINISTRATION OF BLOOD PRODUCTS
Implementation Date	01 October 2016
Purpose	To ensure RS Critical Care Paramedics and Doctors manage HMCAS blood products as per established safety and best practice guidelines.
Responsibility	ALL RETRIEVAL SERVICE FLIGHT STAFF

1. Provision of Blood Products:

- 1.1 HMC Blood Bank provides the following blood products for use by HMCAS Retrieval Service:
 - 1.1.1 Two (2) x packs of three (3) units of 'O' Positive Red Blood Cells in an appropriate container.

2. Storage and Cold Chain Integrity:

- 2.1 The container will be capable of maintaining blood products at the required temperature for a period of up to 12 hours.
- 2.2 Staff must maintain cold chain integrity through sound management and monitoring in accordance with [SOP 5.4.G.3](#).

3. Expansion of Provision:

- 3.1 HMC Blood Bank has the means to scale up the number of packs provided to a maximum of 10 packs upon request.
- 3.2 Other blood products can be provided on a case-by-case basis, including but not limited to 'O' Negative blood and Fresh Frozen Plasma.
- 3.3 Requests for additional packs or provision of non-standard products may only be made by the senior clinician in charge of the case.

4. Procedure:

- 4.1 Requests are made as soon as possible after the requirement has been identified.
- 4.2 The requester must provide a clear statement of the blood products required and a pick-up time.
- 4.3 Persons collecting the blood products must be in possession of Corporate ID.
- 4.4 Whilst it is acknowledged that patient details may be scant or non-existent, all documentation must be completed as fully as possible.
- 4.5 At sign out, the collecting person must confirm that the content of the container is correct.
- 4.6 Blood products are to be handled appropriately including protection from heat and direct sunlight.
- 4.7 The container must not be opened until such time as the blood products are to be transfused.
- 4.8 The decision to transfuse rests with the senior clinician in charge of the case.
- 4.9 Prior to transfusion, the patient's serum haemoglobin must be checked using the iStat analyser.
- 4.10 Prior to transfusion, a 3 ml 'type and screen'/'group and save' specimen must be obtained from the patient and stored in a purple top specimen tube.
- 4.11 The specimen must be labelled with three of the following means of identification (name, HMC number, Date of Birth, QID number).

- 4.12 Blood may only be transfused using a standard giving set incorporating a 170 – 200 micron filter.
- 4.13 Transfusion must be commenced within 12 hours of the container packing time.
- 4.14 Units must be transfused within 4 hours of removing the blood unit from the container.
- 4.15 Details of the transfusion must be recorded as per policy in the Patient Care Report form and on other clinical records as required.
- 4.16 Adverse transfusion reactions are to be documented in the Patient Care Report form and reported to the on-call Transfusion Physician on return. AERS Process with documentation
- 4.17 The patient's condition must be monitored, with observations recorded every 10 minutes from commencement of the transfusion until 30 minutes post transfusion.
- 4.18 Unused units, the container and specimens are to be returned to the Blood Bank at completion of the mission.
- 4.19 Details of the transfusion must be entered into the registry of blood products transfused during aeromedical missions.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Scheduled Ambulance Service (SAS) and Retrieval Services (RS) Operations
Procedure	5.4.F.6 PATIENT HANDOVER & ENDORSEMENT
Implementation Date	01 October 2016
Purpose	To facilitate accurate transfer of information and safe and effective transition of care within HMCAS and to healthcare facilities.
Responsibility	ALL AMBULANCE SERVICE STAFF & MEDICAL DOCTORS

- Handover occurs every time the care for a patient is transferred from one healthcare provider to another, either within the Ambulance Service or to another healthcare facility (HCF). Every patient must be formally handed over and received through verbal and/or written communication.
- For higher acuity cases, both the CCP/AP, as team leader, and the Doctor, as Clinical Leader, should receive report from the sending facility staff.
- For higher acuity cases, both the CCP/AP and the Doctor should take part in giving the handover to the receiving facility. Be sure to include the receiving doctor as well as the receiving RN so that no patient information is missed, and patient care is not compromised.
- Use the following mnemonic to guide verbal handover:

I	Identify the patients age, gender, and time of onset of complaint.
M	Describe the Mechanism of Injury or current Medical Complaint .
I	List the Injuries sustained (trauma patients) or information ab.
S	List the Signs and Symptoms .
T	List the Treatment given and response thereto.
A	List any Allergies the patient has.
M	List any Medications the patient is taking.
B	Summarize the patient's Background , i.e. Past Medical History.
O	Describe any Other issues , e.g. social situation, organ donation.

- On completion of handover, ensure that the receiving healthcare provider signs the Patient Care Report.
- Pre- arrival notification for seriously ill or injured patients:
 - Because we are taking part in stable inter-facility transfers it is unnecessary to contact the Clinical Desk prior to arrival at the receiving facility.
 - The AP driving the ambulance should remain in contact with the control room as per HMCAS [SOP 4.5.C](#).
 - If the patient's condition deteriorates while in transit Clinical Desk should be contacted so that the receiving facility can be notified accordingly.
- Upon arrival at the HCF, the HMCAS treating clinician shall do the following:
 - Avoid using abbreviated words during the handover.
 - Identify and verbally confirm which receiving HCF doctor or nurse will accept responsibility for the continued care of the patient.

- 6.3 Clearly identify self as the treating clinician.
- 6.4 Introduce the patient's family/friends/colleagues and commence with the verbal handover of the patient.
- 6.5 Use the mnemonic "IMIST-AMBO" to guide the patient handover.
- 6.6 Assist the HCF doctor/nurse receiving the patient within clinical scope of practice.
- 6.7 Ensure efficient transfer of the patient to alternate monitoring and treatment devices, minimizing interruption of the clinical treatment being administered to the patient.
- 6.8 The HMCAS treating clinician should briefly re-evaluate the patient's clinical presentation during the handover. If the patient's clinical presentation or parameters differ significantly from the initial handover then this could indicate a pre-morbid state and should be highlighted to the receiving doctor/nurse and documented accordingly in the "Treatment" section of the PCR.
- 6.9 Once the receiving HCF doctor or nurse has indicated that they are satisfied with the handover, ensure that they sign the PCR to acknowledge receipt and responsibility for the patient. If the HCF doctor or nurse's name and HMC Staff number is illegible then request clarification.
- 6.10 Remove HMCAS equipment that is not needed and notify the NCC of unit availability accordingly.

SOP 5.4.G: Retrieval Service Equipment



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Schedule Ambulance Service (SAS) and Retrieval Services (RS) Operations
Procedure	5.4.G.1 USE OF THE PAR Aid STRETCHER
Implementation Date	01 October 2016
Purpose	To ensure safe and proper operation of the ParAid Stretcher.
Responsibility	ALL AMBULANCE SERVICE PERSONNEL

1. The ParAid stretcher must only be used on the new WAS ambulances. Together, the ambulances and the ParAid stretchers are used to transport NICU, PICU and ICU SHAARP transfers.
2. **At the shift start, ensure that the ParAid stretcher is checked and ready for service:**
 - 2.1 Assess the ParAid and equipment for soiling.
 - 2.2 Where there is soiling from body-substances (*pooling or large splashes of body fluids on non-porous equipment or vehicle surfaces and any body fluids on porous equipment or vehicle surfaces*), clean the area following ambulance protocol ([SOP 2.3](#)).
 - 2.3 Check all equipment (e.g. suction, infusion drivers, etc.) that are attached to stretcher.
 - 2.4 Check that oxygen cylinders are full or within acceptable range.
3. **Loading patients onto ParAid Stretcher:**
 - 3.1 The ParAid stretcher should be placed parallel to the hospital stretcher.
 - 3.2 Once all IV lines, ETT, Foley, etc. are secured, use a minimum of 4 people (5 people if patient has ETT) to slide patient onto the ParAid Stretcher.
 - 3.3 Patient should be placed in a position of comfort, covered with blanket or blue sheet, and secured using seatbelts (e.g. chest, waist, and legs). Intubated patients must be temporarily disconnected from the ventilator to allow the seatbelt to be secured over the patient's chest.
4. **Securing Equipment to stretcher:**
 - 4.1 All electro medical devices must be secured to the stretcher prior to moving the patient.
 - 4.1.1 Space Stations are to be secured to the vertical poles under the ParAid. Confirm that individual IV drivers and Syringe Drivers are locked into Space Stations. Ensure that Space Stations are plugged into power source.
 - 4.1.2 Spare Monitor is to be secured in place and plugged into power source.
 - 4.1.3 Ventilator should be mounted at the head of the stretcher with oxygen tubing running on the outside of the stretcher to the oxygen cylinders.
 - 4.1.4 Primary monitor will be hung on the side of the stretcher until it is inside the ambulance where it will be secured in the monitor cabinet.
 - 4.1.5 Suction unit is to be secured with straps.
5. **Loading the ParAid into the ambulance:**
 - 5.1 Stretcher must be loaded onto the lift head first.
 - 5.2 **All 4 brakes need to be locked prior to moving the lift up or down.**
 - 5.3 When the stretcher is locked in place inside the ambulance, main ParAid power must be plugged into ambulance wall outlet.
 - 5.4 When the stretcher is locked into place inside the ambulance, portable oxygen must be switched to the main ambulance oxygen.

6. Perform simple decontamination after every patient encounter:
 - 6.1 Assess equipment and work surfaces for significant visible body-substances soiling; where found, exchange the equipment for reserve equipment.
 - 6.2 Where there is no significant visible soiling, perform simple decontamination on all working surfaces and non-critical equipment (i.e. *equipment that does not penetrate the skin and is not used on broken skin or mucous membranes*), including:
 - 6.2.1 Monitor-defibrillator and attachments
 - 6.2.2 Ventilator, Space Stations
 - 6.2.3 Immobilization equipment
 - 6.2.4 Stretcher and cabinet handles

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.G.2 SECURING OF EQUIPMENT DURING TRANSFER
Implementation Date	01 October 2016
Purpose	To provide guidance on direction on the carriage and stowage of equipment during patient transfers.
Responsibility	RETRIEVAL SERVICE CRITICAL CARE PARAMEDICS & TEAM LEADERS

1. Application:

- 1.1 This SOP applies to the securing of electro-medical equipment on all RS platforms to maximize patient and crew safety.

2. General Principles:

- 2.1 All equipment required for the retrieval is to be secured for transit using appropriate restraints and mounting brackets.
- 2.2 If mounting brackets are not available, they must be obtained from Operations Production.
- 2.3 Oxygen cylinders must be secured in the space provided on respective platforms prior to movement.
- 2.4 If the transfer is being conducted using a Stryker stretcher, oxygen cylinders must be secured to the stretcher to avoid being dropped.

3. Infusion Pumps and Syringe Drivers:

- 3.1 During transport or movement, B-Braun infusion pumps must either be housed in the charging unit and secured to a central pole on the stretcher using the standard bracket or secured safely in the space provided within the stretcher module.
- 3.2 When not in use, B-Braun infusion pumps must be housed in the charging unit.

4. Suction Units:

- 4.1 Suction units are to be secured in the space provided within the stretcher or mounted to a bracket on the ambulance during transportation.

5. Monitors and Defibrillators:

- 5.1 During transport or movement, monitors and defibrillators must be secured to an appropriate bracket on the stretcher or securely attached to the mounting device available in the stretcher module.
- 5.2 When in use on a platform where a stretcher is not needed, the monitor should be secured safely to a fixed object with restraints or placed safely in an overhead compartment.

6. Ventilators:

- 6.1 If used during patient transport, ventilators are to be mounted on the appropriate retaining bracket of the stretcher being used.
- 6.2 Prior to movement of transport, staff must ensure that the ventilator is securely fitted to the bracket.
- 6.3 Where no bracket is available, the ventilator must be appropriately secured to a fixed object in a manner that will prevent unwanted movement and associated risk to the patient.

7. Stowage:

- 7.1 Medical equipment, packs and bags must be stowed safely during transportation.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.G.3 COLD CHAIN MEDICATION USE AND SIGN OUT BY RS CRITICAL CARE PARAMEDICS & DOCTORS
Implementation Date	04 March 2020
Purpose	To ensure RS Critical Care Paramedics and Doctors manage HMCAS cold chain medications as per established safety and best practice guidelines.
Responsibility	RETRIEVAL SERVICES CRITICAL CARE PARAMEDICS & DOCTORS

1. General Processes pertaining to 'Cold Chain' medications:
 - 1.1 Critical Care Paramedics and Doctors working for Retrieval Services should follow an established 'cold chain' policy during the use and transport of medications requiring controlled refrigeration. This policy includes how to withdraw medications from the HMCAS medication refrigerator, packaging, and safe transport of 'cold chain' medications, re-stocking and returning medications after use, and issues requiring escalation or reporting (such as the breach of temperature control range). (Ref. CL 6052).
2. Cold Chain Defined:
 - 2.1 The 'Cold Chain' is a system of transporting and storing vaccines, medications, and reagents within the safe temperature range of 2 to 8 degrees centigrade. The cold chain begins when the item is manufactured, continues through to the storage location, and ends with the use of the item.
 - 2.2 All medications subject to refrigeration should be transported and stored via a 'cold chain' system whenever removed from HMCAS main pharmacy.
3. Management of 'Cold Chain' items:
 - 3.1 Storage of Cold Chain Items:
 - 3.1.1 All cold chain items should be stored in a refrigerator that is used exclusively for medications.
 - 3.1.2 This refrigerator should be frost-free, regularly inspected and preferably a purpose-built medication refrigerator.
 - 3.1.3 Cold chain items should NOT be stored in the inside door shelving as the temperature in this compartment become variable when the door is opened frequently.
 - 3.1.4 Overstocking of the refrigerator should be avoided, as there should be cool air circulating around the stock in the refrigerator.
 - 3.2 Temperature inside refrigerator:
 - 3.2.1 The refrigerator should be kept at between 2 and 8 degrees centigrade.
 - 3.2.2 An electronic, recording thermometer should be placed in every fridge.
 - 3.2.3 The temperature records are to be recorded routinely as part of the continuous quality improvement program of HMCAS Pharmacy.
 - 3.3 Ambient Temperature:
 - 3.3.1 The ambient temperature of the room where medications are stored should be regulated to between 15 and 25 degrees centigrade and must be monitored and manually recorded each day in a temperature-monitoring sheet.

4. Transportation of 'Cold Chain' items:

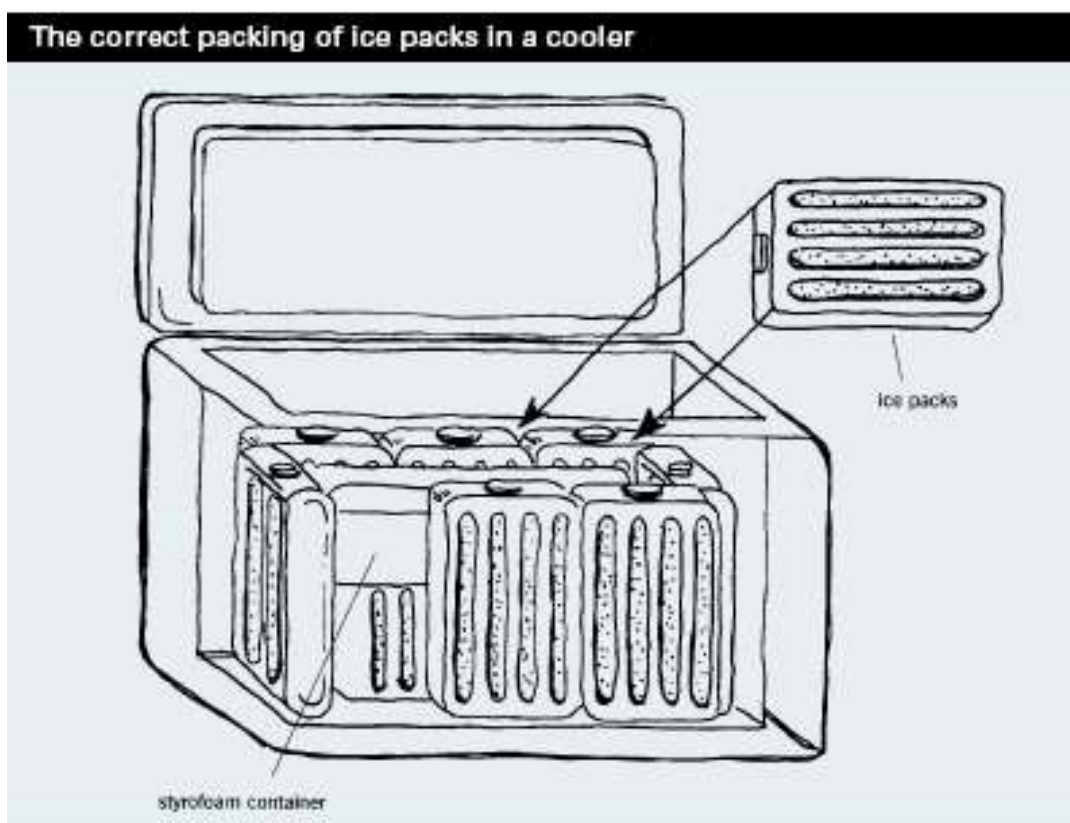
4.1 The following items are required to ensure the integrity of a cold chain system:

- 4.1.1 A refrigerator for storing medications
- 4.1.2 Cold chain monitors
- 4.1.3 A digital, electronic or mercury minimum/maximum thermometer and chart for daily temperature reading
- 4.1.4 Cold boxes for transporting medications
- 4.1.5 Ice packs to keep medications cool
- 4.1.6 Material to separate ice packs from the medications (e.g. shredded paper, cardboard, bubble wrap or Styrofoam)
- 4.1.7 Colorimetric temperature control strips
- 4.1.8 'Cold Chain' Register

4.2 All 'cold chain' items should be carefully packed and transported in insulated containers that contain ice packs. The ice packs should never come in direct contact with the 'cold chain' items as they may freeze the item. They should always have some insulation material between the item being transported and the ice pack.

4.3 The insulated container (cool box) should be:

- 4.3.1 Large enough to store items and icepacks during transport;
- 4.3.2 External surfaces need to be robust and durable;
- 4.3.3 Insulation thickness is at a minimum of 30 mm to as much as 80 mm;
- 4.3.4 Lids should be tight fitting;
- 4.3.5 Have an externally readable thermometer/temperature indicator.



- 4.4 When hand carrying 'cold chain' items over long distances or for prolonged periods of time, there is a risk of fluctuation in temperature and degradation of the integrity of the cold chain items:
- 4.4.1 It is essential that the coolers remain sealed during transport and only opened briefly when the medications are required for use; in addition, externally readable digital thermometers or colorimetric temperature indicators should be visible to the operator at all times.
 - 4.4.2 The cooler should be kept out of direct exposure of extreme heat or cold sources, such as sunlight or heat transferrable surfaces.
 - 4.4.3 The 'cold chain' items should be stored in a refrigerator wherever possible during transit (such as vehicle medication fridges, aircraft refrigerators, hotel refrigerators, etc.) and maintain constant temperature control of between 2 to 8 degrees centigrade throughout the mission. Over prolonged missions, attempts should be made to refresh the cold packs inside the cooler or refrigerate the entire cooler.
 - 4.4.4 In addition to a thermometer, temperature control strips should be placed into every cold chain package prior to commencement of a mission. When the items are inspected prior to use/or arrives at the storage location, the temperature control strip is read to ensure the temperature has remained consistent during the transport. Records should be kept for each mission.
 - 4.4.5 In the event of 'cold chain items' breaching the temperature range for periods greater than 60 minutes, the medications must be deemed unsafe (unless indicated otherwise by specific medication or manufacturer guidelines) and HMCAS pharmacy staff should be notified.

5. Procedure to withdraw 'cold chain' medications:

- 5.1 Retrieval Service Critical Care Paramedic or Doctor identify the cold chain medications and quantity required for the mission or withdraw a "cold chain prepack" and inform the HMCAS pharmacy staff.
- 5.2 The cooler, ice packs and insulation material are prepared and inspected by Retrieval Service staff.
- 5.3 For the cold item that is removed from fridge, the pharmacy staff stick a new label on the medication with a new expiry date determined by international guidelines.
- 5.4 The pharmacy/Retrieval Services staff remove the items from the fridge and place directly into the transport container.
- 5.5 The items are appropriately spaced, insulated from direct contact with the ice bricks and a colorimetric temperature strip is inserted into the cooler prior to ensuring the cooler is properly sealed.
- 5.6 The externally mounted thermometer is inspected for proper function and accuracy.
- 5.7 'Cold Chain' items register is completed and counter signed by both staff.

6. Procedure to return 'cold chain' medications:

- 6.1 Retrieval Service staff informs the pharmacy staff that they have 'cold chain' items to return.
- 6.2 Retrieval Service staff advise the pharmacy staff on any breaches in the cold chain, medication use or 'incidents' that could compromise the items integrity.
- 6.3 Pharmacy staff inspects the externally mounted thermometer and note the duration the items have been outside of a pharmacy controlled environment (such as occasions where medications accompany a mission for several days).

- 6.4 The pharmacy staff opens the cooler and notes any medication use, breakage or “incidents” that may compromise item integrity (e.g. frozen amps, improper insulation, etc.).
- 6.5 Pharmacy staff inspects the colorimetric temperature strip inside the cooler to ensure temperature integrity was maintained throughout transit.
- 6.6 Medications are returned to the fridge or discarded as necessary.
- 6.7 ‘Cold Chain’ items register is completed and counter signed by both staff. A requisition form is completed by the Retrieval Service staff for any items used.
- 6.8 Ice Bricks are returned to the freezer and cooler is placed in storage.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Retrieval Services (RS) Operations
Procedure	5.4.G.4 CONTROLLED MEDICINES MANAGEMENT BY RS CRITICAL CARE PARAMEDICS & MEDICAL DOCTORS
Implementation Date	04 March 2020
Purpose	To ensure Critical Care Paramedics and Medical Doctors manage HMCAS approved controlled and non-controlled medications appropriately.
Responsibility	RETRIEVAL SERVICES CRITICAL CARE PARAMEDICS & MEDICAL DOCTORS

1. General Processes pertaining to Medications:

- 1.1 Critical Care Paramedics and Medical Doctors working for Retrieval Services should follow [SOP 4.3.C](#) and [SOP 4.3 B](#) in regard to all controlled and non-controlled medication use while in the country of Qatar.
- 1.2 This includes how to withdraw medications from the Controlled Medication Storage Case (CMSC), re-stocking of medications, returning medications after use, returning residual controlled medications and issues requiring escalation and reporting.
- 1.3 In line with Ministry of Public Health Policy, any used controlled medication ampoules or vials must be retained and returned to pharmacy as evidence. Where this is not possible (e.g. if the ampoule is inadvertently disposed of and cannot be recovered), an OVA is to be submitted by the CCP on their return.
- 1.4 Under no circumstances should staff expose themselves to the risk of a needle stick injury by attempting to recover used ampoules from a sharps box.

2. Management of Controlled medications outside of Qatar: Regional:

- 2.1 It is understood that controlled medications will need to be taken outside of Qatar due to various mission requirements. This includes but is not limited to; regional retrievals in both fixed and rotor-wing configurations as well as long distance fixed-wing retrievals to Europe, the UK, and the US.
- 2.2 Where fixed or rotor wing missions consist of an “out and back” (e.g. KSA retrieval) the controlled medication policy is identical to that of standard ground operations. See [SOP 4.3.C](#).

3. Management of Controlled medications outside of Qatar: International:

- 3.1 Controlled medications will need to be taken outside of Qatar due to various mission requirements. This includes but is not limited to;
 - 3.1.1 Regional retrievals in both fixed and rotor-wing configurations as well as long distance fixed wing retrievals to Europe, the UK, and the US.
 - 3.1.2 Where fixed-wing missions will be over an extended time (e.g. trips to the UK, US, or Europe) that the medical team will maintain their medications outside of the normal 12-hours shift window.
- 3.2 The provider should ensure that he/she has removed the “Narcotics Letter” from the CMSC with their name on it, ensured that it is in date, and brought with them during the mission.
- 3.3 Based on patient requirement non-controlled medications may be left aboard the Amiri-10 aircraft or other HMCAS owned aircraft (this does not include any commercial flight platform). Non-controlled medications should be secured with either a red pull tab or lock.
- 3.4 Controlled medications should ALWAYS travel with the provider who signed them out of the CMSC and NEVER be left on the aircraft.

- 3.5 Once in designated accommodation, the narcotics box should be locked and placed out of site, either in the provider's suitcase or in the room safe.

SOP 6: Events & Emergency Planning



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	6.1 STAFFING OF EVENTS AND SEALINE CAMPING SEASON
Implementation Date	26 March 2019
Purpose	To outline which licensed clinical staff are eligible to attend high, medium, and low risk events.
Responsibility	ALL HMC AMBULANCE SERVICE GROUP LICENSED AMBULANCE PARAMEDICS OR CRITICAL CARE PARAMEDICS

1. Introduction:

- 1.1 HMCAS is required to ensure that optimal resources are provided to support events and that standards of care are maintained. Therefore, all events will be classified as high, medium, or low risk.
- 1.2 HMCAS will identify which staff may or may not attend an event based on the risk posed by the event to the general public (e.g. high-risk event such as Sealine) and/or risk to reputation of HMCAS (e.g. televised event).
 - 1.2.1 The choice of which staff may work at an event will be reflective of their clinical privileges and the level of risk of critical injury/illness that the event can reasonable be judged to pose. In addition, the risk to the reputation of HMCAS is also an independent factor in identifying what staff may or may not attend an event in a patient facing capacity.
- 1.3 HMCAS scheduling department is responsible for ensuring an appropriate skill mix on events and that staff, with reduced privileges, are not deployed into roles requiring full privileges of either an ambulance paramedic or critical care paramedic without authorization from an Executive Director and/or the Head of Professions or his/her designed deputy.

2. Risk Stratification of Event:

- 2.1 All events are graded either as being either high, medium, or low risk.
- 2.2 Frequently occurring types of events are listed in Table 1.
- 2.3 Events not previously covered by HMCAS will be reviewed by the Events and Emergency Planning team and graded according to the prescribed risk assessment.
- 2.4 Events will be graded in accordance with the following principles:
 - 2.4.1 Risk of injury/illness to public:
 - 2.4.1.1 Likely requirement for care at or above the level of an ambulance paramedic.
 - 2.4.2 Risk to reputation of HMCAS.
- 2.5 Only licensed critical care paramedics who hold **full clinical privileges (level 4 privileges)**, as a critical care paramedic, may undertake any duties as a critical care paramedic.
 - 2.5.1 The exception is where these staff are deployed in an operational managerial position or where these staff are deployed in an operational supportive but 'non-patient facing role'.
- 2.6 Ambulance paramedics who are allocated **clinical privileges as level 2 privileges or level 1 privileges**, are not permitted to cover events designated as high risk (see Table 1).
 - 2.6.1 The exception is where these staff are deployed in an operational managerial position or where these staff are being deployed in an operational supportive 'non-patient facing' role (e.g. communications officer).

- 2.6.2 Or in a foot patrol first responder category.
- 2.7 With the exception of events deemed to be high risk, where both ambulance crew must hold privileges as an ambulance paramedic (**level 3 privileges**), the minimum staffing of an HMCAS ambulances is as follows:
- 2.7.1 One licensed ambulance paramedic with ambulance paramedic **level 3 privileges** (this clinician is deemed to be the senior clinician regardless of the rank of his colleague **IF** his colleague does not hold **level 3 privileges**. The second ambulance paramedic must hold, as a minimum, basic ambulance paramedic privileges (**level 2 privileges**).
- 2.7.2 HMCAS clinicians and/or voluntary staff holding **level 1 privileges** are not permitted to work on a HMCAS ambulance as part of deployment staff at an event.
- 2.8 DHP licensed staff on light duties **ARE NOT** eligible for any events in any capacity without permission of an Executive Director and/or the Head of Professions or his/her designated deputy.
- 2.9 HMCAS clinicians with privileges as a first responder may only be deployed to medium and low risk events in the following roles/capacity:
- 2.9.1 Foot patrol
- 2.9.2 Manning static first aid unit coverage

3. Privileging:

- 3.1 Clinical privileges are outlined in HMCAS policy.

4. Table 1: minimum privileges for staff deployed to event:

Event	Risk	Comment
High risk events		
Sealine	High	Licensed ambulance paramedics employed fulltime within National Command Centre, Health Care Coordination, any support service, mobile doctor services, scheduling, production supervisors and training are NOT ELIGIBLE for high risk events unless they hold FULL PRIVILEGES (level 3 or 4 privileges) , as an ambulance paramedic OR comply with 2.5.1 or 2.6.1. Where there is an agreed requirement for staff to provide crowd-based foot patrol and/or manning of static first aid units then paramedics holding privileges as a basic paramedic or first responder (level 1 or 2 privileges), may be deployed to cover this role.
Events involving any motor vehicle racing (e.g. MotoGP or desert rally).	High	Licensed ambulance paramedics employed fulltime within National Command Centre, Health Care Coordination, any support service, mobile doctor services, scheduling, production supervisors and training are NOT ELIGIBLE for high risk events unless they hold FULL PRIVILEGES (level 3 or 4 privileges) , as an ambulance paramedic OR comply with 2.5.1 or 2.6.1. Where there is an agreed requirement for staff to provide crowd-based foot patrol and/or manning of static first aid units then paramedics holding privileges as a basic paramedic or first responder (level 1 or 2 privileges), may be deployed to cover this role.
Events involving horse or camel racing.	High	Licensed ambulance paramedics employed fulltime within National Command Centre, Health Care Coordination, any support service, mobile doctor services, scheduling, production supervisors and training are NOT ELIGIBLE for high risk events unless they hold FULL PRIVILEGES (level 3 or 4 privileges) , as an ambulance paramedic OR comply with 2.5.1 or 2.6.1. Where there is an agreed requirement for staff to provide crowd-based foot patrol and/or manning of static first aid units then paramedics holding privileges as a basic paramedic or first responder (level 1 or 2 privileges), may be deployed to cover this role.

Any contact sport such as boxing or martial arts.	High	Licensed ambulance paramedics employed fulltime within National Command Centre, Health Care Coordination, any support service, mobile doctor services, scheduling, production supervisors and training are NOT ELIGIBLE for high risk events unless they hold FULL PRIVILEGES (level 3 or 4 privileges), as an ambulance paramedic OR comply with 2.5.1 or 2.6.1. Where there is an agreed requirement for staff to provide crowd-based foot patrol and/or manning of static first aid units then paramedics holding privileges as a basic paramedic or first responder (level 1 or 2 privileges), may be deployed to cover this role.
Work as part of the field of play at sporting event.	High	Licensed ambulance paramedics employed fulltime within National Command Centre, Health Care Coordination, any support service, mobile doctor services, scheduling, production supervisors and training are NOT ELIGIBLE for high risk events unless they hold FULL PRIVILEGES (level 3 or 4 privileges), as an ambulance paramedic OR comply with 2.5.1 or 2.6.1.
Medium risk events		
Gymnastics or events involving diving.	Medium	ALL HMCAS ambulance MUST be staffed by (as a minimum) one fully privileged Ambulance Paramedic (level 3 or 4 privileges) supported by, as a minimum, an ambulance paramedic with privileges as a Basic Paramedic (level 2 privileges). Ideally (although this is not mandatory), the ambulance paramedic holding privileges at the Basic Paramedic level (level 2 privileges should be allocated as the ambulance driver as long as they are qualified as a P1 driver. Basic ambulance paramedics and first responders (level 1 or 2 privileges may be deployed as foot patrol and/or in static first aid units.
National day	Medium	ALL HMCAS ambulance MUST be staffed by (as a minimum) one fully privileged Ambulance Paramedic (level 3 or 4 privileges) supported by, as a minimum, an ambulance paramedic with privileges as a Basic Paramedic (level 2 privileges). Ideally (although this is not mandatory), the ambulance paramedic holding privileges at the Basic Paramedic level (level 2 privileges should be allocated as the ambulance driver as long as they are qualified as a P1 driver. Basic ambulance paramedics and first responders (level 1 or 2 privileges may be deployed as foot patrol and/or in static first aid units.
Low risk events		
Sporting events with spectators (but excluding field of play).	Medium	ALL HMCAS ambulance MUST be staffed by (as a minimum) one fully privileged Ambulance Paramedic (level 3 or 4 privileges) supported by, as a minimum, an ambulance paramedic with privileges as a Basic Paramedic (level 2 privileges). Ideally (although this is not mandatory), the ambulance paramedic holding privileges at the Basic Paramedic level (level 2 privileges should be allocated as the ambulance driver as long as they are qualified as a P1 driver. Basic ambulance paramedics and first responders (level 1 or 2 privileges) may be deployed as foot patrol and/or in static first aid units.
Ramadan and Eid celebrations.	Low	Both ambulance paramedics must, as a minimum, be privileged at the Basic Paramedic level (level 2 privileges) although ideally one will be fully privileged as an ambulance paramedic (level 3 privileges) one of the crew must be qualified as a P1 driver. Basic ambulance paramedics and first responders may be deployed as foot patrol and/or in static first aid units.
Following a risk assessment, via the Office of the Assistant Executive Director Events and Major Incident planning to the Head of Professions/Group Medical Director, the requirements contained within this Standing Operations Procedure may be changed.		

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	6.2 ADVERSE WEATHER PREPAREDNESS, RESPONSE AND RECOVERY
Implementation Date	9 July 2019
Purpose	Establish a coordinated response during adverse weather to ensure the health and safety of Qatar and its population.
Responsibility	ALL HMCAS STAFF AND DIRECTORATES

1. Purpose:

- 1.1 The intent of this procedure is to establish a coordinated response during severe weather such as dust and rain to ensure the health and safety of Qatar and its population. This SOP will ensure that all role players within the HMCAS will provide adequate resources, manage the resources, and ensure that the community has sufficient medical coverage.
- 1.2 Establish internal proactive measures to minimize disruption to key AS services due to severe or adverse weather events based on the following objectives: (Align to ISO 31000:2009; 22301:2013).
 - 1.2.1 Reduce the likelihood of a disruption
 - 1.2.2 Shorten the period of disruption
 - 1.2.3 Limit the impact of a disruption
- 1.3 Establish and outline additional measures by the ambulance service to prepare for, respond to, and recover from severe or extreme weather events. These measures are intended be in conjunction with established procedures outlined in the HMCAS SOP manual, the Ambulance Service Operations Plan (AS-9032), and the Ambulance Service Major Incident Response Guide.

2. Definitions:

- 2.1 **Severe weather:** Dangerous meteorological or hydro-meteorological phenomenon, of varying duration, with risk of causing major damage, serious social disruption, and loss of human life. E.g. Heavy Rainfall and Flooding, High Winds, Tornado, etc.
- 2.2 **Extreme Weather:** Unexpected, unusual, unpredictable, unseasonal weather; weather at the extremes of the historical distribution * (outside the range that has been seen in the past*) E.g. Heat Wave, Cold Wave, etc.
- 2.3 **Lead Time:** The time available for preparation between an announcement or decision and the event it pertains to.

3. Procedures:

- 3.1 Specific Measures and activities are outlined in this document are divided into three activity phases along with the associated event trigger, triggering events must be sufficient enough as to present a threat to life and/or hampering or disrupting regular AS service delivery.
- 3.2 Preparedness:
 - 3.2.1 Event and emergency planning:
 - 3.2.1.1 Ensure the regular conduct and documentation of Hazard Vulnerability Assessment (HVA), Threat Hazard Identification and Risk Assessments (THIRA), and/or Business Impact Analysis (BIA) activities as part of the preparedness planning.
 - 3.2.1.2 Arrange for additional resources in anticipation of the event. ([SOP 4.10.B](#)).

- 3.2.1.3 Prepare or update the severe/extreme weather operational plan.
- 3.2.1.4 Facilitate the preparation of business continuity plans from all HMCAS directorates.

Activity Phase:	Trigger:
I. Preparedness	a. Anticipated seasonal climate and weather changes. (e.g. winter / summer). b. Severe or Extreme weather warning or advisory received. (e.g. 12-72 hours, meteorological prediction, weather forecast, etc.) c. PRIOR warning events
II. Response	a. Daily operations or response operations ongoing sudden severe weather or extreme weather event encountered. b. No prior warning event (e.g. sudden rain and flooding, fog conditions, increasing heat and humidity etc.) c. NO warning events
III. Recovery	a. Weather conditions significantly improved; weather advisory signals normal weather conditions predicted. b. Operations units stood down, normalizing conditions encountered by crews. All Clear.

4. Notification Process:

- 4.1 The On-duty AS-LifeFlight coordinator assigned to NCC will regularly monitor the weather as part of his regularly assigned duties.
- 4.2 Prevailing weather conditions will be updated daily to Oscar-NCC by the LifeFlight coordinator/.
- 4.3 NCC Oscar to assess the need to issue a warning or initiate escalation procedures for a potential weather event. The need for a warning or initiating escalation should be assessed on two major considerations:
- 4.3.1 The likelihood of an event
- 4.3.2 Proceed with planned escalation processes (Policy 9032)
- 4.4 Oscar NCC should notify Silver Command
- 4.5 Silver Command notifies Gold Command and a decision to be made to:
- 4.5.1 Notify On Call personnel
- 4.5.2 Proceed with planned escalation processes (Policy 9032)

- 4.6 Once a decision has been made to issue a warning, Oscar NCC will notify via SMS/ fact 24 all relevant ambulance service directorates. SMS notifications will have a prefix followed by the advisory statement. A statement is either issued as a follow-up message to a warning, watch, or emergency, that may update, extend, or cancel the message it is following up or a notification of significant weather for which no type of advisory, watch, or warning exists. *Ex. SMS message: (Prefix)_**Emergency:** (Statement)_ "Flooding/ dust along xxxx areas, with expected further rain for the next xxxx hours, staff advised....."*
- 4.7 These statements are used in conjunction with specific weather phenomena to convey different levels of risk. In order of increasing risk, these statements are:
- 4.7.1 **Outlook:** A Hazardous Weather Outlook is issued daily to indicate that a hazardous weather or hydrologic event may occur in the next several days. The outlook will include information about potential severe thunderstorms, heavy rain or flooding, winter weather, extremes of heat or cold, etc., that may develop over the next seven days with an emphasis on the first 24 hours of the forecast.
- 4.7.2 **Advisory:** An advisory is issued when a hazardous weather or hydrologic event is occurring, imminent, or likely. Advisories are for "less serious" conditions than warnings that may cause significant inconvenience, and if caution is not exercised could lead to situations that may threaten life or property.
- 4.7.3 **Emergency:** An Emergency is issued when an event that by itself cannot pose a threat to life or property but may indirectly cause other events to happen that may pose a threat to life or property.
- 4.7.4 **Watch:** A watch is used when the risk of a hazardous weather or hydrologic event has increased significantly, but its occurrence, location, or timing is still uncertain. It is intended to provide enough lead time so those who need to set their plans in motion can do so. A watch means that hazardous weather is possible.
- 4.7.5 **Warning:** A warning is issued when a hazardous weather or hydrologic event is occurring, imminent, or likely. A warning statement issued means weather conditions pose a threat to life or property.

5. Silver Command Actions:

- 5.1 Coordinate with NCC/NHICC to stop all hospital IFTs to allow more resources to service the community in the event of disaster from the adverse weather.
- 5.2 Coordinate with NHICC to check with ED HGH Nursing Director if still there is Bed Crisis at ED.
- 5.3 Arrange for Extra Stretcher from HMC AS to be used for patient at ED if needed.
- 5.4 Coordinate with HMCAS Gold On call the Possibility of Divert and Wait for Confirmation.
- 5.5 Activate Silver Production to provide Extra Supply and consumable at ED HGH.
- 5.6 Monitor the turnaround time for the units to provide more resources to service the call demand.

6. Silver Production Actions:

- 6.1 On call Team will coordinate with support services, VCS and schedule to arrange extra staffing, Vehicles and stock.
- 6.2 Extra units will be deployed as per the demand where areas of high call volumes are received, also affected areas where medical coverage need to be placed.
- 6.3 Oscar North Should Respond to ED HGH and Update the Silver Command on the bed availability status and the queuing at ED.

7. Events and Emergency Planning:

- 7.1 During adverse weather rain and dust, the Events and Emergency planning will provide a risk assessment.
- 7.2 HMCAS Events and Emergency planning Division have established the MIR Guide that sets out the roles and responsibilities of the command structure.
- 7.3 The Operational Plan for adverse weather must be distributed to the Silver command as a reference which is updated annually.
- 7.4 Events and Emergency Planning division will provide additional resources. [SOP 4.10.B](#).

8. Operations:

- 8.1 Operations on call will report to Silver Command.
- 8.2 Operational managers must reiterate [SOP 2.1](#) - **PERSONAL PROTECTION & SAFETY – Points 1&3**.
- 8.3 All staff must revert to driving and responding to calls safely following [SOP 4.6.C](#) - **EMERGENCY RESPONSE AND NON- EMERGENCY DRIVING ADVERSE WEATHER CONDITIONS**.
- 8.4 All communications must follow the chain of command.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Events & Emergency Planning
Procedure	6.3 DRILL/EVENT PLANNING & MANAGEMENT
Implementation Date	1 June 2019
Purpose	- To centralise all drill and event requests received by HMCAS in order to ensure that they can be planned, resourced and managed accordingly. - To apply a systematic and uniform approach to the planning, assessment and facilitation of all drill and event requests.
Responsibility	ALL HMCAS DEPARTMENTS

1. HMCAS receives drill and event coverage requests from a broad range of community sectors. These include sporting bodies, schools and universities, commercial and retail entities, and industrial organisations.
2. In order to ensure that each request can be assessed accurately, resourced appropriately and scheduled accordingly, it is essential that the Events & Emergency Planning Section are timeously informed of all Drill and Event coverage requests. It is also crucially important that all Drills and Events are uniformly risk assessed according to the accepted framework.
3. The centralization of all requests will also help assure that all Drills and Events can be assessed, planned, staffed and managed from start to finish. In the event that an issue arises with a Drill or Event, it will also help assure that it can be managed rapidly and efficiently in order to preserve optimal service delivery.
 - 3.1 Directed to Events and Emergency Planning (E&EP) Section:
 - 3.1.1 All requests or info regarding upcoming drills/events must be immediately forwarded to the events and emergency planning department by email to asevent@hamad.qa.
 - 3.1.2 In addition, if the event/drill is imminent (will occur in the next 5 days) a phone call to "ON CALL", event and EP department must be made.
 - 3.2 E&EP will inform all relevant HMCAS sections of upcoming events.
 - 3.3 E&EP will contact the facilitator or organiser of the event/drill and provide confirmation of receipt of the request and solicit further information as required.
 - 3.4 Based on the above, HMCAS will review the event request and either accept or refer coverage.
 - 3.4.1 As required, resources will be requested from relevant HMCAS sections to facilitate coverage of the Drill/Event.
 - 3.5 If coverage of the Drill/Event is accepted, the request will be added to the HMCAS E&EP Events Calendar.
 - 3.6 E&EP will undertake a risk assessment based on the output of form AS-EP-F002-V.2 and schedule a site meeting/visit with the Drill/Event organiser as required. Appropriate representation from all relevant HMCAS sections will be requested as required.
 - 3.7 The output of the risk assessment will be communicated to all relevant HMCAS sections including Scheduling so that the event/drill can be quoted accordingly.
 - 3.8 If accepted, E&EP will prepare and complete an Operations Plan (in consultation with all relevant HMCAS sections). Once reviewed and approved it will be communicated to all applicable HMCAS sections.
 - 3.9 In the interim, workforce planning will allocate staff and resources to cover the event.

- 3.10 Coverage of the Drill/Event will be overseen and coordinated by E&EP. Any issues arising during the coverage of the Drill/Event will be reported by on-scene staff to E&EP Officer/ Manager for further action with other HMCAS Sections notified as required. See weekly HMCAS "ON CALL" list for event personnel on call.
- 3.11 Where required, and if warranted, E&EP will liaise with the Drill/Event Organiser to perform a post-event de-brief where involved HMCAS sections/representatives will be invited.
- 3.12 E&EP will compile the post-exercise report, SWOT report, lessons learned, and after-action report as applicable with consultation/feedback from all involved HMCAS sections.
 - 3.12.1 Once completed, reviewed and approved, the report will be shared with all HMCAS sections.

4. The above workflow is depicted below: Responsibilities, Accountabilities, Communication and Informed roles:

HMC AS Event Management Workflow		EP-F012-v12/2019
INTELLIGENCE	All Event Cover Requests / Event Activity Information From Any source for any event is sent to the: And HMC AS Event Email: Asevent@hamad.qa	
	Event Entered into Event Management Tracking System	SCHEDULING
	Event Entered into Event Calendar	
PLANNING	First Contact / Meeting with event Organizer for: o Event Details o Event Risk Assessment and medical resource recommendation *see note 1	EMERGENCY PLANNING
	If required, Resource availability Approval will be sought from: o Operations o Production (If required) o Scheduling	
	Quotation sent to organizer	SCHEDULING
	Staff are assigned to event and staff details are sent to Event and Emergency Planning	
	Staff Accreditation (access pass) Information sent to event organizer	EMERGENCY PLANNING
	Completed Accreditation Cards collected from event organizer and distributed to HMC AS staff	
	Payment for staff collected and distributed to HMC AS Staff	SCHEDULING
	Payment for Ambulances Collected and sent to HMC AS Finance	
	Event Logistics Payment will be completed according to the event coverage model	
	Event Operations Plan developed as required *see note 3	EMERGENCY PLANNING
	Event Operations Plan Confirmed by Ambulance Service Operations	EMERGENCY SERVICES
	Event Operations Plan Confirmed distributed to: o Major Incident on-call list o All other concerned parties as required	EMERGENCY PLANNING
	Operation Plan Briefing for Venue Supervisor and Service Management *Including NCC if dispatchers are involved	

PREPARATION	Resources are Prepared o Ambulances o Equipment o Personnel	PRODUCTION
	Resources are prepositioned on-site prior to event if required	
RESPONSE	Staff and resources arrive on site, complete and on time	EMERGENCY PLANNING
	Liaison with organizer prior to event for briefing	EMERGENCY PLANNING
	Assistance (if required) to remedy staff/production issues during event *See note 4	VENUE SUPERVISOR
	Liaison with organizer during event	VENUE SUPERVISOR
	Management of Event Resources and the AS Response	VENUE SUPERVISOR
RECOVERY	Liaison with organizer after event for de-briefing	EMERGENCY PLANNING
	Staff and Resources return to EMS base	VENUE SUPERVISOR
	Resources returned to Vehicle Control	VENUE SUPERVISOR
	Event Report / Lessons Learned and Action Plan	EMERGENCY PLANNING
Note-1	Risk assessment and Medical Resource Recommendation In instances where medical resources being provided and paid for by the event organizer do not meet the recommendations outlines in the risk assessment, this shall be outlined in the Operational Plan	
Note-2	Free event cover will only be provided after approval from the HMC Managing Directors Office (Email confirmation from the MD Office is required). The Event and Emergency Planning Team will be responsible for liaising with the MD Office to confirm free events	
Note-3	Operational Plans will be created for events as required	
Note-4	In the instance where there are problems with resources availability (ambulances/equipment/personnel) the venue supervisor will contact Production and Scheduling for assistance and inform events and emergency planning on call.	

SOP 7: Training



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Training Department
Procedure	7.1 ORIENTATION OF ALL HMCAS AMBULANCE PARAMEDIC AND CRITICAL CARE PARAMEDIC STAFF
Implementation Date	10 August 2017
Purpose	To ensure that all HMCAS Staff are on-boarded appropriately and receive consistent orientation and training required by HMC and JCI.
Responsibility	ALL HMCAS AMBULANCE PARAMEDIC AND CRITICAL CARE PARAMEDIC STAFF

1. Minimum Required Orientation:

- 1.1 This Standard Operating Procedure (SOP) describes the minimum required orientation and training for all Hamad Medical Corporation Ambulance Service (HMCAS) Ambulance Paramedic (AP) and Critical Care Paramedic (CCP) staff to be completed during the initial induction to meet HMCAS and Joint Commissions International (JCI) requirements.

2. HMC Human Resources (HR) Orientation:

- 2.1 Upon arrival, all new staff will initially report to HR to complete orientation procedures and documentation. Once the staff member has completed all documentation with HR, he/she will then report to the Training Department for Ambulance Service on-boarding and training.

3. HMCAS On-Boarding:

- 3.1 The new staff member will attend the following lectures:
- 3.1.1 Dress Code
 - 3.1.2 Training Department House Rules
 - 3.1.3 Communications
- 3.2 The new staff member will be issued the following documents:
- 3.2.1 Ambulance Service Orientation Book (Soft copy)
 - 3.2.2 HMCAS Standard Operating Procedures (Soft copy)
 - 3.2.3 HMCAS Clinical Practice Guidelines (Soft copy)
 - 3.2.4 HMCAS Clinical Procedures Manual (Soft copy)
 - 3.2.5 HMCAS Ambulance Paramedic Pre-Course Workbook (Hard copy - AP staff only)
 - 3.2.6 HMCAS Critical Care Paramedic Observational Phase Objectives (Soft copy - CCP staff only)
 - 3.2.7 Corporate Orientation Guideline Document (Hard copy)
 - 3.2.8 Corporate Orientation Checklist (Hard copy)
 - 3.2.9 Facility Orientation Checklist (Hard copy)
 - 3.2.10 Departmental Orientation Checklist (Hard copy)
- 3.3 The following training modules will be completed:
- 3.3.1 HMCAS Infection Control (or HMC e-learning module equivalent)
 - 3.3.2 HMCAS Fire Safety (or HMC e-learning module equivalent)
 - 3.3.3 HMC Environmental Safety

- 3.3.4 HMC Major Incident Response
- 3.3.5 HMCAS Quality Improvement and Patient Safety (or HMC e-learning module equivalent)
- 3.3.6 HMCAS Fall Safety
- 3.3.7 Hospitality World of Welcome
- 3.4 The Training Department will assist the staff with the application for his/her HMC username creation.
- 3.5 The Training Department will facilitate the activation of the staff member's HMC ID card to open doors within HMC facilities.
- 3.6 The Training Department will start a hard file for the new staff member to house all records and documentation pertaining to the above orientation/training. Once the staff member is loaded onto the Scheduling Management System (SMS), the relevant documentation will then be uploaded onto their electronic profile.
- 3.7 The Training Department will liaise with the Uniform Section to get the staff member their uniform and operational equipment.
- 3.8 The Training Department will provide direction and support to new staff applying for their temporary and permanent DHP license.

4. Corporate, Facility and Departmental/Job Specific Orientation Checklists:

- 4.1 The Training Department will assist the new AP and CCP staff with completion of:
 - 4.1.1 HMC Corporate Orientation Checklist
 - 4.1.2 Facility Orientation Checklist
 - 4.1.3 Departmental/Job Specific Orientation Checklist

5. Clinical Orientation and Initial Training:

- 5.1 The training department will provide an orientation for all newly hired Critical Care Paramedics based on their role within 999 ground operations reviewing their clinical skills and scope of practice.
- 5.2 The training department will provide an Ambulance Paramedic Course for all newly hired Ambulance Paramedics who require the course to obtain licensure with the Department of Healthcare Professions.
- 5.3 The clinical orientation will include, but is not limited to, the minimum courses offered by the Ambulance Service required for DHP licensure.
 - 5.3.1 Ambulance Service Resuscitation Course
 - 5.3.2 Ambulance Service Trauma Course

ON BOARDING REQUIREMENTS					
Title	Recertification Period	Delivery	JCI Required	DHP Required	HMCAS Required
Corporate Orientation	N/A	Online	☒	☐	☒
Facility Orientation	N/A	By Training Department	☒	☐	☒
Departmental Orientation	N/A	By Training Department for ground 999.	☒	☐	☒
AP Course	N/A	By Training Department	☐	☒	☒
CCP Induction Program	N/A	By Training Department	☐	☒	☒
All Safety Programs (see SOP 7.3)	SOP 7.3	SOP 7.3	☒	☐	☒

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	7.2 ANNUAL TRAINING REQUIREMENTS AND MINIMUM PATIENT FACING HOURS FOR AMBULANCE PARAMEDICS AND CRITICAL CARE PARAMEDICS
Implementation Date	10 August 2017
Purpose	To ensure that all HMCAS clinical staff comply with CPD requirements as outlined by DHP, JCI, HMCAS.
Responsibility	ALL HMCAS CLINICAL STAFF

1. Introduction:

- 1.1 In order to maintain clinical privileges within Hamad Medical Corporation Ambulance Service (HMCAS), all Ambulance Paramedic (AP), Clinical Care Paramedic (CCP) staff will be required to meet minimum annual training requirements.
- 1.2 The minimum standards are a combination of those requirements set forth by the Department of Healthcare Professions (DHP), Joint Commission International (JCI), and the HMCAS executive management team.

2. Definitions:

- 2.1 Continuous Professional Development (CPD): In the context of HMCAS this is any training, course, or learning activity which meets the standards of DHP and is accredited or accepted by the DHP for CPD hours.
- 2.2 Mandatory HMCAS CPD: In alignment with HMC CPD's mission, our Training Department will strategically developed quarterly mandatory CPD activities to include but are not limited to; Medical, Trauma, Resuscitation, MIR, Paediatric and Obstetric and Gynecological emergencies etc.

3. DHP Minimum Requirements for Ambulance Paramedics and Critical Care Paramedics:

- 3.1 Resuscitation (CPR) Course every 2 years – HMCAS Resuscitation CPD meets this requirement. This activity will be available to attend 1 quarter each year.
- 3.2 Trauma Emergencies Course every 2 years – HMCAS Trauma CPD meets this requirement. This activity will be available to attend 1 quarter each year.
- 3.3 Paediatric Course – HMCAS Paediatric CPD **does NOT** meet this requirement. This requirement is met through completion of APLS/PALS for CCPs or PLS course for APs or equivalent courses as accepted by DHP. These courses require renewal upon certificate expiration.
- 3.4 Patient facing hours – minimum of 24 patient facing hours (2 clinical shifts) per month, with attendance averaged over 12 months. HMCAS deem the following areas of clinical practice as being patient facing for maintenance of clinical hours:
 - 3.4.1 Any 12-hour allocated shift within 999, SAS, Retrieval, or within Special Operations.
 - 3.4.2 Event coverage
 - 3.4.3 A shift allocated to a hospital department.
 - 3.4.4 A shift as the Clinical Command Emergency or Clinical Command Retrieval (CCP only) based within the National Command Centre.
 - 3.4.5 A shift as the Flight Coordinator (CCA only) within the National Command Center.

3.4.6 On-call responsibilities for the provision of clinical advice are applicable exclusively to Silver clinical on-call duties.

3.5 Continuous Professional Development hours – 40 hours per year. 20 Category 1 and 20 category 2/3.

4. HMCAS Minimum Requirements:

4.1 All AP and CCP staff must meet all requirements described in section 1 through 3.

4.2 AP licensed staff: Completion of quarterly mandatory clinical HMCAS CPD activities offered by the HMCAS Training Department with the addition of the Airway CPD and MIR CPD activities once per year. (Total of 6 activities annually).

4.3 CCP licensed staff and Critical Care Assistant (CCA): Completion of all 4 mandatory HMCAS CPD activities offered by the HMCAS Training Department per year (1 per quarter) with the addition of the Airway CPD which will run once a week throughout the year, as well as, the MIR CPD once in the year.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Staff
Procedure	7.3 REQUIRED SAFETY COURSES FOR ALL HMCAS STAFF
Implementation Date	10 August 2017
Purpose	To ensure that all HMCAS Staff are on-boarded appropriately and receive consistent orientation and training required by HMC and JCI.
Responsibility	ALL HMCAS STAFF

1. Mandated Safety Courses:

- 1.1 All HMCAS staff are required to complete and maintain certification of the following online HMCAS Specific Workplace Safety Courses:
 - 1.1.1 As part of initial orientation all HMCAS staff must complete the following courses:
 - 1.1.1.1 Ambulance Service Fire Safety Training
 - 1.1.1.2 Ambulance Service Infection Control Training
 - 1.1.1.3 Ambulance Service Fall Safety Training
 - 1.1.1.4 Ambulance Service Quality Improvement and Patient Safety Training
 - 1.1.1.5 HMC Environmental Safety eLearning Module
 - 1.1.1.6 HMC Major Incidents eLearning Module
 - 1.1.1.7 HMC Infection Prevention and Control Orientation eLearning module
 - 1.1.1.8 HMC Fire Safety eLearning module
 - 1.1.2 Recertification periods for all HMCAS Staff:
 - 1.1.2.1 Annually:
 - 1.1.2.1.1 Ambulance Service Fire Safety (or HMC equivalent)
 - 1.1.2.1.2 Ambulance Service Fall Safety
 - 1.1.2.1.3 HMC Environmental Safety
 - 1.1.2.1.4 HMC Major Incidents
 - 1.1.2.2 Every 2 years:
 - 1.1.2.2.1 Ambulance Service Infection Control (or HMC equivalent).
 - 1.1.2.3 Every 3 years:
 - 1.1.2.3.1 For non-clinical staff, Infection Control should be completed every 3 years.

2. Accessing the HMCAS specific Safety Training Courses:

- 2.1 Ambulance Service training modules as well as HMC modules will be hosted on the eLearning platform, eTaleem.
- 2.2 Staff will receive an automated email alert from the SMS system at 30 days, 15 days and 7 days prior to expiry of the module. Therefore, staff must ensure they have access to their HMC email and ensure their inbox is not full.

3. Compliance:

- 3.1 It is the responsibility of the individual staff member to ensure they remain compliant with the patient safety courses at the intervals defined in section 1.1.2.

- 3.2 Failure to maintain compliance may result in staff being removed from clinical duties until such time as the courses are completed.
- 3.3 All eTaleem course completion certificates must be sent to ASTraining@hamad.qa by email to ensure compliance is updated.

SAFETY COURSES – Must Attain 100% Compliance Within Refresher Period (JCI/Corporate Policy)					
Title	Recertification Period	Delivery	JCI Required	DHP Required	HMCAS Required
HMC Environmental Safety	Annual	Online	☒	☐	☒
HMC Major Incident Response	Annual	Online	☒	☐	☒
HMCAS Fire Safety	Annual	Online	☒	☐	☒
HMCAS Fall Safety	Annual	Online	☒	☐	☒
HMCAS Infection Control	2 years	Online	☒	☐	☒
HMCAS Infection Control – Non-Clinical Staff	3 years	Online	☒	☐	☒
HMCAS QIPS Training	Upon Induction	Online or in Person	☒	☐	☒

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Staff
Procedure	7.4 REPORTING OF COMPLIANCE WITH MANDATORY CLINICAL TRAINING
Implementation Date	10 August 2017
Purpose	To ensure that all HMCAS staff comply with Workplace Safety Training as mandated by Ambulance Service and Corporate Policy.
Responsibility	HMCAS TRAINING DEPARTMENT

1. Reporting Frequency:

- 1.1 A compliance report will be provided to the HMCAS Head of Professions during the first week of each new quarter.

2. Report Content:

- 2.1 Percentage of staff who are compliant with each of the HMCAS Mandatory Continuing Professional Development clinical training modules.
- 2.2 Percentage of staff who are compliant with the HMCAS offered safety courses:
 - 2.2.1 Fire Safety
 - 2.2.2 Infection Control
 - 2.2.3 QPS Training
 - 2.2.4 Fall Safety Training
- 2.3 Environmental Safety will be reported quarterly by the Environmental Safety Officer within HMC's office of Occupational Health and Safety Department.

3. Validation of Clinical Training Compliance Reports:

- 3.1 Completion of training in Trauma, Resuscitation, and Paediatric emergency care is independently verified by the DHP.
- 3.2 Proof of completion must be provided to the DHP for initial licensure and every 2 years thereafter for re-licensure.
- 3.3 Continuing Professional Development courses are accredited by the DHP. Signed attendance records and course content are maintained, as required by DHP, for review and verification.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Staff
Procedure	7.5 SCHEDULING OF CORE HMCAS CPD COURSES FOR APS AND CCPS
Implementation Date	10 August 2017
Purpose	To ensure that staff are scheduled to attend mandatory training courses within their standard paid hours.
Responsibility	ALL HMCAS AMBULANCE PARAMEDICS AND CRITICAL CARE PARAMEDIC STAFF

1. Scheduling of CPD sessions:

- 1.1 The Workforce Management Department is responsible for scheduling staff on the Hamad Medical Corporation Ambulance Service (HMCAS) Continuous Professional Development (CPD) activities, ensuring alignment with operational schedule and to avoid and conflicts.
- 1.2 Each department within the Ambulance Service (AS) should maintain a member within the department to act as a liaison between his/her department, the Training Department and the Scheduling Department with regards to the scheduling of their staff to CPD activities.
- 1.3 AP Staff should be scheduled for each quarterly CPD activity plus an MIR CPD activity once per year, ensuring all staff attend 5 CPD activities annually as part of their annual shift requirements. CCP and CCA staff should be scheduled for each quarterly CPD activity plus an MIR and airway CPD activity once per year, ensuring all staff attend 6 CPD activities annually, with 5 CPD activities as part of their annual shift requirements, with 1 activity as overtime.
- 1.4 It is the responsibility of the supervisors and managers within the department to ensure their staff are allotted time within their mandatory hours to attend the core CPD events.
- 1.5 Staff should monitor their schedule and ensure that they access the online content on eTaleem and complete all activities at least 1 week prior to attending the course.
- 1.6 The Training Department will release a quarterly CPD schedule to all departments.
- 1.7 All scheduling of CPD activities should be done by the Scheduling Department and an updated schedule of events should be sent to the Training Department.
- 1.8 Any queries regarding the scheduling of Training sessions should be taken up with the staff member's department liaison or the Workforce Management Department directly.
- 1.9 All staff should be scheduled for the CPD event a minimum of 1 week prior to the event taking place.

2. Mandated CPD Training:

- 2.1 Each clinical staff member must attend all mandated quarterly CPD activities.

3. Cancellation of Attendance on a Core CPD Event:

- 3.1 The HMCAS mandatory CPD activities are considered as any other normally assigned duty, failure to attend is the same as failing to attend a regular duty shift. All relevant procedures for attendance, cancellation or sick leave are to be followed.
- 3.2 Staff are not permitted to cancel their attendance on a HMCAS Core CPD event unless the cancelation has been approved by their supervisor or manager. The supervisor or manager is responsible for contacting the Scheduling Department to notify of a staff member's cancelation of attendance.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Staff
Procedure	7.6 DRIVER TRAINING
Implementation Date	10 August 2017
Purpose	To ensure that all HMCAS staff comply with required Training prior to driving any HMCAS vehicle.
Responsibility	ALL HMCAS STAFF

1. Minimum Requirement for Operating an HMCAS Emergency Vehicle:

- 1.1 Valid Qatari Driver's License
- 1.2 Safe Driving Orientation

2. Non-Emergency Patient Transport Drivers:

- 2.1 Defined by marked/unmarked vehicles transporting HMCAS patients without emergency lights or sirens.
- 2.2 Staff must complete a Priority 2 (non-emergency response) driving assessment conducted by the HMCAS Training Department.
- 2.3 If a staff member is not successful on the Priority 2 driver training course due to a lack of basic driving skills, they will be requested to gain more experience driving in their personal vehicle and will be evaluated after a set interval of time, usually 6 months.

3. Emergency Patient Transport Driver:

- 3.1 Defined by marked/unmarked vehicles transporting HMCAS staff/patients with use of emergency lights or sirens.
- 3.2 Staff must successfully complete the Priority 1 (emergency response) driving course conducted by the HMCAS Training Department.
- 3.3 The duration of the Priority 1 Driving Course may be adjusted based on the individual's performance during his/her assessment as well as prior Priority 1 driving experience.
- 3.4 Only staff with sufficient past driving experience and basic driving ability are eligible to attend the Priority 1 driving course. This determination is at the sole discretion of the Training Department.

4. Record of Valid Driver's License:

- 4.1 Staff are required to provide the Training Department with an updated copy of his/her valid Qatar Driver's License. Copies should be sent to ASTraining@hamad.qa with his/her corporation number and "Driving License" in the subject line.

5. Break in Service over 6 months:

- 5.1 Staff that have not driven priority 1 for over 6 months will be required to attend a P1 driving refresher program of a minimum of 2 days.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Staff
Procedure	7.7 RETURN TO CLINICAL PRACTICE FOLLOWING A BREAK IN SERVICE
Implementation Date	10 August 2017
Purpose	To ensure that all HMCAS clinical staff attend an appropriate refresher training program and his/her clinical competency is appropriately assessed following a break in service.
Responsibility	ALL HMCAS AMBULANCE PARAMEDIC AND CRITICAL CARE PARAMEDIC STAFF

1. Minimum Patient Facing Requirements:

- 1.1 Staff licensed with the Department of Healthcare Professions (DHP) as an Ambulance Paramedic (AP) or Critical Care Paramedic (CCP) are required to perform a minimum of 2 shifts (12 hours each) per month in a “patient facing” role.
- 1.2 Patient facing hours may include, but are not limited to, shifts on Operational units (EAS or SAS), Retrieval Services, Special Operations, event coverage, Clinical Command Emergency, Clinical Command Retrievals Flight Co-Ordinator or in-hospital shifts.
- 1.3 Clinical on-call roles are considered as being ‘patient facing’.

2. Break in Service:

- 2.1 Any AP or CCP staff member who fails to meet the minimum requirement of 24 hours per month for a period greater than 3 months will be considered to have had a break in service.
- 2.2 Following a break in service the practitioner must undergo a defined training program delivered by the HMCAS Training Department prior to returning to independent clinical duties.
- 2.3 This program will have the outcome of preparing the practitioner for either independent clinical practice or supervised clinical practice.
- 2.4 Staff moving from Scheduled Ambulance Service to Emergency Ambulance Service will also undergo the 2-day refresher program.
- 2.5 In addition to staff demonstrating clinical competency, they must also be current in all mandatory training programs (as described in [SOP 7.4](#)) prior to returning to clinical duties.

3. Clinical Practice Definitions:

- 3.1 Independent Clinical Practice: To practice as a primary member of a response unit and be able to deliver all care and to make all treatment decisions within the appropriate scope of practice without supervision.
- 3.2 Supervised Clinical Practice: To practice as a non-essential member of a response unit and only performing clinical interventions and making treatment decisions under the supervision of an Ambulance Paramedic Educator (APEd), CCP Preceptor, or senior clinical leader.
- 3.3 Observational Practice: To practice as a non-essential member of a response unit while observing the clinical practice of the primary crew. If the staff member is licensed and in good standing with the DHP and HMCAS they may assist with non-invasive patient care at the discretion of the primary crew.

4. Requirements for Returning to Independent Clinical Practice:

- 4.1 Break in Service 3 to 6 months:
 - 4.1.1 CCP Refresher Course

- 4.1.1.1 Total duration: 4 days subject to the individual's training needs.
- 4.1.1.2 Topics and associated skills:
 - 4.1.1.2.1 Resuscitation
 - 4.1.1.2.2 Trauma
 - 4.1.1.2.3 Paediatrics
 - 4.1.1.2.4 RSI skill drill
 - 4.1.1.2.5 Failed and difficult airway drill
- 4.1.2 AP Refresher Course
 - 4.1.2.1 Total duration: 2 days
 - 4.1.2.2 Topics and associated skills:
 - 4.1.2.2.1 Resuscitation
 - 4.1.2.2.2 Trauma
 - 4.1.2.2.3 Paediatrics
 - 4.1.2.2.4 Primary Patient Assessment
 - 4.1.2.2.5 Airway Management
 - 4.1.2.3 If the clinician is unsuccessful during the refresher training an action plan will be drawn up by the HMCAS Training Department that is agreed with the clinician
- 4.2 Break in service 6 to 12 months
 - 4.2.1 CCP Refresher Course
 - 4.2.1.1 Total duration: 5 days
 - 4.2.1.2 Topics and associated skills:
 - 4.2.1.2.1 Resuscitation
 - 4.2.1.2.2 Trauma
 - 4.2.1.2.3 Paediatrics
 - 4.2.1.2.4 RSI skill drill
 - 4.2.1.2.5 Failed and difficult airway drill
 - 4.2.1.2.6 Anaesthetic attachment: 1 shift (8 hours – covering airway management and anaesthetic medications)
 - 4.2.1.3 Clinical Shifts: A minimum of 2 shifts in a high activity hub with a preceptor.
 - 4.2.2 AP Refresher Course
 - 4.2.2.1 Total duration: 5 days
 - 4.2.2.2 Topics and associated skills:
 - 4.2.2.2.1 Resuscitation
 - 4.2.2.2.2 Trauma
 - 4.2.2.2.3 Paediatrics
 - 4.2.2.2.4 Primary Patient Assessment

- 4.2.2.2.5 Airway Management
 - 4.2.2.2.6 Clinical documentation
 - 4.2.2.3 Clinical Shifts: A minimum of 2 shifts in a high activity hub with an APED.
- 4.2.3 If the clinician is unsuccessful during the refresher training an action plan will be drawn up by the HMCAS Training Department that is agreed upon by the clinician.
- 4.3 Break in Service 12 to 24 months:
 - 4.3.1 CCP Refresher Course:
 - 4.3.1.1 Total duration: 11 days
 - 4.3.1.2 Complete mandated CPD modules.
 - 4.3.1.3 Skills to be covered:
 - 4.3.1.3.1 RSI simple and complex
 - 4.3.1.3.2 Failed airway
 - 4.3.1.3.3 Difficult airway
 - 4.3.1.3.4 Pacing and cardioversion
 - 4.3.1.3.5 Procedural sedation
 - 4.3.1.3.6 Normal and complicated delivery
 - 4.3.1.4 Assessment: 1 day (completed by a Training Officer prior to conducting clinical shifts).
 - 4.3.1.5 Anaesthetic attachment: 1 shift (cover airway management and anaesthetic medication).
 - 4.3.1.6 Clinical shifts: Minimum of 4 shifts in a high activity hub with a preceptor.
 - 4.3.1.7 Additional supervised patient contact can be mandated if a need is identified during initial classroom training/preceptor shifts.
 - 4.3.1.8 Final sign off interview with either the Head of Professions or his/her designate.
 - 4.3.2 AP Refresher Course:
 - 4.3.2.1 Total duration: 10 days
 - 4.3.2.2 Complete the mandated CPD modules (or equivalent) and an obstetrics review session.
 - 4.3.2.3 Assessment: 1 day (completed by a Training Officer prior to conducting clinical shifts).
 - 4.3.2.4 Clinical shifts: Minimum of 4 shifts in a high activity hub with a preceptor.
 - 4.3.2.5 Additional supervised patient contact can be mandated if a need is identified during initial classroom training/preceptor shifts.
 - 4.3.2.6 Final sign off interview with either the Head of Training or his/her designate.
- 4.3.3 If the clinician is unsuccessful during the refresher training an action plan will be drawn up by the HMCAS Training Department that is agreed upon by the clinician.
- 4.4 Break in service greater than 24 Months:

- 4.4.1 A CCP or AP that has not completed regular clinical shifts for more than **24 months is mandated by DHP to complete a minimum of 3 months of supervised clinical practice prior to returning to independent clinical practice.**
- 4.4.2 Such staff are required to complete the following prior to return to independent clinical practice:
 - 4.4.2.1 Demonstrate all clinical induction training competencies through theory assessments, skills assessments, and simulations commensurate with their level of licensure (AP or CCP) followed by;
 - 4.4.2.2 3 months or equivalent number of shifts (minimum of 48 shifts) of supervised clinical practice with a preceptor or Ambulance Paramedic Educator.
- 4.4.3 On the completion of all supervised shifts the AP or CCP will undergo a final assessment by either an APED or CCP preceptor.
- 4.4.4 On completion of final assessment all APs will undergo a final sign off interview with the Head of Training or his/her designate and all CCPs will undergo a final sign off interview with the Head of Professions or his/her designate.

5. Requirements for Returning to Observed or Supervised Clinical Practice:

- 5.1 Staff who have had a break in service for greater than 3 months but who will not be returning to independent clinical practice and will not serve as a primary member of the crew are required to undergo a 1-day refresher training program covering:
 - 5.1.1 Equipment review
 - 5.1.2 Safe lifting and moving
 - 5.1.3 Seatbelt use
 - 5.1.4 Reporting process
 - 5.1.5 Assisting in resuscitation

6. Other Mandatory Training:

- 6.1 If staff are not current with their mandatory safety training courses (i.e. fire safety, infection control, etc.) more time may be added to complete these trainings.
- 6.2 Prior to signing off the staff member as being able to return to clinical duties, the training department will ensure the staff member is current with all mandatory trainings.

7. Training Priority:

- 7.1 The training department will offer refresher training in such a way as not to interfere with ongoing primary AP/CCP induction training and/or CPD training.
- 7.2 Priority for refresher training will be given to those staff returning to a full-time clinical role.
- 7.3 Refresher training will be offered dependent on having enough training instructors, venues, and equipment available to conduct the training.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	Ambulance Service Operations
Procedure	7.9 MAINTAINING LICENSURE WITH DHP AND PRIVILEGING
Implementation Date	06 July 2021
Purpose	Outline the responsibilities of all DHP licensed HMCAS in maintaining their license.
Responsibility	ALL HMC AMBULANCE SERVICE GROUP LICENSED, PHYSICIAN, NURSES, AMBULANCE PARAMEDICS, CRITICAL CARE PARAMEDICS, OR EMERGENCY MEDICAL DISPATCHERS

1. Introduction:

- 1.1 The retention of a Department of Healthcare Professions (DHP) license is required for **ALL** Ambulance Service Group staff holding a post where they are listed on **ORACLE** as being in a clinical role or where holding a DHP license is mandated by their job description.
- 1.2 Retention of a license is the responsibility of the individual staff member and failure to retain a license (without prior agreement from your Executive Director) will result in removal from patient facing duties and could result in disciplinary action. Failure to maintain a license will result in a change of terms and conditions of employment and may result in termination of employment.
- 1.3 **ALL** staff are required to ensure that they have completed all relicensing requirements and have uploaded all relevant/required information to DHP **NO LATER** than 60 days prior to the date of license expiry when staff are completing the 5th year of the license cycle.
- 1.4 **ALL** staff are required to ensure that they have completed all relicensing requirements and have uploaded all relevant/required information to DHP **NO LATER** than 30 days prior to the date of license expiry when staff are completing any other year of the license cycle.
- 1.5 **ALL** staff are required to ensure that their DHP registered email account AND their HMC account are checked regularly for licensing, CPD or safety bulletin updates. HMCAS staff are required to manage their email 'in box' to ensure that email is received and responded to in a timely manner.
- 1.6 It is advisable that staff avoid taking holiday, during the relicensing period, in case DHP return their application and they are therefore unable to action any requests. Those taking leave during this period do so at their own risk.

2. Training and appraisal:

- 2.1 The minimum training and education requirements expected of HMC licensed paramedics and CCPs is outlined in [SOP 7.2](#) and [7.3](#).
- 2.2 The minimum training and education requirements expected of an HMC licensed ambulance Emergency Medical Dispatcher is outlined in [SOP 3.29](#).
- 2.3 Staff are reminded that it is their responsibility to ensure that they are fully aware of DHP requirements regarding relicensure. These requirements are published on the DHP website.
 - 2.3.1 To support staff HMCAS maintains a list of essential licensures and relicensure requirements published under the HMC e-learning platform etaleem. This information is for **guidance only** and does not superseded any specific information circulated by DHP. Staff are directed to point 2.3 of this SOP.
 - 2.3.2 Staff are required to ensure that they can demonstrate DHP CPD cycle compliance as well as DHP required CPR courses as per scope of practice during their annual appraisal. Failure to be able to demonstrate compliance may will result in an appraisal

score of no higher than **THREE**. A subsequent action plan to demonstrate a process to complete will be required.

3. Action to be taken if DHP license is delayed (Please refer to Policy HR 3009):

- 3.1 Staff are reminded of point 1.3 of this SOP.
- 3.2 If a staff member has not received their new license within 14 days of when the license expires then the following is to be instigated:
 - 3.2.1 Line manager is to be informed IN PERSON AND VIA EMAIL
 - 3.2.1.1 Line manager will ensure that:
 - 3.2.1.1.1 The staff member is not rostered on a clinical shift from 00.01am on the day the license expires.
 - 3.2.1.1.2 Staff will be placed onto UNPAID leave until which time their license is renewed.
 - 3.2.1.1.3 The case will be reviewed, and a written warning will be issued and an annual appraisal score of a maximum of 2 will be applied REGARDLESS of overall performance within the appraisal period.
 - 3.2.1.1.4 Staff who fail to license for a 2nd time will be referred for disciplinary at Executive Director level.
 - 3.2.2 The Training Department is to be informed IN PERSON AND VIA EMAIL.
- 3.3 Support from the Training Department with relicensure issues:
 - 3.3.1 The Training Department's responsibilities to support staff licensure cease after the initial permanent license is issued.
 - 3.3.2 Any request for assistance with re licensure can only originate from an Operations Manager or higher. Individual staff are not permitted to seek assistance without the authority of their Operations Manager, Senior Operations Manager, Assistant Executive Director or Executive Director otherwise such requests will be denied.

4. Privileging:

- 4.1 This SOP is a supportive document produced as part of the HMC AS privileging policy, HMCAS Clinical Practice Guidelines, and HMCAS Clinical Procedures Manual.
- 4.2 All staff who are licensed with DHP either as an Ambulance Paramedic (AP), Critical Care Paramedic (CCP), or Ambulance Service Medical Dispatcher (ASMD) are privileged via the applicable Head of Professions, through the office of the Group Medical Director. This is regardless of their area of employment within HMCAS as all licensed Ambulance Paramedics, Critical Care Paramedics, and ASMDs are professionally accountable to the office of the applicable Head of Professions.
- 4.3 The degree of privileging is related to the level at which the clinician is licensed with DHP and their anticipated exposure to managing patients. Privileging is a patient safety requirement and does not affect an individual's terms and conditions of employment.
- 4.4 The individuals historical training and/or previous level of privileging is not a factor in the level of privileging that will be awarded. as outlined in point 4.3.
- 4.5 HMCAS staff are not entitled to function outside of their level of privileging except during a major incident on the instruction of Silver Clinical, Silver Command, Group Medical Director, or Gold Command.

- 4.6 HMCAS will operate four levels of privileging for DHP registered AP and CCP staff.
 - 4.6.1 Level 4 (CCP only)
 - 4.6.1.1 CCP's working in essential support roles where patient facing roles are limited will have a reduced scope operating at level 3. The additional authority to administer opiate analgesia, Ketamine analgesia (but not Ketamine sedation) and Midazolam for the control of seizures but not sedation will be reviewed on a case per case basis
 - 4.6.1.2 Level 3 (reflecting Ambulance Paramedic as per DHP scope)
 - 4.6.1.3 Level 2 (reflecting Basic Paramedic as per DHP scope)
 - 4.6.1.4 Level 1 is at the first responder level of care
- 4.7 Regardless of the level of privileging, awarded to an HMC AS clinician, they must retain their license as per their Job title/description (either as an Ambulance Paramedic, Critical Care Paramedic, or ASMD) to meet the terms and conditions of their employment. Unless continued licensure with DHP is no longer a requirement of their continued employment.

SOP 8: Clinical Desk



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services Clinical Desk
Procedure	8.4 CLINICAL DESK PROCESS FLOWS
Implementation Date	06 October 2020
Purpose	To organize and standardize the utilization of process flows for routine processes used on the Clinical Desk.
Responsibility	ALL STAFF ASSIGNED TO THE CLINICAL DESK

1. Background and Purpose:

- 1.1 In undertaking daily operational duties, staff on the Clinical Desk perform tasks that are of a routine nature and interlink with other routine tasks to form routine processes. With the objective of improving standardization of these processes the Clinical Desk shall utilize process flows.

2. Definitions applicable within this SOP:

- 2.1 **'CAD'** shall mean Computer Aided Dispatch and applies to the use of any approved and officially implemented Computer Aided dispatch system used by HMC Ambulance Service including, but not limited to, Najem (NJM) or Valencia.
- 2.2 **'CC: Emergency'** or 'Clinical Coordinator: Emergency' refers to the Critical Care Paramedic employed by HMC Ambulance Service with level 4 clinical privileging and deployed to the Clinical Desk to provide clinical support and hospital pre-arrival notifications for patient facing staff.
- 2.3 **'CC: Retrieval'** or 'Clinical Coordinator: Retrieval' refers to the Critical Care Paramedic employed by HMC Ambulance Service with level 4 clinical privileging and deployed to the Clinical Desk to provide clinical support, transfer triage and coordinate international aeromedical cases.
- 2.4 **'Clinical Desk'** (the desk) means the division under the governance of Clinical Services that includes the Clinical Coordinator: Emergency, Clinical Coordinator: Retrieval, LifeFlight Coordinator, EMD: Retrievals Coordinators within the HMC Ambulance Services Control and Communications Center under the leadership of the SME: Clinical Desk.
- 2.5 **'Process Flow'** means the graphical representation of the steps or phases and the sequence in which they are undertaken when performing a given routine procedure.

3. Procedure:

- 3.1 The Clinical Desk employs the use of process flows to specify the standard approach to routine processes undertaken in daily operations of the Clinical Desk.
 - 3.1.1 These process flows may be set out without an associated SOP unless the process is of a nature that requires a written, detailed SOP.
- 3.2 The authorization of Process Flow implementation will be at the discretion of the Assistant Executive Director of Clinical Services together with the Assistant Executive Director of any other division affected by the Process Flow.
 - 3.2.1 Where approved for implementation without executive resolution, the process flow shall be signed off by the Assistant Executive Directors authorizing its implementation before being published.
 - 3.2.2 Where deemed necessary, the process flow will be submitted for executive resolution through the standard HMC Ambulance Services resolution process.

- 3.3 Process flows that have received resolution for implementation and do not have an associated SOP shall be appended to this SOP and numbered accordingly.
- 3.4 Deviation from process flows:
 - 3.4.1 A Clinical Coordinator on duty at the Clinical Desk as a CC: Emergency or CC: Retrieval may deviate from a process flow only when considered reasonably necessary to provide clinical benefit to the patient.
 - 3.4.2 Notwithstanding 3.4.1 above, any deviation from a resolved process flow must be authorized by a senior manager or on-call commander.
 - 3.4.3 Any deviation from a process flow, by any person, shall be documented in the appropriate CAD system case notes and include a reason for the deviation.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services Clinical Desk
Procedure	8.5 CLINICAL DESK ONBOARDING AND CURRENCY
Implementation Date	20 April 2022
Purpose	To set out the standard onboarding and currency standards for staff assigned to duties on the Clinical Desk.
Responsibility	ALL STAFF ASSIGNED TO THE CLINICAL DESK

1. Background and Purpose:

- 1.1 The purpose of this document is to establish and clarify the requirements and process for the onboarding of new staff and ongoing currency for all staff assigned to a role on the Clinical Desk.
- 1.2 This SOP sets out:
 - 1.2.1 Onboarding standards for newly assigned staff to the Clinical Desk.
 - 1.2.2 Clinical Desk currency standards
 - 1.2.3 Re-onboarding standards for those staff that have not maintained currency in accordance with provisions set out below.

2. Definitions applicable within this SOP:

- 2.1 **‘Clinical Desk’**, (the desk) means the division under the governance of Clinical Services that includes the Clinical Coordinator: Emergency, Clinical Coordinator: Retrieval, LifeFlight Coordinator, EMD: Retrievals Coordinators within the HMC Ambulance Services Control and Communications Center under the leadership of the SME: Clinical Desk.
- 2.2 **‘Clinical Coordination’** means the combined roles of the CC: Emergency and CC: Retrieval who are collectively responsible for delivering clinical coordination and support for HMC Ambulance Services.
- 2.3 **‘LifeFlight Coordinator’** means the combined roles of the CC: Emergency and CC: Retrieval who are collectively responsible for delivering clinical coordination and support for HMC Ambulance Services.
- 2.4 **‘EMD: Retrievals Coordinator’** refers to the Emergency Medical Dispatcher deployed to the Clinical Desk to dispatch, control and coordinate Retrieval resources and missions.
- 2.5 **‘MPDS’** means Medical Priority Dispatch System, sometimes referred to as the Advanced Medical Priority Dispatch System (AMPDS), the system used in HMC Ambulance Service communications centers for structured, standardized and systematic call interrogation and dispatch of appropriate resources as well as standardized provision of pre-arrival medical instructions.

3. Aims of Currency and Orientation:

- 3.1 Currency standards and orientation aim to ensure the maintenance of business continuity regarding:
 - 3.1.1 Equipping newly assigned staff with the knowledge and skills required for their role on the desk, ensuring they feel prepared to meet the daily demands imposed on them.
 - 3.1.2 Minimizing adverse events that may impact the quality of clinical care received by HMC Ambulance Service patients.
 - 3.1.3 Reduction of risks posed to aeromedical missions through ensuring continuous and consistent coordination and tracking of a high standard.

- 3.1.4 Ensuring professional and continual provision of quality service delivery at a high standard.
- 3.1.5 Ensure the maintenance of business continuity through appropriate deployment of skilled well-trained staff.

4. Staff Assignment to the Clinical Desk:

- 4.1 Deployment on the Clinical Desk is a requirement for all consoles (CC: Emergency, CC: Retrieval, LifeFlight Coordinator, EMD: Retrievals Coordinator) to ensure business continuity and sustainable provision of quality service.
- 4.2 Staffing of Clinical Coordination and LifeFlight Coordination is provided by Emergency Operations, Retrieval and LifeFlight sections of Clinical Services, and the EMD: Retrievals Coordinator role is staffed by the Healthcare Coordination Centre.
- 4.3 Staff assignment to roles in the Clinical Desk must comply with the following requirements:
 - 4.3.1 CC: Emergency staff must be a clinically privileged CCP (Level 4) deployed in Emergency Operations and/or LifeFlight with a minimum accumulative time in Charlie/LifeFlight of 6-months.
 - 4.3.2 CC: Retrieval staff must be a clinically privileged CCP (Level 4) deployed in a role undertaking local and international Retrievals with a minimum accumulative time in Retrievals of 6-months and have completed a minimum of 2 international retrievals.
 - 4.3.3 LifeFlight Coordinator staff must be a clinically privileged AP (Level 3) and appointed as a CCA in LifeFlight Operations with a minimum of 6-months of LifeFlight Operations experience.
 - 4.3.4 EMD: Retrievals Coordinator staff must be registered as an Emergency Medical Dispatcher and appointed in the role of dispatcher in HCC with a minimum of 6-months HCC dispatching experience.

5. Onboarding Standards:

5.1 Clinical Coordination:

- 5.1.1 The onboarding of CC: Emergency and CC: Retrieval shall be carried out through a similar process and content to support cross functional performance.
- 5.1.2 Onboarding shall be coordinated by the SME: Clinical Desk, or an appropriate delegated staff member, in collaboration with the relevant division management.
- 5.1.3 Prior to commencement of any shifts on the desk, Clinical Coordinator staff are required to complete the following, unless otherwise agreed by the SME: Clinical Desk.
 - 5.1.3.1 NCC Access approval
 - 5.1.3.2 Either pre-shift briefing on rules and conduct in HMCAS communications centers or orientation training covering the same.
- 5.1.4 Prior to commencement of independent shifts on the desk, Clinical Coordinator staff are required to complete the following, unless otherwise approved by the SME: Clinical Desk:
 - 5.1.4.1 CC: Emergency assigned Staff
 - 5.1.4.1.1 CD Onboarding – CC: Emergency Orientation
 - 5.1.4.1.2 CD Onboarding – CC: Emergency Preceptorship
 - 5.1.4.1.3 LifeNet Alert system familiarization

- 5.1.4.1.4 Cisco Telephone system and IVR familiarization
- 5.1.4.1.5 Any additional training agreed to by the Clinical Services management team.
- 5.1.4.2 CC: Retrieval assigned staff:
 - 5.1.4.2.1 CD Onboarding – CC: Emergency Orientation
 - 5.1.4.2.2 CD Onboarding – CC: Emergency Preceptorship (reduced length)
 - 5.1.4.2.3 CD Onboarding – CC: Retrieval Orientation
 - 5.1.4.2.4 CD Onboarding – CC: Retrieval Preceptorship
 - 5.1.4.2.5 LifeNet Alert system familiarization
 - 5.1.4.2.6 Cisco Telephone system and IVR familiarization
 - 5.1.4.2.7 Valencia CAD Call-taker training
 - 5.1.4.2.8 Any additional training agreed to by the Clinical Services management team.
- 5.1.4.3 Preceptorship shifts will usually be structured as set out below; however, remains subject to management discretion based on the staff members prior service experience, divisional assignments and operational needs:

Assigned Role	Primary Role	Role Supporting	NCC General	Total
CC: Emergency	2 (minimum)	2 (minimum)	1 (experience dependent)	5
CC: Retrieval	3 (minimum)	2 (minimum)	1 (experience dependent)	6

5.2 LifeFlight Coordinators.

- 5.2.1 The onboarding of LifeFlight Coordinators will be closely supported by the LifeFlight division specifically pertaining to expert knowledge areas.
- 5.2.2 Onboarding shall be coordinated by the SME: Clinical Desk, or an appropriate delegated staff member, in collaboration with the relevant division management.
- 5.2.3 Prior to commencement of any shifts on the desk, LifeFlight Coordinator staff are required to complete the following, unless otherwise agreed by the SME: Clinical Desk.
 - 5.2.3.1 NCC Access approval
 - 5.2.3.2 Either pre-shift briefing on rules and conduct in HMCAS communications centers or orientation training covering the same.
- 5.2.4 Prior to commencement of independent shifts on the desk, LifeFlight Coordinator staff are required to complete the following, unless otherwise approved by the SME: Clinical Desk:
 - 5.2.4.1 CD Onboarding – LifeFlight Coordinator Orientation
 - 5.2.4.2 CD Onboarding – LifeFlight Coordinator Preceptorship
 - 5.2.4.3 Cisco Telephone system and IVR familiarization
 - 5.2.4.4 LifeNet Alert system familiarization

- 5.2.4.5 Any additional training agreed to by the Clinical Services management team.
- 5.2.4.6 Preceptorship shifts will usually be structured as set out below; however, remains subject to management discretion based on the staff members prior service experience, divisional assignments and operational needs.

Assigned Role	Primary Role	Role Supporting	NCC General	Total
LifeFlight Coordinator	4 (minimum)	2 (minimum)	1 (experience dependent)	7

5.3 EMD Retrieval Coordinator:

- 5.3.1 The onboarding of EMD: Retrieval Coordinator shall be carried out as a collaborative arrangement between Clinical Desk and Healthcare Coordination Centre management.
- 5.3.2 Prior to commencement of any shifts on the desk, EMD: Retrieval Coordinator staff are required to complete the following, unless otherwise agreed between Clinical Desk and Healthcare Coordination Centre management.
- 5.3.2.1 NCC Access approval
- 5.3.2.2 Either pre-shift briefing on rules and conduct in HMCAS communications centers or orientation training covering the same.
- 5.3.3 Prior to commencement of independent shifts on the desk, EMD: Retrieval Coordinator staff are required to complete the following, unless otherwise agreed between Clinical Desk and Healthcare Coordination Centre management.
- 5.3.3.1 CD Onboarding – EMD: Retrieval Coordinator Orientation
- 5.3.3.2 CD Onboarding – EMD: Retrieval Coordinator Preceptorship
- 5.3.3.3 Cisco Telephone system training (facilitated by HCC)
- 5.3.3.4 Valencia CAD Training (facilitated by HCC)
- 5.3.3.5 Any additional training agreed to by the Clinical Desk and Healthcare Coordination Centre management.
- 5.3.3.6 Preceptorship shifts will usually be structured as set out below; however, remains subject to management discretion based on the staff members prior service experience, divisional requirements and operational needs.

Assigned Role	Sierra Retrieval/Similar	Shifts – focus: Ground Operations (Supervised)	Shifts – focus: Aeromedical Operations (Supervised)	Total
EMD: Retrieval Coordinator	2	8 (minimum)	8 (minimum)	18

6. Currency Standards.

- 6.1 All staff assigned to the desk are required to retain the following minimum currency requirements.
- 6.1.1 CC: Emergency assigned staff will be considered current for duties on the desk if the following are met:
- 6.1.1.1 Hold currency as a Charlie or LifeFlight CCP in Emergency Operations or LifeFlight.

- 6.1.1.2 Has completed Clinical Desk onboarding.
- 6.1.1.3 Has completed a minimum of four (4) Clinical Desk shifts within the preceding two (2) calendar months.
- 6.1.1.4 Has completed the annual CC Governing Document Review.
- 6.1.1.5 Holds current access to:
 - 6.1.1.5.1 The Clinical Desk share drive folder "hmcfs03\Clinical Command Desk".
 - 6.1.1.5.2 NCC Access approval
 - 6.1.1.5.3 Credentialing and access to required information systems.
- 6.1.1.6 Any other requirements agreed to by management.
- 6.1.2 CC: Retrieval assigned staff will be considered current for duties on the desk if the following are met:
 - 6.1.2.1 Holds currency as a Retrieval CCP in Retrieval
 - 6.1.2.2 Has completed Clinical Desk onboarding
 - 6.1.2.3 Has completed a minimum of four (4) Clinical Desk shifts within the preceding two (2) calendar months.
 - 6.1.2.4 Has completed the annual CC Governing Document Review.
 - 6.1.2.5 Holds current access to:
 - 6.1.2.5.1 The Clinical Desk share drive folder "hmcfs03\Clinical Command Desk"
 - 6.1.2.5.2 NCC Access approval
 - 6.1.2.5.3 Credentialing and access to required information systems.
 - 6.1.2.6 Any other requirements agreed to by management.
- 6.1.3 LifeFlight Coordinator assigned staff will be considered current for duties on the desk if the following are met:
 - 6.1.3.1 Hold currency as a LifeFlight CCA in LifeFlight.
 - 6.1.3.2 Has completed Clinical Desk onboarding.
 - 6.1.3.3 Has completed a minimum of four (4) Clinical Desk shifts within the preceding two (2) calendar months.
 - 6.1.3.4 Has completed the annual LFC Governing Document Review
 - 6.1.3.5 Holds current access to:
 - 6.1.3.5.1 The Clinical Desk share drive folder "hmcfs03\Clinical Command Desk"
 - 6.1.3.5.2 NCC Access approval
 - 6.1.3.5.3 Credentialing and access to required information systems.
 - 6.1.3.6 Any other requirements agreed to by management.
- 6.1.4 EMD: Retrieval Coordinator assigned staff will be considered current for duties on the desk if the following are met:
 - 6.1.4.1 Holds currency as a Dispatcher in Healthcare Coordination Centre.

- 6.1.4.2 Has Completed Clinical Desk onboarding
- 6.1.4.3 Has completed a minimum of four (4) Clinical Desk shifts within the preceding two (2) calendar months.
- 6.1.4.4 Holds current access to:
 - 6.1.4.4.1 The Clinical Desk share drive folder “hmcfs03\Clinical Command Desk”
 - 6.1.4.4.2 NCC Access approval
 - 6.1.4.4.3 Credentialing and access to required information systems.
- 6.1.4.5 Any other requirements agreed to by the Clinical Services and HealthcareCoordination Centre management team.

7. Staff Re-onboarding:

- 7.1 Where a staff member has completed onboarding; however, has not maintained the minimum shift currency requirement, they are considered not current for independent duties on the desk.
- 7.2 Staff that are no longer current for independent duties on the desk will be required to undertake re-onboarding in accordance to the re-onboarding matrix (below) unless otherwise agreed by the management team of the Clinical Desk and relevant staff division.
- 7.3 Re-onboarding shall be supported by the division from which the staff member is assigned and shall be coordinated by the SME: Clinical Desk, or an appropriate delegated staff member, in liaison with the assigning division.

Clinical Desk Role	≥ 4 Clinical Desk shifts in preceding 2 months	2-3 Clinical Desk shifts in preceding 2 months	1 Clinical Desk shifts in preceding 2 months	<1 Clinical Desk shifts in preceding 2 months
CC: Emergency CC: Retrieval	Current	1 Preceptorship shift with an appropriate Clinical Coordinator.	- Re-onboarding discussion facilitated by the SME: Clinical Desk. 2 Preceptorship Shifts with an appropriate Clinical Coordinator.	- Specific role re-orientation - CD Currency: CC Governing Document Review. 4 Preceptorship shifts (minimum) with an appropriate Clinical Coordinator.
	Current	1 Preceptorship shift with an appropriate LifeFlight Coordinator.	- Re-onboarding discussion facilitated by the SME: Clinical Desk. 2 Preceptorship Shifts with an appropriate LifeFlight Coordinator.	- Specific role re-orientation - CD Currency: LFC Governing Document Review. 4 Preceptorship shifts (minimum) with an appropriate LifeFlight Coordinator.
LifeFlight Coordinator	Current	1 Preceptorship shift with an appropriate LifeFlight Coordinator.	- Re-onboarding discussion facilitated by the SME: Clinical Desk. 2 Preceptorship Shifts with an appropriate LifeFlight Coordinator.	- Specific role re-orientation - CD Currency: RD Governing Document Review. 4 Preceptorship shifts (minimum) with an appropriate EMD: Retrieval Coordinator.
EMD: Retrievals Coordinator	Current	1 Preceptorship shift with an appropriate EMD: Retrieval Coordinator.	- Re-onboarding discussion facilitated by the SME: Clinical Desk. 2 Preceptorship Shifts with an appropriate EMD: Retrieval Coordinator.	- Specific role re-orientation - CD Currency: RD Governing Document Review. 4 Preceptorship shifts (minimum) with an appropriate EMD: Retrieval Coordinator.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services Clinical Desk
Procedure	8.6 CLINICAL DESK DAILY OPERATIONS
Implementation Date	06 October 2020
Purpose	To standardize and clarify daily routine shift procedures of the Clinical Desk.
Responsibility	ALL STAFF ASSIGNED TO THE CLINICAL DESK

1. Background and Purpose:

- 1.1 The Clinical Desk operates within the National Command Centre and is therefore expected to work in accordance with the general provisions of [SOP 3.9](#) Daily Operations in the Communications Centre's.
- 1.2 The Clinical Desk, however, operates with a different ethos and is staffed differently to the Communication Center's; therefore, this SOP sets out specific provisions that apply to the daily operation of the Clinical Desk.

2. Definitions applicable within this SOP:

- 2.1 **'Clinical Desk'** (the desk) means the division under the governance of Clinical Services that includes the Clinical Coordinator: Emergency, Clinical Coordinator: Retrieval, LifeFlight Coordinator, EMD: Retrievals Coordinators within the HMC Ambulance Services Control and Communications Center under the leadership of the SME: Clinical Desk.
- 2.2 **'CC: Emergency'** or 'Clinical Coordinator: Emergency' refers to the Critical Care Paramedic employed by HMC Ambulance Service with level 4 clinical privileging and deployed to the Clinical Desk to provide clinical support and hospital pre-arrival notifications for patient facing staff.
- 2.3 **'CC: Retrieval'** or 'Clinical Coordinator: Retrieval' refers to the Critical Care Paramedic employed by HMC Ambulance Service with level 4 clinical privileging and deployed to the Clinical Desk to provide clinical support, transfer triage and coordinate international aeromedical cases.
- 2.4 **'LifeFlight Coordinator'** refers to the Critical Care Assistant employed by HMC Ambulance Service with level 3 clinical privileging and deployed to the Clinical Desk to dispatch, control and coordinate LifeFlight resources and missions.
- 2.5 **'EMD: Retrievals Coordinator'** refers to the Emergency Medical Dispatcher deployed to the Clinical Desk to dispatch, control and coordinate Retrieval resources and missions.
- 2.6 **'LifeX'** refers to the Integrated Command and Control System used by NCC-TCC for radio and telephony communications to and from consoles in the NCC-TCC.

3. Commencement of shift:

- 3.1 Clinical Desk shift timings shall be determined by the SME: Clinical Desk in collaboration with relevant divisions and will be reviewed regularly based on operational need.
- 3.2 All staff assigned to the desk, regardless of specific role, shall report for duty at the National Command Centre fifteen minutes prior to the commencement of shift to facilitate endorsement between the oncoming and off-going staff.
- 3.3 Off-going staff shall endorse their shifts occurrences, ongoing or pending issues and cases to the oncoming shift.
- 3.4 Staff must undertake an operational check of their console and workspace at the commencement of shift including, but not limited to:

- 3.4.1 Confirming their HMC workstation and screens are functional and that they can access network services.
- 3.4.2 Login to Cisco Finesse and check that their phoneline is functional for incoming calls, through the IVR (4439 8833), and outgoing calls.
- 3.4.3 Start up and login to all applications necessary to carry out their specific duties for the shift including, as a minimum and as applicable to the console, Valencia CAD, NJM, LifeX, Skytrac, LifeNet and Cerner.

4. Break management and fatigue:

- 4.1 Staff assigned to the Clinical Desk shall be entitled to take fatigue management breaks as needed within accordance to the below set out conditions.
- 4.2 At any given time, the below shall apply:
 - 4.2.1 Where two Clinical Coordinators are on duty, at least one Clinical Coordinator shall be present at the desk at all times.
 - 4.2.1.1 Where only one Clinical Coordinator is on duty, either the Clinical Coordinator or LifeFlight Coordinator must be present at the desk at all times.
 - 4.2.2 Either the LifeFlight Coordinator or CC: Emergency shall be present at the desk at all times.
 - 4.2.3 Where two EMD: Retrievals Coordinators are on duty, at least one EMD: Retrievals Coordinator shall be present at the desk at all times.
 - 4.2.3.1 Where only one EMD: Retrievals Coordinator is on duty, either the CC: Retrieval or EMD: Retrievals Coordinator shall be present at the desk at all times.
- 4.3 When taking a break, staff must notify either one of the Clinical Coordinators that they are going on a break and provide their contact number and intended break location.
- 4.4 Any single break taken by any staff member of the desk may not exceed 45 minutes in duration and should not be excessive in quantity through the duration of the shift.
- 4.5 Breaks are subject to operational demand posed on the desk, and any staff member on a break may be recalled to duty by a Clinical Coordinator, supervisor or manager should the need arise.

5. National Command Centre TCC rules:

- 5.1 All staff assigned to duties on the desk must be familiar with [SOP 3.9](#) Daily Operations in the Communications Centre's, including all provisions applicable to conduct within the HMCAS NCC Communications Centre.
 - 5.1.1 Any uncertainty about what is acceptable inside the NCC-TCC should be raised to the Oscar NCC on duty at the time.

6. Handling and management of infrastructure:

- 6.1 All Clinical Desk staff shall use their own credentials when logging into any applications and systems including, but not limited to, NJM, LifeX, Valencia CAD and Cerner.
- 6.2 Staff must ensure they log-off of all workstations at the completion of their shift prior to leaving their console including NJM, LifeX and HMC computers.
- 6.3 All HMC Ambulance Service workstations used on the Clinical Desk must be restarted once a night shift between the hours of 00:00 and 05:00.

- 6.4 Staff should ensure that the computer applications required, on short notice, as part of their duties is ready for use at any time while at their console with minimal delay.
 - 6.4.1 Workstations should be unlocked and awaiting user input.
 - 6.4.2 Required applications should remain open, logged in and ready for user input.
- 6.5 Staff should not store personal files on Clinical Desk workstations as these are shared operational computers that may, without notice, be sent for urgent upkeep and maintenance.
- 6.6 Multi-user HMC Ambulance Service workstations will be scheduled by the SME: Clinical Desk, in collaboration with the IT Delivery and Transformation Program, for an annual workstation rebuild and other necessary maintenance as required.
- 6.7 Cisco Telephone Failure:
 - 6.7.1 Any failure involving the function of the Clinical Desk telephone system must immediately be escalated to the Oscar HCC and be handled in accordance with [SOP 3.10](#) as guided by the Oscar HCC.
 - 6.7.2 The following persons must be notified immediately:
 - 6.7.2.1 Oscar NCC
 - 6.7.2.2 Consultant Critical Care Paramedic: Effectiveness
 - 6.7.2.3 SME: Clinical Desk
 - 6.7.3 LifeX shall be used as the primary back-up system for all outgoing phone calls until the issue is resolved.
- 6.8 Tetra Radio or Tetra Network Failure:
 - 6.8.1 Any failure involving the function of any Clinical Desk tetra radio or the tetra radio system must immediately be escalated to the Oscar NCC and be handled in accordance with [SOP 3.11](#) as guided by the Oscar NCC.
 - 6.8.2 The following persons must be notified immediately:
 - 6.8.2.1 Oscar HCC
 - 6.8.2.2 Consultant Critical Care Paramedic: Effectiveness
 - 6.8.2.3 SME: Clinical Desk
 - 6.8.3 If the system failure is isolated to the LifeX system, staff shall use the handheld tetra radios as a fallback until LifeX function is restored.
- 6.9 NJM and LifeX failure:
 - 6.9.1 In the event of any NJM or LifeX performance failure the user should attempt to restart the application or the workstation.
 - 6.9.2 If 6.9.1 above does not resolve the issue it must be escalated to the Oscar NCC whom shall arrange the appropriate response to resolve the issue.
 - 6.9.3 Where the failure of NJM or ICCS performance cannot be resolved within 15 minutes, Clinical Desk staff must notify:
 - 6.9.3.1 Consultant Critical Care Paramedic: Effectiveness
 - 6.9.3.2 SME: Clinical Desk
- 6.10 HMC workstation failure:

- 6.10.1 In the event of a failure of normal performance of the HMC Ambulance Service workstation or its accessories, the user must perform a computer restart using the restart option in the windows start menu.
- 6.10.2 If 6.10.1 above does not resolve the issue, contact HICT at 4439 1444 who shall assist remotely to resolve the issue.
- 6.10.3 Where the failure of HMC Workstation performance cannot be resolved within 15-minutes, staff must notify:
 - 6.10.3.1 Oscar HCC/HCC Utilization Team Leader (UTL)
 - 6.10.3.2 Consultant Critical Care Paramedic: Effectiveness
 - 6.10.3.3 SME: Clinical Desk
- 6.11 CareMonX (Valencia) CAD failure:
 - 6.11.1 In the event of any CareMonX CAD performance failure the user should attempt to restart the application or the workstation.
 - 6.11.2 If 6.11.1 above does not resolve the issue it must be escalated to the HCC PTL at 4439 0013 whom shall guide any further action.
 - 6.11.3 Any failure of CareMonX CAD used on the Clinical Desk shall be managed in accordance with [SOP 3.18](#) as guided by the Oscar HCC.
 - 6.11.4 Where the failure of CareMonX CAD performance cannot be resolved within 15-minutes, Clinical Desk staff must notify:
 - 6.11.4.1 Consultant Critical Care Paramedic: Effectiveness
 - 6.11.4.2 SME: Clinical Desk
- 6.12 LifeNet failure:
 - 6.12.1 In the event of any LifeNet performance failure the user should attempt to restart the application and the workstation.
 - 6.12.2 Where the failure of LifeNet performance affects all Lifenet Alert stations on the Clinical Desk preventing transmission of ECGs to the Clinical Desk or to the receiving hospital, a Clinical Coordinator must immediately notify:
 - 6.12.2.1 Consultant Critical Care Paramedic: Effectiveness
 - 6.12.2.2 SME: Clinical Desk
 - 6.12.3 If 6.12.1 above does not resolve the issue and the 'popup' error message requests the user to "...start the LIFENET Alert user interface with administrator privileges..." contact HICT at 4439 1444 and follow the step-by-step guidance outlined in the "Guide for the resolution of Lifenet Alert Application 'Administrator Error'" (See Figure 1 below).
 - 6.12.4 If the troubleshooting steps in 6.12.1 and 6.12.2 do not resolve the issue it must be escalated to the SME: Clinical Desk directly for further guidance.
- 6.13 Skytrac or DaWinci failure:
 - 6.13.1 In the event of any inability to access the Skytrac or DaWinci web applications the LifeFlight Coordinator should restart their browser and/or workstation.
 - 6.13.2 If 6.13.1 above does not resolve the issue and web browsing access is working, it must be immediately escalated to Oscar LifeFlight on-call.
 - 6.13.2.1 If web browsing is not possible the LifeFlight Coordinator should contact the SME: Clinical Desk for further guidance.

6.13.3 Where the failure of Skytrac or DaWinci performance cannot be resolved within 15-minutes, the LifeFlight Coordinator must notify:

6.13.4 Operations Officer: LifeFlight

6.13.5 SME: Clinical Desk

7. Ending shift:

7.1 Staff must ensure all documentation and shift reports are completed for their shift and log-off of all workstations and systems before leaving their console at the end of every shift.

7.2 Off-going staff must complete social cleaning of their console, chair, keyboard and mouse before handing over to the on-coming staff.

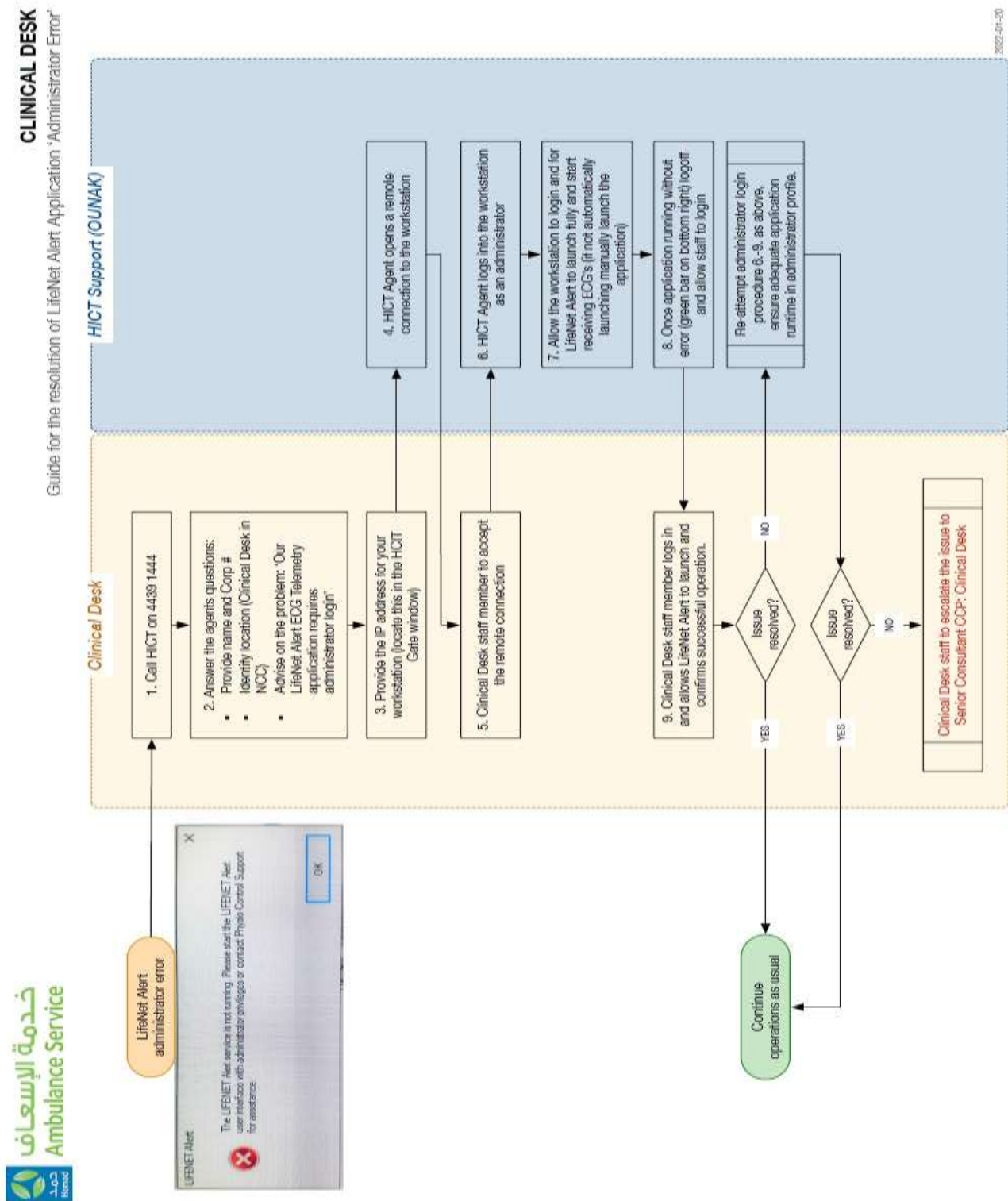
7.3 Off-going staff must provide an endorsement to the on-coming staff including, but not limited to:

7.3.1 Ongoing calls

7.3.2 Documentation of ongoing calls

7.3.3 Any major concerns or issues from their shift

Figure 1: "Guide for the resolution of Lifenet Alert Application 'Administrator Error'"



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services: Clinical Desk
Procedure	8.7 CLINICAL DESK PHONE CALL MANAGEMENT
Implementation Date	06 October 2020
Purpose	To set out the standards for the management of telephone systems and standardize the approach to the handling of inbound and outbound telephone calls of the Clinical Desk.
Responsibility	ALL STAFF ASSIGNED TO THE CLINICAL DESK

1. Background and Purpose:

- 1.1 The staff of the Clinical Desk handle incoming calls for case coordination or clinical consultation of both emergency and non-emergency calls, as well as conducting outbound calls for the same purposes. These calls include staff within the HMC Ambulance Service as well as external stakeholders. This SOP provides clarity and guidance to Clinical Desk staff on the standards for managing these calls as well as the use of the Cisco phone system.

2. Definitions applicable within this SOP:

- 2.1 **'Cisco telephone'** refers to the specific make of telephone used on the Clinical Desk for inbound and outbound calls.
- 2.2 **'Clinical Desk'** (the desk) means the division under the governance of Clinical Services that includes the Clinical Coordinator: Emergency, Clinical Coordinator: Retrieval, LifeFlight Coordinator, EMD: Retrievals Coordinators within the HMC Ambulance Services Control and Communications Center under the leadership of the SME: Clinical Desk.
- 2.3 **'IMIST-AMBO'** refers to the acronym used for patient endorsement as set-out in [SOP 4.8.B](#).
- 2.4 **'IVR'** (Interactive Voice Response), refers to the telephony technology that reads the available pre-programmed options to the caller, who can then select their desired choice on which the system then directs their call to the appropriate console.

3. General Provisions:

- 3.1 Clinical Desk staff shall only use the HMC Ambulance Service Cisco telephones for all incoming and outgoing calls.
- 3.2 All inbound and outbound calls of the Clinical Desk will be recorded for quality assurance and improvement purposes.
- 3.3 Clinical Desk staff must login to the Cisco Agent on their respective workstation and make their number available in the IVR immediately at the start of shift and should only log-off again at the end of shift.
- 3.4 Availability in the IVR must be monitored throughout the shift and staff should only set their number not ready (i.e. unavailable) for the following 'not ready reasons':
 - 3.4.1 Meeting or training – Set 'not ready reason' as "Meeting"
 - 3.4.2 Bathroom use - Set 'not ready reason' as "Personal"
 - 3.4.3 Short break - Set 'not ready reason' as "Break"
 - 3.4.4 Meal break - Set 'not ready reason' as "Meal"
 - 3.4.5 Prayer - Set 'not ready reason' as "Prayer Time"
 - 3.4.6 Leaving the console to coordinate a case directly in person with other persons in NCC - Set 'not ready reason' as "Standby"

- 3.5 EMD: Retrievals Coordinator shall continue to make themselves available and operate under the HCC (9 8555) IVR and adhere to [SOP 3.21](#) and [SOP 3.22](#).
 - 3.5.1 Where calls are routed to EMD: Retrievals Coordinator from the Clinical Desk IVR (9 8833) they should forward the call to an appropriate person as guided within this SOP.
 - 3.5.2 Where it is not possible to forward the call, all relevant information as per section 5 below must be obtained and the appropriate person immediately notified.
- 3.6 All inbound and outbound calls should be conducted using appropriate phone etiquette.
 - 3.6.1 Clinical Desk staff handling any inbound or outbound call should do so in a professional and courteous manner no matter the situation.
 - 3.6.2 Use appropriate language based on the caller's expectations, knowledge, language and cultural background.
 - 3.6.2.1 Use appropriate medical terminology when talking with another medical professional.
 - 3.6.2.2 Avoid words or phrases that may be viewed offensively or be ambiguous for the caller.
 - 3.6.2.3 Avoid sounding abrupt and impatient with the caller.
 - 3.6.3 Maintain an appropriate professional vocal tone and use clear concise communication.
 - 3.6.3.1 Avoid the impression that you are rushing a caller.
 - 3.6.3.2 Do not raise your voice in frustration or unnecessarily prolong a call.
 - 3.6.4 Maintain control of the conversation by politely and professionally directing the caller to answer questions you ask.
 - 3.6.5 Avoid asking a caller to repeat themselves by taking appropriate notes during the call.
 - 3.6.6 Confirm your understanding of information received by summarizing or paraphrasing this back to the caller for their confirmation.

4. Opening the Call:

- 4.1 Inbound calls must be answered as soon as available and should ideally be answered within 15 seconds.
- 4.2 Clinical Desk staff should answer all inbound calls professionally using the phrase, '____ [your role], how may I help you?' (e.g.: 'CC: Emergency, how may I help you?') as a guide.

5. Obtaining and Confirming Information:

- 5.1 Obtain the details and call back information for the caller, where necessary.
 - 5.1.1 Where the caller is ambulance service staff, ask the caller to provide their name and unit call sign.
 - 5.1.2 Where the caller is not ambulance service staff, ask the caller to provide their name, corporation number and contact number.
- 5.2 Obtain the patient's unique identification, such as their QID or HC number, and a secondary confirmation identifier, such as HC number, QID or name, whenever required.
- 5.3 Obtain an IMIST-AMBO endorsement as well as any additional appropriate and applicable clinical or case information.

- 5.3.1 Where a caller from HMC Ambulance Service endorses a patient in a way that does not meet the requirements of [SOP 4.8.B](#) or [SOP 8.8](#), as applicable, staff may politely direct the caller to provide an IMIST-AMBO endorsement.
- 5.4 In all cases, where a caller provides names, corporation numbers, contact information, locations or patient identifiers Clinical Desk staff must reconfirm they have understood and documented this correctly by repeating this back to the caller (closed loop communication).

6. Placing a Caller on Hold:

- 6.1 A caller should not be put on hold unless necessary to manage the case appropriately or resolve the query.
- 6.2 Where a caller is the treating clinician of a critically ill patient or is directly involved in managing a critical situation:
 - 6.2.1 The caller should not be put on hold.
 - 6.2.2 The details should be taken of the caller, the patient and the situation including an IMIST-AMBO report when appropriate and ask them to keep their phone nearby for your call-back. ('Thank you, I will follow this up now and will return your call shortly, please remain close to your phone.'))
 - 6.2.3 Immediately resolve any concerns or endorse the case to the appropriate person to resolve.
 - 6.2.4 Call the caller back to update them on their call and the actions taken, or to be taken.
- 6.3 When placing a caller on hold, ask if you can put them on hold, explain the reason for placing them on hold and advise them to remain on the line.
 - 6.3.1 Use the Hold/Mute function to place the caller on hold.
 - 6.3.2 Check in periodically with the caller when on hold if it is expected to take longer than 1-minute to resolve the concern.
 - 6.3.3 Callers should not be put on hold for longer than necessary.
- 6.4 If unable to place a caller on hold for any reason, follow the procedure as set out at 6.2.2 above.

7. Transferring of Calls:

- 7.1 Calls should only be transferred when resolving the call cannot be achieved without transferring the call.
- 7.2 Always inform the caller that you need to transfer their call and to whom and request them to remain on the line.
- 7.3 Before transferring any call:
 - 7.3.1 Confirm the person to which the call will be transferred is available and ready to receive the call.
 - 7.3.2 Confirm the caller's name and contact number in case the call is dropped during the transfer and you need to call them back.

8. Conferencing Calls:

- 8.1 When necessary, conference calls shall be conducted over the HMC Ambulance Service Cisco System.
- 8.2 Before conducting the conference call, staff must ensure they know all contact numbers and names of participants and are familiar with the objective of the call.

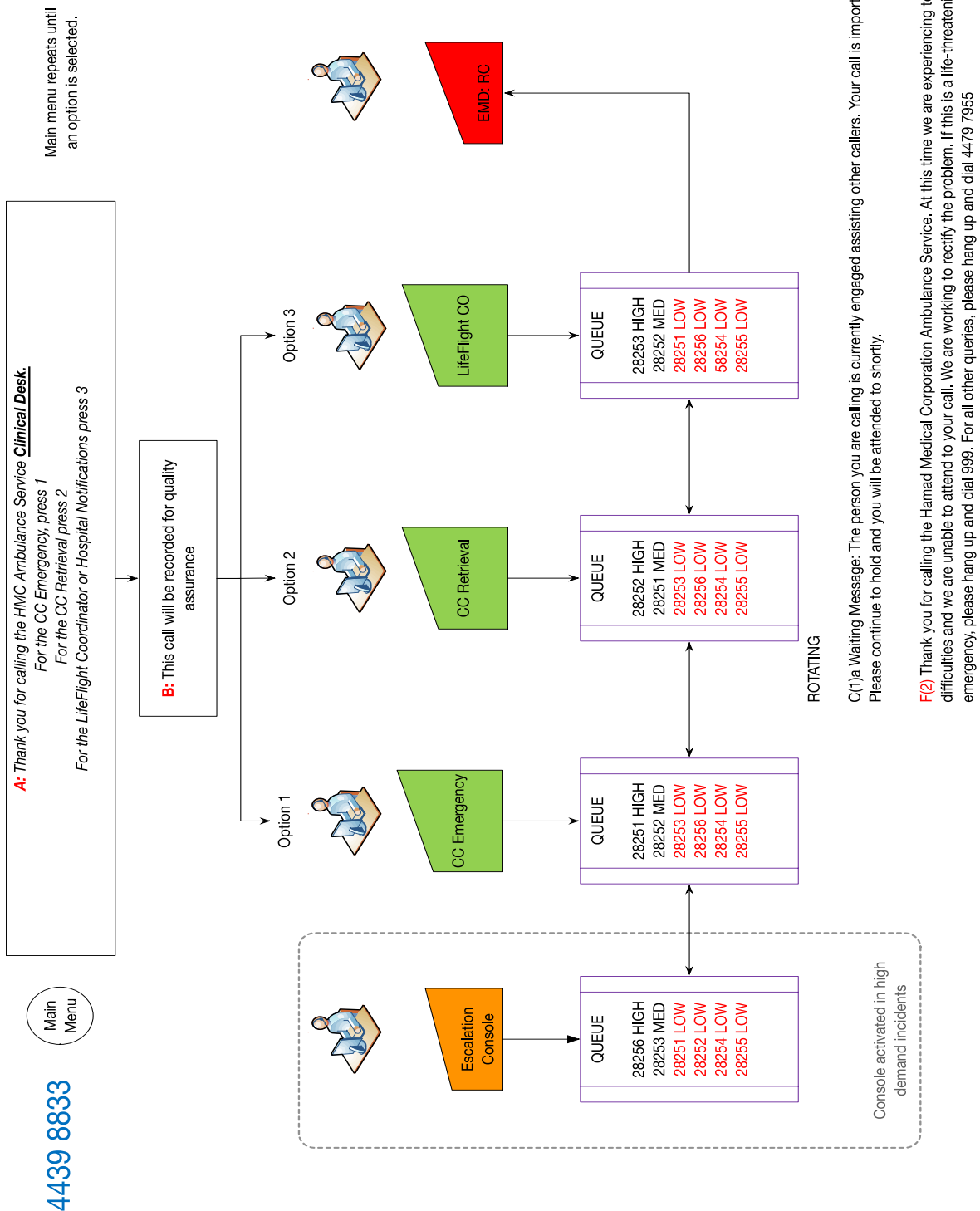
- 8.3 International conference calls will be facilitated by the EMD: Retrievals Coordinator using the approved phone for international dialing. Local conference calls can be facilitated through any HMC Cisco phone at the Clinical Desk.
- 8.4 During conference calls:
 - 8.4.1 Clinical Desk staff hosting/setting-up the conference call should open the call by confirming all callers are online and introducing callers to each other where necessary.
 - 8.4.2 Staff must remain attentive on the call, take adequate notes for documentation purposes and obtain all required information to prevent unnecessary follow-up/repeat calls.
 - 8.4.3 Use the mute function when not actively talking on the call to reduce background noise disrupting other speakers on the call.
 - 8.4.4 When engaged in a conference call, do not put the call on hold as this will put all callers on hold, use the mute function if required.
 - 8.4.5 On completion of the conference call the staff hosting/setting-up the call should close the call, after ensuring all discussion is completed, professionally. (*"Thank you all for participating in this discussion, I appreciate your time. Should there be any changes or updates we will be in further contact, Goodbye."*)

9. Closing the Call:

- 9.1 Clinical Desk staff must end all calls professionally regardless of who the caller is.
- 9.2 Calls shall be closed with a standard script or a reasonably similar appropriate script. (*"Thank you, please call us back if anything changes. Goodbye."*)

10. Clinical Desk IVR System:

- 10.1 Clinical Desk phones are managed with a call distribution system based on the caller's selection for preferred service offering.
- 10.2 Calls will be routed based on caller selection and, where the original selection is unavailable, re-routed based on predetermined call routing priorities.
- 10.3 Clinical Desk staff must ensure their phone is on and working, and that the Cisco Finesse application is logged-in throughout their shift.
- 10.4 Staff must be available in the Cisco Finesse System (IVR availability) at all times they are at their console. When leaving the console, or unavailable for any other reason, they must make themselves unavailable in accordance to 3.4 above.'



Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services Clinical Desk
Procedure	8.8 HOSPITAL PRE-ARRIVAL NOTIFICATION
Implementation Date	01 July 2022
Purpose	Establishes a standardized process and endorsement criteria for all hospital pre-arrival notifications by HMC Ambulance Service.
Responsibility	ALL PATIENT FACING AND CLINICAL DESK STAFF

1. Background and Purpose:

- 1.1 The pre-arrival notification of hospitals for the imminent arrival of pathway or priority 1 patients is necessary to ensure optimal and timely provision of critical care to patients and is a requirement under HMC Ambulance Service Policy.
- 1.2 This SOP aims to standardize the approach to the pre-arrival notification of hospitals including the endorsement from patient facing clinicians to the Clinical Desk and the notification of the receiving facility by the Clinical Desk.

2. Definitions applicable within this SOP:

- 2.1 **'Hospital Pre-arrival Notification'** means the intentional notification of the receiving hospital patient care unit of a patient being transported by HMC Ambulance Service to that unit, prior to the patient's arrival.

3. Requirements for pre-arrival notification:

- 3.1 It is the responsibility of the patient facing Critical Care Paramedic or Ambulance Paramedic to initiate the pre-arrival notification process by notifying the Clinical Desk as soon as reasonably possible.
- 3.2 Pre-arrival notification of receiving hospitals shall be conducted by any clinical staff assigned to the Clinical Desk including the CC: Emergency, CC: Retrieval or LifeFlight Coordinator.
 - 3.2.1 All pre-arrival notifications for LifeFlight cases should be done through the LifeFlight Coordinator unless there is a need for clinical guidance, or the LifeFlight Coordinator is unable to assist for any reason.
 - 3.2.2 All uncomplicated pre-arrival notifications from a treating CCP should be done through the LifeFlight Coordinator unless there is a need for clinical guidance, or the LifeFlight Coordinator is unable to assist for any reason.
 - 3.2.3 Any pre-arrival notification from an Ambulance Paramedic, or where the on-scene CCP requires additional clinical support beyond pre-arrival notification, should be done through the CC: Emergency.
 - 3.2.3.1 Where the CC: Emergency is unable to assist for any reason the CC: Retrieval should facilitate the pre-arrival notification.
- 3.3 Hospital pre-arrival notification must be done, from the Clinical Desk to the hospital, through the approved hospital pre-arrival notification 'hotlines'.
- 3.4 Notification of receiving facilities must be undertaken for any of the following patient groups being transported by HMC Ambulance Service:
 - 3.4.1 Any priority 1 patient regardless of gender or age.
 - 3.4.2 Any pathway patient including, but not limited to, Trauma Bypass, STEMI, CVA and WWRC.

- 3.4.3 Any Interfacility Transfer in which the patient has deteriorated, or any significant changes have been made to the clinical care of the patient.
- 3.4.4 Any other patient group or receiving facility as indicated in SOP, CPG or reasonable request by management.
- 3.5 Notification of the receiving facility shall be undertaken prior to the arrival of the patient at the receiving unit and all available clinical information endorsed to enable the receiving unit to facilitate a safe rapid transition of care.
- 3.6 Hospital pre-arrival notification does not constitute a request for the receiving facility to accept the patient. Unless the receiving facility is on divert, the facility is mandated to accept and receive the patient from the HMC Ambulance Service.

4. Standard endorsement information and structure:

- 4.1 The transporting Critical Care Paramedic or Ambulance Paramedic shall use the standard patient endorsement tool, IMIST-AMBO, in accordance with [SOP 4.8.B](#) when endorsing any patient to the Clinical Desk.
 - 4.1.1 Notification of the Clinical Desk should be through a recognized communication system including Clinical 1 tetra channel or the Clinical Desk phoneline (4439 8833).
 - 4.1.2 Where LifeFlight are unable to communicate on these communication lines due to reception challenges in flight, they may use MS Teams for this purpose.
 - 4.1.2.1 All notification messages on MS Teams must include all IMIST-AMBO information in a standard format.
 - 4.1.2.2 MS Teams notification messages must be received and acknowledged by the LifeFlight Coordinator at the Clinical Desk before the pre-arrival notification is made to the receiving hospital.
- 4.2 The Clinical Desk shall endorse the patient to the receiving hospital using the IMIST-AMBO approach in accordance with [SOP 4.8.B](#) providing the below minimum information for all cases.
 - 4.2.1 Age and Gender of the patient
 - 4.2.2 Chief Complaint or Mechanism of Injury
 - 4.2.3 Level of Consciousness
 - 4.2.4 Cardiorespiratory status
 - 4.2.5 Treatment provided and ongoing
 - 4.2.6 Expected time of arrival
- 4.3 This information (4.2) must be delivered using a standard IMIST-O and ETA bundle structure as a minimum for all cases, and where 'Allergies', 'Medication' or 'Background' are applicable to the specific case, this should also be endorsed.

5. Exceptional endorsement information:

- 5.1 Pre-arrival hospital notification should be done for all patients going to Heart Hospital, regardless of not meeting pre-arrival notification criteria set out in 3.4 above.
- 5.2 Pre-arrival hospital notification of Sidra Hospital for any patient being transported by LifeFlight must include clear notification that the patient is arriving by helicopter.

6. Pre-arrival notification process ([Appendix 8.8.1](#)):

- 6.1 The transporting Critical Care Paramedic or Ambulance Paramedic shall notify the Clinical Desk of all patients requiring pre-arrival notification within a reasonable timeframe to enable pre-arrival notification.
- 6.2 The CC: Emergency, CC: Retrieval or LifeFlight Coordinator shall receive and document the information per 4.3 above and any additional relevant information endorsed from the transporting unit using the approved documentation standard as stipulated by management.
- 6.3 The CC: Emergency, CC: Retrieval or LifeFlight Coordinator shall endorse the patient to the receiving hospital in a timely manner whilst adhering to set pre-arrival notification standards.
 - 6.3.1 All hospital pre-arrival notification calls must be opened professionally with introduction of the caller's name and reason for calling. (*"Good day, this is [your name] of the Clinical Desk, HMC Ambulance Services, may I endorse a patient?"*)
 - 6.3.2 The caller shall confirm they are speaking to the correct receiving facility and unit for all calls before patient endorsement.
 - 6.3.3 The caller shall request and document the receiving person's name and corporation number and note the time of the call.
 - 6.3.4 The caller shall provide the relevant patient and clinical information in accordance with 4.2 and 4.3 above, ensuring a minimum of "IMIST-O & ETA" to the receiving facility.
 - 6.3.5 The caller should advise the receiving facility that the estimated time of arrival is an estimation and is dependent on traffic and road conditions.
 - 6.3.6 The caller shall close the call professionally, document the notification and case information appropriately and, where necessary, advise the transporting unit of any further updates.
- 6.4 The caller shall advise the receiving or transporting unit of any changes to the patient's disposition, treatment or estimated time of arrival should this change after notification has been provided.
- 6.5 Notwithstanding 6.3.1 above, the management of telephone calls for the purposes of hospital pre-arrival notification should adhere to the provisions of [SOP 8.7](#).

7. Hospital refusal of patients during pre-arrival notification:

- 7.1 During pre-arrival notification of the preferred receiving facility, the doctor or nurse may refuse to accept the patient in their unit, referring the patient to another unit or hospital. Reasons for this may range from bed availability to triage concerns and are not within HMCAS control.
- 7.2 Where a pathway patient (STEMI, CVA, Trauma Bypass, WWRC) is being refused at a designated receiving facility or unit:
 - 7.2.1 The Clinical Coordinator or LifeFlight Coordinator doing the notification should advise the unit that the patient meets pathway criteria.
 - 7.2.2 If they continue to refuse or refer the patient, their refusal or referral should be reconfirmed and documented as a refusal in the CC Clinical Shift Report with the Corporation number of the doctor/nurse.
 - 7.2.3 The new receiving hospital or unit should be pre-notified as clinically appropriate.
 - 7.2.4 The Clinical Coordinator or LifeFlight Coordinator handling the case must update the transporting unit of the new disposition.
- 7.3 Where the patient being referred to an alternative facility or unit does not meet pathway criteria:

- 7.3.1 The refusal or referral should be reconfirmed and annotated in “Other” (notes section of the case) on the CC Clinical Shift Report with the Corporation number of the doctor/nurse.
- 7.3.2 The case should be reported under the relevant pathology classification/section in the CC Clinical Shift Report (i.e. do not select refusal).
- 7.3.3 The new receiving hospital or unit should be pre-notified as clinically appropriate.
- 7.3.4 The Clinical Coordinator or LifeFlight Coordinator handling the case must update the transporting unit of the new disposition.

Hamad Medical Corporation Ambulance Service Standard Operating Procedures

Sector	HMC Ambulance Services Clinical Desk
Procedure	8.9 TRANSFER ESCALATION TO THE CLINICAL DESK
Implementation Date	01 June 2022
Purpose	To set out the standard procedure for the escalation of transfers to the Clinical Desk from HCC and NCC with the intention to improve standardization and consistency in quality whilst mitigating unnecessary delays.
Responsibility	ALL STAFF ASSIGNED TO THE CLINICAL DESK, NCC AND HCC

1. Background and Purpose:

- 1.1 To facilitate safe patient transport by ambulance between healthcare facilities, high risk transfer cases need to be evaluated and triaged prior to dispatch of resources; thereby, ensuring that the patient receives the right level of care and transfer platform at the right time.

2. Definitions:

- 2.1 **'Clinical Privileging'** refers to the authorization levels determining what medical treatments HMCAS staff may provide based on their area of operations and clinical exposure as set out in the HMCAS Clinical Practice Guidelines.
- 2.2 **'HMCAS service offering'** refers to operational divisions within HMCAS that provides patient transfer including Charlie, LifeFlight, Retrieval, Interfacility Transfer (IFT) and Alpha.
- 2.3 **'Immediate Transfer'** means an inter-facility transfer of a pathway patient or any patient requiring an emergent intervention immediately on arrival at the receiving facility such as STEMI, Stroke, meeting Trauma Bypass Criteria, rescue PCI, immediate neurological intervention. Formerly known as "push, urgent, time critical" transfers.
- 2.4 **'Level of Care'** means the level of clinical care required to safely transfer a patient which is determined by the clinical condition and ongoing treatment needs of the patient, and clinical privileging levels of clinicians within HMCAS.
- 2.5 **'Scheduled Transfer'** means an inter-facility transfer of a stable patient for bed management, step-down care or for further clinical investigation and monitoring in the receiving facility with a negotiated pick-up time scheduled based on available resources.
 - 2.5.1 **'Scheduled Urgent Transfer'** means a scheduled inter-facility transfer of a stable patient that does not meet pathway criteria or require immediate limb or lifesaving treatment at the receiving facility; however, requires an upgrade in their level of clinical care within the following 2-hours to prevent clinical harm or deterioration.
- 2.6 **'Triage'** means the process of weighing the relevant available clinical and logistic information to decide on the appropriate urgency of transfer, level of care, appropriate HMCAS service offering and patient stability for transfer.

3. General Principles:

- 3.1 The purpose of escalation to the Clinical Desk is primarily for the confirmation of level of care where this is anticipated to exceed the clinical privileging of an Ambulance Paramedic.
- 3.2 Escalation of cases to the Clinical Desk should be closely monitored to prevent unreasonable delays in dispatch, especially where the patient is suspected to require immediate transfer. To minimize any delays case ownership is defined below, where the case owner remains responsible to track escalation and coordination progress until resources are dispatched.
 - 3.2.1 Calls received at the HCC will remain under HCC (UTL) oversight and coordination (ownership) until escalated to the Clinical Desk.

- 3.2.2 Calls received at the NCC will remain under NCC (DTL) oversight and coordination (ownership) until escalated to the Clinical Desk.
- 3.2.3 Cases escalated to the Clinical Desk will remain under the oversight and coordination (ownership) of the Clinical Desk (EMD: Retrievals Coordinator) until endorsed back to the HCC/NCC.

4. **Criteria for interfacility transfer escalation to the Clinical Desk:**

- 4.1 All interfacility transfer cases meeting the following criteria should be escalated to the Clinical Desk:
 - 4.1.1 Any Protocol 45 or Protocol 46 case that is indicated by ProQA for escalation.
 - 4.1.2 Any patient for which the referring facility requests a LifeFlight or helicopter response.
 - 4.1.3 Any patient for which the referring facility requests a level of care that is higher than an Ambulance Paramedic (CCP, Doctor or other specialized transport team).
 - 4.1.4 Any patient for which the referring facility requests specialized equipment for the transfer other than oxygen, ECG/heart monitor or suction.
 - 4.1.5 Any patient identified with clinical 'attachments' or 'drains' beyond the clinical privileging (scope of practice) of an Ambulance Paramedic.
 - 4.1.6 Where the call-taker/dispatcher has specific and relevant clinical concerns about the patient's clinical stability or level-of-care needs.

5. **NCC Escalation process:**

- 5.1 Cases identified for escalation to the Clinical Desk may be escalated directly to an available Clinical Coordinator at the Clinical Desk in NCC.
- 5.2 The call-taker/dispatcher escalating the case to the Clinical Desk must provide:
 - 5.2.1 NJM CFS number
 - 5.2.2 Patient identification in the form of a name, HC#, QID# or passport #.
 - 5.2.3 Referral facility call-back contact number
 - 5.2.4 Outline of the case including referring and receiving facility, and the chief complaint.
 - 5.2.5 Why the case is being escalated (the escalation trigger)
- 5.3 At the time of escalation, the call-taker/dispatcher escalating the case must add a CFS note indicating who the case was escalated to and why it was escalated.
- 5.4 Once the case has been triaged, the Clinical Coordinator must take necessary actions to update the DTL of the triage level of care, preferred HMCAS service offering and priority.
 - 5.4.1 If the case is triaged for immediate CCP transport the CC should update the DTL and advise the appropriate CCP resource required (Charlie/LifeFlight/Retrieval) and, where possible, the CC should update the responding CCP of the known information about the case.
 - 5.4.2 If the case is triaged for scheduled CCP transport, the CC should update the DTL and advise for the case to be handed over to EMD: Retrieval Coordinators to manage further following standard scheduled transfer procedures.
 - 5.4.3 If the case is triaged for Ambulance Paramedic transport, the CC must update the DTL accordingly.

6. HCC Escalation process:

- 6.1 Cases identified for escalation to the Clinical Desk must be escalated directly to the EMD: Retrievals Coordinator seated at the Clinical Desk in NCC.
- 6.2 The call-taker/dispatcher escalating the case to the Clinical Desk must provide:
 - 6.2.1 Valencia CAD trip number
 - 6.2.2 Outline of the case including referring and receiving facility, and the chief complaint.
 - 6.2.3 Why the case is being escalated (the escalation trigger)
- 6.3 Any escalation to the Clinical Desk must be documented in the relevant CAD notes:
 - 6.3.1 The call-taker/dispatcher escalating the case must add a Valencia CAD note, using note type "CD-Escalation", at the time of escalation noting "*Escalated to [name of person contacted]*".
 - 6.3.2 On receiving any escalation from the HCC, the EMD: Retrievals Coordinator must open the case in CAD and add a note "*Received from [name of escalating person]*", using note type "CD-Escalation".
- 6.4 The call-taker/dispatcher escalating the case should not transfer the caller to the EMD: Retrieval Coordinator who shall:
 - 6.4.1 Reconfirm the patient's name and ID from the Valencia CAD.
 - 6.4.2 **Where the caller is a PHCC**, The EMD: Retrievals Coordinator must obtain additional specific information as set out in the "PHCC Escalation Additional Questions" (see [Appendix 8.9.C](#)) and endorse the patient information to the Clinical Coordinator for them to advise on requirement to immediately dispatch a CCP unit.
 - 6.4.3 **Where the caller is an HMC hospital**, the EMD: Retrievals Coordinator should directly endorse the case to a Clinical Coordinator and, if possible, not transfer the caller to the Clinical Coordinator for direct clinical interrogation.
- 6.5 Once the case has been triaged, the EMD: Retrieval Coordinator must take necessary actions to, where appropriate, arrange for a CCP unit from the NCC DTL and update the escalating call-taker/dispatcher.
 - 6.5.1 If the case is triaged for Ambulance Paramedic transport, the EMD: Retrieval Coordinator must update the call-taker/dispatcher that escalated the case and, at the same time, add a note in CAD using the note type "CD-Closed". e.g.: "*Endorsed back to [NAME].*"
 - 6.5.2 If the case is triaged for immediate CCP transport, the EMD: Retrieval Coordinator must liaise with the DTL and, if LifeFlight is required the LifeFlight Coordinator, to get a CCP unit and update the call-taker/dispatcher of the CCP requirement.
 - 6.5.2.1 Once the CCP has changed onto the appropriate Tetra Radio channel the EMD: Retrieval Coordinator should add a note in CAD using the note type "CD-Closed". e.g.: "*CCP arranged, endorsed back to [NAME].*"
 - 6.5.3 If the case is triaged for scheduled CCP transport, the EMD: Retrieval Coordinator must update the call-taker/dispatcher that escalated the case and, at the same time, add a note in CAD using the note type "CD-Closed". e.g.: "*Endorsed back to [NAME].*"
 - 6.5.3.1 Any case triaged for scheduled CCP/Doctor transfer must be handed over to the EMD: Retrieval Coordinator to manage further.

7. Documentation and reporting:

- 7.1 Any case escalated to the Clinical Desk must be appropriately documented in the relevant NJM or Valencia System at the time of escalation. Additional notes with further clinical or operational details may be added as usual in the case notes.
- 7.2 Once feedback is provided following triage, a clear note must be added in the relevant NJM or Valencia System at the time of the feedback indicating the triage decision.
- 7.3 The Clinical Coordinator should ensure that full and adequately detailed clinical notes are added to the NJM or Valencia CAD system documenting the clinical screening findings, consultations and triage decision.
 - 7.3.1 Where the escalated case is a Health Centre case, the Clinical Coordinator is not required to add notes unless they have additional information not already reflected in the ProQA or case details.
- 7.4 EMD: Retrieval Coordinators should ensure that all CCP, doctor or team led transfers are reported in the Retrieval Activity Shift Report.

8. Appendices:

- 8.1 [Appendix 8.9.C: PHCC Escalation Additional Questions](#)

SOP: Appendix



APPENDIX 1.2-1 UNIFORM ENTITLEMENT

1. Standard Blue Uniform (see Table 1)

- 1.1 Patient Transport Responder (SAS)
- 1.2 Ambulance Paramedic (Alpha, IFT, CCA, SEM, Retrieval, Life Patrol and Scheduling)
- 1.3 Extended Blue Uniform (see Table 1.1) is given as:
 - 1.3.1 *Changes to the standard uniform, or
 - 1.3.2 **Additional item according to their duty requirements such as:
 - 1.3.2.1 Critical Care Assistant (rotating in Charlie, Life Flight & NCC)
 - 1.3.2.2 Specialized Emergency Management
 - 1.3.2.3 Retrieval Services
 - 1.3.2.4 Life Patrol

Table 1 Standard Blue Uniform

Item	Quantity	Eligibility
Tactical Paramedic Shirt (Blue)	4	Every 2 years
Trousers (Dark Navy)	4	Every 2 years
Safety Boots (ATAC)	1 pair	Every 2 years
Cap	1	Every 2 years/Optional
Arm Cover	2 pairs	Every 2 years/Optional
Head Scarf (Female Muslim staff)	4	Every 2 years
Tactical Belt	1	Every 4 years
Jacket	1	Every 4 years
Epaulettes	2 pairs	Every 2 years
Reflective Vest	1	Every 4 years / when required (with OVA)
Personal Issued Equipment:		
Helmet with accessories	1	Once per entire Service
Penlight	1	Every 4 years
Stethoscope	1	Once per entire Service
Scissors w/ Holster	1	Once per entire Service
Safety Eyewear	1	Every 4 years / when required (with OVA)
Rescue Gloves	1	Once per entire Service
Face Shield Visor	1	Once per entire Service



Table 1.1 Extended Blue Uniform

Critical Care Assistant (rotating in Charlie, Life Flight & NCC)

Item	Quantity	Eligibility
**Life Flight Suit	2	When required
**Life Flight Boots (HAIX)	1 pair	When required
**Life Flight Helmet	1	When required
**Additional items		



Ambulance Paramedic (Specialized Emergency Management)

Item	Quantity	Eligibility
*Tactical Paramedic Shirt (Blue)	2	Every 2 years
**MIR Polo	2	Every 2 years
*Change in quantity **Additional items		



Ambulance Paramedic (Retrieval Services)

Item	Quantity	Eligibility
**Polo Shirt (Dark Navy)	2	Every 2 years / when required
**Trousers (Khaki)	2	Every 2 years / when required
**Additional items for Staff on Official Mission. Note: Polo Shirt without badges or logo.		



Ambulance Paramedic (Life Patrol)

Item	Quantity	Eligibility
*Tactical Paramedic Shirt (Blue)	1	Every 2 years
*Trousers (Dark Navy)	1	Every 2 years
**Life Patrol Polo	4	Every 2 years
**Life Patrol Pants	4	Every 2 years
**Life Patrol Shoes	1 pair	Every 2 years
**Life Patrol Gloves	1	Every 2 years
**Life Patrol Arm Cover	2 pairs	Every 2 years
**Bike Helmet	1	When Required
*Changes in quantity **Additional items		



2. Standard White Uniform (see Table 2)

2.1 Senior Supervisor (Delta, Romeo, Sierra, Instructor)

2.2 Critical Care Paramedic and Operations Officer

2.3 Operations Manager and Senior Leaders

2.4 Extended White Uniform (see Table 2.1) is given as:

2.4.1 *Changes to the standard uniform, or

2.4.2 **Additional item according to their duty requirements such as:

2.4.2.1 Critical Care Paramedic (rotating in Charlie, Life Flight & NCC)

2.4.2.2 Retrieval Services

Table 2 Standard White Uniform

Item	Quantity	Eligibility
Tactical Paramedic Shirt (White)	5/2	5 shirts initial issuance / 2 shirts after 2 years or when required (with OVA)
Trousers (Dark Navy)	4/2	4 trousers initial issuance/ 2 trousers after 2 years or when required (with OVA)
Safety Boots (ATAC)	1 pair	Every 2 years
Cap	1	Every 2 years/Optional
Arm Cover	2 pairs	Every 2 years/Optional
Head Scarf (Female Muslim staff)	4	Every 2 years
Tactical Belt	1	Every 4 years
Jacket	1	Every 4 years
Epaulettes	2 pairs	Every 2 years
Reflective Vest	1	Every 4 years/ when required (with OVA)
Personal Issued Equipment:		
Helmet with accessories	1	Once per entire Service
Penlight	1	Every 4 years
Stethoscope	1	Once per entire Service
Scissors w/ Holster	1	Once per entire Service
Safety Eyewear	1	Every 4 years / when required (with OVA)
Rescue Gloves	1	Once per entire Service
Face Shield Visor	1	Once per entire Service



Table 2.1 Extended White Uniform

Critical Care Paramedic (rotating in Charlie, Life Flight & NCC)			
Item	Quantity	Eligibility	
**Life Flight Suit	2	When required	
**Life Flight Boots (HAIX)	1 pair	When required	
**Life Flight Helmet	1	When required	
**Additional items			
Critical Care Paramedic (Retrieval Services)			
Item	Quantity	Eligibility	
**Polo Shirt (Dark Navy)	2	Every 2 years / when required	
**Trousers (Khaki)	2	Every 2 years / when required	
**Additional items for Staff on Official Mission. Note: Polo Shirt <u>without</u> badges or logo.			

3. Medical Dispatcher Standard Uniform (NCC and HCC) (Table 3)

- 3.1 Medical Dispatcher, Call Taker
- 3.2 Senior Supervisor, Operations Officer, Operations Manager and Senior Leaders
- 3.3 Medical Dispatcher Extended Uniform (Table 3.1) is given as:
 - 3.3.1 *Changes to the standard uniform, or
 - 3.3.2 **Additional item according to their duty requirements such as:
 - 3.3.2.1 Mobile Command Unit

Table 3 Medical Dispatcher Standard Uniform (NCC and HCC)

Item	Quantity	Eligibility	
Tactical Paramedic Shirt (White) (HCS logo)	4	Every 2 years	
Trousers (Dark Navy)	4	Every 2 years	
Trainer Shoes (ABR)	1 pair	Every 2 years	
Cap	1	Every 2 years/Optional	
Head Scarf (Female Muslim staff)	4	Every 2 years	
Tactical Belt	1	Every 4 years	
Jacket	1	Every 4 years	
Epaulettes	2 pairs	Every 2 years	

Table 3.1 Medical Dispatcher Extended Uniform (NCC and HCC)

Mobile Command Unit

Item	Quantity	Eligibility
**Safety Boots (ATAC)	1 pair	Every 2 years
**Reflective Vest	1	Every 4 years / when required (with OVA)
**Additional items		

4. Support Service Standard Uniform (see Table 4)

4.1 Support Service Staff

4.2 Senior Supervisor, Operations Officer, Operations Manager and Senior Leaders

4.3 Support Service Extended Uniform (see Table 4.1) is given as:

4.3.1 *Changes to the standard uniform, or

4.3.2 **Additional item according to their duty requirements such as:

4.3.2.1 Staff rotating in Ambulance Clinical Shift

Table 4 Support Service Standard Uniform

Item	Quantity	Eligibility
Support Staff Polo (Dark Navy)	5	Every 2 years
Trousers (Dark Navy)	4	Every 2 years
Safety Boots (ATAC) – Clinical Shift & Warehouse Staff	1 pair	Every 2 years
Trainer Shoes (ABR) – Admin duties		
Cap	1	Every 2 years/Optional
Arm Cover	2 pairs	Every 2 years/Optional
Head Scarf (Female Muslim staff)	4	Every 2 years
Tactical Belt	1	Every 4 years
Jacket	1	Every 4 years
Personal Issued Equipment (Non-Clinical Staff):		
Safety Eyewear	1	Every 4 years / when required (with OVA)
Face Shield Visor	1	Once per entire Service
Personal Issued Equipment (Clinical Staff):		
Helmet with accessories	1	Once per entire Service
Penlight	1	Every 4 years
Stethoscope	1	Once per entire Service
Scissors w/ Holster	1	Once per entire Service
Safety Eyewear	1	Every 4 years / when required (with OVA)
Rescue Gloves	1	Once per entire Service
Face Shield Visor	1	Once per entire Service



Table 4.1 Support Service Extended Uniform

Item	Quantity	Eligibility	
Ambulance Paramedic in Support Service			
**Tactical Paramedic Shirt (Blue)	2	Every 2 years	
**Additional items			
Senior Supervisor, Operations Officer and Senior Leaders (Non-Clinical)			
*Trousers (Khaki)	4	Every 2 years	
*Change in trousers colour			
Senior Supervisor, Operations Officer and Senior Leaders (Clinical)			
*Trousers (Khaki)	3	Every 2 years	
*Trousers (Dark Navy)	1	Every 2 years	
**Tactical Paramedic Shirt (White)	1	Every 2 years	
**Additional items			
*Change in quantity			

5. Senior Leaders with Major Incident Response Role (see Table 5)

Table 5

Item	Quantity	Eligibility
Senior Leaders with Major Incident Response Role		
Trousers (Khaki)	2	Every 2 years
Polo Shirt (Dark Navy) Ambulance Service badges	2	Every 2 years
Safety Boots (ATAC) – Clinical shift	1 pair	Every 2 years
Trainer Shoes (ABR) – Admin duties	1 pair	Every 2 years
Tactical Belt	1	Every 4 years
Jacket	1	Every 4 years
Reflective Vest	1	Every 4 years
Face Shield Visor	1	Once per entire Service



6. Admin Personnel (see Table 7)

6.1 Project Coordinator, Executive Secretary, Secretary, Office Clerks, and other office personnel

6.1.1 Staff may wear clothing that is appropriate to the nature of their work. This will usually be business attire (suit, shirt, and tie, Qatari national dress, or female equivalent).

6.1.2 Upon approval of employee's Executive Director, below list of uniform will be provided.

Table 7 Admin Personnel Uniform (upon ED's approval)

Item	Quantity	Eligibility	
Polo Shirt (Dark Navy) - Ambulance Service badges	5/2	5 shirts initial issuance/ 2 shirts after 2 years or when required (with OVA)	
Trousers (Dark Navy)	4/2	4 trousers initial issuance/ 2 trousers after 2 years or when required (with OVA)	
Trainer Shoes (ABR)	1 pair	Every 2 years	
Head Scarf (Female Muslim staff)	4	Every 2 years	
Tactical Belt	1	Every 4 years	

APPENDIX 1.2-2 HMCAS RANK INSIGNIA

Ambulance Service Rank Insignia			
Designation	Epaulette	Helmet Markings	
		Operations	Production
Chief Executive Officer			
Executive Director / Medical Director			
Assistant Executive Director			
Senior Operations Manager / Senior Production Manager			
Operations Manager			
Operations Officer			
Operations Supervisor I / Production Supervisor			
Operations Supervisor II			
Operations Supervisor III			
Ambulance Paramedic Educator			
Ambulance Paramedic			
Patient Transport Responder			
Case Controller / Mentor			
Dispatcher and Call Taker			
Ambulance Service Medical Dispatcher (ASMD) Student			
Ambulance Attendant			
Helmet Colors			
Access and Effectiveness Officers			
Access and Effectiveness Staff			
Production Officers and Staff			

APPENDIX 1.2-3 WASHING INSTRUCTIONS

Washing Instructions

Taclite® Pro Pant Style# 74273	- Wash Inside Out - Machine Wash with Cold Water - Wash Dark Color Separately - Use Mild Detergent - Do Not Bleach - Do Not Use Fabric Softener - Tumble Dry Low - Warm Iron, If Needed - Do Not Dry Clean
Performance Short Sleeve Polo Style# 71049	- Machine Wash with Cold Water - Wash Dark Color Separately - Use Mild Detergent - Do Not Bleach - Do Not Use Fabric Softener - Tumble Dry Low - Do Not Iron - Do Not Dry Clean
Uniform Hat (Ball Cap) Style # 89260	- Hand Wash Only
Tactical Fleece Jacket Style# 48038	- Machine Wash with Cold Water - Wash Dark Colors Separately - Use Mild Detergent - Do Not Bleach - Tumble Dry Low - Do Not Iron - Do Not Dry Clean
Station Non-NFPA Class-A Short Sleeve Shirt Style# 46122	- Wash Inside Out - Machine Wash with Cold water - Wash Dark Colors Separately - Use Mild Detergent - Do Not Bleach - Do Not Use Fabric Softener - Tumble Dry Low - Warm Iron - Do Not Dry Clean

APPENDIX 1.2-4 UNIFORM REQUEST FORM

Uniform Request Form

خدمة الإسعاف
Ambulance Service



Name:	Department:
Corp No:	Mobile No:

Item Description	Type	Size	Quantity
Tactical Shirt			
Tactical Pants			
Shoes			
Tactical Belt			
Jacket			
Polo Shirt			
Cap			
Epaulette			
Female Head Scarf			
For Life Patrol Staff			
Life Patrol Shirt			
Life Patrol Pants			
Ranger Shoes			
Bike Helmet			
Arm Cover			
Personal Issued Equipment			
Protective Helmet			
Stethoscope			
Scissors			
Penlight			
Safety Eyewear			
Rescue Gloves			
Reflective Vest			
Life Flight Suit			

Uniform Section
Contact No: +974 55230313
E-mail: SS-UNIFORM@HAMAD.QA

APPENDIX 1.2-5 END OF SERVICE UNIFORM CHECKLIST



خدمة الإسعاف
Ambulance Service



مؤسسة حمد الطبية
Hamad Medical Corporation
HEALTH - EDUCATION - RESEARCH صحة - تعليم - بحوث

End of Service Uniform Checklist

Name of Staff: _____ Corp. no. _____

Section: _____ Mobile no. _____

Uniform	Size	Quantity	Remarks
Helmet	XXXXX	1	
Reflective Vest	XXXXX	1	
Stethoscope	XXXXX	1	
Scissor w/Holster	XXXXX	1	
Penlight	XXXXX	1	
Safety Gloves	XXXXX	1	
Life Flight Suit			
Bike Helmet			
Rescue Gloves	XXXXX		
Others:	** New Items Requested within 6 months		
Polo Shirt			
Trouser			
Shoes/Boots			
Belt			
Jacket			
Cap			

Staff Support Supervisor: _____ Signature: _____
Date: _____

Uniform Section
Contact No: +974 55230313
E-mail: SS-UNIFORM@HAMAD.QA

APPENDIX 3.14 LIST OF HEALTH FACILITIES TO BE PROCESSED WITH MPDS PROTOCOLS 33 & 37 (NCC COMMUNICATIONS ONLY)



List of Health Facilities to be Processed with MPDS Protocols 33 & 37 (For NCC Communications only)

HOSPITALS TO BE HANDLED BY HCC				
But if obliged to accept at NCC should be processed with Protocol 37				
Al Khor Hospital (AK) مستشفى الخور	Al Rayyan Pediatric Emergency Center (PEC Al Rayaan) مركز الريان لطوارئ الأطفال	Al Saad Pediatric Emergency Center (PEC Al Sadd) مركز السد لطوارئ الأطفال	Al Wakrah Hospital (AW) مستشفى الوكرة	Hamad General Hospital (HG) مستشفى حمد العام
Hazm Mebareek General Hospital (HMGH) مستشفى حزم ميربك العام	Heart Hospital (HH) مستشفى القلب	Sidra Medicine سدرة للطب	The Cuban Hospital (CH) المستشفى الكوبي	Women's Wellness and Research Center (WWRC) مركز صحة المرأة والأبحاث
HOSPITALS TO BE PROCESSED WITH PROTOCOL 33				
HMC Facilities				
Aisha Bint Hamad Al Attiyah Hospital مستشفى عائشة بنت حمد العتيبة	Al Maha Pediatric Specialized Care Center مركز المها للرعاية التخصصية للأطفال	Ambulatory Care Center (ACC) مركز الرعاية الطبية اليومية	Communicable Disease Center (CDC) مركز الأمراض الانتقالية	
HMC Staff Medical Center المركز الطبي للموظفين	Mesaieed General Hospital مستشفى مسعيد العام		National Center for Cancer Care and Research (NCCCR) المركز الوطني لعلاج وأبحاث السرطان	
RH Psychiatry Department "Rawdat Al Khail" قسم الطب النفسي التابع لمستشفى الرميلة	Qatar Rehabilitation Institute (QRI) مركز قطر لإعادة التأهيل		Rumailah Hospital (RH) مستشفى الرميلة	
HMC Pediatric Emergency Centers				
Airport Pediatric Emergency Center (PEC Airport) مركز المطار لطوارئ الأطفال	Al Daayen Pediatric Emergency Center (PEC Al Daayen) مركز الظعنين لطوارئ الأطفال		Al Shamal Pediatric Emergency Center (PEC Al Shamal) مركز الشمال لطوارئ الأطفال	
HMC Dialysis Services				
Al Khor Hospital Dialysis Unit وحدة مستشفى الخور لغسيل الكلى	Al Shahania Dialysis Unit وحدة الشحانية لغسيل الكلى	Al Shamal Dialysis Unit وحدة الشمال لغسيل الكلى	Al Wakra Dialysis Unit وحدة الوكرة لغسيل الكلى	Fahad Bin Jassim Kidney Center مركز فهد بن جاسم لغسيل الكلى
HMC-Others				
Field Hospital Old Industrial Area (FHOIA) المستشفى الميداني للمنطقة الصناعية	Industrial Area Satellite Emergency Department (IASSED) مركز طوارئ المنطقة الصناعية		Ras Laffan Satellite Emergency Department مركز طوارئ رأس لفان	
PRIMARY HEALTHCARE CORPORATION (PHCC) HEALTH CENTERS				
TO BE HANDLED BY HCC				
But if obliged to accept at NCC should be processed with Protocol 33				
PHCC Health Centers-All				
Abu Baker Al-Siddiq HC مركز أبو بكر الصديق الصحي	Abu Nakhla HC مركز أبو نخلة الصحي	Airport HC مركز المطار الصحي	Al Daayen HC مركز الظعنين الصحي	
Al Jumiliya HC مركز الجميلية الصحي	Al Kaaban HC مركز الكعبان الصحي	Al Karaana HC مركز الكرعانة الصحي	Al Khor HC مركز الخور الصحي	
Al Mashaf HC مركز المشاف الصحي	Al Rayyan HC مركز الريان الصحي	Al Ruwais HC مركز الرويس الصحي	Al Sadd HC مركز السد الصحي	
Al Sheehaniya HC مركز الشهبانية الصحي	Al Thumama HC مركز الثمامة الصحي	Al Waab HC مركز الوعب الصحي	Al Wajbah HC مركز الوجبة الصحي	Al Wakra HC مركز الوكرة الصحي
Gharrafat Al Rayyan HC مركز غرافة الريان الصحي	Leabaib HC مركز لعبيب الصحي	Leghwairiya HC مركز لغويرية الصحي	Madinat Khalifa HC مركز مدينة خليفة الصحي	

Mesaimeer HC مركز مسيمير الصحي	Muaither HC مركز معيذر الصحي	Omar Bin Khatab HC مركز عمر بن الخطاب الصحي	Qatar University HC مركز جامعة قطر الصحي	Rawdat Alkhail HC مركز روضة الخيل الصحي		
South Wakra HC مركز جنوب الوكرة الصحي	Umm Al Seneem HC مركز ام السنييم الصحي	Umm Ghuwailina HC مركز أم غويلبنة الصحي	Umm Slal HC مركز أم صلال الصحي	West Bay HC مركز الخليج الغربي الصحي		
Private Hospitals						
Al Emadi Hospital مستشفى العمادي	Al-Ahli Hospital المستشفى الأهلي	Alfardan Medical with Northwestern Medicine (AMNM)	Aman Hospital مستشفى الأمان	American Hospital المستشفى الأمريكي		
Doha Clinic Hospital مستشفى عيادة الدوحة	Marble Medical Center "Lusail" مركز ماربل الطبي	The View Hospital مستشفى "ذا فيو"	Turkish Hospital المستشفى التركي			
QatarEnergy Clinics (formerly Qatar Petroleum "QP")						
Dukhan Medical Center (DMC) مركز دخان الطبي	Mesaieed Medical Center (MMC) مركز مسيعيد الطبي	Qatar Energy Health Center "QatarEnergy Headquarters" مركز قطر للطاقة الصحي "قطر للطاقة المقر الرئيسي"	Ras Laffan Al Madinah Health Center مركز رأس لفان المدينة الصحي	Ras Laffan Medical Center (RLMC) مركز رأس لفان الصحي		
Qatar Red Crescent (QRC) Clinics						
QRC Fereej Abdel Aziz Health Center مركز فريج عبد العزيز الصحي الهلال الأحمر القطري	QRC Al Hemaila Health Center مركز الحميلة الصحي الهلال الأحمر القطري	QRC Workers Health Center, Mesaimeer مركز العمال الصحي، مسيمير الهلال الأحمر القطري	QRC Workers' Health Center Zekreeth مركز العمال الصحي، زكريت الهلال الأحمر القطري			
Military Hospitals/Medical Centers						
Al Attar for Physical Medicine, Rehabilitation and Sports Medicine المختار للطب الفيزيائي، وإعادة التأهيل و الطب الرياضي	Military Medical City Hospital (MMCH) مستشفى المدينة الطبية العسكرية		Muqdam Camp Health Center مركز الخدمات الطبية، معسكر مقدم			
Others						
Abu Samra Border Clinic عيادة منفذ أبو سمرة	Al Udeid Air Base Clinic عيادة قاعدة العديد الجوية	Aspetar Hospital مستشفى سبتار	HIA Medical Centre (Departures Hall) مطار حمد الدولي، المركز الطبي (صالة المغادرون)	HIA Medical Centre (Arrivals Hall) مطار حمد الدولي، المركز الطبي (صالة القادمون)	Mesaieed Hospital "Including (OPD)" مستشفى مسيعيد "يشمل العيادات الخارجية"	Ras Laffan Hospital مستشفى رأس لفان
EXEMPTION IN USING PROTOCOL 33 & 37						
All other non classified private clinics جميع المراكز الطبية الخاصة الغير مدرجة بالقائمة	All Outpatient Departments (OPDs) جميع العيادات الخارجية	Enaya Continuing Care Center "Muaidher" مركز الرعاية المستمرة عناية	Enaya Specialized Care Center "Hamad Medical City" مركز الرعاية التخصصية عناية	Qatar National Blood Donation Center مركز قطر الوطني للتبرع بالدم	HMC Bone and Joint Center مركز العظام والمفاصل	Qatar Metabolic Institute (QMI) "Hamad Medical City" معهد قطر للتمثيل الغذائي

List of Health Facilities to be Processed with MPDS Protocols 33 & 37-Sorted by categories (For NCC Communications only)	
ENGLISH	ARABIC
I. HOSPITALS TO BE HANDLED BY HCC	
But if obliged to accept at NCC should be processed with Protocol 37	
Al Khor Hospital (AK)	مستشفى الخور
Al Rayyan Pediatric Emergency Center (PEC Al Raya'an)	مركز الريان لطوارئ الأطفال
AL Saad Pediatric Emergency Center (PEC Al Sadd)	مركز السد لطوارئ الأطفال
Al Wakrah Hospital (AW)	مستشفى الوكرة
Hamad General Hospital (HG)	مستشفى حمد العام
Hazm Mubareek General Hospital (HMGH)	مستشفى حزم مبارك العام
Heart Hospital (HH)	مستشفى القلب
Sidra Medicine	سيدة للطب
The Cuban Hospital (CH)	المستشفى الكوبي
Women's Wellness and Research Center (WWRC)	مركز صحة المرأة والأبحاث
II. LIST OF CLINICS TO BE PROCESSED WITH MPDS PROTOCOL 33	
A. HMC Facilities	
a. Facilities	
Aisha Bint Hamad Al Attiyah Hospital	مستشفى عائشة بنت حمد العتيبة
Al Maha Pediatric Specialized Care Center	مركز المها للرعاية التخصصية للأطفال
Ambulatory Care Center (ACC)	مركز الرعاية الطبية اليومية
Communicable Disease Center (CDC)	مركز الأمراض الانتقالية
HMC Staff Medical Center	المركز الطبي للموظفين
Mesaieed General Hospital	مستشفى مسيعيد العام
National Center for Cancer Care and Research (NCCCR)	المركز الوطني لعلاج وأبحاث السرطان
Psychiatry Department (RH)"Rawdat Al Khail"	قسم الطب النفسي التابع لمستشفى الرميلة "روضة الخيل"
Qatar Rehabilitation Institute (QRI)	مركز قطر لإعادة التأهيل
Rumailah Hospital (RH)	مستشفى الرميلة
b. Pediatric Emergency Centers	
Airport Pediatric Emergency Center (PEC Airport)	مركز المطار لطوارئ الأطفال
Al Daayen Pediatric Emergency Center (PEC Al Daayen)	مركز الظعابين لطوارئ الأطفال
Al Shamal Pediatric Emergency Center (PEC Al Shamal)	مركز الشمال لطوارئ الأطفال
c. Dialysis Services	
Al Khor Hospital Dialysis Unit	وحدة مستشفى الخور لغسيل الكلى
Al Shahania Dialysis Unit	وحدة الشحانية لغسيل الكلى
Al Shamal Dialysis Unit	وحدة الشمال لغسيل الكلى
Al Wakra Dialysis Unit	وحدة الوكرة لغسيل الكلى
Fahad Bin Jassim Kidney Center	مركز فهد بن جاسم لغسيل الكلى
d. Others	
Field Hospital Old Industrial Area (FHOIA)	المستشفى الميداني للمنطقة الصناعية
Industrial Area Satellite Emergency Department (IASSED)	مركز طوارئ المنطقة الصناعية
Ras Laffan Satellite Emergency Department	مركز طوارئ رأس لفان
B. PRIMARY HEALTHCARE CORPORATION (PHCC) HEALTH CENTERS TO BE HANDLED BY HCC	
But if obliged to accept at NCC should be processed with Protocol 33	
Abu Baker Al Siddiq HC	مركز أبو بكر آل صديق
Abu Nakhla HC	مركز ابونخلة الصحي
Airport HC	مركز المطار الصحي
Al Daayen HC	مركز الظعابين الصحي
Al Jumailiya HC	مركز الجميلية الصحي
Al Kaaban HC	مركز الكعبان الصحي
Al Karaana HC	مركز الكرعانة الصحي
Al Khor HC	مركز الخور الصحي
Al Mashaf HC	مركز المشاف الصحي
Al Rayyan HC	مركز الريان الصحي
Al Ruwais HC	مركز الرويس الصحي
Al Sadd HC	مركز السد الصحي
Al Sheehaniya HC	مركز الشحانية الصحي
Al Thumama HC	مركز الثمامة الصحي
Al Waab HC	مركز الوعب الصحي

Al Wajbah HC	مركز الوجبة الصحي
Al Wakra HC	مركز الوكرة الصحي
Gharrafat Al Rayyan HC	مركز الغرافة الصحي
Leabaib HC	مركز لعيبب الصحي
Al Leghwairiya HC	مركز الغويرية الصحي
Madinat Khalifa HC	مركز مدينة خليفة الصحي
Mesaimeer HC	مركز مسيمير الصحي
Muaither HC	مركز معيدر الصحي
Omar Bin Al Khattab HC	مركز عمر بن الخطاب الصحي
Qatar University HC	مركز جامعة قطر الصحي
Rawdat Al Khail HC	مركز روضة الخيل الصحي
South Wakra HC	مركز جنوب الوكرة الصحي
Umm Al Seneem HC	مركز ام السنيم الصحي
Umm Ghuwailina HC	مركز ام غويلينا الصحي
Umm Slal HC	مركز ام صلال الصحي
West Bay HC	مركز الخليج الغربي الصحي
C. Private Clinics	
Al Emadi Hospital	مستشفى العمادي
Al-Ahli Hospital	المستشفى الأهلي
Alfardan Medical with Northwestern Medicine (AMNM)	الفردان الطبية و نورث وسترن مديس
Aman Hospital	مستشفى الأمان
American Hospital	المستشفى الأمريكي
Doha Clinic Hospital	مستشفى عيادة الدوحة
Marble Medical Center "Lusail"	مركز ماربل الطبي
The View Hospital	مستشفى "ذا فيو"
Turkish Hospital	المستشفى التركي
D. QatarEnergy Clinics (formerly Qatar Petroleum "QP")	
Dukhan Medical Center (DMC)	مركز دخان الطبي
Mesaieed Medical Center (MMC)	مركز مسيعيد الطبي
Qatar Energy Health Center "QatarEnergy Headquarters"	مركز قطر للطاقة الصحي "المقر الرئيسي"
Ras Laffan Al Madinah Health Center	مركز رأس لفان المدينة الصحي
Ras Laffan Medical Center (RLMC)	مركز رأس لفان الصحي
E. Qatar Red Crescent Clinics	
QRC Fereej Abdel Aziz Health Center	مركز فريج عبد العزيز الصحي، الهلال الأحمر القطري
QRC Workers Health Center, Al Hemaila	مركز العمال الصحي الحميلة، الهلال الأحمر القطري
QRC Workers Health Center, Mesaimeer	مركز العمال الصحي مسيمير، الهلال الأحمر القطري
QRC Zikrit Health Center	مركز زكريت الصحي
F. Military Hospitals/Medical Centers	
Al Attar for Physical Medicine, Rehabilitation and Sports Medicine	العتار للطب الفيزيائي، وإعادة التأهيل و الطب الرياضي
Military Medical City Hospital (MMCH)	مستشفى المدينة الطبية العسكرية
Muqdam Camp Health Center	مركز الخدمات الطبية، معسكر مقدم
G. Others	
Abu Samra Border Clinic	عيادة منفذ أبو سمرة
Al Udeid Air Base Clinic	عيادة قاعدة العديد الجوية
Aspetar Hospital	مستشفى سبيتار
HIA Medical Centre (Departures Hall)	مطار حمد الدولي، المركز الطبي (صالة المغادرون)
HIA Medical Centre(Arrivals Hall)	مطار حمد الدولي، المركز الطبي (صالة القادمون)
Mesaieed Hospital "Including (OPD)"	مستشفى مسيعيد "يشمل العيادات الخارجية"
Ras Laffan Hospital	مستشفى رأس لفان
III. EXEMPTION IN USING PROTOCOL 33 & 37	
All other non classified private clinics	جميع المراكز الطبية الخاصة الغير مدرجة بالقائمة
All Outpatient Departments (OPDs)	جميع العيادات الخارجية
Enaya Continuing Care Center "Muaither"	مركز الرعاية المستمرة عناية
Enaya Specialized Care Center "Hamad Medical City"	مركز الرعاية التخصصية عناية
HMC Blood Donor Center	مركز التبرع بالدم - مؤسسة حمد الطبية
HMC Bone and Joint Center "Old OPD Annex"	مركز العظام والمفاصل
Qatar Metabolic Institute (QMI)	معهد قطر للتمثيل الغذائي
COLOR LEGEND	
Facilities not available in the old list	

**List of Health Facilities to be Processed with MTPS Protocols 45, 46 & 47-Sorted by categories
(For HCC Communications only)**

1. LIST OF HEALTH FACILITIES TO BE PROCESSED WITH MTPS: P-45: SPECIALIZED UNSCHEDULED UP CARE TRANSPORTS

1.1 To be used when a Patient is being transported to a higher level of care for evaluation or stabilization of an acute or chronic problem.

1.2 Examples:

- A patient transfer from HGH-ED to HH-Cath lab
- A patient transfer from PEC Al Sadd to HGH Pediatric ICU

1.3 To be exclusively used with all the following PHCC Primary Health Centers:

Abu Baker Al Siddiq HC	مركز أبو بكر آل صديق
Abu Nakhla HC	مركز ابونخله الصحي
Airport HC	مركز المطار الصحي
Al Daayen HC	مركز الظعابين الصحي
Al Jumailiya HC	مركز الجميلية الصحي
Al Kaaban HC	مركز الكعبان الصحي
Al Karaana HC	مركز الكرعانة الصحي
Al Khor HC	مركز الخور الصحي
Al Mashaf HC	مركز المشاف الصحي
Al Rayyan HC	مركز الريان الصحي
Al Ruwais HC	مركز الرويس الصحي
Al Sadd HC	مركز السد الصحي
Al Sheehaniya HC	مركز الشحانية الصحي
Al Thumama HC	مركز الثمامة الصحي
Al Waab HC	مركز الوعب الصحي
Al Wajbah HC	مركز الوجبة الصحي
Al Wakra HC	مركز الوكرة الصحي
Gharrafat Al Rayyan HC	مركز الغرافة الصحي
Leabaib HC	مركز لعبيب الصحي
Al Leghwairiya HC	مركز الغويرية الصحي
Madinat Khalifa HC	مركز مدينة خليفة الصحي
Mesaimmer HC	مركز مسيمير الصحي
Muaither HC	مركز معيذر الصحي
Omar Bin Al Khattab HC	مركز عمر بن الخطاب الصحي
Qatar University HC	مركز جامعة قطر الصحي
Rawdat Al Khail HC	مركز روضة الخيل الصحي
South Wakra HC	مركز جنوب الوكرة الصحي
Umm Al Seneem HC	مركز أم السنينم الصحي
Umm Ghuwailina HC	مركز أم غويلينا الصحي
Umm Slal HC	مركز أم صلال الصحي
West Bay HC	مركز الخليج الغربي الصحي

2. LIST OF HEALTH FACILITIES TO BE PROCESSED WITH MTPS: P-46 SCHEDULED INTERFACILITY TRANSFERS (ROUTINE)

2.1 To be used when a Patient is being transported to or from a medically supervised environment for the purpose of routine treatment, procedure, checkup, palliative care, or relocation. Continuity of current care may be required.

2.2 Examples:

- A patient transfer from Al Wakra Hospital ED to Rumailah Hospital ward
- A patient transfer from Al Khor Hospital ED to HGH Radiology for CT Scan

3. LIST OF HEALTH FACILITIES TO BE PROCESSED WITH MTPS: P-47 MENTAL HEALTH TRANSFERS

3.1 To be used with Patient with diagnosis or symptoms of mental illness being transported for the purpose of medical clearance, mental health evaluation, admission, routine appointment, or relocation.

3.2 Example:

- A patient transfer from HGH ED to Mental Health Hospital (With psychiatric complaint)

ENGLISH	ARABIC	Proposed MPDS Protocol (33/37/Others 1 to 32)	FINAL
II. LIST OF FACILITIES/CLINICS TO BE PROCESSED WITH MPDS PROTOCOL 33			
A. HMC Facilities			
a. Facilities			
Aisha Bint Hamad Al Attiyah Hospital	مستشفى عائشة بنت حمد العتيبة	37	
Mesaieed General Hospital	مستشفى مسيعيد العام	33	
Al Maha Pediatric Specialized Care Center	مركز المها للرعاية التخصصية للأطفال	33	
B. PRIMARY HEALTHCARE CORPORATION (PHCC) HEALTH CENTERS TO BE HANDLED BY HCC			
Al Mashaf HC	مركز المشاف الصحي	33	
Al Sadd HC	مركز السد الصحي	33	
Umm Al Seneem HC	مركز ام السنينم الصحي	33	
C. Private Clinics			
Marble Medical Center "Lusail"	مركز ماريبل الطبي	33	
Aman Hospital	مستشفى الأمان	33	
The View Hospital	مستشفى "ذا فيو"	33	
Alfardan Medical with Northwestern Medicine (AMNM)	الفردان الطبية و نورث وسترن مديسن	33	
D. QatarEnergy Clinics (formerly Qatar Petroleum "QP")			
Qatar Energy Health Center "QatarEnergy Headquarters"	مركز قطر للطاقة الصحي "المقر الرئيسي"	33	
F. Military Hospitals/Medical Centers			
Military Medical City Hospital (MMCH)	مستشفى المدينة الطبية العسكرية	33	

APPENDIX 3.22 INTER-FACILITY PATIENT TRANSFERS

				Valentia CAD Changes
	Response Priority	Chief Complaint - Change 01 June 2018	Chief Complaint - Change 01 Jan 2021	Chief Complaint Definition - Change 01 Jan 2021
INTERFACILITY TRANSFER SERVICE		IFT Heart		Cardiac Pathway, GHG to and from HH
	P1 IFT Emergency	IFT PEC Inside Doha		Al Sadd, Al Rayyan
	P2 IFT Urgent	IFT PEC Outside Doha		Other PEC's
	P3 IFT Routine	IFT Bariatric		Super Bariatric (+ 150 kg)
		IFT Airport		To or from airport
		IFT NICU Transfer		Transfer done by referring facility
		IFT NICU Retrieval		NICU Retrieval using Retrieval and NICU transfer team
		IFT PICU Transfer		Transfer done by referring facility
		IFT PICU Retrieval		PICU Retrieval using Retrieval and PICU transfer team
		IFT Team Transfer		Escort Provided
		IFT Upgrade in Care		e.g. Transported to more Acute Level or E/Ward to ICU
		IFT Transfer Admission		e.g. Ward to ward or service not available
		IFT Procedure Two way		e.g. Tracheostomy change or endoscopy, etc
		IFT Diagnostic Two way		e.g. MRI, CT scan
		IFT Assessment or Consultation		e.g. further EVALUATION or OPD
		IFT Capacity Management		Bed Management
		IFT Isolation		Patients moving to Isolation facility from healthcare facility
PATIENT TRANSPORT SERVICE		PTS Dialysis		Pre and post procedure transport on Dialysis centers
	P5 Scheduled	PTS Discharge		HGH, A&E, HH,CDC,NCCCR,RH,QRI,WWRC,AWH, ALKHOR, CUBAN
	(6 Unscheduled)	PTS Rehabilitation		Physiotherapy, Occupational therapy and Speech therapy
		PTS OPD Appointment		OPD consultations, review and its post transport
		PTS Pvt Hospital Appointment		Discharge cases, clinic cases, Procedure and after procedure transport
		PTS Ministry		Special request for Court, Police Station, MOPH and ISF clinic
		PTS Social		Schools, Special activities and Home to home transport
		PTS Routine Procedure		Tracheostomy clinic, Foot clinic, Warfarin clinic and Swallow Clinic
		PTS Procedure		Non elective procedure, as Endoscopy, colonoscopy, Radiotherapy, PET Scan, MRI, CT Scan, and Laboratory test
		PTS Post procedure		Elective and non elective procedures transport
		PTS Out on Pass		Patients are sending home for a short time by the permission of a doctor.
		PTS Others		Lifting the heavy patients, Special request by VIP
RETRIEVAL	P4 IFT Team	RTVL AMRT		International retrieval using A7-MED Aircraft
		RTVL Commercial		International aeromedical transfer of patients on commercial aircraft. E.g. Qatar Airways or other commercial carrier
		RTVL Patient Assessment		Initial patient visit to confirm eligibility for international travel.
		RTVL Patient Visit		Any subsequent patient visits conducted prior to patient travel (expatriation/repatriation).
		RTVL International		International Aeromedical retrievals using air ambulance or other private aircraft (e.g. Qatar Executive)
		RTVL SHAARP	RTVL CCP	Local CCP led retrieval cases (no doctor/escorting team aboard)
			RTVL SHAARP	Local adult high acuity transfers WITH a SHAARP Doctor
			RTVL Dr (non-SHAARP)	Local high acuity transfers with a doctor that is not a SHAARP doctor.
		RTVL ECMO		Immediate and scheduled ECMO retrievals
		RTVL Regional GCC		Expatriation/repatriation of patient within the GCC region using QEAF aircraft.
		RTVL Team Transfer		Movement of any medical team (HMCAS or other) without a patient, to facilities/airports/accommodations

APPENDIX 3.24 NCC-LF OFFSHORE CHECKLIST



OFFSHORE CASE CHECKLIST



IMPORTANT! this checklist must be completed for each, and every offshore case received by HMC NCC:			
- LifeFlight Coordinator must scan and save the completed document in the Clinical Desk share drive with the tracking sheet. - DTL must send the original copy to the Quality section for review and filing			
Date:		Shift:	
CFS No.:		Time:	
Call-taker Tasks			Yes No
1	Receive the request for an offshore case from Qatar Energy Call Center (A7S)		
2	Call-taker immediately notifies the on-duty DTL of an A7S offshore call.		
3	Obtain the caller's name and address of the patient (designated helideck name & code [e.g.: Delta Explorer – DEXP]).		
4	Accurately select/enter the designated helideck name and code in the NJM CFS.		
5	Reconfirm the designated helideck name and code by repeating this back to the caller.		
6	Identify the potential presence of Communicable Diseases / Hazardous Substance contamination		
	CD (+) Inform the DTL immediately.		
	CD (-) Complete the ProQA protocol 33.		
7	Inform A7S that the LFC will call them shortly for LF activation and close the call. Complete NJM entry and update DTL.		
LifeFlight Coordinator Tasks			Yes No
1	Open LifeFlight mailbox, retrieve the 'Offshore Retrieval Request' form and review key information		
2	Notify assigned LifeFlight unit and request them to start aircraft preparation for offshore response		
3	Open the relevant CFS check designated helideck name & code against the 'Offshore Retrieval Request'		
	<i>Once DTL confirms call-taking completed, LifeFlight Coordinator calls A7S</i>		
4	Add note in NJM of call received and confirm 'Offshore Retrieval Request' form received, reconfirm helideck name and code, refueling availability & location, <i>if contamination reported</i> check decontamination is being done		
5	Notify QEAF engineers & dispatch LifeFlight on Airmed 1, send 'Offshore Retrieval Request' form to flight team on MS Teams with additional important information		
6	Update DTL of LifeFlight dispatch and update NJM status		
7	Notify Oscar LifeFlight on-call of the case, case location and available patient information		
8	Confirm go/no-go, aircraft ETA and operational/logistical details – document in NJM CFS and Flight Mission Information and Tracking Form and update DTL		
9	Call Qatar Energy control room (A7S) and update them on mission plan and ETA		
10	Track mission progress using telecommunication/Skytrac and update NJM CFS and Flight Mission Information and Tracking Form		
NOTIFICATIONS			
DTL Task			Yes No
1	Follow progress with the call-taker (Helideck Name & Code accuracy & documentation in NJM)		
2	Coordinate between the call-taker and the LifeFlight Coordinator.		
3	Keep Oscar NCC updated on mission progress as necessary.		
Oscar NCC Task			Yes No
1	Ensure appropriate updates of the management on-call notification groups.		
2	Oversee the NCC management of the case and provides feedback to the staff if necessary and appropriate.		
Notes:			
Role	Name	Corp. No.	Signature

AS-Communications (NCC) & Clinical Desk - 26 April 2023

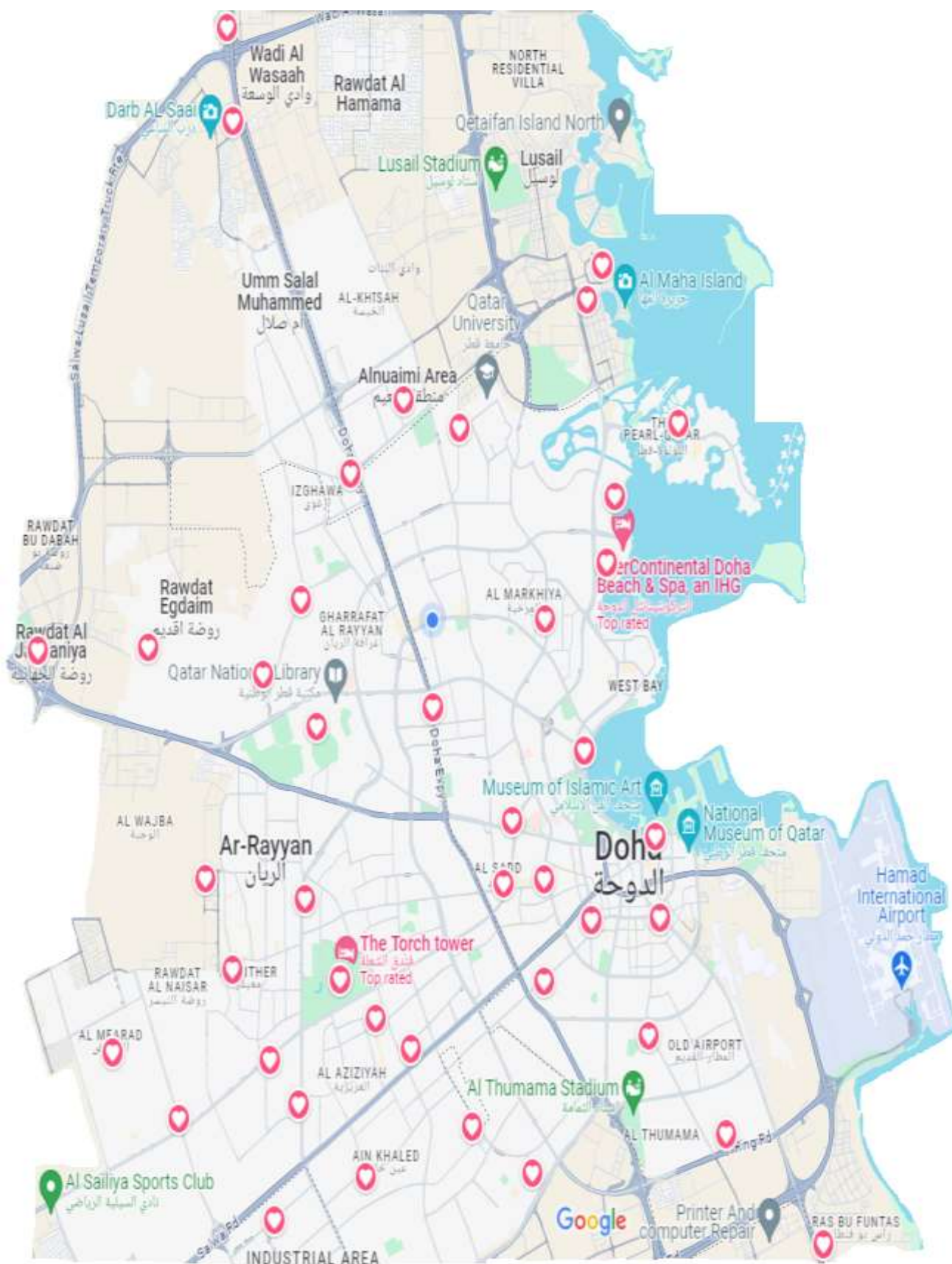
APPENDIX 4.4.A EMERGENCY UNIT DISTRIBUTION – CURRENT HUB AND SPOKE LOCATIONS

Hub	Spoke	Name	Latitude	Longitude	Type
Hub 1	1	Capital Police	25.2758	51.540525	Spoke
Hub 1	2	PEC Al Saad	25.283428	51.50527199	Spoke
Hub 1	3	The Mall	25.251967	51.537081	Spoke
Hub 1	4	Omar Bin Al Khattab PHCC	25.262933	51.505214	Spoke
Hub 1	5	Islamic Museum	25.291961	51.539153	Spoke
Hub 1	6	Fahad Bin Jassim	25.2952238	51.49551055	Spoke
Hub 1	7	AS Hub 1	25.27521401	51.51960499	Hub
Hub 1	Not Ranked	Al Saad PHCC	25.282553	51.492889	Spoke
Hub 2	1	Lusail Qatar Diar	25.39979444	51.51823889	Spoke
Hub 2	2	Umm Salal PHCC	25.43561389	51.41029444	Spoke
Hub 2	3	Immigration	25.31799345	51.47129174	Spoke
Hub 2	4	The Pearl Qatar	25.37493889	51.546125	Spoke
Hub 2	5	AS Hub 2	25.335675	51.50546111	Hub
Hub 2	6	Taibah	25.364831	51.446178	Spoke
Hub 2	7	Exhibition Rail	25.34698889	51.52426667	Spoke
Hub 2	8	Post Office	25.30923611	51.51743056	Spoke
Hub 2	9	Al Gharrafa PHCC	25.33958611	51.43106944	Spoke
Hub 2	10	Leabaib PHCC	25.379639	51.46235299	Spoke
Hub 2	24/7	Qatar Foundation Accommodation	25.32459722	51.41956389	Fixed
Hub 2	24/7	Qatar Foundation Education City	25.31407222	51.43587778	Fixed
Hub 2	Not Ranked	Lusail Vendome	25.40666667	51.52290833	Spoke
Hub 2	Not Ranked	Qatar University PHCC	25.37403333	51.47946111	Spoke
Hub 2	BIKE	Al Qutifia - Katara (Bike Team)	25.36032222	51.52684722	Spoke
Hub 4	1	Wajba PHCC	25.2835556	51.40202779	Spoke
Hub 4	2	Water Tank	25.247228	51.421653	Spoke
Hub 4	3	AL MEARAD	25.24864423	51.37369986	Standby
Hub 4	4	RAWDET GDAYEM	25.330055	51.3845467	Standby
Hub 4	5	Mall of Qatar	25.32936111	51.35089444	Spoke
Hub 4	6	Muaither PHCC	25.2353611	51.39391669	Spoke
Hub 4	7	AS Hub 4	25.27946111	51.43231944	Hub
Hub 4	8	Abu Baker Al-Sidiq PHCC	25.23818611	51.43041667	Spoke
Hub 4	9	AL HUDA SCHOOL	25.26534948	51.41027551	Standby

Hub 4	10	AZIZIA CIVIL DEFENSE	25.2555378	51.45385024	Standby
Hub 4	1B	Al Sheehaniya PHCC	25.36735	51.229228	Spoke
Hub 4	2B	Al Jumailiyah PHCC	25.616922	51.081892	Spoke
Hub 4	3B	Zekreet	25.45892222	50.8957	Spoke
Hub 4	4B	Dukhan	25.418819	50.782881	Spoke
Hub 4	5B	Umm Bab	25.209928	50.82857499	Spoke
Hub 4	BIKE	Aspire (Bike Team)	25.26331111	51.44297222	Spoke
Hub 4	Not Ranked	Jaryan Al Sahm (Near BUGARN)	25.386131	51.349535	Spoke
Hub 5	1	Grand Mall	25.190042	51.468992	Spoke
Hub 5	2	AIN KHALED	25.22367175	51.45091479	Standby
Hub 5	3	Al Hemaila Health Center-Red Crescent	25.16965701	51.39832899	Spoke
Hub 5	4	BARWA CITY	25.19699195	51.50927263	Standby
Hub 5	5	Mesaimeer	25.22446667	51.5017	Spoke
Hub 5	6	AL IKHLAS SCHOOL	25.1988436	51.36528667	Standby
Hub 5	7	STREET 47	25.16260891	51.43873676	Standby
Hub 5	8	AS Hub 5	25.21505833	51.42303056	Hub
Hub 5	9	Kharara Bridge	25.177808	51.32194199	Spoke
Hub 5	10	Qatar Racing Club Street 52	25.17391111	51.47137222	Spoke
Hub 5	11	Hazam Mebareek General Hospital	25.18013333	51.42894444	Spoke
Hub 5	12	Al Waab PHCC	25.24943889	51.46464167	Spoke
Hub 5	1B	Mekaines	25.137833	51.22796944	Spoke
Hub 5	2B	Al Karaana PHCC	25.008969	51.036647	Spoke
Hub 5	3B	Al Karaana (Suda Nathil before)	24.93725	50.99071944	Spoke
Hub 5	4B	Abu Samra	24.75665	50.85244167	Spoke
Hub 5	Not Ranked	Mesaimeer PHCC (MIR, SAS)	25.23373333	51.48331111	Spoke
Hub 5	MIR	LOGISTIC VILLAGE (MIR, MCC)	25.13148175	51.14970547	Spoke
Hub 6	1	AS Hub 6	25.682139	51.49896699	Hub
Hub 6	2	Al Dayyen PHCC	25.56369444	51.45384722	Spoke
Hub 6	3	UM SALAL ALI	25.4543175	51.40854834	Standby
Hub 6	4	Umm Al Amad Spoke	25.51854167	51.40758611	Spoke
Hub 6	5	RAS GAS	25.65198517	51.45490688	Standby
Hub 6	1B	Leghwairiya	25.84446944	51.26722778	Spoke
Hub 6	2B	Al Kaaban Spoke	25.87545278	51.34898889	Spoke
Hub 6	3B	Ras Laffan	25.92602222	51.44173056	Spoke
Hub 6	4B	Al Shamal PHCC	26.12453611	51.20641111	Spoke

Hub 6	Not Ranked	Al Shamal Spoke Station	26.12603056	51.20563333	Spoke
Hub 6	Not Ranked	Al Khor New PHCC	25.66765278	51.48509167	Spoke
Hub 7	1	Al Thumama PHCC	25.232306	51.56074999	Spoke
Hub 7	2	Ras Abu Fontas	25.21013889	51.59041111	Spoke
Hub 7	3	Wukair Kharrara	25.142947	51.512217	Spoke
Hub 7	4	AS Hub 7	25.17360001	51.59659999	Hub
Hub 7	1B	Mesaieed Solar Spoke	24.99216389	51.53531389	Spoke
Hub 7	2B	Sealine Location	24.88348889	51.52380278	Spoke
Hub 7	3B	Sealine Resort	24.86205556	51.51482778	Spoke
Hub 7	4B	Mesaieed Hospital	25.01309444	51.55963889	Spoke
Hub 7	Not Ranked	South Wakra PHCC	25.14999444	51.58821389	Spoke
Hub 7	Not Ranked	Sealine Clinic	24.85406944	51.50925	Spoke
Hub 7	Not Ranked	Al Mashaf	25.17630556	51.58136111	Spoke
Hub 7	Not Ranked	Al Mashaf PHCC	25.186109	51.56731	Spoke

APPENDIX 4.4.B MAP OF CURRENT HUB AND SPOKE DOHA



APPENDIX 4.4.C MAP OF CURRENT HUB AND SPOKE QATAR



APPENDIX 4.4.D UNIT AVAILABILITY ESCALATION PROCEDURES

Escalation Procedures AS 9032, 6.5	Actions
Unit Availability 30 %	➤ NCC OSCAR and OSCAR Command (jointly) to check the status of units delayed on scene or delayed at destination that can be expedited. ➤ Update Silver Command. ➤ Cease P2 IFT from PHCC and Private facilities and utilize SAS Resources Foxtrot units - priority units for being reassigned A1.11 (after 1500 hours) and A8.3.
Escalation to remain in place until a minimum of 30 minutes sustained unit availability >30%	
Unit Availability 25 %	➤ Recall NCC staff to the NCC/HCC - ceasing breaks for NCC/HCC staff.
Unit Availability 20 %	➤ All Actions as per unit availability of 30% plus: ➤ Announce low coverage as general broadcast on the Tetra radio (work with Distribution Team Leaders). ➤ OSCAR NCC to brief call-takers to ensure 'refer low acuity patients' as per PROQA protocol is optimized. ➤ Deploy Production OSCAR to spoke points - respond to P1 calls. ➤ Deploy Production Supervisor to spoke points - respond to P1 calls (unless within 60 minutes of end of shift). ➤ Oscar Command to instigate Cohorting at HGH and expedited handover at other units. ➤ OSCAR access to report to the ED with the 'greatest pressure' to support turn-around. ➤ Check the status of units responding to P2 cases that can be diverted to P1 cases (work with Clinical Desk). ➤ Extend EAS shifts (work with Production Team Leader). ➤ Extend NCC and HCC shifts. ➤ STOP all Interfaculty Transfers (unless critical transfer – clinical desk to re-triage). ➤ Review lower acuity patient transports from Facility to Home and vice versa QRI Physiotherapy, Hydrotherapy, Dentistry, Dermatology, and out-on-pass. ➤ Protocol 33 calls to be re-triaging by clinical desk and potential stood down. ➤ Use of A2.11.
Escalation to remain in place until a minimum of 30 minutes sustained unit availability >30%	
Unit Availability 15 %	➤ All Actions as per unit availability of 30% and 20% plus: ➤ Inform Gold Command. ➤ Silver Communication to attend NCC. ➤ Stop all PTS call except dialysis cases.
Escalation to remain in place until a minimum of 30 minutes sustained unit availability >30%	
Unit Availability 10 %	➤ All Actions as per unit availability of 30%, 20% and 15% plus: ➤ Rural ambulances to be redeployed 'closure' to urban area as per Silver Command. ➤ Silver Clinical to report to 1) ED to support turn-around of 2) NCC to support clinical desk.
Escalation to remain in place until a minimum of 30 minutes sustained unit availability >30%	

APPENDIX 4.7.B-1 DESIGNATED HELICOPTER OPERATING SITES

HOS No.	NAME	AREA	GPS COORDINATES
HOS 1	AL SHAMAL HEALTH CENTRE	SHAMAL/RUWAIS	N26° 08.17' E051° 12.23'
HOS 2	DUKHAN FOOTBALL FIELD	DUKHAN	N25° 25.00' E050° 46.93'
HOS 3	ABU SAMRA BORDER POST	ABU SAMRA	N24° 44.86' E050° 51.09'
HOS 4	MESSAIEED MEDICAL CENTRE	MESSAIEED	N24° 59.94' E051° 33.00'
HOS 5	DRAG RACING TRACK [STREET 52]	INDUSTRIAL AREA	N25° 10.46' E051° 28.51'
HOS 6	AL WAKRA HOSPITAL	WAKRA	N25° 09.96' E051° 35.39'
HOS 7	AISHA BINT HAMAD AL-ATTIYA HOSPITAL	TENBEK	N25° 36.48 E051° 27.73
HOS 8	SEALINE CLINIC	SEALINE	N24° 51.34' E051° 30.50'
HOS 9	RESTRICTED		
HOS 10	CLOSED AS/SD 21-16		
HOS 11	CLOSED AS/SD 21-16		
HOS 12	CUBAN HOSPITAL	DUKHAN	N25° 26.40' E050° 51.62'
HOS 13	QP – RAS LAFFAN	RAS LAFFAN	N25° 51.73' E051° 32.69'
HOS 14	CLOSED AS/SD 21-16		
HOS 15	RESTRICTED		
HOS 16	LUSAIL INTERNATIONAL CIRCUIT	LUSAIL	N25° 29.23 E051° 27.21
HOS 17	RESTRICTED		
HOS 18	AL KHOR HOSPITAL	AL KHOR	N25° 43.01' E051° 31.13'
HOS 19	RESTRICTED		
HOS 20	WEIGH STATION ON Q3 TO ZEKREET [DUKHAN 1]	ZEKREET INDUSTRIAL AREA	N25° 27.51' E050° 53.71'
HOS 21	WEIGH STATION ON Q5 TO ABU SAMRA	NEAR AL RAMZANI PETROL STATION	N24° 54.63' E050° 57.52'
HOS 22	WEIGH STATION ON Q5 TO DOHA	BETWEEN EXIT 42 & 51	N25° 04.13' E051° 06.88'
HOS 23	WEIGH STATION ON Q5 TO ABU SAMRA	PAST RAWDAT RASHED INTERCHANGE	N25° 08.67' E051° 14.88'
HOS 24	WEIGH STATION ON Q5 TO DOHA	PAST RAWDAT RASHED INTERCHANGE	N25° 08.61' E051° 15.01'
HOS 25	WEIGH STATION ON Q5 TO DOHA	AL SALIYA AREA	N25° 11.10' E051° 21.75'
HOS 26	WEIGH STATION ON Q5 TO ABU SAMRA	AL SALIYA AREA	N25° 11.19' E051° 21.74'
HOS 27	WEIGH STATION ON Q1 TO AL SHAMAL	BETWEEN EXIT 48 & 44	N25° 45.74' E051° 21.80'
HOS 28	WEIGH STATION ON Q1 TO DOHA	BETWEEN EXIT 29 & 24	N25° 32.31' E051° 24.73'
HOS 29	WEIGH STATION ON Q1 TO AL SHAMAL	UM SALAL ALI	N25° 28.18' E051° 24.54'

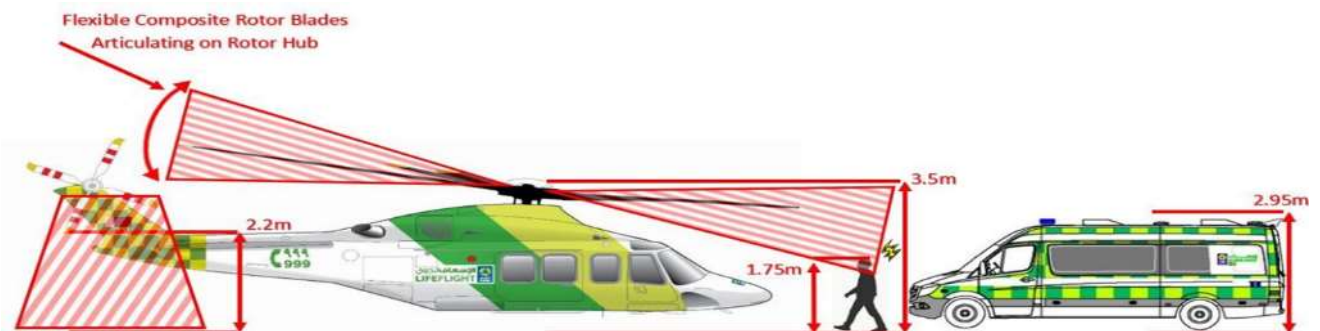
HOS 30	MILITARY AREA – OLD BORDER POST	GRID REF: 3J	N24° 32.35' E051° 4.93'
HOS 31	SEALINE DESERT AREA	SEALINE	N24° 47.20' E051° 27.05'
HOS 32	SEALINE DESERT AREA	SEALINE	N24° 41.43' E051° 24.87'
HOS 33	SEALINE DESERT AREA	SEALINE	N24° 33.68' E051° 20.45'
HOS 34	ANANTARA RESORT	BANANA ISLAND	N25° 17.80' E051° 38.33'
HOS 35	CLOSED AS/SD 21-16		
HOS 36	CLOSED		
HOS 37	CLOSED AS/SD 21-16		
HOS 38	CLOSED AS/SD 21-16		
HOS 39	CLOSED AS/SD 21-16		
HOS 40	HOTEL ECHO	AL RAYYAN	N25° 17.53' E051° 30.06'
HOS 41	CLOSED AS/SD 21-16		
HOS 42	CLOSED AS/SD 21-16		
HOS 43	CLOSED AS/SD 21-16		
HOS 44	CLOSED AS/SD 21-16		
HOS 45	HAMAD PORT ADMIN AREA	MESAIEED	N25° 02.25' E051° 34.78'
HOS 46	CLOSED AS/SD 21-16		
HOS 47	SALWA RESORT	ABU SAMRA	N24° 50.27' E050° 51.49'
HOS 48	SIDRA HOSPITAL	AL GHARAFA	N25° 19.46' E051° 26.76'
HOS 49	HAZM MEBAIREEK GENERAL HOSPITAL	MEBAIREEK	N25° 10.80' E 51° 25.76'
HOS 50	RAS LAFFAN HEALTH CENTRE	RAS LAFFAN	N25° 55.59' E51° 26.59'
HOS 51	RAS LAFFAN HOSPITAL	RAS LAFFAN	N25° 54.76' E51° 24.53'
HOS 52	MESAIEED HOSPITAL	MESAIEED	N25° 0.78' E51° 33.50'
HOS 53	CLOSED AS/SD 21-16		
HOS 54	CLOSED AS/SD 21-16		
HOS 55	ZULAL WELLNESS RESORT	AL RUWAIS	N26° 09.37' E51° 14.65'

HOS List Updated NOV2023

Important LifeFlight Helicopter Safety Rules



- Never approach unless escorted by flight crew
- Never walk near tail rotor area
- Never approach from an uphill slope
- Never direct bright lights at pilots or aircraft
- If directed by flight crew to assist, do so in a crouched manner keeping your head down
- Secure all loose objects in landing zone
- No unauthorized personnel near aircraft
- Protect yourself
- No smoking within 50 meters of aircraft
- No vehicles should approach within 50 meters unless directed by flight crew
- Under no circumstances will any vehicle drive underneath the helicopter blades
- **Follow flight crew instructions at all times**



Safe Approach to a Helicopter



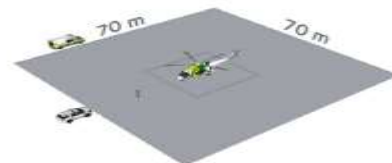
Beware of Rotor Downwash



Landing Zone Officer Signals



Landing area must be clear of Obstacles



TRAUMA BYPASS CRITERIA

If ONE or MORE of the listed criteria are met, you should immediately notify the CTL and transport patients **DIRECTLY** to HGH Trauma Resuscitation Unit, effectively bypassing the HGH RIAMS room/ Bay 1; and/ or bypassing other closer hospitals.

If NONE of the listed criteria are met, transport patient to closest hospital emergency department.

Any deviation from these criteria MUST be authorized through CTL.

ASSESS MECHANISM OF INJURY

Falls

- Adult fall: 6 meters (20ft or 2 stories) or higher
- Child fall: 3 meters or higher (10ft or 1 story) or twice patient's height
- Fall from horse, camel or similar type of animal

Motor vehicle crash with:

- Traumatic death in the same vehicle (except bus or mini-bus type of vehicle)
- Significant intrusion >30cm into the occupant compartment or >45cm at any site
- Ejection (partial or complete) from vehicle (including ATV-type vehicle)
- Person trapped under vehicle and/ or extrication >20min
- Rollover with roof deformity >40cm

Motorcycle accident with speed >30km/h

Pedestrian/ paediatric/ bicyclist struck by vehicle (thrown/ run over/ or with impact with speed >20km/h)

Burns with evidence of trauma OR >20% BSA in adults OR >10% BSA in children OR any significant neck/ facial burns with potential airway compromise.

Hanging or near-hanging with evidence of trauma (eg. hanging fall greater than height of patient or obvious spine deformity).

Drowning or near-drowning with evidence of trauma (eg. diving into shallow water).

ASSESS VITAL SIGNS AND LEVEL OF CONSCIOUSNESS

- Glasgow Coma Scale <15 (consider effects of alcohol or drugs)
- Sustained systolic blood pressure:
 - <90 in adults
 - <age-specific normal value in children
 - <110 if age >65 years.
- Respiratory Rate:
 - <10 OR >29 in adults
 - infant to 1 year old <20
 - any patient needing respiratory assistance

ASSESS ANATOMIC CRITERIA (PATTERN OF INJURY)

- Chest wall instability or deformity (eg. flail chest)
- Traumatic amputation above wrist or ankle
- Penetrating trauma to head, neck, torso or extremities above elbow or knee
- Open and/ or depressed skull fracture
- Suspected pelvic fracture/ unstable pelvis
- Neurological deficit in patients with spinal trauma
- Two or more fractures of either the humerus or femur
- Crushed, de-gloved, mangled or pulseless extremity

ASSESS SPECIAL PATIENT OR SYSTEM CONSIDERATION

Patients who have sustained significant trauma but do not fit any of the criteria above but are:

- Older patients (>65 years)
- Pregnant (>20 weeks)

Is there a reasonable clinical concern despite clearing all of the above criteria AND the CTL is in agreement?

APPENDIX 4.8.M KEY ROLES AND FUNCTIONS OF STAFF COHORTING

ROMEIO

- Ensure cohorting AP's are briefed
- Maximum of four patients to be cohorted unless instructed otherwise by GOLD command
- Ensure ePCR has all patients' details transferred
- Ensure that IMIST-AMBO form completed

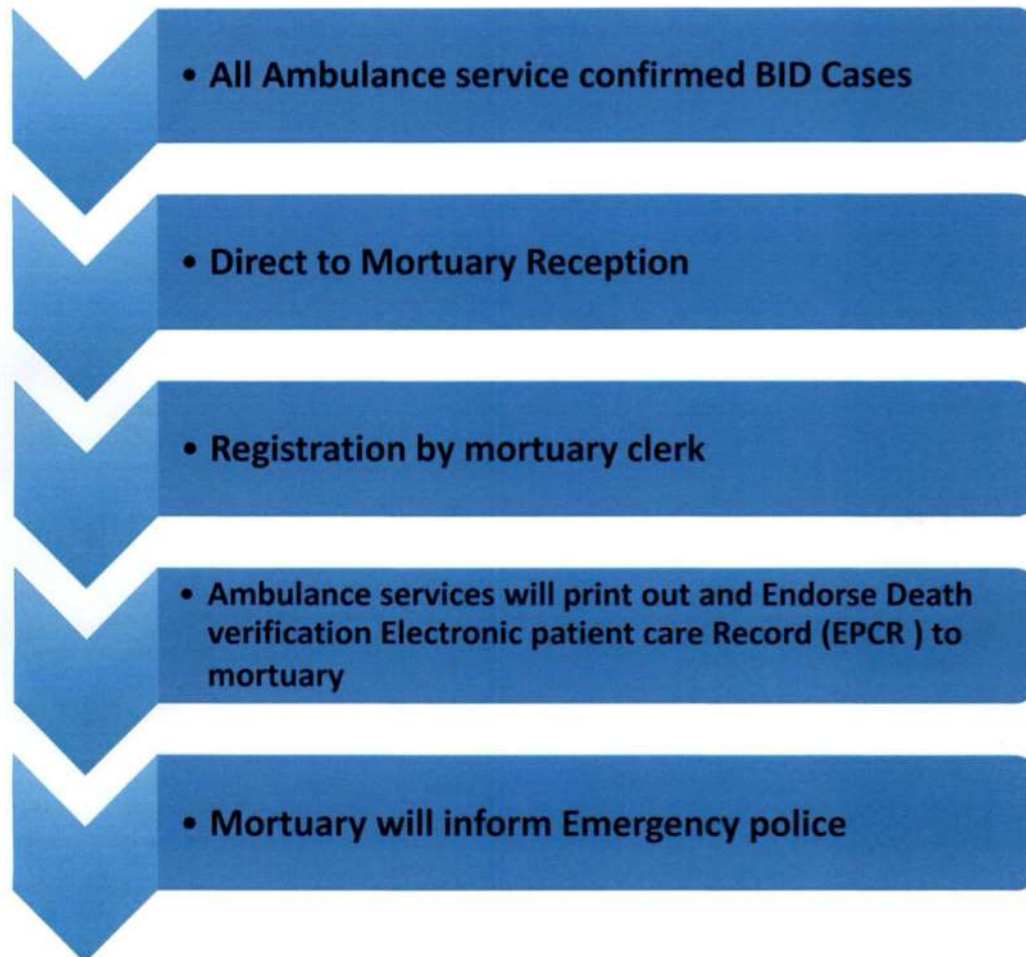
Clinical Documentation Officer

- If not wearing HMCAS identifiable uniform put on Hi-Vis tabard 'saying paramedic'
- Take handover from ambulance crew
 - Ensure ePCR is transferred to your tuff-pad/note book
 - Ensure IMIST AMBO form is completed
 - All medication
 - Complete set of vital signs
 - Crews call sing, date, and time
 - If monitoring is required attach a new LP15 allowing crew to be released
- Ensure you remain in the area, observing patients, at all times once cohorting is commenced UNLESS you are transporting patients to an Emergency Department bed
- Ensure you remain in the area, observing patients, at all times once cohorting is commenced
- Ensure ePCR has all patients' details transferred
- Ensure that IMIST-AMBO form completed
- If patients deteriorate act in accordance with CPG's and ensure events documented on ePCR

Cohorting Ambulance Team

- If not wearing HMCAS identifiable uniform put on Hi-Vis tabard 'saying paramedic'
- Take handover from ambulance crew
 - Ensure ePCR is transferred to your tuff-pad/note book
 - Ensure IMIST AMBO form is completed
 - All medication
 - Complete set of vital signs
 - Crews call sing, date, and time
 - If monitoring is required attach a new LP15 allowing crew to be released
- Ensure you remain in the area, observing patients, at all times once cohorting is commenced UNLESS you are transporting patients to an Emergency Department bed
- Ensure ePCR has all patients' details transferred
- Ensure that IMIST-AMBO form completed
- If patients deteriorate act in accordance with CPG's and ensure events documented on ePCR

BID ADMISSION PROCESS PATHWAY



Note: - All BID cases goes to nearest hospital (HGH, AL-Khor & AL-Wakra)

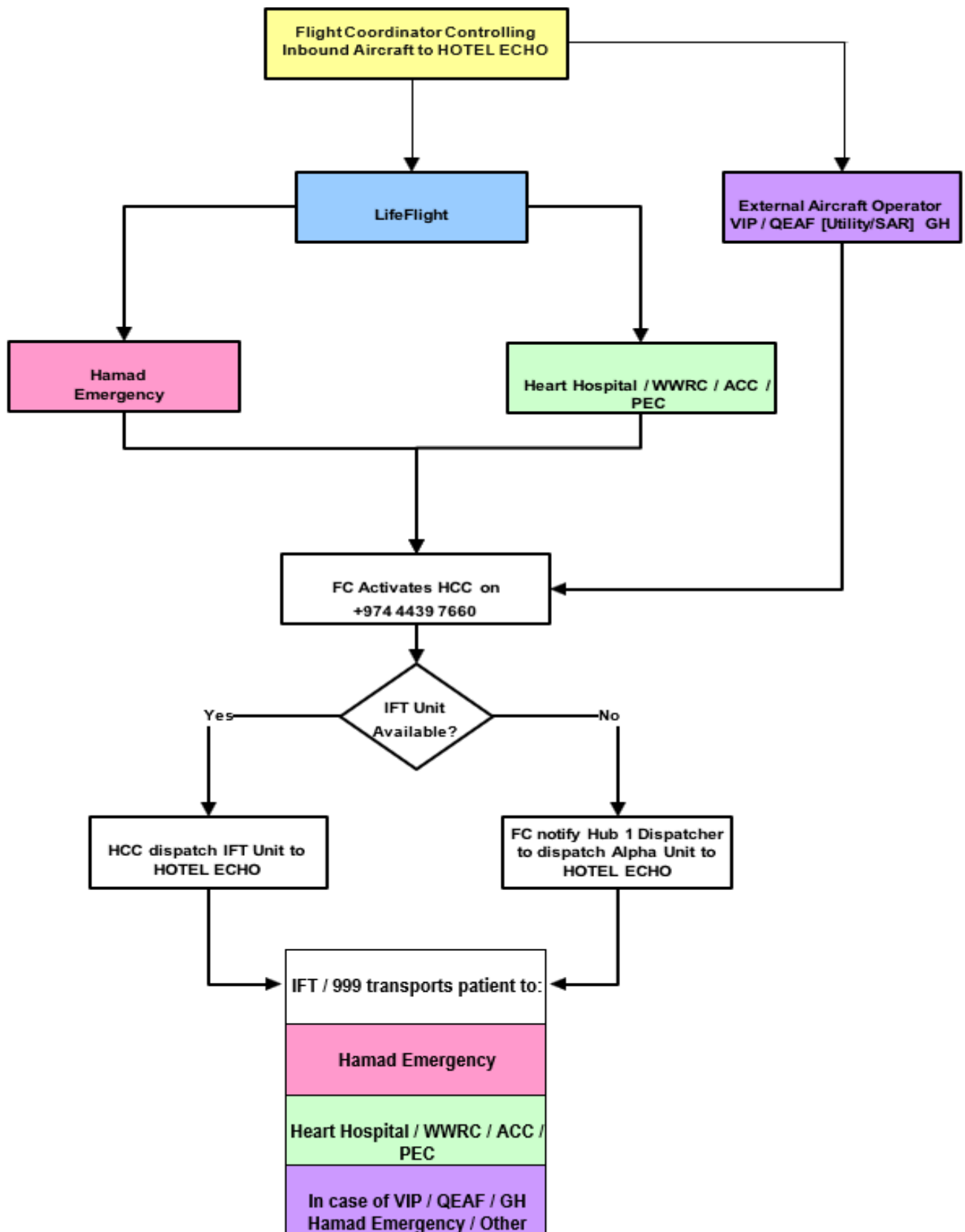
Dr. Loua H. Al Shaikh
Medical Director HMCAS

Dominic William Jenkins
Vice chairman ED-HGH

Sara Khamis Al Hamad
Manager Laboratory Services

Sara Khamis Al Hamad
Manager Laboratory Services
Dept. of Laboratory Medicine & Pathology
003947-HMC

APPENDIX 4.11.A.2 LIFEFLIGHT HOTEL ECHO HELIPAD ACTIVATION PROCESS





Medical Information Form (MEDIF)

MEDIF No:

Application for passenger requiring special assistance or medical clearance

No medical clearance or special type of form is required for those incapacitated customers who only require special assistance at the airport or during the embarkation or disembarkation process.

All questions must be answered in full.

Use BLOCK letters or a typewriter when completing this form.

Use a cross (X) in "Yes" or "No" boxes to indicate appropriate preference.

MEDIF to be completed in English by nominated physician.

MEDA approvals are for Qatar Airways operated flights only.

Notes for the guidance of a medical practitioner are on page 4 & 5. Cabin attendants are not authorized to give special assistance to particular passengers, to the detriment of their service to other passengers. Additionally, they are trained only in FIRST AID and are not permitted to administer any injections, or to give medication.

You must be reasonably satisfied that you are medically fit to fly. If, despite having received medical clearance, you subsequently do not feel fit enough to fly then please inform us immediately. Your booking continues to be subject to our fare rules/booking terms and Conditions of Carriage.

MEDIF forms must be submitted to Qatar Airways no less than 48 hours and no more than 7 days prior to the departure of each flight (outbound and return). In case no new MEDIF is required for the return journey then this will be indicated within Part 2 of this form as "Approved (full journey)."

Fees if any, relevant to the provision of the above information and for carrier-provided special equipment is to be paid by the passenger.

Customers are requested to check with the respective office for an approval prior to travel.

* For Doha (Qatar) station please submit the completed form to the Medical Counter at Qatar Airways Ticketing Implant office in the Ministry of Public Health Tower.

Medical Information Form (MEDIF)



QATAR
AIRWAYS
قطرية
GOING PLACES TOGETHER

PART 1 | MEDIF No:

To be completed by Authorised Agent or QR Sales Office Staff							
A	Passenger Name/Initials Title				Nationality	Languages	Contact Telephone No.
B	Date	Flight No.	From	To	Class	Reservation Status	Booking Reference (PNR)
							Note: Transfer from one flight to another often requires LONGER connecting time.
C	Nature of Incapacitation / Illness				Age	Stretcher Needed?	
					Incubator Needed?	Yes ☐ No ☐	
					Yes ☐ No ☐	All stretcher cases must be accompanied by medical escort	
					Type?	Oxygen Needed? Yes ☐ No ☐	
D	Intended Escort Details						
	Name						
	Additional Languages						
	Escort: Doctor ☐ Medical Team ☐ Nurse ☐ Family or non-medical ☐						
E	PNR of the Escort				Note:		
	Are there any special in-flight arrangements required? Special meals, special seating, extra seat(s), wheelchairs, equipment provision of special equipment for oxygen etc. Please complete Part 2 , if you have your own wheelchair, please specify: details (is it manual or power driven, collapsible, in-collapsible, battery type: spillable / Non-spillable).						
F	Ambulance Arrangement:				Has hospital admission been confirmed at destination port? Yes ☐ No ☐		
	Departure Port Yes ☐ Specify _____ No ☐				Hospital Details: (full name, address and telephone No.)		
	Transit Port Yes ☐ Specify _____ No ☐						
	Destination Port Yes ☐ Specify _____ No ☐						
*Note: All ambulance and hospital arrangements must be arranged by the treating doctor / hospital or family members.							
G	Additional Passenger Information						
H	Do you have a valid FREMEC card?				Yes ☐ No ☐		
	If yes, add below FREMEC data to your reservation requests.						
	If no, (or additional data needed by carrying airline(s)), has physician in attendance completed Part 2 overseas?						
	Number	Issued By	Valid Until	Incapacitation	Limitation		

In order to enable Qatar Airways to confirm my fitness to fly for my booked flights, I hereby consent for Qatar Airways Medical Examiner stated in Part 2 below to disclose the necessary information to Qatar Airways solely for the purpose of determining my fitness to fly and to fulfil my special assistance requirements and hereby release the attending doctor from his/her obligation to maintain my medical information strictly confidential with respect to the disclosure to Qatar Airways, and agree to meet such physician's fees in connection therewith. I reserve the right to revoke my consent to Qatar Airways to process my medical information at any time, but recognize that this may result in my being deemed not fit to fly and being therefore denied carriage on my booked flights. If the flight was already booked, there might be cancellation fees. I agree to notify Qatar Airways if there is any change in the status of my medical condition before my flight to avoid being refused for travel.

Yes, I have read and understood the above statement and hereby consent to the processing of my personal data/the personal data of the minor or person whose legal guardian I am.

Passenger or Guardian's signature

Signed

Date

أنا الموقع أدناه أفوض

(أدخل اسم الطبيب)

أرشد شركة الطيران بالمعلومات المطلوبة من قبل القسم الطبي لديهم لغرض تحديد لياقتي للسفر بطريق الجو وذلك أعني الطبيب من واجب الحفاظ على السرية المهنية بما يتعلق بهذه المعلومات ووافق على تسديد رسوم الطبيب البالغة في هذا الأمر.

وأبني على علم، حال قبول سفري، أن رحلتي ستكون خاضعة لشروط النقل والسعر وهذه الإجراءات المتبعة من قبل الناقل لا تعني تحمل أية مسؤولية خارج شروط السعر ووافق على دفع أية مصروفات وتكاليف تتعلق بسفري لشركة الناقل.

أوافق على إخطار الخطوط الجوية القطرية في حال حدوث أي تغيير في الحالة الصحية أو في الحجز وذلك لتجنب رفض الخطوط الجوية القطرية قبول سفري.

(هناك بحث العاج، يتم مراجعتها، تاريخها و توقيعها من قبل المسافر / أو من ينوب عنه)

أعد قرأت و استوعبت النموذج الطبي

توقيع المسافر أو وكيله

تاريخ

Medical Information Form (MEDIF)



PART 2 | MEDIF No:

To be completed by the PHYSICIAN ATTENDING the incapacitated passenger.										
REQUIREMENTS for Transportation:										
Wheelchair Needed?					Others?					
WCHR ☐		WCHS ☐		WCHC ☐		Stretcher ☐		Incubator* ☐		
(Can climb steps/walk cabin)		(Unable to climb steps/can walk cabin)		(Immobile)		Ventilator* ☐		AC ☐ DC ☐		
MEDA 01	Patient's Family Name/Initials:					Age		Sex F ☐ M ☐		
ATTENDING PHYSICIAN: (Name & Address)										
MEDA 02	Name of Hospital or clinic and specialty					Telephone (Business)				
Mobile:										
MEDICAL DATA: DIAGNOSIS in detail:										
Date of first symptoms:										
Date of diagnosis:										
Date of operation:										
Expected date of delivery:										
Vital Signs		BP		HB		LMP		HT WT		
PROGNOSIS for the flight:										
Narrative: (e.g. late stage disease, unstable, complicated/uncomplicated pregnancy)										
GOOD ☐ GUARDED ☐ POOR ☐										
(No problems anticipated) (Potential problems) (Problems likely)										
MEDA 05	Contagious and communicable disease?									
Yes ☐ No ☐ Specify:										
MEDA 06	Would the physical and/or mental condition of the patient be likely to cause distress or discomfort to other passengers?									
Yes ☐ No ☐ Specify:										
MEDA 07	Can patient use normal aircraft seat with seat belt placed in the upright position when so required?									
Yes ☐ No ☐ Specify:										
MEDA 08	Can patient take care of his/her needs onboard unassisted? (Including meals, visit to toilet, etc.)									
Meals Yes ☐ No ☐ Visit to toilet Yes ☐ No ☐ Specify:										
MEDA 09	If to be ESCORTED, by whom? Doctor ☐ Nurse ☐ Resp. Therapist ☐ Non-medical ☐ Passenger traveling alone ☐ Other (please specify):									
If not, state travel companion or proposed escort by you										
MEDA 10	Does the patient need "OXYGEN" equipment in flight? (If YES, please state rate of flow):									
Continuous Yes ☐ No ☐ Litres Per Minute (Between 2 to 8 LPM) Specify:										
MEDA 11	Does the patient need any MEDICATION, other than self-administered, and/or the use of special apparatus such as respirator, incubator, ventilator, nebulizer, etc.? (Note: all battery operated equipment on board must be dry or non-spillable, otherwise specify)									
MEDA 12	A) On the ground while at the airport(s) Yes ☐ No ☐ Specify:									
B) On board the aircraft Yes ☐ No ☐ Specify:										
MEDA 13	Does the patient need HOSPITALISATION, (if YES, indicate arrangement made, or if none were made, indicate "No action taken")									
(Note: The attending physician and/or Patient is responsible for all arrangements).										
MEDA 14	During long layover or at connecting points Yes ☐ No ☐ Specify:									
B) Upon arrival at destination Yes ☐ No ☐ Specify:										
MEDA 15	Other remarks or information in the interest of your patient's smooth and comfortable transportation									
Yes ☐ No ☐ Specify:										
MEDA 16	Other arrangements made by the Attending Physician:									
Please ensure that all above information is accurate. Once approved, no last minute changes will be entertained. Qatar Airways must be informed of any change in patient's status or requirement at least 48 hours prior to departure.										
I have read, understood and hereby agree to the conditions of the MEDIF form										
Attending Physician's Signature & Stamp					Place		Date			
Qatar Airways Medical Examiner					Remarks					
Approved (one way) ☐ Approved (full journey) ☐ Rejected ☐ Need Details ☐										
Requirements										
Doctor ☐		Stretcher ☐		Incubator ☐		Diaper/Pamper ☐		Sign & Stamp		
Nurse ☐		Wheelchair ☐		Oxygen ☐ LPM		Bassinet ☐				
Non-medical ☐		Ventilator ☐		Other ☐						

* Incubator / ventilator/ambulance arrangements are to be made by the treating doctor / hospital/family members. It is advisable to carry a universal multi-configuration adaptor to ensure compatibility of electrically operated medical equipments with electrical supply outlets on board the aircraft.

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Medical Information Form (MEDIF)



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PART 3

Reduced atmospheric pressure (Cabin air pressure varies greatly during 15-30 minutes after takeoff and before landing. Gas expansion and contraction can cause pain and pressure effects).

Reduction in oxygen tension (The cabin is at a pressure equivalent to an altitude of 6,000 to 8,000 feet and oxygen partial pressure is approximately 20% less than on the ground).

Any medical condition which would render a passenger unable to complete the flight safely without requiring extraordinary medical assistance during the flight is considered unacceptable for air travel.

Conditions usually considered UNACCEPTABLE for air travel (although these are suggested limiting factors, each individual case must be considered on its merits and is dependent on whether or not the passenger is accompanied by a professional escort).

- Anemia of severe degree
- Severe cases of Otitis Media and Sinusitis
- Acute contagious or communicable disease
- Those suffering from Congestive Cardiac Failure or other cyanotic conditions not fully controlled
- Uncomplicated Myocardial infarction within 2 weeks of onset complicated MI within 6 weeks of onset
- Those suffering from severe respiratory disease or recent pneumothorax
- Those with GI lesions which may cause hematemesis, Melena or intestinal obstruction
- Post operative cases:
 - a) Within 10 days of simple abdominal operations
 - b) Within 21 days of chest or invasive eye surgery (not laser)
- Fracture of the Mandible with fixed wiring of the jaw (unless medically) escorted
- Unstable Mental Illness without escort and suitable medication for the journey
- Uncontrolled seizures unless medically escorted
- Uncomplicated Single Pregnancies beyond 35 weeks
- Multiple Pregnancies beyond 32 weeks
- Infants within 7 days of birth
- Introduction of air to body cavities for diagnostic or therapeutic purposes within 7 days prior to air travel

Please carefully read the following frequently asked questions (FAQs) that will help you complete all the necessary information requirements. Failure to provide complete information to the airline will result in unnecessary delays in processing your case for air travel. We thank you for your understanding.

1. What is the purpose of Medical Information Form (MEDIF) ?

MEDIF is used for providing confidential information of passengers requiring special assistance. The information enables Qatar Airways Medical Centre to assess fitness of the passenger for air travel and to determine the use of medical equipment during travel e.g. stretcher, incubator, ventilator etc. It is therefore necessary that information (MEDA 01 to 16) required on MEDIF - PART 2, needs to be completely filled out.

2. What other supporting documents are required to be submitted by the passenger along with the MEDIF?

The documents to be submitted are (in English only):

- a) **Medical Report** - where necessary;
- b) If the passenger needs to be provided continuous oxygen then certification (on a specified format) is required from the attending physician that a non-medical escort/attendant travelling with the passenger is trained to administer oxygen;
- c) Sonography report where and when applicable

3. When do I need to submit a Medical Report along with a MEDIF and in which situations only a Medical Certificate (without MEDIF) will be sufficient?

Medical Report along with MEDIF:

A detailed Medical Report is required under the following situations.

1. Ventilator, Incubator, Stretcher Cases
2. Passenger requiring medical oxygen on board
3. Diabetics with unstable / complicated conditions
4. Asthmatic sufferers requiring oxygen on board
5. Single uncomplicated Pregnancy cases (beginning of 33rd week up to and not beyond 35 weeks)
6. Wheelchair (for completely immobile passengers with recent surgery / medical treatment / illness)
7. Others - where required by QR Doctor

Medical Information Form (MEDIF)



QATAR
AIRWAYS
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GOING PLACES TOGETHER

PART 4

Medical Certificate is sufficient and MEDIF is not required:

A Medical Certificate is required from a qualified doctor for the following.

1. Beginning of 29th week up to 32nd week of single uncomplicated pregnancy
2. Controlled Diabetic passengers on insulin injections
3. Passengers having recent leg fracture with a HALF PLASTER CAST (i.e. boot type - applied below the knee)
4. In case of any recent communicable or infectious ailment. The certificate must state whether the passenger is fit or unfit to air travel and should include precautionary measure/s if necessary.
5. Passengers using portable kidney dialysis machines and/or any medical equipment on-board provided the medical condition is stable and to make advance notice at least 48 hours prior to the date of travel.
6. Medical Certificate must be dated within 10 days of the flight date.

4. What is the difference between a Medical Report and a Medical Certificate?

A Medical Report is more detailed containing diagnosis, a summary of hospitalisation and recent general condition of the patient. A Medical Certificate includes brief diagnosis and states whether the passenger is fit for air travel and must state if any precautionary measures are required.

5. Which language should be used for MEDIF, Medical Report or Medical Certificate?

ENGLISH language is a must for MEDIF, Medical Report or Medical Certificate.

6. Does the Attending Physician OR Qatar Airways Medical Centre approve air travel on Qatar Airways flight?

Qatar Airways Medical Centre is the final authority to approve air travel for any patient. The patient's Attending Physician provides details of patient's medical condition, which help the airline's Doctors in assessing whether the patient is fit for air travel under specific conditions.

7. Which section of MEDIF needs to be filled out by passenger's Attending Physician?

MEDIF - PART 2 needs to be filled out completely by passenger's Attending Physician duly signed and stamped.

8. Why is it important that ALL the information in the section (MEDIF - PART 2) must be completely filled out by the Attending Physician ?

Complete and clear information provided by patient's Attending Physician will help Qatar Airways Doctor to approve the case quickly. For a MEDIF with incomplete and vague information, Qatar Airways Doctor may contact the patient's attending physician for clarification, which will result in delays.

9. What causes major delays in processing MEDIF application?

It has been observed that INCOMPLETE or VAGUE information provided within MEDIF is normally the cause of major delays in processing the application.

10. What are the validity requirements for Medical Certificate and MEDIF form?

- A Medical Certificate must be dated within 10 days of flight date.
- An application using MEDIF must be completed and submitted to Qatar Airways no less than 48 hours and no more than 7 days prior to flight departure.

11. Do I need a new MEDIF for my return journey?

Depending upon the medical condition of the passenger, Qatar Airways may require a new MEDIF prior to the return journey. In case no MEDIF is required for the return journey then this will be indicated within Part 2 as "Approved (full journey)". Although, any change in the medical condition of the passenger after the approval process, should be notified to Qatar Airways at least 48 hours before travel.

Medical Information Form (MEDIF)



Other Important Information:

- a)** Wheelchairs can be provided at most airports if notice is given.
- b)** Any case that Qatar Airways considers to result in jeopardizing the safety or operation of the aircraft will not be accepted by Qatar Airways.
- c)** Particular attention is drawn to the fact that the medical details given at the front of this form must be accurately typed and completed. If at the time of embarkation the condition of the passenger is not as good as the medical details provided, then the air carriage may have to be refused.
- d)** To ensure that your patient is accepted for carriage, it is important that all medical terms provided by the attending physician must be easily readable.
- e)** Wheelchairs with spill able batteries are "Dangerous Goods" and are permitted on passenger aircraft only under certain conditions which can be checked from the airline. In addition certain countries may impose specific restrictions.
- f)** For pregnancy cases, the Attending Physician must specify "Single/Multiple" and "Complicated/Uncomplicated" in Diagnosis in details within MEDA Q3.
- g)** Mentally Deficient passengers. No MEDIF & Medical Report required if the passenger has a pre-approved Frequent Traveler's Medical Card (FREMEC) or in stable condition
- h)** In case of pregnancy, please specify age of gestation and if it is uncomplicated single or multiple / complicated pregnancy. Expectant mother has to be "Fit to Travel" for the time covering the entire journey with no intended/voluntary stopover at the transit point. Stopover is permissible provided the entire period of air travel is within the acceptable number of gestational weeks (not over 32 weeks for Multiple Pregnancy and not over 35 weeks for single pregnancy).

Equipment	Additional Information / Restriction
Stretcher	Weight Limit : The stretcher is designed to bear any passenger weight.
Medical Oxygen	Flow Rate : 2 to 8 Litres per Minute (LPM) Mask : • Standard – Adult-size disposable oxygen therapy mask • Options – Child-size therapeutic mask
Medical Outlet (Electrical)	Airbus* : 115V / 5A / 60 Hz and 220V / 3A / 50 Hz Boeing : 115V / 15A / 60 Hz and 220V / 15A / 50 Hz

I have read and understood this MEDIF FORM

Date
Signature of Passenger or Parent/Legal Guardian

*A319/A320/A321's are not equipped with Electrical outlets.

*All other aircraft are equipped with medical power outlet in Economy Cabin only

*Passenger travelling in Business or First Class Cabin who needs to use medical equipment onboard must ensure that such device is battery operated and has sufficient power to last for the flight duration

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APPENDIX 5.4.A.3.2-2 MEDICAL ATTACHÉ REQUEST FORM

RETRIEVAL SERVICES



خدمة الإسعاف
Ambulance Service

Date:

Dear (Name of Attaché)

The following patient, (Patient name) will be escorted to (Destination) on the (Date). The patient will be traveling with (Nr) family escorts and (Nr) Medical Team members.

Details are as follows:

Date of Departure	ETD (Local Time)	ETA (Local Time)	Airline	Flight number

Please confirm the following is arranged:

1	Ambulance from Airport to Hospital	
2	Ambulance from Hospital to Airport	
3	Transport for Escorts from Airport to Hospital	
4	Transport for Escorts form Hospital to Airport	
5	Hospital Bed Booking Confirmation	
6	Medilift on Arrival	
7	Medilift on Departure	
8	Transportation of Medical Team from Hospital to Hotel	
9	Transportation of Medical Team from Hotel to Hospital	
10	Transportation of Medical Team from Airport to Hotel	
11	Transportation of Patient from Airport to Hotel.	

Thank you and regards

(Name of RD on duty)

Signature:

RS.O.0009v1

Tel: (+974) 55078622
Landline: (+974) 44397368
Email: Retrievals@hamad.qa

P.O.Box 3050
Doha, Qatar
www.hamad.qa

APPENDIX 5.4.A.3.2-3 MEDICAL TEAM MEMBER EMERGENCY CONTACT FORM



Medical Team Member Emergency Contact Details

Unforeseen emergencies can arise during your mission abroad. In order to help ensure your well-being during your travels, and enable an effective response from the corporation if an emergency occurs, please provide the below details.

Rest assured this information will be treated with the highest confidentiality.

Once Completed please send this form to: retrievals@hamad.qa, please include the name of the patient you will be travelling with or their GOAL ID number if known.

Personal Details	
Name	
Corporation Number	
Contact Number While Abroad	
Contact address while abroad (if known) or name of hotel	
Private Medical Insurance Company Name (if applicable)	

Emergency Contact Details	
Emergency Contact Person Name	
Relationship <i>(e.g. Spouse/father/mother)</i>	
Primary Contact Number	
Alternative Contact number	
Contact email	
Physical Address	

APPENDIX 5.4.A.3.2-4 MISSION TASKING FORM



CLINICAL DESK I RETRIEVAL

International Retrieval Tasking

Approval for the international travel of HMC Ambulance Service employees with Hamad Medical Corporation equipment and medical consumables for the purposes of patient retrieval and medical care.

MISSION DETAILS

Goal ID:	From:	To:
Patient HC/ID:	Patient Name:	
Requesting Agency & Contact:		

ITINERARY

Departure Date:	Departure Time:	Departure Flight No.
Return Date:	Return Time:	Return Flight No.

Mission Type: ☐ Commercial ☐ A7-MED ☐ QEAF ☐ LifeFlight ☐ Other: _____

Retrieval Type: ☐ Expatriation ☐ Repatriation ☐ International Transfer I ☐ Seated ☐ Stretcher

CREW

Corp #:	Name:	Mobile #:
Corp #:	Name:	Mobile #:
Corp #:	Name:	Mobile #:
Corp #:	Name:	Mobile #:

CLINICAL

Primary Diagnosis:	
Specific Clinical Concerns/Risks:	
Approving Doctor:	Approval Date:

HMC AMBULANCE SERVICE APPROVAL

Approved by: _____

Designation: _____

Date: _____

Corp #: _____

Signed:

--

Internal Use Only

1 of 1

CD-D022.2.0(20230213)

APPENDIX 5.4.A.3.2-7 ACKNOWLEDGEMENT OF AGREEMENT ON HIGH RISK TRAVEL



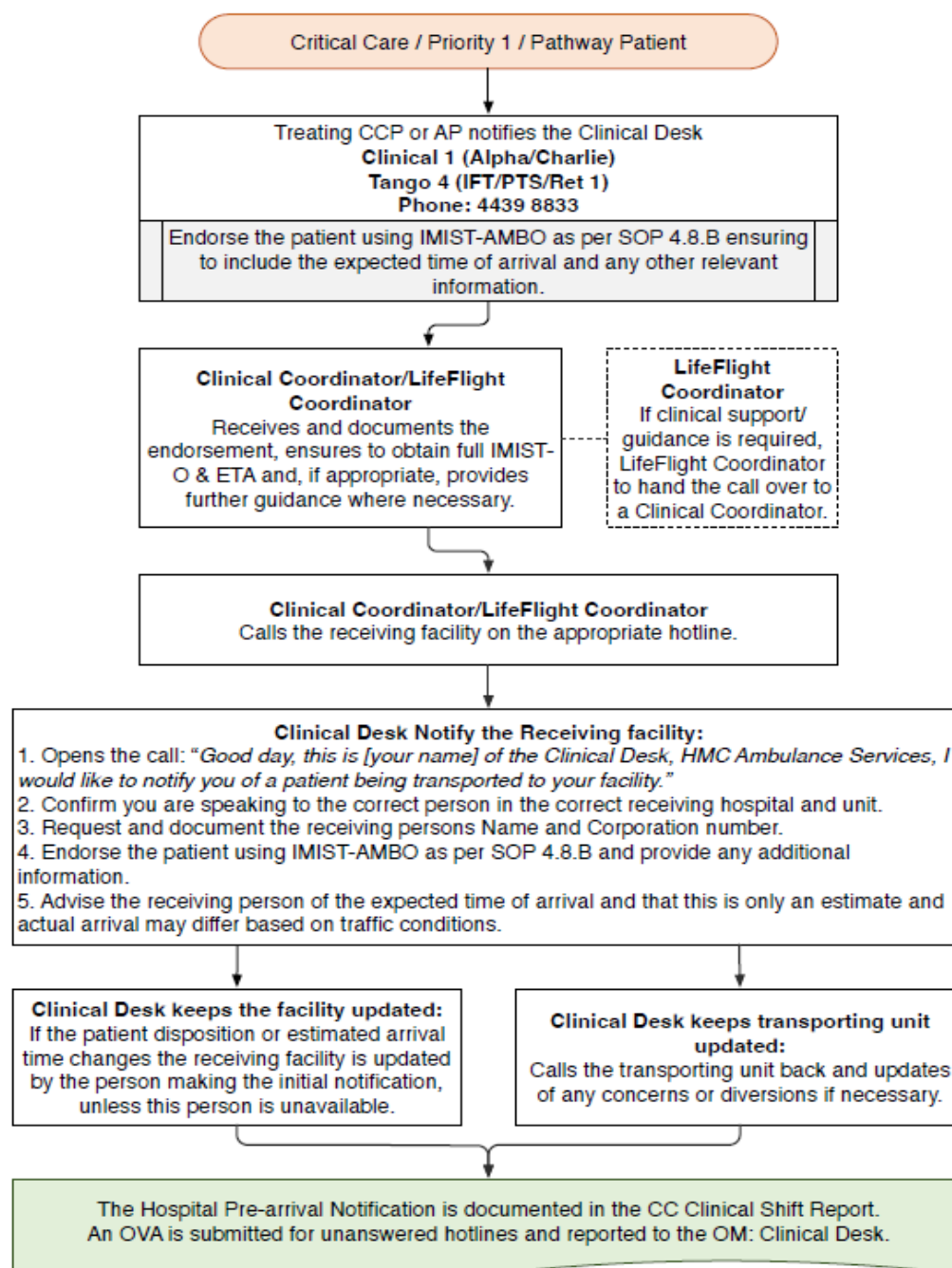
Acknowledgment of Agreement on High-Risk Travel

HC NO.									
PATIENT NAME									
NATIONALITY									
DATE OF BIRTH		/		/					
AGE		GENDER ☐ MALE ☐ FEMALE							

Facility: ☐ HGH ☐ WWRC ☐ RH ☐ NCCCR ☐ AKH ☐ HH ☐ AWH ☐ TCH ☐ CDC ☐ QRI
☐ ACC ☐ HMGH ☐ MHS ☐ HHCS ☐ PNS ☐ Other:

Acknowledgment	أقرار
Patient Health Status	الحالة الصحية للمريض/المريضة
The current patient's medical condition, is at high-risk, does not fit to international travel, but insisting to travel.	نظراً للحالة الصحية للمريض حالياً معرضة لمخاطر عالية وغير ملائمة للسفر الدولي بالطائرة إلا أن المريض/المريضة يُصر على السفر .
The medical transportation team has explained clearly to me, the possibility of in-flight medical incidents / emergencies, which cannot be excluded during commercial air travel on Qatar Airways from Qatar to and the flight is extremely high risk.	قام فريق النقل الطبي بشرح الحالة لي بوضوح حول إمكانية وقوع حوادث أو مستجبات طبية طارئة على متن الطائرة والتي لا يمكن تفاديها أثناء السفر الجوي التجاري على متن الرحلات الجوية القطرية من قطر إلى والرحلة عالية الخطورة .
I acknowledge that I have been fully informed of this agreement, that no attempts to divert the aircraft or perform an emergency landing shall be undertaken by the medical team and Qatar Airways In the event of an in-flight medical incidents / emergencies or death of:	أقر بأنني على علم تام بهذه الاتفاقية وأنه لن يتم القيام بتحويل مسار الطائرة أو القيام بهبوط اضطراري من قبل الفريق الطبي وطاقم الخطوط الجوية القطرية في حال وقوع حوادث / حالات طوارئ طبية على متن الطائرة أو وفاة .
Patient' Name:	اسم المريض :
HC Number:	رقم الملف الطبي للمريض :
I accept and agreed to these conditions of flight and release the escorting medical team and Qatar Airways of any liability:	أقر وأوافق بمحض إرادتي على شروط الرحلة ولا أحمل الفريق الطبي المرافق والخطوط الجوية القطرية من أية مسؤولية .
Signature	التوقيع
Name:	الاسم :
Relationship:	العلاقة :
QID number:	رقم البطاقة الشخصية القطرية :
Signature:	التوقيع :
Date and Time:	التاريخ / الوقت :

APPENDIX 8.8.1 HOSPITAL PRE-ARRIVAL NOTIFICATION PROCESS



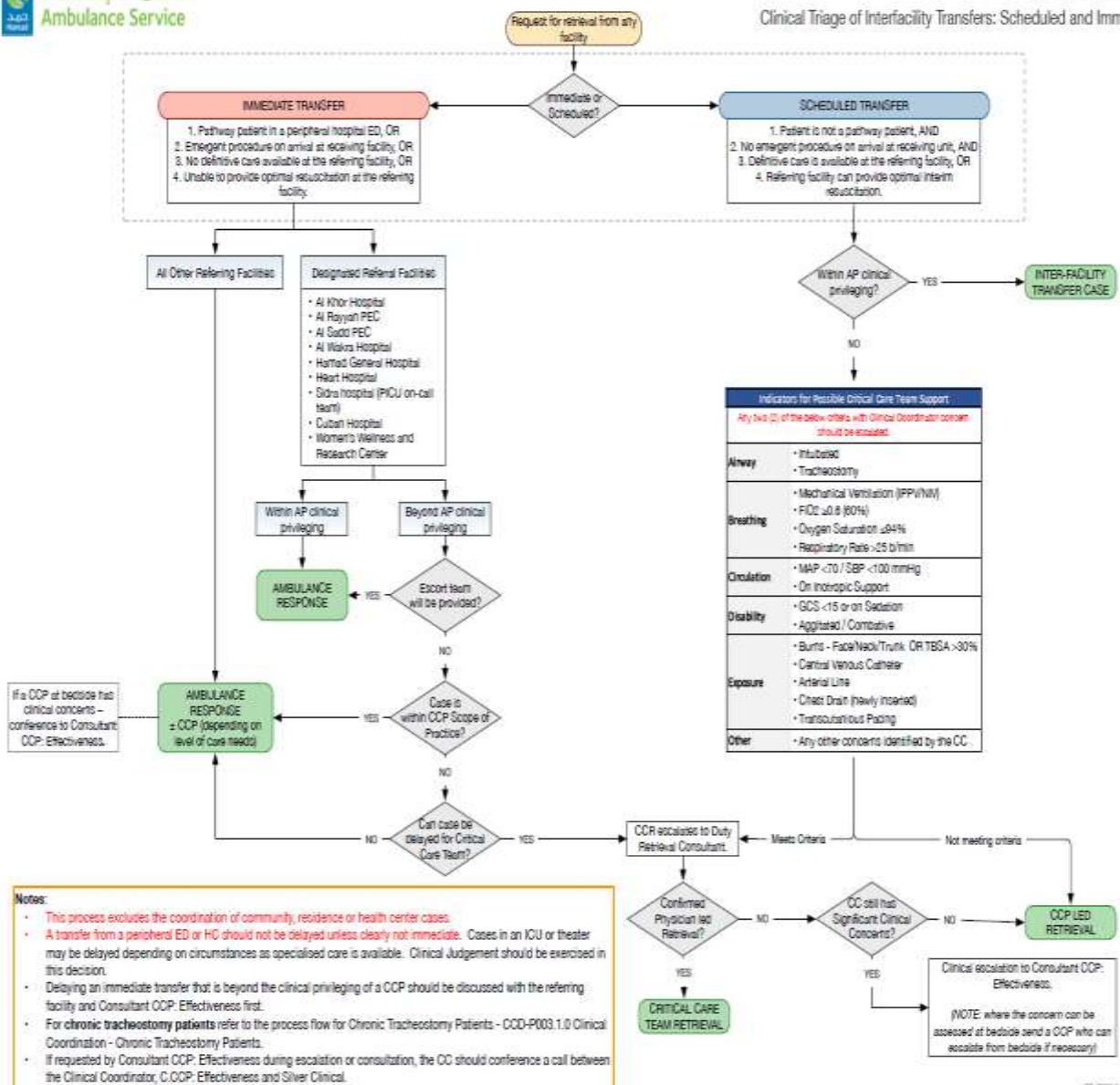
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APPENDIX 8.9.A CLINICAL TRIAGE OF INTERFACILITY TRANSFERS: SCHEDULED AND IMMEDIATE PROCESS FLOW.



CLINICAL DESK

Clinical Triage of Interfacility Transfers: Scheduled and Immediate



CC-P003.1.1 (20220308)

APPENDIX 8.9.B CLINICAL DESK TRANSFER TRIAGE MATRIX

CLINICAL DESK Transfer Triage Matrix

NOTE: This is a guideline and is targeted at the major hospitals (HGH, HH, NCCOR, AWH, AKH, TCH etc.) and does not apply to HC or PHCC facilities.
NOTE: Health Centre's and PHCC's must be managed at a community case with the Clinical Coordinator advising on CCP dispatch, if required, for PHCC's only.

	LOW ACUTY Clinical complexity or ongoing medical care is within the privilege of an Ambulance Paramedic	HIGH ACUTY Clinical complexity or ongoing medical care is within the privilege of a Critical Care Paramedic	CRITICAL CARE Clinical complexity or ongoing medical care exceeds the privilege of a Critical Care Paramedic	Valencia CAD Priority
IMMEDIATE TRANSFER a) Param patient in peripheral facility; b) Referral facility is unable to provide urgently required procedure, definitive care or resuscitation For immediate dispatch P1 Examples: STEMI, CVA, Trauma Bypass, ICH needing urgent neurosurgical intervention, Failed reperfusion needing rescue PCI	1) 999 Alpha unit Nearest available Alpha unit	1) 999 CCP unit (Charlie/LifeFlight) 2) Retrieval unit • Nearest appropriate 999 CCP unit (Charlie/LifeFlight). • Use Retrieval unit whenever possible (i.e. sending Retrieval from Dona to AWH is preferred over sending a Dona Charlie). • Apply dual response strategy if in Hamad Medical City, AKH, CSH (if possible).	1) Retrieval unit 2) 999 CCP unit (Charlie/LifeFlight) • Nearest appropriate CCP unit (Retrieval (first option) (Charlie/LifeFlight)). • Use Retrieval unit whenever possible (i.e. sending Retrieval from Dona to AWH is preferred over sending a Dona Charlie). • If no possibility to delay for Retrieval without causing patient harm - referring facility should provide an escort. • Apply dual response strategy if in Hamad Medical City, AKH, CSH (if possible).	Emergency <15min P1 dispatch
SCHEDULED TRANSFER URGENT SCHEDULED a) Although not immediate the patient requires an upgrade in care as an urgency and cannot wait for an extended period of time Target dispatch time <12 hours Examples: NSTEMI needing PCI (not a rescue PCI), Pneumonia in Respiratory Distress but stable with likelihood of deterioration if delayed more than 3-hours. Septic patient being resuscitated at the referring facility	1) 999 Alpha unit 2) IF unit Next available appropriate unit	1) Retrieval unit 2) 999 CCP unit (20:00-08:00 - C.CCP.E Escalation req) • Next available appropriate 999 CCP unit (Charlie/LifeFlight). • Apply dual response strategy if in Hamad Medical City, AKH, CSH (if possible). • Use Retrieval unit whenever possible (i.e. sending Retrieval from Dona to AWH is preferred over sending a Dona Charlie). • Priority of dispatch should be balanced against service availability and patient needs.	1) Retrieval unit 2) 999 CCP unit (20:00-08:00 - C.CCP.E Escalation req) • Next appropriate CCP unit (Retrieval (first option) (Charlie/LifeFlight)). • Apply dual response strategy if in Hamad Medical City, AKH, CSH (if possible). • Use Retrieval unit whenever possible (i.e. sending Retrieval from Dona to AWH is preferred over sending a Dona Charlie). • Priority of dispatch should be balanced against service availability and patient needs. • CC: Retrieval should escalate to the DRC prior to dispatch if appropriate. • CC: Retrieval should escalate to Silver Clinical if they have specific clinical concerns and where a CCP will transfer the patient without any escort.	Immediate / Urgent <1hour P3 / <30 min P2
SCHEDULED a) Care can be scheduled for transfer based on unit availability Target dispatch time <12-hours Examples: ICU patient going to another facility for best management reasons; Patient transfer to another facility for scheduled procedure the following day; transfer for step down or long-term care.	IF unit Schedule based on appropriate unit availability	1) Retrieval unit 2) 999 CCP unit (20:00-08:00 - C.CCP.E Escalation req) • Use Retrieval unit whenever possible • Where there are 22 pending transfers between 20:00 & 08:00 (pending for dayshift) the CC: Retrieval should consult with Consultant CCP. Effectiveness based on Charlie unit availability • Transfer scheduling should be balanced against service availability and patient needs.	1) Retrieval unit 2) 999 CCP unit (20:00-08:00 - C.CCP.E Escalation req) • Use Retrieval unit whenever possible • Where there are 22 pending transfers between 20:00 & 08:00 (pending for dayshift) the CC: Retrieval should consult with Consultant CCP. Effectiveness based on Charlie unit availability • Transfer scheduling should be balanced against service availability and patient needs. • CC: Retrieval should escalate to the DRC prior to dispatch if appropriate. • CC: Retrieval should escalate to Silver Clinical if they have specific clinical concerns and where a CCP will transfer the patient without any escort.	Pre-scheduled / Scheduled Next day / 14- hours P4/5/6
Level of care	AMBULANCE PARAMEDIC	CRITICAL CARE PARAMEDIC	CRITICAL CARE TEAM	
Level of care: Ambulance Paramedic: IF or Alpha Ambulance Critical Care Paramedic: Retrieval, LifeFlight or a Charlie unit Critical Care Team: CCP unit with an ICU doctor, PICU/ICU team, ECMO team etc.				

SOP ... Appendix ver-1.0, DRAFT 20200306



PHCC ESCALATION ADDITIONAL QUESTIONS – EMD: RETRIEVALS COORDINATOR

Check that the patient's HC number, Name, Age, Gender and Nationality are captured in CAD. *If not, you must confirm this from the referring facility and document it in CAD.*

1. **Confirm the patient's identity** using the QID/HC number and Name.
2. **(If not documented in CAD)** Confirm the patient's **age, gender, and nationality**.
3. What is the **current diagnosis/chief complaint?** (*Why is the patient being seen at the HC?*)
 - a. **(If chest pain)** Is there **ongoing** chest pain **now**?
 - i. Has an **ECG** been done on the patient?
 1. **(Yes)** Are there **ECG changes identified** by the doctor?
4. When did the current **symptoms/complaint start**?
5. What are the patient's current **vital signs?** (*Heart Rate, Blood Pressure, Respiration Rate, Oxygen Saturation, Level of Consciousness*)
6. What **treatment** is the patient **receiving**?
7. **Why** is the patient being **referred** to hospital now?

EMD: Retrievals Coordinator should document this additional information and endorse it to the Clinical Coordinator for a clinical decision on the case or, if necessary, further clinical interrogation.

NOTE: the intention is not to re-ask questions asked during ProQA or to obtain detailed information, rather it is to obtain basic information not elicited in ProQA that can help identify the need for a CCP dispatch.

